

Auszug aus der UAW-Datenbank des BfArM

Stand der Datenbank: 07.03.2016

Einzelfallausdrucke

Diclofenac: Inland, ab 1992, schwere Hautreaktionen

Hinweise zur Interpretation der Daten:

Es handelt sich bei den Berichten sowohl um Spontanmeldungen als auch um Berichte aus systematisierten Untersuchungen, aus Projekten oder aus der Literatur. Die Meldungen sind dem Bundesinstitut für Arzneimittel und Medizinprodukte durch pharmazeutische Unternehmer, durch die Arzneimittelkommissionen der Heilberufe, direkt durch niedergelassene oder in Krankenhäusern tätige Ärzte sowie durch sonstige Meldequellen, z. B. von Patienten, berichtet worden. Es muss beachtet werden, dass es sich um Verdachtsfälle unerwünschter Arzneimittelwirkungen handelt, dass also ein Kausalzusammenhang im Einzelfall nicht sicher belegt ist. Ferner lassen sich auf der Grundlage von Spontanberichten keine Aussagen darüber machen, wie häufig eine bestimmte unerwünschte Wirkung unter der Anwendung eines Arzneimittels vorkommt, und auch vergleichende Angaben darüber, wie viel mal häufiger eine bestimmte unerwünschte Wirkung bei einem Arzneimittel im Verhältnis zu einem anderen Medikament auftritt, sind anhand solcher Berichte kaum möglich.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92900215

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: privacy Age: 80 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19911017				recovered/resolved
2 Epidermal necrolysis	19911017				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acimethin methionine, active	Oral	002 Df dosage form/Day	19910922-19911017	-
2 allvoran diclofenac sodium	Oral	001 Df dosage form/Day	19910922-19911017	-
3 lioresal baclofen	Oral	003 Df dosage form/Day	19910922-19911017	-
4 ludiomil maprotiline hydrochloride	Oral	001 Df dosage form/Day	19910922-19911017	-
5 nifelat retard nifedipine	Oral	002 Df dosage form/Day	19910922-19911017	-
6 rohypnol flunitrazepam	Oral	0.5 Df dosage form/Day	19910922-19911017	-
7 supracombin clemizole penicillin; streptomycin pantothenate; streptomycin sulfate	Oral	/	19911017-19911018	Urinary tract infection, site not specified
8 tensoflux amiloride hydrochloride; bendroflumethiazide	Oral	001 Df dosage form/Day	19910922-19911017	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Paralytic ileus			k.A.	concomitant disease
Quadriplegia			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92002025

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 33 Year	weight (kg): 69	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Angioedema	19920324		3 Day		recovered/resolved
2 Conjunctivitis	19920324		3 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm 100 diclofenac sodium	Rectal	100 Mg milligram(s)/Day	19920324-19920324	Pancreatitis acute
			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92003776

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 51 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Leucopenia	19920514				recovered/resolved
2 Moniliasis	19920514				recovered/resolved
3 Stomatitis	19920514				recovered/resolved
4 Lymphadenopathy	19920514				recovered/resolved
5 Agranulocytosis	19920514				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ambrodoxy ambroxol hydrochloride; doxycycline hyclate	Unknown	/Day	19920511-	-
2 diclac diclofenac sodium	Unknown	/Day	19920511-	-
3 gelonida na acetylsalicylic acid; codeine phosphate; paracetamol	Rectal	/Day	19920511-	-
4 novaminsulfon metamizole	Oral	/Day	199205-	-
5 tempil n acetylsalicylic acid; diphenylpyraline hydrochloride; metamfepramone hydrochloride	Unknown	/Day	19920511-	-
6 treupel comp. codeine phosphate; paracetamol	Rectal	/Day	19920511-	-

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92900471

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 39 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19920612		2 Week		not recovered/not resolved
2 Erythematous rash	19920612		2 Week		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm sf diclofenac sodium	Intravenous (not otherwis	075 Mg milligram(s)/Day	19920609-19920610	Lumbago
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Sciatica			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008030

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: privacy Age: 66 Year weight (kg): 72 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19910625				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cytotec misoprostol	Oral	400 Mg milligram(s)/Day	19910610-19910628	Rheumatoid arthritis
2 diclofenac 100 mg retard diclofenac sodium	Oral	001 Df dosage form/Day	19910621-19910626	
3 fluniget diflunisal	Oral	002 Df dosage form/Day	19910610-19910627	Rheumatoid arthritis
4 gastrosil metoclopramide hydrochloride	Oral	060 Gtt drop(s)/Day	19910617-19910621	
5 ismo 20 isosorbide mononitrate	Oral	040 Mg milligram(s)/Day	19910610-19910628	
6 l-thyroxin levothyroxine sodium	Oral	100 µg microgram(s)/Day	19910610-19910628	
7 tauredon 20 sodium aurothiomalate	Intramuscular	020 Mg milligram(s)/Day	19910617-19910617	

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92008032

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Female	Birth: privacy	Age: 78 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19910906				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 circo-maren nicergoline	Oral	002 Df dosage form/Day	-19910906	-
2 diclofenac 50 mg diclofenac sodium	Oral	100 Mg milligram(s)/Day	19910803-19910906	-
3 dihydralazin dihydralazine	Oral	001 Df dosage form/Day	-19910906	-
4 gastrosil metoclopramide hydrochloride	Oral	/	-19910906	-
5 hydergin forte dihydroergocornine mesilate; dihydroergocristine mesilate; dihydroergocryptine mesilate	Oral	/	-19910910	-
6 ismo 20 isosorbide mononitrate	Oral	040 Mg milligram(s)/Day	-19910910	-
7 lefax dimeticone; silicon dioxide, colloidal	Oral	/	-19910906	-
8 lexotanil bromazepam	Oral	001 Df dosage form/Day	-19910906	-
9 megacillin oral phenoxymethylpenicillin potassium	Oral	/	19910904-19910906	-
10 tensobon comp captopril; hydrochlorothiazide	Oral	/	-19910906	-
11 tranxilium clorazepate dipotassium	Oral	001 Df dosage form/Day	-19910906	-
12 vertigo-vomex dimenhydrinate; nicotinic acid; pyridoxine hydrochloride	Oral	/	-19910906	-

Drugs (concomitant)

13 digimerck 0,1 tabletten digitoxin	Oral	001 Df dosage form/Day	19910906-19910910	-
14 fenistil dimetindene maleate	Intravenous (not otherwis	/	19910906-19910910	-
15 kalium-duriles potassium chloride	Oral	001 Df dosage form/Day	19910906-19910910	-
16 solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis	250 Mg milligram(s)/Day	19910906-19910910	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Essential hypertension, unspecified			k.A.	concomitant disease
Unspecified organic brain syndrome (chronic)			k.A.	concomitant disease
Atrial fibrillation and flutter			k.A.	concomitant disease
Angina pectoris			k.A.	concomitant disease
Other acute and subacute forms of ischaemic heart disease			k.A.	concomitant disease
Other generalized ischaemic cerebrovascular disease			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92900655

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 24 Year	weight (kg): 60	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Jaundice	19910203		12 Day		recovered/resolved
2 Hepatic enzymes increased	19910203		12 Day		recovered/resolved
3 Stevens Johnson syndrome	19910203		12 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	002 Df dosage form/Day	19910103-19910201	Acute nasopharyngitis (common cold)
2 batrafen creme ciclopirox olamine	Topical	/	-	Unspecified disorder of skin and subcutaneous tissue
3 carbamid urea	Topical	/	-	Unspecified disorder of skin and subcutaneous tissue
4 contramutan aconite; adonis vernalis glycosides; belladonna alkaloids; echinacea spp; eupatorium perfoliatum; myristica	Oral	15 Gtt drop(s)/	19910103-19910201	Acute nasopharyngitis (common cold)
5 lidocaine lidocaine	Subcutaneous	1 Df dosage form/	19910119-19910119	Low back pain
6 muskel trancopal chlormezanone	Oral	1 Df dosage form/	19910119-19910119	Low back pain
7 panotile fludrocortisone acetate; lidocaine hydrochloride; neomycin sulfate; polymyxin b sulfate	Topical	3 Df dosage form/	19910102-19910102	Unspecified otitis media
8 prednisolon prednisolone	Unknown	/	19910119-19910119	
9 pyrogenium homeopatics nos	Oral	10 Gtt drop(s)/	19910131-19910131	Acute nasopharyngitis (common cold)
10 resimatil primidone	Oral	003 Df dosage form/Day	-	Epilepsy, unspecified
11 sinupret ascorbic acid; gentiana; primula flowers; rumicis asetos; sambucus flowers; verbena herb	Oral	006 Df dosage form/Day	19910101-	Chronic sinusitis, unspecified
12 volon a haftsalbe triamcinolone acetonide	Buccal	/	-	Candidiasis of mouth
13 voltaren diclofenac sodium	Intramuscular	1 Df dosage form/	19910119-19910119	Low back pain

MedDRA-term	Begin	End	Cont.	Comment
Influenza with other respiratory manifestations			k.A.	concomitant disease

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92900645

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 79 Year	weight (kg): 63	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anorexia	19920806		3 Week		recovered/resolved
2 Application site reaction	19920806		3 Week		recovered/resolved
3 Bullous eruption	19920806		3 Week		recovered/resolved
4 General malaise	19920806		3 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 arthrex cellugel diclofenac sodium	Topical	1 Df dosage form/	19920806- -	Other affections of shoulder region, not elsewhere classified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other specified arthropathy			k.A.	concomitant disease
Heart failure, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-92010246

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 14 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19921010		6 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	002 Df dosage form/Day	19921002-19921007	Pain in limb
			-	

Drugs (concomitant)

2 minocycline minocycline	Oral	050 Mg milligram(s)/Day	-	Other acne
3 skinoren creme azelaic acid	Topical	001 Df dosage form/Day	-	Other acne

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute conjunctivitis			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93003602

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 39 Year	weight (kg): 68	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19921110	deathcause: Vasculitis
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anaphylactic shock	19921109		24 Hour		Unknown
2 Epidermal necrolysis	19921109		24 Hour		Unknown
3 Circulatory failure	19921109		24 Hour		Unknown
4 Pancytopenia	19921109		24 Hour		Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 clinit-n for injection cyanocobalamin; lidocaine; salamidacetic acid	Intramuscular	1 Df dosage form/	19921106-19921106	Rheumatism
2 cuxaflex benzyl nicotinate; camphor	Topical	/Day	19921106-19921108	Rheumatism
3 dexa-clinit injektionsloesung dexamethasone	Intramuscular	1 Df dosage form/	19921106-19921106	Rheumatism
4 diclofenac diclofenac	Oral	/	19921107-19921108	Rheumatism

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Contact dermatitis and other eczema, unspecified cause			k.A.	concomitant disease
Allergy, unspecified, not elsewhere classified			k.A.	concomitant disease
Nondependent abuse of alcohol			k.A.	concomitant disease
Nondependent abuse of tobacco			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93900269

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 54 Year	weight (kg): 82	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme	19910630		16 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ambene for injection aminophenazone; cyanocobalamin; dexamethasone sodium metasulfobenzoate; lidocaine hydrochloride; phenylbutazone sodium	Intramuscular	/	19910626-19910626	Sciatica
2 diazepam diazepam	Oral	/Day	19910619-19910626	-
3 diclofenac diclofenac	Oral	/Day	-	Sciatica
4 dolo-neurobion forte cyanocobalamin; paracetamol; pyridoxine hydrochloride; thiamine mononitrate	Oral	/Day	19910626-	Sciatica
5 fortecortin dexamethasone	Intramuscular	/	19910626-19910626	Unspecified erythematous condition
6 zovirax 200 aciclovir	Oral	/	19910630-	-

Drugs (concomitant)

7 rantudil acemetacin	Oral	/Day	19910630-	Sciatica
			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93900184

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin necrosis	199104				Unknown
2 Injection site reaction	199104				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 delphinac diclofenac sodium	Intramuscular	/	199104-199104	-
2 lokalanaesthetikum	Unknown	/	199104-199104	-

Drugs (concomitant)

3 delphimix triamcinolone diacetate	Intramuscular	/	199104-199104	-
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Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93004890

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 78 Year	weight (kg): 51	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19920812	deathcause: Heart failure, unspecified
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920731				Unknown
2 Epidermal necrolysis	19920731				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aplona apple dried	Oral	/	19920727-19920729	-
2 bepanthen roche panthenol	Oral	008 Df dosage form/Day	19920724-	-
3 briserin clopamide; dihydroergocryptine mesilate; reserpine	Oral	002 Df dosage form/Day	-	-
4 contradol acetylsalicylic acid	Oral	003 Df dosage form/Day	19920727-19920728	-
5 crataegutt novo filmtabletten crataegus extract	Oral	003 Df dosage form/Day	-	-
6 indometacin indometacin	Rectal	001 Df dosage form/Day	19920722-19920730	-
7 kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	3 Df dosage form/Week	-19920724	-
8 karil calcitonin, salmon	Subcutaneous	001 Df dosage form/Day	-	-
9 paspertin metoclopramide hydrochloride	Oral	060 Gtt drop(s)/Day	19920724-	-
10 voltaren diclofenac sodium	Intramuscular	001 Df dosage form/Day	-	-
11 zovirax aciclovir	Topical	/	19920729-	-
12 zyloric allopurinol	Oral	3 Df dosage form/Week	-19920803	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Essential hypertension, unspecified			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n.mammakarzinom-op
Malignant neoplasm of exocervix			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93004891

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 77 Year	weight (kg): 62	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19920704	deathcause: Heart failure, unspecified
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920616				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 anti-phosphat algedrate	Oral	/	-	Renal failure, unspecified
2 depuran anise oil; carum carvi; senna fruit extract	Oral	/	-	Constipation
3 diclofenac diclofenac	Oral	/	19920613-19920613	Bronchitis, not specified as acute or chronic
4 isoket retard 40 isosorbide dinitrate	Oral	/	-	Hypotension, unspecified
5 lactulose lactulose	Oral	/	-	Constipation
6 paracetamol paracetamol	Oral	/	-	Other general symptoms
7 rulid roxithromycin	Oral	/	19920613-19920613	Bronchitis, not specified as acute or chronic

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: z.n.lyell-syndrom
Coronary atherosclerosis			k.A.	concomitant disease
Unspecified adverse effect of drug, medicament and biologi			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93004860

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Female	Birth: privacy	Age: 27 Year	weight (kg): 54	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dermatitis contact	19930326		14 Day		recovering/resolving
2 Bullous eruption	19930326		14 Day		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 arthrex cellugel diclofenac sodium	Topical	1 Df dosage form/	199303- 2 -	Wrist sprains and strains

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93009078

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Female	Birth: privacy	Age: 68 Year	weight (kg): 56	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dermatitis contact	19930505		14 Day		recovered/resolved
2 Bullous eruption	19930505		14 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 effekton diclofenac sodium	Unknown	1 Df dosage form/	19930505- 1 -	Sprains and strains of unspecified site of shoulder and

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93011023

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Female Birth: privacy Age: 85 Year weight (kg): 60 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19921212				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allvoran diclofenac sodium	Oral	001 Df dosage form/Day	-	Gout, unspecified
2 claudicat pentoxifylline	Oral	/	-	Unspecified circulatory system disorder
3 dulcolax bisacodyl	Unknown	/	-	Constipation
4 iso mack isosorbide dinitrate	Oral	040 Mg milligram(s)/Day	-	Heart failure, unspecified
5 isoglaucan clonidine hydrochloride	Ophthalmic	/	-	Glaucoma, unspecified
6 uripurinol-300 allopurinol	Oral	001 Df dosage form/Day	-	Gout, unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93011025

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Female	Birth: privacy	Age: 76 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19930401				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol allopurinol	Oral	001 Df dosage form/Day	19930318-	Hyperuricaemia
2 diclofenac diclofenac	Oral	/	-	
3 novodigal beta-acetyldigoxin	Oral	001 Df dosage form/Day	-	Heart failure, unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Emphysema			k.A.	concomitant disease
Alcohol dependence syndrome			k.A.	concomitant disease
Korsakov's psychosis, alcoholic			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93011100

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 56 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920918		1 Month		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin acetylsalicylic acid	Oral	/	-	Unspecified circulatory system disorder
2 cytotec misoprostol	Oral	/	19920902-	Unspecified functional disorder of stomach
3 defluina peri buflomedil hydrochloride	Oral	/	-	Unspecified circulatory system disorder
4 diclac diclofenac sodium	Oral	/	19920826-19920903	Unspecified arthropathy
5 ibuprofen ibuprofen	Oral	/	19920908-19920914	Unspecified arthropathy
6 indometacin indometacin	Oral	/	19920914-19920918	Unspecified arthropathy
7 methotrexat methotrexate	Oral	/	19920914-19920921	Unspecified arthropathy
8 penicillin benzylpenicillin sodium	Unknown	/	19920917-19920917	Erysipelas
9 rantudil acemetacin	Oral	/	19920903-19920907	Unspecified arthropathy

Drugs (concomitant)

10 acc 200 acetylcysteine	Oral	/	19920918-	Cough
11 agarol paraffin, liquid; phenolphthalein	Oral	/	19920918-	
12 augmentan amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis	/	19920918-19920921	Erysipelas
13 heparin heparin	Subcutaneous	/	19920918-	Thrombosis
14 valoron tilidine hydrochloride	Oral	/	-19920920	
15 voltaren diclofenac sodium	Intramuscular	/	-19920920	Unspecified arthropathy

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other psoriasis			k.A.	concomitant disease
Diabetes mellitus without mention of complication			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-93013371

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythematous rash	19920213				recovered/resolved with sequel
2 Pain	19920213				recovered/resolved with sequel
3 Skin necrosis	19920213				recovered/resolved with sequel
4 Injection site reaction	19920213				recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 effekton diclofenac sodium	Intramuscular	1 Df dosage form/	19920213- -	Osteoarthritis, generalized

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94014126

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 62 Year weight (kg): 93 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 SGPT increased	19931012		13 Day		recovering/resolving
2 SGOT increased	19931012		13 Day		recovering/resolving
3 Hepatotoxic effect	19931012		13 Day		recovering/resolving
4 Stomatitis	19931012		13 Day		not recovered/not resolved
5 Erythroderma	19931012		13 Day		recovering/resolving
6 Gamma-glutamyltransferase incr.	19931012		13 Day		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac 50 heumann diclofenac sodium	Oral	150 Mg milligram(s)/Day	19930921-19930927	Gout acute
			-	

Drugs (concomitant)

2 allopurinol 300 heumann allopurinol	Oral	300 Mg milligram(s)/Day	19930921-19931012	Gout acute
			-	
3 colchicum-dispert colchicine	Oral	006 Df dosage form/Day	19930921-19930923	Gout acute
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other and unspecified hyperlipidaemia			k.A.	concomitant disease
Alcoholic liver damage, unspecified			k.A.	concomitant disease
Hepatitis, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94900202

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 77 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19931008	deathcause: Erythema multiforme
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19931006				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac 100 mg supp. diclofenac sodium	Rectal	250 Mg milligram(s)/	19931006-19931006	Lumbago
			-	

Drugs (concomitant)

2 cardio-longoral	Oral	/	-	
crataegus extract; magnesium hydrogen aspartate; potassium hydrogen aspartate			-	
3 complamin retard	Oral	002 Df dosage form/Day	-	
xantinol nicotinate			-	
4 stadasan 200 schmerztabletten	Oral	/	19931003-19931003	
ibuprofen			-	
5 togonal		/	-	
acetylsalicylic acid; lithium citrate; quinine citrate			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94006177

Serious Yes	Death	Lifethr. Yes	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 36 Year	weight (kg): 84	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19940331				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol allopurinol	Oral	/	- 14 -	Hyperuricaemia
2 diclofenac diclofenac	Oral	/	- 2 -	Pain in joint

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94009104

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 63 Year	weight (kg): 68	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 BUN increased	199409				Unknown
2 Acidosis metabolic	199409				Unknown
3 Semi-coma	199409				Unknown
4 Renal vasculitis	199409				Unknown
5 Thrombocytopenia	199409				Unknown
6 Bleeding gastric ulcer	199409				Unknown
7 Erythema multiforme severe	19940831		5 Week		not recovered/not resolved
8 Stool tarry	199409				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 rewodina diclofenac sodium	Oral	025 Mg milligram(s)/Day	19940824-19940831	Rheumatoid arthritis
2 rocaltrol calcitriol	Oral	001 Df dosage form/Day	19940824-19940831	Renal osteodystrophy

Drugs (concomitant)

3 lasix furosemide	Oral	001 Df dosage form/Day	-	Chronic renal failure
4 prednisolone prednisolone	Oral	100 Mg milligram(s)/Day	-	Rheumatoid arthritis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Amyloidosis			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-94012275

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome					Unknown
2 Shock					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Intramuscular	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95000542

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 81 Year	weight (kg): 65	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19940824	deathcause: Shock without mention of trauma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Necrosis skin	19940823				Unknown
2 Infection	19940823				Unknown
3 Sepsis	19940823				Unknown
4 Fasciitis	19940823				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/	19940821-19940822	Pain in limb
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Old myocardial infarction			k.A.	concomitant disease
Coronary atherosclerosis			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95000560

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 62 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Necrosis skin	19941106				recovered/resolved
2 Lyell syndrome	19941106				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac 100 stada retard diclofenac sodium	Oral	100 Mg milligram(s)/Day	19931101-19941107	Knee pain
			-	

Drugs (concomitant)

2 marcumar phenprocoumon	Oral	003 Mg milligram(s)/Day	19930501-	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Atrial fibrillation and flutter			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95003068

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 52 Year	weight (kg): 63	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19930601				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aprical nifedipine	Oral	001 Df dosage form/Day	19930503-19930529	Cardiac dysrhythmia, unspecified
2 ass 100 acetylsalicylic acid	Oral	001 Df dosage form/Day	19930503-19930529	Coagulation defects, other and unspecified
3 coric lisinopril	Oral	001 Df dosage form/Day	19930503-19930529	Hypertonia
4 diclofenac diclofenac	Intramuscular	/	19930503-19930511	Other and unspecified disc disorder

Drugs (concomitant)

5 chlorhexamed chlorhexidine gluconate	Unknown	/	19930531-	Stomatitis
6 herviros aminoquinuride hydrochloride; tetracaine hydrochloride	Unknown	/	19930531-	Stomatitis
7 penicillin nos penicillin nos	Oral	/	19930531-	Rash and other nonspecific skin eruption

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pure hypercholesterolaemia			k.A.	concomitant disease
Allergy, unspecified, not elsewhere classified			k.A.	concomitant disease
Acute, but ill-defined, cerebrovascular disease			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95003071

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 62 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19930813				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 alivoran ampullen diclofenac sodium	Intramuscular	/	-	
2 mepivacaine mepivacaine	Subcutaneous	/	-	
3 muskel trancopal chlormezanone	Oral	/	-	

Primary source

Qualification: observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95900243

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acifugan allopurinol; benzbromarone	Unknown	/	-	-
2 allopurinol-ratiopharm 300 allopurinol	Oral	/	-	-
3 azulfidine sulfasalazine	Unknown	/	-	-
4 diclofenac diclofenac	Unknown	/	-	-
5 diclophlogont diclofenac sodium	Oral	/	-	-
6 tebesium 300 isoniazid; pyridoxine hydrochloride	Oral	/	-	-
7 valoron n naloxone hydrochloride; tilidine hydrochloride	Unknown	/	-	-
8 voltaren diclofenac sodium	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95900195

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: privacy Age: 80 Year weight (kg): 86 BMI_int: Age fetus:

Patientdeath

deathdate: deathcause: Erythema multiforme

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19930728				Unknown
2 Death NOS	19930811				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aquaphor 40 xipamide	Oral	/	-	-
2 calcium brausetabletten calcium	Oral	/	-	-
3 euphyllin aminophylline	Oral	/	-	-
4 isoptin verapamil hydrochloride	Oral	/	-	-
5 Kortison cortisone acetate	Oral	/	-	-
6 lasix 40 furosemide	Oral	002 Df dosage form/Day	19930723-19930730	-
7 lopirin captopril	Oral	/	-	-
8 myogit diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005918

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 36 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19941216				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allvoran sf diclofenac sodium	Intramuscular	/	-	
2 muskel trancopal compositum chlormezanone; paracetamol	Oral	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95001740

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum					Unknown
2 Erythema multiforme					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Intramuscular	/	-	
diclofenac			-	

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95001742

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme					Unknown
2 Erythema exudativum					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo 50 diclofenac sodium	Unknown	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95900664

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex:	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anaphylactic shock					Unknown
2 Lyell syndrome					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allvoran sf	Intramuscular	/	-	
diclofenac sodium			-	
2 bupivacaine	Intra-articular	/	-	
bupivacaine			-	
3 diclofenac	Topical	/	-	
diclofenac			-	
4 diclofenac	Oral	/	-	
diclofenac			-	
5 volon a	Intramuscular	/	-	
gramicidin; neomycin sulfate; triamcinolone acetonide			-	

Primary source

Qualification: observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95003869

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vasculitis					Unknown
2 Necrosis skin					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren retard diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95901189

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 63 Year weight (kg): 86 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940925				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	/	-	-
2 isoptin verapamil hydrochloride	Oral	/	-	-
3 l-thyroxin 100 levothyroxine sodium	Oral	/	-	-
4 muskel trancopal chlormezanone	Oral	/	-	-
5 neotri triamterene; xipamide	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005911

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 56 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma-GT increased					Unknown
2 Toxic epidermal necrolysis	19921122				Unknown
3 Lymphopenia	19921126				Unknown
4 SGPT increased					Unknown
5 Stevens Johnson syndrome	19921122				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin plus c acetylsalicylic acid; ascorbic acid acetylsalicylic acid	Oral	/	-	-
2 aspirin 100	Oral	/	-	-
3 limptar aminophylline; quinine sulfate	Oral	/	-	-
4 muskel trancopal chlormezanone	Oral	/	-	-
5 normoc bromazepam	Oral	/	-	-
6 reparil for injection escin sodium	Intravenous (not otherwis	/	-	-
7 tramal tramadol hydrochloride	Intravenous (not otherwis	/	-	-
8 tridin calcium citrate; calcium gluconate; sodium fluorophosphate	Oral	/	-	-
9 valium diazepam	Intravenous (not otherwis	/	-	-
10 vitamin-b calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	Intravenous (not otherwis	/	-	-
11 voltaren diclofenac sodium	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005913

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No								

Patient

Initials:	Sex: Female	Birth: privacy	Age: 40 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19931219				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	agarol paraffin, liquid; phenolphthalein	Oral	/	-
2	akrinor injektionsloesung cafedrine hydrochloride; theodrenaline hydrochloride	Intravenous (not otherwis	/	-
3	atropine sulfate atropine sulfate	Intravenous (not otherwis	/	-
4	calcium calcium	Oral	/	-
5	decadron-phosphat 8 mg dexamethasone sodium phosphate	Intravenous (not otherwis	/	-
6	dobutrex dobutamine hydrochloride	Intravenous (not otherwis	/	-
7	dopamin dopamine hydrochloride	Intravenous (not otherwis	/	-
8	dulcolax bisacodyl	Rectal	/	-
9	expafusin calcium chloride; glucose; hetastarch; potassium chloride; sodium chloride; sodium lactate	Intravenous (not otherwis	/	-
10	fentanyl fentanyl	Intravenous (not otherwis	/	-
11	forene isoflurane	Respiratory (inhalation)	/	-
12	fortecortin dexamethasone	Oral	/	-
13	gelonida na acetylsalicylic acid; codeine phosphate; paracetamol	Oral	/	-
14	heparin-natrium heparin sodium	Subcutaneous	/	-
15	hexoral hexetidine	Buccal	/	-
16	imeson nitrazepam	Oral	/	-
17	lactuflo lactulose	Oral	/	-
18	liquidepur anise oil; carum carvi; senna liquid extract	Oral	/	-
19	minirin desmopressin acetate	Nasal	/	-
20	muskel trancopal chlormezanone	Oral	/	-
21	nimotop s nimodipine	Intravenous (not otherwis	/	-
22	onkohaes hetastarch; sodium chloride	Intravenous (not otherwis	/	-
23	optalidon special noc dragees caffeine; dihydroergotamine mesilate; propyphenazone	Oral	/	-
24	optalidon special noc suppositorien dihydroergotamine mesilate; propyphenazone	Rectal	/	-
25	pancuronium pancuronium	Intravenous (not otherwis	/	-
26	paracetamol paracetamol	Rectal	/	-
27	phenhydan phenytoin	Oral	/	-

28	rheohes hetastarch; sodium chloride	Intravenous (not otherwis /	-
29	spizef 2,0 g cefotiam hydrochloride	Intravenous (not otherwis /	-
30	temgesic buprenorphine	Intravenous (not otherwis /	-
31	temgesic-sublingual buprenorphine hydrochloride	Sublingual /	-
32	tramal tramadol hydrochloride	Oral /	-
33	trapanal thiopental sodium	Intravenous (not otherwis /	-
34	valium for injection	Intravenous (not otherwis /	-
35	valium 2 roche	Oral /	-
36	Vitamine nnb vitamins nos	Intravenous (not otherwis /	-
37	voltaren suppositorien 50 mg diclofenac sodium	Rectal /	-
38	vomex a injektionsloesung i.v. dimenhydrinate	Intravenous (not otherwis /	-
39	zantic ranitidine hydrochloride	Oral /	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005914

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 68 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940110				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adumbran oxazepam	Oral	/	-	-
2 ben-u-ron paracetamol	Oral	/	-	-
3 clexane 20 enoxaparin	Subcutaneous	/	-	-
4 cytotec 200 misoprostol	Oral	/	-	-
5 endoxan cyclophosphamide	Oral	/	-	-
6 fluumucil granulat acetylcysteine	Oral	/	-	-
7 gallo-merz cellulase; dehydrocholic acid; dimeticone; pancreatin; turmeric	Oral	/	-	-
8 harzol sitosterols	Oral	/	-	-
9 mucosolvan tropfen ambroxol hydrochloride	Oral	/	-	-
10 muskel trancopal chlormezanone	Oral	/	-	-
11 rantudil forte acemetacin	Oral	/	-	-
12 rohypnol flunitrazepam	Oral	/	-	-
13 strogen uno sabal extract	Oral	/	-	-
14 ultralan-oral 5 fluocortolone	Oral	/	-	-
15 valoron tilidine hydrochloride	Oral	/	-	-
16 voltaren 50 diclofenac sodium	Oral	/	-	-
17 zantic ranitidine hydrochloride	Oral	/	-	-
18 zinacef cefuroxime	Intravenous (not otherwis /		-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-95005917

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 59 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940813				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 almag von ct suspension algedrate; magnesium hydroxide	Oral	/	-	-
2 ambroxol-ratiopharm 75 ambroxol hydrochloride	Oral	/	-	-
3 ass-ratiopharm 500 acetylsalicylic acid	Oral	/	-	-
4 batkaletta n reinecke senna leaf	Oral	/	-	-
5 calcium c ascorbic acid; calcium lactogluconate	Oral	/	-	-
6 diclo 50 von ct diclofenac sodium	Oral	/	-	-
7 isdn 120 stada retard isosorbide dinitrate	Oral	/	-	-
8 muskel trancopal chlormezanone	Oral	/	-	-
9 neuro-ratiopharm filmtabletten cyanocobalamin; pyridoxine hydrochloride; thiamine mononitrate	Oral	/	-	-
10 nife 20 ret von ct nifedipine	Oral	/	-	-
11 nitroglycerin-kaukapseln glyceryl trinitrate	Unknown	/	-	-
12 nitrolingual retard glyceryl trinitrate	Sublingual	/	-	-
13 olynth 0,1% loesung xylometazoline hydrochloride	Nasal	/	-	-
14 propra-ratiopharm 10 propranolol hydrochloride	Oral	/	-	-
15 thilocanfol 1% azidamfenicol	Ophthalmic	/	-	-
16 zocor 10 simvastatin	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96900009

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 36 Year weight (kg): 85 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19940814				Unknown
2 Toxic epidermal necrolysis	19940814				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 buscopan hyoscine butylbromide	Intravenous (not otherwis /		-	
2 musaril tetrazepam	Oral	/	-	
3 muskel trancopal chlormezanone	Oral	/	-	
4 novalglin metamizole	Intravenous (not otherwis /		-	
5 nubain nalbuphine hydrochloride	Intravenous (not otherwis /		-	
6 omnipaque-300 iohexol	Intravenous (not otherwis /		-	
7 paspertin tropfen metoclopramide hydrochloride	Oral	/	-	
8 spasmex trospium chloride	Intravenous (not otherwis /		-	
9 spasmo-cibalgin compositum s codeine phosphate; drofenine hydrochloride; propyphenazone	Unknown	/	-	
10 tarivid ofloxacin	Oral	/	-	
11 tarivid i.v. ofloxacin hydrochloride	Intravenous (not otherwis /		-	
12 tramal tramadol hydrochloride	Oral	/	-	
13 voltaren diclofenac sodium	Rectal	/	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96900246

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 55 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960113				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass 100 acetylsalicylic acid	Oral	/	-	-
2 butazolidin phenylbutazone	Intramuscular	/	-	-
3 diclofenac diclofenac	Intravenous (not otherwis	/	-	-
4 isomonit isosorbide dinitrate	Oral	/	-	-
5 molsihexal molsidomine	Oral	/	-	-
6 nif-ten atenolol; nifedipine	Oral	/	-	-
7 paracodin dihydrocodeine bitartrate	Oral	/	-	-
8 vitamin b12 cyanocobalamin	Intravenous (not otherwis	/	-	-
9 xanef 5 mg enalapril maleate	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96005154

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 84 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19920505				Unknown
2 Toxic epidermal necrolysis	19920505				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adalat t 10	Oral	/	-	
nifedipine			-	
2 diclofenac	Oral	/	-	
diclofenac			-	
3 dimenhydrinate	Oral	/	-	
dimenhydrinate			-	
4 furosemide	Oral	/	-	
furosemide			-	
5 novodigal	Oral	/	-	
beta-acetyldigoxin			-	
6 spiro comp.-ratiopharm	Oral	/	-	
furosemide; spironolactone			-	
7 tms forte	Oral	/	-	
sulfamethoxazole; trimethoprim			-	

Primary source

Qualification:	observestudytype:
Literature:	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96008197

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 75 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19951105				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bikalm zolidem tartrate	Oral	0.5 Df dosage form/Day	-	Sleep disturbances
2 l-thyroxin 75 henning levothyroxine sodium	Oral	075 µg microgram(s)/Day	-	Goitre, unspecified
3 maprotiline hydrochloride maprotiline hydrochloride	Oral	001 Df dosage form/Day	-	Depression
4 muskel trancopal chlormezanone	Oral	001 Df dosage form/Day	-	Backache, unspecified
5 neogama sulpiride	Oral	002 Df dosage form/Day	-	Vertigo
6 novodigal beta-acetyldigoxin	Oral	001 Df dosage form/Day	-	Heart failure, unspecified
7 tredalat acebutolol hydrochloride; nifedipine	Oral	001 Df dosage form/Day	-	Essential hypertension, unspecified
8 voltaren resinat diclofenac resinate	Oral	002 Df dosage form/Day	-	Backache, unspecified

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96008678

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: 64 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	19960418				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96016514

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 33 Year	weight (kg): 68	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19941102		4 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bactrim forte sulfamethoxazole; trimethoprim	Oral	/Day	19941020-19941102	Pneumonia, organism unspecified
2 bifiteral lactulose	Oral	/Day	19941024-19941030	Constipation
3 diclofenac diclofenac	Oral	50 Mg milligram(s)/	19941023-19941023	
4 diflucan 50 fluconazole	Oral	050 Mg milligram(s)/Day	19941005-19941102	Candidiasis of mouth
5 eusaprim forte sulfamethoxazole; trimethoprim	Oral	002 Df dosage form/Day	19941005-19941020	Pneumonia, organism unspecified
6 gentamicin gentamicin	Intravenous (not otherwis	80 Mg milligram(s)/	19941101-19941101	
7 Kodein codeine	Oral	/Day	19941005-19941019	Cough
8 laxoberal abfuehrtropfen sodium picosulfate	Oral	/		Constipation
9 l-polamidon levomethadone	Oral	/	19941020-19941102	Drug dependence of morphine type
10 magnesium aspartate hydrochloride magnesium aspartate hydrochloride	Oral	/	19941021-19941021	
11 muskel trancopal chlormezanone	Oral	/Day	19941020-19941022	
12 oxazepam oxazepam	Oral	/Day	19941025-19941120	
13 rocephin im. ceftriaxone sodium; lidocaine hydrochloride	Intravenous (not otherwis	1 G gram(s)/	19941101-19941101	
14 zovirax aciclovir	Oral	/Day	19941021-19941102	Herpes genital
15 zyloric 300 allopurinol	Oral	/Day	19941021-19941102	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Viral hepatitis B without mention of hepatic coma			k.A.	concomitant disease
Bronchitis, not specified as acute or chronic			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: hiv-infektion mit naeher bezeichneten in
Hepatitis non-A non-B			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
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Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96016515

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 87 Year	weight (kg): 53	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19951121	deathcause: Congestive heart failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19951110				Unknown
2 Toxic epidermal necrolysis	19951110				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol 300 heumann allopurinol	Oral	300 Mg milligram(s)/Day	19951106-19951111	Hyperuricaemia
2 allopurinol 300 stada allopurinol	Oral	300 Mg milligram(s)/Day	19951106-19951111	Hyperuricaemia
3 ambroxol ambroxol	Oral	/	19951016-19951113	Cough
4 amoxicillin amoxicillin	Oral	002 Df dosage form/Day	19951106-19951110	Bronchitis, not specified as acute or chronic
5 capto-puren cor captopril	Oral	/	19951016-19951113	Heart failure, unspecified
6 cerebroforte piracetam	Oral	003 Df dosage form/Day	19951016-19951113	Unspecified mental retardation
7 chloral hydrate chloral hydrate	Oral	500 Mg milligram(s)/Day	19951016-19951113	Sleep disturbances
8 corangin 40 retard isosorbide mononitrate	Oral	040 Mg milligram(s)/Day	19951016-19951113	Heart failure, unspecified
9 diclofenac diclofenac	Oral	50 Mg milligram(s)/	19951016-19951106	Unspecified musculoskeletal disorders and symptoms
10 digimerck digitoxin	Oral	.07 Mg milligram(s)/Day	19951016-19951113	Heart failure, unspecified
11 diuretikum verla hydrochlorothiazide; triamterene	Oral	001 Df dosage form/Day	19951016-19951110	Heart failure, unspecified
12 eunerpan 25 melperone	Oral	025 Mg milligram(s)/Day	19951016-19951113	Sleep disturbances
13 muskel trancopal chlormezanone	Oral	1 Df dosage form/	19951016-19951106	Unspecified musculoskeletal disorders and symptoms

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Osteoarthritis, localized, not specified whether primary or s			k.A.	concomitant disease
Presenile dementia			k.A.	concomitant disease
Renal failure, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96016516

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 35 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctivitis	19951005		4 Week		recovered/resolved
2 Maculopapular rash	19951005		4 Week		recovered/resolved
3 Toxic epidermal necrolysis	19951005		4 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diazepam diazepam	Intravenous (not otherwis	010 Mg milligram(s)/Day	19950918-19950919	Lumbago
2 diclofenac diclofenac	Intravenous (not otherwis	2*1 Df dosage form/	19950918-19950919	Lumbago
3 musaril tetrazepam	Oral	150 Mg milligram(s)/Day	19950928-19951004	Displacement of intervertebral disc, site unspecified, without
4 muskel trancopal compositum chlormezanone; paracetamol	Oral	001 Df dosage form/Day	19950921-19950927	Displacement of intervertebral disc, site unspecified, without
5 piroxicam piroxicam	Intravenous (not otherwis	2 Df dosage form/	19950918-19950918	Lumbago
6 prednisolone prednisolone	Intravenous (not otherwis	1 Df dosage form/	19950918-19950918	Lumbago
7 riopan magaldrate	Oral	003 Df dosage form/Day	19950928-19951004	
8 solutrast iopamidol	Intravenous (not otherwis	1 Df dosage form/	19950929-19950929	
9 tramadol tramadol	Oral	/	19950921-19950927	Displacement of intervertebral disc, site unspecified, without
10 vitamin b12 cyanocobalamin	Intravenous (not otherwis	2*1 Df dosage form/	19950918-19950919	Lumbago
11 voltaren 50 diclofenac sodium	Oral	150 Mg milligram(s)/Day	19950928-19951004	Displacement of intervertebral disc, site unspecified, without

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96016528

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 24 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19951012				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allvoran diclofenac sodium	Intramuscular	/	-	-
2 brexidol piroxicam	Oral	/	-	-
3 muskel trancopal compositum chlormezanone; paracetamol	Unknown	/	-	-
4 voltaren diclofenac sodium	Oral	/	-	-
5 xyloneural lidocaine hydrochloride	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-96020742

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 76 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrolysis	19910824				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol	Oral	/	-	
allopurinol			-	
2 asclepias oligoplex	Oral	/	-	
homeopatics nos			-	
3 diclofenac	Oral	/	-	
diclofenac			-	
4 Homoeopathika	Oral	/	-	
			-	
5 mucosolvan inhalationsloesung	Respiratory (inhalation)	/	-	
ambroxol hydrochloride			-	
6 sultanol inhalationsloesung	Respiratory (inhalation)	/	-	
salbutamol sulfate			-	
7 theophyllin	Oral	/	-	
theophylline			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000100

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 45 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940116				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 apis-homaccord homeopatics nos	Oral	/	-	-
2 ass acetylsalicylic acid	Oral	/	-	-
3 augmentan amoxicillin trihydrate; clavulanic acid	Oral	/	-	-
4 diclo divide diclofenac sodium	Oral	/	-	-
5 paracetamol paracetamol	Oral	/	-	-
6 solidagoren n herbal extracts nos	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000114

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 68 Year	weight (kg): 81	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Circulatory failure					Unknown
2 Toxic epidermal necrolysis	19951026				Unknown
3 Cardiac failure					Unknown
4 Skin erythema	19951006				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-97000114					
1	abnoba viscum injektionsloesung homeopatics nos	Unknown	/	19951019-19951023	Multiple myeloma
2	acifugan allopurinol; benzbromarone	Oral	003 Df dosage form/Day	19950929-	Gout, unspecified
3	allopurinol allopurinol	Oral	/Day	19951017-	Gout, unspecified
4	bifiteral lactulose	Oral	/	19951017-	Constipation
5	develin retard dextropropoxyphene hydrochloride	Oral	002 Df dosage form/Day	19951017-	Other general symptoms
6	diclofenac diclofenac	Oral	150 Mg milligram(s)/Day	19951019-	Gout, unspecified
7	dolantin pethidine hydrochloride	Subcutaneous	/	-	Other general symptoms
8	doxorubicin hydrochloride doxorubicin hydrochloride	Intravenous (not otherwis	15 Mg milligram(s)/	19951023-	Multiple myeloma
9	fortecortin dexamethasone	Intravenous (not otherwis	40 Mg milligram(s)/	19951023-19951025	
10	glucose 5% infusion glucose	Unknown	/	19951018-	Feeding difficulties and mismanagement
11	heparin heparin	Subcutaneous	015 Kiu iu (1000s)/Day	19951017-	Thrombosis
12	impletol caffeine; procaine hydrochloride	Subcutaneous	5 ml millilitre(s)/	19951022-19951025	Other general symptoms
13	kaliumchloridloesung potassium chloride	Intravenous (not otherwis	/	19951018-	Hypokalaemia
14	karil calcitonin, salmon	Subcutaneous	1 Df dosage form/	19951023-	Malignant neoplasm of bone and articular cartilage, site
15	lasix furosemide	Intravenous (not otherwis	/	-	Fluid retention
16	laxoberal abfuehrtropfen sodium picosulfate	Oral	20 Gtt drop(s)/	19951018-	Constipation
17	mst 10 morphine sulfate	Oral	006 Df dosage form/Day	19951017-19951018	Other general symptoms
18	nacl 0,9% sodium chloride	Unknown	/	19951018-19951025	Hyponatraemia
19	novalgin metamizole	Oral	/	-	Other general symptoms
20	paspertin tropfen metoclopramide hydrochloride	Oral	060 Gtt drop(s)/Day	19951021-19951025	Nausea and vomiting
21	solu-decortin prednisolone sodium succinate	Unknown	/	19951025-19951026	
22	sostril ranitidine hydrochloride	Oral	002 Df dosage form/Day	19951017-	Unspecified functional disorder of stomach
23	sterofundin electrolytes nos	Intravenous (not otherwis	/	19951018-	Volume depletion
24	tramal tropfen tramadol hydrochloride	Oral	060 Gtt drop(s)/Day	19950929-	Other general symptoms
25	vincristine sulfate vincristine sulfate	Intravenous (not otherwis	/	19951023-	Multiple myeloma

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000116

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 83 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960106				Unknown
2 Epidermal necrolysis	19960106				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 baldrian valeriana officinalis root	Oral	/	-	-
2 bronchicum ammonia; cimicifuga racemosa root; cortex quebracho; ephedrine hydrochloride; eucalyptus oil; grindelia; menthol; pimpinella root; sodium bromide; thyme; thyme extract	Oral	/	-	-
3 calcium calcium	Oral	/	-	-
4 diclofenac diclofenac	Oral	/	-	-
5 diclo-wolff supp. diclofenac sodium	Oral	/	-	-
6 Hoffmannstropfen ethanol	Oral	/	-	-
7 korodin camphor; convallaria tincture; crataegus extract; menthol; valerian tincture	Oral	/	-	-
8 magnesium citrate magnesium citrate	Oral	/	-	-
9 novalgin metamizole	Intramuscular	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000960

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 66 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin necrosis	199608				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Intramuscular	/	- -	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97000973

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 57 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dyspnoea	19960213				Unknown
2 Rhabdomyolysis	19960213				Unknown
3 Skin erythema	19960213				Unknown
4 Myositis	19960213				Unknown
5 Necrosis skin	19960213				Unknown
6 Skin discolouration	19960213				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dexamethasone dexamethasone	Intramuscular	/	-	-
2 diclofenac diclofenac	Intramuscular	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97900100

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: 45 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Injection site reaction	199509				Unknown
2 Paralysis muscle local skeletal	199509				Unknown
3 Pain	199509				Unknown
4 Skin necrosis	199509				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dexabene	Unknown	/	-	
dexamethasone sodium phosphate			-	
2 diclofenac	Unknown	/	-	
diclofenac			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97001411

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 51 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19950414				Unknown
2 Mucosa vesicle	19950414				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allvoran diclofenac sodium	Intramuscular	/	-	-
2 ambene aluminium glycinate; cyanocobalamin; dexamethasone; phenylbutazone; thiamine hydrochloride	Intramuscular	/	-	-
3 dexabene dexamethasone sodium phosphate	Intramuscular	/	-	-
4 imap fluspirilene	Intramuscular	/	-	-
5 kraeuterlax aloes extract; curcuma xanthorrhiza; mandragora; senna leaf; sodium phosphate dibasic dodecahydrate; sodium phosphate monobasic (anhydrate)	Oral	/	-	-
6 musaril tetrazepam	Oral	/	-	-
7 thomapyrin acetylsalicylic acid; caffeine; paracetamol	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002136

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 71 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatic enzymes increased	19960725				Unknown
2 Stevens Johnson syndrome	19960725				Unknown
3 Conjunctivitis	19960725				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 briserin n clopamide; reserpine	Oral	/	-	
2 voltaren emulgel diclofenac diethylamine	Topical	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97002596

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Application site reaction					Unknown
2 Necrosis skin					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm sf	Intramuscular	/	-	
diclofenac sodium			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006279

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 83 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19950924				Unknown
2 Toxic epidermal necrolysis	19950924				Unknown
3 Vesiculobullous rash	19950924				Unknown
4 Skin disorder	19950924				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cor tensobon	Oral	/	-	
captopril			-	
2 isoptin mite	Oral	/	-	
verapamil hydrochloride			-	
3 trental infusionsloesung	Intravenous (not otherwis	/	-	
pentoxifylline			-	
4 voltaren 50	Oral	/	-	
diclofenac sodium			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006350

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 26 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	19951002				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acic 250 pi aciclovir sodium	Intravenous (not otherwis	/	-	
2 aciclovir-ratiopharm 800 aciclovir	Oral	/	-	
3 carbamazepine carbamazepine	Oral	/	-	
4 cytotec 200 misoprostol	Oral	/	-	
5 esberitox apis mellifica; baptisia; crotalus; echinacea spp; lachesis; silicon dioxide, colloidal; thuja	Oral	/	-	
6 tramadol tramadol	Oral	/	-	
7 voltaren 50 diclofenac sodium	Oral	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97006351

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 43 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960222				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bromazaniol bromazepam	Oral	/	-	-
2 novalgin tropfen metamizole sodium	Oral	/	-	-
3 paracefan clonidine hydrochloride	Oral	/	-	-
4 paracetamol paracetamol	Oral	/	-	-
5 rivotril clonazepam	Oral	/	-	-
6 tegretal suspension carbamazepine	Oral	/	-	-
7 voltaren diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-97007453

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering	19970814				Unknown
2 Skin reaction localised	19970814				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98000010

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 66 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19950513				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adalat nifedipine	Oral	/	-	-
2 atosil promethazine hydrochloride	Oral	/	-	-
3 ben-u-ron paracetamol	Unknown	/	-	-
4 cytotec 200 misoprostol	Oral	/	-	-
5 diclophlogont ampullen diclofenac sodium	Intramuscular	/	-	-
6 digimerck digitoxin	Intravenous (not otherwis	/	-	-
7 elantan 20 isosorbide mononitrate	Oral	/	-	-
8 furosemide furosemide	Intravenous (not otherwis	/	-	-
9 gelonida acetylsalicylic acid; codeine phosphate; paracetamol	Oral	/	-	-
10 isoptin verapamil hydrochloride	Intravenous (not otherwis	/	-	-
11 lasix 40 furosemide	Oral	/	-	-
12 neurocil levomepromazine maleate	Oral	/	-	-
13 novodigal beta-acetyldigoxin	Intravenous (not otherwis	/	-	-
14 supracombin sulfamethoxazole; trimethoprim	Oral	/	-	-
15 tranxilium clorazepate dipotassium	Oral	/	-	-
16 valoron n naloxone hydrochloride; tilidine hydrochloride	Oral	/	-	-
17 voltaren diclofenac sodium	Unknown	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98900275

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 46 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	19980122				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 effekton-creme diclofenac sodium	Topical	/	19980122-19980128	Sciatica

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98003748

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 91 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19960606				Unknown
2 Stevens Johnson syndrome	19960606				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 rewodina	Oral	/	-	
diclofenac sodium			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98900371

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Male	Birth: privacy	Age: 67 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering	19960508				Unknown
2 Toxic epidermal necrolysis	19960503				Unknown
3 Skin erythema	19960503				Unknown
4 Itching	19960503				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 berodual dossier-aerosol fenoterol hydrobromide; ipratropium bromide	Respiratory (inhalation)	/	-	
2 ciprobay 500 ciprofloxacin hydrochloride	Oral	/	-	
3 decortin-h prednisolone	Oral	/	-	
4 diclofenac diclofenac	Rectal	/	-	
5 dipidolor piritramide	Unknown	/	-	
6 elobact 250 cefuroxime axetil	Oral	/	-	
7 gramaxin cefazolin sodium	Intravenous (not otherwis	/	-	
8 imeson nitrazepam	Oral	/	-	
9 metamizole metamizole sodium	Oral	/	-	
10 moronal suspension nystatin	Unknown	/	-	
11 noctamid lormetazepam	Oral	/	-	
12 pulmicort budesonide	Respiratory (inhalation)	/	-	
13 refobacin gentamicin sulfate	Intravenous (not otherwis	/	-	
14 scandicain 1% mepivacaine hydrochloride	Intrapleural	/	-	
15 supramycin chlortetracycline hydrochloride; tetracycline hydrochloride	Intrapleural	/	-	
16 tarivid ofloxacin	Oral	/	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98004998

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 44 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pancreatitis	19980108				Unknown
2 Pustular rash	199801				Unknown
3 Skin vasculitis NOS	199801				Unknown
4 Raised liver enzymes	199801				Unknown
5 Pancreas enzymes increased	199801				Unknown
6 Inflammatory oedema reaction	199801				Unknown
7 Abdominal discomfort	199801				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Intramuscular	/	-	
diclofenac			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98005926

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 37 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lip swelling	19970523				Unknown
2 Oral mucosa swollen	19970518				Unknown
3 Epidermal necrolysis	19970518				Unknown
4 Stevens Johnson syndrome	19970518				Unknown
5 Scrotal pain	19970524				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aspirin "bayer" acetylsalicylic acid	Unknown	/	-	-
2 voltaren diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006120

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 72 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19971206				Unknown
2 Itching	19971206				Unknown
3 Blistering	19971206				Unknown
4 Toxic epidermal necrolysis	19971206				Unknown
5 Macular rash	19971206				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	adumbran	Oral	/	-
	oxazepam			-
2	arelix	Intravenous (not otherwis	/	-
	piretanide			-
3	arthotec forte	Oral	/	-
	diclofenac sodium; misoprostol			-
4	atosil tropfen	Oral	/	-
	promethazine hydrochloride			-
5	bromuc	Intravenous (not otherwis	/	-
	acetylcysteine			-
6	corvaton retard	Oral	/	-
	molsidomine			-
7	diazepam	Oral	/	-
	diazepam			-
8	durazanil 6	Oral	/	-
	bromazepam			-
9	heparin	Intravenous (not otherwis	/	-
	heparin			-
10	imap	Intramuscular	/	-
	fluspirilene			-
11	lasix	Intravenous (not otherwis	/	-
	furosemide			-
12	noctamid	Oral	/	-
	lormetazepam			-
13	novalgin tropfen	Oral	/	-
	metamizole sodium			-
14	paspertin tropfen	Oral	/	-
	metoclopramide hydrochloride			-
15	psyquil	Intravenous (not otherwis	/	-
	triflupromazine			-
16	saroten retard	Oral	/	-
	amitriptyline hydrochloride			-
17	scandicain 1%	Subcutaneous	/	-
	mepivacaine hydrochloride			-
18	tegretal 400	Oral	/	-
	carbamazepine			-
19	thioctacid 600	Intravenous (not otherwis	/	-
	thioctic acid			-
20	voltaren dispers	Oral	/	-
	diclofenac sodium			-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006992

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 35 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19970217				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac "ratiopharm" diclofenac sodium	Oral	/	-	-
2 thomapyrin acetylsalicylic acid; caffeine; paracetamol	Oral	/	-	-
3 unacid ampicillin sodium; sulbactam sodium	Intravenous (not otherwis	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006993

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 36 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19970218				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclophlogont diclofenac sodium	Intramuscular	/	-	
2 piroxicam "ratiopharm" piroxicam	Oral	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98006997

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 77 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19970329				Unknown
2 Stevens Johnson syndrome	19970329				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acesal acetylsalicylic acid	Oral	/	-	-
2 allopurinol 100 allopurinol	Oral	/	-	-
3 jenafenac 75 i.m. diclofenac sodium	Intramuscular	/	-	-
4 triampur forte hydrochlorothiazide; triamterene	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007012

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 59 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19971219				Unknown
2 Stomatitis	19971227				Unknown
3 Conjunctivitis	19971227				Unknown
4 Leucopenia	19971227				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diazepam	Oral	/	-	
diazepam			-	
2 diclofenac	Oral	/	-	
diclofenac			-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98007396

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 36 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19970820				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 colchysat buerger colchicine	Oral	/	-	-
2 diclofenac-ratiopharm 50 diclofenac sodium	Oral	/	-	-
3 lorafem 400 loracarbef	Oral	/	-	-
4 naproxen stada 500 naproxen	Oral	/	-	-
5 zyloric 300 allopurinol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008087

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 71 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19960719				Unknown
2 Toxic epidermal necrolysis	19960719				Unknown
3 Conjunctivitis	19960719				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclophlogont-50 diclofenac sodium	Oral	/	-	-
2 lasix 40 furosemide	Oral	/	-	-
3 noctazepam oxazepam	Oral	/	-	-
4 prontopyrin plus caffeine; paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008314

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 59 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthema	19970429				Unknown
2 Blistering	19970429				Unknown
3 Red eye	19970429				Unknown
4 Stomatitis	19970429				Unknown
5 Toxic epidermal necrolysis	19970429				Unknown
6 Stevens Johnson syndrome	19970429				Unknown
7 Mucosal inflammation	19970429				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 caffeine caffeine	Oral	/	-	-
2 diclofenac diclofenac	Intramuscular	/	-	-
3 novalgin metamizole	Oral	/	-	-
4 paracetamol paracetamol	Oral	/	-	-
5 propyphenazone propyphenazone	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008331

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 69 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19970425				Unknown
2 Toxic epidermal necrolysis	19970425				Unknown
3 Stevens Johnson syndrome	19970425				Unknown
4 Conjunctivitis	19970425				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 actrapid insulin human	Intravenous (not otherwis	/	-	-
2 adalat retard nifedipine	Oral	/	-	-
3 dexamethasone dexamethasone	Oral	/	-	-
4 ebrantil urapidil	Intravenous (not otherwis	/	-	-
5 fluimucil acetylcysteine	Oral	/	-	-
6 fortecortin dexamethasone	Intravenous (not otherwis	/	-	-
7 gramaxin cefazolin sodium	Intravenous (not otherwis	/	-	-
8 isoket 20 isosorbide dinitrate	Oral	/	-	-
9 nac 200 acetylcysteine	Oral	/	-	-
10 pepdul i.v. famotidine	Intravenous (not otherwis	/	-	-
11 phenhydan phenytoin	Intravenous (not otherwis	/	-	-
12 refobacin gentamicin sulfate	Ophthalmic	/	-	-
13 tegretal 400 retard carbamazepine	Oral	/	-	-
14 thrombophob-7500 heparin	Subcutaneous	/	-	-
15 voltaren resinat diclofenac resinate	Oral	/	-	-
16 zentropil phenytoin	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98009253

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 71 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blisters	19980716				Unknown
2 Itching	19980710				Unknown
3 Petechia	19980710				Unknown
4 Exanthema	19980710				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 anafranil retard clomipramine hydrochloride	Unknown	/	-	-
2 pepdul famotidine	Oral	/	-	-
3 voltaren resinat diclofenac resinate	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008653

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 54 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19980117				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclophlogont sl diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-98008657

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 33 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980304				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allohexal 300 allopurinol	Oral	/	-	
2 diclofenac 75 mg diclofenac sodium	Intramuscular	/	-	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001079

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stomatitis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac al 25 diclofenac sodium	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002397

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 65 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin reaction	19981205				Unknown
2 Necrosis skin	19981205				Unknown
3 Vasculitis	19981205				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	
diclofenac			-	
2 ferro-folgamma	Oral	/	-	
ascorbic acid; cyanocobalamin; ferrous sulfate; folic acid			-	
3 marcumar	Oral	/	-	
phenprocoumon			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000792

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 51 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever	19981021				Unknown
2 Blepharoconjunctivitis	19981027				Unknown
3 Stevens Johnson syndrome	19981017				Unknown
4 Skin erythema	19981018				Unknown
5 Cheilitis	19981021				Unknown
6 Nasal inflammation	19981021				Unknown
7 Lip swelling	19981018				Unknown
8 Buccal inflammation	19981021				Unknown
9 Red eye	19981017				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclophlogont-50 diclofenac sodium	Oral	/	-	-
2 oxazepam oxazepam	Oral	/	-	-
3 saroten retard kapseln 25 mg amitriptyline hydrochloride	Oral	/	-	-
4 sempera liquid itraconazole	Oral	/	-	-
5 zentropil phenytoin	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000793

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 70 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19980907				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm 75 sl diclofenac sodium	Oral	/	-	
2 milurit 300 allopurinol	Oral	/	-	
3 penicillin benzylpenicillin sodium	Oral	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99000802

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 47 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 LDH increased	19981213				Unknown
2 SGPT increased	19981213				Unknown
3 Stevens Johnson syndrome	19981212				Unknown
4 Epidermal necrolysis	19981212				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allobeta 300 allopurinol	Oral	/	-	-
2 diclofenbeta uno diclofenac sodium	Oral	/	-	-
3 herz ass-ratiopharm acetylsalicylic acid	Oral	/	-	-
4 meditonsin h homeopatics nos	Oral	/	-	-
5 paracetamol paracetamol	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001177

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 75 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19951103				Unknown
2 Leukocytosis	19951102				Unknown
3 LDH increased serum	19951103				Unknown
4 Exanthema	19951103				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylcysteine acetylcysteine	Oral	/	-	-
2 adalat retard nifedipine	Oral	/	-	-
3 aquaphor xipamide	Oral	/	-	-
4 cynt moxonidine	Oral	/	-	-
5 diclofenac diclofenac	Rectal	/	-	-
6 novodigal mite beta-acetyldigoxin	Oral	/	-	-
7 oedemase furosemide	Oral	/	-	-
8 sostril ranitidine hydrochloride	Oral	/	-	-
9 valoron n loesung zum einnehmen naloxone hydrochloride; tilidine hydrochloride	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001427

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 89 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19981126				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 adumbran oxazepam	Oral	/	-	
2 diclofenac diclofenac	Oral	/	-	
3 salbutamol salbutamol	Respiratory (inhalation)	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001473

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19980817				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bactrim sulfamethoxazole; trimethoprim	Intravenous (not otherwis	/	-	
2 mcp-tropfen metoclopramide hydrochloride	Oral	/	-	
3 voltaren emulgel	Topical	/	-	
4 voltaren 100 zaepfchen	Rectal	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001898

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 26 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 19941111	deathcause: Hepatic coma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Coma hepatic	19941101				Unknown
2 Sepsis	19941101				Unknown
3 Stomatitis	19941101				Unknown
4 Stevens Johnson syndrome	19941101				Unknown
5 Exanthema	19941101				Unknown
6 Blistering	19941101				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 atosil tropfen promethazine hydrochloride	Oral	/	19941025-19941026	Unspecified functional disorder of stomach
2 augmentan amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis	/	19941023-19941029	Unspecified infectious and parasitic diseases
3 ben-u-ron paracetamol	Rectal	/	19941021-19941023	Pyrexia of unknown origin
4 bifiteral lactulose	Oral	/	19941023-19941031	Unspecified chronic liver disease without mention of
5 dopamin dopamine hydrochloride	Intravenous (not otherwis	/	19941027-	Renal failure, unspecified
6 furorese furosemide	Oral	/	19941028-19941029	Oedema
7 humatin paromomycin sulfate	Oral	/	19941023-19941031	Encephalopathy, unspecified
8 konakion phytomenadione	Unknown	/	-	Deficiency of vitamin K
9 lasix furosemide	Intravenous (not otherwis	/	19941024-19941031	Oedema
10 novalgin injektionsloesung metamizole	Intravenous (not otherwis	/	19941030-19941030	Pyrexia of unknown origin
11 quantalan colestyramine	Oral	/	19941031-	Cirrhosis of liver without mention of alcohol
12 rocephin ceftriaxone sodium	Intravenous (not otherwis	/	19941030-	Unspecified infectious and parasitic diseases
13 voltaren diclofenac sodium	Rectal	/	19941101-19941101	Other general symptoms

Drugs (concomitant)

14 l-polamidon levomethadone	Unknown	/	-	
15 maalox aluminium hydroxide; aluminium hydroxide gel; magnesium hydroxide	Unknown	/	19941005-19941006	Unspecified functional disorder of stomach

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug dependence of morphine type			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001899

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 62 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19941106				Unknown
2 Toxic epidermal necrolysis	19941106				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclophlogont-50 diclofenac sodium	Oral	/	- -	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001908

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 81 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Gamma GT increased	19941212				Unknown
2 LDH increased					Unknown
3 Erythema multiforme severe	19941212				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	
diclofenac			-	
2 esidrix	Oral	/	-	
hydrochlorothiazide			-	
3 meaverin	Intracardiac	/	-	
mepivacaine hydrochloride			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99001912

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 37 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19940727				Unknown
2 SGPT increased	19940729				Unknown
3 SGOT increased	19940729				Unknown
4 Leucopenia	19940729				Unknown
5 Leukocytosis	19940729				Unknown
6 LDH increased	19940729				Unknown
7 Gamma GT increased	19940729				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Unknown	/	-	-
2 diclofenac-ratiopharm sf diclofenac sodium	Intramuscular	/	-	-
3 paracetamol paracetamol	Unknown	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99005518

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 64 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Conjunctivitis	19990628				Unknown
2 Fever	19990628				Unknown
3 Skin disorder	19990628				Unknown
4 Toxic epidermal necrolysis	19990628				Unknown
5 Malaise	19990628				Unknown
6 Erythema	19990628				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 antra mups omeprazole magnesium	Oral	/	-	
2 azulfidine sulfasalazine	Oral	/	-	
3 clont metronidazole	Oral	/	-	
4 concor 10 bisoprolol fumarate	Oral	/	-	
5 diclofenac diclofenac	Oral	/	-	
6 dytide h hydrochlorothiazide; triamterene	Oral	/	-	
7 klacid 250 clarithromycin	Oral	/	-	
8 mobec 7,5 mg meloxicam	Oral	/	-	
9 tri.-thiazid "stada" hydrochlorothiazide; triamterene	Oral	/	-	
10 zyloric 300 allopurinol	Oral	/	-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002532

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 47 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19990326				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass 500 acetylsalicylic acid	Oral	/	-	-
2 diclofenac diclofenac	Oral	/	-	-
3 doxycycline doxycycline	Oral	/	-	-
4 paracodin n-tropfen dihydrocodeine thiocyanate	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002548

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							

Patient

Initials:	Sex: Female	Birth: privacy	Age: 77 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis					Unknown
2 Leucopenia	19990522				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol	Oral	/	-	
allopurinol			-	
2 duravolten	Oral	/	-	
diclofenac sodium			-	
3 esidrix	Oral	/	-	
hydrochlorothiazide			-	
4 furosemide	Oral	/	-	
furosemide			-	
5 paracetamol	Oral	/	-	
paracetamol			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99002890

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 21 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug eruption	19990727				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 rewodina diclofenac sodium	Topical	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99006185

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bullous eruption					Unknown
2 Fever					Unknown
3 Lyell syndrome					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99006493

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 48 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Unconsciousness	199503				Unknown
2 Anaphylactic shock	199503				Unknown
3 Quincke's oedema	199503				Unknown
4 Conjunctivitis	199503				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm sf diclofenac sodium	Intramuscular	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99003584

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 71 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19990824				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Intramuscular	/	-	
2 xylocaine lidocaine hydrochloride	Intra-articular	/	-	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99004052

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 70 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19970725				Unknown
2 Toxic epidermal necrolysis	19970725				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 rewodina dual 75 kaps diclofenac sodium	Oral	/	- -	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99004067

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 68 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Creatinine increased	19990227				Unknown
2 Stevens Johnson syndrome	19990227				Unknown
3 Raised liver enzymes	19990227				Unknown
4 Lymphopenia	19990227				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acc 200 acetylcysteine	Oral	/	-	-
2 adumbran oxazepam	Oral	/	-	-
3 arlevert cinnarizine; dimenhydrinate	Oral	/	-	-
4 bepanthen dexpanthenol	Oral	/	-	-
5 furorese furosemide	Oral	/	-	-
6 gastrosil metoclopramide hydrochloride	Oral	/	-	-
7 globocef cefetamet pivoxil hydrochloride	Oral	/	-	-
8 godamed acetylsalicylic acid	Oral	/	-	-
9 voltaren dispers diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-99005384

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 69 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vasculitis	19991110				Unknown
2 Exanthema	19991110				Unknown
3 Skin necrosis	19991110				Unknown
4 Granulocytopenia	1999				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm 75 sl diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00000398

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 58 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin thickening	20000120				Unknown
2 Drug eruption	20000120				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aponal doxepin hydrochloride	Oral	/	-	-
2 diclac 50 diclofenac sodium	Oral	/	-	-

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00500119

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes		Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	/	-	-
2 lasix 20 furosemide	Oral	/	-	-
3 musaril tetrazepam	Oral	/	-	-

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00003922

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 64 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Rigors	19991217				Unknown
2 Fever	19991217				Unknown
3 Burn	19991217				Unknown
4 Exanthema	19991217				Unknown
5 Blisters	19991218				Unknown
6 Allergic reaction	19991217				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001048

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 39 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Desquamation	20000117				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00005243

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 49 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dermatitis necrotizing	20000304				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 monoflam 75 mg diclofenac sodium	Intramuscular	/	-	-

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001502

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 79 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lymphocytes raised	19991108				Unknown
2 SGOT increased	19991106				Unknown
3 SGPT increased	19991106				Unknown
4 LDH increased	19991108				Unknown
5 Fever	19991030				Unknown
6 Blisters					Unknown
7 Conjunctivitis					Unknown
8 Toxic epidermal necrolysis	19991104				Unknown
9 Stevens Johnson syndrome	19991104				Unknown
10 Gamma GT increased	19991106				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 arelix ace piretanide; ramipril	Oral	/	-	-
2 carbostesin bupivacaine hydrochloride	Intravenous (not otherwis	/	-	-
3 claforan cefotaxime sodium	Intravenous (not otherwis	/	-	-
4 dytide h hydrochlorothiazide; triamterene	Oral	/	-	-
5 mst 10 morphine sulfate	Oral	/	-	-
6 novaminsulfon metamizole	Oral	/	-	-
7 orelox cefepodoxime proxetil	Oral	/	-	-
8 pk-merz amantadine sulfate	Intravenous (not otherwis	/	-	-
9 tegretal carbamazepine	Oral	/	-	-
10 voltaren resinat diclofenac resinate	Oral	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00200088

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 88 Year	weight (kg): 69	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	19990726		37 Day		recovered/resolved
2 Exanthema	19990726		37 Day		recovered/resolved
3 Buccal inflammation	19990726		37 Day		recovered/resolved
4 Ulcer lip	19990726		37 Day		recovered/resolved
5 Blistering	19990726		37 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 antra 20 omeprazole	Oral	001 Df dosage form/Day	19990707- -	Unspecified functional disorder of stomach
2 atosil tropfen promethazine hydrochloride	Oral	/	19990725-19990725 -	Other and unspecified special symptoms or syndromes, not
3 chloraldurat 500 chloral hydrate	Oral	001 Df dosage form/Day	19990715- -	Sleep disturbances
4 clexane enoxaparin	Subcutaneous	/	19990708-19990714 -	Embolism and thrombosis of unspecified site
5 corangin isosorbide mononitrate	Oral	001 Df dosage form/Day	19990715- -	Coronary atherosclerosis
6 corvaton molsidomine	Oral	001 Df dosage form/Day	19990713-19990714 -	Coronary atherosclerosis
7 fraxiparin nadroparin calcium	Subcutaneous	/	19990715- -	Thrombosis
8 furorese furosemide	Oral	100 Mg milligram(s)/Day	-19990714 -	Heart failure, unspecified
9 lanitop metildigoxin	Oral	001 Df dosage form/Day	19990715- -	Heart failure, unspecified
10 oxazepam oxazepam	Oral	1 Df dosage form/	-19990712 -	Sleep disturbances
11 pres 2,5 mg enalapril maleate	Oral	2.5 Mg milligram(s)/Day	-19990714 -	Essential hypertension, unspecified
12 sedariston hypericum extract; melissa extract; valerian tincture	Oral	/	19990723-19990726 -	Other and unspecified special symptoms or syndromes, not
13 tramal tramadol hydrochloride	Oral	/	19990715-19990726 -	Other general symptoms
14 unat torasemide	Oral	001 Df dosage form/Day	19990715-19990727 -	Heart failure, unspecified
15 voltaren resinat diclofenac resinate	Oral	002 Df dosage form/Day	19990719-19990726 -	Other general symptoms
16 zyloric 100 allopurinol	Oral	001 Df dosage form/Day	-19990728 -	Hyperuricaemia

Drugs (concomitant)

17	bifiteral lactulose	Oral	1 Df dosage form/	19990701-19990721	Constipation
18	dulcolax bisacodyl	Rectal	1 Df dosage form/	19990707-19990707	Constipation
19	isoket isosorbide dinitrate	Oral	002 Df dosage form/Day	-19990714	Coronary atherosclerosis
20	liquemin heparin	Intravenous (not otherwis 2.5 Kiu iu (1000s)/Day		-19990708	Pulmonary embolism
21	magnesium stoffangabe (teilweise) unklar	Oral	001 Df dosage form/Day	19990717-	Disorders of magnesium metabolism
22	novodigal beta-acetyldigoxin	Oral	001 Df dosage form/Day	-19990714	Heart failure, unspecified
23	paspertin metoclopramide hydrochloride	Oral	/	19990703-19990703	Nausea and vomiting
24	rantudil forte acemetacin	Oral	002 Df dosage form/Day	19990726-	Other general symptoms
25	sostril ranitidine hydrochloride	Intravenous (not otherwis 1 Df dosage form/		19990630-19990630	Unspecified disorder of stomach and duodenum
26	tilidine hydrochloride tilidine hydrochloride	Oral	035 Gtt drop(s)/Day	19990701-19990712	Other general symptoms
27	tutofusin amino acids nos; electrolytes nos; vitamins nos	Intravenous (not otherwis /		19990630-19990630	Volume depletion
28	x-prep senna leaf	Oral	/	19990707-19990707	Constipation

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fracture of shaft or unspecified part of femur, closed			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-00001677

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 78 Year	weight (kg): 69	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Shock without mention of trauma
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20000210				Unknown
2 Pyrexia abnormal	20000210				Unknown
3 Water excessive loss of	20000210				Unknown
4 Sepsis	20000210				Unknown
5 Pancytopenia	20000210				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol	Oral	/	-	Hyperuricaemia
allopurinol			-	
2 amoxicillin	Oral	/	-	Pyrexia of unknown origin
amoxicillin			-	
3 cefotaxime	Oral	/	-	Pyrexia of unknown origin
cefotaxime			-	
4 diclofenac	Oral	/	-	Other general symptoms
diclofenac			-	
5 godamed 100	Oral	/	-	Coagulation defects, other and unspecified
acetylsalicylic acid			-	
6 voltaren resinat	Oral	/	-	Other general symptoms
diclofenac resinate			-	
7 zyloric 300	Oral	/	-	Hyperuricaemia
allopurinol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Influenza with other respiratory manifestations			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

case: DE-BFARM-01000203

Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes					

Patient

Initials: Sex: Male Birth: privacy Age: 33 Year weight (kg): 107 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000218		19 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol allopurinol	Oral	001 Df dosage form/Day	20000208-20000218	Hyperuricaemia
2 emser pastillen cetylpyridinium chloride; emser salt; menthol	Oral	001 Df dosage form/Day	20000201-20000208	Throat pain
3 voltaren resinat diclofenac resinate	Oral	001 Df dosage form/Day	20000208-20000218	Pain in joint

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland Type: Spontaneous report case: DE-BFARM-01000218

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes Yes Yes Yes

Patient

Initials: Sex: Female Birth: privacy Age: 45 Year weight (kg): 65 BMI_int: 22.49 Age fetus:

Patientdeath

deathdate: deathcause: Shock without mention of trauma

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	19990114		12 Day		Unknown
2 Leucopenia	19990125				Unknown
3 Stomatitis	19990118				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin amoxicillin	Oral	/Day	19990103-19990110 8 Day	Ulcer of lower limbs, except decubitus ulcer
2 antra omeprazole	Oral	/Day	19990103-19990112 10 Day	Chronic ulcer of skin, unspecified site
3 diclofenac diclofenac	Oral	/Day	19990103-19990112 10 Day	Osteoarthritis, localized, not specified whether primary or
4 erythromycin erythromycin	Oral	/Day	19981222-19990104 14 Day	Unspecified arthropathy

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lupus erythematosus			k.A.	concomitant disease
Allergy			k.A.	concomitant disease
Unspecified disorder of metabolism			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 09.05.2012 DE-SZ09-PHHY2012DE038377

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE038377 is an initial literature report was received on 04 May 2012. The author discussed about Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus. This case referred to a 45-year-old female patient (Tab 2, No 10). This patient's medical history included systemic lupus erythematosus (1980), antiphospholipid syndrome, exacerbation of systemic lupus erythematosus (1995) with extensive skin and liver involvement. She had atypical targets flat and widespread spots on her skin. She had drug history of prednisolone/prednisone and azathioprine as immunosuppressants. The patient was on treatment with amoxicillin and diclofenac (manufacturer, dose route, formulation and indication were unknown for both drugs) from an unreported date. On an unreported date in Jan 1999, she was diagnosed with definite toxic epidermal necrolysis (TEN). Involvement of skin (included atypical targets flat, spots widespread), palms and sole and mucous membranes (lips, oral) was reported. She was hospitalized on an unreported date. Treatment received in response to the event was unknown. On an unreported date the patient died of TEN. The author stated that potential cause of TEN was possibly amoxicillin and diclofenac.

Sender's comment:

Serious literature report, for amoxicillin: Toxic epidermal necrolysis [death], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

For diclofenac: Toxic epidermal necrolysis [death], already listed in the Core Data Sheet. All literature reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Comment: Amoxicillin and diclofenac are known to cause toxic epidermal necrolysis (TEN). Based on review of the available information, causal association with both the suspected medications cannot be ruled out. TEN is labelled with a fatal outcome for diclofenac and non-fatal outcome for amoxicillin.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01003607

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes

Patient

Initials: Sex: Female Birth: privacy Age: 63 Year weight (kg): 85 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pain	20010104		4 Hour		Unknown
2 Necrosis skin	20010125				not recovered/not resolved
3 Discoloration skin	20010104				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac diclofenac sodium	Intramuscular	1 Df dosage form/	20000420-20010104	Osteoarthritis, localized, not specified whether primary or

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Rheumatoid arthritis			k.A.	concomitant disease
Osteoarthritis, localized, not specified whether primary or s			k.A.	concomitant disease
Osteoporosis			k.A.	concomitant disease
Other specified forms of osteochondropathy			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01003854

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 35 Year	weight (kg): 76	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dermatitis contact	20010504				Unknown
2 Bullous eruption	20010504				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/	20010502- -	Synovitis and tenosynovitis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Epilepsy grand mal			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01004277

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 34 Year	weight (kg): 59	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Renal failure acute	20010606				recovering/resolving
2 Creatinine blood increased	20010606				recovering/resolving
3 Overdose effect	20010603				Unknown
4 Conjunctivitis	20010606				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren zaepfchen diclofenac sodium	Rectal	/	20010603-20010606	Backache, unspecified
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol dependence syndrome			k.A.	concomitant disease
Hypercalcemia			k.A.	concomitant disease
Duodenal ulcer, unspecified as acute or chronic, without me			k.A.	concomitant disease
Essential hypertension, unspecified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01004540

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: 33 Year weight (kg): 70 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	200003				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	/	-	Gout, unspecified

Drugs (concomitant)

2 allopurinol allopurinol	Oral	/	-	Gout, unspecified
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Asthma bronchial			k.A.	concomitant disease
Vasculitis			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01004543

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 40 Year weight (kg): 75 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pruritis	20010620				recovering/resolving
2 Skin erythema	20010620				recovering/resolving
3 Allergic reaction	20010620				recovering/resolving
4 Edema at application site	20010620				recovering/resolving
5 Pain	20010620				recovering/resolving
6 Bullous eruption	20010620				recovering/resolving
7 Gait disorder	20010620				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	002 Df dosage form/Day	20010620-20010621	Arthralgia
			-	

Primary source

Qualification: Consumer or other non health professional observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-01008523

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 27 Year	weight (kg): 74	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000608		33 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol allopurinol	Oral	/	20000520- -	Gout, unspecified
2 diclofenac diclofenac	Oral	/	20000520- -	Other general symptoms

Drugs (concomitant)

3 lisino loratadine	Oral	/	20000513- -	Allergic rhinitis due to other allergen
4 pantozol pantoprazole sodium	Oral	/	20000513- -	Oesophagitis NOS
5 uniphyllin theophylline	Oral	/	20000513- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other specified hypertrophic and atrophic conditions of skin			k.A.	concomitant disease
Unspecified arthropathy			k.A.	concomitant disease
Allergic asthma			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02001075

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 31 Year	weight (kg): 72	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	20011007				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	150 Mg milligram(s)/	20011001-20011006	Other general symptoms
2 viramune nevirapine	Oral	200 Mg milligram(s)/Day	20010923-20011009	HIV infection NOS

Drugs (concomitant)

3 combivir lamivudine; zidovudine	Oral	002 Df dosage form/Day	20010906-20011009	HIV infection NOS
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			k.A.	concomitant disease
Osteitis deformans without mention of bone tumour			k.A.	concomitant disease
Influenza with other respiratory manifestations			k.A.	concomitant disease

Primary source

Qualification: observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005286

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Eyelid edema	20011217				Unknown
2 Blepharoconjunctivitis	20011217				Unknown
3 Drug maladministration	20011217				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Intraocular	/	20011217-20011217	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005472

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 41 Year	weight (kg): 70	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20000613				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Intramuscular	/	-	Low back pain
diclofenac			-	
2 paracetamol	Oral	/	-	
paracetamol			-	

Drugs (concomitant)

3 omeprazol	Oral	/	-
omeprazole			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Alcohol dependence syndrome			k.A.	concomitant disease
Alcoholic fatty liver			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005507

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 61 Year	weight (kg): 80	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20010208		22 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Oral	/	-	Other general symptoms
2 benzodiazepine	Unknown	/	-	Other, mixed, or unspecified drug abuse
3 diclofenac diclofenac	Oral	/	-	Other general symptoms
4 thomapyrin acetylsalicylic acid; caffeine; paracetamol	Oral	/	-	Other general symptoms

Drugs (concomitant)

5 antra 20 mg omeprazole	Oral	/	-	Abnormality of secretion of gastrin
6 beloc-zok mite metoprolol succinate	Oral	/	-	Essential hypertension, unspecified
7 omnic 0,4 tamsulosin hydrochloride	Oral	/	-	Hyperplasia of prostate

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02005417

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes

Patient

Initials: Sex: Female Birth: privacy Age: 31 Year weight (kg): 62 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bullous eruption	20000625		4 Week		recovered/resolved
2 Toxic epidermal necrolysis	20000625		4 Week		recovered/resolved
3 Ulcerative stomatitis	20000625		4 Week		recovered/resolved
4 Stevens Johnson syndrome	20000625		4 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 furorese furosemide	Oral	040 Mg milligram(s)/Day	20000618-20000618	Frequency of urination and polyuria
2 rewodina diclofenac sodium	Oral	100 Mg milligram(s)/Day	20000608-20000618	Lumbago
3 Urbason methylprednisolone	Intravenous (not otherwis	1.5 G gram(s)/Day	20000621-20000623	Lupus erythematosus
4 vioxx rofecoxib	Oral	013 Mg milligram(s)/Day	20000619-20000623	Lumbago

Drugs (concomitant)

5 bvk roche tablet biotin; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine mononitrate	Oral	003 lu international unit(s)	20000623-	
6 imurek azathioprine	Oral	100 Mg milligram(s)/Day	20000530-20000619	Lupus erythematosus

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Other specified disorders of skin			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02014497

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes

Patient

Initials: Sex: Male Birth: privacy Age: 55 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bullous eruption	1998				recovered/resolved
2 Burns first degree	1998				recovered/resolved
3 Medication error					Unknown
4 Dermatitis	1998				recovered/resolved
5 Application site reaction	1998				recovered/resolved
6 Vesicular rash	1998				recovered/resolved
7 Skin necrosis	1998				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 iontophorese	Unknown	/	-	
2 voltaren emulgel diclofenac diethylamine	Topical	/	1998-1998	

Drugs (concomitant)

3 Acetylsalizylsaeure acetylsalicylic acid	Unknown	/	-	
4 corvo enalapril maleate	Oral	/	-	
5 insulin insulin	Unknown	/	-	Diabetes mellitus
6 metohexal metoprolol tartrate	Oral	/	-	Essential hypertension, unspecified
7 omnic tamsulosin hydrochloride	Oral	/	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Coronary atherosclerosis			Unknown	concomitant disease
Peripheral vascular disease, unspecified			Unknown	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-02014713

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes

Patient

Initials: Sex: Female Birth: privacy Age: 75 Year weight (kg): 62 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin erythema	20021203		5 Week		recovered/resolved
2 Application site reaction	20021203		5 Week		recovered/resolved
3 Bullous eruption	20021203		5 Week		recovered/resolved
4 Dermatitis	20021203		5 Week		recovered/resolved
5 Pruritis	20021203		5 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/	20021202-20021202	Neck pain

Primary source

Qualification: Consumer or other non health professional observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03001863

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 23 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nephritis interstitial	20021008				recovering/resolving
2 Drug eruption	20020913				Unknown
3 Renal failure acute	20021008				recovering/resolving
4 Creatinine blood increased	20021008				Unknown
5 Urea increased	20021008				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	150 Mg milligram(s)/Day	20020910-20020913	-
2 voltaren zaepfchen diclofenac sodium	Rectal	200 Mg milligram(s)/Day	20020909-20020909	Other specified complications of procedures, not elsewhere

Drugs (concomitant)

3 cortisone cortisone	Unknown	/	-	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lupus erythematosus			k.A.	concomitant disease
Hepatitis, unspecified			k.A.	concomitant disease
Myocarditis, unspecified			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: nasennebenhohlen- u. mandel-operation
Myalgia and myositis, unspecified			k.A.	concomitant disease
Asthma bronchial			k.A.	concomitant disease
Peritonsillar abscess			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03007542

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes

Patient

Initials: Sex: Female Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin necrosis	20020109				recovered/resolved
2 Medication error	20020109				recovered/resolved
3 Injection site inflammation	20020109				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Subcutaneous	1 Df dosage form/	20020109- -	Ischias

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03014760

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 20 Year	weight (kg): 94	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20010914		26 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dexa-gentamicin augentropfen dexamethasone sodium phosphate; gentamicin sulfate	Intraocular	/	-	Other and unspecified conjunctivitis
2 diclac diclofenac sodium	Oral	/	-	Synovitis and tenosynovitis

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03015398

Serious Yes	Death	Lifethr. Yes	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 16 Year	weight (kg): 57	BMI_int: 18.19	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20010408		39 Day		recovered/resolved
2 Thrombocytopenia	200104		6 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-03015398

1	ben-u-ron paracetamol	Oral	500 Mg milligram(s)/Day	20010321-20010328 8 Day	
2	ben-u-ron paracetamol	Oral	001 G gram(s)/Day	20010331-20010331 -	
3	ben-u-ron paracetamol	Rectal	001 G gram(s)/Day	20010408-20010408 -	
4	ben-u-ron paracetamol	Rectal	500 Mg milligram(s)/Day	20010320-20010320 -	Other general symptoms
5	chloral hydrate chloral hydrate	Rectal	/	20010327-20010330 4 Day	
6	chloral hydrate chloral hydrate	Oral	002 G gram(s)/Day	20010327-20010327 -	Other and unspecified special symptoms or syndromes, not
7	dibro-be mono potassium bromide	Oral	/	20010328-20010409 13 Day	Epilepsy, unspecified
8	diclofenac diclofenac	Oral	150 Mg milligram(s)/Day	20010323-20010326 4 Day	Other general symptoms
9	ergenyl valproate sodium	Intravenous (not otherwis	800 Mg milligram(s)/Day	20010329-20010329 -	
10	ergenyl valproate sodium	Intravenous (not otherwis	800 Mg milligram(s)/Day	20010327-20010327 -	Epilepsy grand mal
11	frisium clobazam	Oral	010 Mg milligram(s)/Day	20010327-20010328 2 Day	Other and unspecified special symptoms or syndromes, not
12	heparin sodium heparin sodium	Oral	002 Mg milligram(s)/Day	20010321-20010321 -	Embolism and thrombosis of unspecified site
13	heparin sodium heparin sodium	Oral	002 Mg milligram(s)/Day	20010326-20010330 5 Day	
14	luminal phenobarbital	Intravenous (not otherwis	600 ml milliilitre(s)/Day	20010320-20010320 -	Epilepsy grand mal
15	novalgin metamizole	Oral	500 Mg milligram(s)/Day	20010324-20010324 -	
16	novalgin metamizole	Oral	500 Mg milligram(s)/	20010321-20010321 -	Other general symptoms
17	phenhydan phenytoin	Intravenous (not otherwis	/	20010326-20010328 3 Day	
18	phenhydan phenytoin	Intravenous (not otherwis	/	20010320-20010322 3 Day	Epilepsy grand mal
19	rivotril clonazepam	Intravenous (not otherwis	2.5 Mg milligram(s)/Day	20010327-20010327 -	Epilepsy grand mal
20	tavor lorazepam	Oral	002 Mg milligram(s)/Day	20010408-20010408 -	
21	tavor lorazepam	Oral	002 Mg milligram(s)/Day	20010322-20010322 -	Epilepsy grand mal
22	timonil retard carbamazepine	Oral	100 lu international unit(s)	20010323-20010409 18 Day	Embolism and thrombosis of unspecified site
23	zentropil phenytoin	Oral	/	20010329-20010408 11 Day	Epilepsy, unspecified

Drugs (concomitant)				
24	aciclovir aciclovir	Intravenous (not otherwis 1.5 G gram(s)/Day	20010320-20010324 5 Day	Herpes simplex without mention of complication
25	decortin-h prednisolone	Oral 100 Mg milligram(s)/Day	20010408-20010408 -	Rash and other nonspecific skin eruption
26	dolantin pethidine hydrochloride	Intravenous (not otherwis 015 Mg milligram(s)/Day	20010323-20010323 -	Other general symptoms
27	dolantin pethidine hydrochloride	Intravenous (not otherwis 020 Mg milligram(s)/Day	20010408-20010408 -	
28	kavacur kava-kava rhizoma	Oral 045 Mg milligram(s)/Day	20010312-20010320 9 Day	Other and unspecified special symptoms or syndromes, not
29	paediafusin calcium chloride dihydrate; glucose; magnesium chloride hexahydrate; malic acid; sodium acetate; sodium glycerophosphate	Intravenous (not otherwis 600 ml millilitre(s)/Day	20010320-20010323 4 Day	
30	ringer lactado "grifols" calcium chloride dihydrate; potassium chloride; sodium chloride; sodium lactate	Intravenous (not otherwis 680 ml millilitre(s)/Day	20010323-20010324 2 Day	
31	tbg Basisloesung	Intravenous (not otherwis 300 ml millilitre(s)/Day	20010408-20010409 2 Day	
32	tbg Basisloesung	Intravenous (not otherwis 900 ml millilitre(s)/Day	20010409-20010409 -	
33	valium diazepam	Intravenous (not otherwis 010 Mg milligram(s)/Day	20010408-20010409 2 Day	Partial epilepsy, with impairment of consciousness
34	zyrtec cetirizine hydrochloride	Oral 010 Mg milligram(s)/Day	20010408-20010408 -	Rash and other nonspecific skin eruption

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Partial epilepsy, without mention of impairment of conscious			k.A.	concomitant disease
Allergic rhinitis, cause unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03017228

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes Yes

Patient

Initials: Sex: Male Birth: privacy Age: 58 Year weight (kg): 84 BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Agranulocytosis	20030630				recovered/resolved
2 Pyrexia abnormal	20030630				recovered/resolved
3 Leucopenia	20030630				recovered/resolved
4 Thrush	20030630				recovered/resolved
5 Stomatitis	20030630				recovered/resolved
6 Diarrhea	20030630				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	150 Mg milligram(s)/Day	20030612-20030707	Osteoarthritis, localized, not specified whether primary or
2 novaminsulfon metamizole	Oral	001 G gram(s)/Day	20030612-20030707	Osteoarthritis, localized, not specified whether primary or

Drugs (concomitant)

3 pantozol pantoprazole sodium	Oral	040 Mg milligram(s)/Day	20030612-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: z.n.tep-implantation
unbekannt			k.A.	concomitant disease: z.n.tep-wechsel
unbekannt			k.A.	concomitant disease: z.n.nekr.pankreatitis 1994
Goitre, unspecified			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
 Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-03901094

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Male Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Neck stiffness	200311				Unknown
2 Joint swelling inflammatory	200311				Unknown
3 Rash desquamating	200311				Unknown
4 Infection	200311				Unknown
5 Hypokinesia	200311				Unknown
6 Pain in limb	200311				Unknown
7 Fever	200311				Unknown
8 Borrelia infection NOS	200311				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	/	200311-20031120 -	Arthralgia

Drugs (concomitant)

2 co-trimoxazol sulfamethoxazole; trimethoprim	Unknown	/	200311-20031120 -	Arthralgia
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Arrhythmia			k.A.	concomitant disease
Other general symptoms			k.A.	concomitant disease

Primary source

Qualification: observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000337

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 23 Year	weight (kg): 65	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema multiforme severe	20020124				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 codipront codeine; phenyltoloxamine	Oral	/	-	Cough
2 diclo-puren 50 diclofenac sodium	Unknown	/	-	Pain in limb
3 paracetamol 500 paracetamol	Oral	/	-	Pain in limb
4 tramal tramadol hydrochloride	Unknown	/	-	
5 zithromax azithromycin	Oral	/	-	Influenza with other respiratory manifestations

Drugs (concomitant)

6 orfiril long 300 mg valproate sodium	Oral	/	-	Epilepsy, unspecified
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Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000341

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 30 Year	weight (kg): 78	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20020128		29 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ambene aluminium glycinate; cyanocobalamin; dexamethasone; phenylbutazone; thiamine hydrochloride	Subcutaneous	/	-	Displacement of intervertebral disc, site unspecified, without
2 diclofenac diclofenac	Oral	/	-	Other general symptoms
3 tramal tramadol hydrochloride	Oral	/	-	Other general symptoms

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000602

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 66 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/	-	-

Drugs (concomitant)

2 amitriptylin-neuraxpharm amitriptyline hydrochloride	Unknown	/	-	-
3 durogesic 50 ug/h fentanyl	Unknown	/	-	-
4 dytide h hydrochlorothiazide; triamterene	Unknown	/	-	-
5 enahexal enalapril maleate	Unknown	/	-	-
6 enbrel etanercept	Unknown	/	-	-
7 isozid isoniazid	Unknown	/	-	-
8 laxoberal sodium picosulfate	Unknown	/	-	-
9 metex 7,5 methotrexate sodium	Unknown	/	-	-
10 novalgin filmdabletten metamizole	Oral	/	-	-
11 omep 10 mg omeprazole	Unknown	/	-	-
12 prednisolon prednisolone	Unknown	/	-	-
13 sevredol morphine sulfate	Unknown	/	-	-

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000532

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 56 Year	weight (kg): 75	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug eruption	20031102		5 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo kd 75 diclofenac sodium	Oral	2 Df dosage form/	20031023-20031023	Sciatica
2 dolovisano m dragees mephenesin	Oral	1 Df dosage form/	20031023-20031023	Sciatica
3 procain procaine hydrochloride	Unknown	1 ml millilitre(s)/	20031030-20031030	Sciatica
4 procain-loges 1% procainamide hydrochloride	Unknown	1 ml millilitre(s)/	20031023-20031023	Sciatica

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04000888

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Female Birth: privacy Age: 67 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum	20031125		3 Week		recovered/resolved
2 Oedema	20031125		3 Week		recovered/resolved
3 Dyspnoea	20031125		3 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 bextra valdecoxib	Oral	001 Df dosage form/Day	20031108-20031113	Pain in joint
2 tavor 0,5 tabletten lorazepam	Oral	001 Df dosage form/Day	20031108-20031112	
3 voltaren diclofenac sodium	Oral	001 Df dosage form/Day	20031124-	Pain in joint

Drugs (concomitant)

4 nexium esomeprazole	Unknown	/	-
5 ranitidine ranitidine	Unknown	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Rheumatism, unspecified and fibrositis			k.A.	concomitant disease

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04001156

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 51 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20040124				Unknown
2 Itching	20040124				Unknown
3 Eruption	20040124				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Transdermal	/	-	Enthesopathy of elbow region
2 elmendos 25 mg tabletten lamotrigine	Oral	025 Mg milligram(s)/Day	20040117-20040122	Manic-depressive psychosis, other and unspecified
3 elmendos 50 mg tabletten lamotrigine	Oral	050 Mg milligram(s)/Day	20040123-200401	Manic-depressive psychosis, other and unspecified

Drugs (concomitant)

4 aponal doxepin hydrochloride	Oral	150 Mg milligram(s)/Day	20031224-
5 thyreotom levothyroxine sodium; liothyronine	Oral	001 Df dosage form/Day	20031211-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: nikotin use

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-04900153

Patient

Initials: Sex: Female Birth: privacy Age: 53 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dermatitis medicamentosa	200305				recovered/resolved
2 Skin erythema	200305				recovered/resolved
3 Papulosquamous rash	200305				recovered/resolved
4 Lymphadenopathy	200305				recovered/resolved
5 Pruritis	200305				recovered/resolved
6 Hypersensitivity	200305				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 tetrazepam	Unknown	/	-	Ischias
tetrazepam			-	
2 voltaren	Unknown	/	-	Ischias
diclofenac sodium			-	

Drugs (concomitant)

3	concor bisoprolol fumarate	Oral	/	-	Essential hypertension, unspecified
4	dexamethasone dexamethasone	Intravenous (not otherwis	/	200305-	Ischias
5	tramal tramadol hydrochloride	Intravenous (not otherwis	/	200305-	Ischias
6	vitamin b12 cyanocobalamin	Intravenous (not otherwis	/	200305-	Ischias

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04003543

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 62 Year	weight (kg): 94	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Flu-like symptoms	20040213			1 Day	not recovered/not resolved
2 Heartburn					not recovered/not resolved
3 Lip swelling	20040229			17 Day	not recovered/not resolved
4 Marginal keratitis					not recovered/not resolved
5 Pain of skin	20040221			9 Day	not recovered/not resolved
6 Rash pruritic	200402				not recovered/not resolved
7 Skin disorder	20040225			13 Day	not recovered/not resolved
8 Skin reaction	20040221			9 Day	not recovered/not resolved
9 Vesicle	20040228			16 Day	not recovered/not resolved
10 Mucosal erosion	20040229			17 Day	not recovered/not resolved
11 Stevens Johnson syndrome	20040221	20040312	21 Day	11 Day	not recovered/not resolved
12 CRP increased	20040301			18 Day	not recovered/not resolved
13 ESR increased	20040305			22 Day	Unknown
14 Allergic exanthema	20040215	20040218	4 Day	3 Day	not recovered/not resolved
15 Pruritus	20040221			9 Day	not recovered/not resolved
16 Creatinine increased	20040301			18 Day	not recovered/not resolved
17 Transaminases increased	20040301			18 Day	not recovered/not resolved
18 Chills	20040220			8 Day	not recovered/not resolved
19 Conjunctivitis					not recovered/not resolved
20 Eye redness	20040302			19 Day	not recovered/not resolved
21 Fever	20040220			8 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fenistil dimetindene maleate	Intravenous (not otherwis	1 Df dosage form/Day Solution for injection	20040216-20040216	Itching 1440 Minute
2 PARACETAMOL paracetamol	Oral	/ Tablet	20040216-20040218	Fever 3 Day
3 PARACETAMOL paracetamol	Oral	/ Tablet	20040212-20040212	Fever 1440 Minute
4 prednisolon prednisolone	Oral	/	20040216-20040227	Allergic exanthema 12 Day
5 solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis	250 Mg milligram(s)/Day Solution for injection	20040216-20040216	Exanthema 1440 Minute
6 VOLTAREN RESINAT diclofenac resinate	Oral	2 Df dosage form/Day Capsule	20040213-20040213	Fever 1440 Minute

Drugs (concomitant)

7 concor plus bisoprolol fumarate; hydrochlorothiazide	Unknown	/	-	Hypertension
8 KETEK telithromycin	Oral	1 Df dosage form/Day	20040224-20040224	Purulent bronchitis 1440 Minute

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Dermatitis			Unknown	due to drugs and medicaments taken internally
Hypertension arterial			Unknown	concomitant disease
Upper respiratory tract infection			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 06.02.2008 DE-002147023-PHNU2004DE01207

Sender diagnosis: not reported

Narrative

Initial report received on 12 Mar 2004. Physician reported about patient who took one Voltaren (diclofenac) dose, about 10-12 days ago, for flu symptoms. Shortly thereafter, the patient showed an "extreme" skin reaction, and antibiotic treatment with Ketek (telithromycin) was started. The patient was hospitalized on 1 Mar 2004, and high dose prednisolone over 3 days (starting with 250mg/d, then tapering) was given. Due to affection of the conjunctiva, a dermatological consultation was requested, and Ketek (telithromycin) was stopped. Histology confirmed the diagnosis of Stevens Johnson syndrome. The patient remained on topical treatment, and the condition greatly improved during the following days. The patient's discharge was foreseen for 12 Mar 2004. Both Voltaren and Ketek were named as suspected drugs.

Follow up received on 24 Mar 2004 (written documentation from internal specialist): The patient took Voltaren on 22 Feb 2004, for rheumatic pains due to upper respiratory tract infection. Stevens Johnson syndrome started on ("about") 23 Feb 2004 and lasted for "more than 14 days". According to reporter, pruritic rash started after Voltaren use, and the picture of Stevens Johnson syndrome occurred one day after starting Ketek therapy. On an attached lab sheet, the following "lab data relevant for the AE" were reported: increased creatinine, increased transaminases, CRP increase and ESR increase. The condition did not abate after stopping therapy (no rechallenge since). Outcome was indicated as "not yet recovered". The reporting physician rated causality for Voltaren as probable (Ketek was not explicitly named as suspect drug).

Follow up received on 4 Feb 2008 from RegiSCAR project (internal case number 9150136). The Voltaren drug was specified as Voltaren Resinat (diclofenac, colestyramin); and according to this information (in contrast to previous data), Voltaren Resinat was used on 13 Feb 2004, for fever. No previous Voltaren drug use was recorded in this follow up. Fenistil (dimetinden maleate) was used on 16 Feb 2004, due to pruritus. On 13 Feb 2004, flu like symptoms occurred; and allergic exanthema was present from 15 until 18 Feb 2004. Chills and fever occurred on 20 Feb, and skin pains, pruritus, generalized macules and spots were visible on 21 Feb 2004. Purulent bronchitis was seen on 23 Feb, and skin changes further spread on 25 Feb 2004. The patient later also presented heartburn, marginal keratitis, eye redness, lip swellings and mucosal erosions. The first vesicle was seen on 28 Feb 2004. The patient was hospitalised on 1 Mar 2004. Outcome was reported as "not yet recovered". By the expert from the reporting center, the following drugs were mentioned as suspected (i.e. with a causality of at least possible): Voltaren Resinat, Fenistil inj., paracetamol, Solu-Decortin (prednisolone-21-hydrogensuccinate), and prednisolone; Ketek was not regarded as suspected. As other contributing or causal factor, flu infection was mentioned.

Sender's comment:

Serious adverse event [hospitalisation], Stevens-Johnson syndrome assessed as listed according to the Core Data Sheet.
Reporting healthcare professional causality is suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04900261

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 77 Year	weight (kg): 78	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Myalgia	20040329				recovering/resolving
2 Rhabdomyolysis	20040329				recovering/resolving
3 Asthenia	20040329				recovering/resolving
4 Oedema peripheral	20040329				recovering/resolving
5 Creatine phosphokinase increased	20040329				recovering/resolving
6 Confusion	20040429				recovering/resolving
7 Blisters	20040405				Unknown
8 Tiredness	20040329				recovering/resolving
9 LDH increased	20040329				recovering/resolving
10 SGOT increased	20040329				recovering/resolving
11 Lyell syndrome	20040405				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 novalgine metamizole	Oral	/	-20040416 -	Closed fracture of lumbar vertebra without mention of
2 voltaren resinat diclofenac resinate	Oral	/	-20040331 -	Other general symptoms

Drugs (concomitant)

3 Acetylsalizylsaeure acetylsalicylic acid	Unknown	100 Mg milligram(s)/Day	- -	
4 actonel risedronic acid	Unknown	/	- -	
5 Biphosphonat	Unknown	/	- -	
6 kuerbisoeel cucurbita pepo seed oil	Unknown	/	- -	
7 rofecoxib rofecoxib	Unknown	025 Mg milligram(s)/Day	20040416- -	Other general symptoms
8 tramal tramadol hydrochloride	Oral	200 Mg milligram(s)/Day	20040429-20040430 -	Closed fracture of lumbar vertebra without mention of

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Abdominal pain			k.A.	concomitant disease
Osteoporosis			k.A.	concomitant disease
Hypertension arterial			k.A.	concomitant disease
unbekannt			k.A.	concomitant disease: schmerzsyndrom
Closed fracture of lumbar vertebra without mention of spina			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04007415

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 71 Year	weight (kg): 94	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin vasculitis NOS	20040328		21 Day		recovered/resolved
2 Nephritis interstitial	20040328		21 Day		recovered/resolved
3 Renal failure acute	20040328		21 Day		recovered/resolved
4 Fever	20040328		21 Day		recovered/resolved
5 Malaise	20040328		21 Day		recovered/resolved
6 Weakness generalized	20040328		21 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	50 Mg milligram(s)/	-	Backache, unspecified
diclofenac			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Acute bronchitis			k.A.	concomitant disease
Hypertension arterial			k.A.	concomitant disease
Atrial fibrillation and flutter			k.A.	concomitant disease
Right bundle branch block			k.A.	concomitant disease
Hyperparathyroidism secondary			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04010789

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 58 Year	weight (kg): 87	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Anaphylaxis	20040427		1 Week		recovered/resolved
2 Drug eruption	20040427		1 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclac diclofenac sodium	Oral	/	20040427-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergy, unspecified, not elsewhere classified			k.A.	concomitant disease

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04013785

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Female Birth: privacy Age: 80 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering	20041009				not recovered/not resolved
2 Erythema	20041009				not recovered/not resolved
3 Allergic skin reaction	20041009				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	002 Df dosage form/Day	20041009-20041010	-

Drugs (concomitant)

2 venostasin-gel escin; heparin	Topical	/	-	-
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Primary source

Qualification: Pharmacist observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-04014894

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 63 Year	weight (kg): 100	BMI_int: 34.6	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pulse rate increased	20040927		5 Hour	1 Day	recovered/resolved
2 Sweating attack	20040927		5 Hour	1 Day	recovered/resolved
3 Drug eruption	20040930			4 Day	recovered/resolved
4 Low blood pressure	20040927		5 Hour	1 Day	recovered/resolved
5 Rash	20040927			1 Day	recovered/resolved
6 Anaphylactic shock	20040927		5 Hour	1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	1 Df dosage form/Day	20040927- -	Low back pain

Drugs (concomitant)

2 allopurinol allopurinol	Oral	300 Mg milligram(s)/Day	-	Hyperuricaemia
3 codiovan hydrochlorothiazide; valsartan	Oral	1 Df dosage form/Day	-	Essential hypertension, unspecified
4 hct hexal 25 mg hydrochlorothiazide	Oral	0.5 Df dosage form/Day	-	Essential hypertension, unspecified
5 metoprolol metoprolol	Oral	200 Mg milligram(s)/Day	-	Essential hypertension, unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cough			Unknown	
Hyperuricaemia			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05001295

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.

Patient

Initials: Sex: Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin reaction localised					Unknown
2 Exfoliation of skin					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm gel	Topical	/	-	
diclofenac sodium			-	

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05002223

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 67 Year	weight (kg): 92	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug eruption	20050205				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac 50 mg diclofenac sodium	Oral	/Day	20050203-	-

Drugs (concomitant)

2 delix 5 plus hydrochlorothiazide; ramipril	Oral	001 Df dosage form/Day	200311-20050125	Thrombosis
3 plavix 75 mg filmdabletten clopidogrel sulfate	Oral	001 Df dosage form/Day	200311-20050125	Hypertonia
4 xalacom latanoprost; timolol	Ophthalmic	001 Df dosage form/Day	200406-20050205	Glaucoma, unspecified

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	concomitant disease: zustand n. pericardresektion
unbekannt			k.A.	concomitant disease: zustand n. hirnstamminfarkt

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05003033

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
 Yes

Patient

Initials: Sex: Female Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Necrosis skin					Unknown
2 Abscess					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Intramuscular	/	-	-

Primary source

Qualification:

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-05005285

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell's syndrome					fatal
2 Sepsis NOS					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	Low back pain
diclofenac			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Z rheumatol 2003,vol.62:474-75	

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05006363

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No				Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 50 Year	weight (kg): 70	BMI_int: 24.22	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug eruption	20050318			1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Voltaren Resinat diclofenac resinate		/	20050318-20050318	Pain management
			-	

Drugs (concomitant)

2 amoclav 875 mg + 125 mg tabletten amoxicillin trihydrate; clavulanate potassium	2 Df dosage form/Day	20050316-	Pneumonia NOS
		-	
3 Pantozol 20 mg pantoprazole sodium	1 Mg milligram(s)/Day	20050318-	
		-	
4 Zop zopiclone	1 Mg milligram(s)/Day	-	Sleep disorder NOS
		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Nondependent abuse of tobacco			Yes	
Hypersensitivity NOS			Yes	
Pneumonia NOS			Yes	
Pneumothorax NOS			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

case: DE-BFARM-05008833

Patient

Reactions

Drugs (suspected or interacting)

Primary source

166 / 652

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-05014574

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Female Birth: privacy Age: 73 Year weight (kg): 74 BMI_int: 27.18 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Glaucoma					recovering/resolving
2 Weight increased	20050919				recovering/resolving
3 Condition aggravated	20050919				recovering/resolving
4 Visual acuity reduced	20050919				recovering/resolving
5 Conjunctivitis	20051111				recovering/resolving
6 Hypertension	20050919				recovering/resolving
7 Intraocular pressure increased	20050919				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Voltaren Resinat diclofenac resinate		1 Df dosage form/Day	20050918-20050918	-
2 Voltaren Resinat diclofenac resinate		1 Df dosage form/Day	20050916-20050916	-
3 Voltaren Resinat diclofenac resinate	Oral	75 Mg milligram(s)/Day	20050914-20050914	Intervertebral disc protrusion
4 Voltaren Resinat diclofenac resinate	Oral	1 Df dosage form/Day	20051022-20051023	2 Day
5 Voltaren Resinat diclofenac resinate	Oral	1 Df dosage form/Day	20051108-20051110	3 Day

Drugs (concomitant)

6 algaphan aminophenazone; dextropropoxyphene hydrochloride		/	-	Intraocular pressure increased
7 Kaliumchlorid potassium chloride		/	-	Intraocular pressure increased
8 tim-ophtal timolol maleate		/	-	Intraocular pressure increased
9 trusopt dorzolamide hydrochloride		/	-	Intraocular pressure increased
10 xalatan latanoprost	Ophthalmic	/	-	Intraocular pressure increased

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Glaucoma			Yes	glaucoma with scotoma on the right eye
Blood potassium decreased			Yes	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06006917

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 52 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Trichophyton infection					recovered/resolved
2 Herpes zoster					recovered/resolved
3 Eosinophil count increased	200608				Unknown
4 Skin disorder					recovered/resolved
5 Acute generalised exanthematous pustulosis	200608				recovered/resolved
6 Pyrexia					recovered/resolved
7 Streptococcus serology positive					recovered/resolved
8 C-reactive protein increased					recovered/resolved
9 Leucocytosis NOS					recovered/resolved
10 Swelling face					recovered/resolved
11 Malaise					recovered/resolved
12 GPT increased					recovered/resolved
13 GOT increased					recovered/resolved
14 GGT increased					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	Arthropathy
diclofenac			-	
2 phenoxymethylpenicillin potassium	Oral	/	-	Nasopharyngitis
phenoxymethylpenicillin potassium			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Birte Prange, Alessandra Marini, et al. "Acute localized exanthematous pustulosis (ALEP)" J Dtsch Dermatol Ges 2005: (Vol. 3): 210-212	

Narratives (nur bei elektronischem Eingang)

Merck KGaA, 24.05.2006 DE-Merck KGaA-6022315

Sender diagnosis: not reported

Narrative

Literature report received on 28 Apr 2006.

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A 52-year-old female patient developed acute localised exanthematous pustulosis after intake of penicillin and diclofenac. She presented with a common cold with sore throat, and one day after penicillin therapy, she complained of generalised malaise, redness and swelling in front of the right ear and on the right cheek as well as in the area of the right lower eyelid, and fever. At the time of presentation, she showed a large, painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localised exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with penicillin and diclofenac was negative. The reporters state that the event was probably caused by penicillin or diclofenac.

Note: The authors do not suspect an interaction between the both drugs taken by the patient. It is not clear which one of either drug was causative in the end.

Sender's comment:

Temporal relationship between suspected drug and adverse reaction: plausible
 Pharmacological relationship between suspected drug and adverse reaction: plausible
 Observed reaction is a sign/symptom of underlying disease: not probable
 History of observed event: no
 Other risk factors present: probable
 Sufficiency of documentation: sufficient
 Overall company causality assessment of relationship drug / adverse reaction: possible

The events are known as possible side effects of diclofenac.

The suspected "penicillin" was not further specified, e.g. if penicillin G or penicillin V or another derivate was concerned.

Reporter's comment:

ratiopharm GmbH, 14.07.2006 DE-RATIOPH-2006R072034

Sender diagnosis: not reported

Narrative

A 52-year-old female patient developed acute localised exanthematous pustulosis after intake of Penicillin and Diclofenac. She presented with a common cold with sore throat, and one day after Penicillin therapy, she complained of generalised malaise, redness and swelling in front of the right ear and on the right cheek as well as in the area of the right lower eyelid, and fever. At the time of presentation, she showed a large, painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localised exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with Penicillin and Diclofenac was negative. The reporters state that the event was probably caused by Penicillin or Diclofenac. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

The term "acute localized exanthematous pustulosis (ALEP)" is a new possible entity of AGEP and not literally listed in the RSI of Diclofenac and Penicillin. But as various skin reactions up to serious reactions like SJS are listed in both RSIs, this information about severe possible skin reactions is considered to be sufficient and covering also the ADR ALEP. This is in accordance to the intention of informing the prescribing physician about a large spectrum of possible side effects whereas ALEP is a new diagnosis, known mainly in dermatology speciality.

Reporter's comment:

ratiopharm GmbH, 14.07.2006 DE-RATIOPH-2006R072034

Sender diagnosis: not reported

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Reporter's comment:SANOFI-SYNTHELABO, 12.09.2006 DE-SANOFI-SYNTHELABO-A04200600858

Sender diagnosis: not reported

Narrative

Reaction as reported: acute generalised exanthema pustulosis
LLT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799
PT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799

Reaction as reported: increase of eosinophils
LLT: EOSINOPHIL COUNT INCREASED / 10014945
PT: EOSINOPHIL COUNT INCREASED / 10014945

Narrative:

Initial report received 30-Aug-2006

A hospital physician reported via phone about a 69-year-old female patient who was treated with terbinafine HCL (Octosan) at a dose of 250 mg. Indication not known. After several days of treatment (onset: unknown), she presented acute generalised exanthema (pustulosis) with increase of eosinophils. No medical history, underlying diseases or concomitant medication reported.

At the time of report no outcome was mentioned.

Further information will be requested.

Sender's comment:

07 -SEP-2006 SA Germany - Pharmacovigilance

All reported clinical events are considered locally labelled. Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:SANOFI-SYNTHELABO, 04.10.2006 DE-SANOFI-SYNTHELABO-A04200600858

Sender diagnosis: not reported

Narrative

Reaction as reported: acute generalised exanthema pustulosis (AGEP)
LLT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799
PT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799

Reaction as reported: increase of eosinophils
LLT: EOSINOPHIL COUNT INCREASED / 10014945
PT: EOSINOPHIL COUNT INCREASED / 10014945

Narrative:

Initial report received 30-Aug-2006

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At the time of report no outcome was mentioned.

Further information will be requested.

Addendum received on 20-Sep-2006

The patient was hospitalised in August 2006 because of acute generalised exanthema pustulosus (AGEP). Octosan (terbinafine HCL) was discontinued. The patient was treated with prednisone at a dose of 80 mg daily without success. Again occurrence of pustules on the whole body mainly on extremities. In the 3rd week dose was increased to 90 mg. No change of patients situation. Dosage was reduced to maintenance dose of 10 mg in week five of hospitalisation.

Local treatment with betamethasone hydrogenphosphate-dinatrium ointment. In week six the patient is still hospitalised. Outcome: ongoing.

Results of smear of pustules, skin and foot showed massive staphylococcus aureus. X-ray of the thorax was negative. Sonography of upper abdomen was negative. Inguinal sonography showed reactive lymph nodes. Microscopic result of skin of neck: showed suspicion of drug induced exanthema.

Configured erythema on the whole inguiment.

The reporting physician assessed the case as "highly probable".

No further information is awaited.

Sender's comment:

22-SEP-2006 SA Germany - Pharmacovigilance

All reported clinical events are considered locally labelled. Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06007255

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 71 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering					Unknown
2 Skin red					Unknown
3 Burning sensation skin					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel	Topical	/	-	
diclofenac diethylamine			-	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06016435

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 51 Year	weight (kg): 60	BMI_int: 21.01	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Atelectasis	20060402				not recovered/not resolved
2 Blood bicarbonate decreased	20060402				not recovered/not resolved
3 Bradycardia					not recovered/not resolved
4 Chills	20060401				not recovered/not resolved
5 Conduction disorder	20060402				not recovered/not resolved
6 CRP increased					not recovered/not resolved
7 PO2 decreased	20060402				not recovered/not resolved
8 PTT prolonged	20060402				not recovered/not resolved
9 Pulmonary congestion					not recovered/not resolved
10 Respiratory insufficiency	20060402				not recovered/not resolved
11 Retrosternal pain	20060403				not recovered/not resolved
12 Sinus tachycardia	20060402				not recovered/not resolved
13 Speech disorder	20060402				not recovered/not resolved
14 Stenocardia					not recovered/not resolved
15 Vasodilatation					not recovered/not resolved
16 Vomiting	20060401				not recovered/not resolved
17 Weakness	20060402				not recovered/not resolved
18 Negative T wave	20060402				not recovered/not resolved
19 Base excess increased	20060402				not recovered/not resolved
20 Drug intolerance	20060402				not recovered/not resolved
21 Quick's value decreased	20060402				not recovered/not resolved
22 Stomatitis	20060401		4 Day		not recovered/not resolved
23 Diarrhoea	20060401		4 Day		not recovered/not resolved
24 Urinary infection	20060401		4 Day		not recovered/not resolved
25 Thrombocytopenia	20060401		4 Day		not recovered/not resolved
26 Pleural effusion	20060402		4 Day		not recovered/not resolved
27 Pneumonitis	20060401		4 Day		not recovered/not resolved
28 Pulmonary embolism	20060401		4 Day		not recovered/not resolved
29 Postoperative complication	20060401		4 Day		not recovered/not resolved
30 Hematoma					not recovered/not resolved
31 Hyperventilation	20060402				not recovered/not resolved
32 Hypokalemia	20060402				not recovered/not resolved
33 Leukocytosis	20060402				not recovered/not resolved
34 Nausea	20060401				not recovered/not resolved
35 Oral pain	20060402				not recovered/not resolved
36 Oxygen saturation decreased	20060402				not recovered/not resolved
37 Anemia	20060402				not recovered/not resolved
38 Anxiety					not recovered/not resolved
39 Arterial embolism NOS	20060402				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	Celebrex 200 mg Hartkapseln celecoxib	Oral	200 Mg milligram(s)/Day	20060331-20060331 Pain management 1440 Minute
2	Dulcolax bisacodyl	Rectal	2 Df dosage form/Day	20060401-20060401 Obstipation 1440 Minute
3	LAXOBERAL sodium picosulfate		/	-
4	mono-embolex certoparin sodium		/	-
5	Voltaren 50 Zaepfchen diclofenac sodium	Rectal	100 Mg milligram(s)/Day	20060401-20060401 Pain management 1440 Minute

Drugs (concomitant)

6	anaesthetika		/	-
7	HEPARIN heparin	Intravenous (not otherwis	/	-
8	Hormone		/	-
9	REFLUDAN lepirudin		/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Uterine myoma			Yes	Uterus myomatosus
Platelet count decreased			Yes	
Pulmonary embolism			Yes	
Hallux malleus			Unknown	Hallux malleus operation of both feet in 2000
Hypermenorrhea			Unknown	
Suprapubic catheter insertion			Unknown	
Vaginal hysterectomy			Unknown	Vaginal hysterectomy with plasty and installation of a suprapubic catheter due to hypermenorrhea

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		
Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 12.07.2006 DE-002147023-GXKR2006DE02293

Sender diagnosis: not reported

Narrative

Initial report received on 21 Apr 2006 from German Health Authority, originating source anesthesiologist, BfArM number 06016435. The patient was treated with Celebrex (celecoxib) on 31 Mar 2006, and on 1 Apr 2006 with Dulcolax (bisacodyl). On 1 Apr 2006, also Voltaren suppositories were started, as a postoperative analgetic. Under the section for "other comments" (not in the adverse event section), a "parallel suspicion of HIT II (clinically acute pulmonary arterial embolism and thrombocyte drop unter Mono Embolex)" was mentioned. No HIT-test findings were known so far, and no further data were given. Furthermore, on 1 Apr 2006, patient showed stomatitis and diarrhea, lasting for 4 days. Antibacterial and antimycotic local therapy was initiated. Outcome for these events was indicated as "not yet recovered".

Follow-up report received from the anesthesiologist via German HA (BfArM no. 06016435) on 18 May 2006: An ICU transfer report was provided where the following diagnoses were listed: postoperative acute pulmonary artery embolism with associated pleuropneumonia and pleural effusion (right), condition after vaginal hysterectomy due to hypermenorrhea, uterine myoma with postoperative urinary infection and secondary thrombocytopenia, diarrhea, stomatitis during drug intolerance (Dulcolax suppositories (bisacodyl), Voltaren suppositories (diclofenac), Celebrex (celecoxib), Laxoberal (sodium picosulfate)), hypokalemia. On 30 Mar 2006 the patient with a history of hallux malleus operation of both feet in 2000 was hospitalized electively for uterus extirpation due to hypermenorrhea with uterine myoma. At admission the patient did not report about any ongoing drug therapy but stated later that she received a hormonal once every three months injection (not specified). Additionally nicotine abuse had been existent until about three weeks before hospitalization. At anamnesis the patient showed no pain and no cardiopulmonary disorder. ECG was normal except a mild bradycardiac sinus rhythm. Preoperative lab data were normal except anemia. On 31 Mar 2006 vaginal hysterectomy with plasty and installation of a suprapubic catheter was performed with normal anesthesia (not specified). The intra- and postoperative course was normal

except nausea and vomiting on the first postoperative day. In the evening of 01 Apr 2006 the patient complained about chills and fever of 39.8°C with general weakness, speech disorder and showed mild dyspnea on the next morning. She was transferred to operative ICU with sinus tachycardia (HF 115bpm), mild right heart conduction delay and negative T wave (lead V4-V6) in the ECG. Blood gas analysis showed a partial respiratory insufficiency with compensating hyperventilation (pH 7.47, PCO₂ 31.0mmHg, base excess 0.1, blood bicarbonate 22.4mmol/l, PO₂ 66.1mmHg, oxygen saturation 93.9%). Further lab analysis showed the following abnormal findings: Hb 9.3g/dl, leukocytosis of 12800/Åµl, decreased thrombocyte count of 120000/Åµl (preoperative value: 170000/Åµl), hypokalemia of 3.2mmol/l, QuickâĶs value 43%, PTT 52sec, D dimer 3.6Åµg/ml. Additionally the patient complained about pain in oral cavity, lips and tongue associated with speech disorders. In parallel watery stools occurred which were associated with hypokalemia. After abdominal sonography a small pleural effusion was suspected. Abdominal spiral CT analysis confirmed mild bilateral pleural effusions and showed a mass of about 8cm near left iliac bone with mild increased abdominal fluid accumulation which was regarded as hematoma. Thoracic X-ray analysis was normal except a small discoid atelectasis (left). As the complaints were regarded as related to the diarrhea balanced infusion therapy (not specified) was performed. Additionally the patient received prophylaxis with a low molecular heparin (not specified, it remained unclear if Mono-Embolex was given at this time) and a proton pump inhibitor (not specified). Bacterial and viral stool analysis was negative. On 03 Apr 2006 diarrhea abated whereas stomatitis was still ongoing and was treated with chlorhexidine (not specified) and Candiohermal (nystatin) mouth rinses and benzocaine lozenges (not specified). At examination the patient complained about retrosternal pain during deep breathing. Blood gas analysis showed a distinct partial respiratory insufficiency (pH 7.46, PCO₂ 38.2mmHg, base excess -0.1, blood bicarbonate 24.5mmol/l, PO₂ 48.0mmHg, oxygen saturation 89.8%): Right heart conduction delay was ongoing whereas repolarization disturbances were no longer detectable. Lung scintigraphy indicated two small triangle-like perfusion defects of the right lung (ventrally and dorsally) and acute pulmonary embolism was suspected. Duplex sonography of leg veins showed no indication for deep vein thrombosis. Intravenous heparin (not specified) administration was performed. Due to a decrease of thrombocyte count and the fact that a heparin-induced thrombocytopenia could not be excluded the patient was switched to Refludan (lepirudin) after change of central venous catheter and infusion system on 04 Apr 2006. On 06 Apr 2006 subcutaneous injection of heparin (not specified, 7500IU three times daily) was re-initiated based on the results of heparin-induced thrombocytopenia- and ELISA-testing (therefore suspicion of HIT was deleted as event). In the further course the patient showed a central and basal pulmonary congestion with pleural effusions and possible infiltrates which was regarded as matching to a post embolic pneumonia. On 05 Apr 2006 therapy with ampicillin and sulbactam (not specified, first iv and since 07 Apr 2006 oral administration) was started as the patient showed subfebrile temperatures, a mild increase of leukocytes and a positive urotube test for coagulase-negative staphylococci. During this therapy body temperature and CRP level decreased, but leukocytosis aggravated to 16600/Åµl. Furthermore the patient received elastic stockings and was mobilized carefully. Till transfer from ICU the patient showed a clear dyspnea on exertion whereas radiological findings and oxygenation were improving. On 07 Apr 2006 the patient was transferred. On this day partial respiratory insufficiency was still ongoing (pH 7.45, PCO₂ 34.1mmHg, base excess -0.1, blood bicarbonate 24.5mmol/l, PO₂ 54.9mmHg, oxygen saturation 91.5%) and in the morning the patient had complained about stenocardia-like symptoms with dyspnea, retrosternal pain and anxiety. Although troponin T was normal and D dimer was decreasing ECG analysis showed a negative T wave (V2). Therefore trans-thoracic echocardiography was performed and showed dilatation of vena cava and small bilateral pleural effusions. At this time stomatitis was clearly resolving and abdominal examination was normal. As the last bowel movement had already been on 03 Apr 2006 metoclopramid (not specified) and Movicol (macrogol) were prescribed as a relationship between Dulcolax suppositories and Laxoberal and the stomatitis and diarrhea was suspected. It remained unclear when the patient received Voltaren suppositories and Mono-Embolex.

Significant follow up due to internal review on 11 Jul 2006: Novartis Comment for certoparin was changed.

Sender's comment:

For certoparin:

Serious spontaneous report (hospitalization) assessed as unlisted for postoperative urinary infection, stomatitis during drug intolerance, diarrhea, mild bradycardic sinus rhythm, leukocytosis and anemia according to the Core Data Sheet Information.

However, alternative causes (underlying condition, co-suspected drug) provide a possible explanation for the reported events.

Serious spontaneous report (hospitalization) assessed as listed for postoperative acute pulmonary artery embolism associated with pleuropneumonia and pleural effusion, hematoma, nausea and vomiting according to the Core Data Sheet Information.

All spontaneous reports are considered suspected for reporting purposes.

For diclofenac:

Serious spontaneous report (hospitalization) assessed as unlisted for postoperative acute pulmonary artery embolism associated with pleuropneumonia and pleural effusion, postoperative urinary infection, hematoma, mild bradycardic sinus rhythm and leukocytosis according to the Core Data Sheet Information. However other alternative causes (underlying condition, other suspected drugs) provide a possible explanation for the reported adverse events.

All spontaneous reports are considered suspected for reporting purposes.

Serious spontaneous report (hospitalization) assessed as listed for stomatitis during drug intolerance, diarrhea, anemia, nausea and vomiting according to the Core Data Sheet Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 08.05.2007 DE-002147023-GXKR2006DE02293

Sender diagnosis: not reported

Narrative

Initial report received on 21 Apr 2006 from German Health Authority, originating source anesthesiologist, BfArM number 06016435. The patient was treated with Celebrex (celecoxib) on 31 Mar 2006, and on 1 Apr 2006 with Dulcolax (bisacodyl). On 1 Apr 2006, also Voltaren suppositories were started, as a postoperative analgetic. Under the section for "other comments" (not in the adverse event section), a "parallel suspicion of HIT II (clinically acute pulmonary arterial embolism and thrombocyte drop unter Mono Embolex)" was mentioned. No HIT-test findings were known so far, and no further data were given. Furthermore, on 1 Apr 2006, patient showed stomatitis and diarrhea, lasting for 4 days. Antibacterial and antimycotic local therapy was initiated. Outcome for these events was indicated as "not yet recovered".

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As the complaints were regarded as related to the diarrhea balanced infusion therapy (not specified) was performed. Additionally the patient received prophylaxis with a low molecular heparin (not specified, it remained unclear if Mono-Embolex was given at this time) and a proton pump inhibitor (not specified). Bacterial and viral stool analysis was negative. On 03 Apr 2006 diarrhea abated whereas stomatitis was still ongoing and was treated with chlorhexidine (not specified) and Candiohermal (nystatin) mouth rinses and benzocaine lozenges (not specified). At examination the patient complained about retrosternal pain during deep breathing. Blood gas analysis showed a distinct partial respiratory insufficiency (pH 7.46, PCO₂ 38.2mmHg, base excess -0.1, blood bicarbonate 24.5mmol/l, PO₂ 48.0mmHg, oxygen saturation 89.8%): Right heart conduction delay was ongoing whereas repolarization disturbances were no longer detectable. Lung scintigraphy indicated two small triangle-like perfusion defects of the right lung (ventrally and dorsally) and acute pulmonary embolism was suspected. Duplex sonography of leg veins showed no indication for deep vein thrombosis. Intravenous heparin (not specified) administration was performed. Due to a decrease of thrombocyte count and the fact that a heparin-induced thrombocytopenia could not be excluded the patient was switched to Recludan (lepirudin) after change of central venous catheter and infusion system on 04 Apr 2006. On 06 Apr 2006 subcutaneous injection of heparin (not specified, 7500IU three times daily) was re-initiated based on the results of heparin-induced thrombocytopenia- and ELISA-testing (therefore suspicion of HIT was deleted as event). In the further course the patient showed a central and basal pulmonary congestion with pleural effusions and possible infiltrates which was regarded as matching to a post embolic pneumonia. On 05 Apr 2006 therapy with ampicillin and sulbactam (not specified, first iv and since 07 Apr 2006 oral administration) was started as the patient showed subfebrile temperatures, a mild increase of leukocytes and a positive urotube test for coagulase-negative staphylococci. During this therapy body temperature and CRP level decreased, but leukocytosis aggravated to 16600/Åµl. Furthermore the patient received elastic stockings and was mobilized carefully. Till transfer from ICU the patient showed a clear dyspnea on exertion whereas radiological findings and oxygenation were improving. On 07 Apr 2006 the patient was transferred. On this day partial respiratory insufficiency was still ongoing (pH 7.45, PCO₂ 34.1mmHg, base excess -0.1, blood bicarbonate 24.5mmol/l, PO₂ 54.9mmHg, oxygen saturation 91.5%) and in the morning the patient had complained about stenocardia-like symptoms with dyspnea, retrosternal pain and anxiety. Although troponin T was normal and D dimer was decreasing ECG analysis showed a negative T wave (V2). Therefore trans-thoracic echocardiography was performed and showed dilatation of vena cava and small bilateral pleural effusions. At this time stomatitis was clearly resolving and abdominal examination was normal. As the last bowel movement had already been on 03 Apr 2006 metoclopramid (not specified) and Movicol (macrogol) were prescribed as a relationship between Dulcolax suppositories and Laxoberal and the stomatitis and diarrhea was suspected. It remained unclear when the patient received Voltaren suppositories and Mono-Embolex.

Significant follow up due to internal review on 11 Jul 2006: Novartis Comment for certoparin was changed.

Sender's comment:

For certoparin:

Serious spontaneous report (hospitalization) assessed as unlisted for postoperative urinary infection, stomatitis during drug intolerance, diarrhea, mild bradycardic sinus rhythm, leukocytosis and anemia according to the Core Data Sheet Information.

However, alternative causes (underlying condition, co-suspected drug) provide a possible explanation for the reported events.

Serious spontaneous report (hospitalization) assessed as listed for postoperative acute pulmonary artery embolism associated with pleuropneumonia and pleural effusion, hematoma, nausea and vomiting according to the Core Data Sheet Information.

All spontaneous reports are considered suspected for reporting purposes.

For diclofenac:

Serious spontaneous report (hospitalization) assessed as unlisted for postoperative acute pulmonary artery embolism associated with pleuropneumonia and pleural effusion, postoperative urinary infection, hematoma, mild bradycardic sinus rhythm and leukocytosis according to the Core Data Sheet Information. However other alternative causes (underlying condition, other suspected drugs) provide a possible explanation for the reported adverse events.

All spontaneous reports are considered suspected for reporting purposes.

Serious spontaneous report (hospitalization) assessed as listed for stomatitis during drug intolerance, diarrhea, anemia, nausea and vomiting according to the Core Data Sheet Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06062913

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 66 Year	weight (kg): 79	BMI_int: 29.73	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Rash vesicular	20060326		9 Day	3 Day	recovered/resolved
2 Pruritus	20060326		9 Day	3 Day	recovered/resolved
3 Blister	20060326	20060403	9 Day	3 Day	recovered/resolved
4 Burns second degree	20060326		9 Day	3 Day	recovered/resolved
5 Pain	20060326		9 Day	3 Day	recovered/resolved
6 Skin burning sensation	20060326		9 Day	3 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Voltaren Emulgel diclofenac diethylamine	Topical	/	20060324-20060327 4 Day	Joint dislocation

Drugs (concomitant)

2 diclofenac 50 mg diclofenac sodium	/	-	-	Pain in extremity
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fall	200603		No	
Edema peripheral	200603		Yes	swelling of foot

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 09.05.2007 DE-002147023-PHNU2006DE01601

Sender diagnosis: not reported

Narrative

Initial consumer report received on 06 April 2006. The patient has been treated with Voltaren (diclofenac) Emulgel applications due to foot swelling and pain and developed burning, skin rash, with wet and itching blisters. The physician diagnosed burn of II. degree. After adequate therapy the skin was dry and peeled. Condition improved.

Follow up combining information received on 19 Apr 2006 (short letter from patient) and on 21 Apr 2006 (documentation sheets filled out by physician). The patient stated that she forwarded the follow up request to the treating physician. She added that the skin reaction was accompanied by severe pain over several days and nights. Voltaren Emulgel was used from 24 Mar until 27 Mar 2006 due to upper leg distorsion. The treating physician described the event as tension bullae which started on 26 Mar 2006 and lasted until 3 Apr 2006. The patient fully recovered after stopping therapy. Causality was rated as definite, and the reporter excluded other causative factors.

Non-significant follow up received on 8 Sep 2006. No new medical data were included.

Non-significant follow up received on 20 Nov 2006 (mail from patient). No new medical data were received

Sender's comment:

Serious spontaneous report, 'tension bullae' (medically significant), assessed as unlisted according to the Basic Prescribing Information due to greater specificity/severity.

The following term (rash, eczema, erythema, dermatitis contact, dermatitis bullous) is already included in the Basic Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06017982

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 51 Year	weight (kg): 60	BMI_int: 21.01	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Thrombocytopenia					not recovered/not resolved
2 Watery diarrhoea	20060401		4 Day		recovered/resolved
3 Heparin-induced thrombocytopenia type II					not recovered/not resolved
4 Hypokalaemia	20060401		4 Day		recovered/resolved
5 Stomatitis	20060401		4 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 dulcolax bisacodyl	Rectal	/ Suppository	20060401- -	Laxative supportive care
2 DULCOLAX bisacodyl	Rectal	50 Mg milligram(s)/ Suppository	20060401- -	Laxative supportive care
3 voltaren diclofenac sodium	Rectal	50 Mg milligram(s)/ Suppository	20060401- -	Analgesic medication supportive therapy

Drugs (concomitant)

4 mono-embolex certoparin sodium	/	- -
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Uterine myoma			k.A.	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 26.04.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-02032DE

Sender diagnosis: not reported

Narrative

This is a health-care professional confirmed report from the german registries (BfArM).

Initial information has been received from a physician concerning a 56-year-old female, german patient.

The patient has a history of myomatous uterus, it is reported that there are no concomitant diseases.

The patient was placed on Dulcolax, 2 suppositories daily since 01-April-06 as postoperative laxative. She also took Voltaren (diclofenac) suppository, 100 mg daily since 01-April-06 as postoperative analgetics and Celebrex (celecoxib), 200 mg daily as analgetic prevention since 31-March-06.

Information on further concomitant drugs was not provided.

On 01-April-06 the patient developed stomatitis and diarrhea, which lasted for 4 days. Therapy included mouth irrigation with candio-hermal (nystatin) and chlorhexidine and lozenges with benzocaine. Additionally there was a suspicion of HIT II (heparin induced thrombocytopenia type II) with an acute pulmonary arterial embolism and a drop of thrombocytes during therapy of Mono-embolex (certoparin-sodium).

The adverse events have been described as not to be life-threatening.

Laboratory analyses without finding. A HIT test was performed but there was no result available at the moment.

Information on the treatment of the events was not provided.

At the moment of this report the patient was not yet recovered.

Further information is not expected.

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim
 Comment on case 2006-DE-02032DE / DULCOLAX

Although the serious event DIARRHEA is listed for the drug in question, other factors have been reported in this case which may equally plausibly have caused the event (co-medication). The extent to which the suspect drug and/or the confounding factors may have contributed to the occurrence of the event(s) is not clear.

Heparin induced thrombocytopenia type II with its symptoms / outcome ACUTE PULMONARY ARTERIAL EMBOLISM and DROP OF THROMBOCYTES is clearly not related to Dulcolax.

No change in the company core safety information or other measures are recommended at this time.

JUNKERN

Case assessment created by JU on 25 Apr 2006

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 26.05.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-02032DE

Sender diagnosis: not reported

Narrative

This is a health-care professional confirmed report from the German registries (BfArM).

Initial information has been received from a physician concerning a 56-year-old female, German patient.

The patient has a history of myomatous uterus, it is reported that there are no concomitant diseases.

The patient was placed on Dulcolax, 2 suppositories daily since 01-April-06 as postoperative laxative. She also took Voltaren (diclofenac) suppository, 100 mg daily since 01-April-06 as postoperative analgetics and Celebrex (celecoxib), 200 mg daily as analgetic prevention since 31-March-06.

Information on further concomitant drugs was not provided.

On 01-April-06 the patient developed stomatitis and diarrhea, which lasted for 4 days. Therapy included mouth irrigation with Candio-Hermal (nystatin) and chlorhexidine and lozenges with benzocaine. Additionally there was a suspicion of HIT II (heparin induced thrombocytopenia type II) with an acute pulmonary arterial embolism and a drop of thrombocytes during therapy of Mono-embolex (certoparin-sodium).

The adverse events have been described as not to be life-threatening.

Laboratory analyses without finding. A HIT test was performed but there was no result available at the moment.

Information on the treatment of the events was not provided.

At the moment of this report the patient was not yet recovered.

Further information is not expected.

This is a spontaneous health-care professional confirmed report from Germany.

Follow up information (discharge letter from the department of anaesthesia) has been received via the German Health Authority concerning the 51-year-old female patient on 18-May-2006.

As concomitant medication hormone therapy and contraceptives were reported (no further details). Additionally an abuse of nicotine was reported that the patient stopped three weeks before the admission. The patient has a history of operation of hammer toes in 2000.

The patient was hospitalised for vaginal hysterectomy due to uterus myomatosis with hypermenorrhea on 30-March-2006 and operated on 31-March-2006. On the first postoperative day she experienced nausea, vomiting and chills and on the next day a temperature of 39.8 degree combined with asthenia, feeling of lalopathy and slight dyspnea. The patient was transferred to the intensive care unit. A partial respiratory insufficiency, thrombocytopenia and hypokalemia were detected. The patient complained about pain in the area of oral cavity, lips and tongue accompanied by lalopathy. Additionally profuse watery stools with hypokalemia occurred.

The patient obtained an infusion for balancing of electrolyte state and for prophylaxis low molecular-weight heparin and proton pump inhibitor. In addition the patient was treated with chlorhexidine, Candio-Hermal solution (nystatin) and benzocaine lozenges.

A microbiological stool analysis for clostridium difficile toxin A and B, adenovirus and rotavirus was performed without findings.

On 03-April-2006 the diarrhea stopped, but the symptoms of stomatitis continued.

Due to retrosternal pain and a significant partial respiratory insufficiency diagnostic measures were carried out leading to the diagnosis of acute pulmonary arterial embolism combined with postembolic pneumonitis and pleural effusion. Therapy included heparin which was stopped due to suspicion of HIT. Additionally a postoperative bacterial urinary tract infection was diagnosed and treated with antibiotics.

The stomatitis improved at the moment of relocation.

A suspicion of drug intolerance against Dulcolax supp. (bisacodyl), Laxoberal (sodium picosulfate), Voltaren supp. (diclofenac) and Celebrex (celecoxib) was reported.

The reporter rated the causal relationship between the laxatives (Dulcolax and Laxoberal) and the stomatitis and diarrhea as probable. There is no information in this report on administration details for the medication.

Further information is not expected, case is closed.

Sender's comment:

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 29.05.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-02032DE

Sender diagnosis: not reported

Narrative

This is a health-care professional confirmed report from the german registries (BfArM).

Initial information has been received from a physician concerning a 56-year-old female, german patient.

The patient has a history of myomatous uterus, it is reported that there are no concomitant diseases.

The patient was placed on Dulcolax, 2 suppositories daily since 01-April-06 as postoperative laxative. She also took Voltaren (diclofenac) suppository, 100 mg daily since 01-April-06 as postoperative analgetics and Celebrex (celecoxib), 200 mg daily as analgetic prevention since 31-March-06.

Information on further concomitant drugs was not provided.

On 01-April-06 the patient developed stomatitis and diarrhea, which lasted for 4 days. Therapy included mouth irrigation with candio-hermal (nystatin) and chlorhexidine and lozenges with benzocaine. Additionally there was a suspicion of HIT II (heparin induced thrombocytopenia type II) with an acute pulmonary arterial embolism and a drop of thrombocytes during therapy of Mono-embolex (certoparin-sodium).

The adverse events have been described as not to be life-threatening.

Laboratory analyses without finding. A HIT test was performed but there was no result available at the moment.

Information on the treatment of the events was not provided.

At the moment of this report the patient was not yet recovered.

Further information is not expected.

This is a spontaneous health-care professional confirmed report from Germany.

Follow up information (discharge letter from the department of anaesthesia) has been received via the German Health Authority concerning the 51-years-old female patient on 18-May-2006.

As concomitant medication hormone therapy and contraceptives were reported (no further details). Additionally an abuse of nicotine was reported that the patient stopped three weeks before the admission. The patient has a history of operation of hammer toes in 2000.

The patient was hospitalised for vaginal hysterectomy due to uterus myomatosus with hypermenorrhea on 30-March-2006 and operated on 31-March-2006. On the first postoperative day she experienced nausea, vomiting and chills and on the next day a temperature of 39,8 degree combined with asthenia, feeling of lalopathy and slight dyspnea. The patient was transferred to the intensive care unit. A partial respiratory insufficiency, thrombocytopenia and hypokalemia were detected. The patient complained about pain in the area of oral cavity, lips and tongue accompanied by lalopathy. Additionally profuse watery stools with hypokalemia occurred.

The patient obtained an infusion for balancing of electrolyte state and for prophylaxis low molecular-weight heparin and proton pump inhibitor. In addition the patient was treated with chlorhexidine, Candio-Hermal solution (nystatin) and benzocain lozenges.

A microbiological stool analysis for clostridium difficile toxin A and B, adenovirus and rotavirus was performed without findings.

On 03-April-2006 the diarrhea stopped, but the symptoms of stomatitis continued.

Due to retrosternal pain and a significant partial respiratory insufficiency diagnostic measures were carried out leading to the diagnosis of acute pulmonary arterial embolism combined with postembolic pneumonitis and pleural effusion. Therapy included heparin which was stopped due to suspicion of HIT. Additionally a postoperative bacterial urinary tract infection was diagnosed and treated with antibiotics.

The stomatitis improved at the moment of relocation.

A suspicion of drug intolerance against Dulcolax supp. (bisacodyl), Laxoberal (sodiumpicosulfat), Voltaren supp. (diclofenac) and Celebrex (celecoxib) was reported.

The reporter rated the causal relationship between the laxatives (Dulcolax and Laxoberal) and the stomatitis and diarrhea as probable. There is no information in this report on administration details for the medication.

Further information is not expected, case is closed.

Sender's comment:

Regional Centre Drug Safety
Boehringer Ingelheim
Comment on case 2006-DE-02032DE / DULCOLAX

Although the serious event diarrhea is listed for the drug in question, other factors have been reported in this case which may equally plausibly have

caused the event (co-medication). The extent to which the suspect drug and/or the confounding factors may have contributed to the occurrence of the event(s) is not clear.

Acute pulmonary arterial embolism and drop of thrombocytes it is more plausibly explained by the surgery, additionally in the context of other risk factors for embolism (smoker, hormone therapy, contraceptive co-medication). It remains unclear from the information available if a HIT existed which could also be a cause of this event. The postoperative urinary tract infection is considered not to be related to the product in question.

No change in the company core safety information or other measures are recommended at this time.

JUNKERN

Case assessment created by JU on 26 May 2006

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 12.06.2006 DE-Boehringer Ingelheim GmbH, Germany-2006-DE-02032DE

Sender diagnosis: not reported

Narrative

This is a health-care professional confirmed report from the German registries (BfArM).

Initial information has been received from a physician concerning a 56-year-old female, German patient.

The patient has a history of myomatous uterus, it is reported that there are no concomitant diseases.

The patient was placed on Dulcolax, 2 suppositories daily since 01-April-06 as postoperative laxative. She also took Voltaren (diclofenac) suppository, 100 mg daily since 01-April-06 as postoperative analgetics and Celebrex (celecoxib), 200 mg daily as analgetic prevention since 31-March-06.

Information on further concomitant drugs was not provided.

On 01-April-06 the patient developed stomatitis and diarrhea, which lasted for 4 days. Therapy included mouth irrigation with Candio-Hermal (nystatin) and chlorhexidine and lozenges with benzocaine. Additionally there was a suspicion of HIT II (heparin induced thrombocytopenia type II) with an acute pulmonary arterial embolism and a drop of thrombocytes during therapy of Mono-embolex (certoparin-sodium).

The adverse events have been described as not to be life-threatening.

Laboratory analyses without finding. A HIT test was performed but there was no result available at the moment.

Information on the treatment of the events was not provided.

At the moment of this report the patient was not yet recovered.

Further information is not expected.

This is a spontaneous health-care professional confirmed report from Germany.

Follow up information (discharge letter from the department of anaesthesia) has been received via the German Health Authority concerning the 51-years-old female patient on 18-May-2006.

As concomitant medication hormone therapy and contraceptives were reported (no further details). Additionally an abuse of nicotine was reported that the patient stopped three weeks before the admission. The patient has a history of operation of hammer toes in 2000.

The patient was hospitalised for vaginal hysterectomy due to uterus myomatous with hypermenorrhea on 30-March-2006 and operated on 31-March-2006. On the first postoperative day she experienced nausea, vomiting and chills and on the next day a temperature of 39,8 degree combined with asthenia, feeling of lalopathy and slight dyspnea. The patient was transferred to the intensive care unit. A partial respiratory insufficiency, thrombocytopenia and hypokalemia were detected. The patient complained about pain in the area of oral cavity, lips and tongue accompanied by lalopathy. Additionally profuse watery stools with hypokalemia occurred.

The patient obtained an infusion for balancing of electrolyte state and for prophylaxis low molecular-weight heparin and proton pump inhibitor. In addition the patient was treated with chlorhexidine, Candio-Hermal solution (nystatin) and benzocaine lozenges.

A microbiological stool analysis for clostridium difficile toxin A and B, adenovirus and rotavirus was performed without findings.

On 03-April-2006 the diarrhea stopped, but the symptoms of stomatitis continued.

Due to retrosternal pain and a significant partial respiratory insufficiency diagnostic measures were carried out leading to the diagnosis of acute pulmonary arterial embolism combined with postembolic pneumonitis and pleural effusion. Therapy included heparin which was stopped due to suspicion of HIT. Additionally a postoperative bacterial urinary tract infection was diagnosed and treated with antibiotics.

The stomatitis improved at the moment of relocation.

A suspicion of drug intolerance against Dulcolax supp. (bisacodyl), Laxoberal (sodiumpicosulfat), Voltaren supp. (diclofenac) and Celebrex (celecoxib) was reported.

The reporter rated the causal relationship between the laxatives (Dulcolax and Laxoberal) and the stomatitis and diarrhea as probable. There is no information in this report on administration details for the medication.

Further information is not expected, case is closed.

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim
Comment on case 2006-DE-02032DE / DULCOLAX

Although the serious event diarrhea is listed for the drug in question, other factors have been reported in this case which may equally plausibly have caused the event (co-medication). The extent to which the suspect drug and/or the confounding factors may have contributed to the occurrence of the event(s) is not clear.

Acute pulmonary arterial embolism and drop of thrombocytes it is more plausibly explained by the surgery, additionally in the context of other risk factors for embolism (smoker, hormone therapy, contraceptive co-medication). It remains unclear from the information available if a HIT existed which could also be a cause of this event. The postoperative urinary tract infection is considered not to be related to the product in question.

No change in the company core safety information or other measures are recommended at this time.

JUNKERN

Case assessment created by JU on 26 May 2006

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-06023982

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes							Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiorgan failure
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute renal failure					fatal
2 Toxic epidermal necrolysis					fatal
3 ARDS					fatal
4 Septic shock					fatal
5 Toxic shock syndrome					fatal
6 Multiorgan failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	Lumbago
diclofenac			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Spornraft-Ragaller P,Ragaller M,Meurer M. Toxische epidermale Nekrolyse nach NSAR-Einnahme. Z Rheumatol 2003;62:474-475.	
Qualification:	Physician	observestudytype:
Literature:	Spornraft-Ragaller P,Theilen H,Gottschlich G,Ragaller M. Behandlung der toxischen epidermalen Nekrolyse: Erfahrungen bei 9 Patienten unter Beruecksichtigung von intravenoesen Immunglobulinen. Hautarzt 2006;57:3:185-194.	

Narratives (nur bei elektronischem Eingang)

Boehringer Ingelheim GmbH, Germany, 22.05.2006 DE-Boehringer Ingelheim GmbH, Germany-2004-DE-03237GD

Sender diagnosis: not reported

Narrative

A 68-year-old female patient developed a severe bullous drug reaction in the form of toxic epidermal necrolysis (TEN) after taking oral diclofenac. According to information from the patient's relatives efflorescences had appeared first on the face and then spread to the upper chest. When she was admitted to the intensive care unit 9 days after the start of the first symptoms the somnolent patient already had generalised erythema over about 70 % of her body surface with confluent blisters and involvement of the oral mucosa. The case history revealed that at the time these symptoms appeared the patient had been taking diclofenac intermittently for lumbago.

There were large areas of confluent erythema multiforme(EM)-like foci with blisters forming in the center leaving extensive erosions. It finally covered 85 % (erythema) or 60 % (blisters) of her body surface. Histological examination of the skin revealed almost complete epidermal necrolysis. It was not possible to control the disease in spite of specific antibiotics and treatment with high-dosed glucocorticoids; after developing acute renal failure and ARDS the patient died of septic-toxic shock with multiorgan failure. Intravenous immunoglobulins (IVIG) were not administered because the condition was so far advanced - the pathological changes had been present for over 5 days when the patient was admitted to hospital. The authors state: "In the case of the patient described here, diclofenac is considered to be the most probable cause."

Sender's comment:Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 23.05.2006 DE-Boehringer Ingelheim GmbH, Germany-2004-DE-03237GD

Sender diagnosis: not reported

Narrative

A 68-year-old female patient developed a severe bullous drug reaction in the form of toxic epidermal necrolysis (TEN) after taking oral diclofenac. According to information from the patient's relatives efflorescences had appeared first on the face and then spread to the upper chest. When she was admitted to the intensive care unit 9 days after the start of the first symptoms the somnolent patient already had generalised erythema over about 70 % of her body surface with confluent blisters and involvement of the oral mucosa. The case history revealed that at the time these symptoms appeared the patient had been taking diclofenac intermittently for lumbago.

There were large areas of confluent erythema multiforme(EM)-like foci with blisters forming in the center leaving extensive erosions. It finally covered 85 % (erythema) or 60 % (blisters) of her body surface. Histological examination of the skin revealed almost complete epidermal necrolysis.

It was not possible to control the disease in spite of specific antibiotics and treatment with high-dosed glucocorticoids; after developing acute renal failure and ARDS the patient died of septic-toxic shock with multiorgan failure. Intravenous immunoglobulins (IVIG) were not administered because the condition was so far advanced - the pathological changes had been present for over 5 days when the patient was admitted to hospital.

The authors state: "In the case of the patient described here, diclofenac is considered to be the most probable cause."

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06038416

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 91 Year	weight (kg): 63	BMI_int: 23.14	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Mucous membrane disorder NOS	200607		4 Week		not recovered/not resolved
2 Exanthematic drug eruption	200607		4 Week	1 Month	not recovered/not resolved
3 Erythroderma	200607		4 Week		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 rewodina diclofenac sodium	Oral	100 Mg milligram(s)/Day tablet	200606- -	Spinal disorder

Drugs (concomitant)

2 acerbon 5 mg lisinopril	Oral	5 Mg milligram(s)/Day tablet	- -	Hypertension arterial
3 ass 100 acetylsalicylic acid		/ -	- -	Coronary artery disease
4 ass 100 acetylsalicylic acid	Oral	100 Mg milligram(s)/Day tablet	- -	Arrhythmia absoluta
5 fosamax 70 mg alendronate sodium	Oral	70 Mg milligram(s)/Week	- -	Osteoporosis
6 torem 10 torasemide	Oral	10 Mg milligram(s)/ Day tablet	- -	Hypertension arterial

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Dementia			Unknown	
Mitral insufficiency			Unknown	
Atrial fibrillation			Unknown	
Coronary artery disease			Unknown	
Candidiasis of mouth			Unknown	
Osteoporosis			Unknown	
Hypertension arterial			Unknown	
MRSA wound infection			Unknown	
Arrhythmia absoluta			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

PLIVA Research and Development Ltd., 05.03.2007 DE-PLIVA Research and Development Ltd.-107#01#2006-01710

Sender diagnosis: not reported

Narrative

This spontaneous report was received on 22 Aug 2006 from a physician.

A 91-year-old female patient experienced exanthematic drug eruption including erythrodermia during treatment with Rewodina 50 mg tablets for degeneration of cervical intervertebral disc.

Concomitant medication included Acerbon 5 mg tablets, Torem 10 mg tablets, Fosamax 70 mg tablets and ASS 100 mg tablets.

Concomitant disease included arterial hypertension, osteoporosis, arrhythmia absoluta, coronary artery disease, dementia, mitral insufficiency, oral candida and MRSA infection.

The reporter stated the outcome as not recovered. The causality is stated as probable.

Sender's comment:

Exanthematic drug eruption and Erythrodermia are described sufficiently in the current SmPC of Rewodina 50 mg.

Risk minimizing measures are not necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06060394

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	Yes	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 53 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Cross reactive drug hypersensitivity	200301				not recovered/not resolved
2 Bleeding time prolonged	20060116				Unknown
3 Dizziness	2002				not recovered/not resolved
4 Drug hypersensitivity					not recovered/not resolved
5 Dyspnoea exertional	2002				not recovered/not resolved
6 Exanthem	20020713				not recovered/not resolved
7 Fall	20051119				not recovered/not resolved
8 Food allergy					not recovered/not resolved
9 Haematoma	200511				not recovered/not resolved
10 Haematoma	20060523				Unknown
11 Hepatic neoplasm	2005				not recovered/not resolved
12 Hypersensitivity reaction	2002				not recovered/not resolved
13 Hypertension arterial					Unknown
14 Increased tendency to bruise	20051119				not recovered/not resolved
15 Wound healing delayed	200511				not recovered/not resolved
16 Urticaria	2002				not recovered/not resolved
17 Tachycardia paroxysmal					Unknown
18 Sleep disturbance	2002				not recovered/not resolved
19 Skin thinness	20060116				Unknown
20 Skin haemorrhage	20060116				Unknown
21 Ringing in ears	2002				not recovered/not resolved
22 Proteinuria	20051223				Unknown
23 Proteinuria	20051024				Unknown
24 Post procedural bleeding	2005				Unknown
25 Palpitations	2002				not recovered/not resolved
26 Oedema					Unknown
27 Laceration	20051119				not recovered/not resolved
28 Allergic skin reaction	20071105				recovering/resolving
29 Chest pain	20071105				Unknown
30 Thyroid nodule	20061108				Unknown
31 Blood pressure inadequately controlled	20071105	20071119	15 Day		recovered/resolved
32 Bechterew's disease acute episode	20061108				Unknown
33 Lipid metabolism disorder	20051024				Unknown
34 Cystitis haemorrhagic	20051223				Unknown
35 Stevens-Johnson syndrome	20020713				not recovered/not resolved
36 Angioedema	2002				not recovered/not resolved
37 Anaphylactic reaction	2002				not recovered/not resolved
38 Vasculitis	2002				not recovered/not resolved
39 Lyell's syndrome	20020713				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ATACAND candesartan cilexetil	Oral	12 Mg milligram(s)/Day 245	20071105-200711 -	Loss of control of blood sugar
2	diclofenac diclofenac	Unknown	/	200301-2003 -	Bechterew's disease
3	VIOXX rofecoxib	Oral	25 Mg milligram(s)/Day 245	200207-20020718 -	Bechterew's disease
4	VIOXX rofecoxib	Oral	25 Mg milligram(s)/Day 245	200204-200204 -	Bechterew's disease

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Dyspnoea exertional			No	
Overweight			Yes	
Rheumatic fever			No	
Sudden hearing loss			No	
Hyperlipidaemia			Yes	
Heart racing			No	
Tachycardia paroxysmal			No	
Dizziness			No	
Tooth extraction			No	
Thrombophilia			Yes	
Myomectomy			No	
Familial risk factor			Yes	
Comminuted fracture			No	
Abrasion NOS			No	
Endocarditis			No	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

009507513, 19.10.2006 DE-MERCK-0512DEU00105

Sender diagnosis: not reported

Narrative

Information has been received from a case in litigation. A lawyer, a physician (via German Agency), a medical expert, medical records and an allergologist reported about a female patient with positive familial thrombophilic diathesis (also reported by the medical expert as with evidence of arterial thrombophilia) and with a history in uterus myoma enucleatio, endocarditis after rheumatic attack (1978), abrasion (1991) tooth extraction with postprocedural bleeding of 10-15 min (2001) (also reported by the patient as in 2005), comminuted fracture of right leg (1994) who in April 2002 was placed on therapy with rofecoxib, 25 mg tablet, 25 mg, once a day for the treatment of Bechterew's disease.

Prior medication included acemetacin (RANTUDIL) forte, prednisolone 4 mg / 8 mg daily and diclofenac 100 mg supp. as necessary, which were all well tolerated, even in high dosages. Ecg from 03-MAY-2002 was inconspicuous. Laboratory tests from 12-JUN-2006 showed normal copper, transferrin, iron, transferrin saturation and ferritin. The reporting physician explicit mentioned these examinations before therapy with rofecoxib, conflicting to reported therapy start in April 2002. There were no indications of existing allergies / intolerances, skin changes or skin hemorrhage, coagulopathy etc. before therapy with rofecoxib.

According to the allergologist rofecoxib was well tolerated during the first cycle of treatment in April 2002. In a further cycle starting in July 2002 (also reported by the medical expert as a continuous 3 month therapy, starting in April 2002) the patient developed massive skin rash (also reported by the medical expert as massive skin reactions compatible with Stevens-Johnson-syndrome and Lyell's syndrome) on 13-JUL-2002. In addition the medical expert reported about hypersensitivity reactions with angioedema and urticaria, anaphylactic/anaphylactoid reactions, vasculitis and palpitations (onset date not reported). According to the lawyer, she additionally suffered from ringing in ears, dizziness, sleep disturbances and exertional dyspnea. On 18-JULY-2002 the patient was evaluated for the first time by the allergologist. That same day therapy with rofecoxib was discontinued. The patient was treated with prednisolone. Repeat evaluation, done on 22-JUL-2003 showed no further exanthema. In the following weeks the patient's symptoms slowly improved.

In January 2003, the patient was placed on therapy with diclofenac suppository (dose and duration unknown) for the treatment of an acute episode of

Bechterew's disease.

Subsequently, she developed generalized itching exanthem of the whole body (also reported by the lawyer as generalized itching exanthem with cracking of skin and painful skin inflammations, interpreted as cross allergy with diclofenac) and was therefore evaluated again by the allergologist. Diclofenac was immediately discontinued. The patient was treated with cortisone. In the further course ("for any length of time" reported by the allergologist) the patient's symptoms persisted. The allergologist stated that the patient has developed generalized psoriasis-like drug-induced exanthem caused by analgetic drug intolerance (conflicting with the information reported by the medical expert: reported as Stevens-Johnson-syndrome and Lyell's syndrome).

The medical expert stated that, in the further course, the persisting Stevens-Johnson syndrome led to extreme tissue and vascular vulnerability so that any even minor trauma caused massive hematoma and bleedings which were almost impossible to suppress. In addition, the patient developed heparin allergy and multiple food allergies (onset unknown).

Diagnostic evaluation ruled out pathological hemopoiesis, coagulation disorder and immunological deficits of cellular immune function. A hereditary disorder of the plasmatic coagulation factors as well as fibrinolysis and its inhibitors was also excluded. A homocysteine C677 mutation (MTHFR polymorphism) and a heterocysteine positive mutation GP 1a C807 were considered signs of an arterial thrombophilia. Based on this result, the massive occurrence of hematoma, bleedings and wound healing disturbances was attributed to an allergic/toxic process.

In 2005, the patient was diagnosed with tumorous changes in the liver. There were conflicting information received during nephrologic evaluation, done on 07-DEC-2005: There was no evidence of hepatic neoplastic structure.

The patient's status was still massively compromised by generalized itching exanthema, slowly healing wounds, scars and hematoma. Due to her extreme bleeding tendency, her quality of life was considerably reduced. Normal daily activities involved the risk of life-threatening concealed or external hemorrhage.

On 24-OCT-2005 the patient was diagnosed with lipid metabolism disorder and glomerular proteinuria. In addition tooth extraction in 2005 (also reported by the patient two tooth extractions in 2005) with post procedural bleeding of 10-15 min was documented.

On 19-NOV-2005 the patient fell in front of her house and developed a laceration under left knee joint. Within the next 2 days the patient developed expanded haematoma of left lower leg and a 10 x 1.5 cm big laceration. An adequate surgical wound could not be performed due to increased tissue vulnerability. It was reported that the patient still had decreased mobility due to delayed wound healing.

Evaluation summary written on 08-December-2005 contained following diagnosis: paroxysmal tachycardia and arterial hypertension (onset date not reported).

On 23-DEC-2005 the patient was diagnosed with hemorrhagic cystitis and mixed type proteinuria. In addition it was documented that the patient complained about edema.

In the further course the patient was diagnosed with haemorrhage of forearms including very thin skin and prolonged in vivo bleeding time up to 15 min (16-JAN-2006). The haematologist stated that these findings (prolonged in vivo bleeding time, by normal amount of thrombocytes and normal function) suggest soonest a possible vascular related disturbance of primary function of haemostasis.

On 02-MAY-2006 the patient was diagnosed with severe affinity of hematoma formation. Outcome regarding edema, tooth extraction with post procedural bleeding,

lipid metabolism disorder, glomerular proteinuria, paroxysmal tachycardia and arterial hypertension, hemorrhagic cystitis and mixed type proteinuria, haemorrhage of forearms including very thin skin, prolonged in vivo bleeding time up to 15 min and severe affinity of hematoma formation was not reported.

The reporting medical expert felt that the hypersensitivity reactions with angioedema and urticaria, the anaphylactic/anaphylactoid reactions, vasculitis, palpitations, Stevens-Johnson-syndrome/Lyell's syndrome, heparin allergy and cross allergy with diclofenac were related to therapy with rofecoxib. He did not rate drug relationship regarding the other symptoms.

The lawyer felt that also ringing in ears, dizziness, sleep disturbances and exertional dyspnea were related to rofecoxib.

The allergologist felt that psoriasis-like drug-induced exanthem was related to therapy with rofecoxib.

The physician felt that fall, laceration, haematoma, tissue vulnerability and delayed wound healing were related to therapy with rofecoxib.

Vasculitis and Stevens-Johnson-syndrome/Lyell's syndrome were considered to be immediately life-threatening and disabling.

Additional information has been requested.

Information was received on 20-DEC-2005 which pertained to an event that occurred 3 years following the discontinuation of rofecoxib in July 2002. This information contained the following adverse experience: tumorous changes in the liver (2005).

Follow up information received on 19-JUN-2006 which pertained to events that occurred following the discontinuation of rofecoxib on 18-Jul-2002. This information contained the following adverse experiences:

edema (onset date not reported)

lipid metabolism disorder and glomerular proteinuria (24-OCT-2005)

mixed type proteinuria (23-DEC-2005)

very thin skin (16-JAN-2006)

Sender's comment:

Diese Meldung erhielten wir erst nach Bekanntwerden der Marktrücknahme von VIOXX°. Eine Anwältin berichtet, dass sich bei einer Mandantin unter der Therapie mit VIOXX° eine sehr schwer verlaufende chronische Form eines Steven-Johnson/Lyell-Syndroms, sowie eine Vaskulitis entwickelten. Die übrigen berichteten Ereignisse sind nicht schwerwiegend. Die genannten schwerwiegenden Ereignisse waren in der Fach- und Gebrauchsinformation von VIOXX° beschrieben. Die uns bisher vorliegenden Informationen erlauben keine Kausalitätsbewertung. Wir werden den Fall weiter abklären und unaufgefordert wieder berichten. MSD hat am 30. September 2004 VIOXX° freiwillig weltweit vom Markt zurückgenommen.

FOLLOW-UP(1): Die neuen Informationen enthalten keine weiteren Daten, die zur Klärung der Ursache der berichteten schwerwiegenden Ereignisse führen bzw. zu einer Änderung unseres ursprünglichen Kommentars bezüglich der Kausalitätsbewertung Anlass geben.

FOLLOW-UP(2): Der Allergologe, der den Hautauschlag der Patientin im Juni 2002 behandelt hatte, hatte ein generalisiertes makulopapuläres Exanthem diagnostiziert. Die Patientin wurde ambulant mit Prednisolon 40 mg am Tag und Bethamethason 0,1 % Creme behandelt. Die Diagnose und Therapie schließen ein kritisches Krankheitsbild wie Steven-Johnson- oder Lyell-Syndrom aus. Angaben oder Dokumentation einer Ätiologieabklärung (Allergietestung, Hautbiopsien, immunologische Differentialdiagnostik etc.) fehlen. Ohne diese Angaben bleibt ein Kausalzusammenhang mit VIOXX° auch weiterhin unklar.

FOLLOW-UP(3): Die neu eingetroffenen Informationen beziehen sich auf die medizinische Vorgeschichte der Patientin und bringen keine neuen Aspekte für die Beurteilung der Ereignisse, die unter der Gabe von VIOXX° auftraten. Eigenverantwortliche Maßnahmen sind nicht erforderlich.

Reporter's comment:

009507513, 23.08.2007 DE-MERCK-0512DEU00105

Sender diagnosis: not reported

Narrative

Information has been received from a case in litigation. A lawyer, a physician (via German Agency), a medical expert, medical records and an allergologist reported about a female patient with positive familial thrombophilic diathesis (also reported by the medical expert as with evidence of arterial thrombophilia) and with a history in uterus myoma enucleatio, endocarditis after rheumatic attack (1978), abrasion (1991) tooth extraction with postprocedural bleeding of 10-15 min (2001) (also reported by the patient as in 2005), comminuted fracture of right leg (1994), paroxysmal tachycardia (1984), dizziness (1984) and heart racing (1984) who, in April 2002, was placed on therapy with rofecoxib, 25 mg tablet, 25 mg, once a day for the treatment of Bechterew's disease.

Prior medication included acemetacin (RANTUDIL) forte, prednisolone 4 mg/8 mg daily and diclofenac 100 mg supp. as necessary, which were all well tolerated, even in high dosages. Ecg from 03-MAY-2002 was inconspicuous. On 12-JUN-2006, laboratory analysis revealed normal copper, transferrin, iron, transferrin saturation and ferritin. The reporting physician explicitly mentioned these examinations before therapy with rofecoxib, conflicting with the reported therapy start in April 2002. There was no evidence of existing allergies/intolerances, skin changes or skin hemorrhage, coagulopathy etc. before therapy with rofecoxib.

According to the allergologist, rofecoxib was well tolerated during the first cycle of treatment in April 2002. In a further cycle starting in July 2002 (also reported by the medical expert as a continuous 3-month therapy, starting in April 2002) the patient developed massive skin rash (also reported by the medical expert as massive skin reactions compatible with Stevens-Johnson syndrome and Lyell's syndrome) on 13-JUL-2002. The medical expert also reported about hypersensitivity reactions with angioedema and urticaria, anaphylactic/anaphylactoid reactions, vasculitis and palpitations (onset date not reported). According to the lawyer, the patient additionally suffered from ringing in ears, dizziness, sleep disturbances and exertional dyspnea. On 18-JULY-2002, the patient was evaluated for the first time by the allergologist. That same day, therapy with rofecoxib was discontinued. The patient was treated with prednisolone. On 22-JUL-2003, repeat evaluation showed no further exanthema. In the following weeks the patient's symptoms slowly improved.

In January 2003, the patient was placed on therapy with diclofenac suppository (dose and duration unknown) for the treatment of an acute episode of Bechterew's disease. Subsequently, she developed generalized itching exanthema of the whole body (also reported by the lawyer as generalized itching exanthema with cracking of skin and painful skin inflammations, interpreted as cross allergy with diclofenac) and was again evaluated by the allergologist. Diclofenac was immediately discontinued. The patient was treated with cortisone. In the further course ("for any length of time" reported by the allergologist), the patient's symptoms persisted. The allergologist stated that the patient had developed generalized psoriasis-like drug-induced exanthema caused by analgetic drug intolerance (conflicting with the information reported by the medical expert: reported as Stevens-Johnson syndrome and Lyell's syndrome).

The medical expert stated that, in the further course, the persisting Stevens-Johnson syndrome led to extreme tissue and vascular vulnerability so that any even minor trauma caused massive hematoma and bleedings which were almost impossible to suppress. The patient also developed heparin allergy and multiple food allergies (onset unknown). Diagnostic evaluation ruled out pathological hemopoiesis, coagulation disorder and immunological

deficits of cellular immune function. A hereditary disorder of the plasmatic coagulation factors as well as fibrinolysis and its inhibitors was also excluded. A homocysteine C677 mutation (MTHFR polymorphism) and a heterozygote positive mutation GP Ia C807 were considered signs of an arterial thrombophilia. Based on this result, the massive occurrence of hematoma, bleedings and wound healing disturbances was attributed to an allergic/toxic process.

In 2005, the patient was diagnosed with a tumorous changes in the liver. Conflicting information was received during nephrologic evaluation, done on 07-DEC-2005 : There was no evidence of hepatic neoplastic structure.

The patient's status was still massively compromised by generalized itching exanthema, slowly healing wounds, scars and hematoma. Due to her extreme bleeding tendency, her quality of life was considerably reduced. Normal daily activities involved the risk of life-threatening concealed or external hemorrhage. On 24-OCT-2005, the patient was diagnosed with lipid metabolism disorder and glomerular proteinuria. A tooth extraction in 2005 (also reported by the patient as "two tooth extractions in 2005") with post procedural bleeding of 10-15 min was documented.

On 19-NOV-2005, the patient fell in front of her house and sustained a laceration under her left knee joint. Within the next 2 days the patient developed expanded hematoma of her left lower leg and a laceration of 10x1.5 cm. An adequate surgical wound treatment could not be performed due to increased tissue vulnerability. It was reported that the patient still had decreased mobility due to delayed wound healing.

Evaluation summary written on 08-DEC-2005 contained the following diagnoses: paroxysmal tachycardia and arterial hypertension (onset date not reported).

On 23-DEC-2005, the patient was diagnosed with hemorrhagic cystitis and mixed type proteinuria. It was also documented that the patient complained about edema. In the further course, the patient was diagnosed with hemorrhage of forearms including very thin skin and prolonged in vivo bleeding time up to 15 min (16-JAN-2006). The hematologist stated that these findings (prolonged in vivo bleeding time in the context of normal amount of thrombocytes and normal function) suggest most likely a possible vascular related disturbance of primary function of hemostasis.

On 02-MAY-2006, the patient was diagnosed with severe affinity of hematoma formation. Outcome regarding edema, tooth extraction with post procedural bleeding, lipid metabolism disorder, glomerular proteinuria, paroxysmal tachycardia and arterial hypertension, hemorrhagic cystitis and mixed type proteinuria, hemorrhage of forearms including very thin skin, prolonged in vivo bleeding time up to 15 min and severe affinity of hematoma formation was not reported.

The reporting medical expert felt that the hypersensitivity reactions with angioedema and urticaria, the anaphylactic/anaphylactoid reactions, vasculitis, palpitations, Stevens-Johnson syndrome/Lyell's syndrome, heparin allergy and cross allergy with diclofenac were related to therapy with rofecoxib; he did not rate the drug relationship regarding the other symptoms. The allergologist felt that psoriasis-like drug-induced exanthema was related to therapy with rofecoxib. The physician felt that fall, laceration, hematoma, tissue vulnerability and delayed wound healing were related to therapy with rofecoxib. The lawyer felt that ringing in ears, dizziness, sleep disturbances and exertional dyspnea were as well related to rofecoxib.

Vasculitis and Stevens-Johnson syndrome/Lyell's syndrome were considered to be immediately life-threatening and disabling.

Information was received on 20-DEC-2005 which pertained to an event that occurred 3 years following the discontinuation of rofecoxib in July 2002. This information contained the following adverse experience: tumorous changes in the liver (2005).

Follow up information received on 19-JUN-2006 which pertained to events that occurred following the discontinuation of rofecoxib on 18-Jul-2002. This information contained the following adverse experiences: edema (onset date not reported), lipid metabolism disorder and glomerular proteinuria (24-OCT-2005), mixed type proteinuria (23-DEC-2005), very thin skin (16-JAN-2006).

Additional information has been requested.

Sender's comment:

Diese Meldung erhielten wir erst nach Bekanntwerden der Marktrücknahme von VIOXX°. Eine Anwältin berichtet, dass sich bei einer Mandantin unter der Therapie mit VIOXX° eine sehr schwer verlaufende chronische Form eines Steven-Johnson/Lyell-Syndroms, sowie eine Vaskulitis entwickelten. Die übrigen berichteten Ereignisse sind nicht schwerwiegend. Die genannten schwerwiegenden Ereignisse waren in der Fach- und Gebrauchsinformation von VIOXX° beschrieben. Die uns bisher vorliegenden Informationen erlauben keine Kausalitätsbewertung. Wir werden den Fall weiter abklären und unaufgefordert wieder berichten. MSD hat am 30. September 2004 VIOXX° freiwillig weltweit vom Markt zurückgenommen.

FOLLOW-UP(1): Die neuen Informationen enthalten keine weiteren Daten, die zur Klärung der Ursache der berichteten schwerwiegenden Ereignisse führen bzw. zu einer Änderung unseres ursprünglichen Kommentars bezüglich der Kausalitätsbewertung Anlass geben.

FOLLOW-UP(2): Der Allergologe, der den Hautauschlag der Patientin im Juni 2002 behandelt hatte, hatte ein generalisiertes makulopapuläres Exanthem diagnostiziert. Die Patientin wurde ambulant mit Prednisolon 40 mg am Tag und Bethamethason 0,1 % Creme behandelt. Die Diagnose und Therapie schließen ein kritisches Krankheitsbild wie Steven-Johnson- oder Lyell-Syndrom aus. Angaben oder Dokumentation einer Ätiologieabklärung (Allergietestung, Hautbiopsien, immunologische Differentialdiagnostik etc.) fehlen. Ohne diese Angaben bleibt ein Kausalzusammenhang mit VIOXX° auch weiterhin unklar.

FOLLOW-UP(3): Die neu eingetroffenen Informationen beziehen sich auf die medizinische Vorgeschichte der Patientin und bringen keine neuen Aspekte für die Beurteilung der Ereignisse, die unter der Gabe von VIOXX® auftraten. Eigenverantwortliche Maßnahmen sind nicht erforderlich.

Reporter's comment:

009507513, 17.07.2008 DE-MERCK-0512DEU00105

Sender diagnosis: not reported

Narrative

Information has been received from a case in litigation. A lawyer, a physician (via German Agency), a medical expert, several medical records and an allergologist reported about a female patient with positive familial thrombophilic diathesis (also reported by the medical expert as with evidence of arterial thrombophilia) and with a history in uterus myoma enucleatio, endocarditis after rheumatic attack (1978), abrasion (1991) tooth extraction with postprocedural bleeding of 10-15 min (2001) (also reported by the patient as in 2005), comminuted fracture of right leg (1994), paroxysmal tachycardia (1984), dizziness (1984) and heart racing (1984) who, in April 2002, was placed on therapy with rofecoxib, 25 mg tablet, 25 mg, once a day for the treatment of Bechterew's disease.

Prior medication included acemetacin (RANTUDIL) forte, prednisolone 4 mg/8 mg daily and diclofenac 100 mg supp. as necessary, which were all well tolerated, even in high dosages. Ecg from 03-MAY-2002 was inconspicuous. On 12-JUN-2006, laboratory analysis revealed normal copper, transferrin, iron, transferrin saturation and ferritin. The reporting physician explicitly mentioned these examinations before therapy with rofecoxib, conflicting with the reported therapy start in April 2002. There was no evidence of existing allergies/intolerances, skin changes or skin hemorrhage, coagulopathy etc. before therapy with rofecoxib.

According to the allergologist, rofecoxib was well tolerated during the first cycle of treatment in April 2002. In a further cycle starting in July 2002 (also reported by the medical expert as a continuous 3-month therapy, starting in April 2002) the patient developed massive skin rash (also reported by the medical expert as massive skin reactions compatible with Stevens-Johnson syndrome and Lyell's syndrome) on 13-JUL-2002. The medical expert also reported about hypersensitivity reactions with angioedema and urticaria, anaphylactic/anaphylactoid reactions, vasculitis and palpitations (onset date not reported). According to the lawyer, the patient additionally suffered from ringing in ears, dizziness, sleep disturbances and exertional dyspnea. On 18-JULY-2002, the patient was evaluated for the first time by the allergologist. That same day, therapy with rofecoxib was discontinued. The patient was treated with prednisolone. On 22-JUL-2003, repeat evaluation showed no further exanthema. In the following weeks the patient's symptoms slowly improved.

In January 2003, the patient was placed on therapy with diclofenac suppository (dose and duration unknown) for the treatment of an acute episode of Bechterew's disease. Subsequently, she developed generalized itching exanthema of the whole body (also reported by the lawyer as generalized itching exanthema with cracking of skin and painful skin inflammations, interpreted as cross allergy with diclofenac) and was again evaluated by the allergologist. Diclofenac was immediately discontinued. The patient was treated with cortisone. In the further course ("for any length of time" reported by the allergologist), the patient's symptoms persisted. The allergologist stated that the patient had developed generalized psoriasis-like drug-induced exanthema caused by analgetic drug intolerance (conflicting with the information reported by the medical expert: reported as Stevens-Johnson syndrome and Lyell's syndrome).

The medical expert stated that, in the further course, the persisting Stevens-Johnson syndrome led to extreme tissue and vascular vulnerability so that any even minor trauma caused massive hematoma and bleedings which were almost impossible to suppress. The patient also developed heparin allergy and multiple food allergies (onset unknown). Diagnostic evaluation ruled out pathological hemopoiesis, coagulation disorder and immunological deficits of cellular immune function. A hereditary disorder of the plasmatic coagulation factors as well as fibrinolysis and its inhibitors was also excluded. A homocysteine C677 mutation (MTHFR polymorphism) and a heterocysteine positive mutation GP Ia C807 were considered signs of an arterial thrombophilia. Based on this result, the massive occurrence of hematoma, bleedings and wound healing disturbances was attributed to an allergic/toxic process.

In 2005, the patient was diagnosed with a tumorous changes in the liver. Conflicting information was received during nephrologic evaluation, done on 07-DEC-2005 : There was no evidence of hepatic neoplastic structure.

The patient's status was still massively compromised by generalized itching exanthema, slowly healing wounds, scars and hematoma. Due to her extreme bleeding tendency, her quality of life was considerably reduced. Normal daily activities involved the risk of life-threatening concealed or external hemorrhage. On 24-OCT-2005, the patient was diagnosed with lipid metabolism disorder and glomerular proteinuria. A tooth extraction in 2005 (also reported by the patient as "two tooth extractions in 2005") with post procedural bleeding of 10-15 min was documented.

On 19-NOV-2005, the patient fell in front of her house and sustained a laceration under her left knee joint. Within the next 2 days the patient developed expanded hematoma of her left lower leg and a laceration of 10x1.5 cm. An adequate surgical wound treatment could not be performed due to increased tissue vulnerability. It was reported that the patient still had decreased mobility due to delayed wound healing.

Evaluation summary written on 08-DEC-2005 contained the following diagnoses: paroxysmal tachycardia and arterial hypertension (onset date not reported).

On 23-DEC-2005, the patient was diagnosed with hemorrhagic cystitis and mixed type proteinuria. It was also documented that the patient complained about edema. In the further course, the patient was diagnosed with hemorrhage of forearms including very thin skin and prolonged in vivo bleeding time up to 15 min (16-JAN-2006). The hematologist stated that these findings (prolonged in vivo bleeding time in the context of normal amount of thrombocytes and normal function) suggest most likely a possible vascular related disturbance of primary function of hemostasis.

On 02-MAY-2006, the patient was diagnosed with severe affinity of hematoma formation. Outcome regarding edema, tooth extraction with post procedural bleeding, lipid metabolism disorder, glomerular proteinuria, paroxysmal tachycardia and arterial hypertension, hemorrhagic cystitis and mixed type proteinuria, hemorrhage of forearms including very thin skin, prolonged in vivo bleeding time up to 15 min and severe affinity of hematoma formation was not reported.

On 08-NOV-2006, the patient developed an acute episode of Bechterew's disease and therefore was hospitalized. Upon admission, she was diagnosed with thyroid nodule. Outcome was not reported.

On 05-NOV-2007, the patient developed loss of blood pressure control and therefore was hospitalized. Upon admission, she was diagnosed with thoracic pain. She received treatment with candesartan cilexetil (ATACAND) 8 mg tablet, 4 mg three times a day (therapy initiated on 05-NOV-2007). Because she developed an allergic skin reaction to angiotensin II blocker, therapy with candesartan cilexetil (ATACAND) was discontinued (date unknown) and she received treatment with cortisone. On 19-NOV-2007, she was recovering from allergic skin reaction to angiotensin II blocker and recovered from loss of blood pressure control. The outcome of the thoracic pain was not reported.

The reporting medical expert felt that the hypersensitivity reactions with angioedema and urticaria, the anaphylactic/anaphylactoid reactions, vasculitis, palpitations, Stevens-Johnson syndrome/Lyell's syndrome, heparin allergy and cross allergy with diclofenac were related to therapy with rofecoxib; he did not rate the drug relationship regarding the other symptoms. The allergologist felt that psoriasis-like drug-induced exanthema was related to therapy with rofecoxib. The physician felt that fall, laceration, hematoma, tissue vulnerability and delayed wound healing were related to therapy with rofecoxib. The lawyer felt that ringing in ears, dizziness, sleep disturbances and exertional dyspnea were as well related to rofecoxib. The physicians did not rate the drug relationship concerning the acute episode of Bechterew's disease, thyroid nodule, loss of blood pressure control, thoracic pain and allergic skin reaction to angiotensin II blocker.

Vasculitis and Stevens-Johnson syndrome/Lyell's syndrome were considered to be immediately life-threatening and disabling.

Information was received on 20-DEC-2005 which pertained to an event that occurred 3 years following the discontinuation of rofecoxib in July 2002. This information contained the following adverse experience: tumorous changes in the liver (2005).

Follow up information received on 19-JUN-2006 which pertained to events that occurred following the discontinuation of rofecoxib on 18-Jul-2002. This information contained the following adverse experiences:
edema (onset date not reported), lipid metabolism disorder and glomerular proteinuria (24-OCT-2005), mixed type proteinuria (23-DEC-2005), very thin skin (16-JAN-2006).

All available medical records will be provided upon request.

Additional information has been requested.

Sender's comment:

Diese Meldung erhielten wir erst nach Bekanntwerden der Marktrücknahme von VIOXX°. Eine Anwältin berichtet, dass sich bei einer Mandantin unter der Therapie mit VIOXX° eine sehr schwer verlaufende chronische Form eines Steven-Johnson/Lyell-Syndroms, sowie eine Vaskulitis entwickelten. Die übrigen berichteten Ereignisse sind nicht schwerwiegend. Die genannten schwerwiegenden Ereignisse waren in der Fach- und Gebrauchsinformation von VIOXX° beschrieben. Die uns bisher vorliegenden Informationen erlauben keine Kausalitätsbewertung. Wir werden den Fall weiter abklären und unaufgefordert wieder berichten. MSD hat am 30. September 2004 VIOXX° freiwillig weltweit vom Markt zurückgenommen.

FOLLOW-UP(1): Die neuen Informationen enthalten keine weiteren Daten, die zur Klärung der Ursache der berichteten schwerwiegenden Ereignisse führen bzw. zu einer Änderung unseres ursprünglichen Kommentars bezüglich der Kausalitätsbewertung Anlass geben.

FOLLOW-UP(2): Der Allergologe, der den Hautauschlag der Patientin im Juni 2002 behandelt hatte, hatte ein generalisiertes makulopapuläres Exanthem diagnostiziert. Die Patientin wurde ambulant mit Prednisolon 40 mg am Tag und Bethamethason 0,1 % Creme behandelt. Die Diagnose und Therapie schließen ein kritisches Krankheitsbild wie Steven-Johnson- oder Lyell-Syndrom aus. Angaben oder Dokumentation einer Ätiologieabklärung (Allergietestung, Hautbiopsien, immunologische Differentialdiagnostik etc.) fehlen. Ohne diese Angaben bleibt ein Kausalzusammenhang mit VIOXX° auch weiterhin unklar.

FOLLOW-UP(3): Die neu eingetroffenen Informationen beziehen sich auf die medizinische Vorgeschichte der Patientin und bringen keine neuen Aspekte für die Beurteilung der Ereignisse, die unter der Gabe von VIOXX° auftraten. Eigenverantwortliche Maßnahmen sind nicht erforderlich.

FOLLOW-UP (4):

Die neuen Informationen enthalten keine weiteren Daten, die zur Klärung der Ursache der bisher berichteten schwerwiegenden Ereignisse führen bzw. zu einer

Reporter's comment:

009507513, 16.07.2009 DE-MERCK-0512DEU00105

Sender diagnosis: not reported

Narrative

Information has been received from a case in litigation. A lawyer, a physician (via German Agency), a medical expert, several medical records and an allergologist reported about a female patient with positive familial thrombophilic diathesis (also reported by the medical expert as with evidence of arterial thrombophilia), overweight, hyperlipidaemia and familial risk factor and with a history in uterus myoma enucleatio, endocarditis after rheumatic attack (1978), abrasion (1991) tooth extraction with postprocedural bleeding of 10-15 min (2001) (also reported by the patient as in 2005), comminuted fracture of right leg (1994), paroxysmal tachycardia (1984 and 1996), dizziness (1984), heart racing (1984), progredient dyspnoea on exertion (in 1996), sudden hearing loss in 1994, who, in April 2002, was placed on therapy with rofecoxib, 25 mg tablet, 25 mg, once a day for the treatment of Bechterew's disease.

Information has been received from a physician concerning a female with thrombophilia, bechterew's disease, hyperlipidaemia, overweight and familial risk factor and a history of myomectomy, endocarditis, abrasion nos, comminuted fracture, tooth extraction, tachycardia paroxysmal, dizziness, heart racing, tachycardia paroxysmal, dyspnoea exertional, rheumatic fever, endocarditis and sudden hearing loss who in April 2002, was placed on therapy with rofecoxib, 25 mg tablet, 25 mg, once a day for the treatment of bechterew's disease (duration not reported). Concomitant therapy included diclofenac and candesartan cilexetil (ATACAND). In 2002 the patient experienced hypersensitivity reaction, angioedema, urticaria, anaphylactic/anaphylactoid reactions, vasculitis, palpitations, ringing in ears, dizziness, sleep disturbances and exertional dyspnea.

Prior medication included acemetacin (RANTUDIL) forte, prednisolone 4 mg/8 mg daily and diclofenac 100 mg supp. as necessary, which were all well tolerated, even in high dosages. Ecg from 03-MAY-2002 was inconspicuous. On 12-JUN-2006, laboratory analysis revealed normal copper, transferrin, iron, transferrin saturation and ferritin. The reporting physician explicitly mentioned these examinations before therapy with rofecoxib, conflicting with the reported therapy start in April 2002. There was no evidence of existing allergies/intolerances, skin changes or skin hemorrhage, coagulopathy etc. before therapy with rofecoxib.

According to the allergologist, rofecoxib was well tolerated during the first cycle of treatment in April 2002. In a further cycle starting in July 2002 (also reported by the medical expert as a continous 3-month therapy, starting in April 2002) the patient developed massive skin rash (also reported by the medical expert as massive skin reactions compatible with Stevens-Johnson syndrome and Lyell's syndrome) on 13-JUL-2002. The medical expert also reported about hypersensitivity reactions with angioedema and urticaria, anaphylactic/anaphylactoid reactions, vasculitis and palpitations (onset date not reported). According to the lawyer, the patient additionally suffered from ringing in ears, dizziness, sleep disturbances and exertional dyspnea. On 18-JULY-2002, the patient was evaluated for the first time by the allergologist. That same day, therapy with rofecoxib was discontinued. The patient was treated with prednisolone. On 22-JUL-2003, repeat evaluation showed no further exanthema. In the following weeks the patient's symptoms slowly improved.

In January 2003, the patient was placed on therapy with diclofenac suppository (dose and duration unknown) for the treatment of an acute episode of Bechterew's disease. Subsequently, she developed generalized itching exanthema of the whole body (also reported by the lawyer as generalized itching exanthema with cracking of skin and painful skin inflammations, interpreted as cross allergy with diclofenac) and was again evaluated by the allergologist. Diclofenac was immediately discontinued. The patient was treated with cortisone. In the further course ("for any length of time" reported by the allergologist), the patient's symptoms persisted. The allergologist stated that the patient had developed generalized psoriasis-like drug-induced exanthema caused by analgetic drug intolerance (conflicting with the information reported by the medical expert: reported as Stevens-Johnson syndrome and Lyell's syndrome).

The medical expert stated that, in the further course, the persisting Stevens-Johnson syndrome led to extreme tissue and vascular vulnerability so that any even minor trauma caused massive hematoma and bleedings which were almost impossible to suppress. The patient also developed heparin allergy and multiple food allergies (onset unknown). Diagnostic evaluation ruled out pathological hemopoiesis, coagulation disorder and immunological deficits of cellular immune function. A hereditary disorder of the plasmatic coagulation factors as well as fibrinolysis and its inhibitors was also excluded. A homocysteine C677 mutation (MTHFR polymorphism) and a heterocysteine positive mutation GP Ia C807 were considered signs of an arterial thrombophilia. Based on this result, the massive occurrence of hematoma, bleedings and wound healing disturbances was attributed to an allergic/toxic process.

In 2005, the patient was diagnosed with a tumorous changes in the liver. Conflicting information was received during nephrologic evaluation, done on 07-DEC-2005 : There was no evidence of hepatic neoplastic structure.

The patient's status was still massively compromised by generalized itching exanthema, slowly healing wounds, scars and hematoma. Due to her extreme bleeding tendency, her quality of life was considerably reduced. Normal daily activities involved the risk of life-threatening concealed or external hemorrhage. On 24-OCT-2005, the patient was diagnosed with lipid metabolism disorder and glomerular proteinuria. A tooth extraction in 2005 (also reported by the patient as "two tooth extractions in 2005") with post procedural bleeding of 10-15 min was documented.

On 19-NOV-2005, the patient fell in front of her house and sustained a laceration under her left knee joint. Within the next 2 days the patient developed expanded hematoma of her left lower leg and a laceration of 10x1.5 cm. An adequate surgical wound treatment could not be performed due to increased tissue vulnerability. It was reported that the patient still had decreased mobility due to delayed wound healing.

Evaluation summary written on 08-DEC-2005 contained the following diagnoses: paroxysmal tachycardia and arterial hypertension (onset date not reported).

On 23-DEC-2005, the patient was diagnosed with hemorrhagic cystitis and mixed type proteinuria. It was also documented that the patient complained about edema. In the further course, the patient was diagnosed with hemorrhage of forearms including very thin skin and prolonged in vivo bleeding time up to 15 min (16-JAN-2006). The hematologist stated that these findings (prolonged in vivo bleeding time in the context of normal amount of thrombocytes and normal function) suggest most likely a possible vascular related disturbance of primary function of hemostasis.

On 02-MAY-2006, the patient was diagnosed with severe affinity of hematoma formation. Outcome regarding edema, tooth extraction with post procedural bleeding, lipid metabolism disorder, glomerular proteinuria, paroxysmal tachycardia and arterial hypertension, hemorrhagic cystitis and mixed type proteinuria, hemorrhage of forearms including very thin skin, prolonged in vivo bleeding time up to 15 min and severe affinity of hematoma formation was not reported.

On 08-NOV-2006, the patient developed an acute episode of Bechterew's disease and therefore was hospitalized. Upon admission, she was diagnosed with thyroid nodule. Outcome was not reported.

On 05-NOV-2007, the patient developed loss of blood pressure control and therefore was hospitalized. Upon admission, she was diagnosed with thoracic pain. She received treatment with candesartan cilexetil (ATACAND) 8 mg tablet, 4 mg three times a day (therapy initiated on 05-NOV-2007). Because she developed an allergic skin reaction to angiotensin II blocker, therapy with candesartan cilexetil (ATACAND) was discontinued (date unknown) and she received treatment with cortisone. On 19-NOV-2007, she was recovering from allergic skin reaction to angiotensin II blocker and recovered from loss of blood pressure control. The outcome of the thoracic pain was not reported.

The reporting medical expert felt that the hypersensitivity reactions with angioedema and urticaria, the anaphylactic/anaphylactoid reactions, vasculitis, palpitations, Stevens-Johnson syndrome/Lyell's syndrome, heparin allergy and cross allergy with diclofenac were related to therapy with rofecoxib; he did not rate the drug relationship regarding the other symptoms. The allergologist felt that psoriasis-like drug-induced exanthema was related to therapy with rofecoxib. The physician felt that fall, laceration, hematoma, tissue vulnerability and delayed wound healing were related to therapy with rofecoxib. The lawyer felt that ringing in ears, dizziness, sleep disturbances and exertional dyspnea were as well related to rofecoxib. The physicians did not rate the drug relationship concerning the acute episode of Bechterew's disease, thyroid nodule, loss of blood pressure control, thoracic pain and allergic skin reaction to angiotensin II blocker.

Vasculitis and Stevens-Johnson syndrome/Lyell's syndrome were considered to be immediately life-threatening and disabling.

Information was received on 20-DEC-2005 which pertained to an event that occurred 3 years following the discontinuation of rofecoxib in July 2002. This information contained the following adverse experience: tumorous changes in the liver (2005).

Follow up information received on 19-JUN-2006 which pertained to events that occurred following the discontinuation of rofecoxib on 18-Jul-2002. This information contained the following adverse experiences:

edema (onset date not reported), lipid metabolism disorder and glomerular proteinuria (24-OCT-2005), mixed type proteinuria (23-DEC-2005), very thin skin (16-JAN-2006).

All available medical records will be provided upon request.

Additional information has been requested.

Sender's comment:

Diese Meldung erhielten wir erst nach Bekanntwerden der Marktrücknahme von VIOXX°. Eine Anwältin berichtet, dass sich bei einer Mandantin unter der Therapie mit VIOXX° eine sehr schwer verlaufende chronische Form eines Steven-Johnson/Lyell-Syndroms, sowie eine Vaskulitis entwickelten. Die übrigen berichteten Ereignisse sind nicht schwerwiegend. Die genannten schwerwiegenden Ereignisse waren in der Fach- und Gebrauchsinformation von VIOXX° beschrieben. Der Allergologe, der den Hautauschlag der Patientin im Juni 2002 behandelt hatte, hatte ein generalisiertes makulopapuläres Exanthem diagnostiziert. Die Patientin wurde ambulant mit Prednisolon 40 mg am Tag und Bethamethason 0,1 % Creme behandelt. Die Diagnose und Therapie schließen ein kritisches Krankheitsbild wie Steven-Johnson- oder Lyell-Syndrom aus. Angaben oder Dokumentation einer Ätiologieabklärung (Allergietestung, Hautbiopsien, immunologische Differentialdiagnostik etc.) fehlen. Ohne diese Angaben bleibt ein Kausalzusammenhang mit VIOXX° auch weiterhin unklar. Die uns bisher vorliegenden Informationen erlauben keine Kausalitätsbewertung. Ein kausaler Zusammenhang zwischen den neu dokumentierten schwerwiegenden Ereignissen (einem akuten Schub des bekannten M. Bechterew (nicht gelabelled) und einer hypertensiven Entgleisung (gelabelled) - beide min. 4 Jahre nach der letzten VIOXX°-Einnahme aufgetreten) und der VIOXX°

Therapie scheint aufgrund des zeitlichen Zusammenhangs sehr unwahrscheinlich. Wir werden den Fall weiter abklären und unaufgefordert wieder berichten. MSD hat am 30. September 2004 VIOXX[®] freiwillig weltweit vom Markt zurückgenommen.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-06061370

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 56 Year	weight (kg): 60	BMI_int: 20.76	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin necrosis	20060424	20061002	5 Month	5 Day	recovered/resolved
2 Drug eruption	20060424	20061002	5 Month	5 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Oral	50 Mg milligram(s)/Day	20060420-20061002	Pain 5 Month

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 27.12.2006 DE-002147023-PHNU2006DE01973

Sender diagnosis: not reported

Narrative

Initial physician report received on 12 May 2006. A patient has been treated with Voltaren (diclofenac) 25 2x daily on demand. After 8 days of administration skin exanthema appeared. Voltaren was discontinued immediately but the exanthema deteriorated into skin necrosis (2cm length). The physician asked Novartis about known relationship to Voltaren and skin necrosis.

Follow up report received on 14 Dec 2006. Voltaren was started on 20 Apr 2006. Adverse reaction started on 24 Apr to 02 Oct 2006. Therapy with Voltaren was discontinued on 02 Oct 2006. The patient recovered. The causality was assessed as probable.

Sender's comment:

Serious spontaneous report [medically significant] already listed in the Basic Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07002096

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 67 Year	weight (kg): 63	BMI_int: 23.14	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blisters	20070117			1 Day	recovered/resolved
2 Erythema	20070117			1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN RESINAT diclofenac resinate	Oral	1 Df dosage form/Day	20070117-	-

Drugs (concomitant)

2 DYNASTAT parecoxib sodium	/	20070116-	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bee sting hypersensitivity			Unknown	Bee venom allergy
Aortic valve insufficiency			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 29.01.2007 DE-002147023-PHNU2007DE00641

Sender diagnosis: not reported

Narrative

Initial report received on 22 Jan 2007 and 23 Jan 2007 (phone call on 23 Jan 2007). Preliminary scarce information from physician and nurse. This patient started treatment with Voltaren Resinat (diclofenac, colestyramin) on 17 Jan 2007, for unstated reasons. She had received a Dynastat (parecoxib) infusion on the day before. On 17 Jan 2007, shortly after Voltaren Resinat initiation, patient showed erythema at wrists, fingers and arms ("like burns"). After the patient developed large blisters, she was admitted to hospital on 18 Jan 2007. A causal relationship with Voltaren Resinat was addressed, due to the close temporal relationship, while the Dynastat infusion was not suspected as causally related.

Sender's comment:

Serious spontaneous report (hospitalisation), Blisters already listed in the Basis Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:NOVARTIS, 20.02.2007 DE-002147023-PHNU2007DE00641

Sender diagnosis: not reported

Narrative

Initial report received on 22 Jan 2007 and 23 Jan 2007 (phone call on 23 Jan 2007). Preliminary scarce information from physician and nurse. This patient started treatment with Voltaren Resinat (diclofenac, colestyramin) on 17 Jan 2007, for unstated reasons. She had received a Dynastat (parecoxib) infusion on the day before. On 17 Jan 2007, shortly after Voltaren Resinat initiation, patient showed erythema at wrists, fingers and arms ("like burns"). After the patient developed large blisters, she was admitted to hospital on 18 Jan 2007. A causal relationship with Voltaren Resinat was addressed, due to the close temporal relationship, while the Dynastat infusion was not suspected as causally related.

Follow up received on 15 Feb 2007 (written documentation from physician). Further patient details were given. On 17 Jan 2007, i.e. the first day of treatment, patient showed swelling and redness in the area of eyes, front, and both arms, as well as skin blisters at her wrists. After stopping medication, the patient made a full recovery. No hospital discharge date was mentioned. The reporting physician assessed Voltaren Resinat as causally related, thereby excluding other causations.

Sender's comment:

Serious spontaneous report (hospitalisation), Blisters already listed in the Basis Prescribing Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07004211

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 74 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Allergic reaction					recovered/resolved
2 Skin reaction					recovered/resolved
3 Exanthematic drug eruption					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo 100 retard - 1 a pharma diclofenac sodium	Oral	2 Df dosage form/Day	- 1 Day	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

case: DE-BFARM-07004225

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Initials: Sex: Female Birth: privacy Age: 44 Year weight (kg): BMI int: Age fetus:

Reactions

MedDRA LLT		Begin	End	duration	TtO	outcome
1	Skin reaction localised	20070125			1 Day	recovered/resolved
2	Skin red	20070125			1 Day	recovered/resolved
3	Blisters	20070125			1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 rewodina schmerzgel	Topical	2 Df dosage form/Day	20070125-20070128	
diclofenac sodium			4 Day	

Drugs (concomitant)

2	amineurin 25 amitriptyline hydrochloride	25 Mg milligram(s)/Day	-
3	diclofenac 75 mg diclofenac sodium	150 Mg milligram(s)/Day	-
4	lyrica pregabalin	150 Mg milligram(s)/Day	-
5	oxygesic oxycodone hydrochloride	120 Mg milligram(s)/Day	-

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07008436

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 48 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	200702				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine		/	200702- 1440 Minute	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug allergy			Unknown	Ibuprofen allergy

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 27.03.2007 DE-002147023-PHNU2007DE01265

Sender diagnosis: not reported

Narrative

Initial physician report received on 20 March 2007. The patient had been treated with Voltaren (Diclofenac) Schmerzgel and after the first administration Feb 2007 he developed Steven Johnson Syndrome which led to hospitalization. Preliminary allergy against Ibuprofen was well known. Patient practiced selfmedication with Voltaren Schmerzgel without physician consulting. No further information available.

Sender's comment:

Serious spontaneous report, (hospitalisation), Stevens-Johnson syndrome assessed as unlisted according to the Basic Prescribing Information. However other alternative causes (other drugs), allergy against Ibuprofen was well known provide a possible explanation for the reported adverse event.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07040910

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema					Unknown
2 Actinic keratosis					Unknown
3 Inflammation					Unknown
4 Skin scaly					Unknown
5 Edema					Unknown
6 Serous discharge					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Topical	/	-	
diclofenac			21600 Minute	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	British Journal of Dermatology; Patel M, Ulrich C, Forschner T; 156 (3); 2007; 57-61; Genetically determined susceptibility to COX-2 inhibitors - a report of exaggerated responders to diclofenac 3% gel in the treatment of actinic keratoses	
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 21.06.2007 DE-002147023-PHBS2007DE10086

Sender diagnosis: not reported

Narrative

Literature report received on 18 Jun 2007. (5 of 10) This patient was applied twice a day with diclofenac gel (manufacturer name unknown) for an unknown indication during 15 days. He was advised to due strict sun protective measures. He experienced severe actinic keratosis with inflammation, erythema, scaling, edema and exudation. The patch test for diclofenac was negative. The outcome of the vent was not reported.

Sender's comment:

The available information was considered inadequate to fully assess the case.

Serious spontaneous report, (medically significant), Actinic keratosis assessed as unlisted according to the Basic Prescribing Information. The information provided in this individual case does not warrant a change to the Basic Prescribing Information text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07040918

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 72 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Serous discharge					Unknown
2 Erythema					Unknown
3 Edema					Unknown
4 Skin scaly					Unknown
5 Actinic keratosis					Unknown
6 Inflammation					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Topical	/	-	
diclofenac			1440 Minute	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	British Journal of Dermatology; Patel M, Ulrich C, Forschner T; 156 (3); 2007; 57-61; Genetically determined susceptibility to COX-2 inhibitors - a report of exaggerated responders to diclofenac 3% gel in the treatment of actinic keratoses	
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 21.06.2007 DE-002147023-PHBS2007DE10087

Sender diagnosis: not reported

Narrative

Literature report received on 18 Jun 2007. (9 of 10) This patient was applied twice a day with diclofenac gel (manufacturer name unknown) for an unknown indication during 1 day. He was advised to due strict sun protective measures. He experienced severe actinic keratosis with inflammation, erythema, scaling, edema and exudation. The patch test for diclofenac was negative. The outcome of the event was not reported.

Sender's comment:

The available information was considered inadequate to fully assess the case.

Serious spontaneous report, (medically significant), Actinic keratosis assessed as unlisted according to the Basic Prescribing Information. The information provided in this individual case does not warrant a change to the Basic Prescribing Information text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07040943

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 69 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Serous discharge					Unknown
2 Edema					Unknown
3 Skin scaly					Unknown
4 Erythema					Unknown
5 Actinic keratosis					Unknown
6 Inflammation					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Topical	/	-	
diclofenac			1440 Minute	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	British Journal of Dermatology; Patel M, Ulrich C, Forschner T; 156 (3); 2007; 57-61; Genetically determined susceptibility to COX-2 inhibitors - a report of exaggerated responders to diclofenac 3% gel in the treatment of actinic keratoses	
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 21.06.2007 DE-002147023-PHBS2007DE10088

Sender diagnosis: not reported

Narrative

Literature report received on 18 Jun 2007. (10 of 10) This patient was applied twice a day with diclofenac gel (manufacturer name unknown) for an unknown indication during 1 day. He was advised to due strict sun protective measures. He experienced severe actinic keratosis with inflammation, erythema, scaling, edema and exudation. The patch test for diclofenac was negative. The outcome of the vent was not reported.

Sender's comment:

The available information was considered inadequate to fully assess the case.

Serious spontaneous report, (medically significant), Actinic keratosis assessed as unlisted according to the Basic Prescribing Information. The information provided in this individual case does not warrant a change to the Basic Prescribing Information text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07040969

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 69 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Actinic keratosis					Unknown
2 Inflammation					Unknown
3 Erythema					Unknown
4 Skin scaly					Unknown
5 Edema					Unknown
6 Serous discharge					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Topical	/	-	
diclofenac			77760 Minute	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	British Journal of Dermatology; Patel M, Ulrich C, Forschner T; 156 (3); 2007; 57-61; Genetically determined susceptibility to COX-2 inhibitors - a report of exaggerated responders to diclofenac 3% gel in the treatment of actinic keratoses	
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 21.06.2007 DE-002147023-PHBS2007DE10081

Sender diagnosis: not reported

Narrative

Literature report received on 18 Jun 2007. (6 of 10) This patient was applied twice a day with diclofenac gel (manufacturer name unknown) for an unknown indication during 54 days. He was advised to due strict sun protective measures. He experienced severe actinic keratosis with inflammation, erythema, scaling, edema and exudation. The patch test for diclofenac was negative. The outcome of the event was not reported.

Sender's comment:

The available information was considered inadequate to fully assess the case.

Serious literature report, (medically significant), Actinic keratosis assessed as unlisted according to the Basic Prescribing Information. The information provided in this individual case does not warrant a change to the Basic Prescribing Information text. The topic will be monitored closely.

All literature reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-07040984

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 72 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Actinic keratosis					Unknown
2 Serous discharge					Unknown
3 Inflammation					Unknown
4 Erythema					Unknown
5 Skin scaly					Unknown
6 Edema					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Topical	/	-	
diclofenac			44640 Minute	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	British Journal of Dermatology; Patel M, Ulrich C, Forschner T; 156 (3); 2007; 57-61; Genetically determined susceptibility to COX-2 inhibitors - a report of exaggerated responders to diclofenac 3% gel in the treatment of actinic keratoses	
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 21.06.2007 DE-002147023-PHBS2007DE10078

Sender diagnosis: not reported

Narrative

Literature report received on 18 Jun 2007. (5 of 10) This patient was applied twice a day with diclofenac gel (manufacturer name unknown) for an unknown indication during 31 days. He was advised to due strict sun protective measures. He experienced severe actinic keratosis with inflammation, erythema, scaling, edema and exudation. The patch test for diclofenac was negative. The outcome of the event was not reported.

Sender's comment:

The available information was considered inadequate to fully assess the case.
 Serious literature report, (medically significant), Actinic keratosis assessed as unlisted according to the Basic Prescribing Information. The information provided in this individual case does not warrant a change to the Basic Prescribing Information text. The topic will be monitored closely.

All literature reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07066724

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Female Birth: privacy Age: 65 Year weight (kg): 68 BMI_int: 24.98 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Swelling of tongue	20070622			5 Day	recovered/resolved
2 Mouth ulcer	20070622			5 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo 75 diclofenac sodium	Oral	2 Df dosage form/Day	20070618-20070621 4 Day	Periarthritis scapulohumeralis

Primary source

Qualification: Physician

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland Type: Spontaneous report case: DE-BFARM-07072184

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes Yes Yes Yes Yes Yes Yes

Patient

Initials: Sex: Male Birth: privacy Age: 39 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Rhabdomyolysis					fatal
2 Purpura fulminans					fatal
3 Stevens-Johnson syndrome					fatal
4 Shock					fatal
5 DIC					fatal
6 Multi organ failure					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Intramuscular	75 Mg milligram(s)/ Injection	-	Common cold syndrome
diclofenac			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Common cold			Yes	

Primary source

Qualification: Physician observestudytype:
Literature: Hengge U, Jochum C, Tschakarjan E, Maschke J, Erbel R, Schmid K, Otterbach F. Purpura fulminans - Fatale Folge einer alltäglichen Therapie?. Hautarzt 2002;53:483-487.

Narratives (nur bei elektronischem Eingang)

Merck KGaA, 13.08.2007 DE-Merck KGaA-6036615

Sender diagnosis: not reported

Narrative

On 06-Jul-2007 this spontaneous case report was received from German literature:

U.R. Hengge, C. Jochum, E. Tschakarjan, J. Maschke, R. Erbel, K.W. Schmid, F. Otterbach;
Purpura fulminans - Fatale Folge einer alltäglichen Therapie?
[purpura fulminans - fatal consequence of a common therapy?]
journal: Hautarzt 2002, 53; pages 483-487
Springer-Verlag

Purpura fulminans
A fatal consequence of a widely used medication?

Abstract
A 39-year old man developed purpura fulminans, a manifestation of disseminated intravascular coagulopathy, with rhabdomyolysis following the i. m. injection of diclofenac.

Case Report

History of Present Illness
This patient had apparently caught a common cold from his 3-year-old twin daughters in the course of a week. He complained of severe (muscle) aches and pains, fever of up to 40°C and chills and, therefore, his physician gave him an intramuscular injection of Diclofenac-75 into the gluteus maximus muscle. The very same night he developed ecchymoses over large areas of the lower extremities, toes and fingers. Within 12 hours, the patient, having

developed respiratory insufficiency requiring mechanical ventilation, was admitted to the ICU for severe shock.

Findings

Dermatologic Findings

There were mostly sharply delineated, livid ecchymoses with erythematous margins. Some skin areas were not affected. On the abdomen, the patient developed dirty-gray erythema with numerous blisters, some of them confluent. Nikolski's direct sign could be elicited. Some of the confluent blisters were eroded and covered with a hemorrhagic crust. The palms of the hands also showed blisters measuring 1-2 cm across. The fingers and toes showed evidence of blackish-blue hemorrhage, developing into dry, leathery gangrene on subsequent days. In addition, erosions were seen in the face and oral cavity, on and in the nose, the aural helices, the lips, and on the scrotum. Approximately 45% of the entire skin surface area was involved, with about 15% of the SSA showing evidence of exfoliation. Interestingly, the ecchymoses had stopped spreading after the first 24 hours.

Histology

Biopsy revealed subepidermal cleaving under a necrotic epidermis, and sweat gland necroses. Most dermal blood vessels were lined with fibrin clots, and there were huge numbers of erythrocytes in the upper dermis. At one site, the dermis beneath a fresh blister showed evidence of segmentally structured acute granulocytic vasculitis in the area of small venules.

Diagnostic Workup

While the patient had no history of recent travel to endemic areas, hemorrhagic fever was ruled out by excluding Hanta, Dengue, Ebola, and Marburg viruses by antibody testing and electron microscopic blood examination. This was also motivated by the fact that the patient was bleeding profusely from catheter/needle entry sites and body orifices. VZV, HSV, and mycoplasma serology also revealed no abnormalities. Culture studies of tracheal secretion, blood, and CSF produced no evidence of meningococci (Waterhouse-Friderichsen syndrome) or any other bacteria.

Abnormal Laboratory Tests

WBC count 39.6/nL; differential: neutrophils 80%, lymphocytes 11%; Hb 8.0 g/dL, platelets 23/nL; CRP 53.7 mg/dL, total protein 3.3 g/dL, creatinine 2.4 mg/dL, GOT 916 U/L, GPT 190 U/L, LDH 5,423 U/L, CK 12,441 U/L, troponin 2.9 mg/dL, prothrombin level 18%, PTT 108.5 sec, serum myoglobin 2,466 microgram/L, D-dimers 17.0 microgram/L, CH50 282 mU/mL, fibrinogen 415 mg%, ATIII 69%.

Treatment

Based on a provisional diagnosis of Waterhouse-Friderichsen syndrome, we prescribed broad (spectrum) antibiotic therapy with high-dose penicillin, doxycycline, cefuroxime, and gentamicin. As no infectious pathogens had been detected and after the histology findings were back, we prescribed high-dose corticosteroid therapy (500 or 1,000 mg of prednisolone/day) based on a revised provisional diagnosis of a severe toxic ADR. Disseminated intravascular coagulation (DIC) additionally necessitated multiple transfusions of fresh frozen plasma, packed red cells, and platelet concentrates. Low-dose heparin was administered as well. No new skin efflorescences appeared with this treatment.

Further Clinical Course

Despite maximum intensive care therapy, hemodynamic stabilization was only achieved with high-dose dopamine and/or norepinephrine and replacement of 30 liters of fluid. Subsequently the patient developed worsening respiratory insufficiency and massive whole body edema. In the course of treatment, he also developed septic temperatures, *Aspergillus* bronchopneumonia, and septic *Aspergillus* myocarditis. Moreover, there was progressive renal failure in the presence of rhabdomyolysis (myoglobin 80,000 microgram/L). On day 16, the patient developed massive pulmonary hemorrhage with obstruction of the main bronchi, as evidenced by rigid bronchoscopy. Bronchoscopy performed shortly before death revealed large-area exfoliation of the bronchial mucosa, leading to blockage of the air-carrying bronchi. As a result, oxygenation was no longer possible even with 100% oxygen and despite maximum PEEP, and the patient died in multiple organ failure.

Autopsy Findings

Autopsy revealed severe hemorrhagic necrotizing bronchopneumonia with evidence of *Aspergillus* mycelium in the right upper lung lobe. Also diagnosed was severe hemorrhagic necrotizing tracheobronchitis with profuse intrabronchial and endotracheal hemorrhage. Shock lung, shock kidney, shock liver, and septic myocarditis with evidence of *Aspergillus* mycelium led to the patient's death. On the skin and mucous membranes, we made the pathological anatomical diagnosis of purpura fulminans. In conclusion, the patient was

diagnosed as having suffered multiple organ failure, initially toxic, thereafter septic, in the presence of purpura fulminans with severe hemorrhagic necrotizing bronchopneumonia and Aspergillus myocarditis.

Author's comment:

Given the acute nature of the clinical picture, a case of purpura fulminans after dosing with diclofenac for a febrile infection was presented after ruling out testable differential diagnoses (hemorrhagic fever and Waterhouse-Friderichsen syndrome).

The uniqueness of this case, i.e., not extensively hemorrhagic subepidermal blistering and mucosal involvement, initially suggested a diagnosis of Steven-Johnson Syndrom/toxic epidermal nekrolysis overlap syndrome with associated DIC and rhabdomyolysis. The acute onset of purpura along with a preceding common cold and rapid-onset symmetric necroses of the toes and fingers, however, suggest that a diagnosis of purpura fulminans is likely. However, it cannot be definitively determined whether this patient suffered the acute [para]infectious form or rather drug induced PF associated with dosing with diclofenac. Failure to detect a bacterial pathogen along with failure of broad (spectrum) antibiotic therapy might also be compatible with a viral trigger, but the usual respiratory viruses (influenza and parainfluenza virus, adenovirus, respiratory syncytial virus, rhinovirus, etc.) failed to be detected serologically.

In the present case of severe epidermal necrosis, especially with sweat gland involvement, drug-induced coma or use of illicit drugs (such as barbiturates or ecstasy) should be considered as well. However, in our patient this had been ruled out by intense drug screening because he was a senior employee at a drug rehabilitation clinic.

Sender's comment:

Literature case report concerning a 39-year old male patient who developed purpura fulminans (PF), Stevens-Johnson-Syndrom, a shock with manifestation of disseminated intravascular coagulopathy and with rhabdomyolysis leading to multi-organ-failure following the single IM injection of diclofenac (no brand name stated). Concomitant virus-infection was known.

Temporal relationship was considered as compatible.

According to the reporter however, it cannot be definitively determined whether this patient suffered acute infectious form or rather drug induced PF associated with diclofenac. Rhabdomyolysis could be caused by virusinfection or electrolyte disturbances as well.

Company causality was assessed as possible.

Purpura fulminans, Stevens Johnson Syndrom and shock are listed for diclofenac as per company reference document; rhabdomyolysis, DIC and multi-organ failure as single terms are not listed, but expected consequences of listed shock and inhibition of thrombocyte aggregation.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07079887

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 68 Year	weight (kg): 85	BMI_int: 30.85	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis				1095 Day	Unknown
2 Skin exfoliation				1095 Day	recovered/resolved
3 Skin edema				1095 Day	recovered/resolved
4 Blister				1095 Day	recovered/resolved
5 Necrosis					recovered/resolved
6 Surgery					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo diclofenac	Oral	150 Mg milligram(s)/	2004- -	Analgesia
2 diclo 1A Pharma diclofenac	Oral	75 Mg milligram(s)/Day	- 1440 Minute	Analgesia

Drugs (concomitant)

3 BISOPROLOL bisoprolol	Unknown	/	- -
4 Kaliumjodid potassium iodide	Unknown	/	- -
5 Kaliumjodid potassium iodide	Unknown	/	- -
6 levothyroxine levothyroxine sodium	Unknown	/	- -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Arthrosis			Unknown	

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 30.08.2007 DE-SZ09-GXKR2007DE07160

Sender diagnosis: not reported

Narrative

Initial spontaneous report received from a consumer on 20 Aug 2007. The patient with an unknown medical history developed skin reactions after a long term treatment (over approximately 3 years) with Diclo 75 SL - 1A Pharma (diclofenac sodium) for analgesia and arthrosis. On examination the patient had burn-like vesication below the mamilla, accumulation of liquid underneath the skin, and the skin peeled off. Necrolysis was diagnosed by the patient's family doctor and the patient was referred to a gynecologist. An ambulant operation was necessary and performed by the gynecologist. The outcome of the patient was unknown.

Local case-ID: 2007-209

Sender's comment:

Serious spontaneous report (medically significant) already listed in the Core Data Sheet Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 05.09.2007 DE-SZ09-GXKR2007DE07160

Sender diagnosis: not reported

Narrative

Initial spontaneous report received from a consumer on 20 Aug 2007. The patient with an unknown medical history developed skin reactions after a long term treatment (over approximately 3 years) with Diclo 75 SL - 1A Pharma (diclofenac sodium) for analgesia and arthrosis. On examination the patient had burn-like vesication below the mamilla, accumulation of liquid underneath the skin, and the skin peeled off. Necrolysis was diagnosed by the patient's family doctor and the patient was referred to a gynecologist. An ambulant operation was necessary and performed by the gynecologist. The outcome of the patient was unknown.

Local case-ID: 2007-209

Significant FU received from treating physician on 27 Aug 2007: Further patient details provided. The patient took only one tablet of Diclo 75 SL - 1 A Pharma (diclofenac) and was on a former long-term treatment with diclofenac since 2004. The physician diagnosed necrosis and performed an extirpation. The patient recovered. The physician stated that the causal relationship is possible.

Local case ID: 2007-209

Sender's comment:

Serious spontaneous report (medically significant) already listed in the Core Data Sheet Information.

All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07082756

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No		No						

Patient

Initials:	Sex: Female	Birth: privacy	Age: 47 Year	weight (kg): 57	BMI_int: 20.94	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dermatitis medicamentosa	20070511	20070630	51 Day	9 Day	recovered/resolved
2 Myalgia	20070511			9 Day	recovered/resolved
3 Ache	20070511			9 Day	recovered/resolved
4 LDH increased	20070511			9 Day	recovered/resolved
5 CRP increased	20070511			9 Day	recovered/resolved
6 Immunoglobulin E increased	20070511			9 Day	recovered/resolved
7 Erythrocyte sedimentation rate increased	20070511			9 Day	recovered/resolved
8 Arthralgia	20070511			9 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm 100 diclofenac sodium		150 Mg milligram(s)/Day	20070503-20070511 9 Day	Radiculopathy

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07093725

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg): 70	BMI_int: 22.09	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell syndrome	20070920	20070927	8 Day	2 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	150 Mg milligram(s)/Day	20070916-20070917	Neck pain
2 KEPPRA levetiracetam	Oral	1500 Mg milligram(s)/Day	20070919-20070922	Complex partial seizures
			2 Day	
			4 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Complex partial seizures			Unknown	
CK increased			Unknown	
Transaminases increased			Yes	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

UCB GmbH, 08.10.2007 DE-UCBSA-8027151

Sender diagnosis: not reported

Narrative

A physician reported that a 44-year-old black-African male patient, was hospitalized in Sep-2007 due to complex partial seizures. He received Keppra (levetiracetam) 1000 mg and two days after the start of Keppra, he got a generalized "frazzle-exanthema" comparable to a Lyell syndrome. Keppra was immediately stopped on 22-Sep-2007. The exanthema did not worsen since then. The patient switched to Frisium (clobazam) against seizures. The reporter stated that the skin of the patient's head was like a single scab. The patient was still hospitalized at the time of this report.

Sender's comment:

BLACK-AFRICAN PATIENT DEVELOPED GENERALIZED FRAZZLE EXANTHEMA. NO DATA ON RELEVANT TESTS AND PRESENCE OF MUCOSAL LESIONS.

Reporter's comment:UCB GmbH, 14.12.2007 DE-UCBSA-8027151

Sender diagnosis: not reported

Narrative

A physician reported that a 44-year-old black-African male patient, was hospitalized in Sep-2007 due to complex partial seizures. He received Keppra

(levetiracetam) 1000 mg and two days after the start of Keppra, he got a generalized "frazzle-exanthema" comparable to a Lyell syndrome. Keppra was immediately stoppped on 22-Sep-2007. The exanthema did not worsen since then. The patient switched to Frisium (clobazam) against seizures. The reporter stated that the skin of the patient's head was like a single scab. The patient was still hospitalized at the time of this report.

Additional information was received from the neurologist on 03-Dec-2007.

The reaction, described as a generalized skin allergy with bullous skin detachment (dry), began on 20-Sep-2007. An allergic skin reaction to diclofenac was pre-existing and improving but exacerbated notably under Keppra monotherapy.

The patient took diclofenac 75 mg twice daily from 16-Sep-2007 to 17-Sep-2007 for neck pain.

The patient had a medical history of elevated transaminases which were resolving and also a history of increased CK.

The patient fully recovered from the event after drug withdrawal and initiation of antiallergic therapy (cortisone + loratadine).

The neurologist assessed the causal relationship between Keppra and the event as possible.

Sender's comment:

BLACK-AFRICAN PATIENT DEVELOPED GENERALIZED FRAZZLE EXANTHEMA. NO DATA ON RELEVANT TESTS AND PRESENCE OF MUCOSAL LESIONS. PATIENT ALLERGIC TO DICLOFENAC. EVENT RESOLVED AFTER DISCONTINUATION OF BOTH LEVETIRACETAM AND DICLOFENAC.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-07126992

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		Yes

Patient

Initials:	Sex:	Birth: privacy	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac basics 50mg magensaftresistent	Oral	/	-	Lumbar pain
diclofenac sodium			-	

Primary source

Qualification: observestudytype:

Literature: P Spornraft-Ragaller, M Ragaller, M Meurer. Toxic epidermal necrolysis induced by NSAID. Z Rheumatol.. 2003; 62(5); 474-5

Narratives (nur bei elektronischem Eingang)RANBAXY, 19.12.2007 DE-RANBAXY-2007RR-12124

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2007RR-12124 from GERMANY refers to a 68 year old female who developed toxic epidermal necrolysis following treatment with diclofenac which was fatal.

The case was reported in the literature.

The subject received the following suspect product:

Diclofenac: orally, for lumbar pain.

The patient developed severe toxic epidermal necrolysis following oral administration of diclofenac.

On presentation, the patient had generalised erythema of 70%.

The patient had multiforme exudative erythema with purulent vesicles and large erosive areas. Erythema invasion reached 85%, 60% vesicles of the entire body surface, respectively.

Skin histology revealed almost total epidermic necrolysis.

Despite treatment with specific antibiotic treatment and high-dose glucocorticoid therapy, the patient died because of multi-organ failure and septic shock. The patient also received intravenous immunoglobulin.

The outcome of the event was reported as "fatal" at the time of this report.

The case was reported as serious because the patient died.

Sender's comment:Reporter's comment:

The case is rated as serious, expected and possible with diclofenac use.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08001542

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		Yes

Patient

Initials:	Sex:	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin tumor NOS					Unknown
2 Stomatitis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclofenac retard-ratiopharm 100(Retardkapseln)	Oral	/	-	
diclofenac sodium		Prolonged-release capsule, hard	-	

Drugs (concomitant)

2 opipramol	/	-	
opipramol		-	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

ratiopharm GmbH, 09.01.2008 DE-RATIOPH-2007R127513

Sender diagnosis: not reported

Narrative

Reported terms: Tumour on tongue(Neoplasm skin) Oral inflammation (Stomatitis)

Case reference number 2007R127513 is a spontaneous case report sent by a pharmacist which refers to a unk of unknown age.

The patient's past medical history included no medical records. There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started taking Diclofenac retard-ratiopharm 100(Retardkapseln) (DICLOFENAC), indication unknown on an unknown date. The patient has been exposed to the following concomitant drug(s): Opipramol (OPIPRAMOL), indication unknown. The patient developed Tumour on tongue (Neoplasm skin)(start date ?) and Oral inflammation (Stomatitis)(start date ?).

The following action was taken: for Diclofenac retard-ratiopharm 100(Retardkapseln) (DICLOFENAC) - Unknown

The outcome for Tumour on tongue (Neoplasm skin) is Unknown. The outcome for Oral inflammation (Stomatitis) is Unknown.

Rechallenge information for Diclofenac retard-ratiopharm 100(Retardkapseln) is not applicable. Dechallenge information for Diclofenac retard-ratiopharm 100(Retardkapseln) is not applicable.

The reporter's assessment for drug Diclofenac retard-ratiopharm 100(Retardkapseln) related with Oral inflammation is as follows: Unknown. The reporter's assessment for drug Diclofenac retard-ratiopharm 100(Retardkapseln) related with Tumour on tongue is as follows: Unknown. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08008088

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 66 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20030201	deathcause: Toxic epidermal necrolysis
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pain	20021213				not recovered/not resolved
2 Dry cough	20030102	20030105	4 Day	20 Day	Unknown
3 Sleep disorder	20030102	20030105	4 Day	20 Day	Unknown
4 Blister	20030105		28 Day		fatal
5 Macule	20030105		28 Day		fatal
6 Nikolsky's sign	20030105		28 Day		fatal
7 Oral ulceration	20030105		28 Day		fatal
8 Pain of skin	20030105		28 Day		fatal
9 Tachycardia	20021217			4 Day	Unknown
10 Edema	20021221			8 Day	Unknown
11 Constipation	20021227			14 Day	Unknown
12 Hypertension	20021219			6 Day	Unknown
13 Hypertensive crisis	20021219			6 Day	Unknown
14 Fever	20021216			30 Day	Unknown
15 Infection					Unknown
16 Sepsis					Unknown
17 Mucosal erosion	20030109			27 Day	fatal
18 Rash	20030105			23 Day	fatal
19 Exanthema	20030105			23 Day	fatal
20 Macular rash	20030105			23 Day	fatal
21 Toxic epidermal necrolysis	20030105			23 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	BAYOTENSIN nitrendipine	Oral	/	20021219-20021219 Hypertension 1440 Minute
2	BAYOTENSIN nitrendipine		/	20030103-20030103 Hypertension 1440 Minute
3	BAYOTENSIN nitrendipine	Oral	/	20021229-20021230 Hypertension 2 Day
4	BAYOTENSIN AKUT nitrendipine	Oral	2 ml millilitre(s)/	20021219-20021219 Hypertensive crisis -
5	CIPROBAY ciprofloxacin hydrochloride	Intravenous (not otherwis	200 Mg milligram(s)/	20021216-20021230 Sepsis 15 Day
6	ciprobay 100 ciprofloxacin lactate	Intravenous (not otherwis	/ Solution for injection	20021216-20021230 Sepsis 15 Day
7	CLEXANE enoxaparin	Subcutaneous	/	20021218- Thrombosis prophylaxis -
8	HEPARIN heparin	Subcutaneous	/	19921229-19921229 Thrombosis prophylaxis 1440 Minute
9	HEPARIN heparin	Subcutaneous	/	-20021217 Thrombosis prophylaxis -
10	INSULIN insulin	Subcutaneous	/	20021212-20021212 Diabetes mellitus 1440 Minute
11	INSULIN insulin		/	20021220-20030101 Diabetes mellitus 13 Day
12	insuman rapid insulin human; isophane insulin		/	20021230-20021230 Diabetes mellitus 1440 Minute
13	insuman rapid insulin human; isophane insulin	Intravenous (not otherwis	/	20021226-20021228 Diabetes mellitus 3 Day
14	insuman rapid insulin human; isophane insulin	Intravenous (not otherwis	/	20021217-20021217 Diabetes mellitus 1440 Minute
15	INSUMAN RAPID insulin human	Subcutaneous	/ Solution for injection	20021217-20021217 Diabetes mellitus -
16	Kontrastmittel		/	20021230-20030101 3 Day
17	Kontrastmittel		/	20030107-20030109 3 Day
18	LANTUS insulin glargine	Subcutaneous	/	20021219- Diabetes mellitus -
19	LORZAAR PLUS hydrochlorothiazide; losartan potassium	Oral	/	20030102-20030109 Hypertension 8 Day
20	NEXIUM esomeprazole magnesium	Oral	80 Mg milligram(s)/Day	20021219- Prophylaxis -
21	NITROLINGUAL-SPRAY glyceryl trinitrate	Oral	/ Cutaneous Spray	20021214-20021214 Hypertensive crisis -
22	NITROLINGUAL-SPRAY N glyceryl trinitrate		/	20021219-20021219 Hypertension 1440 Minute
23	NITROLINGUAL-SPRAY N glyceryl trinitrate	Oral	/	20021214-20021214 Hypertension 1440 Minute
24	NITROLINGUAL-SPRAY N glyceryl trinitrate		/	20021230-20021230 Hypertension 1440 Minute
25	NOVALGIN metamizole	Oral	/	20021230-20030101 Pain 3 Day
26	NOVALGIN metamizole	Oral	/	20030107-20030109 Pain 3 Day
27	NOVALGIN metamizole	Oral	30 Gtt drop(s)/ Irrigation solution	20021230-20030101 Pain 3 Day
28	silomat clobutinol hydrochloride	Oral	/	- Dry cough -

29	silomat clobutinol hydrochloride	Oral	/ Irrigation solution	20030101-20030103 Dry cough 3 Day
30	silomat clobutinol hydrochloride	Oral	/	20030101-20030103 Dry cough 3 Day
31	TRAMAL tramadol hydrochloride	Oral	/	20021213-20021213 Pain 1440 Minute
32	VOLTAREN DISPERS diclofenac sodium		/ Dispersible tablet	20021214-20021215 Joint pain 2 Day
33	VOLTAREN DISPERS diclofenac sodium		/ Dispersible tablet	20021217- Joint pain -

Drugs (concomitant)

34	alt-insulin insulin	Subcutaneous	16 lu international unit(s)/ Solution for injection	20021220-20030101 Diabetes mellitus 13 Day
35	ANTRA omeprazole	Intravenous (not otherwis /	Solution for injection	20021214-20021219 Prophylactic 6 Day
36	AUGMENTAN amoxicillin trihydrate; clavulanic acid	Intravenous (not otherwis 4.4 G gram(s)/Day	Solution for injection	20021213-20021216 Sepsis 4 Day
37	BELOC metoprolol tartrate	Intravenous (not otherwis 10 Mg milligram(s)/	Solution for injection	20021217-20021217 Tachycardia -
38	BELOC ZOK metoprolol succinate	Oral	/	20021218-20030109 Hypertension 23 Day
39	BENURON paracetamol	Rectal	/	20021214-20021214 Fever -
40	BIFITERAL SIRUP lactulose	Oral	/	20030104-20030104 Constipation -
41	decortin prednisolone	Oral	/	20021021-20021213 Connective tissue disorder 54 Day
42	Diazepam diazepam	Rectal	5 Mg milligram(s)/ Suppository	20021212-20021212 Agitation -
43	Elektrolytloesung amitriptyline hydrochloride	Intravenous drip	/	20021214-20021215 Fluid replacement NOS 2 Day
44	gentamicin gentamicin	Intravenous (not otherwis 240 Mg milligram(s)/	Solution for injection	20021220-20021223 Infection 4 Day
45	Insulin human insulin human	Intravenous (not otherwis 50 lu international unit(s)/	Solution for injection	20021219-20021219 Diabetes mellitus -
46	JONOSTERIL calcium chloride dihydrate; glucose; potassium chloride; sodium chloride	Intravenous drip	2 l litre(s)/ Solution for infusion	20021218-20021218 Fluid replacement NOS -
47	LACRIMAL polyvinyl alcohol	Other	12 Gtt drop(s)/Day Eye drops	20021016-20021214 Eye dryness 60 Day
48	LASIX furosemide	Oral	500 Mg milligram(s)/	20021221-20021221 Edema -
49	LASIX furosemide	Oral	/	20021221-20021221 Edema 1440 Minute
50	lipofundin glycerol; lecithin; soya oil	Intravenous drip	0.9 l litre(s)/ Solution for infusion	20021216-20021216 Parenteral nutrition -
51	Loesung zur parenteralen Ernaehrung	Intravenous drip	/	20021214-20030101 Parenteral nutrition 19 Day
52	nacl sodium chloride	Intravenous drip	250 ml millilitre(s)/ Solution for infusion	20021220-20021223 Therapeutic procedure 4 Day
53	NORVASC amlodipine	Oral	/	20021001-20021214 Hypertension 75 Day
54	PANTOZOL pantoprazole sodium	Oral	/	20021016-20021214 Prophylactic 60 Day
55	Paracetamol paracetamol	Rectal	1 G gram(s)/ Suppository	20021214-20021215 Fever 2 Day
56	PCM paracetamol	Oral	1 G gram(s)/ Effervescent tablet	20021219-20021219 Fever -
57	pepsin pepsin	Oral	/	20021210-20021214 Gastrointestinal disorder 5 Day
58	PERFALGAN paracetamol	Intravenous (not otherwis 1 G gram(s)/	Solution for injection	20021216-20021216 Fever -
59	pflanzliche Zubereitung	Oral	/	20030102-20030105 Sleep disorder 4 Day
60	TAMBOCOR flecainide acetate	Intravenous (not otherwis /	Solution for injection	20021218-20021218 Atrial fibrillation -

Drugs (concomitant)

61	tavegil clemastine	Oral	/	20030106-20030109	Exanthema 4 Day
62	tim-ophtal timolol maleate	Other	/ Eye drops	20021216-20030109	Cataract 25 Day
63	VITALIPIDE ergocalciferol; phytomenadione; retinol palmitate; tocopherol	Intravenous drip	/ Solution for infusion	20021216-20021217	Vitamin supplementation 2 Day
64	Vitamine nmb vitamins nos	Intravenous (not otherwis	/ Solution for infusion	20021219-20021230	Vitamin supplementation 12 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Tachycardia			k.A.	tachycardia
Gastric disorder			k.A.	stomach disorder
Hypertension			Unknown	
Stomach discomfort			Unknown	
Atrial fibrillation			Unknown	
Diabetes mellitus			Unknown	
Sharp's syndrome			Unknown	Suspected sharp syndrome
Cataract			Unknown	
Eye dryness			Unknown	
Pain			Unknown	
Collagenosis			Unknown	of mixed collagenosis (differential diagnosis Lupus, with increased ANA titer 1:3200)
ANA increased			Unknown	
Staphylococcal sepsis			Unknown	staphylococcus sepsis
Agitation			Unknown	
Lupus erythematosus			k.A.	Concomitant disease
Constipation			k.A.	constipation
Dry cough			k.A.	dry cough
Edema NOS			k.A.	edema
Exanthema			k.A.	exanthema
Fever			k.A.	fever
Hypertensive crisis			k.A.	hypertensive crisis
Infection			k.A.	suspected infection
Mixed connective tissue disease			k.A.	suspected sharp syndrome
Sepsis			k.A.	suspected sepsis
Sleep disorder			k.A.	sleeping disorder

Primary source

Qualification: Physician observestudytype: Other studies
Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 18.01.2008 DE-002147023-PHNU2008DE00540

Sender diagnosis: not reported

Narrative

Initial report received on 14 Jan 2008 (report from Intensive Monitoring Project, Schäpf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9140003). This multimorbid and polymedicated patient had a history of staphylococcal sepsis and other complications in early Dec 2002. Hospitalizations from 16 Oct 2002 until 15 Nov 2002, 8 Nov 2002 until 6 Dec 2002, and 8 Dec 2002 until 11 Jan 2003 were recorded (but not further described or commented). Voltaren Dispers (diclofenac) was used in dosages between thrice daily and once daily on 15 Dec 2002 and on a daily basis since 17 Dec 2002, for joint pain. The patient had received multiple drugs prior and during the following weeks, including parenteral nutrition, volume substitution, vitamin substitution, stomach protection etc. On 16 Dec 2002, patient showed fever, the diagnosis of sepsis was made, and patient was set on Ciprobay (ciprofloxacin) therapy. Tachycardia on 17 Dec 2002 led to Beloc (atenolol) administration. Subsequently, the patient

received diabetes mellitus medication, thrombosis prophylaxis and (later) vitamin substitution and stomach protection. Due to hypertensive crisis episodes on 19 Dec 2002, 30 Dec 2002, and 2 Jan 2003, respectively, patient was treated with Bayotensin (nitrendipin). A three day period from 20 until 22 Dec 2002 of i.v. gentamicin therapy was due to a suspected infection (which was not further commented). On 21 Dec 2002, edemas occurred and were treated with Lasix (furosemide). Constipation was noted on 27 Dec 2002 and 4 Jan 2003, respectively, and Bifiteral (lactulose) was used as a drug. On 30 Dec 2002, a colon contrast medium was administered, but the procedure was not further described. Novalgin (metamizol) was given on 30 and 31 Dec 2002 and 7 and 8 Jan 2003, respectively, for pain. Dry cough, sleeping disorder, and exanthema were noted during the first 2 weeks of Jan 2003 and treated accordingly. On 5 Jan 2003, generalized spots were noted. The first vesicle was seen on 9 Jan 2003. By 11 Jan 2003, skin pains and erosive mucosae were noted. Fever up to 39.0 degrees Celsius occurred on 12 Jan 2003. The further course was not further described. The patient died on 1 Feb 2003 (event duration was recorded as 1 Feb 2003); as adverse event, Toxic Epidermal necrolysis with maculae was documented. Cause of death and further diagnoses were not reported. As suspect drugs, the following were recorded: Voltaren Dispers (very possibly related), Altinsulin (insuline), Nitrolingual Spray (nitroglycerin), Ciprobay (ciprofloxacin), Insuman rapid (insulin), Clexane (enoxaparin-sodium), Bayotensin akut (nitrendipin), Lantus (human insulin), Nexium (esomeprazol), contrast medium (NOS), Novalgin (metamizol), Silomat (clobutinol), and Tramal (tramadol). As for event causality, the expert from the skin reaction center noted a suspicion of mixed collagenosis (differential diagnosis Lupus, with increased ANA titer 1:3200) as other causal factor.

Sender's comment:

Serious adverse event report [medically significant] "sepsis and tachycardia" assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs, therapeutic interventions, underlying conditions, etc.] provide a possible explanation for the reported adverse event.

Serious adverse event [death] "toxic epidermal necrolysis" assessed as listed according to the Core Data Sheet.

Serious adverse event [death] "hypertensive crisis and tachycardia" assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected. Reporting healthcare professional causality is suspected.

All other events were assessed as non-serious.

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 25.02.2008 DE-AVENTIS-200810454DE

Sender diagnosis: not reported

Narrative

Initial report

This case of toxic epidermal necrolysis (TEN) with maculae was received from the German health authority (DE-BFARM-08008088). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9140003). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of TEN was made by an expert panel who assessed it as a "definite case" of TEN. Causal relationship was assessed by the experts in June 2003 using an algorithm.

As for metamizole (Novalgin) the causality was assessed as "well possible", for human insulin (Insuman rapid), insulin glargine (Lantus) and enoxaparin (Clexane) it was assessed as "possible". For the other suspected drugs causality was assessed as :

"well possible": diclofenac, ciprofloxacin

"possible": losartan + hydrochlorothiazide, glycerol nitrate, nitrendipine, esomeprazole, contrast media, clobutinol

Not suspected were: diazepam, amoxicillin + clavulanic acid, amlodipine, pepsin, electrolyte solution, flecainid, pantoprazole, heparin sodium, insulin, metoprolol, prednisolone, timolol, polyvinyl alcohol, tramadol, paracetamol, omeprazole, nutritional solution, soya oil, propacetamol, vitamins, human insulin, gentamicin, sodium chloride, furosemide, lactulose, valerian root + hops cone, clemastine.

"suspected mixed collagenosis / differential diagnosis: lupus (ANA 1:3200)" were considered possible causes of TEN; infection was not suspected.

Epicrisis:

Patient: female, 66 years

Medical history: hypertension with hypertensive crises, atrial fibrillation, diabetes mellitus, grey cataract, suspicion of SHARP syndrome (suspicion of mixed collagenosis / differential diagnosis: lupus), suspected sepsis / staphylococcus sepsis.

From 16-Oct-2002 to 06-Dec-2002 hospitalised on the internal ward. Re-admitted to internal ward on 08-Dec-2002.

On 29-Oct-2002 the patient was diagnosed to suffer from staphylococcus sepsis.

On 17-Dec-2002, from 26- to 28-Dec-2002 and on 30-Dec-2002 the patient was treated with human insulin (Insuman rapid) for diabetes (dosages: 4 IU, 16IU, 6 IU, and 8 IU, respectively), from 18-Dec-2002 to at least 09-Jan-2003 she received applications of enoxaparin as prophylaxis of thrombosis (dosages 2x80mg and 1x40mg, respectively); from 19-Dec-2002 to at least 09-Jan-2003 she received injections of insulin glargine for diabetes mellitus (dosages: 12 IU, 14 IU, and 8 IU, respectively), and from 30-Dec-2002 to 01-Jan-2003 and from 07-Jan-2003 to at least 09-Jan-2003 she was treated with metamizole sodium for pain (dosage: 30 drops). Before and during hospitalisation the patient was also treated with a lot of other drugs.

On 05-Jan-2003 the patient developed generalised maculae.

On 09-Jan-2003, the first skin blisters were detected.

On 11-Jan-2003 the patient complained of skin pain, Nikolsky's sign was positive, erosions of the oral mucosa were detected. She was obviously transferred to a different ward (date of discharge from internal ward).

The outcome was reported as "death", date of end of event was reported as "01-Feb-2003".

Laboratory: no values provided.

Sender's comment:

21-FEB-2008 Sanofi-Aventis Company Comment: Based only on the compatible temporal relationship the causal role of the suspected company drugs (metamizole sodium, insulin human, insulin glargine and enoxaparin-sodium) cannot be excluded. However, the intake of numerous other drugs as well as the patient's underlying connective tissue disease may be discussed as possible alternative causes of the reported reaction.

19-FEB-2008 Sanofi Aventis Germany: TOXIC EPIDERMAL NECROLYSIS - FATAL is considered locally unlabelled for metamizole sodium (due to fatal outcome) as well as for insulin human, insulin glargine and enoxaparin-sodium. The facts presented here however are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the products. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

Bayer Vital GmbH, 29.02.2008 DE-BAYER-200813479GPV

Sender diagnosis: not reported

Narrative

This post-marketing study report was received on 18-Feb-2008 by BFARM, the regulatory authority of Germany via a physician, referring to a 66 year-old female.

This is a solicited report from the Documentation Center for Severe Skin Reaction in Germany.

Patient had concomitant conditions of agitation, suspected sepsis, hypertension, stomach disorder, atrial fibrillation, diabetes mellitus, suspected sharp syndrome, grey cataract, eye dryness, fever, staphylococcus bacteria, tachycardia, hypertensive crisis, suspected infection, edema, constipation, pain NOS, dry cough, sleeping disorder, and exanthema. Concomitant drugs have been given. For details see section "Concomitant Drugs" at the end of this report.

Patient received:

- Ciprobay for sepsis, Therapy dates: 16-DEC-2007 - 30-DEC-2007 with two different intravenous dosages, Formulation: 200 mg and 400 mg intravenous
- Lozaar plus for hypertension, Therapy dates: 21-Oct-2002 - 13-Dec-2002 and 02-Jan-2003 - Unknown, Formulation: orally
- Nitrolingual-Spray for hypertensive crisis, Therapy dates: 14-Dec-2002, 19-Dec-2002 and on 30-Dec-2002, Formulation: 2 puffs Respiratory (inhalation) and 4 puffs Respiratory (inhalation on 30-Dec-2002)
- Voltaren dispers for joint pain, Therapy dates: 14-Dec-2002 and on 17-Dec-2002 till unknown, Formulation: orally
- Insuman rapid for diabetes mellitus, Therapy dates: 17-Dec-2002, 26-Dec-2002 to 27-Dec-2002, 30-Dec-2002, Formulation: 4 - 16 IE subcutaneous injection
- Clexane for thrombosis prophylaxis, Therapy dates: 18-Dec-2002 to unknown, Formulation: 40 - 160 mg subcutaneous injection
- Bayotensin for hypertensive crisis, Therapy dates: 19-Dec-2002, 29-Dec-2002 to 30-Dec-2002, 03-Jan-2003, Formulation: 1 - 2 ml orally
- Lantus for diabetes mellitus, Therapy dates: 19-Dec-2002 to unknown, Formulation: 8 - 14 IE subcutaneous injection
- Nexium for ulcer protection, Therapy dates: 19-Dec-2002 to unknown, Formulation: 80 mg orally
- contrast agent (NOS) for colon contrast enema, Therapy date: 30-Dec-2002, Formulation: unknown
- Novalgin for pain, Therapy dates: 30-Dec-2002 to 31-Dec-2002, 07-Jan-2003 to unknown, Formulation: 30 drops orally
- Silomat for dry cough, Therapy dates: 01-Jan-2003 to 03-Jan-2003, Formulation: 20 - 40 guttae orally

The patient received in-patient treatment in the internal unit of a hospital between 16-Oct-2002 and 11-Jan-2003.

On 05-Jan-2003, 17 days after starting treatment with Bayotensin, 10 weeks and 6 days after starting treatment with Lorzaar plus, 17 days after starting treatment with Nitrolingual-Spray, 22 days after treatment with Voltaren dispers, 19 days after starting treatment with Insuman rapid, 18 days after starting treatment with Clexane, 17 days after starting treatment with Lantus, 17 days after starting treatment with Nexium, 6 days after treatment with contrast agent NOS, 6 days after starting treatment with Novalgin, and 4 days after starting treatment with Silomat patient experienced FATAL TOXIC EPIDERMAL NECROLYSIS (generalized maculae, skin blister, skin pain, Nikolsky's sign, oral mucosa erosion, fever up to 39 °C) considered

serious due to death. Patient had not recovered, follow-up not necessary.

Patient died on an unknown date. Autopsy information is unknown. The patient died from TOXIC EPIDERMAL NECROLYSIS as stated by the reporter.

For the drugs Ciprobay and Bayotensin the assessment was possibly related.

Sender's comment:

The reported event TOXIC EPIDERMAL NECROLYSIS (fatal) is listed in the international product information for Ciprobay (ciprofloxacin) and unlisted for Bayotensin (nitrendipin) and considered serious due to death. Based on the information received for this case an association of Ciprobay and of Bayotensin with the reported event cannot be excluded due to a compatible temporal relationship (both given within the time range of 3 weeks prior the event). According to the source document the patient was suffering from staphylococci sepsis and several concomitant diseases, which required multi drug treatment and therefore makes it impossible to identify the most suspected drug.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08008191

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 80 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20031110	20031207	28 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 fenistil		/	20031006-	
dimetindene maleate			-	
2 FUROSEMIDE		120 Mg milligram(s)/Day	20031014-	
furosemide			-	
3 gelonida	Oral	1 Df dosage form/Day	-	
acetylsalicylic acid; codeine phosphate; paracetamol			-	
4 PARACETAMOL		500 Mg milligram(s)/Day	-	
paracetamol			-	
5 PHENYTOIN		0.1 Mg milligram(s)/Day	-	
phenytoin			-	
6 RANITIDINE		300 Mg milligram(s)/Day	20031014-	
ranitidine			-	
7 voltaren emulgel	Topical	1 Df dosage form/Day	-	
diclofenac diethylamine		Gel	-	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.01.2008 DE-NOVNCHHQ-PHNU2008DE00541

Sender diagnosis: not reported

Narrative

Initial report received on 14 Jan 2008 (report from Intensive Monitoring Project, Schäpf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9150097). This patient was treated with several drugs during summer 2003 and autumn 2003, for unstated reasons. Voltaren Emulgel (diclofenac) was used once daily, but therapy duration was not given. Fenistil (dimetinden-maleate) was started on 6 Oct 2003 (indication and therapy duration unknown). On 10 Nov 2003, patient suffered from Stevens Johnson syndrome. The event lasted until 7 Dec 2003, and the patient made a complete recovery. As suspect drugs, Voltaren Emulgel, Fenistil, furosemid, ranitidine, paracetamol, Gelonida (paracetamol, codeinphosphate), and phenytoin were mentioned. No further information with respect to other causative factors could be given.

Sender's comment:

Serious adverse event report [medically significant] "Stevens-Johnson syndrome" assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs: co-suspect] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

This is a serious adverse event (medically significant hazard). The event Steven-Johnson syndrome was assessed as unlabelled according to the product information. However other alternative cause (concomitant medications) provides a possible explanation for the reported adverse event. Reporting healthcare professional causality was suspected.

Reporter's comment:

NOVARTIS, 22.01.2008 DE-002147023-PHNU2008DE00541

Sender diagnosis: not reported

Narrative

Initial report received on 14 Jan 2008 (report from Intensive Monitoring Project, SchÃ¶pf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9150097). This patient was treated with several drugs during summer 2003 and autumn 2003, for unstated reasons. Voltaren Emulgel (diclofenac) was used once daily, but therapy duration was not given. Fenistil (dimetinden-maleate) was started on 6 Oct 2003 (indication and therapy duration unknown). On 10 Nov 2003, patient suffered from Stevens Johnson syndrome. The event lasted until 7 Dec 2003, and the patient made a complete recovery. As suspect drugs, Voltaren Emulgel, Fenistil, furosemid, ranitidine, paracetamol, Gelonida (paracetamol, codeinphosphate), and phenytoin were mentioned. No further information with respect to other causative factors could be given.

Sender's comment:

Serious adverse event report [medically significant] "Stevens-Johnson syndrome" assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs: co-suspect] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

This is a serious adverse event (medically significant hazard). The event Steven-Johnson syndrome was assessed as unlabelled according to the product information. However other alternative cause (concomitant medications) provides a possible explanation for the reported adverse event. Reporting healthcare professional causality was suspected.

Reporter's comment:

NOVARTIS, 24.01.2008 DE-SZ09-PHNU2008DE00541

Sender diagnosis: not reported

Narrative

Initial report received on 14 Jan 2008 (report from Intensive Monitoring Project, SchÃ¶pf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9150097). This patient was treated with several drugs during summer 2003 and autumn 2003, for unstated reasons. Voltaren Emulgel (diclofenac) was used once daily, but therapy duration was not given. Fenistil (dimetinden-maleate) was started on 6 Oct 2003 (indication and therapy duration unknown). On 10 Nov 2003, patient suffered from Stevens Johnson syndrome. The event lasted until 7 Dec 2003, and the patient made a complete recovery. As suspect drugs, Voltaren Emulgel, Fenistil, furosemid, ranitidine, paracetamol, Gelonida (paracetamol, codeinphosphate), and phenytoin were mentioned. No further information with respect to other causative factors could be given.

Sender's comment:

Serious adverse event report [medically significant] "Stevens-Johnson syndrome" assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs: co-suspect] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

This is a serious adverse event (medically significant hazard). The event Steven-Johnson syndrome was assessed as unlabelled according to the product information. However other alternative cause (concomitant medications) provides a possible explanation for the reported adverse event. Reporting healthcare professional causality was suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08008772

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 66 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Headache	20030331			21 Day	not recovered/not resolved
2 Chills	20030330			20 Day	Unknown
3 Swelling face	20030331			21 Day	not recovered/not resolved
4 Swallowing difficult	20030331			21 Day	not recovered/not resolved
5 Itching	20030331			21 Day	not recovered/not resolved
6 Skin disorder	20030331			21 Day	not recovered/not resolved
7 Cystitis acute	20030318			8 Day	Unknown
8 Lip erosion	20030402			23 Day	not recovered/not resolved
9 Stevens-Johnson syndrome	20030331			21 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ass acetylsalicylic acid	Oral	100 Mg milligram(s)/	19980101-	Thrombosis prophylaxis
2 cotrimoxazol sulfamethoxazole; trimethoprim		/	20030324-20030329	Cystitis
3 ibuprofen ibuprofen	Oral	/	20000101-	Sciatica
4 OFLOX-AZU ofloxacin	Oral	100 Mg milligram(s)/Day	20030317-20030322	Urinary infection
5 voltaren dispers diclofenac sodium	Oral	/Day Dispersible tablet	20030311-	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			Unknown	
Ischial neuralgia			Unknown	
Sleep disorder			Unknown	
Bronchitis			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.01.2008 DE-002147023-PHNU2008DE00542

Sender diagnosis: not reported

Narrative

Initial report received on 14 Jan 2008 (report from Intensive Monitoring Project, Schäpf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9150022). This patient was treated with several drugs during 2003 and 2004. Voltaren Dispers (diclofenac) was used irregularly since 11 Mar 2003. On 18 Mar 2003 a acute cystitis was reported (treated withOflox-Azu from 17 Mar 2003 to 22 Mar 2003 and Cotrimazole from 24 Mar 2003 to 29 Mar 2003). On 30 Mar 2003 chills were reported and on 31 Mar 2003 the patient developed headache, difficulty swallowing, face swelling, itching and generalized targets and spots. On 02 Apr 2003 the patient was hospitalized with lip erosions. A diagnosis of Steven-Johnson-Syndrome was made. Outcome was reported as not yet recovered. Voltaren Dispers, Oflox-Azu and Cotrimazole were reported as suspected. Outcome was reported as not yet recovered.

Sender's comment:

Serious adverse event [medically significant] "Stevens-Johnson syndrome" assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Cystitis was assessed as not suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08007532

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 58 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20031124	deathcause: Stevens-Johnson syndrome
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Fever					Unknown
2 Blister					Unknown
3 Mucosal erosion					Unknown
4 Lip erosion					Unknown
5 Stevens-Johnson syndrome	20031031				fatal
6 Erythema diffuse					Unknown
7 Burn					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	addel chromic chloride; cupric chloride dihydrate; ferric chloride hexahydrate; manganese chloride tetrahydrate; potassium iodide; sodium fluoride; sodium molybdate; sodium selenite; zinc chloride	Intravenous (not otherwis /	-	-	
2	aminofusin amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis /	-	-	
3	bepanthen dexpanthenol	Unknown /	-	-	Drug use for unknown indication
4	carvedilol carvedilol	Oral /	-	-	Cardiac insufficiency
5	ciprobay ciprofloxacin hydrochloride	Intravenous (not otherwis 400 Mg milligram(s)/ Injection	-	-	Infection
6	ciprobay ciprofloxacin hydrochloride	Oral 800 Mg milligram(s)/Day Solution for injection	20031026-20031028	3 Day	Infection NOS
7	clont metronidazole	Intravenous (not otherwis 500 Mg milligram(s)/	-	-	Infection
8	colchicine colchicine	Oral /	-	-	Pericardial effusion
9	corneregel dexpanthenol	Ophthalmic /	-	-	
10	dexpanthenol dexpanthenol	Ophthalmic /	-	-	
11	diclofenac diclofenac	Oral /	-	-	Pain
12	dipidolor piritramide	Subcutaneous / Injection	20031029-	-	Pain
13	dopamin dopamine hydrochloride	Intravenous (not otherwis /	-	-	Unspecified circulatory system disorder
14	fortimel nutrients nos	Intravenous (not otherwis /	-	-	Parenteral nutrition
15	gelafundin calcium chloride dihydrate; polygelatine succinate; sodium chloride	Intravenous (not otherwis 500 ml millilitre(s)/	-	-	
16	glucose glucose	Intravenous (not otherwis /	20031026-	-	
17	haes hetastarch; sodium chloride	Unknown 500 ml millilitre(s)/	-	-	
18	humanalbumin albumin human	Unknown 100 ml millilitre(s)/	-	-	
19	insulin insulin	Subcutaneous / Injection	20031003-	-	Diabetes mellitus
20	KLACID FORTE clarithromycin	Oral 1000 Mg milligram(s)/Day	20031015-	-	Infection
21	lactulose lactulose	Oral 20 ml millilitre(s)/ Oral solution	-	-	Liver failure
22	lipoveno glycerol; lecithin; soya oil	Intravenous (not otherwis 250 ml millilitre(s)/	-	-	Parenteral nutrition
23	liquemin heparin	Subcutaneous / Injection	20031003-	-	Thrombosis prophylaxis
24	pantozol pantoprazole sodium	Oral 40 Mg milligram(s)/	-	-	Prophylaxis
25	refobacin gentamicin sulfate	Unknown /	-	-	Drug use for unknown indication
26	sab simplex dimeticone	Oral 20 ml millilitre(s)/	-	-	
27	solu-decortin prednisolone sodium succinate	Intravenous (not otherwis 500 Mg milligram(s)/	-	-	

			-	
28	soluvit	Intravenous (not otherwis /	-	
	vitamins nos		-	
29	tocopherol	Intravenous (not otherwis /	-	Parenteral nutrition
	tocopherol		-	
30	vancomycin	Intravenous (not otherwis 1 G gram(s)/	-	Inflammation
	vancomycin		-	
31	vioxx	Oral 25 Mg milligram(s)/	-	Pain
	rofecoxib		-	
32	vitalipid	Intravenous (not otherwis /	-	Parenteral nutrition
	vitamins nos		-	
33	zienam	Oral 500 Mg milligram(s)/	-	
	cilastatin sodium; imipenem		-	

Drugs (concomitant)

34	adrenalin epinephrine	Subcutaneous	/	20031003-	
			Injection	-	
35	amino acids nos amino acids nos		/	-	
				-	
36	aminofusin hepar acetylcysteine; amino acids nos; magnesium chloride anhydrous; potassium chloride; sodium glycerophosphate; sorbitol	Oral	1000 ml millilitre(s)/	-	
			Oral solution	-	
37	amphotericin b amphotericin b	Respiratory (inhalation)	10 Mg milligram(s)/	-	Antifungal prophylaxis
				-	
38	ampicillin ampicillin	Intravenous (not otherwis	2 G gram(s)/	-	
				-	
39	anexate flumazenil	Subcutaneous	.5 Mg milligram(s)/	-	
			Injection	-	
40	clinimix amino acids nos		1000 ml millilitre(s)/	-	Parenteral nutrition
				-	
41	cordarex amiodarone hydrochloride	Intravenous (not otherwis	/	20031003-	Cardiac insufficiency
			Injection	-	
42	cordarex amiodarone hydrochloride	Oral	200 Mg milligram(s)/	-	Cardiac insufficiency
				-	
43	corotrop milrinone	Intravenous (not otherwis	/	20031003-	Cardiac insufficiency
			Injection	-	
44	disoprivan propofol	Intravenous (not otherwis	/	20031003-	Sedation
			Injection	-	
45	elzogram cefazolin sodium	Intravenous (not otherwis	2 G gram(s)/	20031002-	Prophylaxis
			Injection	-	
46	esmeron rocuronium bromide	Intravenous (not otherwis	50 Mg milligram(s)/	20031003-	
			Injection	-	
47	fluimucil acetylcysteine	Intravenous (not otherwis	300 Mg milligram(s)/	20031002-	Reaction unevaluable
			Injection	-	
48	fluimucil acetylcysteine	Respiratory (inhalation)	150 Mg milligram(s)/	-	Reaction unevaluable
				-	
49	haldol haloperidol	Intravenous (not otherwis	/	-	Restlessness
				-	
50	magnesium sulfate magnesium sulfate	Intravenous (not otherwis	50 ml millilitre(s)/	20031002-	
				-	
51	nitroglycerin glyceryl trinitrate	Intravenous (not otherwis	/	20031002-	Cardiac insufficiency
			Injection	-	
52	noradrenaline norepinephrine	Intravenous (not otherwis	/	20031011-	
			Injection	-	
53	pantozol pantoprazole sodium	Intravenous (not otherwis	40 Mg milligram(s)/	20031003-	Prophylaxis
			Injection	-	
54	paspertin metoclopramide hydrochloride	Subcutaneous	20 Mg milligram(s)/	-	
			Injection	-	
55	pk-merz amantadine sulfate	Oral	250 ml millilitre(s)/	-	Psychotic disorder
				-	
56	ppsb factor ii (prothrombin); factor ix complex human; factor vii (proconvertin); factor x (stuart prower factor)	Intravenous (not otherwis	1000 lu international unit(s)	20031003-	Antifibrinolysis
				-	
57	prostigmin neostigmine bromide		/	-	
				-	
58	protamin protamine hydrochloride	Intravenous (not otherwis	5000 lu international unit(s)	20031003-	
			Injection	-	
59	tavor lorazepam	Intravenous (not otherwis	1 Mg milligram(s)/	-	Restlessness

Drugs (concomitant)

60	TAZOBAC	Intravenous (not otherwise)	4.5 G gram(s)/	-	-	Intestinal operation
	piperacillin sodium; tazobactam sodium		Injection	-		
61	ulcogant	Oral	/	20031002-	-	Prophylaxis
	sucralfate			-		

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain			Unknown	
Liver failure			Unknown	
Intestinal operation			No	
Inflammation			Unknown	
Infection			Unknown	
Pericardial effusion			Unknown	
Diabetes mellitus			Unknown	
Thrombosis prophylaxis			Unknown	
Cardiac insufficiency			Unknown	
Prophylaxis			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

ROCHE, 24.01.2008 DE-ROCHE-541285

Sender diagnosis: not reported

Narrative

REGISCAR STUDY (STUDY TITLE UNKNOWN)

A 58-YEAR-OLD MALE PATIENT DEVELOPED FATAL STEVENS-JOHNSON SYNDROME DURING PARTICIPATION IN THE ABOVE STUDY.

THE PATIENT WAS TAKING MULTIPLE CONCOMITANT MEDICATIONS. HIS MEDICAL HISTORY INCLUDED CARDIAC INSUFFICIENCY, STOMACH PROTECTION, INTESTINAL RESTORATION, THROMBOSIS PROPHYLAXIS, DIABETES MELLITUS, PAIN, LIVER FAILURE, INFECTION, INFLAMMATION, AND PERICARDIAL EFFUSION. HE WAS ALSO HOSPITALISED ON MANY OCCASIONS FOR CARDIOLOGY, SURGERY, THORAX SURGERY, AND INTERNAL MEDICINE.

ON 03 OCTOBER 2003, THE PATIENT COMMENCED INTRAVENOUS HEPARIN SODIUM (LIQUEMIN), DOSAGE AND FREQUENCY NOT REPORTED. ON AN UNKNOWN DATE, THE PATIENT STARTED TREATMENT WITH CARVEDILOL (DILATREND), 3.12 MG 1-0-1, 6.25MG 1-0-1. ON 31 OCTOBER 2003, THE PATIENT EXPERIENCED DIFFUSE ERYTHEMA, AND THE FIRST BLISTER ON THE SKIN APPEARED. THE SAME DAY, HE WAS HOSPITALISED IN THE INTENSIVE CARE UNIT. THE NEXT DAY, HE DEVELOPED A FEVER. ON 02 NOVEMBER 2003, THE PATIENT WAS TRANSFERRED TO THE CENTRE FOR PATIENTS WITH BURNS, AND ON THIS DAY HE HAD LIP AND ORAL EROSIONS. HE WAS DISCHARGED FROM THE CENTRE ON 19 NOVEMBER 2003. ON AN UNKNOWN DATE, THE PATIENT DIED FROM STEVENS-JOHNSON SYNDROME. IT WAS NOT REPORTED WHETHER AN AUTOPSY WAS PERFORMED. IT WAS NOT REPORTED WHETHER TREATMENT WITH HEPARIN SODIUM AND CARVEDILOL WAS ONGOING AT THE TIME OF THE EVENT.

THE INVESTIGATOR ASSESSED THE EVENT OF FATAL STEVENS-JOHNSON SYNDROME AS POSSIBLE RELATED TO TREATMENT WITH HEPARIN SODIUM AND CARVEDILOL. THE INVESTIGATOR ALSO PROVIDED A CAUSAL RELATIONSHIP WITH OTHER SUSPECTED DRUGS WITH THE EVENT WHICH INCLUDED INSULIN, PIRITRAMIDE (DIPIDOLOR), GLUCOSE, SIMETICONE (SAB SIMPLEX), LACTULOSE, CIPROFLOXACIN (CIPROBAY), GENTAMICIN (REFOBACIN), DEXPANTHENOL (BEPANTHEN), CLARITHROMYCIN (KLACID), METRONIDAZOLE (CLONT), DICLOFENAC, VANCOMYCIN, CILASTATIN/IMIPENEM (ZIENAM), AMINOFUSION, COLCHICINE, DEXPANTHENOL (CORNEREDEL), PREDNISONE HEMISUCCINATE (SOLU-DECORTIN), ROFECOXIB (VIOXX), FORTIMEL, GELAFUNDIN, NUTRIA-TWIN, ALBUMIN HUMAN, HETASTARCH (HAES), DOPAMINE, GLYCINE MAX (LIPOVENOES), ADDEL, SOLUVIT, PANTOPRAZOLE (PANTOZOL) AND VITALIPID.

NO FURTHER INFORMATION WAS PROVIDED.

Sender's comment:

CIOMS TEXT: FATAL STEVENS-JOHNSON SYNDROME IS NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (CDS) FOR

CARVEDILOL, AND IS NOT LISTED IN THE SWISS PRESCRIBING INFORMATION (PI) FOR HEPARIN SODIUM. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN THE REPORTED EVENT AND TREATMENT WITH HEPARIN SODIUM AND CARVEDILOL IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM. HOWEVER POSSIBLE ALTERNATIVE EXPLANATIONS INCLUDE THE CONCOMITANT MEDICATIONS OF CIPROFLOXACIN, CLARITHROMYCIN, DICLOFENAC, AND VANCOMYCIN.; ***LOCAL ASSESSMENT: DZH-INTERVIEW-NR.: 9140087

FATAL STEVENS-JOHNSON SYNDROME = NOT LABELED FOR CARVEDILOL AND FOR HEPARIN SODIUM
CIPROFLOXACIN, CLARITHROMYCIN, DICLOFENAC, AND VANCOMYCIN = RELATED

EINZELFALLBEOBACHTUNG. KEIN HANDLUNGSBEDARF.

Reporter's comment:

009507513, 28.01.2008 DE-MERCK-0801DEU00097

Sender diagnosis: not reported

Narrative

Information has been received from a registry, documenting at least 95% of all serious skin reactions that occur in Germany, concerning a 58-year-old hospitalized male patient with diabetes mellitus, pain, infection, constipation, congestive cardiac failure congestive, exophthalmos, other disorders of intestine, pericardial effusion and hyposphagma who, on 17-OCT-2003, was placed on therapy with imipenem/cilastatin sodium, 500-500 mg injection (form), 500-500 mg, three times a day (indication not reported).

Suspect secondary therapy included: carvedilol (DILATREND), tablet, various dosages for the treatment of congestive heart failure (start 12-OCT-2003), ciprofloxacin hydrochloride (CIPROBAY), injection, 400 mg twice a day iv for the treatment of infection (04-OCT-2003 - 17-OCT-2003 and 26-OCT-2003 - 28-OCT-2003), clarithromycin (KLACID), 500 mg twice a day po, for the treatment of infection (start 15-OCT-2003), colchicine, po, for the treatment of pericardial infusion (dose not reported; 17-OCT-2003 - 18-OCT-2003 and 20-OCT-2003 - 29-OCT-2003), diclofenac, 1 unit twice a day for the treatment of pain (17-OCT-2003 - 18-OCT-2003), heparin, injection as thrombosis prophylaxis (dose not reported; start 03-OCT-2003), neomycin, 9-12 gram daily po for the treatment of unspecified intestinal disorder (start 11-OCT-2003), pantoprazole sodium (PANTOZOL), 40 mg tablet, 40 mg twice a day as gastric ulcer prophylaxis (start 16-OCT-2003), simethicone (SAB SIMPLEX), liquid, 20 mL three times a day po, for the treatment of constipation (start 18-OCT-2003) and vancomycin iv (dose and indication not reported) (17-OCT-2003 - 22-OCT-2003).

Other prior and concomitant therapy included acetylcysteine (FLUIMUCIL), insulin, piritramide (DIPIDOLOR), dextrose, lactulose, gentamicinSO4 (REFOBACIN), dexpanthenol (BEPANTHEN), metronidazole (CLONT), amino acids (unspecified) (+) dextrose, dexpanthenol (CORNEREDEL), prednisolone sodium succinate (SOLU-DECORTIN-H), rofecoxib (MSD), carbohydrates (unspecified) (+) fat (unspecified) (+) minerals (unspecified) (+) protein (unspecified) (+) vitamins (unspecified) (FORTIMEL), electrolytes (unspecified) (+) succinylated gelatin (GELAFUNDIN), amino acids (unspecified), albumin human, pentastarch (HAES-STERIL), dopamine HCl, soybean oil (LIPOVENOS), chromic chloride (+) cupric chloride (+) ferric chloride (+) manganese chloride (+) potassium iodide (+) sodium fluoride (+) sodium molybdate (+) sodium selenite (+) zinc chloride (ADDEL N), ascorbic acid (+) biotin (+) folic acid (+) vitamin B complex (SOLUVIT) and ergocalciferol (+) phytonadione (+) vitamin A (VITALIPID).

On 28-OCT-2003, therapy with imipenem/cilastatin sodium was finished.

On 31-OCT-2003, the patient developed diffuse erythema and first blister on his skin. That same day, he was transferred to the ICU. On 01-NOV-2003, the patient developed fever and, on 02-NOV-2003, mucosal erosions of lips and mouth, he was diagnosed with Stevens-Johnson Syndrome and transferred to a special ward for burn patients. On 24-NOV-2003, the patient died of Stevens-Johnson syndrome.

The reporting physician felt that Stevens-Johnson syndrome and the patient's subsequent death was possibly related to therapy with cilastatin sodium (+) imipenem and/or carvedilol (DILATREND) and/or ciprofloxacin hydrochloride (CIPROBAY) and/or clarithromycin (KLACID) and/or colchicine and/or diclofenac and/or heparin and/or neomycin and/or pantoprazole sodium (PANTOZOL) and/or simethicone (SAB SIMPLEX) and/or vancomycin iv.

Additional information is not expected.

Sender's comment:

Steven-Johnson syndrome is a serious labeled event for therapy with ZIENAM°. This case-report was forwarded to us by the German documentation center for severe skin reactions. A 58 year old male patient was diagnosed with Stevens-Johnson syndrome three days after ZIENAM° had been discontinued. This patient had received multiple concomitant medications, several of which have a comparable potential to ZIENAM° to be causally associated with SJS. Therefore, a causative role of ZIENAM° in the development of SJS cannot be determined. Self initiated measurements are not

indicated.

Reporter's comment:

GlaxoSmithKline, 04.02.2008 DE-GlaxoSmithKline-B0505721A

Sender diagnosis: not reported

Narrative

This 58 year old male subject was enrolled in a non-GSK study.

From an unspecified date, the subject received carvedilol. From 03 October 2003 the subject received heparin sodium.

Medical conditions at the time of the event included cardiac insufficiency, diabetes mellitus, gastric protection, infection, inflammation, intestinal operation, liver failure, pain, pericardial effusion and thrombosis prophylaxis. Concomitant medications included insulin, piritramide, dextrose, lactulose, simethicone, ciprofloxacin hydrochloride, gentamicin sulphate, Bepanthen, clarithromycin, metronidazole, diclofenac, vancomycin, Zienam, Aminofusin, colchicine, dexpanthenol, prednisolone sodium succinate, rofecoxib, Fortimel, Gelafundin, unknown, human albumin, pentastarch, dopamine, soybean oil, electrolytes, Soluvit, vitamin E and pantoprazole, protamine sulphate, rocuronium bromide, factor ix, cephalosin sodium, nitroglycerine, propofol, adrenaline, amiodarone, noradrenaline, pantoprazole, milrinone, sucralfate, magnesium sulfate, acetylcysteine, Tazobac, parenteral nutrition, amino acid, metoclopramide hydrochloride, neostigmine methylsulphate, amantadine, amphotericin, flumazenil, haloperidol, lorazepam and ampicillin trihydrate.

On 31 October 2003, the subject developed Stevens Johnson syndrome. He also experienced diffuse erythema, skin blister, fever, burn, lip erosion and oral erosion. The subject was hospitalised and was discharged on 19 November 2003. The subject died on an unknown date due to Stevens Johnson syndrome. It was unknown whether an autopsy was performed. It was unknown whether treatment with carvedilol and heparin sodium was ongoing at the time of the event. The investigator reported the Stevens Johnson syndrome as possibly related to treatment with carvedilol and heparin sodium and provided a causal relationship with other suspected drugs insulin, piritramide, dextrose, lactulose, simethicone, ciprofloxacin hydrochloride, gentamicin sulphate, Bepanthen, clarithromycin, metronidazole, diclofenac, vancomycin, Zienam, Aminofusin, colchicine, dexpanthenol, prednisolone sodium succinate, rofecoxib, Fortimel, Gelafundin, unknown, human albumin, pentastarch, dopamine, soybean oil, electrolytes, Soluvit, vitamin E and pantoprazole.

Sender's comment:

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

001307602, 25.02.2008 DE-ABBOTT-08P-062-0439153-00

Sender diagnosis: not reported

Narrative

Report from Germany of Stevens-Johnson syndrome, burn injury, and fever coincident with CLARITHROMYCIN (KLACID) therapy. On 02 Oct 2003, the patient began CEFAZOLIN SODIUM for post surgery prophylaxis, GLYCERYL TRINITRATE for heart failure, RINGER-LACTATE for fluid substitution, SUCRALFATE for stomach protection, MAGNESIUM SULFATE for substitution, and ACETYLCYSTEINE for secretolysis. On 03 Oct 2003, the patient began PANTOZOL therapy for stomach protection, PROPOFOL for sedation, GFP INFUSION and GLUCOSE as a substitution, EPINEPHRINE and NOREPINEPHRINE as an intraoperative, AMIODARONE HYDROCHLORIDE and MILRINONE for heart failure, INSULIN for diabetes, PIRITRAMIDE for pain, and HEPARIN SODIUM for thrombosis prophylaxis. On 03 Oct 2003, single doses of PROTAMINE as an intraoperative, ROCURONIUM BROMIDE for change of tubes, PROTHROMBINEX for antifibrinolysis, and ERYTHROCYTE CONCENTRATE as an intraoperative were given. On 04 Oct 2003, a single dose of ERYTHROCYTE CONCENTRATE was given. On 04 Oct 2003, the patient began CIPROFLOXACIN for an infection, PIP/TAZO for sanitation of intestine, and LACTULOSE for liver failure. On 04 Oct 2003, CEFAZOLIN SODIUM and GFP INFUSION were discontinued. On 05 Oct 2003, a single dose of MAGNESIUM BIOMED was given for substitution and PROPOFOL was discontinued. On 05 Oct 2003, the patient began CLINIMIX and AMINO-HEPAR for nutrition, METOCLOPRAMIDE HYDROCHLORIDE for emesis, and DIMETICONE for constipation. On 06 Oct 2003, a single dose of GFP INFUSION was given. On 07 Oct 2003, the patient began AMANTADINE SULFATE for transitional psychosis and AMPHOTERICIN B for thrush, and a single dose of NEOSTIGMINE BROMIDE was given for atonia of intestine. On 07 Oct 2003, the patient began FLUIMICIL INHALATION for secretolysis. On 07 Oct 2003, NOREPINEPHRINE was discontinued. On 08 Oct 2003, the patient began CARVEDILOL for heart failure and CLINIMIX was discontinued. On 09 Oct 2003, the patient began FENTAMICIN SULFATE and DEXPANTHENOL for exophthalmus. On 09 Oct 2003, a single dose of FLUMAZENIL was given for diagnosis and EPINEPHRINE was discontinued. On 10 Oct 2003, RINGER-LACTATE and CARVEDILOL were discontinued. On 11 Oct 2003, AMIODARONE HYDROCHLORIDE and AMINO-HEPAR were discontinued, and a single dose of NOREPINEPHRINE was given. On 12 Oct 2003, the patient began bland diet for nutrition and CARVEDILOL was restarted. On 14 Oct 2003, the patient began AMPICILLIN for an unknown indication. On 14 Oct 2003, single doses of HALOPERIDOL and LORAZEPAM were given for restlessness, and PANTOZOL therapy was discontinued. On 15 Oct 2003, the patient began KLACID therapy and METRONIDAZOLE for an infection. On 15 Oct 2003, MILRINONE, SUCRALFATE, MAGNESIUM SULFATE, PIRITRAMIDE, PIP/TAZO, METOCLOPRAMIDE HYDROCHLORIDE, DIMETICONE, AMPHOTERICIN B, bland diet, AMPICILLIN, GLUCOSE, and FLUIMICIL INHALATION were discontinued. On 16 Oct 2003, the patient began AMINOFUSIN HEPAR SINE for substitution. On 16 Oct 2003, PANTOZOL therapy was restarted and METRONIDAZOLE was discontinued. On 17 Oct 2003, CIPROFLOXACIN and ACETYLCYSTEINE were discontinued. On 17 Oct 2003, the patient began DICLOFENAC therapy for pain, COLCHICIN therapy for pericardial effusion, and VANCOMYCIN and PRIMAXIN for inflammation. On 18 Oct 2003, DICLOFENAC and COLCHICIN therapies were discontinued. On 18 Oct 2003, the patient began CORNEREGEL AUGENGEL for an eye hemorrhage and DIMETICONE was restarted. On 19 Oct 2003, a single dose of PREDNISOLONE SODIUM SUCCINATE for an unknown indication was given. On 19 Oct 2003, the patient began ROFECOXIB for pain. On 20 Oct 2003, COLCHICIN therapy was restarted and ROFECOXIB was discontinued. On 22 Oct 2003, VANCOMYCIN was discontinued. On 23 Oct 2003, INSULIN and AMINOFUSIN HEPAR SINE were discontinued. On 24 Oct 2003, GENTAMICIN SULFATE was discontinued. On 26 Oct 2003, CIPROFLOXACIN and GLUCOSE were restarted, and LACTULOSE was discontinued. On 28 Oct 2003, the patient began FORTIMEL for nutrition. On 28 Oct 2003, METRONIDAZOLE was restarted and CIPROFLOXACIN and VANCOMYCIN were discontinued. On 29 Oct 2003, COLCHICIN therapy and GLUCOSE were discontinued, and a single dose of PIRITRAMIDE was given. On 29 Oct 2003, the patient began NUTRI TWIN for nutrition and a single dose of GELAFUNDIN was given for substitution. On 30 Oct 2003, the patient began DOPAMINE HYDROCHLORIDE for stabilization of circulation, INTRALIPID and VITALIPID for nutrition, and ADDEL N and SOLUVIT for substitution. On 30 Oct 2003, a single dose of HUMAN ALBUMIN for nutrition and a single dose of HAES INFUSION 10% for substitution were given. On 31 Oct 2003, the patient experienced diffuse erythema and the first blister at the skin. On 01 Nov 2003, the patient developed fever. On 02 Nov 2003, the patient experienced erosions at the lips and at the oral mucosa. The patient was diagnosed with Stevens-Johnson syndrome. From 02 Nov 2003 to 19 Nov 2003, the patient was treated in the center for burn injury. In Nov 2003, PANTOZOL therapy, HEPARIN SODIUM, DIMETICONE, CARVEDILOL, DEXPANTHENOL, METRONIDAZOLE, FORTIMEL, DOPAMINE HYDROCHLORIDE, INTRALIPID, ADDEL N, and SOLUVIT, VITALIPID were discontinued. On an unreported date, the patient was treated again with NUTRI TWIN. On 24 Nov 2003, the patient died. Cause of death was Stevens-Johnson syndrome. No discharged summary was available. The Stevens-Johnson syndrome was considered life-threatening and involved hospitalization and the burn injury was considered medically significant. The reporting physician believed the Stevens-Johnson syndrome was possibly related to KLACID therapy, PANTOZOL, DICLOFENAC, COLCHICIN, CIPROFLOXACIN, HEPARIN SODIUM, DIMETICONE, CARVEDILOL, NEBACETIN, VANCOMYCIN, and PRIMAXIN. The reporting physician believed the Stevens-Johnson syndrome was not related to PROTAMINE HYDROCHLORIDE, ROCURONIUM BROMIDE, PROTHROMBINEX, CEFAZOLIN SODIUM, ERYTHROCYTE CONCENTRATE, GLYCERYL TRINITRATE, PROPOFOL, GFP INFUSION, EPINEPHRINE, RINGER-LACTATE, AMIODARONE HYDROCHLORIDE, NOREPINEPHRINE, MILRINONE, SUCRALFATE, MAGNESIUM SULFATE, ACETYLCYSTEINE, PIP/TAZO, MAGNESIUM BIOMED, CLINIMIX, AMINO-HEPAR, METOCLOPRAMIDE HYDROCHLORIDE, NEOSTIGMINE BROMIDE, AMANTADINE SULFATE, AMPHOTERICIN B, FLUMAZENIL, bland diet, HALOPERIDOL, LORAZEPAM, AMPICILLIN, and FLUIMICIL INHALATION. The reporting physician can not assess the causality between the Stevens-Johnson syndrome and INSULIN, PIRITRAMIDE, LACTULOSE, GENTAMICIN SULFATE, DEXPANTHENOL, METRONIDAZOLE, AMINOFUSIN HEPAR SINE, PREDNISOLONE SODIUM SUCCINATE, ROFECOXIB, FORTIMEL,

GELAFUNDIN, NUTRI TWIN, HUMAN ALBUMIN, HAES INFUSION 1 %, DOPAMINE HYDROCHLORIDE, INTRALIPID, ADDEL N, SOLUVIT, VITALIPID, GLUCOSE, and CORNEREGEL AUGENGEL. PANTOZOL, DICLOFENAC, COLCHICIN, CIPROFLOXACIN, HEPARIN SODIUM, DIMETICONE, CARVEDILOL, NEBACETIN, VANCOMYCIN, PRIMAXIN, PROTAMINE HYDROCHLORIDE, ROCURONIUM BROMIDE, PROTHROMBINEX, CEFAZOLIN SODIUM, ERYTHROCYTE CONCENTRATE, GLYCERYL TRINITRATE, PROPOFOL, GFP INFUSION, EPINEPHRINE, RINGER-LACTATE, AMIODARONE HYDROCHLORIDE, NOREPINEPHRINE, MILRINONE, SUCRALFATE, MAGNESIUM SULFATE, ACETYLCYSTEINE, PIP/TAZO, MAGNESIUM BIOMED, CLINIMIX, AMINO-HEPAR, METOCLOPRAMIDE HYDROCHLORIDE, NEOSTIGMINE BROMIDE, AMANTADINE SULFATE, AMPHOTERICIN B, FLUMAZENIL, bland diet, HALOPERIDOL, LORAZEPAM, AMPICILLIN, FLUIMICIL INHALATION, INSULIN, PIRITRAMIDE, LACTULOSE, GENTAMICIN SULFATE, DEXPANTHENOL, METRONIDAZOLE, AMINOFUSIN HEPAR SINE, PREDNISOLONE SODIUM SUCCINATE, ROFECOXIB, FORTIMEL, GELAFUNDIN, NUTRI TWIN, HUMAN ALBUMIN, HAES INFUSION 1 %, DOPAMINE HYDROCHLORIDE, INTRALIPID, ADDEL N, SOLUVIT, VITALIPID, GLUCOSE, and CORNEREGEL AUGENGEL were also considered suspect.

Sender's comment:

Abbott Germany Assessment: 25.02.2008, Klacid(R) forte 500 mg - 8982 / 08P-062-0439153-00, PA
In den Angaben zum Arzneimittel weist der Zulassungsinhaber auf Stevens-Johnson-Syndrom hin.

Reporter's comment:

Bayer Vital GmbH, 04.03.2008 DE-BAYER-200813650GPV

Sender diagnosis: not reported

Narrative

This post-marketing study report was received on 20-Feb-2008 by the regulatory authority of Germany, referring to a 58 year-old male.

This is a solicited report from the Documentation Center for Severe Skin Reaction in Germany.

Concomitant drugs have been given. For details see section "Concomitant Drugs" at the end of this report.

Patient received:

- Ciprobay (ciprofloxacin) for infection nos. Administration began on 04-Oct-2003, for a duration of 14 days at a total daily dose of 800 MG orally, and also administered on 26-Oct-2003, for a duration of 3 days at a total daily dose of 800 MG orally
- Diclofenac (diclofenac) for pain nos. Administration began on 17-Oct-2003, for a duration of 2 days, total daily dose was not reported.
- Vancomycin (vancomycin) for inflammation nos. Administration began on 17-Oct-2003, for a duration of 6 days, total daily dose was not reported.
- Heparin (heparin) for thrombosis prophylaxis. Administration began on 03-Oct-2003, for a duration of 7 weeks and 4 days, total daily dose was not reported.
- Sab Simplex (dimeticone, activated) for obstipation. Administration began on 05-Oct-2003, for a duration of 11 days at a total daily dose of 60 ML orally, and also administered on 18-Oct-2003, for a duration of 5 weeks and 3 days at a total daily dose of 60 ML orally.
- Dilatrend (carvedilol) for cardiac insufficiency. Administration began on 08-Oct-2003, for a duration of 3 days at a total daily dose of 6.25 MG orally, and also administered on 12-Oct-2003, for a duration of 6 weeks and 2 days at a total daily dose of 12.5 MG orally.
- Neomycin (neomycin) for intestinal decontamination. Administration began on 11-Oct-2003, for a duration of 6 weeks and 3 days at a total daily dose of 12 G orally.
- Klacid (clarithromycin) for infection nos. Administration began on 15-Oct-2003, for a duration of 5 weeks and 6 days at a total daily dose of 1000 MG orally.
- Pantozol (pantoprazole sodium) for gastric protection. Administration began on 16-Oct-2003, for a duration of 5 weeks and 5 days at a total daily dose of 80 MG orally.
- Zienam for inflammation nos. Administration began on 17-Oct-2003, for a duration of 12 days at a total daily dose of 1500 MG orally.
- Colchicin (colchicine) for pericardial effusion. Administration began on 17-Oct-2003, for a duration of 2 days, total daily dose was not reported, and also administered on 20-Oct-2003, for a duration of 10 days, total daily dose was not reported.

On 31-Oct-2003, 27 days after starting treatment with Ciprobay, 14 days after starting treatment with Diclofenac and Vancomycin, 4 weeks after starting treatment that was ongoing with Heparin, 26 days after starting treatment that was ongoing with Sab Simplex, 23 days after starting treatment that was ongoing with Dilatrend, 20 days after starting treatment that was ongoing with Neomycin, 16 days after starting treatment that was ongoing with Klacid, 15 days after starting treatment that was ongoing with Pantozol, 14 days after starting treatment with Zienam, and 14 days after starting treatment with Colchicin patient experienced STEVEN-JOHNSON-SYNDROME (diffuse erythema, skin blistering, fever, erosive lips, erosive oral mucosa) considered serious due to hospitalization, and death.

Patient died on 24-Nov-2003. Autopsy information is unknown. The patient died from Stevens-Johnson-syndrome 24 days after onset of the event as stated by the reporter.

The investigator considered the diagnosis FATAL STEVEN-JOHNSON-SYNDROME to be associated.

Sender's comment:

FATAL STEVENS-JOHNSON-SYNDROME, considered serious due to fatal outcome, is listed in the International Product Information for CIPROBAY (ciprofloxacin), however unlisted for VANCOMYCIN and DICLOFENAC. Based on the information received for this case, a causal relation between the reported event and treatment with ciprofloxacin cannot be excluded due to a compatible temporal relationship. STEVENS-JOHNSON SYNDROME has also been reported related to treatment with vancomycin and imipenem, which were administered concomitantly, and with macrolides like clarithromycin, which was given for a duration of 2 weeks until the occurrence of the first clinical symptoms of STEVENS-JOHNSON-SYNDROME. The reported event has also been described after treatment with betablockers like carvediol, with pantoprazol, and with neomycin, all of which administered in a close temporal relation to the occurrence of first signs and symptoms of STEVENS-JOHNSON-SYNDROME. A causal or contributive role of diclofenac, which is known to induce the reported event, may also be discussed, however, it was given for a duration of only two days, 2 weeks prior to the occurrence of the first clinical signs of STEVENS-JOHNSON-SYNDROME.

Reporter's comment:

Bayer Vital GmbH, 05.03.2008 DE-BAYER-200813650GPV

Sender diagnosis: not reported

Narrative

This post-marketing study report was received on 20-Feb-2008 by the regulatory authority of Germany, referring to a 58 year-old male.

This is a solicited report from the Documentation Center for Severe Skin Reaction in Germany.

Concomitant drugs have been given. For details see section "Concomitant Drugs" at the end of this report.

Patient received:

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- Vancomycin (vancomycin) for inflammation nos. Administration began on 17-Oct-2003, for a duration of 6 days, total daily dose was not reported.
- Heparin (heparin) for thrombosis prophylaxis. Administration began on 03-Oct-2003, for a duration of 7 weeks and 4 days, total daily dose was not reported.
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- Klacid (clarithromycin) for infection nos. Administration began on 15-Oct-2003, for a duration of 5 weeks and 6 days at a total daily dose of 1000 MG orally.
- Pantozol (pantoprazole sodium) for gastric protection. Administration began on 16-Oct-2003, for a duration of 5 weeks and 5 days at a total daily dose of 80 MG orally.
- Zienam for inflammation nos. Administration began on 17-Oct-2003, for a duration of 12 days at a total daily dose of 1500 MG orally.
- Colchicin (colchicine) for pericardial effusion. Administration began on 17-Oct-2003, for a duration of 2 days, total daily dose was not reported, and also administered on 20-Oct-2003, for a duration of 10 days, total daily dose was not reported.

On 31-Oct-2003, 27 days after starting treatment with Ciprobay, 14 days after starting treatment with Diclofenac and Vancomycin, 4 weeks after starting treatment that was ongoing with Heparin, 26 days after starting treatment that was ongoing with Sab Simplex, 23 days after starting treatment that was ongoing with Dilatrend, 20 days after starting treatment that was ongoing with Neomycin, 16 days after starting treatment that was ongoing with Klacid, 15 days after starting treatment that was ongoing with Pantozol, 14 days after starting treatment with Zienam, and 14 days after starting treatment with Colchicin patient experienced STEVEN-JOHNSON-SYNDROME (diffuse erythema, skin blistering, fever, erosive lips, erosive oral mucosa) considered serious due to hospitalization, and death.

Patient died on 24-Nov-2003. Autopsy information is unknown. The patient died from Stevens-Johnson-syndrome 24 days after onset of the event as stated by the reporter.

The investigator considered the diagnosis FATAL STEVEN-JOHNSON-SYNDROME to be associated.

Sender's comment:

FATAL STEVENS-JOHNSON-SYNDROME, considered serious due to fatal outcome, is listed in the International Product Information for CIPROBAY (ciprofloxacin), however unlisted for VANCOMYCIN and DICLOFENAC. Based on the information received for this case, a causal relation between the reported event and treatment with ciprofloxacin cannot be excluded due to a compatible temporal relationship. STEVENS-JOHNSON SYNDROME has also been reported related to treatment with vancomycin and imipenem, which were administered concomitantly, and with macrolides like clarithromycin, which was given for a duration of 2 weeks until the occurrence of the first clinical symptoms of STEVENS-JOHNSON-SYNDROME. The reported event has also been described after treatment with betablockers like carvediol, with pantoprazol, and with neomycin, all of which administered in a close temporal relation to the occurrence of first signs and symptoms of STEVENS-JOHNSON-SYNDROME. A causal or contributive role of diclofenac, which is known to induce the reported event, may also be discussed, however, it was given for a duration of only two days, 2 weeks prior to the occurrence of the first clinical signs of STEVENS-JOHNSON-SYNDROME.

Reporter's comment:

001307602, 13.03.2008 DE-ABBOTT-08P-062-0439153-00

Sender diagnosis: not reported

Narrative

Report from Germany of Stevens-Johnson syndrome, burn injury, and fever coincident with CLARITHROMYCIN (KLACID FORTE) therapy. On 02 Oct 2003, the patient began CEFAZOLIN SODIUM for post surgery prophylaxis, GLYCERYL TRINITRATE for heart failure, RINGER-LACTATE for fluid substitution, SUCRALFATE for stomach protection, MAGNESIUM SULFATE for substitution, and ACETYLCYTEINE for secretolysis. On 03 Oct 2003, the patient began PANTOZOL therapy for stomach protection, PROPOFOL for sedation, GFP INFUSION and GLUCOSE as a substitution, EPINEPHRINE and NOREPINEPHRINE as an intraoperative, AMIODARONE HYDROCHLORIDE and MILRINONE for heart failure, INSULIN for diabetes, PIRITRAMIDE for pain, and HEPARIN SODIUM for thrombosis prophylaxis. On 03 Oct 2003, single doses of PROTAMINE as an intraoperative, ROCURONIUM BROMIDE for change of tubes, PROTHROMBINEX for antifibrinolysis, and ERYTHROCYTE CONCENTRATE as an intraoperative were given. On 04 Oct 2003, a single dose of ERYTHROCYTE CONCENTRATE was given. On 04 Oct 2003, the patient began CIPROFLOXACIN for an infection, PIP/TAZO for sanitation of intestine, and LACTULOSE for liver failure. On 04 Oct 2003, CEFAZOLIN SODIUM and GFP INFUSION were discontinued. On 05 Oct 2003, a single dose of MAGNESIUM BIOMED was given for substitution and PROPOFOL was discontinued. On 05 Oct 2003, the patient began CLINIMIX and AMINO-HEPAR for nutrition, METOCLOPRAMIDE HYDROCHLORIDE for emesis, and DIMETICONE for constipation. On 06 Oct 2003, a single dose of GFP INFUSION was given. On 07 Oct 2003, the patient began AMANTADINE SULFATE for transitional psychosis and AMPHOTERICIN B for thrush, and a single dose of NEOSTIGMINE BROMIDE was given for atonia of intestine. On 07 Oct 2003, the patient began FLUIMICIL INHALATION for secretolysis. On 07 Oct 2003, NOREPINEPHRINE was discontinued. On 08 Oct 2003, the patient began CARVEDILOL for heart failure and CLINIMIX was discontinued. On 09 Oct 2003, the patient began FENTAMICIN SULFATE and DEXPANTHENOL for exophthalmus. On 09 Oct 2003, a single dose of FLUMAZENIL was given for diagnosis and EPINEPHRINE was discontinued. On 10 Oct 2003, RINGER-LACTATE and CARVEDILOL were discontinued. On 11 Oct 2003, AMIODARINE HYDROCHLORIDE and AMINO-HEPAR were discontinued, and a single dose of NOREPINEPHRINE was given. On 12 Oct 2003, the patient began bland diet for nutrition and CARVEDILOL was restarted. On 14 Oct 2003, the patient began AMPICILLIN for an unknown indication. On 14 Oct 2003, single doses of HALOPERIDOL and LORAZEPAM were given for restlessness, and PANTOZOL therapy was discontinued. On 15 Oct 2003, the patient began KLACID therapy and METRONIDAZOLE for an infection. On 15 Oct 2003, MILRINONE, SUCRALFATE, MAGNESIUM SULFATE, PIRITRAMIDE, PIP/TAZO, METOCLOPRAMIDE HYDROCHLORIDE, DIMETICONE, AMPHOTERICIN B, bland diet, AMPICILLIN, GLUCOSE, and FLUIMICIL INHALATION were discontinued. On 16 Oct 2003, the patient began AMINOFUSIN HEPAR SINE for substitution. On 16 Oct 2003, PANTOZOL therapy was restarted and METRONIDAZOLE was discontinued. On 17 Oct 2003, CIPROFLOXACIN and ACETYLCYSTEINE were discontinued. On 17 Oct 2003, the patient began DICLOFENAC therapy for pain, COLCHICIN therapy for pericardial effusion, and VANCOMYCIN and PRIMAXIN for inflammation. On 18 Oct 2003, DICLOFENAC and COLCHICIN therapies were discontinued. On 18 Oct 2003, the patient began CORNEREGEL AUGENGEL for an eye hemorrhage and DIMETICONE was restarted. On 19 Oct 2003, a single dose of PREDNISOLONE SODIUM SUCCINATE for an unknown indication was given. On 19 Oct 2003, the patient began ROFECOXIB for pain. On 20 Oct 2003, COLCHICIN therapy was restarted and ROFECOXIB was discontinued. On 22 Oct 2003, VANCOMYCIN was discontinued. On 23 Oct 2003, INSULIN and AMINOFUSIN HEPAR SINE were discontinued. On 24 Oct 2003, GENTAMICIN SULFATE was discontinued. On 26 Oct 2003, CIPROFLOXACIN and GLUCOSE were restarted, and LACTULOSE was discontinued. On 28 Oct 2003, the patient began FORTIMEL for nutrition. On 28 Oct 2003, METRONIDAZOLE was restated and CIPROFLOXACIN and VANCOMYCIN were discontinued. On 29 Oct 2003, COLCHICIN therapy and GLUCOSE were discontinued, and a single dose of PIRITRAMIDE was given. On 29 Oct 2003, the patient began NUTRI TWIN for nutrition and a single dose of GELAFUNDIN was given for substitution. On 30 Oct 2003, the patient began DOPAMINE HYDROCHLORIDE for stabilization of circulation, INTRALIPID and VITALIPID for

nutrition, and ADDEL N and SOLUVIT for substitution. On 30 Oct 2003, a single dose of HUMAN ALBUMIN for nutrition and a single dose of HAES INFUSION 10% for substitution were given. On 31 Oct 2003, the patient experienced diffuse erythema and the first blister at the skin. On 01 Nov 2003, the patient developed fever. On 02 Nov 2003, the patient experienced erosions at the lips and at the oral mucosa. The patient was diagnosed with Stevens-Johnson syndrome. From 02 Nov 2003 to 19 Nov 2003, the patient was treated in the center for burn injury. In Nov 2003, PANTOZOL therapy, HEPARIN SODIUM, DIMETICONE, CARVEDILOL, DEXPANTHENOL, METRONIDAZOLE, FORTIMEL, DOPAMINE HYDROCHLORIDE, INTRALIPID, ADDEL N, and SOLUVIT, VITALIPID were discontinued. On an unreported date, the patient was treated again with NUTRI TWIN. On 24 Nov 2003, the patient died. Cause of death was Stevens-Johnson syndrome. No discharged summary was available. The Stevens-Johnson syndrome was considered life-threatening and involved hospitalization and the burn injury was considered medically significant. The reporting physician believed the Stevens-Johnson syndrome was possibly related to KLACID therapy, PANTOZOL, DICLOFENAC, COLCHICIN, CIPROFLOXACIN, HEPARIN SODIUM, DIMETICONE, CARVEDILOL, NEBACETIN, VANCOMYCIN, and PRIMAXIN. The reporting physician believed the Stevens-Johnson syndrome was not related to PROTAMINE HYDROCHLORIDE, ROCURONIUM BROMIDE, PROTHROMBINEX, CEFAZOLIN SODIUM, ERYTHROCYTE CONCENTRATE, GLYCERYL TRINITRATE, PROPOFOL, GFP INFUSION, EPINEPHRINE, RINGER-LACTATE, AMIODARONE HYDROCHLORIDE, NOREPINEPHRINE, MILRINONE, SUCRALFATE, MAGNESIUM SULFATE, ACETYLCYSTEINE, PIP/TAZO, MAGNESIUM BIOMED, CLINIMIX, AMINO-HEPAR, METOCLOPRAMIDE HYDROCHLORIDE, NEOSTIGMINE BROMIDE, AMANTADINE SULFATE, AMPHOTERICIN B, FLUMAZENIL, bland diet, HALOPERIDOL, LORAZEPAM, AMPICILLIN, and FLUIMICIL INHALATION. The reporting physician can not assess the causality between the Stevens-Johnson syndrome and INSULIN, PIRITRAMIDE, LACTULOSE, GENTAMICIN SULFATE, DEXPANTHENOL, METRONIDAZOLE, AMINOFUSIN HEPAR SINE, PREDNISOLONE SODIUM SUCCINATE, ROFECOXIB, FORTIMEL, GELAFUNDIN, NUTRI TWIN, HUMAN ALBUMIN, HAES INFUSION 1 %, DOPAMINE HYDROCHLORIDE, INTRALIPID, ADDEL N, SOLUVIT, VITALIPID, GLUCOSE, and CORNEREGEL AUGENGEL. PANTOZOL, DICLOFENAC, COLCHICIN, CIPROFLOXACIN, HEPARIN SODIUM, DIMETICONE, CARVEDILOL, NEBACETIN, VANCOMYCIN, PRIMAXIN, PROTAMINE HYDROCHLORIDE, ROCURONIUM BROMIDE, PROTHROMBINEX, CEFAZOLIN SODIUM, ERYTHROCYTE CONCENTRATE, GLYCERYL TRINITRATE, PROPOFOL, GFP INFUSION, EPINEPHRINE, RINGER-LACTATE, AMIODARONE HYDROCHLORIDE, NOREPINEPHRINE, MILRINONE, SUCRALFATE, MAGNESIUM SULFATE, ACETYLCYSTEINE, PIP/TAZO, MAGNESIUM BIOMED, CLINIMIX, AMINO-HEPAR, METOCLOPRAMIDE HYDROCHLORIDE, NEOSTIGMINE BROMIDE, AMANTADINE SULFATE, AMPHOTERICIN B, FLUMAZENIL, bland diet, HALOPERIDOL, LORAZEPAM, AMPICILLIN, FLUIMICIL INHALATION, INSULIN, PIRITRAMIDE, LACTULOSE, GENTAMICIN SULFATE, DEXPANTHENOL, METRONIDAZOLE, AMINOFUSIN HEPAR SINE, PREDNISOLONE SODIUM SUCCINATE, ROFECOXIB, FORTIMEL, GELAFUNDIN, NUTRI TWIN, HUMAN ALBUMIN, HAES INFUSION 1 %, DOPAMINE HYDROCHLORIDE, INTRALIPID, ADDEL N, SOLUVIT, VITALIPID, GLUCOSE, and CORNEREGEL AUGENGEL were also considered suspect.

Follow-up information received on 04 Mar 2008: Suspect drug information and adverse event information were added or revised. It was clarified that the trade name for CLARITHROMYCIN was KLACID FORTE and not KLACID as previously reported. It was clarified that on 30 Oct 2003, the patient received single dose of HUMAN ALBUMIN as a substitution and not as nutrition as previously reported. On an unknown date, the KLACID FORTE therapy was discontinued. It was unknown if an autopsy was performed. The reporting physician stated that in very rare cases of Stevens-Johnson syndrome an autopsy had been performed. The reporting physician also stated that statistic results of examination had shown that Stevens-Johnson syndrome can be considered cause of death if the patient died within six weeks after the onset of Stevens-Johnson syndrome. In this case, the onset of Stevens-Johnson syndrome was on 31 Oct 2003 and the patient died on 24 Nov 2003. So, Stevens-Johnson syndrome can be considered the cause of death. The reporting physician believed the Stevens-Johnson syndrome and PREDNISOLONE SODIUM SUCCINATE, ROFECOXIB, FORTIMEL, GELAFUNDIN, NUTRI TWIN, HUMAN ALBUMIN, HAES INFUSION 1%, DOPAMINE HYDROCHLORIDE, INTRALIPID, ADDEL N, SOLUVIT, and VITALIPID were questionably related.

Sender's comment:

Abbott Germany Assessment: 25.02.2008, Klacid(R) forte 500 mg - 8982 / 08P-062-0439153-00, PA
In den Angaben zum Arzneimittel weist der Zulassungsinhaber auf Stevens-Johnson-Syndrom hin.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08007496

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 81 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Urinary tract infection	20050321				Unknown
2 Stevens-Johnson syndrome	20050323	20050411	20 Day	18 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-08007496

1	ADUMBRAN oxazepam	Oral	10 Mg milligram(s)/	20050309-20050309 Sedation 1 Day
2	ATACAND candesartan cilexetil	Oral	4 Mg milligram(s)/Day	20050306-20050326 Hypertension arterial 21 Day
3	CALCIVIT D calcium carbonate; colecalciferol	Oral	1 Df dosage form/Day	20050307-20050322 Osteoporosis 16 Day
4	decortin prednisone	Oral	0.5 Df dosage form/Day	20050309- Pain -
5	decortin prednisone	Oral	0.5 Df dosage form/Day	20050307-20050307 Osteoporosis 1 Day
6	fenistil-retard dimetindene maleate	Oral	1 Df dosage form/Day	20050321- Pruritus -
7	FOSAMAX alendronate sodium	Oral	70 Mg milligram(s)/Week	20050306-20050326 Osteoporosis 21 Day
8	FRAXIPARIN nadroparin calcium	Subcutaneous	0.3 Mg milligram(s)/Day	20050307-20050322 Thromboembolism prophylaxis 16 Day
9	MUSARIL tetrazepam	Oral	2 Df dosage form/Day	20050306-20050313 Sedation 8 Day
10	NEXIUM esomeprazole magnesium	Oral	/	20050306-20050307 Prophylaxis against 134 gastrointestinal ulcer
11	NEXIUM esomeprazole magnesium	Oral	/Day	20050309-20050312 Prophylaxis against 134 gastrointestinal ulcer
12	NEXIUM esomeprazole magnesium	Oral	/Day	20050323- Prophylaxis against 134 gastrointestinal ulcer
13	OMEPR omeprazole	Oral	20 Mg milligram(s)/Day	20050306-20050306 Prophylaxis against 1 Day gastrointestinal ulcer
14	OMEPR omeprazole	Oral	20 Mg milligram(s)/Day	20050309-20050322 Prophylaxis against 14 Day gastrointestinal ulcer
15	PASPERTIN metoclopramide hydrochloride	Oral	20 Gtt drop(s)/	20050318-20050321 Nausea 4 Day
16	PASPERTIN metoclopramide hydrochloride	Oral	20 Gtt drop(s)/	20050312-20050313 Nausea 158 2 Day
17	PRACTO-CLYSS sodium phosphate dibasic; sodium phosphate monobasic (anhydrate)	Rectal	/	20050318-20050318 Obstipation 242 1 Day
18	STILNOX zolpidem	Oral	1 Df dosage form/Day	20050306-20050322 Initial insomnia 17 Day
19	talvosilen forte codeine phosphate; paracetamol	Oral	2 Df dosage form/Day	20050316-20050319 Pain 4 Day
20	talvosilen forte codeine phosphate; paracetamol	Oral	2 Df dosage form/Day	20050321-20050321 Pain 1 Day
21	tarivid ofloxacin	Oral	400 Mg milligram(s)/Day	20050321-20050325 Urinary tract infection 5 Day
22	voltaren retard diclofenac sodium	Oral	3 Df dosage form/Day	20050306-20050313 Pain 8 Day
23	vomex a dimenhydrinate	Rectal	1 Df dosage form/	20050313-20050313 Nausea 242 1 Day

Drugs (concomitant)				
24	ADUMBRAN oxazepam	Oral	10 Mg milligram(s)/	20050322-20050322 Sedation 1 Day
25	aspirin protect acetylsalicylic acid	Oral	1 Df dosage form/Day	20050228-20050228 Thromboembolism prophylaxis 1 Day
26	BIFITERAL SIRUP lactulose	Oral	/	20050317-20050317 Obstipation 1 Day
27	BLOPRESS candesartan cilexetil	Oral	4 Mg milligram(s)/Day	20050306-20050306 Hypertension arterial 1 Day
28	BLOPRESS candesartan cilexetil	Oral	4 Mg milligram(s)/Day	20050327- Hypertension arterial -
29	BLOPRESS PLUS candesartan cilexetil; hydrochlorothiazide	Oral	1 Df dosage form/Day	20050228-20050307 Hypertension arterial 8 Day
30	calcium d3 calcium carbonate; colecalciferol	Oral	1 Df dosage form/Day	20041201-20050307 Osteoporosis 3 Month
31	CALCIUM D3-RATIOPHARM calcium; colecalciferol	Oral	/	20050228-20050307 Osteoporosis 8 Day
32	CEFAZOLIN cefazolin	Intravenous (not otherwis	2 G gram(s)/Day	20050308-20050308 Antibiotic prophylaxis 1 Day
33	CETIRIZIN cetirizine	Oral	1 Df dosage form/Day	20050306-20050306 Skin reaction 1 Day
34	NITROFURANTOIN RET nitrofurantoin	Oral	2 Df dosage form/Day	20050324-20050327 Urinary tract infection 4 Day
35	PARACETAMOL paracetamol	Oral	1 Df dosage form/	20050327-20050327 Fever 1 Day
36	predni h tablinen prednisolone	Oral	5 Mg milligram(s)/Day	20050228-20050307 Pain 8 Day
37	PREDNI TABLINEN prednisolone	Oral	2.5 Mg milligram(s)/Day	20050306-20050306 Skin reaction 1 Day
38	THROMBOESPRTIZE	Subcutaneous	/	20050323-20050327 Thromboembolism prophylaxis 5 Day
39	VALORON N RET naloxone hydrochloride; tilidine hydrochloride	Oral	300 Mg milligram(s)/Day	20050306-20050311 Pain 6 Day
40	voltaren resinat diclofenac resinate	Oral	2 Df dosage form/Day	20050306-20050306 Pain 1 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Yes	
Constipation			Yes	
Hospitalisation			Yes	
Initial insomnia			Yes	
Nausea			Yes	
Pain			Yes	
Skin disorder			Yes	
Surgery			Yes	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			
Qualification:	Other Health Professional	observestudytype:	
Literature:			

Narratives (nur bei elektronischem Eingang)

009507513, 25.01.2008 DE-MERCK-0801DEU00099

Sender diagnosis: not reported

Narrative

Information has been received from a registry (documenting serious skin reactions that occur in Germany) concerning an 81 year old female with pain (due to osteoporosis), initial insomnia, arterial hypertension, nausea and constipation who, on 06-MAR-2005, was hospitalized for a not further specified surgery. That same day, she was treated with single doses of cetirizine 10 mg PO and prednisolone 5 mg PO for a not further specified skin disorder.

On 06-MAR-2005, the patient was also placed on therapy with alendronate sodium, 70 mg tablet, 70 mg once a week for the treatment of osteoporosis.

Secondary suspect therapies included diclofenac sodium (VOLTAREN) 100 mg tablet, 100 mg four times a day, given from 06-MAR-2005 until 13-MAR-2005 for the treatment of pain; tetrazepam (MUSARIL) 50 mg tablet, 50 mg three times a day, given from 06-MAR-2005 until 13-MAR-2005 for sedation; zolpidem tartrate (STILNOX), 10 mg tablet, 10 mg once a day, given from 06-MAR-2005 until 22-MAR-2005 for the treatment of initial insomnia; omeprazole (OMEP) 20 mg tablet, 20 mg once a day, given on 06-MAR-2005 and from 09-MAR-2005 until 22-MAR-2005 for gastric ulcer prophylaxis; candesartan cilexetil (ATACAND) 8 mg tablet, 4 mg once a day, given from 06-MAR-2005 until 26-MAR-2005 for the treatment of arterial hypertension; esomeprazole magnesium (NEXIUM), tablet, 30 mg twice a day, given on 06-MAR-2005, from 09-MAR-2005 until 12-MAR-2005 and from 23-MAR-2005 onwards for gastric ulcer prophylaxis; nadroparin calcium (FRAXIPARIN), injection, 0.3 mg s.c. once a day, given from 07-MAR-2005 until 22-MAR-2005 for thrombosis prophylaxis; prednisone (DECORTIN) tablet twice a day (dose unknown), given on 07-MAR-2005 and from 09-MAR-2005 onwards for the treatment of pain due to osteoporosis; metoclopramide hydrochloride (PASPERTIN) oral solution, 20 drops as needed, given from 12-MAR-2005 until 13-MAR-2005 and from 18-MAR-2005 until 21-MAR-2005 for the treatment of nausea; dimenhydrinate (VOMEX A), 150 mg suppositories, 150 mg rectal administration as needed, given on 13-MAR-2005 for the treatment of nausea; acetaminophen (+) codeine phosphate (TALVOSILEN) 500-30 mg capsule, 500-30 mg twice a day, given from 16-MAR-2005 until 19-MAR-2005 and on 21-MAR-2005 for the treatment of pain; sodium phosphate, dibasic (+) sodium phosphate, monobasic (PRACTO-CLYSS, dose unknown), rectal administration on 18-MAR-2005 for the treatment of constipation; ofloxacin (TARIVID) 400 mg tablet, 400 mg once a day, given from 21-MAR-2005 until 25-MAR-2005 for the treatment of urinary tract infection.

Concomitant therapy included hydrochloride (+) tilidine hydrochloride (VALORON N), calcium phosphate, dibasic (+) cholecalciferol (CALCIVIT), oxazepam (ADUMBRAN), lactulose (BIFITERAL), dimethindene maleate (FENISTIL).

On 08-MAR-2005, the planned surgery was performed.

On 23-MAR-2005, during her stay in a rehabilitation clinic, the patient developed diffuse skin redness. On 25-MAR-2005, she developed fever. On 26-MAR-2005, she developed pruritus, pain of skin and macules. On 26-MAR-2005 therapy with alendronate sodium was discontinued. On 28-MAR-2005, she developed blisters. Diagnosis of Stevens-Johnson syndrome with onset 23-MAR-2005 was made. On 29-MAR-2005, the patient was transferred to the dermatologic ward; on 01-APR-2005, she was transferred to the burn unit. The outcome was reported as "not recovered".

The reporting physician felt that Stevens-Johnson syndrome was probably related to diclofenac and tetrazepam and possibly related to alendronate, zolpidem tartrate, omeprazole, candesartan, esomeprazole, nadroparin calcium, prednisone, metoclopramide, dimenhydrinate, acetaminophen (+) codeine phosphate, sodium phosphate, dibasic (+) sodium phosphate, monobasic and ofloxacin. Acute cystitis (urinary tract infection) was also suspected.

Stevens-Johnson syndrome prolonged hospitalization.

Information received on 22-JAN-2008 contained the following adverse experience: urinary tract infection (21-MAR-2005). Additional information is not expected.

Sender's comment:

Stevens-Johnson syndrome (SJS) is a serious labeled event for therapy with FOSAMAX°. This case-report was forwarded to us by the documentation center of severe skin reactions. This 81 year old female was diagnosed with Stevens-Johnson syndrome after taking FOSAMAX° for around 20 days. According to the expert dermatologists the primary suspect drugs as trigger of the skin reaction were diclofenac and tetrazepam. Furthermore several concomitant medications were assessed to be potentially causally associated with SJS along with FOSAMAX°. As a result it is not possible to determine the role of FOSAMAX°. SJS is however described in the current labeling. Self initiated measures are not indicated.

Reporter's comment:

AZPROD0000, 26.02.2008 DE-AstraZeneca-2008PK00363

Sender diagnosis: not reported

Narrative

{STEVENS-JOHNSON SYNDROME}

A report has been received from the Centre for Documentation of Severe Skin Reactions in Germany via the German Health Authority (BfArM) concerning an 79 year old female patient. The patient was being treated with ATACAND (candesartan cilexetil) QD oral for hypertension arterial, NEXIUM ORAL (esomeprazole magnesium), OMEP (omeprazole), DICLOFENAC SODIUM SR (diclofenac sodium), MUSARIL (tetrazepam), STILNOX (zolpidem), FOSAMAX (alendronate sodium), FRAXIPARIN (nadroparin calcium), DECORTIN (prednisone), metoclopramide hydrochloride, dimenhydrinate, talvosilen forte, practo-clyss (sodium phosphate) and TARIVID (ofloxacin).

Concomitant drugs included acetylsalicylic acid for prophylaxis of thrombosis, prednisolone for pain due to osteoporosis, blopress plus for hypertension arterial, calcium d3-ratiopharm for osteoporosis, diclofenac resinate for pain, cetirizine for skin reaction, prednisolone acetate for skin reaction, valoron n ret for pain, candesartan cilexetil for hypertension arterial, calcivit d for osteoporosis, cefazolin for intraoperative prophylaxis of infection, oxazepam for sedation, lactulose for obstipation, dimetindene maleate for pruritus, thrombosespritze for prophylaxis of thrombosis, nitrofurantoin for urinary tract infection and paracetamol for fever.

The patient experienced first skin reaction on 05-Mar-2005. On 08-Mar-2005 she underwent an unspecified operation. Since 23-Mar-2005 she suffered from diffuse redness, followed by fever, pruritus, pain in skin, and blisters on 28-Mar-2005. Stevens-Johnson Syndrome was diagnosed on 23-Mar-2005.

The patient has not yet recovered.

The event was considered serious due to hospitalization.

The reporter considered the event possibly related to candesartan cilexetil, esomeprazole magnesium, omeprazole, diclofenac sodium, tetrazepam, zolpidem, alendronate sodium, nadroparin calcium, prednisone, metoclopramide hydrochloride, dimenhydrinate, talvosilen forte, sodium phosphate and ofloxacin.

Sender's comment:

Stevens-Johnson Syndrome is not listed for candesartan but has been associated with use of esomeprazole. It is difficult to assess causality based on the information provided because several other co-suspected drugs may also have been causative for the event. Patient started therapy with Blopress (candesartan) again on 27-Mar-2005.

Report according to article 42 and 63b AMG. Stevens-Johnson Syndrome is not listed for candesartan but has been associated with use of esomeprazole. It is difficult to assess causality based on the information provided because several other co-suspected drugs may also have been causative for the event. Patient started therapy with Blopress (candesartan) again on 27-Mar-2005.

Causality assessment: unassessable

NO

Reporter's comment:

Merck KGaA, 27.02.2008 DE-Merck KGaA-6040903

Sender diagnosis: not reported

Narrative

This is a case from the Schoepf registry / intensified monitoring programme being called RegiSCAR study (# 9160272) that is coordinated by the Dermatological University Hospital Freiburg, Germany, and that registers and evaluates all hospitalized cases of severe skin reactions such as toxic epidermal necrolysis (TEN), Stevens-Johnson syndrome (SJS), acute generalised exanthematous pustulosis (AGEP) and drug reaction with eosinophilia and systemic symptoms (DRESS) in 6 European countries, and that documents the demographic characteristics, the role of infection and drug intake in the past history of the severe skin reaction.

The report was received via German Health Authority (#08007496) on 19 Feb 2008.

An 81-year-old female patient was treated with Decortin (prednisone) on 07 Mar 2005 and since 09 Mar 2005 for pain due to osteoporosis. On 05 Mar 2005, she experienced questionable skin alterations. From 06 Mar 2005 to 11 Apr 2005 she was hospitalized. From 06 Mar 2005 to 23 Mar 2005, she was on the surgery ward since she had an operation on 08 Mar 2005 (for an unknown reason). From 23 Mar 2005 to 29 Mar 2005, she was on rehabilitation ward. During this time Steven-Johnson Syndrome (SJS), ascertained case, was diagnosed on 28 Mar 2005 (index day) beginning with diffuse redness on 23 Mar 2005. On 25 Mar 2005, she developed fever. On 26 Mar 2005, she experienced pruritus, painful skin, cockades and spots.

On 28 Mar 2005, she developed blister on the skin (index day). From 29 Mar 2005 to 01 Apr 2005, she was in the dermatology department and from 01 Apr 2005 to 11 Apr 2005, she was admitted to the department for skin burns.

Medical history: Not reported.

As concurrent infection an acute cystitis was suspected for which she was treated with Tarivid (ofloxacin) from 24 Mar 2005 to 27-Mar 2005 and from 24 Mar 2005 to 27 Mar 2005 with Nitrofurantoin (nitrofurantoin).

Other suspected drugs were:

Valoron N retard (tilidin, naloxone) and Voltaren retard (diclofenac) for pain, Musaril (tetrazepam) for sedation, Stilnox (zolpidem) for sleep disturbances, Omep (omeprazole) for gastric ulcer prophylaxis, Atacand (candesartan cilexetil) for arterial hypertension, Fosamax (alendronate sodium) for osteoporosis, Nexium (esomeprazole) for gastric ulcer prophylaxis, Fraxiparin (heparin-calcium) for thrombosis prophylaxis, Calcivit D (calcium carbonate, colecalciferol) for osteoporosis, Adumbran (oxazepam) for sedation, Paspertin (metoclopramide hydrochloride) and Vomex A (dimenhydrinate) for nausea, Talvosilen forte (paracetamol, codeine) for pain not further specified, Bifiteral (lactulose) and Practo-Clyss (sodium hydrogenphosphate) for obstipation, Tarivid (ofloxacin) for suspected urinary tract infection and Fenistil retard (dimetindene maleate) for pruritus. Concomitant drugs were: Aspirin protect (acetylsalicylic acid) for thrombosis prophylaxis, Predni H Tablinen (prednisolone) for pain due to osteoporosis, Blopress plus (candesartan, hydrochlorothiazide) and Blopress (candesartan) for arterial hypertension, Calcium D3-ratiopharm (calcium carbonate, colecalciferol) for osteoporosis, Voltaren Resinat (diclofenac colestyramine) for pain not further specified, Cetirizin (cetirizine) for skin changes, Predni Tablinen (prednisone) for skin changes, Cefazolin (cefazolin) for intraoperative infection prophylaxis, thrombosis prophylaxis (not further specified), Nitrofurantoin retard (nitrofurantoin) for suspected urinary tract infection and Paracetamol (paracetamol) for fever.

The documentation of the patient was then transferred to the expert panel of the Schoepf registry. This case was evaluated by the expert panel of Prof. Schoepf in Mar 2006. There the diagnosis of Stevens-Johnson syndrome (SJS) was ascertained and suspected drugs and concomitant drugs were identified.

The outcome was reported as not yet recovered on 11 Apr 2005 (time of reporting).

The assessment of causal relationship between the suspected drugs and the occurrence of severe skin reaction is recorded in a rating scale from 1-8 scores (1= definite, 2= very probable, 3= probable, 4= likely; 5= possible; 6= doubtful; 7= no relationship, 8= unlikely).

The expert panel assessed the Steven-Johnson Syndrome as likely related to Voltaren retard. and Musaril, as possibly related to Stilnox, Omep, Atacand, Fosamax, Nexium, Fraxiparin, Decortin, Paspertin, Vomex, Talvosilen forte, Practo-Clyss and Tarivid, as doubtfully related to Valoron N retard, Calcivit D, Adumbran, Bifiteral and Fenistil.

No relationship was assumed by the expert panel for Aspirin protect, Predni H Tablinen, Blopress plus, Blopress, Calcium D3-ratiopharm, Voltaren Resinat, Cetirizin, Predni Tablinen, Cefazolin, thrombosis prophylaxis (not further specified), Nitrofurantoin and Paracetamol and therefore, these drugs were not considered as suspected drugs but as concomitant drugs.

Furthermore, the expert panel indicated that acute cystitis might have contributed to the SJS as well.

Sender's comment:

Case from the Schoepf Registry / Intensified Monitoring Programme for hospitalized cases of severe skin reactions at the University of Freiburg in Germany (RegiSCAR study) being reported by Health Authority.

Ascertained case of Stevens-Johnson syndrome (SJS) in an 81 year old female patient taking Decortin (prednisone) and 30 further drugs. Decortin was indicated for pain due to osteoporosis.

Already two days before initiation of Decortin, questionable skin reactions were recorded, which escalated 2 weeks later into a diffuse erythema. After another two days, the patient presented with fever and the following day with pruritus, painful skin, cockades and spots on the skin. About three weeks after initiation of Decortin, skin blisters emerged.

Co-suspected drugs were: Valoron N retard (tilidin, naloxone), Voltaren retard (diclofenac), Musaril (tetrazepam), Stilnox (zolpidem), Omep (omeprazole), Atacand (candesartan cilexetil), Fosamax (alendronate sodium), Nexium (esomeprazole), Fraxiparin (heparin-calcium), Calcivit D (calcium carbonate, colecalciferol), Adumbran (oxazepam), Paspertin (metoclopramide hydrochloride), Vomex A (dimenhydrinate), Talvosilen forte (paracetamol, codeine), Bifiteral (lactulose), Practo-Clyss (sodium hydrogenphosphate), Tarivid (ofloxacin) and Fenistil retard (dimetindene maleate). Concomitant drugs were: Aspirin protect (acetylsalicylic acid), Predni H Tablinen (prednisolone), Blopress plus (candesartan, hydrochlorothiazide), Blopress (candesartan), Calcium D3-ratiopharm (calcium carbonate, colecalciferol), Voltaren Resinat (diclofenac colestyramine), Cetirizin (cetirizine), Predni Tablinen (prednisone), Cefazolin (cefazolin), thrombosis prophylaxis (not further specified), Nitrofurantoin retard (nitrofurantoin) and Paracetamol (paracetamol).

Relevant medical history: pain, osteoporosis, arterial hypertension.

The expert panel assessed the Steven-Johnson Syndrome as likely related to Voltaren retard. and Musaril, as possibly related to St

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08008800

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 69 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20050131			17 Day	not recovered/not resolved
2 Agitation	20050116			15 Day	Unknown
3 Anxiety	20050116				Unknown
4 Arthritic pains	20050119				Unknown
5 Gout acute	20050119			15 Day	Unknown
6 Hyperuricaemia	20050131			15 Day	Unknown
7 Sleep disturbance	20050124			15 Day	Unknown
8 Arthritis				15 Day	Unknown
9 Blister	20050204			21 Day	Unknown
10 Cardiac insufficiency				15 Day	Unknown
11 Conjunctivitis	20050131			17 Day	Unknown
12 Dysphagia	20050201			18 Day	Unknown
13 Fear				15 Day	Unknown
14 Hypertension				15 Day	Unknown
15 Hyperthyroidism				15 Day	Unknown
16 Macule	20050131			17 Day	Unknown
17 Nikolsky's sign	20050205			22 Day	Unknown
18 Pain				15 Day	Unknown
19 Pain of skin	20050129			15 Day	Unknown
20 Pruritus	20050131			17 Day	Unknown
21 Pulmonary edema				15 Day	Unknown
22 Restlessness				15 Day	Unknown
23 Toxic epidermal necrolysis	20050131			17 Day	not recovered/not resolved
24 Lip erosion	20050202			19 Day	Unknown
25 Nasal mucosal disorder	20050206			23 Day	Unknown
26 Genital erosion	20050206			23 Day	Unknown
27 Oral mucosa erosion	20050202			19 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-08008800

1	ASS acetylsalicylic acid	Oral	100 Mg milligram(s)/Day Tablet	20050115-20050131 Apoplexy 17 Day
2	augmentan amoxicillin trihydrate; clavulanic acid	Oral	2625 Mg milligram(s)/Day Tablet	20050114-20050123 Infection prophylaxis 10 Day
3	BELOC ZOK MITE metoprolol succinate	Oral	95 Mg milligram(s)/Day Tablet	20050115- Hypertension -
4	BELOC-ZOK metoprolol	Oral	2* / Day	20050115-20050131 Hypertension 17 Day
5	CARBIMAZOL carbimazole	Oral	1 Df dosage form/Day Tablet	20050115-20050131 Hyperthyroidism 17 Day
6	cibadrex benazepril hydrochloride; hydrochlorothiazide	Oral	10 Mg milligram(s)/Day Tablet	20050125-20050131 Hypertension 7 Day
7	COLCHICIN colchicine	Oral	1.2 Mg milligram(s)/ Tablet	20050119-20050122 Gout 4 Day
8	COLCHICIN colchicine	Oral	1.2 Mg milligram(s)/ Tablet	20050127-20050130 Gout 4 Day
9	DELIX PLUS hydrochlorothiazide; ramipril	Oral	2.5 Mg milligram(s)/Day Tablet	20050116-20050131 Hypertension 16 Day
10	EUNERPAN melperone	Oral	25 Mg milligram(s)/Day Tablet	20050124-20050131 Sleep disorder 8 Day
11	HEPARIN heparin sodium	Intravenous (not otherwis	/Day Solution for infusion	20050114-20050131 Thrombosis prophylaxis 18 Day
12	IRENAT sodium perchlorate	Oral	360 Df dosage form/Day Oral drops	20050115-20050128 Hyperthyroidism 14 Day
13	tavor lorazepam	Oral	1 Mg milligram(s)/Day Tablet	20050116-20050131 Anxiety disorder 16 Day
14	TORASEMIDE torasemide	Oral	10 Mg milligram(s)/	20050115-20050131 Heart insufficiency 17 Day
15	unat torasemide	Oral	2* /Day Tablet	20050115- Cardiac failure congestive -
16	voltaren diclofenac sodium	Oral	2* / Day	20050127-20050130 Arthritis 4 Day
17	voltaren diclofenac sodium	Oral	2* / Day	20050119-20050120 Arthritis 2 Day
18	voltaren resinat diclofenac resinate	Oral	2 Df dosage form/Day Tablet	20050127-20050130 Arthritic pains 4 Day
19	voltaren resinat diclofenac resinate	Oral	2 Df dosage form/Day Tablet	20050119-20050120 Arthritic pains 2 Day
20	ZOCOR simvastatin	Oral	20 Mg milligram(s)/Day Tablet	20050115- Hyperlipidaemia -
21	ZOCOR simvastatin	Oral	20 Mg milligram(s)/ Day	20050115-20050131 Hyperlipidaemia 17 Day

Drugs (concomitant)

22	ALLOPURINOL allopurinol	Oral	100 Mg milligram(s)/Day Tablet	20050131-20050131	Hyperuricaemia 1 Day
23	BELOC metoprolol	Intravenous (not otherwise)	5 Mg milligram(s)/Day Injection	20050115-20050115	Post MI 1 Day
24	CORDAREX amiodarone	Intravenous (not otherwise)	300 Mg milligram(s)/Day Injection	20050115-20050115	Heart insufficiency 1 Day
25	DELIX ramipril	Oral	2.5 Mg milligram(s)/Day Tablet	20050115-20050115	Hypertension 1 Day
26	fenistil dimetindene maleate	Intravenous (not otherwise)	2 Df dosage form/ -	20050131-20050131	Rash pruritic -
27	LASIX furosemide	Oral	40 Mg milligram(s)/Day Tablet	20000101-20050114	Oedema 5 Year
28	LASIX furosemide	Intravenous (not otherwise)	80 Mg milligram(s)/ Injection	20050115-20050117	Lung oedema 3 Day
29	MARCUMAR phenprocoumon	Oral	2* / Day Tablet	19950101-20050114	Apoplexy 10 Year
30	Morphin morphine	Subcutaneous	2.5 Mg milligram(s)/Day Injection	20050115-20050115	Pain 1 Day
31	sortis atorvastatin calcium	Oral	20 Mg milligram(s)/Day Tablet	20050114-20050115	Hyperlipidaemia 2 Day
32	TAGAMET cimetidine	Intravenous (not otherwise)	2 Df dosage form/ -	20050131-20050131	Exanthema -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Heart insufficiency			Unknown	
Gout			Unknown	
Pruritus			Unknown	
Sleep disorder			Unknown	
Anxiety disorder			Unknown	
Apoplexy			No	
Cardiac failure congestive			Yes	
Hospitalisation			Yes	
Hypertension			Yes	
Hyperthyroidism			Yes	
Myocardial infarction			No	
Hyperlipidaemia			Unknown	
Exanthema			Unknown	
Arthritis			Unknown	
Hyperlipidemia			Yes	
Pulmonary oedema			Yes	
Pain			Yes	
Oedema			Yes	

Primary source

Qualification: Physician observestudytype: Other studies
Literature:

Narratives (nur bei elektronischem Eingang)

009507513, 28.01.2008 DE-MERCK-0801DEU00102

Sender diagnosis: not reported

Narrative

Information has been received from a registry (documenting serious skin reactions that occur in Germany) concerning a 69-year-old male patient hospitalized (admission on 14-JAN-20005) with oedema, pain, cardiac failure congestive, hypertension, suspicion of pulmonary oedema and

hyperthyroidism and a history of apoplexy and myocardial infarction who, on 15-JAN-2005, was placed on therapy with simvastatin, 20 mg tablet, 20 mg, once a day for the treatment of hyperlipidaemia (duration not reported).

Suspect secondary therapies included amoxicillin/clavulanate potassium (AUGMENTAN), 875 mg tablet, 875 mg, three times a day for the prophylaxis of infection (therapy initiated on 14-JAN-2005, stopped on 23-JAN-2005); heparin i.v., 25.000 I.U.; 25.000 I.U./24h for prophylaxis of embolism (therapy initiated on 14-JAN-2005); sodium perchlorate (IRENAT), drops, 40 drops, three times a day for the treatment of hyperthyroidism (therapy initiated on 15-JAN-2005, stopped on 28-JAN-2005); metoprolol succinate (BELOC ZOK MITE), 47.5 mg tablet, 47.5 mg, three times daily for the treatment of hypertension (therapy initiated on 15-JAN-2005); torsemide (UNAT), 10 mg tablet, (dose unclear) for the treatment of cardiac failure congestive (therapy initiated on 15-JAN-2005); carbimazole, tablet, 1 tablet daily (dose not reported) for the treatment of hyperthyroidism (therapy initiated on 15-JAN-2005); aspirin, 100 mg tablet, 100 mg, once daily for treatment after apoplexy (therapy initiated on 15-JAN-2005); lorazepam (TAVOR), 0.5 mg tablet, 0.5 mg, twice daily for the treatment of agitation and anxiety (therapy initiated on 16-JAN-2005); hydrochlorothiazide (+) ramipril (DELIX PLUS), 2.5/12.5 mg tablet, 2.5/12.5 mg once a day for the treatment of hypertension (therapy initiated on 16-JAN-2005); diclofenac resinate (VOLTAREN RESINAT), tablet, one tablet twice a day for the treatment of arthritic pains (therapy initiated on 19-JAN-2005, stopped on 20-JAN-2005, restarted on 27-JAN-2005 and stopped on 30-JAN-2005); colchicine, 0.2 mg tablet, 0.2 mg six times daily for the treatment of gout attack (therapy initiated on 19-JAN-2005, stopped on 22-JAN-2005, restarted on 27-JAN-2005 and stopped on 30-JAN-2005); melperone HCl (EUNERPAN), 25 mg tablet, 25 mg once daily for the treatment of sleep disturbances (therapy initiated on 24-JAN-2005) and benazepril HCl/HCTZ (CIBADREX), 10 mg tablet, 5 mg once a day for the treatment of hypertension (therapy initiated on 25-JAN-2005).

Other concomitant therapy included phenprocoumon (MARCUMAR), furosemide (LASIX (furosemide)), atorvastatin calcium (SORTIS), amiodarone hydrochloride (CORDAREX), morphine, ramipril (DELIX), metoprolol tartrate (BELOC), furosemide (LASIX (furosemide)) and allopurinol.

On 31-JAN-2005, the patient developed pruritus, purpura, blotches and conjunctivitis. He was diagnosed with transitional form of stevens-johnson syndrome/toxic epidermal necrolysis. On 01-FEB-2005, he developed dysphagia. On 02-FEB-2005, he developed erosions of lips and oral mucosa. On 04-FEB-2005, he developed skin blisters. On 05-FEB-2005, he developed Nikolsky's signs. On 06-FEB-2005, he developed erosions of genital mucosa and nasal mucosa. Action taken was not reported. The patient's transitional form of stevens-johnson syndrome/toxic epidermal necrolysis persisted.

The reporting physician felt that transitional form of stevens-johnson syndrome/toxic epidermal necrolysis was immediately life-threatening and possibly related to therapy with simvastatin and amoxicillin/clavulanate potassium (AUGMENTAN) and heparin and sodium perchlorate (IRENAT) and metoprolol succinate (BELOC ZOK MITE) and torsemide (UNAT) and carbimazole and aspirin and lorazepam (TAVOR) and hydrochlorothiazide (+) ramipril (DELIX PLUS) and diclofenac resinate (VOLTAREN RESINAT) and colchicine and melperone HCl (EUNERPAN) and benazepril HCl/HCTZ (CIBADREX).

Information received on 22-JAN-2008 contained the following adverse experiences: agitation (onset date 16-JAN-2005); anxiety (onset date 16-JAN-2005); arthritic pains (onset date 19-JAN-2005); acute gout attack (onset date 19-JAN-2005); hyperuricaemia (onset date 31-JAN-2005) and sleep disturbance (onset date 24-JAN-2005).

Additional information is not expected.

Sender's comment:

Stevens-Johnson syndrome is a serious unlabeled event for therapy with ZOCOR®. This patient is multimorbid and was on a variety of concomitant medications. Whether ZOCOR® played any causative role in this scenario, cannot be assessed based on the available data. Self initiated measures are not indicated.

Reporter's comment:

GlaxoSmithKline, 07.02.2008 DE-GlaxoSmithKline-D0055997A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of toxic epidermal necrolysis in a 69-year-old male patient who received Amoxicillin trihydrate + potassium clavulanate (Augmentan) tablet over a period of 10

days for prophylaxis for infection.

The patient's past medical history included apoplexy. Concurrent medical conditions included cardiac infarction, edema and hyperlipidemia.

Co-suspect medications included diclofenac sodium (Voltaren resinat, given with interruptions), Heparin, sodium perchlorate (Irenat), metoprolol succinate (Beloc zok mite), torasemide (Unat), simvastatin (Zocor), carbimazole (Carbimazol), acetylsalicylic acid (ASS), lorazepam (Tavor), hydrochlorothiazide + ramipril (Delix plus) and Multiple other medication. Concurrent medications included Multiple medication.

On 14 January 2005 the patient started Amoxicillin trihydrate + potassium clavulanate (oral) at 875 mg three times per day. All co-suspect medication was started on the same or the next day. Amoxicillin trihydrate + potassium clavulanate was given until 23 January 2005.

Approximately 15 days after starting and six days after stopping Amoxicillin trihydrate + potassium clavulanate, on 29 January 2005, the patient experienced painful skin. On 31 January 2005 the patient developed itching, macule and conjunctivitis, on 01 February 2005 dysphagia, on 02 February 2005 lip erosion and oral mucosa erosion, on 04 February 2005 skin blister, on 05 February 2005 nikolsky's sign and on 06 February 2005 erosion of genital mucosa and nasal mucosa.

The patient was diagnosed with toxic epidermal necrolysis and stevens johnson syndrome.

The physician considered the events to be life threatening.

Additionally between 15 and 31 January 2005 treatment was started for pain, cardiac insufficiency, status post cardiac infarction, hypertension, possible pulmonary edema, hyperthyreosis, restlessness, fear, pain due to arthritis, gout episode, sleep disturbance and hyperuricaemia. The onset of these events was not reported.

At the time of reporting, the events were unresolved.

The reporting physician considered the events were possibly related to treatment with Amoxicillin trihydrate + potassium clavulanate and all co-suspect medication.

Sender's comment:

Auf Stevens-Johnson-Syndrom und toxische epidermale Nekrolyse wird in unsere Produktinformation zu Augmentan hingewiesen. Die Ursache der Ereignisse im vorliegenden Fall ist auf der Basis der zu dem Fall vorliegenden Informationen nicht zu beurteilen.

Aufgrund dieses Einzelfalles bleibt das Nutzen/Risikoverhaeltnis unveraendert und eigenverantwortliche Massnahmen sind derzeit nicht notwendig.

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

ROCHE, 13.02.2008 DE-ROCHE-545516

Sender diagnosis: not reported

Narrative

REGISCAR
INTENSIVE MONITORING STUDY

THIS CASE WAS REPORTED BY A PHYSICIAN AND CONCERNS A 69-YEAR-OLD MALE PATIENT WHO EXPERIENCED STEVENS-JOHNSON-SYNDROME AND TOXIC EPIDERMAL NECROLYSIS WHILST RECEIVING TORASEMIDE FOR HEART INSUFFICIENCY, AMOXICILLIN/CLAVULANIC ACID (AUGMENTAN) FOR INFECTION PROPHYLAXIS, HEPARIN SODIUM (HEPARIN) FOR EMBOLIEPROPH, SODIUM PERCHLORATE (IRENAT) FOR HYPERTHYROIDISM, METOPROLOL (BELOC-ZOK) FOR HYPERTENSION, SIMVASTATIN (ZOCOR) FOR HYPERLIPIDAEMIA, CARBIMAZOLE (CARBIMAZOL) FOR HYPERTHYROIDISM, ASPIRIN (ASS) FOR APOPLEXY, FLUCANAZOLE/LORAZEPAM/OXYBUTYNIN/SIMVASTATIN (TAVOR) FOR RESTLESSNESS/ANXIETY, HYDROCHLOROTHIAZIDE/RAMIPRIL (DELIX PLUS) FOR HYPERTENSION, DICLOFENAC (VOLTAREN) FOR PAIN OF ARTHRITIS, COLCHICINE (COLCHICIN) FOR GOUT ATTACK, MELPERONE (EUNERPAN) FOR SLEEPING DISORDER AND BENAZEPRIL/HYDROCHLOROTHIAZIDE (CIBADREX) FOR HYPERTENSION.

ON 14 JANUARY 2005, THE PATIENT COMMENCED ORAL AMOXICILLIN/CLAVULANIC ACID (875 MG THRICE DAILY) AND INTRAVENOUS HEPARIN SODIUM (25000 IE DAILY). THE SAME DAY, HE WAS HOSPITALISED. THE FOLLOWING DAY, HE COMMENCED ORAL SODIUM PERCHLORATE (120 DROPS), ORAL METOPROLOL (TWICE DAILY), ORAL TORASEMIDE (60 MG DAILY), ORAL SIMVASTATIN (20 MG ONCE DAILY), ORAL CARBIMAZOLE (ONCE DAILY) AND ORAL ASPIRIN (100 MG ONCE DAILY). ON 16 JANUARY 2005, HE COMMENCED ORAL FLUCANAZOLE/LORAZEPAM/OXYBUTYNIN/SIMVASTATIN (0.5 MG TWICE DAILY) AND ORAL HYDROCHLOROTHIAZIDE/RAMIPRIL (2.5 MG ONCE DAILY). ON 19 JANUARY 2005, ORAL DICLOFENAC (TWICE DAILY) AND ORAL COLCHICINE (1.2MG) WERE STARTED. THE FOLLOWING DAY, DICLOFENAC WAS DISCONTINUED. ON 22 JANUARY 2005, COLCHICINE WAS DISCONTINUED. THE FOLLOWING DAY, AMOXICILLIN/CLAVULANIC ACID WAS DISCONTINUED. ON 24 JANUARY 2005, ORAL MELPERONE (25 MG ONCE DAILY) WAS COMMENCED. THE FOLLOWING DAY, ORAL BENAZEPRIL/HYDROCHLOROTHIAZIDE (0.5 MG DAILY) WAS STARTED. ON 27 JANUARY 2005, DICLOFENAC AND COLCHICINE WERE RESTARTED. ON 28 JANUARY 2005, SODIUM PERCHLORATE WAS DISCONTINUED. THE FOLLOWING DAY, HE HAD SKIN PAIN. ON 30 JANUARY 2005, DICLOFENAC AND COLCHICINE WERE DISCONTINUED. ON 31 JANUARY 2005, THE PATIENT HAD HIS FIRST OCCURRENCE OF BLISTERS AND EROSIONS. HE EXPERIENCED LIFE-THREATENING STEVENS-JOHNSON SYNDROME AND TOXIC EPIDERMAL NECROLYSIS. HE EXPERIENCED PRURITUS, COCKADES AND SPOTS AND CONJUNCTIVITIS. THE SAME DAY, TORASEMIDE, HEPARIN SODIUM, METOPROLOL, SIMVASTATIN, CARBIMAZOLE, ASPIRIN, FLUCANAZOLE/LORAZEPAM/OXYBUTYNIN/SIMVASTATIN, HYDROCHLOROTHIAZIDE/RAMIPRIL, MELPERONE AND BENAZEPRIL/HYDROCHLOROTHIAZIDE WERE DISCONTINUED. ON 1 FEBRUARY 2005, HE HAD DYSPHAGIA. THE FOLLOWING DAY, HE HAD LIP AND ORAL MUCOSA EROSIONS (CO-MANIFESTATION). THE SAME DAY, THE PATIENT WAS DISCHARGED. ON 4 FEBRUARY 2005,

HE HAD SKIN BLISTERS. THE NEXT DAY, HE WAS NIKOLSKI POSITIVE. ON 6 FEBRUARY 2005, THE PATIENT HAD ENDOMETRIUM AND PARATHYROID GLAND EROSIONS.

AT THE TIME OF THIS REPORT, THE EVENTS WERE PERSISTING.

THE INVESTIGATOR ASSESSED THE EVENTS AS POSSIBLY RELATED TO THE USE OF TORASEMIDE, HEPARIN SODIUM, SODIUM PERCHLORATE, METOPROLOL, SIMVASTATIN, CARBIMAZOLE, ASPIRIN, FLUCANAZOLE/LORAZEPAM/OXYBUTYNIN/SIMVASTATIN, HYDROCHLOROTHIAZIDE/RAMIPRIL, COLCHICINE, MELPERONE AND BENAZEPRIL/HYDROCHLOROTHIAZIDE. THE INVESTIGATOR ASSESSED THE DRUG-EVENT RELATIONSHIP FOR AMOXICILLIN/CLAVULANIC ACID AND DICLOFENAC AND THE EVENTS TO BE VERY WELL POSSIBLE.

NO FURTHER INFORMATION WAS AVAILABLE.

Sender's comment:

CIOMS TEXT: STEVENS-JOHNSON-SYNDROME AND TOXIC EPIDERMAL NECROLYSIS ARE NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (CDS) FOR TORASEMIDE. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN THE REPORTED EVENTS AND TREATMENT WITH TORASEMIDE IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM.

HOWEVER, POSSIBLE ALTERNATIVE EXPLANATIONS INCLUDES CO-SUSPECTS: CARBIMAZOL, ASPIRIN AND DICLOFENAC SODIUM AND NON-SUSPECTS: AMIODARONE AND ALLOPURINOL.; ***LOCAL ASSESSMENT: INTERVIEW NR. 9160253

STEVENS-JOHNSON-SYNDROME AND TOXIC EPIDERMAL NECROLYSIS = NOT LABELED

ALTERNATIVE EXPLANATIONS = CARBIMAZOL, ASPIRIN AND DICLOFENAC AND AMIODARONE AND ALLOPURINOL

EINZELFALLBEOBACHTUNG. KEIN HANDLUNGSBEDARF.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08008924

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 46 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20031224	20040126	34 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 BUSCOPAN hyoscine butylbromide	Rectal	/ 242	20031215-20031225 11 Day	Spasms
2 COTRIMHEXAL FORTE sulfamethoxazole; trimethoprim	Oral	/Day 245	-	Urinary tract infection
3 diclac diclofenac sodium	Rectal	100 Mg milligram(s)/Day 242	20031209-20031213 5 Day	Pain
4 FRAGMIN dalteparin	Subcutaneous	/	-	Thrombosis prophylaxis
5 herbalife products herbal extracts nos	Oral	/	20031218-20031225 8 Day	
6 METOCLOPRAMIDE metoclopramide	Oral	60 Gtt drop(s)/Day	20031202-20031215 14 Day	Nausea
7 Multivitamine ascorbic acid; ergocalciferol; folic acid; nicotinamide; panthenol; retinol; riboflavin; thiamine hydrochloride	Oral	/	20031215-20031225 11 Day	
8 PASPERTIN metoclopramide hydrochloride	Oral	/	20031201-20031201 -	Nausea
9 solidago virgaurea solidago virgaurea	Oral	/	20031218-20031225 8 Day	

Drugs (concomitant)

10	AMBROHEXAL ambroxol hydrochloride	Intravenous (not otherwis /	20031128-20031201 4 Day
11	amino acids nos amino acids nos	Intravenous (not otherwis /	20031128-20031201 4 Day
12	catapresan clonidine	Intravenous (not otherwis /	20031128-20031130 3 Day
13	clont metronidazole	Intravenous (not otherwis /	20031128-20031130 3 Day
14	DIPIDOLOR piritramide	Intravenous (not otherwis /	20031128-20031130 3 Day
15	dulcolax bisacodyl	Rectal /	20031128-20031128 -
16	glucose glucose	Intravenous (not otherwis /	20031128-20031201 4 Day
17	kalinor potassium chloride	Oral /	20031201-20031201 -
18	Kaliumchlorid potassium chloride	Intravenous (not otherwis /	20031128-20031129 2 Day
19	KONAKION phytomenadione	Intravenous (not otherwis /	20031129-20031130 2 Day
20	LASIX furosemide	Intravenous (not otherwis /	20031128-20031129 2 Day
21	Natriumchlorid sodium chloride	Intravenous (not otherwis /	20031128-20031205 8 Day
22	NOVALGIN metamizole	Oral /	20031128-20031208 11 Day
23	NOVALGIN metamizole	/	20031202-20031203 2 Day
24	NOVALGIN metamizole	Intravenous (not otherwis /	20031202-20031203 2 Day
25	PLANUM temazepam	Oral /	20031205-20031210 6 Day
26	RANITIC ranitidine hydrochloride	Intravenous (not otherwis /	20031128-20031201 4 Day
27	sterofundin electrolytes nos	Intravenous (not otherwis /	20031128-20031203 6 Day
28	tavanic levofloxacin	Intravenous (not otherwis /	20031128-20031206 9 Day
29	tavor lorazepam	Oral /	20031201-20031201 -
30	TRAMAL tramadol hydrochloride	Oral /	20031202-20031208 7 Day
31	TRAMAL tramadol hydrochloride	Intravenous (not otherwis /	20031202-20031203 2 Day
32	Vitamine nnb vitamins nos	Intravenous (not otherwis /	20031130-20031130 -

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Peritonitis	20031124		No	Purulent peritonitis

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

PFIZER INC, 28.01.2008 DE-PFIZER INC-2008005518

Sender diagnosis: not reported

Narrative

This is a report from the German Documentation Centre of Severe Skin Reactions, a sponsored project set up to collect suspected drug induced severe skin reactions leading to hospital admission.

This 46-year-old female patient was hospitalized on 24Nov2003 due to purulent peritonitis. She was discharged on 15Dec2003. During hospitalization, she was treated with Fragmin P (dalteparin) due to prophylaxis of thrombosis (from 28Nov2003 to 15Dec2003), CotrimHEXAL Forte (once on 16Dec2003), Paspertin (on 01Dec2003), MCP (metoclopramide) (from 02Dec2003 to 15Dec2003), Diclac (from 09Dec2003 to 13Dec2003), Buscopan (from 15Dec2003 to 25Dec2003), Nephrisol mono (solidago vigaurea; from 18Dec2003 to 25Dec2003), unspecified diuretic tea (from 18Dec2003 to 25Dec2003), an unspecified contraceptive injection (start date unknown, ongoing), and an unspecified multivitamin effervescent tablet (from 15Dec2003 to 25Dec2003). Other concomitant medications in the period from 28Nov 2003 to 26Dec2003 included: Dulcolax, Lasix, KCl, Dipidolor, Clont, Catapresan, AmbroHEXAL, glucose, Ranitic, aminoacids NOS, Sterofundin, NaCl, Tavanic, Novalgin, Konakion, vitamins NOS, Tavor, Kalinor, Tramal and Planum. On 24Dec2003, she developed pruritus and pain of her skin. On 25Dec2003, she presented reddening of her eyes and swelling of her lips, and on 26Dec2003, fever up to 38.0° C. Diagnosis of Erythema exsudativum multiforme (EEM) was made, and she was hospitalized on 26Dec2003. As documented, the EEM lasted until 26Jan2004. The outcome of the EEM was reported as recovering. Per documentation the experts of the documentation centre assessed the causal relationship as "probable" with Cotrim forte, and as "possible" with Fragmin P, Paspertin, metoclopramide, Diclac, Buscopan, Nephrisol mono and the unspecified diuretic tea (consisting of betulae folium, flor. calendulae, herba equiseti, fruc. foeniculi and rhiz. graminis), and as "questionable" with the unspecified contraceptive injection and the multivitamin effervescent tablets.

Company Clinical Evaluation:

The event occurred within a plausible temporal sequence since the start of the Fragmin administration. However, an independent causality assessment is not possible since the multiple concomitant medications administered may have contributed to the onset of the reported event.

Sender's comment:Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08009364

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Nasal ulcer	20041030				Unknown
2 Oral ulceration	20041030				Unknown
3 Oral mucosa erosion	20041103			11 Day	not recovered/not resolved
4 Erythroderma	20041107			15 Day	not recovered/not resolved
5 Fever	20041028			5 Day	not recovered/not resolved
6 Rash	20041030			7 Day	not recovered/not resolved
7 Stevens Johnson syndrome	20041030			7 Day	not recovered/not resolved
8 Macule	20041030				Unknown
9 Abrasions	20041103			11 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc acetylcysteine	Intravenous (not otherwis /	20041023-	Hyperuricemia
2	aciclovir aciclovir	Intravenous (not otherwis /	20041025-20041027 3 Day	
3	ALLOPURINOL allopurinol	Oral /	20041024-20041028 5 Day	Acute renal failure
4	altinsulin insulin	Intravenous (not otherwis 50 lu international unit(s)/l	20041023-	
5	ARTERENOL norepinephrine hydrochloride	Intravenous (not otherwis /	20041025-20041030 6 Day	
6	ass acetylsalicylic acid	Oral /	20041024-20041101 9 Day	
7	atrovent ipratropium bromide	Respiratory (inhalation) /	20041030-	COPD
8	atrovent ipratropium bromide	Respiratory (inhalation) /	20041023-	COPD
9	BENALAPRIL enalapril	Oral /	20020101-20041023 2 Year	Hypertension
10	CEFUROXIM cefuroxime sodium	Intravenous (not otherwis /	20041027-20041031 5 Day	Rash
11	clinomel amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; lipids nos	Intravenous (not otherwis /	20041025-20041025 1440 Minute	Nutritional supplement
12	clinomel amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; lipids nos	Intravenous (not otherwis /	20041030-	Nutritional supplement
13	diclofenac diclofenac	Oral /	20020101-	Pain
14	DOBUTAMIN dobutamine hydrochloride	Intravenous (not otherwis /	20041029-	
15	EBRANTIL urapidil	Intravenous (not otherwis /	20041023-20041023 1440 Minute	Hypertension
16	EUPHYLONG theophylline	Oral /	20041025-20041025 1440 Minute	COPD
17	EUPHYLONG theophylline	Oral /	20041023-20041023 1440 Minute	COPD
18	fenta fentanyl	Intravenous (not otherwis /	20041023-20041023 1440 Minute	Pain
19	fresubin carbohydrates nos; fatty acids nos; minerals nos; proteins nos; vitamins nos	Oral /	20041026-20041030 5 Day	Nutritional supplement
20	FRISIUM clobazam	Oral /	20041023-20041023 1440 Minute	Sedation
21	FUROSEMIDE furosemide	Intravenous (not otherwis /	20041027-20071029 3 Year	Nausea
22	FUROSEMIDE furosemide	Intravenous (not otherwis 60 Mg milligram(s)/Day	20041024-20041025 2 Day	Nausea
23	glucose glucose	Intravenous (not otherwis /	20041020-20041101 13 Day	
24	haes-steril hetastarch; sodium chloride	Intravenous (not otherwis /	20041023-20041023 1440 Minute	
25	haes-steril hetastarch; sodium chloride	Intravenous (not otherwis /	20041028-20041031 4 Day	
26	HEPARIN heparin	Intravenous (not otherwis /	20041023-	Thrombosis prophylaxis
27	Hydrokortison hydrocortisone	Intravenous (not otherwis /	20041030-	

			-	
28	kcl potassium chloride	Intravenous (not otherwis /	20041023-20041028	Hypokalemia 6 Day
29	KLACID clarithromycin	Oral /	20041027-20041030	Rash 4 Day
30	klean-prep macrogol; potassium chloride; sodium bicarbonate; sodium chloride; sodium sulfate anhydrous	Intravenous (not otherwis /	20041103-20041103	Bowel discomfort 1440 Minute
31	Natriumbikarbonat sodium bicarbonate	Intravenous (not otherwis /	-	-
32	Natriumbikarbonat sodium bicarbonate	Intravenous (not otherwis /	20041102-20041103	2 Day
33	NORADRENALINE norepinephrine	Intravenous (not otherwis /	20041023-20041023	1440 Minute
34	NORADRENALINE norepinephrine	Intravenous (not otherwis /	20041029-	-
35	OPTIPECT KODEIN codeine	Oral /	20020101-	COPD -
36	ORFIRIL valproic acid	Intravenous (not otherwis /	20041023-20041023	Epilepsy 1440 Minute
37	panthenol panthenol	Intravenous (not otherwis /	20041029-	-
38	PANTOZOL pantoprazole	Oral 1 Df dosage form/	20041024-20041028	Prophylaxis 5 Day
39	PANTOZOL pantoprazole sodium	Intravenous (not otherwis / 231	20041023-20041023	Prophylaxis 1440 Minute
40	PARACETAMOL paracetamol	Rectal /	20041024-20041025	Pain 2 Day
41	PASPERTIN metoclopramide hydrochloride	Intravenous (not otherwis /	20041029-	Infection -
42	PHENYTOIN phenytoin	Intravenous (not otherwis /	20041025-20041025	Epilepsy 1440 Minute
43	PHENYTOIN phenytoin	Oral /	20041026-20041028	Epilepsy 3 Day
44	PHENYTOIN phenytoin	Intravenous (not otherwis /	20041028-20041102	Epilepsy 6 Day
45	PRES enalapril maleate	Oral /	20041023-20041023	Renal failure 1440 Minute
46	PROPOFOL propofol	Intravenous (not otherwis /	20041023-20041027	Sedation 5 Day
47	prostigmin neostigmine bromide	Intravenous (not otherwis /	20041029-	-
48	salbutamol salbutamol	/	20041023-	COPD -
49	SOLOSIN theophylline	Oral /	20041025-20041027	COPD 3 Day
50	SYNACTHEN tetracosactide	Intravenous (not otherwis /	20041030-20041030	Epilepsy 1440 Minute
51	tavor lorazepam	Intravenous (not otherwis 1 Mg milligram(s)/Day	20041023-20041025	Sedation 3 Day
52	tazobac piperacillin sodium; tazobactam sodium	Intravenous (not otherwis /	20041030-20041031	Rash 2 Day
53	thomaejonin calcium chloride dihydrate; magnesium chloride hexahydrate; potassium chloride; sodium acetate; sodium chloride	Intravenous (not otherwis /	20041023-20041029	7 Day

54	tracutil minerals nos	Intravenous (not otherwis /	20041030-	
55	trapanal thiopental sodium	Intravenous (not otherwis / 231	20041026-20041031	Sedation 6 Day
56	VALPROATE SODIUM valproate sodium	Intravenous (not otherwis /	20041023-	Epilepsy
57	vancomycin vancomycin	Intravenous (not otherwis /	20041029-20041029	Rash 1440 Minute
58	XANEF enalapril maleate	Oral /	20041024-20041027	Renal failure 4 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Epilepsy			Unknown	
Status epilepticus			Unknown	
Increased bronchial secretion			Unknown	
Enteroclysis			Unknown	
Blood sugar level fluctuation			Unknown	
Volume depletion			Unknown	
Poor peripheral circulation			Unknown	
Hypokalemia			Unknown	
Hyperuricemia			Unknown	
Rhinitis			Unknown	
Renal insufficiency			Unknown	
Pruritus			Unknown	
Hypertension			Unknown	
Pain			Unknown	
Nausea			Unknown	
Infection			Unknown	
Exanthema			Unknown	
COPD			Unknown	
Cardiac insufficiency			Unknown	
Bacteraemia			Unknown	
Acute renal failure			Unknown	
Pneumonia			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

009507513, 29.01.2008 DE-MERCK-0801DEU00104

Sender diagnosis: not reported

Narrative

Information has been received from a registry, documenting at least 95% of all serious skin reactions that occur in Germany, concerning a 70-year-old hospitalized male patient with arterial hypertension, chronic obstructive pulmonary disease, pain, epilepsy, status epilepticus (reason for hospitalization on 23-OCT-2004), renal insufficiency, hypokalaemia, blood glucose fluctuation, cardiovascular instability, increased bronchial secretion, hyperuricaemia, acute renal failure, infection and nausea who, on 24-OCT-2004, was placed on therapy with enalapril maleate, 5 mg tablet, 5-10 mg daily for the treatment of renal insufficiency.

Prior and concomitant suspect secondary therapy included enalapril maleate (PRES, licensee), tablet, 1.5 mg once given on 23-OCT-2004 for the treatment of renal insufficiency, enalapril maleate (BENALAPRIL, NOD), 1 unit twice a day po for the treatment of arterial hypertension (01-JAN-2002 until 23-OCT-2004), diclofenac taken as needed for the treatment of pain (dose not reported; 01-JAN-2002 until 23-OCT-2004), valproate sodium

(ORFIRIL), injection, 600 mg iv given once on 23-OCT-2004, for the treatment of status epilepticus and valproate sodium injection 400 - 5000 mg/h for the treatment of epilepsy (started on 23-OCT-2004, stop date not reported), urapidil (EBRANTIL), injection, 250 mg/h iv for the treatment of arterial hypertension (on 23-OCT-2004), nitrendipine (BAYOTENSIN), injection, 1 unit iv once on 23-OCT-2004 for the treatment of arterial hypertension, fentanyl citrate (FENTA), injection, 2.5 mg/h iv on 23-OCT-2004 for the treatment of pain, clobazam (FRISIUM), injection, 1 unit iv once on 23-OCT-2004 for sedation, lorazepam (TAVOR), injection, 1 mg once iv on 23-OCT-2004 and on 25-OCT-2004 for sedation, propofol, injection, 1000 mg/h iv from 23-OCT-2004 until 27-OCT-2004 for sedation, pentastarch (HAES-STERIL), infusion, given iv on 23-OCT-2004 and from 28-OCT-2004 until 31-OCT-2004 for fluid replacement (dose not reported), insulin, injection, 60 IU/h iv for the treatment of blood glucose fluctuation (started on 23-OCT-2004, stop date not reported), norepinephrine hydrochloride, injection, 5-25 mg/h iv given on 23-OCT-2004 and from 29-OCT-2004 onwards (stop date not reported) for the treatment of cardiovascular instability, heparin, injection, 2500 IE/h iv as thrombosis prophylaxis (started on 23-OCT-2004, stop date not reported), acetylcysteine, injection, 300 mg iv given daily for the treatment of increased bronchial secretion (stop date not reported), ipratropium bromide (ATROVENT), inhalant, for the treatment of COPD (dose not reported) from 23-OCT-2004 until 27-OCT-2004 and from 30-OCT-2004 onwards (stop date not reported), albuterol, inhalant, for the treatment of COPD (dose not reported, started on 23-OCT-2004, stop date not reported), pantoprazole sodium (PANTOZOL), injection, given iv on 23-OCT-2004 and from 28-OCT-2004 onwards as gastric ulcer prophylaxis (dose and stop date not reported) and pantoprazole sodium (PANTOZOL), 1 unit daily po from 24-OCT-2004 until 28-OCT-2004 as gastric ulcer prophylaxis, theophylline (EUPHYLONG), tablet, 575 mg given po on 24-OCT-2004, 375 mg theophylline given po 25-OCT-2004 and theophylline (SOLOSIN), solution, 40 drops given po daily (from 26-OCT-2004 until 27-OCT-2004), all three for the treatment of COPD, acetaminophen, suppository, given from 24-OCT-2004 until 25-OCT-2004 for the treatment of pain (dose not reported), allopurinol, 300 mg daily po from 24-OCT-2004 until 28-OCT-2004 for the treatment of hyperuricaemia, furosemide (FURO), injection, 20 mg iv three times a day (from 24-OCT-2004 until 25-OCT-2004) and 500 mg/h iv given (from 27-OCT-2004 until 29-OCT-2004) for the treatment of acute renal failure, aspirin, 100 mg po daily from 24-OCT-2004 until 01-NOV-2004 (indication not reported), acyclovir, injection, 500 mg iv given from 25-OCT-2004 until 27-OCT-2004 (indication not reported), phenytoin, injection, 750 mg/h iv given on 25-OCT-2004 and from 28-OCT-2004 onwards for the treatment of epilepsy (stop date not reported and phenytoin, 350 mg daily po from 26-OCT-2004 until 28-OCT-2004 for the treatment of epilepsy and thiopental sodium (TRAPANAL), injection, 1000-2500 mg/h iv given from 26-OCT-2004 until 01-NOV-2004 for sedation.

Other concomitant therapy included codeine, potassium chloride, dextrose (+) sodium chloride (+) sodium lactate (+) sodium acetate (+) calcium chloride (THOMAEJONIN), amino acids (unspecified) (+) dextrose (+) electrolytes (unspecified) (+) lipids (unspecified) (CLINOMEL), nutritional supplements (FRESUBIN), clarithromycin (KLACID), cefuroxime, vancomycin, dobutamine, neostigmine bromide (PROSTIGMIN), metoclopramide hydrochloride (PASPERTIN), panthenol, cosyntropin (SYNACTHEN), piperacillin sodium (+) tazobactam sodium (TAZOBAC), dextrose, sodium bicarbonate, hydrocortisone, minerals (unspecified) (TRACUTIL) and human albumin human.

On 27-OCT-2004, therapy with enalapril maleate was stopped.

On 28-OCT-2004, the patient developed fever and, on 30-OCT-2004, spots, on 04-NOV-2004, tongue erosions, on 05-NOV-2004, erosions of the nasal mucosa, on 06-NOV-2004, erosions of the oral mucosa and on 07-NOV-2004, generalized erythrodermia. Final diagnosis was Stevens-Johnson syndrome. Further course of events was not provided. The patient was reported as not recovered, however, a stop date of the AE was specified with 21-DEC-2004.

The reporting physician felt that Stevens-Johnson syndrome was possibly related to therapy with enalapril maleate (MSD) and/or enalapril maleate (PRES, licencee) and/or enalapril maleate (BENALAPRIL, NOD) and/or diclofenac and/or valproate sodium and/or urapidil (EBRANTIL), and/or nitrendipine (BAYOTENSIN) and/or fentanyl citrate (FENTA) and/or clobazam (FRISIUM and/or lorazepam (TAVOR) and/or propofol, and/or pentastarch (HAES-STERIL), and/or insulin, and/or norepinephrine hydrochloride and/or heparin and/or acetylcysteine and/or ipratropium bromide (ATROVENT), and/or albuterol, and/or pantoprazole sodium iv and po and/or theophylline (EUPHYLONG), tablet and solution and/or furosemide (FURO) and/or aspirin, and/or acyclovir and/or thiopental sodium (TRAPANAL) and that Stevens-Johnson syndrome was probably related to allopurinol and/or phenytoin.

Additional information is not expected.

Upon internal review, Stevens-Johnson syndrome was determined to be an Other Important Medical Event.

Sender's comment:

Stevens-Johnson-syndrome is a serious labeled event for therapy with XANEF°. In this setting of severe underlying conditions and multiple drug therapy the role of enalapril cannot be rated. The safety profile of the drug remains unchanged. Self-initiated measures are not indicated.

Reporter's comment:

Sender diagnosis: not reported

Narrative

Initial report

This case of Stevens-Johnson syndrome (SJS) was received from the German health authority (DE-BFARM-08017209). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9160240). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of SJS was made by an expert panel who assessed it as a "probable case" of SJS. Causal relationship was assessed by the experts in Mar. 2006 using an algorithm.

As for norepinephrine (Arterenol), theophylline (Solosin) and clobazam (Frisium) the causality was assessed as "possible". For the other suspected drugs causality was assessed as:

"probable": allopurinol, phenytoin

"possible": diclofenac, valproinic acid, urapidil, nitrendipine, enalapril, fentanyl, lorazepam, propofol, polyhydroxyethyl starch + sodium chloride, insulin, heparin sodium, acetylcysteine, ipatropium bromide, salbutamol, pantoprazole, theophylline (from another company), paracetamol, furosemide, acetylsalicylic acid, aciclovir, thiopental

Not suspected were: enalapril, codeine, potassium chloride, electrolyte infusion, norepinephrine (of other company), soya oil + glucose + calcium chloride, proteins + fats + carbon hydrates + vitamins + trace elements, clarithromycin, cefuroxime, vancomycin, dobutamine, neostigmin, metoclopramide, dexpanthenol, tetracosactide, piperacillin + tazobactam, glucose, sodium, hydrocortisone, trace elements, retinol + colecalciferol + alpha-tocopherol, albumin, dimetindene, blood components nos, metamizole, amidotrizoate, imipenem + cilastatin, amiodarone, metoprolol, magnesium salt, levetiracetam, xylometazoline, electrolytes.

Pneumonia and bacteremia were considered possible cause of SJS.

Epicrisis:

Patient: male, 70 years

Medical history: arterial hypertension, epilepsy, renal insufficiency, chronic obstructive pulmonary disease, hyperuricemia, hypokalemia, pneumonia, bacteremia.

The patient was hospitalised on 23-Oct-2004 due to epileptic state.

He was treated with clobazam (one tablet) on 23-Oct-2004 for tranquillisation, with theophylline 40 drops/day from 25-Oct-2004 to 27-Oct-2004 for chronic obstructive pulmonary disease, and with norepinephrine 5mg/hour from 25-Oct-2004 to 30-Oct-2004 for stabilisation for circulation. Before and during hospitalisation the patient was also treated with a lot of other drugs.

On 28-Oct-2004 the patient developed fever (not considered onset of event).

On 30-Oct-2004 generalised maculae were detected.

On 03-Nov-2004 the patient developed erosions of his tongue, on 04-Nov-2004 erosions of his nasal mucosa and on 06-Nov-2004 erosions of his oral mucosa.

On 07-Nov-2004 generalised erythrodermia was diagnosed.

The patient had not yet recovered on 21-Dec-2004.

Sender's comment:

29-Feb-2008 Sanofi Aventis Company Comment:

The case report presented here is properly documented, and the temporal relationship between the event and administration of norepinephrine hydrochloride, theophylline and clobazam seems plausible. Dechallenge and rechallenge is not applicable as concomitant medications were altered at the same time. In addition, concomitant drugs as reported here and/or pneumonia and bacteremia may be considered an alternative cause to explain the event.

29-FEB-2008 Sanofi Aventis Germany: STEVENS-JOHNSON SYNDROME is considered locally unlabelled for the suspected products. The facts presented here, however, are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the products. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

Wyeth-Pharma GmbH, 07.03.2008 DE-WYE-G01144608

Sender diagnosis: not reported

Narrative

Information was received from a registry via a regulatory authority regarding a 70-year-old male patient who received Tavor (lorazepam injection) therapy and experienced Stevens-Johnson syndrome.

MEDICAL HISTORY:

The patient's concurrent illnesses include epilepsy, cardiac failure and chronic obstructive pulmonary disease.

PRODUCT DETAILS:

Indication for Tavor was restlessness. The dose regimen was 1 mg 1 time per day (intravenous) from 23-Oct-2004 to 23-Oct-2004 and then was 1 mg 1 time per day (intravenous) from 25-Oct-2004 to 25-Oct-2004. Therapy was permanently discontinued.

Additional suspect medication included enalapril, Optipect Kodein (codeine), diclofenac, Orfiril (valproate sodium), Ebrantil (urapidil), Bayotensin (nitrendipine), Pres (enalapril maleate), fentanyl, Frisium (clobazam), propofol, potassium chloride, Haes-Steril (hydroxyethylcellulose/sodium chloride), Insulin (unspec), valproate sodium, Noradrenaline (norepinephrine), Heparin Sodium, acetylcysteine, Atrovent (ipratropium bromide), salbutamol, Pantozol (pantoprazole injection), Euphyllong (theophylline), paracetamol, Xanef (enalapril maleate), allopurinol, Pantozol (pantoprazole tablet, delayed release), Furo (furosemide), acetylsalicylic acid, theophylline, aciclovir, Solosin (theophylline), Arterenol (norepinephrine hydrochloride), phenytoin, Clinomel (amino acids/carbohydrates/minerals/vitamins), Fresubin (carbohydrates nos/fatty acids nos/minerals nos/proteins nos/vitamins nos), Trapanal (thiopental sodium), Klacid (clarithromycin), cefuroxime, Thomaejonin (calcium chloride dihydrate/magnesium chloride hexahydrate/potassium chloride/sodium acetate/sodium chloride), vancomycin, dobutamine, Prostigmin (neostigmine metilsulfate injection), Paspertin (metoclopramide hydrochloride), panthenol and Synacthen (tetracosactide). Wyeth Pharma is not the marketing authorisation holder for Pantozol (pantoprazole) in Germany.

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient had already been hospitalized since 23-Oct-2004 because of status epilepticus. He experienced fever (non-serious) on 28-Oct-2004 and Stevens-Johnson syndrome (Stevens-Johnson syndrome) on 30-Oct-2004.

The event was described as follows: 'On 30-Oct-2004 the patient experienced generalized blotchy changes of skin. On 03-Nov-2004 he experienced erosions of tongue. On 04-Nov-2004 he experienced erosions of nasal mucous membrane. On 06-Nov-2004 the patient experienced erosions of oral mucous membrane. On 07-Nov-2004 the patient experienced generalized erythrodermia.'

The patient received antibiotic treatment with Tazobac (piperacillin/ tazobactam) from 30-Oct-2004 to 31-Oct-2004 because of an infection. On 01-Nov-2004 he developed pneumonia and he received Zienam (cilastatin sodium/ imipenem) therapy. His exanthema was treated with hydrocortisone starting on 30-Oct-2004 and additionally he received Fenistil (dimetindene maleate) one time on 31-Oct-2004 for pruritus. Metamizol was administered for pain relief on 01-Nov-2004. The patient had to be reanimated on 02-Nov-2004 wherefore he received Cordarex (amiodarone hydrochloride) and Beloc (metoprolol tartrate) for his heart insufficiency. His epilepsy was treated with Keppra (levetiracetam) from 02-Nov-2004 and Otriven (xylometazoline hydrochloride) was administered for rhinitis. An X-ray of the gastrointestinal tract was performed on 01-Nov-2004 wherefore he received Gastrografin (meglumine amidotrizoate/sodium amidotrizoate). On 03-Nov-2004 a colonic irrigation was carried out. Further treatment included nutritional supplement and electrolyte substitutional therapy.

The patient has not recovered yet.

TEST RESULTS:

Body temperature (results: Body temperature increased) was done on 28-Oct-2004. X-ray gastrointestinal tract (results: unknown) was done on 01-Nov-2004.

RELATEDNESS ASSESSMENT:

The dermatological review committee has categorized Tavor as possibly related to skin reactions. Tavor has an assessment of 5 (range 1 stands for certain relationship to event, range 8 unlikely relationship). The same assessment was provided for diclofenac, Orfiril (valproate sodium), Ebrantil (urapidil), Bayotensin (nitrendipine), Pres (enalapril maleate), fentanyl, Frisium (clobazam), propofol, Haes-Steril (hydroxyethylcellulose/sodium chloride), Insulin (unspec), valproate sodium, heparin sodium, acetylcysteine, Atrovent (ipratropium bromide), salbutamol, Pantozol (pantoprazole injection), Euphyllong (theophylline), paracetamol, Xanef (enalapril maleate), Pantozol (pantoprazole tablet, delayed release), Furo (furosemide), acetylsalicylic acid, theophylline, aciclovir, Solosin (theophylline), Arterenol (norepinephrine hydrochloride) and Trapanal (thiopental sodium). The dermatological review committee categorized phenytoin and allopurinol as likely related to Stevens-Johnson syndrome. All other suspect drugs got a lower degree of probability. According to dermatological review committee the drugs given for treatment since 30-Oct-2004 have no temporal relationship to reaction or are not known for causing Stevens-Johnson syndrome.

Sender's comment:

WYETH COMMENT

INITIAL (v1) 28-Feb-2008

STEVENS JOHNSON SYNDROME unter Tavor

Bericht vom Dokumentationszentrum für schwere Hautreaktionen in Freiburg.

Ein 70-jähriger Patient wurde auf Grund eines Status epilepticus am 23-Oct-2004 stationär aufgenommen. Zur Beruhigung erhielt der Patient am 23-Oct-2004 und 25-Oct-2004 Tavor Injektionslösung. Am 30-Oct-2004 zeigte der Patient dann Stevens Johnson Syndrom, zunächst durch generalisierte Flecken, dann mit Erosionen von Zunge, Nasen- und Mund-Schleimhaut und schließlich durch generalisierte Erythrodermie. Der Patient wurde noch nicht wiederhergestellt. Der Zusammenhang zwischen dem Stevens Johnson Syndrom und Tavor wurde nach dem Bewertungsalgorithmus des Dokumentationszentrums als möglich beurteilt.

Ein möglicher Zusammenhang kann nicht ausgeschlossen werden.

Stevens Johnson Syndrom ist in der Fachinformation nicht aufgeführt und wurde daher als unbekannt eingestuft.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Birte Kruse

Michael Pöger

Reporter's comment:

ratiopharm GmbH, 07.03.2008 DE-RATIOPH-2008R021838

Sender diagnosis: not reported

Narrative

Reported terms: Stevens-Johnson-Syndrome(Stevens-Johnson syndrome)

Case reference number 2008R021838 is a spontaneous case report from the RegiSCAR-Studie, sent by the Dokumentationszentrum für schwere Hautkrankheiten (dZh), Uniklinik Freiburg, Germany. It refers to a male aged 70 years.

The patient's past medical history included Enterocolitis (Enterocolitis). There is no information about risk factors. Concomitant diseases include Pain (Pain), COPD (Chronic obstructive pulmonary disease), Hypertension arterial (Hypertension), Status epilepticus (Status epilepticus), Hypertension (Hypertension), Blood sugar level fluctuation (Blood glucose fluctuation), Poor peripheral circulation (Poor peripheral circulation), Increased bronchial secretion (Increased bronchial secretion), Pneumonia (Pneumonia), Rhinitis (Rhinitis), Hypokalemia (Hypokalaemia), Acute renal failure (Renal failure acute), Epilepsy (Epilepsy), Volume depletion (Hypovolaemia), Renal insufficiency (Renal failure), Hyperuricemia (Hyperuricaemia) and Infection (Infection). No prior drugs are known to have been taken by the patient.

The patient started taking Theophyllin retard-ratiopharm 375 mg (Retardkapseln) (THEOPHYLLINE), indication COPD on 25-Oct-2004 and Aciclovir-ratiopharm 500 mg p.i. (Durchstechflaschen) (ACICLOVIR), indication unknown on 25-Oct-2004. The patient has been exposed to the following concomitant drug(s): Orfiril InjLsg 600mg (VALPROIC ACID), indication Status epilepticus, Optipect Kodein forte (CODEINE), indication COPD, Diclofenac (DICLOFENAC), indication Pain, Ebrantil InjLsg 250mg/h (URAPIDIL), indication Hypertension, Bayotensin InjLsg Akut (NITRENDIPINE), indication Hypertension, Benalaprill (ENALAPRIL), indication Hypertension arterial, Pres 1.5mg (ENALAPRIL), indication Renal insufficiency, Fenta InjLsg 2.5mg/h (FENTANYL), indication Pain, Paspertin (METOCLOPRAMIDE), indication Nausea, Prostigmin (NEOSTIGMINE), indication unknown, ACC InjLsg 3000mg (ACETYLCYSTEINE), indication Increased bronchial secretion, Paracetamol Supp b.B. (PARACETAMOL), indication Pain, Allopurinol (ALLOPURINOL), indication Hyperuricemia, Cernevit (VITAMINS), indication Nutritional support, Albumin (ALBUMIN), indication Protein abnormal NOS, Zienam (IMIPENEM AND ENZYME INHIBITOR), indication Pneumonia, Metamizol (METAMIZOLE SODIUM), indication Pain, Blutproduktinf. (BLOOD AND RELATED PRODUCTS), indication Drug therapy, Mg-Mineralien (MAGNESIUM OROTATE), indication Drug therapy, Furo InjLsg 20mg (FUROSEMIDE), indication Acute renal failure, Otriven (XYLOMETAZOLINE), indication Rhinitis, ASS 100mg (ACETYLSALICYLIC ACID), indication Boost of blood flow, Solosin Tropfen (THEOPHYLLINE), indication COPD, KleanPrep (MACROGOL, COMBINATIONS), indication Enterocolitis, Frisium (CLOBAZAM), indication Sedation, Propofol InjLsg 1000mg/h (PROPOFOL), indication Sedation, Heparin InjLsg 2500 IE/h (HEPARIN), indication Thrombosis prophylaxis, HAES InfLsg 10% (BLOOD SUBSTITUTES AND PLASMA PROTEIN FRACTIONS), indication Volume depletion, Panthenol (DEXPANTHENOL), indication Drug therapy, Clacid 250mg (CLARITHROMYCIN), indication Infection, Atrovent Inhalat LSO 0.25% (IPRATROPIUM BROMIDE), indication COPD, Pantozol InjLsg (PANTOPRAZOLE), indication Gastritis prophylaxis, Clinomel InfLsg 2.2% 30ml (ALL OTHER NON-THERAPEUTIC PRODUCTS), indication Parenteral nutrition, Salbutamol Inhalat (SALBUTAMOL), indication COPD, Tazobac (PIPERACILLIN AND ENZYME INHIBITOR), indication Infection, Trapanal InjLsg 1000-2500 mg/h (THIOPENTAL), indication Sedation, Thomaëjonin InfLsg HG5 (ELECTROLYTES WITH CARBOHYDRATES), indication Volume depletion, Glucose (CARBOHYDRATES), indication Drug therapy, Beloc (METOPROLOL), indication Cardiac insufficiency, KCl InfLsg 14.9% 100mmol/h (POTASSIUM CHLORIDE), indication Hypokalemia, Valproat InjLsg 400-5000 mg/h (VALPROIC ACID), indication Epilepsy, Tavor InjLsg 1mg (LORAZEPAM), indication Sedation, Altinsulin InjLsg 50IE/h (INSULIN (HUMAN)), indication Blood sugar level fluctuation, Noradrenalin InjLsg 5mg/h, 25 mg/h (NOREPINEPHRINE), indication Stabilisation of circulation, Dobutamin (DOBUTAMINE), indication Stabilisation of circulation, Vancomycin (VANCOMYCIN), indication Infection, Xanef (ENALAPRIL), indication Renal insufficiency, Synacthen (TETRACOSACTIDE), indication Epilepsy, Phenytoin InjLst 750mg/h (PHENYTOIN), indication Epilepsy, Cefuroxim InjLsg (CEFUROXIME), indication Infection, NaHCO₃ (SODIUM BICARBONATE), indication Drug therapy, Hydrocortison (HYDROCORTISONE), indication Exanthema, Tracutil (COMBINATIONS OF ELECTROLYTES), indication Nutritional support, Arterenol InjLsg 5mg/h (NOREPINEPHRINE), indication Stabilisation of circulation, Fresubin plus Sonde (ALL OTHER NON-THERAPEUTIC PRODUCTS), indication Enteral nutrition, Gastrografin (DIATRIZOIC ACID), indication Scan with contrast, Cordarex (AMIODARONE), indication Resuscitation, Keppra (LEVETIRACETAM), indication Epilepsy, Fenistil (DIMETINDENE), indication Pruritus and Euphyllong (THEOPHYLLINE), indication COPD. The patient developed Stevens-Johnson-Syndrome (Stevens-Johnson syndrome)(start date 30-Oct-2004).

The following action was taken: for Theophyllin retard-ratiopharm 375 mg (Retardkapseln) (THEOPHYLLINE) - Drug withdrawn for Aciclovir-ratiopharm 500 mg p.i. (Durchstechflaschen) (ACICLOVIR) - Drug withdrawn.

The outcome for Stevens-Johnson-Syndrome (Stevens-Johnson syndrome) is Recovered / Resolved.

Rechallenge information for Theophyllin retard-ratiopharm 375 mg (Retardkapseln) is not applicable. Rechallenge information for Aciclovir-ratiopharm 500 mg p.i. (Durchstechflaschen) is not applicable. Dechallenge information for Theophyllin retard-ratiopharm 375 mg (Retardkapseln) is not applicable. Dechallenge information for Aciclovir-ratiopharm 500 mg p.i. (Durchstechflaschen) is not applicable.

The reporter's assessment for drug Aciclovir-ratiopharm 500 mg p.i. (Durchstechflaschen) related with Stevens-Johnson-Syndrome is as follows: Possible. The reporter's assessment for drug Theophyllin retard-ratiopharm 375 mg (Retardkapseln) related with Stevens-Johnson-Syndrome is as follows: Possible. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

Steven-Johnson-Syndrom (SJS) is not SPC/RSI-listed neither for Theophyllin retard-ratiopharm 375 mg (Retardkapseln) nor for Aciclovir-ratiopharm 500 mg (solution for injection), respectively.

Generally, it cannot be excluded that reported symptomatic is related to Theophyllin and/or Aciclovir, however, the patient used multiple drugs concomitantly, which cannot be excluded for causing SJS, respectively. This is the first case including SJS in association with Theophyllin as well for Aciclovir after parenteral administration, in our Pharmacovigilance database. For Aciclovir after oral administration one case including erythema exsudativum multiforme as well one case including SJS were reported to the MAH so far.

Based on this evidence i.e. the use of multiple drugs with known potentiation of induction of a SJS (e.g. valproic acid, phenytoin, clarithromycin...), at present no actions are considered necessary.

Note: Within the case the outcome of patient was reported to be "not recovered/not resolved" but on the other hand reaction end date of SJS was given to be 21.12.2004. Therefore, the outcome is assessed to be "recovered/resolved" (at the time of this report). No further informations are available.

Reporter's comment:

Fresenius Kabi, 10.03.2008 DE-Fresenius Kabi-2008030051

Sender diagnosis: not reported

Narrative

Verbatim of the Reporter:

Stevens-Johnson-Syndrome (SJS)

(Was assessed as probable case according to the assessment of experts March 2006).

Case narrative:

The case was reported by the BfArM (health authority Germany) with Worldwide Reference Number DE-BFARM-08017209 and was detected in an epidemiological study (RegiSCAR Studie) in Germany.

On October 23rd, 2004 a 70-year-old male patient was admitted to the hospital due to status epilepticus. HAES steril 10% (dosage unknown) was administered on the same day and also from 28.10.2004 until 30.10.2004. On 30.10.2004 a Stevens-Johnson-Syndrome occurred and 04.11.2004 the patient suffered from erosions of the tongue, on 06.11. and on 07.11.2004 erosions of mouth, oral and nasal mucosa and generalized erythrodermia.

Medical history: Status epilepticus.

Concomitant medication:

Assessed as probable related to reaction according to the assessment of experts 03/2006: Phenytoin iv. 750 mg/h 25.10.04 and 28.10. - 03.11.04 and 350 mg/day per oral on 26.10. - 28.10.04; Allopurinol 300 mg/day per oral on 24.10. - 28.10.04.

Assessed as possibly related to reaction according to the assessment of experts 03/2006:

Trapanal (thiopental), Diclofenac, Orfiril (valproin acid), Ebrantil (urapidil), Bayotensin (nitredipin), Pres (enalapril), Fenta (fentanyl), Frisium (clobazam), Propofol, Altinsulin (insulin), Valproat (valproin acid), Heparin, ACC (acetylcystein), Atrovent (ipratropiumbromid), Salbutamol, Pantozol (pantoprazole), Euphyllong (theophylline), Paracetamol, Xanef (enalapril), Furo (furosemide), ASS (acetylsalicyl acid), Theophylline, Aciclovir, Solosin (theophylline) and Arterenol (norepinephrine).

Due to the huge number of co-medication irrelevant to assessment of causality with HAES steril 10% we decided to include in our database only such co-medication that has been suspected as being probably causative (allopurinol and phenytoin).

Sender's comment:

According to the reporter's assessment the reaction is possibly related to the administration of HAES-steril 10%.

We agree to the reporter's assessment. The reaction occurred in plausible temporal relationship to the administration of HAES steril 10%. However, concomitant medication allopurinol and phenytoin are more likely causative.

It can not be excluded that HES may have caused the event, as the reason for the occurrence of the first symptoms and the worsening remains unclear. However no similar other cases from spontaneous reporting or clinical studies - not deriving from this special epidemiologic study - are known for starches.

Steven-Johnson-Syndrome is not labeled as a possible side effect of hydroxyethyl starch.

Reporter's comment:

001307602, 17.03.2008 DE-ABBOTT-08P-062-0441536-00

Sender diagnosis: not reported

Narrative

This report received from Nycomed; reference number: 80493.

Report from Germany of Stevens Johnson syndrome coincident with CLARITHROMYCIN (KLACID) therapy.

This report was received from the German Documentation centre of severe skin reactions (ref. no. 9160240) via the German Health authority (BfArM ref. no. DE-BFARM-08017209).

This is a serious case due to hospitalization.

An elderly male patient experienced Stevens-Johnson syndrome while receiving PANTOPRAZOLE (PANTOZOL i.v. and PANTOZOL unknown dose p.o) for ulcer prophylaxis, urapidil (EBRANTIL 250 mg/h i.v) for hypertension, THEOPHYLLINE for COPD and TRAPANAL for sedation. Further 61 suspected drugs were administered.

On 21-Oct-2004 patient had been admitted to hospital due to status epilepticus.

On 28-Oct-2004 patient developed fever.

On 30-Oct-2004 patient developed generalized skin spots.

On 03-Nov-2004 patient developed tongue erosions.

On 04-Nov-2004 patient developed erosions of nasal mucous membranes.

On 06-Nov-2004 patient experienced erosions of oral mucous membranes.

On 07-Nov-2004 patient experienced generalized erythrodermia.

The outcome was reported as still ongoing at the time of the report.

Concomitant diseases included status epilepticus, epilepsy, hypertension, COPD, renal insufficiency, hyperuricaemia, pain, acute renal failure, infection, hypokalemia, nausea, pruritus, pneumonia, cardiac insufficiency, rhinitis and bacteraemia.

Sender's comment:

Abbott Germany Assessment: 17.03.2008, Klacid(R) - 08P-062-0441536-00 (0), PA
Die in diesem Fallbericht geschilderte Symptomatik ist in den Angaben zum Arzneimittel in adaequater Form beschrieben.

Reporter's comment:

Wyeth-Pharma GmbH, 18.03.2008 DE-WYE-G01144608

Sender diagnosis: not reported

Narrative

New version created to add Business Partner (Nycomed) Ref No (80493).

Information was received from a registry via a regulatory authority regarding a 70-year-old male patient who received Tavor (lorazepam injection) therapy and experienced Stevens-Johnson syndrome.

MEDICAL HISTORY:

The patient's concurrent illnesses include epilepsy, cardiac failure and chronic obstructive pulmonary disease.

PRODUCT DETAILS:

Indication for Tavor was restlessness. The dose regimen was 1 mg 1 time per day (intravenous) from 23-Oct-2004 to 23-Oct-2004 and then was 1 mg 1 time per day (intravenous) from 25-Oct-2004 to 25-Oct-2004. Therapy was permanently discontinued.

Additional suspect medication included enalapril, Optipect Kodein (codeine), diclofenac, Orfiril (valproate sodium), Ebrantil (urapidil), Bayotensin (nitrendipine), Pres (enalapril maleate), fentanyl, Frisium (clobazam), propofol, potassium chloride, Haes-Steril (hydroxyethylcellulose/sodium chloride), Insulin (unspec), valproate sodium, Noradrenaline (norepinephrine), Heparin Sodium, acetylcysteine, Atrovent (ipratropium bromide), salbutamol, Pantozol (pantoprazole injection), Euphyllong (theophylline), paracetamol, Xanef (enalapril maleate), allopurinol, Pantozol (pantoprazole tablet, delayed release), Furo (furosemide), acetylsalicylic acid, theophylline, aciclovir, Solosin (theophylline), Arterenol (norepinephrine hydrochloride), phenytoin, Clinomel (amino acids/carbohydrates/minerals/vitamins), Fresubin (carbohydrates nos/fatty acids nos/minerals nos/proteins nos/vitamins nos), Trapanal (thiopental sodium), Klacid (clarithromycin), cefuroxime, Thomaesonin (calcium chloride dihydrate/magnesium chloride hexahydrate/potassium chloride/sodium acetate/sodium chloride), vancomycin, dobutamine, Prostigmin (neostigmine metilsulfate injection), Paspertin (metoclopramide hydrochloride), panthenol and Synacthen (tetracosactide). Wyeth Pharma is not the marketing authorisation holder for Pantozol (pantoprazole) in Germany.

CONCOMITANT THERAPY:

Concomitant medications were not reported.

EVENT DETAILS:

The patient had already been hospitalized since 23-Oct-2004 because of status epilepticus. He experienced fever (non-serious) on 28-Oct-2004 and Stevens-Johnson syndrome (Stevens-Johnson syndrome) on 30-Oct-2004.

The event was described as follows: 'On 30-Oct-2004 the patient experienced generalized blotchy changes of skin. On 03-Nov-2004 he experienced erosions of tongue. On 04-Nov-2004 he experienced erosions of nasal mucous membrane. On 06-Nov-2004 the patient experienced erosions of oral mucous membrane. On 07-Nov-2004 the patient experienced generalized erythrodermia.'

The patient received antibiotic treatment with Tazobac (piperacillin/ tazobactam) from 30-Oct-2004 to 31-Oct-2004 because of an infection. On 01-Nov-2004 he developed pneumonia and he received Zienam (cilastatin sodium/ imipenem) therapy. His exanthema was treated with hydrocortisone starting on 30-Oct-2004 and additionally he received Fenistil (dimetindene maleate) one time on 31-Oct-2004 for pruritus. Metamizole was administered for pain relief on 01-Nov-2004. The patient had to be reanimated on 02-Nov-2004 wherefore he received Cordarex (amiodarone hydrochloride) and Beloc (metoprolol tartrate) for his heart insufficiency. His epilepsy was treated with Keppra (levetiracetam) from 02-Nov-2004 and Otriven (xylometazoline hydrochloride) was administered for rhinitis. An X-ray of the gastrointestinal tract was performed on 01-Nov-2004 wherefore he received Gastrografin (meglumine amidotrizoate/sodium amidotrizoate). On 03-Nov-2004 a colonic irrigation was carried out. Further treatment included nutritional supplement and electrolyte substitutional therapy.

The patient has not recovered yet.

TEST RESULTS:

Body temperature (results: Body temperature increased) was done on 28-Oct-2004. X-ray gastrointestinal tract (results: unknown) was done on 01-Nov-2004.

RELATEDNESS ASSESSMENT:

The dermatological review committee has categorized Tavor as possibly related to skin reactions. Tavor has an assessment of 5 (range 1 stands for certain relationship to event, range 8 unlikely relationship). The same assessment was provided for diclofenac, Orfiril (valproate sodium), Ebrantil (urapidil), Bayotensin (nitrendipine), Pres (enalapril maleate), fentanyl, Frisium (clobazam), propofol, Haes-Steril (hydroxyethylcellulose/sodium chloride), Insulin (unspec), valproate sodium, heparin sodium, acetylcysteine, Atrovent (ipratropium bromide), salbutamol, Pantozol (pantoprazole injection), Euphyllong (theophylline), paracetamol, Xanef (enalapril maleate), Pantozol (pantoprazole tablet, delayed release), Furo (furosemide), acetylsalicylic acid, theophylline, aciclovir, Solosin (theophylline), Arterenol (norepinephrine hydrochloride) and Trapanal (thiopental sodium). The dermatological review committee categorized phenytoin and allopurinol as likely related to Stevens-Johnson syndrome. All other suspect drugs got a lower degree of probability. According to dermatological review committee the drugs given for treatment since 30-Oct-2004 have no temporal relationship to reaction or are not known for causing Stevens-Johnson syndrome.

Sender's comment:**WYETH COMMENT**

1. FOLLOW UP (v2) 10-MAR-2008

STEVENS JOHNSON SYNDROME unter Tavor

Neue Version wurde erstellt um die Business Partner (Nycomed) Ref No (80493) einzufügen.

Bericht vom Dokumentationszentrum für schwere Hautreaktionen in Freiburg.

Ein 70-jähriger Patient wurde auf Grund eines Status epilepticus am 23-Oct-2004 stationär aufgenommen. Zur Beruhigung erhielt der Patient am 23-Oct-2004 und 25-Oct-2004 Tavor Injektionslösung. Am 30-Oct-2004 zeigte der Patient dann Stevens Johnson Syndrom, zunächst durch generalisierte

Flecken, dann mit Erosionen von Zunge, Nasen- und Mund-Schleimhaut und schliesslich durch generalisierte Erythrodermie. Der Patient wurde noch nicht wiederhergestellt. Der Zusammenhang zwischen dem Stevens Johnson Syndrom und Tavor wurde nach dem Bewertungsalgorithmus des Dokumentationszentrums als möglich beurteilt.

Ein möglicher Zusammenhang kann nicht ausgeschlossen werden.

Stevens Johnson Syndrom ist in der Fachinformation nicht aufgeführt und wurde daher als unbekannt eingestuft.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Massnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Dr. Peter Gores

Reporter's comment:

NOVARTIS, 19.03.2008 DE-002147023-PHBS2008DE03039

Sender diagnosis: not reported

Narrative

Initial report was received from German Health Authority (08017209) via Nycomed 80493) on 07 Mar 2008. This elderly patient with status epilepticus, epilepsy, hypertension, COPD, renal insufficiency, acute renal failure, infection, hypokalaemia, pneumonia, cardiac insufficiency and bacteremia started treatment with Pantozol (pantoprazole) for ulcer prophylaxis on 24 Oct 2004 and diclofenac (manufacturer name unknown) on 01 Jan 2002 for pain valporate (manufacturer name unknown) on 23 Oct 2004 for epilepsy, vancomycin (manufacturer name unknown) on 29 Oct 2004 for rash, Synacthen (tetracosactide) for epilepsy. There are 45 other suspected drug added in the system and 11 other suspected drug are not coded in the system due to space problem: Cernevit (vitamins), abumin, Fenistil (dimethindene), blutproduct, metamizol, Gastrogratin (meglumine aminotrizonate, sodium aminotrizolate), Zienam (cistastatin sodium), Cordarex (amiodarone), Beloc (metoprolol), Mg-Mineralien (mineral supplement), Keppa (levetiracetam), Otriven (xylometazoline). On 22 Oct 2004 he was admitted to hospital due to status epilepticus. On 30 Nov 2007 Steven Johnson syndrome was diagnosed with the symptoms of fever, generalized skin spots, tongue erosion, nasal mucous membrane, and erosion of oral mucous membrane and generalized erythrodermia. Pantozol treatment was discontinued on 28 Oct 2004. The outcome of the event was reported as unchanged. The reporter suspected a causal relationship between the event and Pantozol and the other 59 drugs.

Sender's comment:

For Pantozol and diclofenac:

Serious spontaneous report, Stevens-Johnson syndrome [hospitalisation], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

For Synacthen:

Serious spontaneous report, Stevens-Johnson syndrome [hospitalisation], assessed as unlisted according to the Core Data Sheet.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

APHQWEB, 25.03.2008 DE-Nycomed GmbH-80493

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions (ref no. 9160240) via the German Health authority (BfArM ref. no. DE-BFARM-08017209).

This is a serious case due to hospitalisation.

A elderly male patient experienced Stevens-Johnson syndrome while receiving pantoprazole (Pantozol i.v. and Pantozol unknown dose p.o.) for ulcer prophylaxis, urapidil (Ebrantil 250mg/h i.v.) for hypertension, theophylline for COPD and Trapanal for sedation. Further 61 suspected drugs were administered.

On 22-OCT-2004 patient had been admitted to hospital due to status epilepticus.

On 28-OCT-2004 patient developed fever.

On 30-OCT-2004 patient developed generalized skin spots.

On 03-NOV-2004 patient developed tongue erosions.

On 04-NOV-2004 patient developed erosions of nasal mucous membranes.

On 06-NOV-2004 patient experienced erosions of oral mucous membranes.

On 07-NOV-2004 patient experienced generalized erythrodermia.

The outcome was reported as still ongoing at the time of the report.

Concomitant diseases included status epilepticus, epilepsy, hypertension, COPD, renal insufficiency, hyperuricaemia, pain, acute renal failure, infection, hypokalaemia, nausea, pruritus, pneumonia, cardiac insufficiency, rhinitis and bacteraemia. The Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels ranging from 1=definite to 8=improbable). Pantoprazole, urapidil, theophylline (Theophyllin 375mg) and thiopental were rated as possible due to this algorithm. Allopurinol and phenytoin were considered to be highly suspected drugs and the causal relation was rated as probable due to this algorithm.

STEVENS-JOHNSON-SYNDROME:

This patient developed Stevens-Johnson syndrome approximately eight days after first injection of pantoprazole and urapidil, four days after introduction of thiopental, five days after first intake of theophylline and six days after first intake of pantoprazole p.o. Concomitantly, the patient received multiple other drugs (approximately further 60 suspected drugs).

Stevens-Johnson syndrome is a rare skin disorder, characterized by mucosal erosions, skin blisters, often accompanied by sore throat, malaise and fever. It can be drug-induced or due to infectious diseases.

Several drugs have been observed to cause Stevens-Johnson syndrome (SJS).

Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. SJS is not expected for urapidil, theophylline and thiopental.

Temporal relationship plausible, time to onset reasonable for pantoprazole, urapidil, thiopental and theophylline. Mockenhaupt et al (2008) suggest, that Stevens-Johnson syndrome often begins more than four and up to 28 days after the initiation of the responsible medication.

Sender's comment:

The patient received multiple other suspected drugs (allopurinol and phenytoin had been rated as probable by the reporter's algorithm). In addition, the patient concomitantly suffered from an infection (bacteraemia and pneumonia) as well as from renal failure, which could be a contributing factor due to impaired drug metabolism.

However, although temporal relationship to pantoprazole, urapidil, thiopental and theophylline therapy is reasonable, it is hard to determine a single event as trigger for the reported event in this severely ill patient treated with various medications.

As a causal relationship cannot be fully ruled out, Nycomed conservatively assesses this case as possibly related to pantoprazole, urapidil, thiopental and theophylline.

This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole, urapidil, thiopental and theophylline and has no effect on the status of these drugs as reflected by the formal product characteristics.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08013455

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 79 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Necrolysis epidermal toxic (Lyell type)	20050410	20050422	13 Day		fatal
2 Stevens Johnson syndrome	20050410	20050422	13 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol	Oral	1 Df dosage form/ Day	-	Gout
allopurinol			-	
2 Voltaren Dispers	Oral	1 Df dosage form/ Day	20050311-	Pain NOS
diclofenac sodium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension			Unknown	
Hyperlipidaemia NOS			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08014066

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 67 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20030321	deathcause: Toxic epidermal necrolysis
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Eyes red	20030304		18 Day		fatal
2 Toxic epidermal necrolysis	20030304		18 Day		fatal
3 Necrolysis epidermal	20030304				fatal
4 Blister	20030304		18 Day		fatal
5 Exanthema	20030304		18 Day		fatal
6 Fever	20030304		18 Day		fatal
7 Macule	20030304		18 Day		fatal
8 Oral ulceration	20030304		18 Day		fatal
9 Pain of skin	20030304		18 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	100 Mg milligram(s)/	20030127-	Pain
diclofenac			-	
2 diclofenac	Oral	50 Mg milligram(s)/	20030107-	Pain
diclofenac			-	
3 novalgine	Oral	/	20030127-	Pain
metamizole			-	
4 NOVALGIN /00039501/	Oral	3* /Day	20030127-20030307	Pain
metamizole			40 Day	
5 saroten	Oral	25 Mg milligram(s)/	20030127-	Depression
amitriptyline hydrochloride			-	

Drugs (concomitant)

6	DUROGESIC fentanyl	Transdermal	/	Medicated plaster	20030107-20030307 Pain 60 Day
7	FORTECORTIN /00016001/ dexamethasone	Oral	/		20030210-20030307 Pain management 26 Day
8	Haldol haloperidol	Oral	5 Gtt drop(s)/Day	Irrigation solution	20030127-20030307 Agitation 40 Day
9	LAXOBERAL sodium picosulfate	Oral	/	Irrigation solution	20030203-20030307 Constipation 33 Day
10	MOVICOL /01053601/ macrogol; potassium chloride; sodium bicarbonate; sodium chloride	Oral	/	Oral Solution	20030305-20030306 Constipation 2 Day
11	NEURONTIN gabapentin	Oral	/		20030127-20030307 Pain management 40 Day
12	Nexium mups esomeprazole	Oral	40 Mg milligram(s)/Day		20030203-20030307 Prophylactic 33 Day
13	PANTOZOL /01263202/ pantoprazole sodium	Oral	40 Mg milligram(s)/Day		20021018-20030209 Prophylactic 3 Month
14	SEVREDOL morphine sulfate	Oral	60 Mg milligram(s)/Day		20021118-20030213 Pain 88 Day
15	voltaren resinat diclofenac resinate	Oral	/		20030107-20030209 Tumor pain 34 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Agitation			Unknown	agitation
Constipation			Unknown	constipation
Pain NOS			Unknown	pain NOS
Radiation therapy			Unknown	Concomitant disease
Carcinoma of the prostate metastatic			Unknown	Concomitant disease
Tumor pain			Unknown	tumor pain

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

Sanofi-Aventis Deutschland GmbH, 25.02.2008 DE-AVENTIS-200810490DE

Sender diagnosis: not reported

Narrative**Initial report**

This case of toxic epidermal necrolysis (TEN) with maculae was received from the German health authority (DE-BFARM-08014066) . It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9150017). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of TEN was made by an expert panel who assessed it as a "definite case" of TEN. Causal relationship was assessed by the experts in June 2003 using an algorithm.

As for metamizole (Novalgin) the causality was assessed as "well possible". For the other suspected drugs causality was assessed as:

"probable": diclofenac 100mg , "well possible": diclofenac 50mg and possible :amitriptyline

Not suspected were: diclofenac-colestyramine (taken until 09-Feb-2003), pantoprazole, morphine, fentanyl, gabapentin, sodium picosulfate, esomeprazole, haloperidol, dexamethasone, electrolytes

Metastatic prostate carcinoma and radiation therapy were considered possible causes of TEN.

Epicrisis:

Patient: male, 67 years

Medical history: metastatic prostatic carcinoma, depression, agitation.

From 26-Feb-2003 to 05-Mar-2003 the patient underwent percutaneous radiation therapy of cervical spine (discontinued due to occurrence of exanthema under treatment).

From 07-Jan-2003 to 27-Feb-2003 the patient was treated with diclofenac 150mg/day orally for pain; On 28-Feb-2003 therapy was changed from tablets to suppositories. On 27-Jan-2003 he started treatment with metamizole 500mg/4h (three times daily); this treatment lasted at least until 07-Mar-2003. During that time the patient was also treated with a lot of other drugs.

On 04-Mar-2003 the patient developed target lesions and maculae.

On 07-Mar-2003 fever was diagnosed, the patient's eyes were red. He was hospitalised.
 On 08-Mar-2003 first skin blister was detected, the patient suffered from exanthema.
 On 09-Mar-2003 the patient developed blisters on his trunk and complained about skin pain.
 On 11-Mar-2003 the patient's back was affected extensively, the patient had fever.
 On 19-Mar-2003 erosions of oral mucosa were detected.
 The outcome was reported as "death", date of end of event was reported as 21-Mar-2003.
 Laboratory: no values provided.

Sender's comment:

21-FEB-2008 Sanofi-Aventis Company Comment: Due to the compatible temporal relationship the causal role of metamizole cannot be excluded. However, the concomitant medication as well as the underlying metastatic carcinoma of prostate and radiation therapy could be discussed as possible alternative causes.

21-FEB-2008 Sanofi Aventis Germany: TOXIC EPIDERMAL NECROLYSIS - FATAL is considered locally unlabelled for metamizole sodium (due to fatal outcome). The facts presented here however are not very suggestive of a causal relationship (please refer to the intl. evaluator's comment), and thus do not imply a change in the state of the scientific knowledge for the product. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

Bayer Vital GmbH, 29.02.2008 DE-BAYER-200813598GPV

Sender diagnosis: not reported

Narrative

Source of report and patient demography:

This is a solicited report by a health professional from the Documentation Center for Severe Skin Reaction and was received via Germany's health authority on 20-Feb-2008, referring to a 67 year-old male.

Medical and drug history:

Concomitant conditions: tumor pain, pain NOS, constipation, and agitation.
 Concomitant drugs have been given. For details see section "Concomitant Drugs".

Suspect drugs, timing and conditions surrounding the onset of the reactions:

The patient received

- Saroten (amitriptyline hydrochloride) for depression from 27-Jan-2003 to 07-Mar-2003 (latest documented administration) at a total daily dose of 12.5 mg oral;
- Diclofenac (diclofenac) for pain NOS from 07-Jan-2003 to 07-Mar-2003 at a total daily dose of 150 mg oral, and from 28-Feb-2003 to 06-Mar-2003 at a total daily dose of 100 mg rectal;
- Novalgine (metamizole sodium) for pain NOS from 27-Jan-2003 to 07-Mar-2003 (latest documented administration) at a total daily dose of 1500 mg oral.

The progression of the events and their outcome in the patient:

On 04-Mar-2003, the patient experienced FATAL TOXIC EPIDERMAL NECROLYSIS (symptoms: target lesions, maculae, fever, eye redness, first skin blister, exanthema, blistering trunk, skin pain, extensive skin affection of the back, fever, erosive oral mucosa), which was considered serious due to death.

On 05-Mar-2003, the patient experienced EXANTHEMA DURING RADIOTHERAPY, which was considered non-serious. The outcome of this event was not reported.

Relevant details of fatal outcome:

Patient died on 21-Mar-2003. Autopsy information is unknown. The patient died from FATAL TOXIC EPIDERMAL NECROLYSIS as stated by the reporter.

The original reporter's clinical assessment:

For the drug Saroten (amitriptyline hydrochloride): possible; Diclofenac (diclofenac) oral: very possible, Diclofenac (diclofenac) rectal: probable and concerning Novalgine (metamizole), very possible, which is co-suspected by him.

Possible alternative explanation: metastatic prostate carcinoma, radiation therapy, no infection.

Sender's comment:

FATAL TOXIC EPIDERMAL NECROLYSIS considered serious as leading to patient's death and EXANTHEMA DURING RADIOTHERAPY considered non-serious are neither listed in the international product information of Saroten (amitriptyline hydrochloride), nor in the international product information of Diclofenac (diclofenac). Based on the information received a temporal relationship is given and thus a causal association cannot be excluded. However, the patient who suffered from metastatic prostate carcinoma received concomitant percutaneous radiotherapy, being cited as possible alternative explanation by the reporter, which may have mainly contributed to the patient's events.

Reporter's comment:

H Lundbeck A/S, 11.03.2008 DE-LUNDBECK-DKLU1031244

Sender diagnosis: not reported

Narrative

BAYER CASE ID: 200813598GPV

Case reference number DKLU1031244, is a clinical trial case from a Non-HLU Survey, the solicited report was received from a physician of the center of severe skin reactions in Germany (study id: bayo3211 / bayo3211/pms) via the regulatory health authority of Germany (ICSR ID: DE-BAYER-200813598GPV). The case was evaluated by Bayer and submitted by H. Lundbeck A/S based on a pharmacovigilance agreement signed for a co-development programme.

This case concerns a 67-year-old male administered suspected drugs: Saroten (amitriptyline hydrochloride) for depression from 27/Jan/2003 to 07/Mar/ 2003 (latest documented administration) at a total daily dose of 12.5 mg, oral; DICLOFENAC (diclofenac) for pain NOS from 07/Jan/2003 to 07/Mar/2003 at a total daily dose of 150 mg oral, and from 28/Feb/2003 to 06/Mar/2003 at a total daily dose of 100 mg, rectal; NOVALGIN (metamizole sodium) for pain NOS from 27/Jan/2003 to 07/Mar/ 2003 (latest documented administration) at a total daily dose of 1500 mg, oral.

On 04/Mar/2003, approximately five weeks after first dose of Saroten the patient experienced Toxic Epidermal Necrolysis Transitional (symptoms: target lesions, maculae, fever, eye redness, first skin blister, exanthema, blistering trunk, skin pain, extensive skin affection of the back, fever, erosive oral mucosa).

On 05/Mar/2003, the patient experienced exanthema during radiotherapy, which was considered non-serious. The outcome of this event was not reported.

The patient died on 21/Mar/2003. The cause of death was fatal toxic epidermal necrolysis transitional. Autopsy information is unknown.

The patient's medical history comprised tumor pain, pain NOS, constipation, and agitation.

Concomitant medication included:

Name: Voltaren resinat (DICLOFENAC RESINATE), Dates: 07-Jan.2003 to 08-Feb-2003, Duration: 4 weeks 5 days, Indication: tumor pain.

Name: Pantozol (PANTOPRAZOLE SODIUM), Dose: 40 MG, Route: ORAL, Dates: 18-Oct-2002 to 08-Feb-2003, D; uration: 16 weeks days, Indication: unknown indication.

Name: Sevedol (MORPHINE SULFATE), Dose: 60 MG, Route: ORAL, Dates: 18-Nov-2002 to 12-Feb-2003, Duration: 12 weeks 3 days, Indication: pain NOS.

Name: Durogesic (FENTANYL), Route: TOPICAL, Dates: 07-Jan-2003 to ??-??-2003, Duration: Unknown, Indication: pain NOS.

Name: Neurontin (GABAPENTIN), Dose: 300 MG, Route: UNKNOWN, Dates: 27-Jan-2003 to ??-??-2003, Duration: Unknown, Indication: unknown indication.

Name: Laxoberal (SODIUM PICOSULFATE), Route: ORAL, Dates: 03-Feb-2003 to ??-??-2003, Duration: Unknown, Indication: constipation.

Name: Nexium (ESOMEPRAZOLE MAGNESIUM), Dose: 40 MG, Route: ORAL, Dates: 03-Feb.2003 to ??-??-2003, Duration: Unknown, Indication: unknown indication.

Name: Haldol (HALOPERIDOL), Dates: 27-Jan-2003 to ??-??-2003, Duration: Unknown, Indication: agitation.

Name: FORTECORTIN [DEXAMETHASONE] (DEXAMETHASONE [DEXAMETHASONE]), Route: UNKNOWN, Dates: 10-Feb.2003 to

2003, Duration: Unknown, Indication: unknown indication.

Name: Movicol , Route: ORAL, Dates: 06-Mar-2003 to 06-Mar-2003, Duration: 1 day, Indication: constipation.

Saroten on was discontinued on 07/Mar/2003.

Other relevant tests: Taken from unstructured information:

Until 26-Feb-2003 patient received percutaneous radiotherapy of cervical spine due to metastatic prostate carcinoma.

Radiotherapy was discontinued on 05-Mar-2003.

The reporter to be possibly related to Saroten (amitriptyline).

The original reporters assessed the event as follows: For the drug Saroten (amitriptyline hydrochloride): possible; Diclofenac (diclofenac) oral: very possible, Diclofenac (diclofenac) rectal: probable and concerning Novalgin (metamizole) , very possible, which is co-suspected by him.

Possible alternative explanation; metastatic prostate carcinoma, radiation therapy, no infection.

Additional information has been requested and if new relevant information is received the case will be re-submitted.

Sender's comment:

Temporal relationship possible. The pathogenesis of TEN is yet unknown, but an involvement of inflammatory cytokines and the death receptor Fas and its ligand FasL has been suggested (Allergol Int. 2006 Mar;55(1):9-16.). However it is still unknown why inflammation is triggered as a response to various drugs. Confounding factors: Patient received numerous drugs and was immunologically challenged and received radiation therapy at the same time. . Company causality: Cannot be ruled out. Listedness equals expectedness. Causal relationship cannot be ruled out. This single, individual case report does not change the benefit-risk ratio of the drug. No measures are currently considered necessary by the MAH

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08014098

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20030417	deathcause: Toxic epidermal necrolysis
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Macule	20030404			37 Day	Unknown
2 Toxic epidermal necrolysis	20030404	20030417	14 Day	39 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ALDACTONE spironolactone	Oral	/ Tablet	20030227-20030408 41 Day	Ascites
2 AQUAPHOR xipamide	Oral	20 Mg milligram(s)/Day Tablet	20030304- -	Edema
3 CALCILAC KT calcium carbonate; colecalciferol	Oral	2 Df dosage form/Day Tablet	20030222- -	Osteoporosis
4 diclofenac diclofenac	Oral	1 Df dosage form/Day Tablet	20030403-20030403 1 Day	Pain
5 HEPA-MERZ-SIRUP ornithine aspartate	Oral	6 Df dosage form/Day Syrup	20030222- -	Liver cirrhosis
6 ibuprofen ibuprofen	Oral	1 Df dosage form/ Tablet	20030214- -	Pain
7 KONAKION N phytomenadione	Oral	30 Gtt drop(s)/Day	20030222- -	Vitamin K deficiency
8 MCP TROPFEN metoclopramide	Oral	108 Gtt drop(s)/Day	20030222- -	Reflux esophagitis
9 milgamma benfotiamine; cyanocobalamin	Oral	3 Df dosage form/Day	20030222- -	Vitamin B complex deficiency
10 MILGAMMA benfotiamine; cyanocobalamin	Oral	3 Df dosage form/Day Tablet	20030222- -	Vitamin B6 deficiency
11 NEXIUM esomeprazole magnesium	Oral	20 Mg milligram(s)/Day Modified-release tablet	20030225- -	Helicobacter infection
12 NOVAMINSULFON metamizole sodium	Oral	1 Df dosage form/	20030214- -	Pain
13 OBSIDAN propranolol hydrochloride	Oral	37.5 Mg milligram(s)/Day Tablet	20030312- -	Portal vein pressure increased
14 OMEP omeprazole	Oral	40 Mg milligram(s)/Day TABLET	20030319-20030408 21 Day	Prophylaxis against gastrointestinal ulcer
15 ULCOGANT sucralfate	Oral	4 Df dosage form/Day Oral suspension	20030224- -	Gastrointestinal ulcer

Drugs (concomitant)

16	ALDACTONE spironolactone	Unknown	100 Mg milligram(s)/Day	-
17	ALDACTONE spironolactone	Unknown	150 Mg milligram(s)/Day	-
18	ALDACTONE spironolactone	Unknown	100 Mg milligram(s)/Day	-
19	calcilac folic acid	Unknown	/Day	20030222-
20	calcilac folic acid	Unknown	2* /Day	20030222-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ascites			No	
Gastrointestinal ulcer NOS			No	
Liver cirrhosis			No	
Oedema			No	
Oesophagitis			No	
Osteoporosis			No	
Pain NOS			No	
Vitamin B complex deficiency			No	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

ROCHE, 13.02.2008 DE-ROCHE-545518

Sender diagnosis: not reported

Narrative

INTENSIVE MONITORING

THIS CASE WAS REPORTED BY A PHYSICIAN AND CONCERNS A 67-YEAR-OLD FEMALE PATIENT EXPERIENCED FATAL TOXIC EPIDERMAL NECROLYSIS WHILE TAKING PHYTOMENADIONE (KONAKION) FOR VITAMIN K SUBSTITUTION, SPIRONOLACTONE (ALDACTONE) FOR ASCITES, IBUPROFEN FOR PAINS, METAMIZOLE (NOVAMINSULFON) FOR PAINS, FERROUS GLYCINE/PROPRANOLOL (OBSIDAN) FOR DECREASE OF BLOOD PRESSURE, CERESIN/GLYCEROL/LEVOMENOL/LIQUID PARAFFIN/PANTHENOL/WOOL ALCOHOL/XIPAMIDE/YELLOW SOFT PARAFFIN (AQUAPHOR) FOR OEDEMA, ORNITHINE (HEAP-MERZ) FOR LIVER CIRRHOSIS, METOCLOPRAMIDE (MCP TROPFEN) FOR PEPTIC OESOPHAGITIS, SUCRALFATE (ULCOGANT) FOR GASTROINTESTINAL ULCERS, ESOMEPRAZOLE (NEXIUM) FOR ERADICATION, CALCIUM CARBONATE/COLECALCIFEROL (CALCILAC KT) FOR OSTEOPOROSIS, BENFOTIAMINE/CYANOCOBALAMIN/PYRIDOXINE/THIAMINE (MILGAMMA) FOR "VITAMIN B1/B6 DEFICIT", OMEPRAZOLE (OMEP) FOR GASTRIC PROTECTION AND DICLOFENAC FOR PAINS.

IT WAS REPORTED THAT THE PATIENT HAD RECEIVED DICLOFENAC IN THE PAST AND WAS WELL TOLERATED.

ON 14 FEBRUARY 2003, THE PATIENT COMMENCED ORAL IBUPROFEN (TWICE A DAY AS NEEDED) AND ORAL METAMIZOLE (TWICE A DAY AS NEEDED). ON 22 FEBRUARY 2003, THE PATIENT COMMENCED ORAL PHYTOMENADIONE ("3X10 DROPS"), ORAL ORNITHINE ("1-1-1 BAG, 2-2-2 BAG"), ORAL METOCLOPRAMIDE ("3X36 DROPS"), ORAL CALCIUM CARBONATE/COLECALCIFEROL ("1-0-1,1-0-0") AND ORAL BENFOTIAMINE/CYANOCOBALAMIN/PYRIDOXINE/THIAMINE (THREE TIMES DAILY). ON 24 FEBRUARY 2003, THE PATIENT COMMENCED ORAL SUCRALFATE ("2X1 BAG, 4X1 BAG") AND THE FOLLOWING DAY STARTED ORAL ESOMEPRAZOLE (20 MG IN THE MORNING). ON 27 FEBRUARY 2003, THE PATIENT COMMENCED ORAL SPIRONOLACTONE ("2X50MG, 3X50MG AND 2X50MG"). ON 4 MARCH 2003, THE PATIENT COMMENCED ORAL CERESIN/GLYCEROL/LEVOMENOL/LIQUID PARAFFIN/PANTHENOL/WOOL ALCOHOL/XIPAMIDE/YELLOW SOFT PARAFFIN (20 MG IN THE MORNING). ON 12 MARCH 2003, THE PATIENT COMMENCED ORAL FERROUS GLYCINE/PROPRANOLOL ("3X1/2"). ON 19 MARCH 2003, THE PATIENT COMMENCED ORAL OMEPRAZOLE (40 MG). ON 3 APRIL 2003, THE PATIENT RECEIVED ORAL DICLOFENAC (ONE DOSE FORM). ON 4 APRIL 2003, THE PATIENT EXPERIENCED LIFE-THREATENING TOXIC EPIDERMAL NECROLYSIS (CO-MANIFESTATION INCLUDED LIPS AND ORAL MUCOSA EROSIONS). THE FOLLOWING DAY, SHE HAD SKIN PAIN AND FIRST SKIN MODIFICATIONS. ON 6 APRIL 2003, THE PATIENT HAD FIRST SKIN EROSION. ON 7 APRIL 2003, THE PATIENT HAD GENITAL AND NASAL EROSION. ON 8 APRIL 2003, SPIRONOLACTONE AND OMEPRAZOLE WERE DISCONTINUED. ON 9 APRIL 2003, THE PATIENT WAS HOSPITALISED AND TREATMENT WITH PHYTOMENADIONE, IBUPROFEN, METAMIZOLE, FERROUS GLYCINE/PROPRANOLOL, CERESIN/GLYCEROL/LEVOMENOL/LIQUID PARAFFIN/PANTHENOL/WOOL ALCOHOL/XIPAMIDE/YELLOW SOFT PARAFFIN, ORNITHINE,

METOCLOPRAMIDE, SUCRALFATE, ESOMEPRAZOLE, CALCIUM CARBONATE/COLECALCIFEROL AND EFOTIAMINE/PYRIDOXIN WAS DISCONTINUED. ON 17 APRIL 2003, THE PATIENT DIED.

THE PHYSICIAN ASSESSED THE DRUG-EVENT RELATIONSHIP BETWEEN TOXIC EPIDERMAL NECROLYSIS AND SPIRONOLACTONE, METAMIZOLE, FERROUS GLYCINE/PROPRANOLOL, CERESIN/GLYCEROL/LEVOMENOL/LIQUID PARAFFIN/PANTHENOL/WOOL ALCOHOL/XIPAMIDE/YELLOW SOFT PARAFFIN, SUCRALFATE, ESOMEPRAZOLE AND OMEPRAZOLE AS POSSIBLE RELATED AND TO PHYTOMENADIONE, IBUPROFEN, ORNITHINE, METOCLOPRAMIDE, CALCIUM CARBONATE/COLECALCIFEROL AND BENFOTIAMINE/CYANOCOBALAMIN/PYRIDOXINE/THIAMINE AS QUESTIONABLY RELATED AND TO DICLOFENAC AS VERY WELL POSSIBLY RELATED. THE PHYSICIAN FURTHER ASSESSED THE FATAL TOXIC EPIDERMAL NECROLYSIS AS NOT RELATED TO THE INFECTION BUT POSSIBLY RELATED TO LIVER CIRRHOSIS.

NO FURTHER INFORMATION WAS AVAILABLE.

Sender's comment:

CIOMS TEXT: FATAL TOXIC EPIDERMAL NECROLYSIS IS NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (ISPI) FOR PHYTOMENADIONE. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN TOXIC EPIDERMAL NECROLYSIS AND TREATMENT WITH PHYTOMENADIONE IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM. HOWEVER, A POSSIBLE ALTERNATIVE EXPLANATION FOR THE EVENT INCLUDES THE CONCOMITANT USE OF IBUPROFEN, FERROUS GLYCINE/PROPRANOLOL, ESOMEPRAZOLE AND DICLOFENAC.

FATAL TOXIC EPIDERMAL NECROLYSIS IS NOT LISTED IN THE COMPANY REFERENCE DOCUMENT (IDS) FOR SPIRONOLACTONE. BASED ON THE INFORMATION RECEIVED FOR THIS CASE, A CAUSAL RELATIONSHIP BETWEEN TOXIC EPIDERMAL NECROLYSIS AND TREATMENT WITH SPIRONOLACTONE IS ASSESSED AS POSSIBLE. COMPANY CAUSALITY WAS ASSESSED BASED ON THE NARANJO ALGORITHM. HOWEVER, A POSSIBLE ALTERNATIVE EXPLANATION FOR THE EVENT INCLUDES THE CONCOMITANT USE OF IBUPROFEN, FERROUS GLYCINE/PROPRANOLOL, ESOMEPRAZOLE AND DICLOFENAC.; ***LOCAL ASSESSMENT: INTERVIEW NR. 9150023

FATAL TOXIC EPIDERMAL NECROLYSIS = NOT LABELED FOR PHYTOMENADIONE AND FOR SPIRONOLACTONE

ALTERNATIVE EXPLANATION INCLUDES CONCOMITANT USE OF IBUPROFEN, FERROUS GLYCINE/PROPRANOLOL, ESOMEPRAZOLE AND DICLOFENAC

EINZELFALLBEOBACHTUNG. KEIN HANDLUNGSBEDARF.

Reporter's comment:

AZPROD0000, 22.02.2008 DE-AstraZeneca-2008PK00355

Sender diagnosis: not reported

Narrative

{TOXIC EPIDERMAL NECROLYSIS}

A report was received from the German documentation centre for severe skin reactions via the German health authority BfArM concerning a 68 year old female patient. The patient was being treated with NEXIUM ORAL (esomeprazole magnesium) 20 mg QD oral for hp eradication, OMEP (omeprazole), diclofenac, ULCOGANT (SUCRALFATE) (sucralfate), AQUAPHOR TABLET (xipamide), OBSIDAN (PROPRANOLOL) (propranolol hydrochloride), metamizole and ALDACTONE (spironolactone).

Concomitant drugs included ibuprofen for pain, lactulose for liver cirrhosis, metoclopramide for reflux esophagitis, phytomenadione for vitamin k substitution, calcilac kt for osteoporosis and milgamma for vitamine b1/b6 deficiency.

After starting treatment on 25-FEB-2003 the patient experienced lip and oral mucosa erosions on 04-Apr-2003, initial skin reactions with pain on 05-Apr-2003, genital and nasal as well as general skin erosions on 06-Apr-2003 and positive Nikolsky phenomenon on 07-Apr-2003. She was hospitalised on 09-Apr-2003 and diagnosis of toxic epidermal necrolysis was made. The patient died from the disease on 17-APR-2003.

Experts of the documentation centre for severe skin reactions assessed a causal relationship to esomeprazole, omeprazole, sucralfate, xipamide, propranolol, metamizole, diclofenac and spironolactone as possible.

Serious criterion was death.

Sender's comment:

Toxic epidermal necrolysis is listed as a possible adverse event for esomeprazole. The case is poorly documented and the patient was treated with several suspect drugs. Thus it is not possible to establish an unequivocal causal relationship to esomeprazole. Report according to article 63b AMG:

The case is incompletely documented. Toxic epidermal necrolysis is listed as a possible adverse event for esomeprazole in the German product information, but the patient was treated with several other drugs which are also suspected. Thus it is impossible to establish a causal relationship to a single compound.

Causality assessment: UnassessableNO

Reporter's comment:

Merck KGaA, 22.02.2008 DE-Merck KGaA-6040908

Sender diagnosis: not reported

Narrative

This is a case from the Schoepf registry at the Dermatological University Hospital Freiburg, Germany which registers and evaluates all cases of hospitalisation for SJS/TEN in Germany and the drugs given in temporal relationship with the adverse event received via the German Health Authority (# 08014098) on 19 Feb 2008.

**

A 68-year-old female patient was treated with Ulcogant suspension (sucralfate, 2x1, 4x1) for gastric ulcer since 24 Feb 2003.

On 02 Apr 2003 she experienced lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin. She showed symptoms of Nikolski phenomena. On 09 Apr 2003 she was hospitalized and Steven-Johnson Syndrome was diagnosed.

On 17 Apr 2003 the event stopped and the patient died on an unknown date.

Medical history included cirrhosis of liver.

Other suspected drugs Aldactone (spironolactone), Ibuprofen (ibuprofen), Novaminsulfon drops (metamizol), Obsidan (propranolol), Aquaphor (xipamid), Hepa-Merz-Syrup (lactulose), MCP drops (metoclopramid), Nexium (esomeprazole), Konakion N (phytomenadion), Calcilac KT (CaCO₃, colecalciferol), Milgamma (benfotiamin, pyridoxin), Omep (omeprazole) and Diclofenac (diclofenac.)

The reporter assessed Steven-Johnson-Syndrome as probably related to Diclofenac and as possibly related to Ulcogant, Aldactone, Novaminsulfon, Obsidan, Aquaphor, Nexium and Omep. Ibuprofen, Hepa-Merz-Syrup, MCP drops, Konakion N, Calcilac KT, Milgamma were assessed as doubtful related to SJS.

Sender's comment:

A 68-year-old female developed toxic epidermal necrolysis (TEN) with fatal outcome 39 days after introduction of ulcogant (sucralfat, 2 or 4 dosage forms/d - conflicting information received) for ulcerations of upper digestive tract, 49 days after introduction of ibuprofen (taken as required) for pain and 1 day after having taken diclofenac once (dose not provided) for pain.

She was hospitalized 5 days after first signs of TEN. Actions taken on drugs not provided. She died later.

Co-suspected drugs are: Aldactone (spironolactone, dosage between 25 and 150 mg - not clearly stated) for ascites, started 36d prior to- and stopped 4d after occurrence of event; metamizol (taken as required) started 36d prior to event for pain; Obsidan (propranolol, 37,5 mg/d) for hypertension of V. portae, start 23d prior to event; Aquaphor (xipamide, 20 mg/d) for oedema, start 31d prior to event; Hepa-Merz Sirup (lactulose, dosage not provided) for hepatic cirrhosis, start 31d prior to event; metoclopramide (dosage not provided) for oesophageal reflux, start 31d prior to event; Konakion N (phytomenadion, dosage not provided) for substitution, start 31d prior to event; Calcilac KT (calciumcarbonate / colecalciferol, dosage not provided) for osteoporosis, start 31d prior to event; Milgamma (benfotiamin/pyridoxin, dosage not provided) for hypo-aminosis, start 31d prior to event; Esomeprazole (20mg/d) for helicobacter pylori eradication, start 38d prior to event; Omep (omeprazole, 40 mg/d) as prophylaxis, start 15d prior to - and discontinued 4d after occurrence of event; Concomitant diseases: Liver cirrhosis possible (not clearly stated).

Company comment: Event serious, temporal relationship compatible, latencies: 39d for ulcogant, 49d for ibuprofen, 1d for diclofenac. Event is unlisted for ulcogant but listed for ibuprofen and diclofenac as per company reference documents. Alternative explanation: Metamizol, esomeprazole and omeprazole may have contributed (event listed). Liver cirrho

Reporter's comment:

Lilly Deutschland GmbH, 04.03.2008 DE-ELI LILLY AND COMPANY-DE200802005173

Sender diagnosis: not reported

Narrative

This spontaneous case reported by a physician via a regulatory agency concerns a 68 year old female patient of unknown origin.

The medical history of the patient was not reported. Concomitant medications included spironolactone, ibuprofen, metamizol, propranolol, lactulose, metoclopramide, sucralfate, esomepromazole, phytomenadion, colecalciferol with calcium carbonate, benfotiamin with pyridoxin, omeprazole and diclofenac, all for unreported indications.

The patient received xipamide (Aquaphor), 20mg, orally, once daily, in the morning, for the treatment of oedema, beginning on 04-Mar-2003. On 04-Apr-2003, approximately one month after starting xipamide, the patient experienced lips erosions and MSH erosions (to be clarified). On 05-Apr-2003, the patient experienced pain of skin, first skin changes. On 06-Apr-2003, the patient experienced first erosions of the skin, erosions genital and nasal. On 07-Apr-2003, the patient experienced Nikolsky signs. On 09-Apr-2003, the patient was hospitalized. On 04-Apr-2003, the patient was reported to have developed toxic epidermal necrolysis with macula. The patient died on an unreported date. Cause of death was not specified. No information on corrective treatment or laboratory data was provided. It was not reported if an autopsy was performed. It was not reported if xipamide was continued til death.

No reporter contact, lot number not provided.

The reporting physician did not provide an opinion of relatedness between the event toxic epidermal necrolysis and xipamide.

Sender's comment:

Bewertung - 4. März 2008

Verdachtsfall: schwerwiegend und unbekannt

Der Kausalzusammenhang ist denkbar.

Allerdings bewirkt der Fall keine Aenderung des wissenschaftlichen Erkenntnisstandes. Weitergehende Massnahmen nach § 29 Abs. 1 AMG werden nicht fuer erforderlich gehalten.

Reporter's comment:

SANOFI-SYNTHELABO, 07.03.2008 DE-SANOFI-SYNTHELABO-A01200802511

Sender diagnosis: not reported

Narrative

Reaction as reported: toxic epidermal necrolysis with lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin

LLT: TOXIC EPIDERMAL NECROLYSIS / 10044223

PT: TOXIC EPIDERMAL NECROLYSIS / 10044223

Narrative:

Case received from the Health authorities through Chugai Pharm (2008CSK_00050) and Merck (6040908) on 25-Feb-2008.

This is a case from the Schoepf registry at the dermatological University Hospital Freiburg which registers and evaluates all cases of hospitalization for Stevens-Johnson-syndrome (SJS)/ toxic epidermal necrolysis (TEN) in Germany and the drugs in temporal relationship with the adverse event received via the German Health authority on 19-Feb-2008.

A 68-year-old female patient had been treated with sucralfate, for gastric ulcer since 24-Feb-2003. On 04-Apr-2003, she experienced lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin. She showed symptoms of Nikolski phenomena. On 09-Apr-2003, she was hospitalized and Steven-Johnson syndrome was diagnosed.

On 17-Apr-2003, the reaction stopped and the patient died on an unknown date.

Medical history included cirrhosis of liver.

Other suspected drugs: spironolactone, ibuprofen (Ibuprofen), metamizol (Novaminsulfon drops), propranolol, xipamid, lactulose, metoclopramid, esomeprazole, phytomenadion, calcium carbonate + colecalciferol, benfotiamin + pyridoxin, omeprazole and diclofenac (Diclofenac).

The reporter assessed Stevens-Johnson-syndrome as: probably related to diclofenac and as possibly related to sucralfate, spironolactone, metamizol, propranolol, xipamid, esomeprazole, omeprazole and doubtful for ibuprofen, lactulose, metoclopramid, phytomenadion and calcium carbonate + colecalciferol, benfotiamin + pyridoxine.

Evaluation by Merck:

A 68-year-old female developed toxic epidermal necrolysis (TEN) with fatal outcome 39 days after introduction of sucralfate (Ulcogant, 2 or 4 dosages forms/d conflicting information received) for ulcerations of upper digestive tract, 49 days after introduction of ibuprofen (taken as required) for pain and 1 day after having taken diclofenac once (dose not provided) for pain.

She was hospitalized 5 days after first signs of TEN. Action taken on drugs was not provided. She died later.

Co-suspected drugs were: spironolactone (dosage between 25 and 150mg, not clearly stated) for ascites, started 36 d prior to and stopped 4 d after occurrence of the reaction; metamizol (taken as required) started 36 d prior to reaction for pain; propranolol 37.5 mg/d for hypertension of V. portae, started 23d prior to reaction; xipamide 20 mg/d for oedema, started 31 d prior to event; lactulose for hepatic cirrhosis, start 31 d prior to reaction; metoclopramide for oesophageal reflux, start 31 d prior to reaction, phytomenadion for substitution, start 31 d prior to event; calciumcarbonate+ colecalciferol for osteoporosis, start 31 d prior to reaction; benfotiamin + pyridoxine for hypo-vitaminosis, start 31 d prior to reaction; esomeprazole (20 mg/d) for helicobacter pylori eradication, start 38 d prior to reaction; omeprazole 40 mg/d as prophylaxis, start 15 d prior to and discontinued 4d after occurrence of reaction;

Concomitant disease: liver cirrhosis possible (not clearly stated).

Company comment: reaction serious, temporal relationship compatible, latencies: 39 d for sucralfate, 49 d for ibuprofen, 1 d for diclofenac. reaction unlisted for sucralfate but listed for ibuprofen and diclofenac as per company reference documents.

Alternative explanation: Metamizol, esomeprazole and omeprazole may have contributed (reaction listed). Liver cirrhosis may have contributed to fatal outcome.

Causality: Possible for all drugs.

This case was also captured in Aventis database with reference 200810959EU.

Sender's comment:

Reporter's comment:

Merck KGaA, 18.04.2008 DE-Merck KGaA-6040908

Sender diagnosis: not reported

Narrative

This is a case from the Schoepf registry at the Dermatological University Hospital Freiburg, Germany which registers and evaluates all cases of hospitalisation for SJS/TEN in Germany and the drugs given in temporal relationship with the adverse event received via the German Health Authority (# 08014098) on 19 Feb 2008.

**

A 68-year-old female patient was treated with Ulcogant suspension (sucralfate, 2x1, 4x1) for gastric ulcer since 24 Feb 2003.

On 02 Apr 2003 she experienced lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin. She showed symptoms of Nikolski phenomena. On 09 Apr 2003 she was hospitalized and Steven-Johnson Syndrome was diagnosed.

On 17 Apr 2003 the event stopped and the patient died on an unknown date.

Medical history included cirrhosis of liver.

Other suspected drugs Aldactone (spironolactone), Ibuprofen (ibuprofen), Novaminsulfon drops (metamizol), Obsidan (propranolol), Aquaphor (xipamid), Hepa-Merz-Syrup (lactulose), MCP drops (metoclopramid), Nexium (esomeprazole), Konakion N (phytomenadion), Calcilac KT (CaCO₃, colecalciferol), Milgamma (benfotiamin, pyridoxin), Omeprazole (omeprazole) and Diclofenac (diclofenac.)

The reporter assessed Steven-Johnson-Syndrome as probably related to Diclofenac and as possibly related to Ulcogant, Aldactone, Novaminsulfon,

Obsidan, Aquaphor, Nexium and Omep. Ibuprofen, Hepa-Merz-Syrup, MCP drops, Konakion N, Calcilac KT, Milgamma were assessed as doubtful related to SJS.

Correction of company assessment on 11 Apr 2008.

The event was not the cause of the death, the event resolved but the patient died on unknown date of unknown cause.

Sender's comment:

A 68-year-old female developed toxic epidermal necrolysis (TEN) with fatal outcome 39 days after introduction of ulcogant (sucralfat, 2 or 4 dosage forms/d - conflicting information received) for ulcerations of upper digestive tract, 49 days after introduction of ibuprofen (taken as required) for pain and 1 day after having taken diclofenac once (dose not provided) for pain. She was hospitalized 5 days after first signs of TEN. Actions taken on drugs not provided. She died later. Co-suspected drugs are: Aldactone (spironolactone, dosage between 25 and 150 mg - not clearly stated) for ascites, started 36d prior to- and stopped 4d after occurrence of event; metamizol (taken as required) started 36d prior to event for pain; Obsidan (propranolol, 37,5 mg/d) for hypertension of V. portae, start 23d prior to event; Aquaphor (xipamide, 20 mg/d) for oedema, start 31d prior to event; Hepa-Merz Sirup (lactulose, dosage not provided) for hepatic cirrhosis, start 31d prior to event; metoclopramide (dosage not provided) for oesophageal reflux, start 31d prior to event; Konakion N (phytomenadion, dosage not provided) for substitution, start 31d prior to event; Calcilac KT (calciumcarbonate / colecalciferol, dosage not provided) for osteoporosis, start 31d prior to event; Milligamma (benfotiamin/pyridoxin, dosage not provided) for hypo-vitaminosis, start 31d prior to event; Esomeprazole (20mg/d) for helicobacter pylori eradication, start 38d prior to event; Omep (omeprazole, 40 mg/d) as prophylaxis, start 15d prior to - and discontinued 4d after occurrence of event; Concomitant diseases: Liver cirrhosis possible (not clearly stated). Company comment: Event serious, temporal relationship compatible, latencies: 39d for ulcogant, 49d for ibuprofen, 1d for diclofenac. Event is unlisted for ulcogant but listed for ibuprofen and diclofenac as per company reference documents. Alternative explanation: Metamizol, esomeprazole and omeprazole may have contributed (event listed). Liver cirrhosis

Reporter's comment:

SANOFI-SYNTHELABO, 25.04.2008 DE-SANOFI-SYNTHELABO-A01200802511

Sender diagnosis: not reported

Narrative

Reaction as reported: toxic epidermal necrolysis with lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin
LLT: TOXIC EPIDERMAL NECROLYSIS / 10044223
PT: TOXIC EPIDERMAL NECROLYSIS / 10044223

Narrative:

Corrected CIOMS on 23-Apr-2008: after internal review by the partner, the reaction was identified as toxic epidermal necrolysis and not Stevens-Johnson syndrome. The outcome was fatal, the patient died from the reaction.

Case received from the Health authorities through Chugai Pharm (2008CSK_00050) and Merck (6040908) on 25-Feb-2008.

This is a case from the Severe Cutaneous Adverse Reactions registry at the dermatological University Hospital Freiburg which registers and evaluates all cases of hospitalization for Stevens-Johnson-syndrome (SJS)/ toxic epidermal necrolysis (TEN) in Germany and the drugs in temporal relationship with the adverse event received via the German Health authority on 19-Feb-2008.

A 68-year-old female patient had been treated with sucralfate, for gastric ulcer since 24-Feb-2003. On 04-Apr-2003, she experienced lip erosions, oral stomatitis, nasal and genital erosions, skin reactions and pain of skin. She showed symptoms of Nikolski phenomena. On 09-Apr-2003, she was hospitalized and Steven-Johnson syndrome was diagnosed.

On 17-Apr-2003, the reaction stopped and the patient died on an unknown date.

Medical history included cirrhosis of liver.

Other suspected drugs: spironolactone, ibuprofen (Ibuprofen), metamizol (Novaminsulfon drops), propranolol, xipamid, lactulose, metoclopramid, esomeprazole, phytomenadion, calcium carbonate + colecalciferol, benfotiamin + pyridoxin, omeprazole and diclofenac (Diclofenac).

The reporter assessed Stevens-Johnson-syndrome as: probably related to diclofenac and as possibly related to sucralfate, spironolactone, metamizol, propranolol, xipamid, esomeprazole, omeprazole and doubtful for ibuprofen, lactulose, metoclopramid, phytomenadion and calcium carbonate + colecalciferol, benfotiamin + pyridoxine.

Evaluation by Merck:

A 68-year-old female developed toxic epidermal necrolysis (TEN) with fatal outcome 39 days after introduction of sucralfate (Ulcogant, 2 or 4 dosages forms/d conflicting information received) for ulcerations of upper digestive tract, 49 days after introduction of ibuprofen (taken as required) for pain and 1 day after having taken diclofenac once (dose not provided) for pain.

She was hospitalized 5 days after first signs of TEN. Action taken on drugs was not provided. She died later.

Co-suspected drugs were: spironolactone (dosage between 25 and 150mg, not clearly stated) for ascites, started 36 d prior to and stopped 4 d after occurrence of the reaction; metamizol (taken as required) started 36 d prior to reaction for pain; propranolol 37.5 mg/d for hypertension of V. portae, started 23d prior to reaction; xipamide 20 mg/d for oedema, started 31 d prior to event; lactulose for hepatic cirrhosis, start 31 d prior to reaction; metoclopramide for oesophageal reflux, start 31 d prior to reaction, phytomenadion for substitution, start 31 d prior to event; calciumcarbonate+ colecalciferol for osteoporosis, start 31 d prior to reaction; benfotiamin + pyridoxine for hypo-vitaminosis, start 31 d prior to reaction; esomeprazole (20 mg/d) for helicobacter pylori eradication, start 38 d prior to reaction; omeprazole 40 mg/d as prophylaxis, start 15 d prior to and discontinued 4d after occurrence of reaction;

Concomitant disease: liver cirrhosis possible (not clearly stated).

Company comment: reaction serious, temporal relationship compatible, latencies: 39 d for sucralfate, 49 d for ibuprofen, 1 d for diclofenac. reaction unlisted for sucralfate but listed for ibuprofen and diclofenac as per company reference documents.

Alternative explanation: Metamizol, esomeprazole and omeprazole may have contributed (reaction listed). Liver cirrhosis may have contributed to fatal outcome.

Causality: Possible for all drugs.

This case was also captured in Aventis database with reference 200810959EU.

Corrected CIOMS on 23-Apr-2008.

The reaction was toxic epidermal necrolysis and not Stevens-Johnson syndrome. The outcome was fatal, the patient died from the reaction.

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08014364

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 84 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema	20050828	20050926	30 Day	606 Day	recovered/resolved
2 Urinary tract infection	20050823	20050926	35 Day	601 Day	recovered/resolved
3 Sleep disorder	20050825	20050825	1 Day	603 Day	recovered/resolved
4 Skin disorder	20050828	20050926	30 Day	606 Day	recovered/resolved
5 Pustule	20050828	20050926	30 Day	606 Day	recovered/resolved
6 Pruritus	20050827	20050926	31 Day	605 Day	recovered/resolved
7 Lip ulceration	20050829	20050926	29 Day	607 Day	recovered/resolved
8 Infection	20050823	20050926	35 Day	601 Day	recovered/resolved
9 Hypokalemia	20050824	20050926	34 Day	602 Day	recovered/resolved
10 Dry cough	20050821			599 Day	recovered/resolved
11 Disturbance in attention	20050822	20050825	4 Day	600 Day	recovered/resolved
12 Depression	20050824			602 Day	recovered/resolved
13 Constipation	20050824	20050926	34 Day	602 Day	recovered/resolved
14 Vigilance decreased	20050822			1 Day	Unknown
15 Fever	20050826	20050926	32 Day	604 Day	recovered/resolved
16 Dyspnea	20050822	20050926	36 Day	600 Day	recovered/resolved
17 Lip erosion	20050828	20050926	30 Day		recovered/resolved
18 Stevens Johnson syndrome	20050828	20050926	30 Day	606 Day	recovered/resolved
19 Nikolsky's sign	20050828	20050926	30 Day		recovered/resolved
20 Macule	20050828	20050926	30 Day		recovered/resolved
21 Stevens-Johnson syndrome	20050828	20050926	30 Day		recovered/resolved
22 Blister	20050828	20050926	30 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc 600 mg acetylcysteine	Oral	600 Mg milligram(s)/Day	20050821- -	Decreased bronchial secretion
2	ciprobay ciprofloxacin hydrochloride	Oral	2 Df dosage form/Day	20050822-20050824 3 Day	Urinary tract infection
3	COTRIM FORTE "HEUMANN" sulfamethoxazole; trimethoprim	Oral	2 Df dosage form/Day	20050824-20050827 4 Day	Urinary tract infection
4	fraxiparin nadroparin calcium	Subcutaneous	0.3 ml millilitre(s)/Day Solution for injection	20050821- -	Thrombosis prophylaxis
5	fraxiparin Injlsq 0,3 nadroparin calcium	Subcutaneous	/	20050821- -	Thrombosis prophylaxis
6	NOVALGIN /00039501/ metamizole	Oral	25 Gtt drop(s)/Day Irrigation solution	20050401-20050821 4 Month	Pain
7	novalgin injektionsloesung metamizole	Intravenous (not otherwis	1 Df dosage form/Day Solution for injection	20050824-20050825 2 Day	Ache
8	novalgin tropfen metamizole sodium	Oral	/	20050401-20050821 4 Month	Ache
9	voltaren resinat diclofenac resinate	Oral	2 Df dosage form/Day Capsule	20050822-20050828 7 Day	Pain
10	VOLTAREN RESINAT diclofenac resinate	Oral	2 Df dosage form/Day Capsule	20040101-20050821 19 Month	Pain

Drugs (concomitant)						
11	ALPHA-VIBOLEX thioctic acid	Oral	300 Mg milligram(s)/Day Capsule	20050301-20050822	5 Month	Prophylaxis
12	CASODEX bicalutamide	Oral	50 Mg milligram(s)/Day Tablet	20030702-	-	Prostate carcinoma
13	CETIRIZIN cetirizine hydrochloride	Oral	10 Mg milligram(s)/Day Tablet	20050827-	-	Prophylaxis
14	DECORTIN prednisone	Oral	10 Mg milligram(s)/Day Tablet	20050615-	-	Chronic obstructive pulmonary disease
15	ELOBACT cefuroxime axetil	Oral	/Day Tablet	20050514-20050814	3 Month	Urinary tract infection
16	ferro sanol ferroglycine sulfate complex	Oral	1 Df dosage form/Day	20050301-	-	Prophylaxis
17	ibuprofen ibuprofen	Oral	/ Tablet	20040101-	-	Analgesic abuse
18	kalinor potassium chloride	Oral	10800 Mg milligram(s)/Day Tablet	20050824-	-	Hypokalemia
19	klyisma disodium hydrogenorthophosphate; potassium sorbate; sodium phosphate monobasic (anhydrate)	Unknown	/	20050824-20050824	1 Day	Constipation
20	LASIX /00032601/ furosemide	Oral	40 Mg milligram(s)/Day Tablet	20050301-	-	Hypertension arterial
21	NEURONTIN gabapentin	Oral	300 Mg milligram(s)/Day	20050822-	-	
22	NEURONTIN gabapentin	Oral	300 Mg milligram(s)/Day Capsule	20050629-20050821	54 Day	Drug use for unknown indication
23	Oxazepam oxazepam	Oral	/ Tablet	20050825-20050825	-	Sleep disorder
24	oxygen oxygen	Respiratory (inhalation)	4 l litre(s)/Day Inhalation Gas	20050821-20050823	3 Day	Chronic obstructive pulmonary disease
25	PANTOZOL /01263202/ pantoprazole sodium	Oral	20 Mg milligram(s)/Day Tablet	20050301-	-	Prophylaxis
26	RINGER calcium chloride; potassium chloride; sodium chloride; sodium chloride compound injection		/Day Nebuliser solution	20050823-20050827	5 Day	Prophylaxis
27	solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwise)	100 Mg milligram(s)/Day Solution for infusion	20050827-20050827	1440 Minute	Skin disorder
28	SPIRIVA tiotropium bromide	Oral	1 Df dosage form/Day Inhalation vapour, solution	20050301-	-	COPD
29	SYMPAL dexketoprofen	Oral	100 Mg milligram(s)/Day Tablet	20050301-20050821	5 Month	Pain
30	TOGAL CLASSIC acetylsalicylic acid; lithium citrate; quinine citrate	Oral	/Day	20040101-	-	Drug abuse
31	ZOLOFT /01011402/ sertraline hydrochloride	Oral	100 Mg milligram(s)/Day Tablet	20020420-	-	Depression

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cystitis			Unknown	
Prostate cancer			Unknown	
Iron deficiency			Unknown	
Chronic pulmonary heart disease			Unknown	chronic pulmonary obstructive disease
Pain NOS			Unknown	pain NOS
Urinary tract infection			Unknown	rezidivierend
Ache			Unknown	
Analgesic abuse			Unknown	
COPD			Unknown	
Hypertension arterial			Unknown	
Gastritis prophylaxis			Unknown	
Depression reactive			Unknown	
Thrombosis prophylaxis	20050821		Unknown	
Constipation	20050824	20050824	Unknown	
Hypokalaemia	20050824		Unknown	

Primary source

Qualification: Physician observestudytype: Other studies
 Literature:

Narratives (nur bei elektronischem Eingang)

Sanofi-Aventis Deutschland GmbH, 25.02.2008 DE-AVENTIS-200810479DE

Sender diagnosis: not reported

Narrative**Initial report**

This case of toxic Stevens-Johnson syndrome (SJS) was received from the German health authority (DE-BFARM-08014364). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9150175). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of SJS was made by an expert panel, who assessed it as a "probable case" of SJS. Causal relationship was assessed by the experts in March 2006 using an algorithm.

This case concerned an 84-year-old male patient, whose medical history was not provided. This patient experienced SJS from 28-Aug-2005 until 26-Sep-2005.

The following non-suspected drugs are known to have been taken by the patient prior to the event:

Cefuroxime axetil, dextetopfen, thioctic acid, ibuprofen, ferrous glycine sulfate, gabapentin, tiotropium bromide, prednisone, furosemide, pantoprazole sodium, sertraline hydrochloride, bicalutamide, oxygen, sodium chloride compound injection, potassium chloride, oxazepam, prednisolone sodium succinate and cetirizine hydrochloride. Indications were specified with recurrent urinary tract infection, pain, substitution, analgetica abuse, analgetica abuse, chronic obstructive pulmonary disease, arterial hypertension, gastric prophylaxis, reactive depression, prostate carcinoma, dyspnoea, thrombosis prophylaxis, secretolysis, obstipation, hypokalemia, sleep disorder, skin disorder and pruritus prophylaxis.

Suspected drugs: Metamizole drops (Novalgin Tropfen) 25 drops, intake from 01-Apr-2005 to 21-AUG-2005 and once on 28-Aug-2005. Metamizole injection solution (Novalgin Injektionsloesung), injection on 28-Aug-2005 (dose not provided), indication of metamizole: Pain.

Diclofenac resinate (indication: analgetic abuse), intake from 01-Jan-2004 to 25-Aug-2005. Ciprofloxacin hydrochloride (indication: urinary tract infection), intake from 22-Aug-2005 to 24-Aug-2005. Heparin-fraction, calcium salt (indication: prophylaxis of thrombosis) intake from 21-Aug-2005).

Acetylcysteine (indication: secretolysis) intake from 21-Aug-2005. Cotrim forte (indication: urinary tract infection) intake from 24-Aug-2005 until 27-Aug-2005.

The patient was hospitalised to internal ward on 22-Aug-2005 (cause not reported) and transferred to dermatological ward on 29-Aug-2005.

On 22-Aug-2005 the patient suffered from dyspnoea and decrease of vigilance, one day later urinary tract infection was diagnosed. Fever occurred on 26-Aug-2005. On 28-Aug-2005 the symptoms of SJS spots and skin blisters, positive Nikolsky's sign and erosions of lips were assigned.

Causality assessment for metamizole drops (Novalgin Tropfen) was given with "possible", metamizole injection solution (Novalgin Injektionsloesung) with "well possible".

For the other suspected drugs causality was assessed as:

"possible" for diclofenac resinate, heparin-fraction, calcium salt and acetylcysteine, "well possible" for ciprofloxacin hydrochloride, "probable" for cotrim forte.

Cystitis and prostate carcinoma considered "possible" causes of SJS.

Laboratory: no values provided.

Sender's comment:

21-FEB-2008 Sanofi-Aventis Company Comment: Due to the compatible temporal relationship the causal role of metamizole cannot be excluded. However, the numerous concomitant medication as well as the underlying cystitis and prostatic carcinoma could be discussed as possible alternative causes of the reported reaction.

21-FEB-2008 Sanofi Aventis Germany: STEVENS JOHNSON SYNDROME is considered locally labelled for metamizole sodium (Novalgin). Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status of the suspected drug are not deemed necessary.

Reporter's comment:

NOVARTIS, 25.02.2008 DE-002147023-PHNU2008DE01008

Sender diagnosis: not reported

Narrative

Initial report received on 18 Feb 2008 (report from Intensive Monitoring Project, Schoepf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9170332). This multimorbid and polymedicated patient had hospitalizations from 22 Aug 2005 until 29 Aug 2005 and 29 Aug 2005 outcome date not reported. Voltaren resinat (diclofenac) was used for pain. The patient had received multiple drugs prior and during the following weeks. On 22 Aug 2005, patient showed dyspnoe, disturbance in attention, on 23 Aug urinary tract infection, on 26 Aug fever with erythema and pustules on skin on 28 Aug 2005, on 29 Aug 2005 lip ulceration was found. Further events were skin disorder, pruritus. Dry cough was treated with Acetylcystein. Constipation 24 Aug 2005, hypocalcemia, depression. The diagnosis of toxic epidermal lysis (Steven Johnson Syndrom) was made. Patient was preliminary set on Ciprobay (ciprofloxacin) and Cotrim forte (trimethoprim, sulfamethoxacol) therapy due to urinary infection with fever on 26 Aug 2005. The patient received thrombosis prophylaxis with fraxiparin and due to hypertension he received Lasix (furosemid). Constipation was noted on 24 Dec 2005. Novalgin (metamizol) was given on 01 Apr 2005 and Voltaren since 01 Jan 2004, respectively, for pain. Sleeping disorder with disturbance in attention was reported on 25 Aug 2005. The patient recovered. As suspect drugs, the following were recorded: Voltaren resinat, Ciprobay (ciprofloxacin), Fraxiparin (Heparin Calcium), Novalgin (metamizol), Acetylcystein, Cotrim forte (trimethoprim, sulfamethoxacol).

Sender's comment:

Serious adverse event report [hospitalisation], Urinary tract infection, Hypokalaemia assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.. Reporting healthcare professional causality is suspected.

Serious adverse event [hospitalisation], Sleep disorder, Dyspnoea, Depression, Stevens-Johnson-syndrome, Constipation assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

NOVARTIS, 25.02.2008 DE-002147023-PHNU2008DE01008

Sender diagnosis: not reported

Narrative

Initial report received on 18 Feb 2008 (report from Intensive Monitoring Project, Schoepf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9170332). This multimorbid and polymedicated patient had hospitalizations from 22 Aug 2005 until 29 Aug 2005 and 29 Aug 2005 outcome date not reported. Voltaren resinat (diclofenac) was used for pain. The patient had received multiple drugs prior and during the following weeks. On 22 Aug 2005, patient showed dyspnoe, disturbance in attention, on 23 Aug urinary tract infection, on 26 Aug fever with erythema and pustules on skin on 28 Aug 2005, on 29 Aug 2005 lip ulceration was found. Further events were skin disorder, pruritus. Dry cough was treated with Acetylcystein. Constipation 24 Aug 2005, hypocalcemia, depression. The diagnosis of toxic epidermal lysis (Steven Johnson Syndrom) was made. Patient was preliminary set on Ciprobay (ciprofloxacin) and Cotrim forte (trimethoprim, sulfamethoxacol) therapy due to urinary infection with fever on 26 Aug 2005. The patient received thrombosis prophylaxis with fraxiparin and due to hypertension he received Lasix (furosemid). Constipation was noted on 24 Dec 2005. Novalgin (metamizol) was given on 01 Apr 2005 and Voltaren since 01 Jan 2004, respectively, for pain. Sleeping disorder with disturbance in attention was reported on 25 Aug 2005. The patient recovered. As suspect drugs, the following were recorded: Voltaren resinat,

Ciprobay (ciprofloxacin), Fraxiparin (Heparin Calcium), Novalgin (metamizol), Acetylcystein, Cotrim forte (trimethoprim, sulfamethoxacol).

Sender's comment:

Serious adverse event report [hospitalisation], Urinary tract infection, Hypokalaemia assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.. Reporting healthcare professional causality is suspected.

Serious adverse event [hospitalisation], Sleep disorder, Dyspnea, Depression, Stevens-Johnson-syndrome, Constipation assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

NOVARTIS, 26.02.2008 DE-002147023-PHNU2008DE01008

Sender diagnosis: not reported

Narrative

Initial report received on 18 Feb 2008 (report from Intensive Monitoring Project, Schoepf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9170332). This multimorbid and polymedicated patient had hospitalizations from 22 Aug 2005 until 29 Aug 2005 and 29 Aug 2005 outcome date not reported. Voltaren resinat (diclofenac) was used for pain. The patient had received multiple drugs prior and during the following weeks. On 22 Aug 2005, patient showed dyspnoe, disturbance in attention, on 23 Aug urinary tract infection, on 26 Aug fever with erythema and pustules on skin on 28 Aug 2005, on 29 Aug 2005 lip ulceration was found. Further events were skin disorder, pruritus. Dry cough was treated with Acetylcystein. Constipation 24 Aug 2005, hypocalcemia, depression. The diagnosis of toxic epidermal lysis (Steven Johnson Syndrom) was made. Patient was preliminary set on Ciprobay (ciprofloxacin) and Cotrim forte (trimethoprim, sulfamethoxacol) therapy due to urinary infection with fever on 26 Aug 2005. The patient received thrombosis prophylaxis with fraxiparin and due to hypertension he received Lasix (furosemid). Constipation was noted on 24 Dec 2005. Novalgin (metamizol) was given on 01 Apr 2005 and Voltaren since 01 Jan 2004, respectively, for pain. Sleeping disorder with disturbance in attention was reported on 25 Aug 2005. The patient recovered. As suspect drugs, the following were recorded: Voltaren resinat, Ciprobay (ciprofloxacin), Fraxiparin (Heparin Calcium), Novalgin (metamizol), Acetylcystein, Cotrim forte (trimethoprim, sulfamethoxacol).

Sender's comment:

Serious adverse event report [hospitalisation], Urinary tract infection, Hypokalaemia assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.. Reporting healthcare professional causality is suspected.

Serious adverse event [hospitalisation], Sleep disorder, Dyspnea, Depression, Stevens-Johnson-syndrome, Constipation assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

NOVARTIS, 26.02.2008 DE-002147023-PHNU2008DE01008

Sender diagnosis: not reported

Narrative

Initial report received on 18 Feb 2008 (report from Intensive Monitoring Project, Schoepf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9170332). This multimorbid and polymedicated patient had hospitalizations from 22 Aug 2005 until 29 Aug 2005 and 29 Aug 2005 outcome date not reported. Voltaren resinat (diclofenac) was used for pain. The patient had received multiple drugs prior and during the following weeks. On 22 Aug 2005, patient showed dyspnoe, disturbance in attention, on 23 Aug urinary tract infection, on 26 Aug fever with erythema and pustules on skin on 28 Aug 2005, on 29 Aug 2005 lip ulceration was found. Further events were skin disorder, pruritus. Dry cough was treated with Acetylcystein. Constipation 24 Aug 2005, hypocalcemia, depression. The diagnosis of toxic epidermal lysis (Steven Johnson Syndrom) was made. Patient was

preliminary set on Ciprobay (ciprofloxacin) and Cotrim forte (trimethoprim, sulfamethoxacol) therapy due to urinary infection with fever on 26 Aug 2005. The patient received thrombosis prophylaxis with fraxiparin and due to hypertension he received Lasix (furosemid). Constipation was noted on 24 Dec 2005. Novalgin (metamizol) was given on 01 Apr 2005 and Voltaren since 01 Jan 2004, respectively, for pain. Sleeping disorder with disturbance in attention was reported on 25 Aug 2005. The patient recovered. As suspect drugs, the following were recorded: Voltaren resinat, Ciprobay (ciprofloxacin), Fraxiparin (Heparin Calcium), Novalgin (metamizol), Acetylcystein, Cotrim forte (trimethoprim, sulfamethoxacol).

Sender's comment:

Serious adverse event report [hospitalisation], Urinary tract infection, Hypokalaemia assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.. Reporting healthcare professional causality is suspected.

Serious adverse event [hospitalisation], Sleep disorder, Dyspnea, Depression, Stevens-Johnson-syndrome, Constipation assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

Bayer Vital GmbH, 29.02.2008 DE-BAYER-200813559GPV

Sender diagnosis: not reported

Narrative

This post-marketing study report was received on 19-Feb-2008 by BFARM, the regulatory authority of Germany, referring to an 84 year-old male.

This is a solicited report from the Documentation Center for Severe Skin Reaction in Germany.

Patient had concomitant conditions of pain NOS, recurrent urinary tract infection, chronic obstructive pulmonary disease, arterial hypertension, reactive depression, and prostate carcinoma. Concomitant drugs have been given. For details see section "Concomitant Drugs" at the end of this report.

Patient received:

- Ciprobay (ciprofloxacin) for urinary tract infection, Administration began on 22-Aug-2005, for a duration of 2 days, Formulation: orally
- Voltaren Resinat (diclofenac resinate) for analgetic therapy (resulting in analgetic abusos), administration began on 01-Jan-2004, for a duration of approximately 1 year and 8 months, Formulation: as needed; one tablet orally
- Novalgin (metamizole sodium) for pain nos, administration began on 01-Apr-2004, for a duration of approximately 1 year and 4 months, Formulation: orally drops, and also administered on 28-Aug-2005, Formulation: intravenous injection
- Fraxiparin (nadroparin calcium) for thrombosis prophylaxis, administered on 21-Aug-2005, Formulation: subcutaneous
- ACC 600 (acetylcysteine) for secretolysis, administered on 21-Aug-2005, Formulation: orally tablet
- Cotrim Forte for urinary tract infection, administration began on 24-Aug-2005, for a duration of 3 days, Formulation: orally

The patient was hospitalized from 22-Aug-2005 to 29-Aug-2005 in internal medicine unit, since 29-Aug-2005 dermatological unit.

On 22-Aug-2005, on the same day of starting treatment that was ongoing with Ciprobay, approximately 1 year and 8 months after starting treatment that was ongoing with Voltaren Resinat, approximately 1 year and 4 months after starting treatment that was ongoing with Novalgin, 1 day after starting treatment with Fraxiparin, 1 day after starting treatment with ACC 600, and 2 days before starting treatment that was ongoing with Cotrim Forte patient experienced DYSPNEA considered serious due to hospitalization. Event was treated with oxygen. Outcome unknown at time of report. During the same period patient experienced HYPOVIGILANCE, considered non-serious. Outcome unknown at time of report. Six days later on 28-Aug-2005, patient experienced STEVENS-JOHNSON-SYNDROME (hypovigilance, fever, maculae, blistering, positive Nikolsky's sign, erosive lips) considered serious due to important medical event, and hospitalization. Event was treated with Solu-Decortin H (prednisolone), Ceterizin. Patient recovered after 4 weeks and 2 days.

The investigator considered the diagnoses STEVENS-JOHNSON-SYNDROME and DYSPNEA to be associated.

Sender's comment:

The reported events STEVENS-JOHNSON-SYNDROME and DYSPNOE are listed in the international product information for Ciprobay (ciprofloxacin) and considered serious due to prolonged hospitalisation. The reported event HYPOVIGILANCE is unlisted and non-serious. Based on the information received for this case a causal role of Ciprobay (ciprofloxacin) in the occurrence of the reported events cannot be completely excluded due to a compatible temporal relationship. However, the event DYSPNOE might be more likely explained by an exacerbation of the patient's concomitant

obstructive lung disease in context with the urinary tract infection. Oxygen desaturation could have lead to HYPOVIGILANCE then. DYSPNOE and HYPOVIGILANCE might have been also the reason for discontinuation of Ciprobay after only 2 days of therapy. Regarding temporal relations in this case several drugs could be involved in the development of the STEVENS-JOHNSON-SYNDROME (e.g. Nsaids, Co-trim forte). The short therapy duration of 2 days and the discontinuation of Ciprobay 5 days prior the event onset suggest that in this case the STEVENS-JOHNSON-SYNDROME should be attributed more likely to one of the other medications, which were ongoing or discontinued shortly before onset.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08015101

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 71 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20030618	20030723	36 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac diclofenac	Oral	1 Df dosage form/	20030522-	Ache
2 spiro spironolactone	Oral	/	20030522-	Diuresis

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08015199

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exsudativum multiforme	20031015	20031031	17 Day		recovered/resolved
2 Flu	20031013	20031017	5 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	1 Df dosage form/	19960101-	Pain NOS
diclofenac			-	
2 mediabet 500 mg	Oral	/Day	20030908-	Diabetes mellitus
metformin hydrochloride			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Cardiac failure			Yes	
Arterial hypotension			Yes	
Pruritus			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08015243

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 35 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pyelonephritis	20031207				Unknown
2 Stevens Johnson syndrome	20031210	20040116	38 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol 300 mg		300 Mg milligram(s)/Day	-	Nephrolithiasis
allopurinol			-	
2 diclofenac supp. heumann	Rectal	/Day	20031111-	Pain NOS
diclofenac sodium			-	
3 urol mono		3* /Day	20031111-	Nephrolithiasis
solidaginis extract			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Conjunctivitis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08015815

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 29 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema	20041111			73072 Day	recovered/resolved
2 Erythema exsudativum multiforme	20041111	20041125	15 Day	24 Day	recovered/resolved
3 Eye inflammation	20041111			18 Day	recovered/resolved
4 Pruritus	20041111			18 Day	recovered/resolved
5 Mucosal erosion	20041111			24 Day	recovered/resolved
6 Blister	20041111			24 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMBROXOL ambroxol	Oral	/	20041107- -	Cold
2 Diclac 75 mg diclofenac	Oral	75 Mg milligram(s)/Day Gastro-resistant tablet	20041019-20041111 24 Day	Pain
3 Kodein codeine	Oral	/	20041107- -	Cough

Drugs (concomitant)

4 jodid potassium iodide	Oral	/	20030301- -	
5 PARACETAMOL paracetamol	Oral	/	20041107- -	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Goiter nodular			Unknown	
Influenza like illness	20041105		Unknown	
Cough			Unknown	
Mycoplasma infection NOS			Unknown	
Herpes infection	20041101		Unknown	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 28.02.2008 DE-SZ09-GXKR2008DE01831

Sender diagnosis: not reported

Narrative

Registry report from the Freiburg Documentation Center for Severe Skin Reactions received on 20 Feb 2008 from the German health authority (BfArM case number: DE-BFARM-08015815): This patient with a medical history of mycoplasma infection, herpes eruption and influenza like illness developed an erythema exsudativum multiforme majus with pruritus, blister, eye inflammation, cochades, blister, lip and oral mucosal erosions during treatment with Diclac (diclofenac) for pain taken at a daily dosage of 75 mg from 19 Oct 2004 until 11 Nov 2004. Concomitantly the patient received several other drugs, which were considered suspected in question. The patient had an herpes infection on 01 Nov 2004 and an influenza like illness occurred on 05 Nov 2004. On 11 Nov 2004 the patient developed the above mentioned adverse events and was hospitalised. The adverse event lasted until 25 Nov 2004 and the patient completely recovered. The reporter assessed the causality between adverse event and administration of Diclac as possible. The causality between adverse event and underlying diseases (mycoplasmen infection, herpes) was assessed as probably related

by the reporter.

Sender's comment:

Serious adverse event report of erythema multiforme (hospitalisation) assessed as listed according to the Core Data Sheet Information. Reporting physician causality is suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08015838

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Male Birth: privacy Age: 37 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Mucosal erosion	20041009			27 Day	not recovered/not resolved
2 Stevens Johnson syndrome	20041006			24 Day	not recovered/not resolved
3 Conjunctivitis	20041008			26 Day	not recovered/not resolved
4 Pain of skin	20041006			24 Day	not recovered/not resolved
5 Hyperuricaemia	20040917			5 Day	Unknown
6 Vesicles	20041010			28 Day	not recovered/not resolved
7 Rash (nonspecific)	20041007			25 Day	not recovered/not resolved
8 Pruritus	20041006			24 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol	Oral	300 Mg milligram(s)/Day	20040917-20041010	Hyperuricaemia
allopurinol			24 Day	
2 Nexium mups 40 mg magensaftresistente Tabletten	Oral	40 Mg milligram(s)/Day	20040913-20041010	Reflux oesophagitis
esomeprazole magnesium			28 Day	
3 Voltaren Resinat	Oral	1 Df dosage form/Day	20040913-20041010	Chronic polyarthritis
diclofenac resinate		Capsule	28 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Reflux oesophagitis			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 13.02.2008 DE-002147023-PHNU2008DE00811

Sender diagnosis: not reported

Narrative

Initial report received on 04 Feb 2008 (report from Intensive Monitoring Project, Schäpf-Mockenhaupt-SCAR Project = RegiSCAR project, interview number 9160190). This patient was treated with Voltaren Resinat (diclofenac, colestyramin) since 13 Sep 2004, for chronic polyarthritis, and Nexium (esomeprazole) was started on the same day, for reflux oesophagitis. Allopurinol was initiated on 17 Sep 2004, due to hyperuricemia (hyperuricemia was not mentioned as adverse event on the documentation sheet). On 6 Oct 2004, patient complained about pains of skin and pruritus; and on the following day, cocards and spots were visible. Conjunctivitis started on 8 Oct 2004, and mouth mucosal erosions were seen by 9 Oct. The patient was hospitalized on 10 Oct 2004, with skin vesicles and lip and nasal erosions. The diagnosis of Stevens Johnson syndrome (confirmed case) was made (onset date 6 Oct 2004). Outcome was reported as "not yet recovered". As suspect drugs, the expert from the skin reaction center noted Voltaren Resinat, Nexium and allopurinol; other factors were not suspected as causes.

Sender's comment:

Serious adverse event [hospitalisation], Stevens-Johnson syndrome assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Serious adverse event [medically significant], Hyperuricaemia assessed as unlisted according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08016966

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	No	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Necrolysis epidermal toxic (Lyell type)	20050118	20050129	12 Day	28 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aquaphor xipamide	Oral	20 Mg milligram(s)/Day Tablet	20041204-20050109 19 Day	Oedema
2 diclofenac diclofenac	Oral	2 Df dosage form/Day	20040105-	-
3 furosemide furosemide	Oral	80 Mg milligram(s)/Day	20050109-	-
4 lactulose al sirup lactulose	Oral	30 Mg milligram(s)/Day Syrup	20041221-	-
5 pantozol pantoprazole sodium	Oral	80 Mg milligram(s)/Day	20050109-	-
6 spironolacton spironolactone	Oral	100 Mg milligram(s)/Day	20041204-	-

Drugs (concomitant)

7 CEFTRIAXONE ceftriaxone	Unknown	/	20050111-	Peritonitis
8 fenistil dimetindene maleate	Unknown	/	20050118-	Erythema
9 spironolacton spironolactone	Unknown	50 Mg milligram(s)/Day	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hepatic cirrhosis			Unknown	C3-dekomp. Leberzirrhose
Peritonitis			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

STADA Arzneimittel AG, 03.03.2008 DE-STADA-009693

Sender diagnosis: not reported

Narrative

This report was received from the German Center of Severe Skin Reactions (interview no. 9160238) via German Authority (case no. DE-BFARM-08016966).

A 44-year-old male patient received diclofenac (orally, 1 DF twice daily, from 2004-Jan-05 to 2005-Jan-09, indication pain), xipamide (orally, 0.5 DF in the morning, from 2004-Dec-21 to 2005-Jan-09, indication edema/ascites), spironolactone (orally, 50 mg once or twice daily, from 2004-Dec-21 to 2005-Jan-19, indication ascites), lactulose (orally, 10 mL [corrected from 10 mg] three times daily, from 2004-Dec-21 to 2005-Jan-19, indication hepatic encephalopathy prophylaxis), furosemide (orally, 40 mg twice daily, from 2005-Jan-09 to 2005-Jan-19, indication ascites), pantozole (40 mg twice daily, from 2005-Jan-09 to 2005-Jan-19, indication gastric ulcer), amino acids/acetic acid (drip, from 2005-Jan-11 to 2005-Jan-13, indication liver cirrhosis), and ceftriaxone (injection, from 2005-Jan-11 to 2005-Jan-18, indication peritonitis).

On 2005-Jan-18, the patient developed toxic epidermal necrolysis with maculae which prolonged hospitalization. The symptoms started 2005-Jan-18

with skin pain and deteriorated on 2005-Jan-19 with bullae. On 2005-Jan-24 the patient developed erosions of lips and on 2007-Jan-25 of oral mucosa. Actions regarding the event included intensive care unit from 2005-Jan-21 to 2005-Jan-24. The patient died on 2005-Jan-29. An autopsy was not reported. Concomitant medication included dimetindene.

The reporter assessed the causal relationship between the adverse reaction and ceftriaxone with score 4, between the adverse reaction and diclofenac, xipamide, spironolactone, lactulose, furosemide and pantoprazole with score 5. He suspected explicitly ceftriaxone and diclofenac.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

DICLOFENAC

The MAH/MAA assessed the causal relationship between the suspected adverse reaction and diclofenac as probably related based on the temporal association and the known safety profile of diclofenac which includes severe skin reactions like TEN which is expected according to the current reference SPC.

SPIRONOLACTONE

The MAH/MAA assessed the causal relationship between the suspected adverse reaction and spironolactone as possibly related based on the temporal association. Toxic epidermal necrolysis is unexpected according to the current reference SPC.

LACTULOSE

The MAH/MAA assessed the causal relationship between the suspected adverse reaction and lactulose as unlikely related based on the known safety profile of lactulose. Toxic epidermal necrolysis is unexpected according to the current reference SPC.

FUROSEMIDE

The MAH/MAA assessed the causal relationship between the suspected adverse reaction and furosemide as possibly related based on the temporal association and the known safety profile of furosemide which includes skin reactions. Toxic epidermal necrolysis is unexpected according to the current reference SPC.

CEFTRIAZONE

The MAH/MAA assessed the causal relationship between the suspected adverse reaction and ceftriazone as probably related based on the temporal association and the known safety profile of ceftriazone which includes severe skin reactions. TEN is expected according to the current reference SPC.

This case does not change the overall benefit-risk balance of the medicinal products.

Reporter's comment:

Lilly Deutschland GmbH, 05.03.2008 DE-ELI LILLY AND COMPANY-DE200803000051

Sender diagnosis: not reported

Narrative

This spontaneous case reported by a physician via a regulatory agency concerns a 47 year old male patient of unknown origin.

The medical history of the patient was not reported. Concomitant medications included diclofenac, lactulose, furosemide, spironolactone, pantoprazole, all for an unreported indications, dimetindene for erythema and ceftriazone for peritonitis.

The patient received xipamide (Aquaphor), 20mg half tablet in the morning, for the treatment oedema, beginning on 22-Dec-2004. On 09-Jan-2005, xipamide was discontinued. On 18-Jan-2005, ten days after stopping xipamide, the patient experienced pain of skin, cockades and spots; on 19-Jan-2005 the patient developed skin bubbles; on 20-Jan-2005 the patient had fever up to 39.8 degrees; on 24-Jan-2005 the patient developed erosions of lips and on 25-Jan-2005 the patient developed MSH (to be clarified) erosions. On an unspecified date the patient was diagnosed toxic epidermal necrolysis with macula and died on an unreported date. No cause of death was reported. It was not reported if an autopsy was performed. No Laboratory data was provided.

No reporter contact, lot number not provided.

No opinion of relatedness was provided by the reporting physician.

Sender's comment:

Bewertung - 5. März 2008

Verdachtsfall: schwerwiegend und unbekannt

Der Kausalzusammenhang ist denkbar.

Allerdings bewirkt der Fall keine Aenderung des wissenschaftlichen Erkenntnisstandes. Weitergehende Massnahmen nach § 29 Abs. 1 AMG werden nicht fuer erforderlich gehalten.

Reporter's comment:

APHQWEB, 11.03.2008 DE-Nycomed GmbH-80483

Sender diagnosis: not reported

Narrative

This report was received from the German Documentation centre of severe skin reactions (ref no. 9160238) via the German Health authority (BfArM ref. no. DE-BFARM-08016966).

A middle-aged male patient experienced toxic epidermal necrolysis with maculae while receiving pantoprazole (Pantozol 40 mg p.o.) for stomach ulcer. Further 9 suspected drugs were administered.

09-Jan-2005 to 20-Jan-2005: hospitalisation, internal medicine.

From 11-Jan to 18-Jan, the patient received antibiotic treatment with ceftriaxon for peritonitis.

18-Jan-2005: patient developed pain of the skin, cockades and maculae.

19-Jan-2005: patient developed skin blisters

20-Jan-2005 to 21-Jan-2005: hospitalisation in dermatology department. Patient developed fever up to 39.8°C.

21-Jan-2005 to 24-Jan-2005: intensive care unit. On 24-Jan patient developed erosions of lips and on 25-Jan erosions of the oral mucosa.

The reaction was rated as life-threatening and the patient died. No information regarding autopsy report was provided.

Concomitant diseases included pain, oedema, abdominal exudate, stomach ulcer, liver cirrhosis, peritonitis and erythema. The Intensive Monitoring Project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels ranging from 1=definite to 8=improbable). Causality to pantoprazole was rated as "possible", diclophenac and ceftriaxon were considered as highly suspected and assessed as "possible" or "well possible" respectively due to this algorithm.

This patient developed toxic epidermal necrolysis with maculae after having been on pantoprazole treatment for approximately 9 days. Concomitantly, the patient received several other drugs (9 further suspected drugs).

Toxic epidermal necrolysis is a rare skin disorder, characterized by mucosal erosions, skin blisters, fever. It can be drug-induced or due to infectious diseases. Pre-existing liver or renal disorders may have an influence on the event.

Several drugs have been observed to cause toxic epidermal necrolysis. Toxic bullous skin reactions up to acute disseminated epidermal necrolysis have also been observed in very rare cases with pantoprazole and other proton pump inhibitors. Although toxic epidermal necrolysis is considered labelled for pantoprazole, this event was considered to be unlabeled, since pantoprazole cases with "fatal outcome" are considered as unlabelled. However, the mortality is high in general for patients suffering from Stevens-Johnson syndrome and toxic epidermal necrolysis (20-25%; Mockenhaupt et al 2008).

Temporal relationship basically plausible. Mockenhaupt et al (2008) suggest, that toxic epidermal necrolysis often begins more than four and up to 28 days after the initiation of the responsible medication. The patient had been on treatment with pantoprazole for approximately nine days before onset of the event. The patient received multiple other suspected drugs (ceftriaxon and diclophenac had been considered as highly suspected drugs. In addition, the patient concomitantly suffered from peritonitis and liver cirrhosis.

Sender's comment:

Although temporal relationship to pantoprazole therapy seems reasonable, there are some factors that point to a more reasonable relationship to the event (i.e. antibiotic treatment with ceftriaxon, diclophenac therapy and peritonitis). The pre-existing liver cirrhosis with possible impact on drug

metabolism could have been a contributing factor. However, according to the multiple factors that were present in this case, it is difficult to determine a single cause for onset of toxic epidermal necrolysis with maculae.

Therefore Nycomed agrees with the reporter and conservatively assesses as possible. This report is not considered to have any influence on current knowledge regarding the safety profile of pantoprazole and has no effect on the status of the drug as reflected by the formal product characteristics.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08017073

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 56 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blister	20041221			6 Day	recovering/resolving
2 Conjunctivitis	20041221			6 Day	recovering/resolving
3 Erythema	20041220			5 Day	recovering/resolving
4 Fever	20041220			5 Day	recovering/resolving
5 Lip swelling	20041219			4 Day	recovering/resolving
6 Mucosal inflammation	20041221			6 Day	recovering/resolving
7 Pain of skin	20041220			5 Day	recovering/resolving
8 Toxic epidermal necrolysis	20041221			6 Day	recovering/resolving
9 Lip erosion	20041220			5 Day	recovering/resolving
10 Mucosal erosion	20041221			6 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol	Oral	300 Mg milligram(s)/Day	20041125-	Hyperuricaemia
allopurinol			-	
2 diclac	Oral	100 Mg milligram(s)/Day	20041216-	Coxitis
diclofenac			-	
3 GLUCOPHAGE	Oral	/	20030601-	Diabetes mellitus
metformin			-	
4 levothyroxine	Oral	200 µg microgram(s)/Day	20030501-	
levothyroxine			-	
5 paracetamol comp	Oral	1 Df dosage form/Day	20041115-	Pain
paracetamol			-	
6 zolpidem	Oral	10 Mg milligram(s)/Day	20041201-	Disorder sleep
zolpidem			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypothyroidism			Yes	
Coxitis			Yes	
Cold			Unknown	
Hyperuricaemia			Yes	
Sleep disorder			Yes	
Diabetes mellitus non-insulin-dependent			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 29.02.2008 DE-SZ09-GXKR2008DE01894

Sender diagnosis: not reported

Narrative

Initial registry report from the Freiburg Documentation Centre for Severe Skin Reactions received on 25 Feb 2008 from the German health authority

(BfArM case number: DE-BFARM-08017073): This patient with medical history of diabetes mellitus, hypothyroidism, hyperuricaemia and sleep disorder developed Toxic epidermal necrolysis (TEN) under treatment with Diclac (diclofenac) taken for coxitis pain at a daily dosage of 100 mg since 16 Dec 2004. Concomitantly several other drugs were taken which were considered causative: Among others L-Thyrox (levothyroxine, MAH unknown), allopurinol (MAH unknown), zolpidem (MAH unknown), Glucophage (metformine) and Paracetamol Comp (paracetamol/codeine) were considered causative. The patient had a common cold on 18 Dec 2004 and developed on 19 Dec 2004 a TEN with the first symptom of lip swelling. On the next day fever up to 39.5 degree Celsius, pain of skin, cockades and maculae as well as lip erosions occurred. On 21 Dec 2004 the patient was hospitalized with blisters, Nikolsky's sign, conjunctivitis and oral mucosal erosions. On the next day the genital mucosa was reddened. The adverse event lasted until 17 Feb 2005, but at the time of the report the patient had not yet recovered. The reporter assessed the causality between onset of adverse event and administration of diclofenac and zolpidem as possible, for levothyroxine as questionable and for allopurinol as very probable. The causal relationship between adverse event and cold assessed the reporter as possible.

Sender's comment:

For diclofenac and allopurinol:

Serious adverse event report (life-threatening) of Toxic epidermal necrolysis, assessed as listed according to the Core Data Sheet Information. Reporting physician causality is suspected.

For levothyroxine and zolpidem:

Serious adverse event report (life-threatening) of Toxic epidermal necrolysis, assessed as unlisted according to the Core Data Sheet Information. However, other alternative causes (other drugs, common cold) provide a possible explanation for the reported adverse event. Reporting physician causality is suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08017001

Serious Yes	Death Yes	Lifethr. Yes	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 71 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20041231	deathcause: Toxic epidermal necrolysis
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20041212			11 Day	fatal
2 Lip erosion	20041212		20 Day		fatal
3 Oral ulceration	20041212		20 Day		fatal
4 Nikolsky's sign	20041212		20 Day		fatal
5 Macule	20041212		20 Day		fatal
6 Eyes red	20041212		20 Day		fatal
7 Blister	20041212		20 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CEFUROXIM cefuroxime	Intravenous (not otherwis	4.5 G gram(s)/Day	20041130-20041202	Infection prophylaxis
2 clexane enoxaparin	Subcutaneous	0.4 Mg milligram(s)/Day	20041125-20041202	Thromboembolism prophylaxis
3 NEURONTIN gabapentin	Oral	300 Mg milligram(s)/Day	20041201-20041202	Pain
4 NEUROTININ gabapentin	Oral	300 Mg milligram(s)/Day	20041201-20041202	Pain
5 NEXIUM esomeprazole magnesium	Oral	/ Modified-release tablet	20041202-20041210	Gastric pain
6 NOVALGIN metamizole	Oral	20 Gtt drop(s)/Day	20041126-20041126	Pain
7 NOVALGIN metamizole	Oral	20 Gtt drop(s)/Day	20041206-20041210	Pain
8 NOVALGIN metamizole	Oral	20 Gtt drop(s)/ Irrigation solution	20041126-20041126	Pain
9 PASPERTIN metoclopramide hydrochloride	Oral	15 Gtt drop(s)/Day	20041206-20041210	Nausea
10 TRAMAL tramadol hydrochloride	Intravenous (not otherwis	/24 Hour Injection	20041129-20041129	Pain
11 voltaren - slow release diclofenac sodium	Oral	100 Mg milligram(s)/Day	20041202-20041203	Pain
12 voltaren retard diclofenac sodium	Oral	100 Mg milligram(s)/Day	20041202-20041203	Pain
13 voltaren retard diclofenac sodium	Oral	100 Mg milligram(s)/Day	20041202-20041203	Coxarthrosis
14 ZOCOR simvastatin	Oral	40 Mg milligram(s)/Day	20041202-20041210	Hypercholesteremia
15 ZYLORIC allopurinol	Oral	300 Mg milligram(s)/Day	20041209-20041210	Tumour lysis syndrome

Drugs (concomitant)

16	ESIDRIX hydrochlorothiazide	Oral	12.5 Mg milligram(s)/Day	-20041210	Hypertension
17	KALINOR /00031402/ potassium chloride	Oral	/Day Effervescent granules	20041130-20041204	Electrolyte replacement
18	kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	1 Df dosage form/Day	20041126-20041127	Supplementation therapy
19	kalinor-brausetabletten potassium carbonate; potassium citrate	Oral	1 Df dosage form/Day	20041130-20041204	Supplementation therapy
20	kalium-duriles potassium chloride	Oral	6 Df dosage form/Day	20041205-20041210	Supplementation therapy
21	MUTAFLOR escherichia coli	Oral	/	20041201-20041211	Meteorism
22	OXAZEPAM oxazepam	Oral	/	20041125-20041125	Sleep disorder
23	PAN-OPHTAL dexpanthenol	Topical	/ Eye ointment	20041208-20041211	Eye pain
24	Pan-Ophtal Augensalbe dexpanthenol		/	20041208-20041211	Eye pain
25	pres plus enalapril maleate; hydrochlorothiazide	Oral	/	-20041210	Hypertension
26	VINCRISTIN vincristine	Intravenous (not otherwis	2 Mg milligram(s)/Day	20041210-20041210	Lymphoma

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bone marrow biopsy			No	
Fever	20041225		Yes	
Gastric lymphoma			Yes	
Pruritus	20041210		Yes	
B-cell lymphoma			No	High-malignant B-cell lymphoma of stomach
Tumor lysis syndrome			No	
Eye pain			No	
Nausea			No	
Gastric pain			No	
Hypercholesterolemia			No	
Hip arthrosis			No	
Meteorism			No	
Hypertension			No	
Sleep disorder			No	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

PFIZER INC, 19.02.2008 DE-PFIZER INC-2008011019

Sender diagnosis: not reported

Narrative

This is a report from the German Documentation Centre of Severe Skin Reactions, a sponsored project set up to collect suspected drug induced severe skin reactions leading to hospital admission. This physician reports that this 71 year old female patient has a medical history of sleep disorder, hypertension, meteorism, arthrosis of the hip, hypercholesterolemia, gastric pain, nausea, pain of the eyes, tumor lysis syndrome and high-malignant B-cell lymphoma of the stomach. The patient developed fever up to 39.4 degree Centigrade on 25Nov2004 and was hospitalized on 25Nov2004. On 30Nov2004 an incisional biopsy of the iliac crest was performed. She was treated with Neurontin (gabapentin) 300 mg per day due to pain from 01Dec2004 until 02Dec2004. Fever up to 40.4 degree Centigrade was diagnosed on 02Dec2004. On 10Dec2004 she developed pruritus and on 12Dec2004 first blisters of the skin, cockades and spots. Diagnosis of toxic epidermal necrolysis with macula was made on 12Dec2004. Nikolski

phenomenon was positive on 15Dec2004. On 16Dec2004 she developed reddening of the eyes and erosions of the lips and oral mucosa. The event persisted until 31Dec2004. The final outcome of the event was reported as "death". Date of death not documented. Information on medication is available from 14Nov2004 until 12Dec2004. In Mar2005, experts assessed the causal relationship as "very possible" with Novalgine (metamizole) (from 26Nov2004 until 10Dec2004), Cefuroxim (from 30Nov2004 until 02Dec2004), Voltaren (diclofenac) (from 02Dec2004 until 03Dec2004), Zyloric (allopurinol) (from 09Dec2004 until 10Dec2004 as "possible" with Neurontin (from 01Dec2004 until 02Dec2004), Clexane (heparin-fraction) (from 25Nov2004 until 02Dec2004), Tramal (tramadol) (on 29Nov2004), Zocor (simvastatin) (from 02Dec2004 until 10Dec2004), Nexium (from 02Dec2004 until 10Dec2004), Paspertin (from 06Dec2004 until 10Dec2004), as "questionable" with Esidrix (hydrochlorothiazide) (from 25Nov2004 until 10Dec2004), Pres plus (enalapril/hydrochlorothiazide) (from 25Nov2004 until 10Dec2004), Kalinor (potassium chloride) (from 26Nov2004 until 27Nov2004 and 30Nov2004 until 04Dec2004), Mutaflor (from 01Dec2004 until 11Dec2004), Kalium-Duriles (from 05Dec2004 until 10Dec2004), Pan-Ophtal (from 08Dec2004 until 11Dec2004), Vincristin (on 10Dec2004), as "not related" with Oxazepam (on 25Nov2004). The event was documented as life-threatening. Per documentation, previous treatment with Esidrix, Novalgine drops, Tramal and Mutaflor was tolerated well by the patient. A possible relationship is suspected by the experts with the high-malignant B-cell lymphoma of the stomach.

Company Clinical Evaluation

Based on the available information, a possible contributory role of the suspect drug gabapentin to the reported event of TOXIC EPIDERMAL NECROLYSIS can not be excluded; however, patient was also taking other drugs that could have contributed to the reported event. A medically meaningful assessment cannot be provided at this time due to the very limited information. This individual report does not modify the benefit/risk profile of the suspect drug. This case will be reassessed once additional information is available

Sender's comment:

Reporter's comment:

GRUNENTHAL, 27.02.2008 DE-GRUNENTHAL-2008 10813

Sender diagnosis: not reported

Narrative

Manufacturer Control Number: 2008-10813

This solicited case from GERMANY was reported by a health professional via regulatory authority, was received at GRT on 25-FEB-2008 and concerns a 71- year- old female patient (height unknown / weight unknown).

The relevant medical history included: B-cell lymphoma , Pain , Sleep disorder , Thrombosis prophylaxis , Hypertension , Electrolyte substitution therapy , Infection prophylaxis , Flatulence , Osteoarthritis , Hypercholesterolaemia , Abdominal pain upper , Nausea , Eye pain , Tumour lysis syndrome.

The relevant concomitant medication included:

OXAZEPAM / OXAZEPAM;
HYDROCHLOROTHIAZIDE / ESIDRIX;
ENALAPRIL MALEATE, HYDROCHLOROTHIAZIDE / PRES PLUS;
POTASSIUM CHLORIDE / KALINOR;
ESCHERICHIA COLI / MUTAFLOR;
POTASSIUM CHLORIDE / KALIUM-DURILES;
METOCLOPRAMIDE HYDROCHLORIDE / PASPERTIN;
DEXPANTHENOL / PAN-OPHTAL;
VINCISTINE SULFATE / VINCISTIN;

This case was reported by the Documentation Centre for Severe Skin Reactions (Freiburg/ Germany) via BfArM and concerns a 71-year-old multimorbid female patient on multimедication (incl. tramadol). The medical history included B-cell lymphoma of the stomach with high malignancy. On 25-Nov-2004, the patient developed fever up to 39.4 degrees Celsius and was hospitalized. On 30-Nov-2004, the patient underwent biopsy of the iliac crest. On 02-Dec-2004, fever up to 40.4 degrees Celsius developed. On 10-Dec-2004, pruritus occurred followed by skin blisters, cockades and

maculae on 12-Dec-2004. Nikolski + on 15-Dec-2004. On 16-Dec-2004, the patient experienced reddening of the eyes and erosions of the lips and the oral mucosa.

The reported symptoms were considered to be life-threatening. The patient died.

The expert committee considered metamizole, cefuroxime, diclofenac and allopurinol to be the most suspect drugs followed by tramadol and further drugs. B-cell-lymphoma of high malignancy was considered to have a possible causal relationship with the skin reactions.

No further information was requested.

Third party case numbers

E2B Authority Number: DE-BFARM-08017001
(GLS)-DE-GRT: DE-08-DS-0124

Sender's comment:

Toxic epidermal necrolysis is assessed as serious due to the criteria fatal, life-threatening and hospitalization.

Toxic epidermal necrolysis is not listed in the reference safety information of tramadol hydrochloride.

Toxic epidermal necrolysis is assessed to be unlikely related to the administration of tramadol hydrochloride as the event is not listed for tramadol, the patient received tramadol seemingly only once about two weeks before the occurrence of the event and was certainly multi-medicated- thus, it is deemed more likely that other medications, such as cefuroxim, which are known to be able to trigger very severe skin reactions, contributed to the patient's event.

The information in this single case does not appear to alter the favourable benefit to risk ratio of tramadol hydrochloride.

Reporter's comment:

AZPROD0000, 28.02.2008 DE-AstraZeneca-2008PK00405

Sender diagnosis: not reported

Narrative

{TOXIC EPIDERMAL NECROLYSIS WITH MACULAE (EVIDENCED CASE)}

A report has been received from the Centre for Documentation of Severe Skin Reactions in Germany via the German Health Authority (BfArM) concerning a 71 year old female patient who experienced an evidenced case of toxic epidermal necrolysis with maculae.

Suspected medication included NEXIUM ORAL (esomeprazole magnesium) QD oral for gastric pain, CLEXANE (enoxaparin sodium), NOVALGIN (metamizole sodium), TRAMAL (tramadol hydrochloride), cefuroxime, gabapentin, VOLTAREN (diclofenac), simvastatin, PASEPTIN (metoclopramide hydrochloride) (metoclopramide hydrochloride) and ZYLORIC (allopurinol).

Concomitant drugs included oxazepam for sleep disorder, hydrochlorothiazide and enalapril+hydrochlorothiazide for hypertension, escherichia coli for meteorismus, potassium and potassium chloride for substitution, dexpanthenol ointment for pain in the eyes and vincristine for lymphoma.

Concomitantly the patient was hospitalised because of malign B-cell gastric lymphoma and fever on 25-Nov-2004. On 30-Nov2004 a punch biopsy of the iliac crest was performed. On 02-Dec-2004 her fever increased up to 40 degrees Celsius and on 10-Dec-2004 she suffered from pruritus and two days later cutaneous blisters, blisters and diffuse redness appeared. On 15-Dec-2004 she had a positive Nikolsky's sign. On the next day she suffered from red eyes, lip erosions and oral mucosa erosions.

The further course of the finally diagnosed of experienced toxic epidermal necrolysis with maculae has not been reported and the patient died from the event on 31-Dec-2004.

The reporting experts from the documentation centre for severe skin reactions suspected metamizole sodium, cefuroxime, diclofenac and allopurinol to have very possibly been causative of the toxic epidermal necrolysis with maculae and esomeprazole magnesium, enoxaparin sodium, tramadol hydrochloride, gabapentin, simvastatin and clopramide hydrochloride were considered to have possibly been related to the event.

The event was considered serious due to life threatening and death.

Sender's comment:

Toxic epidermal necrolysis is listed as a possible adverse event for esomeprazole. However, all events with fatal outcome are considered unlisted by AstraZeneca. The case is poorly documented and the patient was treated with several suspect drugs. Thus it is not possible to establish an unequivocal causal relationship to esomeprazole. Report according to article 63b and 42 AMG

Toxic epidermal necrolysis is listed as a possible adverse event for esomeprazole. However, all events with fatal outcome are considered unlisted by AstraZeneca. The case is poorly documented and the patient was treated with several suspect drugs. Thus it is not possible to establish an unequivocal causal relationship to esomeprazole.

Causality assessment: unassessableNO

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 03.03.2008 DE-AVENTIS-200810548DE

Sender diagnosis: not reported

Narrative

Initial report

This case of toxic epidermal necrolysis (TEN) with maculae was received from the German health authority (DE-BFARM-08017001). It was reported by the German Documentation Centre for Severe Skin Reactions in Freiburg / Breisgau, Germany (code-no. 9160224). The centre is conducting a prospective study on the incidence and drug aetiology of severe skin reactions. Diagnosis of TEN was made by an expert panel who assessed it as a "definite case" of TEN. Causal relationship was assessed by the experts in Mar. 2005 using an algorithm.

As for metamizole (Novalgin) the causality was assessed as "well possible", for enoxaparin (Clexane) it was assessed as "possible". For the other suspected drugs causality was assessed as:

"well possible": cefuroxime, diclofenac, allopurinol

"possible": tramadol, gabapentin, simvastatin, esomeprazole, metoclopramide

Not suspected were: oxazepam, hydrochlorothiazide, enalapril + hydrochlorothiazide, potassium, Escherichia coli Nissl, dexpanthenol, vincristin.

Highly malignant B-cell lymphoma of the stomach was considered possible cause of TEN.

Epicrisis:

Patient: female, 71 years

Medical history: Highly malignant B-cell lymphoma of the stomach, hypertension, hip arthrosis, hypercholesterolemia, tumorlysis syndrome.

On 25-Nov-2004 the patient was hospitalised with fever of 39.4 degrees Celsius. On 30-Nov-2004 a biopsy of the iliac crest was performed. On 02-Dec-2004 the fever increased to 40.4 degrees Celsius.

The patient was treated with metamizole 20 drops on 26-Nov-2004 and from 06-Dec-2004 to 10-Dec-2004 for pain. She received enoxaparin 1x0.4 mg/day from 25-Nov-2004 to 02-Dec-2004 as prophylaxis of embolism. Before and during hospitalisation the patient was also treated with a lot of other drugs.

On 10-Nov-2004 the patient complained about pruritus (not considered onset of TEN). All drugs with the exception of Escherichia coli and dexpanthenol eye ointment were discontinued.

On 12-Nov-2004 skin blisters, target lesions and maculae were detected.

On 15-Nov-2004 Nikolsky's sign was positive.

On 16-Dec-2004 the patient developed red eyes and erosions of lips and oral mucosa.

The outcome was reported as "death", date of end of event was reported as 31-Dec-2004.

Laboratory: no values provided.

Sender's comment:

27-FEB-2008 Sanofi-Aventis Company Comment: Due to the compatible temporal relationship the causal role of metamizole sodium and/or enoxaparin cannot be excluded. However, the numerous concomitant medications as well as the underlying lymphoma could be discussed as possible alternative causes of reaction.

27-FEB-2008 Sanofi Aventis Germany: TOXIC EPIDERMAL NECROLYSIS - FATAL was considered locally unlabelled for enoxaparin (Clexane) and metamizole sodium (due to life-threatening character of the reaction). To date, this report does not imply a change in the state of the scientific knowledge for the products. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08027908

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthema	20070317			747 Day	fatal
2 Blisters	20050331			31 Day	fatal
3 Pain of skin	20050329			29 Day	fatal
4 Pruritus	20050328			28 Day	fatal
5 Skin blotches	20050323			23 Day	fatal
6 Nikolsky's sign	20050404			35 Day	fatal
7 Yersinia infection NOS	20050331			31 Day	fatal
8 Toxic epidermal necrolysis	20050329			29 Day	fatal
9 Stevens Johnson syndrome	20050329			29 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-08027908

1	ACTRAPID HUMAN insulin human	Subcutaneous	6 G gram(s)/	20050324-20050329 Diabetes mellitus 6 Day
2	AVALOX moxifloxacin hydrochloride	Intravenous (not otherwis	400 Mg milligram(s)/	20050331-20050331 1440 Minute
3	BENURON paracetamol	Oral	500 Mg milligram(s)/	20050315-20050315 Fever 1440 Minute
4	BENURON paracetamol	Oral	500 Mg milligram(s)/	20050307-20050307 Fever 1440 Minute
5	BENURON paracetamol	Oral	500 Mg milligram(s)/	20050322-20050323 Fever 2 Day
6	BENURON paracetamol	Rectal	1 Df dosage form/	20050305-20050305 Fever 1440 Minute
7	BENURON paracetamol	Rectal	1 Df dosage form/	20050319-20050319 Fever 1440 Minute
8	CALCIUM calcium	Oral	2 Df dosage form/Day	20050324- -
9	CALCIUM calcium		2 Df dosage form/Day	20050308-20050309 2 Day
10	CALCIUM GLUCONATE calcium gluconate	Intravenous (not otherwis	1 Df dosage form/	20050323-20050323 1440 Minute
11	CANDIO-HERMAL nystatin	Topical	1 Df dosage form/	20050326-20050326 Candidiasis 1440 Minute
12	CANDIO-HERMAL nystatin	Topical	/	20050317-20050317 Candidiasis 1440 Minute
13	CORANGIN isosorbide mononitrate		1 Df dosage form/	20050327-20050702 3 Month
14	DECORTIN prednisone	Intravenous (not otherwis	/	20050330- -
15	EUGLUCON glibenclamide	Oral	1.75 Mg milligram(s)/	- -
16	EUSAPRIM sulfamethoxazole; trimethoprim	Oral	2 Df dosage form/Day	20050307-20050314 Infection 8 Day
17	FENISTIL (NCH) dimetindene maleate	Intravenous (not otherwis	/	20050328-20050328 Skin disorder 1440 Minute
18	HAES-STERIL hetastarch; sodium chloride	Intravenous (not otherwis	/	20050304-20050305 2 Day
19	HEPARIN heparin	Subcutaneous	150305 Df dosage form/	- -
20	HEPARIN heparin	Subcutaneous	1 Df dosage form/	20050222-20050307 Thrombosis prophylaxis 14 Day
21	IMODIUM loperamide hydrochloride	Oral	2 Df dosage form/	20050325-20050326 Diarrhea 2 Day
22	IMODIUM loperamide hydrochloride	Oral	2 Df dosage form/	20050315-20050315 Diarrhea 1440 Minute
23	IMODIUM loperamide hydrochloride	Oral	/	20050308-20050308 Diarrhea 1440 Minute
24	INTRAMIN G acetylcysteine; amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; glucose monohydrate; potassium chloride; sodium glycerophosphate	Intravenous (not otherwis	500 ml millilitre(s)/	20050331-20050331 1440 Minute
25	INTRAMIN G acetylcysteine; amino acids nos; calcium chloride dihydrate; electrolytes nos; glucose; glucose monohydrate; potassium chloride; sodium glycerophosphate	Intravenous (not otherwis	500 ml millilitre(s)/	20050323-20050324 2 Day
26	KONAKION phytomenadione	Oral	5 Gtt drop(s)/	20050309-20050310

27	KONAKION phytomenadione	Oral	10 Gtt drop(s)/	2 Day 20050314-20050316
28	MANINIL glibenclamide	Oral	/	3 Day 20050211-20050303 Diabetes mellitus
29	MARCUMAR phenprocoumon	Oral	/	21 Day 20050305-20050308
30	MULTIBIONTA ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; thiamine hydrochloride; tocopheryl acetate	Intravenous (not otherwis 1 Df dosage form/		4 Day 20000322-20050322 5 Year
31	MYOCHOLINE bethanechol	Oral	75 Mg milligram(s)/Day	20050210- -
32	NACL sodium chloride	Intravenous (not otherwis /		20050329- -
33	NACL sodium chloride	Intravenous (not otherwis /		20050318-20050324 7 Day
34	NACL sodium chloride	Intravenous (not otherwis 500 ml millilitre(s)/		20050318- -
35	NOVALGIN metamizole	Intravenous (not otherwis 1 Df dosage form/		20050131-20050531 Pain 4 Month
36	NOVALGIN metamizole	Oral	80 Gtt drop(s)/Day	20050309-20050314 Pain 6 Day
37	PANTOPRAZOLE SODIUM pantoprazole sodium	Intravenous (not otherwis 80 Mg milligram(s)/Day		20050318- -
38	PANTOZOL pantoprazole	Oral	/	20050301- -
39	PASPERTIN metoclopramide hydrochloride	Oral	60 Gtt drop(s)/Day	20050310-20050318 Nausea 9 Day
40	PASPERTIN metoclopramide hydrochloride	Intravenous (not otherwis 1 Df dosage form/		20050322-20050322 Nausea 1440 Minute
41	RIOPAN magaldrate	Oral	/	20070309- -
42	RIOPAN magaldrate		/	20050307-20050307 Abdominal pain 1440 Minute
43	ROCEPHIN ceftriaxone sodium	Intravenous (not otherwis 2 G gram(s)/		20050322-20050325 Infection 4 Day
44	SAROTEN amitriptyline hydrochloride	Oral	/	20050301-20050318 Depression 18 Day
45	sortis atorvastatin calcium	Oral	1 Df dosage form/	20040930-20050303 Hypercholesterolaemia 5 Month
46	THIOCTACID thioctic acid	Intravenous (not otherwis 600 Mg milligram(s)/		20050325-20050327 Polyneuropathy 3 Day
47	Urbason methylprednisolone	Intravenous (not otherwis /		20050331-20050331 1440 Minute
48	Urbason methylprednisolone	Intravenous (not otherwis /		20050322-20050329 8 Day
49	VOLTARENE RESINATE diclofenac resinate	Oral	2 Df dosage form/	20050214-20050309 Pain 24 Day
50	WATER FOR INJECTION water for injection	Intravenous (not otherwis 100 ml millilitre(s)/		20050323-20050323 1440 Minute
51	XUSAL levocetirizine dihydrochloride	Oral	/	20050329- -
52	XYLIT xylitol	Intravenous (not otherwis /		20050323-20050323 1440 Minute

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Collagenosis			Unknown	
Yersinia infection			Unknown	
Pruritus			Unknown	
Dyspnoea			Unknown	
Polyneuropathy			Unknown	
Emesis			Unknown	
Gastric pain			Unknown	
Hypercholesterolaemia			Unknown	
Pain			Unknown	
Depression			Unknown	
Atonic bladder (neurogenic)			Unknown	
Thrombosis			Unknown	
Fever			Unknown	
Diabetes mellitus			Unknown	
Diarrhea			Unknown	
Hypocalcemia			Unknown	
Nausea			Unknown	

Primary source

Qualification: Other Health Professional observestudytype:

Literature:

Qualification: observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 27.02.2008 DE-002147023-PHBS2008DE02561

Sender diagnosis: not reported

Narrative

The patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol therapy was initiated. On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Sender's comment:

For Pantozol and Voltaren Resinate:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information. It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

For Corangin:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

For Fenistil:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 28.02.2008 DE-NOVNCHHQ-PHBS2008DE02561

Sender diagnosis: not reported

Narrative

The patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol therapy was initiated. On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Sender's comment:

For Pantozol and Voltaren Resinate:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

For Corangin:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

For Fenistil:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 03.03.2008 DE-002147023-PHBS2008DE02561

Sender diagnosis: not reported

Narrative

The patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol therapy was initiated. On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with

sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Significant correction on narrative and suspected drugs was done after internal revue on 22 Feb 2008. Initial report received from Health Authority (08014064) via Nycomed (8331) on 22 Feb 2008. This elderly patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol (pantoprazole) therapy was initiated and on 14 Mar 2007 Voltaren Resinate (diclofenac resinate) (Other suspected drugs are: pantoprazole sodium, Maninil (glibenclamide), Sortis (atrovastatin), Saroten (amitriptyline), Clexane (heparin) Myocholine (bethanechol), Haes (hyetellose), Marcumar (phenprocoumon), Benuron (paracetamol), Euglucon (glibenclamide), Imodium (loperamide), calcium, Novalgin (metamizole sodium monohydrate), Konakion (phytomenadione), Paspertin (metoclopramide), Candio-Hermal (nystatin), NaCl (sodium chloride), Multibionta (ascorbic acid, calcium pantothenate, cyanocobalamin, nicotinamide, ...), Rocephin (ceftriaxone), Urbason (methylprednisolone), Xylit (xylitol), Ca-Gluconat (calcium gluconate), Aqua ad (water for injection), Intramin (cetylcysteine, amino acids, electrolytes, glucose, sodium ...), Actrapid (insulin), Thioctacid (thioctic acid), Corangin (isosorbide mononitrate), Fenistil (dimetindene), Xusal (levocetirizine dihydrochloride), Decortin (prednisone), Avalox (moxifloxacin). On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Sender's comment:

For Pantozol and Voltaren Resinate:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

For Corangin:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

For Fenistil:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 04.03.2008 DE-SZ09-PHBS2008DE02561

Sender diagnosis: not reported

Narrative

The patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol therapy was initiated. On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of

unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Significant correction on narrative and suspected drugs was done after internal revue on 22 Feb 2008. Initial report received from Health Authority (08014064) via Nycomed (8331) on 22 Feb 2008. This elderly patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol (pantoprazole) therapy was initiated and on 14 Mar 2007 Voltaren Resinate (diclofenac resinate) (Other suspected drugs are: pantoprazole sodium, Maninil (glibenclamide), Sortis (atrovastatin), Saroten (amitriptyline), Clexane (heparin) Myocholine (bethanechol), Haes (hyetellose), Marcumar (phenprocoumon), Benuron (paracetamol), Euglucon (glibenclamide), Imodium (loperamide), calcium, Novalgin (metamizole sodium monohydrate), Konakion (phytomenadione), Paspertin (metoclopramide), Candio-Hermal (nystatin), NaCl (sodium chloride), Multibionta (ascorbic acid, calcium pantothenate, cyanocobalamin, nicotinamide, ...), Rocephin (ceftriaxone), Urbason (methylprednisolone), Xylit (xylitol), Ca-Gluconat (calcium gluconate), Aqua ad (water for injection), Intramin (cetylcysteine, amino acids, electrolytes, glucose, sodium ...), Actrapid (insulin), Thioctacid (thioctic acid), Corangin (isosorbide mononitrate), Fenistil (dimetindene), Xusal (levocetirizine dihydrochloride), Decortin (prednisone), Avalox (moxifloxacin). On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Sender's comment:

For Pantozol and Voltaren Resinate:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

For Corangin:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

For Fenistil:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

NOVARTIS, 04.03.2008 DE-NOVNCHHQ-PHBS2008DE02561

Sender diagnosis: not reported

Narrative

The patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol therapy was initiated. On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was

discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Significant correction on narrative and suspected drugs was done after internal revue on 22 Feb 2008. Initial report received from Health Authority (08014064) via Nycomed (8331) on 22 Feb 2008. This elderly patient was hospitalized from 10 Feb 2005 until 23 Feb 2005 and from 26 Feb 2005 to 01 Mar 2005 in a department for internal medicine (reason for admission unknown). On 01 Mar 2005 Pantozol (pantoprazole) therapy was initiated and on 14 Mar 2007 Voltaren Resinate (diclofenac resinate) (Other suspected drugs are: pantoprazole sodium, Maninil (glibenclamide), Sortis (atrovastatin), Saroten (amitriptyline), Clexane (heparin) Myocholine (bethanechol), Haes (hyetellose), Marcumar (phenprocoumon), Benuron (paracetamol), Euglucon (glibenclamide), Imodium (loperamide), calcium, Novalgin (metamizole sodium monohydrate), Konakion (phytomenadione), Paspertin (metoclopramide), Candio-Hermal (nystatin), NaCl (sodium chloride), Multibionta (ascorbic acid, calcium pantothenate, cyanocobalamin, nicotinamide, ...), Rocephin (ceftriaxone), Urbason (methylprednisolone), Xylit (xylitol), Ca-Gluconat (calcium gluconate), Aqua ad (water for injection), Intramin (cetylcysteine, amino acids, electrolytes, glucose, sodium ...), Actrapid (insulin), Thioctacid (thioctic acid), Corangin (isosorbide mononitrate), Fenistil (dimetindene), Xusal (levocetirizine dihydrochloride), Decortin (prednisone), Avalox (moxifloxacin). On 09 Mar 2005 Riopan therapy was started. On 04 Mar 2005 she was transferred into neurological department due to not reported reason. From 07 Mar 2007 to 14 Mar 2007 she received antibiotic treatment with sulfamethoxazol/trimethoprim due to suspected infection and on 22 Mar 2005, antibiotic treatment with ceftriaxone was initiated due to infection of unknown source. The patient repeatedly received paracetamol due to fever. On 17 Mar 2007 she experienced exanthema. Pantozol treatment was discontinued and pantoprazole was started. On 23 Mar 2005 she experienced cockades and skin spots. On 28 Mar 2005 she developed pruritus on 29 Mar 2005 she experienced skin pain. On 31 Mar 2005 she experienced skin blisters. The patient was transferred to an intensive care unit. A Yesinia infection was suspected. On 04 Apr 2005 Nikolsky's sign was positive and she was transferred to a center of burn injury. The patient died on an unknown date. No autopsy reported was provided. The reporter suspected a causal relationship between the event and Pantozol. Diagnosis of overlapping Stevens-Johnson / Toxic Epidermal Necrolysis was made.

Sender's comment:

For Pantozol and Voltaren Resinate:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

It is Novartis' policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

For Corangin:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

For Fenistil:

Serious spontaneous report, T.E.N. [death] assessed as unlisted according to the Basic Prescribing Information.

However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08030339

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Macule	20050118			10 Day	fatal
2 Lip erosion	20050124			16 Day	fatal
3 Fever	20050120			12 Day	fatal
4 Toxic epidermal necrolysis	20050118			10 Day	fatal
5 Oral mucosa erosion	20050125			17 Day	fatal
6 Pain of skin	20050118			10 Day	fatal
7 Blisters	20050119			11 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aminosteril amino acids nos; vitamins nos	Intravenous (not otherwis /		20050111-20050113	Liver cirrhosis
2 aquaphor xipamide	Oral	5 Df dosage form/Day	20041221-20050109	
3 CEFTRIAXONE ceftriaxone	Intravenous (not otherwis /	Injection	20050111-20050118	Peritonitis
4 diclofenac diclofenac	Oral	2 Df dosage form/Day	20040105-20050109	
5 fenistil dimetindene maleate	Topical	/	20050118-	Erythema
6 fenistil dimetindene maleate	Oral	/	20050118-	Erythema
7 FUROSEMIDE furosemide	Oral	80 Mg milligram(s)/Day	20050109-	
8 LACTULOSE lactulose	Oral	30 Mg milligram(s)/Day	20041221-	Hepatic encephalopathy prophylaxis
9 PANTOZOL pantoprazole	Oral	80 Mg milligram(s)/Day	20050109-	Gastric ulcer
10 spironolacton spironolactone	Oral	50 Mg milligram(s)/	20041221-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Liver cirrhosis			Unknown	
Oedema			Unknown	
Erythema			Unknown	
Peritonitis			Unknown	
Gastric ulcer			Unknown	the patient also had abdominal exudate
Pain			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 07.03.2008 DE-002147023-PHBS2008DE02868

Sender diagnosis: not reported

Narrative

Initial report received from the German documentation centre of severe skin reactions (9160238) via Health Authority (DE-BFARM-08016966) and Nycomed (80483). This patient had a relevant history of pain, oedema, abdominal exudate, stomach ulcer, liver cirrhosis, peritonitis and erythema. The patient was treated with Pantozol 40 mg (pantoprazole sodium sesquihydrate) for gastric ulcer since 09 Jan 2005. Further suspected drugs were diclofenac (manufacturer unknown), xipamide, spironolactone, lactulose, furosemide, aminosteril, and ceftriaxone and dimetindene maleate. The patient experienced toxic epidermal necrolysis with maculae while receiving Pantozol. From 09 Jan 2005 to 20 Jan 2005, the patient was hospitalized (internal medicine). From 11 Jan 2005 to 18 Jan 2005, the patient received antibiotic treatment with ceftriaxone for peritonitis. The patient developed pain of the skin, cockades and maculae on 18 Jan 2005 and skin blisters on 19 Jan 2005. From 20 Jan 2005 to 21 Jan 2005, the patient was hospitalized in a dermatology department. He developed fever up to 39.8°C. From 21 Jan 2005 to 24 Jan 2005, he was in an intensive care unit. On 24 Jan 2005, the patient experienced erosions of lips and on 25 Jan 2005 erosions of the oral mucosa. The reaction was rated as life-threatening and the patient died. No information regarding autopsy report was provided. The causality was assessed as possible for Pantozol by the reporter.

Sender's comment:

Comment for Pantozol and diclofenac:

Serious spontaneous report Toxic epidermal necrolysis [death], assessed as unlisted according to the Core Data Sheet.

It is Novartis policy to assess a case with a fatal outcome as unlisted unless the label specifically mentions death as a possible outcome of the listed risks. All spontaneous reports are considered suspected for reporting purposes.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08029718

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 17 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Application site pruritus					not recovered/not resolved
2 Blistering					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel	Topical	/	-	
diclofenac diethylamine			-	

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08027236

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes		No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 89 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20060509	deathcause: Toxic epidermal necrolysis
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20060430	20060509	10 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ARCOXIA etoricoxib	Oral	90 Mg milligram(s)/Day 245	20060429- -	Pain in hip
2 dexamethasone dexamethasone	Subcutaneous	/Day 120	20060421-20060421 1 Day	Pain
3 diclofenac Inj.Lsg. diclofenac	Subcutaneous	/Day 120	20060421-20060421 1 Day	Pain
4 meloxicam 7,5 mg meloxicam	Oral	/	20060421-20060428 8 Day	Back pain
5 novalgin Inj.Lsg. 1 g metamizole	Intravenous (not otherwis	1 G gram(s)/Day 120	20060428- -	Pain
6 sympal Injektionsloesung dexketoprofen trometamol	Subcutaneous	/Day 120	20060421-20060421 1 Day	Pain

Drugs (concomitant)

7 calcium carbonate / cholecalciferol calcium carbonate; colecalciferol	Oral	/	20060429- -	Osteoporosis
8 calcium chloride / dextrose / magnesium chlorid	Intravenous (not otherwis	/	20060428- -	
9 DUROGESIC fentanyl	Topical	/3 Day	20060429- -	Pain
10 Elektrolyte nnb calcium gluconate; magnesium carbonate; potassium chloride; sodium chloride	Intravenous (not otherwis	/	20060428- -	
11 FOSAMAX alendronate sodium	Oral	70 Mg milligram(s)/7 Day 245	20060429- -	Osteoporosis
12 Kaliumchlorid potassium chloride	Intravenous (not otherwis	/	20060428- -	
13 MONO EMBOLEX certoparin sodium	Subcutaneous	0.5 ml millilitre(s)/Day 120	20060428- -	
14 PASPERTIN metoclopramide hydrochloride	Intravenous (not otherwis	1 /Day 120	20060428- -	Nausea
15 POLYBION INJECTION vitamins nos	Intravenous (not otherwis	1 /Day 120	20060428- -	
16 vomex a dimenhydrinate	Intravenous (not otherwis	1 /Day 120	20060429-20060429 1 Day	Nausea

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain in hip			Yes	
Osteoporosis			Yes	
Nausea			Yes	
Hospitalisation			Yes	
Back pain			Yes	
Total hip replacement			No	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

009507513, 12.03.2008 DE-MERCK-0803DEU00031

Sender diagnosis: not reported

Narrative

Information has been received from a registry, documenting at least 95% of all serious skin reactions that occur in Germany, concerning an 89-year-old, hospitalized (on 28-APR-2006) female patient with pain, back pain, nausea and osteoporosis who, on 29-APR-2006, was placed on therapy with etoricoxib, 90 mg tablet, 90 mg, once a day for the treatment of pain (duration not reported).

Suspect secondary therapy included: dexketoprofen tromethamine (SYMPAL), subcutaneous injection, 1 dose unit once on 21-APR-2006 for the treatment of pain, dexamethasone (+) diclofenac, subcutaneous injection, 1 dose unit once on 21-APR-2006 for the treatment of pain, meloxicam, 7.5 mg, po for the treatment of back pain (taken from 21-APR-2006 until 28-APR-2006)(total daily dose unknown) and dipyrone (NOVALGIN), injection, 1 gram daily iv for the treatment of pain (start date 28-APR-2006).

Other concomitant therapy included certoparin sodium (MONO EMBOLEX), metoclopramide hydrochloride (PASPERTIN), calcium chloride (+) dextrose (+) magnesium chloride (+) sodium chloride (+) sodium lactate (+) potassium chloride (STEROFUNDIN), electrolytes (unspecified), dexpanthenol (+) vitamin B complex (POLYBION INJECTION), dimenhydrinate (VOMEX A), alendronate sodium (MSD), fentanyl (DUROGESIC) and calcium carbonate (+) cholecalciferol.

On 30-APR-2006, the patient developed extensive skin reddening and blisters. Action taken regarding therapy with etoricoxib and dipyrone was not provided. On 01-MAY-2006, the patient developed difficulty in swallowing which got worse on 02-MAY-2006. On 03-MAR-2006, Nikolsky's sign was positive and the patient developed mucosal erosions of mouth. That same day, the patient was transferred to the dermatological ward. She was diagnosed with toxic epidermal necrolysis (TEN). The further course of events and the outcome were not provided.

The reporting physician felt that TEN prolonged hospitalization and was probably not related to therapy with etoricoxib but possibly to dexketoprofen tromethamine and/or dexamethasone (+) diclofenac and/or dipyrone and probably to meloxicam.

Additional information has been requested.

Sender's comment:

Toxic epidermal necrolysis (TEN) is a serious labeled event for therapy with ARCOXIA°. This 89-year-old hospitalized female developed TEN one day after she was placed on therapy with ARCOXIA° for back pain. She received numerous concomitant medications. In view of the temporal correlation between the concomitant drugs and the TEN and the experience with these drugs regarding severe skin reactions, they are primary suspects for the TEN and a causal link with ARCOXIA° is doubtful. The safety profile of ARCOXIA° is unchanged. Self initiated measures are not indicated.

Reporter's comment:

009507513, 30.04.2008 DE-MERCK-0803DEU00031

Sender diagnosis: not reported

Narrative

07.03.2016

BfArM - Pharmacovigilance

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Information has been received from a registry, documenting at least 95% of all serious skin reactions that occur in Germany, concerning an 89-year-old female patient hospitalized for hip-TEP surgery (on 28-APR-2006) with back pain, nausea and osteoporosis who, on 29-APR-2006, was placed on therapy with etoricoxib, 90 mg tablet, 90 mg, once a day for the treatment of pain in hip (duration not reported).

Suspect secondary therapy included: dexketoprofen tromethamine (SYMPAL), subcutaneous injection, 1 dose unit once on 21-APR-2006 for the treatment of pain, dexamethasone (+) diclofenac, subcutaneous injection, 1 dose unit once on 21-APR-2006 for the treatment of pain, meloxicam, 7.5 mg, po for the treatment of back pain (taken from 21-APR-2006 until 28-APR-2006)(total daily dose unknown) and dipyrone (NOVALGIN), injection, 1 gram daily iv for the treatment of pain (start date 28-APR-2006).

Other concomitant therapy included certoparin sodium (MONO EMBOLEX), metoclopramide hydrochloride (PASPERTIN), calcium chloride (+) dextrose (+) magnesium chloride (+) sodium chloride (+) sodium lactate (+) potassium chloride (STEROFUNDIN), electrolytes (unspecified), dexpanthenol (+) vitamin B complex (POLYBION INJECTION), dimenhydrinate (VOMEX A), alendronate sodium (MSD), fentanyl (DUROGESIC) and calcium carbonate (+) cholecalciferol.

On 30-APR-2006, the patient developed extensive skin reddening and blisters. Action taken regarding therapy with etoricoxib and dipyrone was not reported. On 01-MAY-2006, the patient developed difficulty in swallowing which got worse on 02-MAY-2006. On 03-MAY-2006, Nikolsky's sign was positive and the patient developed erosions of oral mucosa. That same day, the patient was transferred to the dermatological ward. She was diagnosed with toxic epidermal necrolysis (TEN). On 09-MAY-2006, the patient died. The cause of death was toxic epidermal necrolysis (TEN).

The reporting physician felt that TEN prolonged hospitalization and was probably not related to therapy with etoricoxib but possibly to dexketoprofen tromethamine and/or dexamethasone (+) diclofenac and/or dipyrone and probably to meloxicam.

Additional information is not expected.

Sender's comment:

Toxic epidermal necrolysis (TEN) is a serious labeled event for therapy with ARCOXIA°. This 89-year-old hospitalized female developed TEN one day after she was placed on therapy with ARCOXIA° for back pain. She received numerous concomitant medications. In view of the temporal correlation between the concomitant drugs and the TEN and the experience with these drugs regarding severe skin reactions, they are primary suspects for the TEN and a causal link with ARCOXIA° is doubtful. The safety profile of ARCOXIA° is unchanged. Self-initiated measures are not indicated.

Reporter's comment:

FIRST LOOK

Country: Deutschland Type: Spontaneous report case: DE-BFARM-08040340

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes Yes

Patient

Initials: Sex: Female Birth: privacy Age: 20 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pruritus					not recovered/not resolved
2 Exanthema					not recovered/not resolved
3 Skin peeling					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm gel	Topical	/	20080319-20080320	
diclofenac sodium		Gel	2 Day	

Primary source

Qualification: Pharmacist observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

ratiopharm GmbH, 18.04.2008 DE-RATIOPH-2008R042898

Sender diagnosis: not reported

Narrative

Reported terms: Exanthema(Rash) Pruritus(Pruritus) Skin peeling(Skin exfoliation)

Case reference number 2008R042898 is a spontaneous case report sent by a pharmacist to the AMK der Apotheker. It refers to a female aged 20 years.

The patient's past medical history included no medical records. There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started using Diclofenac-ratiopharm Gel (DICLOFENAC), indication unknown on 19-Mar-2008. The patient developed Exanthema (Rash), Pruritus (Pruritus) and Skin peeling (Skin exfoliation) after two day use.

The following action was taken: for Diclofenac-ratiopharm Gel (DICLOFENAC) - Drug withdrawn

The outcome for Exanthema (Rash), for Pruritus (Pruritus) and for Skin peeling (Skin exfoliation) is Not Recovered / Not Resolved.

Rechallenge / Dechallenge information for Diclofenac-ratiopharm Gel is not applicable.

The reporter's assessment for drug Diclofenac-ratiopharm Gel related with Skin peeling, with Pruritus and with Exanthema is as follows: Unknown. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

Skin peeling is not exactly worded in our RSI, however it is covered by the chapter side effects: "local skin reactions, such as skin redness, pruritus, a burning sensation, skin rash, sometimes with pustules or wheals." Causal relationship between suspected drug and events is rated as "æœpossibleâ€ .

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08045149

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 69 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 ESR increased				2 Day	recovered/resolved
2 Fever				2 Day	recovered/resolved
3 Hypertension					Unknown
4 Leukocytosis				2 Day	recovered/resolved
5 Lipoma					Unknown
6 Rash pustular				2 Day	recovered/resolved
7 Skin biopsy abnormal				2 Day	recovered/resolved
8 Vertigo					Unknown
9 Nodule				2 Day	recovered/resolved
10 Skin inflammation					Unknown
11 Metaplasia					not recovered/not resolved
12 Lipomatosis					not recovered/not resolved
13 Acute generalized exanthematous pustulosis					recovered/resolved
14 Erythroderma					Unknown
15 Cyst of kidney					Unknown
16 Metaplasia				2 Day	recovered/resolved
17 AGEF					recovered/resolved
18 Patch test positive				2 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclofenac sodium	Intramuscular	/	-	Analgesia
diclofenac sodium		Solution for injection	-	
2 Metamizol	Unknown	/	-	Analgesia
metamizole sodium			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension			Unknown	
Vertigo			Unknown	
Cyst of kidney			Unknown	
Psoriasis			Yes	
Adipositas			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Eyerich K, Traidl-Hoffmann C, Albert A, Kerzl R, Rombold S, Darsow U, et al. Lipomatous Metaplasia after Severe and Chronic Cutaneous Inflammation. Dermatology 2008; 217: 52-55	

Narratives (nur bei elektronischem Eingang)

BETA, 21.04.2008 DE-BETA-7829/08L

Sender diagnosis: not reported

Narrative

This is a report from literature. A 69-year-old female patient with history of psoriasis developed acute generalised exanthematic pustulosis (AGEP) after treatment with metamizole and diclofenac. The patient was admitted to hospital with a suberythrodermia. A generalised pustular exanthema,

accompanied by fever $>39^{\circ}\text{C}$, leukocytosis and increased erythrocyte sedimentation rate was noted. Histological examination of skin biopsies revealed subcorneal and spongiform pustules and a diagnosis of AGEP was made. It was discovered that she had received metamizole the day before occurrence of the skin rash and additional diclofenac was applied intramuscularly 2 days before pustules became apparent. Corrective treatment was given and the cutaneous manifestations declined. The authors state "metamizole seems to be the most likely drug causative for this severe immune reaction. And after avoidance of the suspected analgesics metamizole and diclofenac the clinical condition is stable."

Sender's comment:

Assessment for Diclofenac: serious / AGEP: unexpected / MAH measures to reduce drug risk: increased monitoring

Reporter's comment:

ratiopharm GmbH, 24.04.2008 DE-RATIOPH-2008R043316

Sender diagnosis: not reported

Narrative

A 69-year-old female patient with history of psoriasis developed acute generalised exanthematic pustulosis (AGEP) after treatment with metamizole and diclofenac. The patient was admitted to hospital with a suberythrodermia. A generalised pustular exanthema, accompanied by fever $>39^{\circ}\text{C}$, leukocytosis and increased erythrocyte sedimentation rate was noted. Histological examination of skin biopsies revealed subcorneal and spongiform pustules and a diagnosis of AGEP was made. It was discovered that she had received metamizole the day before occurrence of the skin rash and additional diclofenac was applied intramuscularly 2 days before pustules became apparent. Corrective treatment was given and the cutaneous manifestations declined. Within 2 months, she developed a second course of AGEP after repeated treatment with metamizole. Histology of skin site biopsies showed an atrophic epidermis and mature adipocytes were detected. A diagnosis of lipomatous metaplasia, possibly as a consequence of the severe and chronic inflammation of the skin due to psoriasis and AGEP was made. Patch testing of analgesics revealed positive reactions to metamizole and indomethacin. The authors state "metamizole seems to be the most likely drug causative for this severe immune reaction. And after avoidance of the suspected analgesics metamizole and diclofenac the clinical condition is stable." CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

AGEP is not literally listed in the RSIs of Metamizole or Diclofenac, but is regarded to be covered by the listed severe skin reactions including SJS and TEN.

Lipomatous metaplasia is unlisted. Metaplasia usually occurs during regeneration after chronic inflammation. According to the authors, lipomatous metaplasia was possibly a consequence of the severe and chronic inflammation of the skin due to psoriasis and AGEP. As lipomatous metaplasia was no direct adverse effect of the suspected drugs, but either the consequence of a drug-induced skin reaction or the chronic inflammation of the skin caused by the underlying disease of the patient, psoriasis, no actions with regard to the labelling are considered necessary.

Causal relationship between AGEP and Metamizole is assessed as probable by the MAH due to positive rechallenge and positive patch testing.

Alternative explanation: psoriasis was present as risk factor.

Causal relationship between AGEP and Diclofenac is assessed as possible by the MAH with plausible alternative explanations: metamizole and psoriasis.

Causal relationship between lipomatous metaplasia and the analgesics is assessed as possible by the MAH with a plausible alternative explanation: chronic skin inflammation due to the underlying psoriasis.

Reporter's comment:

AbZ Pharma GmbH, 24.04.2008 DE-ABZ-2008A043329

Sender diagnosis: not reported

Narrative

A 69-year-old female patient with history of psoriasis developed acute generalised exanthematic pustulosis (AGEP) after treatment with metamizole

and diclofenac. The patient was admitted to hospital with a suberythrodermia. A generalised pustular exanthema, accompanied by fever $>39^{\circ}\text{C}$, leukocytosis and increased erythrocyte sedimentation rate was noted. Histological examination of skin biopsies revealed subcorneal and spongiform pustules and a diagnosis of AGEP was made. It was discovered that she had received metamizole the day before occurrence of the skin rash and additional diclofenac was applied intramuscularly 2 days before pustules became apparent. Corrective treatment was given and the cutaneous manifestations declined. Within 2 months, she developed a second course of AGEP after repeated treatment with metamizole. Histology of skin site biopsies showed an atrophic epidermis and mature adipocytes were detected. A diagnosis of lipomatous metaplasia, possibly as a consequence of the severe and chronic inflammation of the skin due to psoriasis and AGEP was made. Patch testing of analgesics revealed positive reactions to metamizole and indomethacin. The authors state "metamizole seems to be the most likely drug causative for this severe immune reaction. And after avoidance of the suspected analgesics metamizole and diclofenac the clinical condition is stable." CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

AGEP is not literally listed in the RSI of Diclofenac, but is regarded to be covered by the listed severe skin reactions including SJS and TEN. Lipomatous metaplasia is unlisted. Metaplasia usually occurs during regeneration after chronic inflammation. According to the authors, lipomatous metaplasia was possibly a consequence of the severe and chronic inflammation of the skin due to psoriasis and AGEP. As lipomatous metaplasia was no direct adverse effect of the suspected drugs, but either the consequence of a drug-induced skin reaction or the chronic inflammation of the skin caused by the underlying disease of the patient, psoriasis, no actions with regard to the labelling are considered necessary. According to the authors, Metamizole was the most likely cause for AGEP in this patient. Causal relationship between AGEP and Diclofenac is assessed as possible by the MAH with plausible alternative explanations: metamizole (positive rechallenge and positive patch test) and psoriasis. Causal relationship between lipomatous metaplasia and the analgesics is assessed as possible by the MAH with a plausible alternative explanation: chronic skin inflammation due to the underlying psoriasis.

Reporter's comment:

Krewel Meuselbach GmbH, 24.04.2008 DE-KREWEL-000685

Sender diagnosis: not reported

Narrative

A 69-year-old female patient with history of psoriasis developed acute generalised exanthematic pustulosis (AGEP) after treatment with metamizole and diclofenac. The patient was admitted to hospital with a suberythrodermia. A generalised pustular exanthema, accompanied by fever $>39^{\circ}\text{C}$, leukocytosis and increased erythrocyte sedimentation rate was noted. Histological examination of skin biopsies revealed subcorneal and spongiform pustules and a diagnosis of AGEP was made. It was discovered that she had received metamizole the day before occurrence of the skin rash and additional diclofenac was applied intramuscularly 2 days before pustules became apparent. Corrective treatment was given and the cutaneous manifestations declined. Within 2 months, she developed a second course of AGEP after repeated treatment with metamizole. Histology of skin site biopsies showed an atrophic epidermis and mature adipocytes were detected. A lipomatous metaplasia, possibly as a consequence of the severe and chronic inflammation of the skin due to psoriasis and AGEP was made. Patch testing of analgesics revealed positive reactions to metamizole and indomethacin. The authors state "metamizole seems to be the most likely drug causative for this severe immune reaction. And after avoidance of the suspected analgesics metamizole and diclofenac the clinical condition is stable."

NOTE: Other risk factors: Co-suspected diclofenac, untreated psoriasis.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

This German literature case report describes a severe allergic skin reaction after treatment with metamizole and diclofenac. Allergic skin reactions like fixed drug eruption and toxic epidermal necrolysis are listed for Nopain our metamizole containing medicinal product. Metaplasia which is not expressis verbis listed for Nopain was probably a consequence of the dermal inflammation. The causality has been assessed as possible. The medical history of psoriasis probably contributed to the occurrence and severity of the skin reaction. Further measures are not necessary.

Reporter's comment:

MENARINIP, 28.04.2008 DE-MEN-12074

Sender diagnosis: not reported

Narrative

This case identified within the international literature, involved a 69-year-old female patient with history of psoriasis, who developed acute generalised exanthematic pustulosis (AGEP) after treatment with metamizole and diclofenac. The patient was admitted to hospital with a suberythrodermia. A generalised pustular exanthema, accompanied by fever $>39^{\circ}\text{C}$, leukocytosis and increased erythrocyte sedimentation rate was noted. Histological examination of skin biopsies revealed subcorneal and spongiform pustules and a diagnosis of AGEP was made. It was discovered that she had received metamizole the day before occurrence of the skin rash and additional diclofenac was applied intramuscularly 2 days before pustules became apparent. Corrective treatment was given and the cutaneous manifestations declined.

The authors state "metamizole seems to be the most likely drug causative for this severe immune reaction. And after avoidance of the suspected analgesics metamizole and diclofenac the clinical condition is stable."

Sender's comment:

Due to the close temporal relationship, we consider the event as possible related to the intake of diclofenac and metamizol. Especially AGEP is not listed in the SPC, but other serious skin diseases like Stevens-Johnson-Syndrome and Lyell-Syndrome are known. No measures are deemed necessary.

Reporter's comment:NOVARTIS, 29.04.2008 DE-002147023-PHBS2008DE04154

Sender diagnosis: not reported

Narrative

Literature report received on 21 Apr 2008. This patient was admitted to hospital with a suberythrodermia twice within 2 months. She showed a generalized pustular exanthema on the head, trunk and in the intertriginous areas, accompanied by fever $>39^{\circ}\text{C}$, leukocytosis of about $30 \times 10^3/\text{ul}$ and increased erythrocyte sedimentation rate (70 mm/h). Histological examination of skin biopsies taken from the abdomen revealed subcorneal and spongiform pustules which led us to the diagnosis of acute generalized exanthematic pustulosis (AGEP). We diagnosed a lipomatous metaplasia of the dermis and hypothesize that this metaplasia occurred as a consequence of the severe and chronic inflammation of the skin. The medical history revealed that she had received metamizole the day before occurrence of the skin rash in both cases. In addition, concerning the first episode, diclofenac (manufacturer name unknown) was applied intramuscularly 2 days before pustules became apparent. After treatment of the first episode of the AGEP with corticosteroids (prednisolone in declining therapy scheme for 20 days), the cutaneous manifestations declined. Now static, coalescing lucent nodules ranging from red to yellow-brownish appeared. They were located preferentially on the thighs and upper arms. According to the patient's assertion she had never noticed these skin alterations until a few days before. Notably, the skin rash was neither itchy nor painful. Histology of skin site biopsies of both thighs and upper arms showed an atrophic epidermis with a washy basal membrane zone, a sparse lymphocytic perivascular infiltrate around the upper plexus and a prominent vascularization of the papillary dermis. Right beneath the papillary dermis, a well-vascularized band of fatty tissue consisting of mature adipocytes was detected. Immunohistochemical stainings of CD68 excluded postinflammatory xanthomization, while S100 staining marked mature adipocytes. We diagnosed a lipomatous metaplasia, possibly as a consequence of the severe and chronic inflammation of the skin due to psoriasis and AGEP. The further medical history revealed that she had suffered from psoriasis since the age of 20. The chronically relapsing psoriasis was tolerated by the patient. An appropriate antipsoriatic therapy was rejected except for a tar-containing topical drug in the 1950s. The medical history did not reveal any allergy, especially not one against drugs. Besides adiposity, the physical examination was without pathological findings. Laboratory tests of peripheral blood showed normal parameters of the liver and kidneys; serum lipids were in the normal range with subnormal triglycerides. Immunoelectrophoresis and phenotyping of peripheral lymphocytes were performed without pathological findings. Both an abdominal ultrasound and a computed tomography of the cervix, thorax, abdomen and pelvis revealed no pathological findings. Medical investigations by our cardiology, ear, nose and throat and urology specialists revealed merely hypertension, vertigo of unknown origin and cystic kidneys. Four weeks after the last ingestion of corticosteroids, the patient underwent allergological diagnosis. Patch testing of analgetics revealed positive reactions to metamizole and indomethacin 48 and 72 h after application. Prick testing of analgetics was negative and had not shown any evidence of a type I allergy. The patient rejected an oral provocation with metamizole and diclofenac. However, acetylsalicylic acid, celecoxib and tramadol were tolerated orally as alternative analgetic drugs.

Sender's comment:

Serious spontaneous report [medically significant] acute generalised exanthematous pustulosis, renal cyst assessed as unlisted according to the Core Data Sheet.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

All remaining reported leading events were assessed as non-serious by the Novartis Medical Safety Physician.

Reporter's comment:

Sanofi-Aventis Deutschland GmbH, 02.05.2008 DE-AVENTIS-200811905EU

Sender diagnosis: not reported

Narrative

Case received from a literature article through our local affiliate (Lit NOV 02/08) on 21-Apr-2008.

A 69-year-old woman was admitted to hospital with a suberythrodermia twice within 2 months. She showed a generalized pustular exanthema on the head, trunk and in the intertriginous areas, accompanied by fever > 39 degrees C, leukocytosis of about $30 \times 10^3/\text{mL}$ and increased erythrocyte sedimentation rate (70 mm/h). Histological examination of skin biopsies taken from the abdomen revealed subcorneal and spongiform pustules which led to the diagnosis of acute generalized exanthematic pustulosis (AGEP). The medical history revealed that she had received metamizole the day before occurrence of the skin rash in both cases. In addition, concerning the first episode, diclofenac was applied intramuscularly 2 days before pustules became apparent. After treatment of the first episode of the AGEP with corticosteroids (prednisolone intravenously, initial dose 250 mg in declining therapy scheme for 20 days), the cutaneous manifestations declined.

Now static, coalescing lucent nodules ranging from red to yellow-brownish appeared. They were located preferentially on the thighs and upper arms. According to the patient's assertion she had never noticed these skin alterations until a few days before. Notably, the skin rash was neither itchy nor painful. Histology of skin site biopsies of both thighs and upper arms showed an atrophic epidermis with a washy basal membrane zone, a sparse lymphocytic perivascular infiltrate around the upper plexus and a prominent vascularisation of the papillary dermis. Right beneath the papillary dermis, a well-vascularized band of fatty tissue consisting of mature adipocytes was detected. Immunohistochemical stainings of CD68 excluded postinflammatory xanthomization, while S100 staining marked mature adipocytes. The authors diagnosed a lipomatous metaplasia, possibly as a consequence of the severe and chronic inflammation of the skin due to psoriasis and AGEP.

The further medical history revealed that she had suffered from psoriasis since the age of 20. The chronically relapsing psoriasis was tolerated by the patient. An appropriate antipsoriatic therapy was rejected except for a tar-containing topical drug in the 1950s. The medical history did not reveal any allergy, especially not one against drugs.

Besides adiposity, the physical examination was without pathological findings. Laboratory tests of peripheral blood showed normal parameters of the liver and kidneys; serum lipids were in the normal range with subnormal triglycerides. Immunoelectrophoresis and phenotyping of peripheral lymphocytes were performed without pathological findings. Both an abdominal ultrasound and a computed tomography of the cervix, thorax, abdomen and pelvis revealed no pathological findings. Medical investigations by cardiology, ear, nose and throat and urology specialists revealed merely hypertension, vertigo of unknown origin and cystic kidneys.

Four weeks after the last ingestion of corticosteroids, the patient underwent allergological diagnosis. Patch testing of analgetics revealed positive reactions to metamizole and indomethacin 48 and 72 h after application. Prick testing of analgetics was negative and did not show any evidence of a type I allergy. The patient rejected an oral provocation with metamizole and diclofenac. However, acetylsalicylic acid, celecoxib and tramadol were tolerated orally as alternative analgetic drugs.

Follow up with skin biopsies every 3 months did not show any new clinical or histological changes.

The authors state "metamizole seems to be the most likely drug causative for this severe immune reaction. In the skin lipomatous metaplasia is very uncommon. After avoidance of the suspected analgesics metamizole and diclofenac, and adequate therapy of the psoriasis the clinical condition is stable."

Please note: This case has also been entered into the legacy Sanofi database with the number A01200804507.

Sender's comment:

Sanofi aventis company comment dated 29-APR-2008:

The chronology, i.e. occurrence of AGEF 1 day after metamizole administration with positive rechallenge, and results of patch testing are suggestive of a causal role of metamizole in the occurrence of AGEF. The respective role of long standing untreated psoriasis and AGEF in the onset of lipomatous metaplasia is difficult to assess.

02-MAY-2008 Sanofi Aventis Germany: AGEF and DIFFUSE LIPOMATOUS METAPLASIA are considered locally unlabelled for metamizole (Novalgin). At present, the evaluation of this report does not imply a change in the state of the scientific knowledge for the product. Measures according to Para 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

SANOFI-SYNTHELABO, 05.05.2008 DE-SANOFI-SYNTHELABO-A01200804507

Sender diagnosis: not reported

Narrative

Reaction as reported: acute generalized exanthematous pustulosis

LLT: ACUTE GENERALIZED EXANTHEMATOUS PUSTULOSIS / 10062372

PT: ACUTE GENERALISED EXANTHEMATOUS PUSTULOSIS / 10048799

Reaction as reported: diffuse lipomatous metaplasia of the dermis

LLT: LIPOMATOSIS / 10061228

PT: LIPOMATOSIS / 10061228

Reaction as reported: diffuse lipomatous metaplasia of the dermis

LLT: METAPLASIA / 10054949

PT: METAPLASIA / 10054949

Narrative:

Case received from a literature article through our local affiliate (Lit NOV 02/08) on 21-Apr-2008.

A 69-year-old woman was admitted to hospital with a suberythrodermia twice within 2 months. She showed a generalized pustular exanthema on the head, trunk and in the intertriginous areas, accompanied by fever > 39 degrees C, leukocytosis of about $30 \times 10^3/\text{mL}$ and increased erythrocyte sedimentation rate (70 mm/h). Histological examination of skin biopsies taken from the abdomen revealed subcorneal and spongiform pustules led to the diagnosis of acute generalized exanthematous pustulosis (AGEP). The medical history revealed that she had received metamizole the day before occurrence of the skin rash in both cases. In addition, concerning the first episode, diclofenac was applied intramuscularly 2 days before pustules became apparent. After treatment of the first episode of the AGEF with corticosteroids (prednisolone intravenously, initial dose 250 mg in declining therapy scheme for 20 days), the cutaneous manifestations declined.

Now static, coalescing lucent nodules ranging from red to yellow-brownish appeared. They were located preferentially on the thighs and upper arms. According to the patient's assertion she had never noticed these skin alterations until a few days before. Notably, the skin rash was neither itchy nor painful. Histology of skin site biopsies of both thighs and upper arms showed an atrophic epidermis with a washy basal membrane zone, a sparse lymphocytic perivascular infiltrate around the upper plexus and a prominent vascularisation of the papillary dermis. Right beneath the papillary dermis, a well-vascularized band of fatty tissue consisting of mature adipocytes was detected. Immunohistochemical stainings of CD68 excluded postinflammatory xanthomization, while S100 staining marked mature adipocytes. The authors diagnosed a lipomatous metaplasia, possibly as a consequence of the severe and chronic inflammation of the skin due to psoriasis and AGEF.

The further medical history revealed that the patient had suffered from psoriasis since the age of 20. The chronically relapsing psoriasis was tolerated by the patient. An appropriate antipsoriatic therapy was rejected except for a tar-containing topical drug in the 1950s. The medical history did not reveal any allergy, especially not one against drugs.

Besides adiposity, the physical examination was without pathological findings. Laboratory tests of peripheral blood showed normal parameters of the liver and kidneys; serum lipids were in the normal range with subnormal triglycerides. Immunoelectrophoresis and phenotyping of peripheral lymphocytes were performed without pathological findings. Both an abdominal ultrasound and a computed tomography of the cervix, thorax, abdomen and pelvis revealed no pathological findings. Medical investigations by cardiology, ear, nose and throat and urology specialists revealed merely hypertension, vertigo of unknown origin and cystic kidneys.

Four weeks after the last ingestion of corticosteroids, the patient underwent allergological diagnosis. Patch testing of analgetics revealed positive reactions to metamizole and indomethacin 48 and 72 h after application. Prick testing of analgetics was negative and did not show any evidence of a

type I allergy. The patient rejected an oral provocation with metamizole and diclofenac. However, acetylsalicylic acid, celecoxib and tramadol were tolerated orally as alternative analgetic drugs.

Follow up with skin biopsies every 3 months over 1 year did not show any new clinical or histological changes.

The authors state "metamizole seems to be the most likely drug causative for this severe immune reaction. In the skin lipomatous metaplasia is very uncommon. After avoidance of the suspected analgesics metamizole and diclofenac, and adequate therapy of the psoriasis the clinical condition is stable."

An associated case has been also created for metamizole in the database of products from aventis origin with the ref 200811905EU.

Sender's comment:

Winthrop Arzneimittel Germany E2B Evaluator's comment 22 Apr 2008 < Allergic skin reactions like fixed drug eruption, maculopapulous exanthema, as well as other slight skin reactions, possibly with generalisation are listed in the SPC of metamizole containing products of Winthrop Arzneimittel GmbH. AGEP and lipomatous metaplasia are unlisted. The causal connection between the application of metamizole and the genesis of the skin reaction is regarded as possible.>

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 18.08.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-04813GD

Sender diagnosis: not reported

Narrative

A woman with a history of acute generalized exanthematic pustulosis (AGEP) caused by metamizole is described. Furthermore, she had suffered from an untreated psoriasis since the age of 20. After an adequate therapy of both psoriasis and AGEP, a lipomatous metaplasia of the dermis was diagnosed and hypothesized that this metaplasia occurred as a consequence of the severe and chronic inflammation of the skin.

A 69-year-old woman was admitted to hospital with a suberythrodermia twice within 2 months. She showed a generalized pustular exanthema on the head, trunk and in the intertriginous areas, accompanied by fever >39 degC, leukocytosis of about $30 \times 10^3/\text{mcl}$ and increased erythrocyte sedimentation rate (70 mm/h). Histological examination of skin biopsies taken from the abdomen revealed subcorneal and spongiform pustules which led us to the diagnosis of AGEP. The medical history revealed that she had received metamizole the day before occurrence of the skin rash in both cases. In addition, concerning the first episode, diclofenac was applied intramuscularly 2 days before pustules became apparent. After treatment of the first episode of the AGEP with corticosteroids (prednisolone intravenously, initial dose 250 mg in declining therapy scheme for 20 days), the cutaneous manifestations declined. Now static, coalescing lucent nodules ranging from red to yellow-brownish appeared. They were located preferentially on the thighs and upper arms. According to the patient's assertion she had never noticed these skin alterations until a few days before. Notably, the skin rash was neither itchy nor painful. Histology of skin site biopsies of both thighs and upper arms showed an atrophic epidermis with a washy basal membrane zone, a sparse lymphocytic perivascular infiltrate around the upper plexus and a prominent vascularization of the papillary dermis. Right beneath the papillary dermis, a well-vascularized band of fatty tissue consisting of mature adipocytes was detected. Immunohistochemical stainings of CD68 excluded postinflammatory xanthomization, while S100 staining marked mature adipocytes. A lipomatous metaplasia was diagnosed, possibly as a consequence of the severe and chronic inflammation of the skin due to psoriasis and AGEP. The further medical history revealed that she had suffered from psoriasis since the age of 20. The chronically relapsing psoriasis was tolerated by the patient. An appropriate antipsoriatic therapy was rejected except for a tar-containing topical drug in the 1950s. The medical history did not reveal any allergy, especially not one against drugs. Besides adiposity, the physical examination was without pathological findings. Laboratory tests of peripheral blood showed normal parameters of the liver and kidneys; serum lipids were in the normal range with subnormal triglycerides. Immunoelectrophoresis and phenotyping of peripheral lymphocytes were performed without pathological findings. Both an abdominal ultrasound and a computed tomography of the cervix, thorax, abdomen and pelvis revealed no pathological findings. Medical investigations revealed merely hypertension, vertigo of unknown origin and cystic kidneys. Four weeks after the last ingestion of corticosteroids, the patient underwent allergological diagnosis. Patch testing of analgetics revealed positive reactions to metamizole and indomethacin 48 and 72 h after application. Prick testing of analgetics was negative and did not show any evidence of a type I allergy. The patient rejected an oral provocation with metamizole and diclofenac. However, acetylsalicylic acid, celecoxib and tramadol were tolerated orally as alternative analgetic drugs.

Authors' statement: 'The patient suffered from a chronically inflamed skin due to an untreated psoriasis for nearly 50 years. At least twice she developed an AGEP meeting both the clinical and the histological criteria. Following medical examination and positive

patch testing ... metamizole seems to be the most likely drug causative for this severe immune reaction ... With an adequate therapy of the psoriasis and avoidance of the suspected analgetics metamizole and diclofenac, the clinical condition of the skin is stable, as is the dispersal and quantity of the lipomatous metaplasia.'

Sender's comment:

Reporter's comment:

Boehringer Ingelheim GmbH, Germany, 19.08.2008 DE-Boehringer Ingelheim GmbH, Germany-2008-DE-04813GD

Sender diagnosis: not reported

Narrative

A woman with a history of acute generalized exanthematic pustulosis (AGEP) caused by metamizole is described. Furthermore, she had suffered from an untreated psoriasis since the age of 20. After an adequate therapy of both psoriasis and AGEP, a lipomatous metaplasia of the dermis was diagnosed and hypothesized that this metaplasia occurred as a consequence of the severe and chronic inflammation of the skin.

A 69-year-old woman was admitted to hospital with a suberythrodermia twice within 2 months. She showed a generalized pustular exanthema on the head, trunk and in the intertriginous areas, accompanied by fever >39 degC, leukocytosis of about $30 \times 10^3/\text{mcl}$ and increased erythrocyte sedimentation rate (70 mm/h). Histological examination of skin biopsies taken from the abdomen revealed subcorneal and spongiform pustules which led us to the diagnosis of AGEP. The medical history revealed that she had received metamizole the day before occurrence of the skin rash in both cases. In addition, concerning the first episode, diclofenac was applied intramuscularly 2 days before pustules became apparent. After treatment of the first episode of the AGEP with corticosteroids (prednisolone intravenously, initial dose 250 mg in declining therapy scheme for 20 days), the cutaneous manifestations declined. Now static, coalescing lucent nodules ranging from red to yellow-brownish appeared. They were located preferentially on the thighs and upper arms. According to the patient's assertion she had never noticed these skin alterations until a few days before. Notably, the skin rash was neither itchy nor painful. Histology of skin site biopsies of both thighs and upper arms showed an atrophic epidermis with a washy basal membrane zone, a sparse lymphocytic perivascular infiltrate around the upper plexus and a prominent vascularization of the papillary dermis. Right beneath the papillary dermis, a well-vascularized band of fatty tissue consisting of mature adipocytes was detected. Immunohistochemical stainings of CD68 excluded postinflammatory xanthomization, while S100 staining marked mature adipocytes. A lipomatous metaplasia was diagnosed, possibly as a consequence of the severe and chronic inflammation of the skin due to psoriasis and AGEP. The further medical history revealed that she had suffered from psoriasis since the age of 20. The chronically relapsing psoriasis was tolerated by the patient. An appropriate antipsoriatic therapy was rejected except for a tar-containing topical drug in the 1950s. The medical history did not reveal any allergy, especially not one against drugs. Besides adiposity, the physical examination was without pathological findings. Laboratory tests of peripheral blood showed normal parameters of the liver and kidneys; serum lipids were in the normal range with subnormal triglycerides. Immunoelectrophoresis and phenotyping of peripheral lymphocytes were performed without pathological findings. Both an abdominal ultrasound and a computed tomography of the cervix, thorax, abdomen and pelvis revealed no pathological findings. Medical investigations revealed merely hypertension, vertigo of unknown origin and cystic kidneys. Four weeks after the last ingestion of corticosteroids, the patient underwent allergological diagnosis. Patch testing of analgetics revealed positive reactions to metamizole and indomethacin 48 and 72 h after application. Prick testing of analgetics was negative and did not show any evidence of a type I allergy. The patient rejected an oral provocation with metamizole and diclofenac. However, acetylsalicylic acid, celecoxib and tramadol were tolerated orally as alternative analgetic drugs.

Authors' statement: 'The patient suffered from a chronically inflamed skin due to an untreated psoriasis for nearly 50 years. At least twice she developed an AGEP meeting both the clinical and the histological criteria. Following medical examination and positive

patch testing ... metamizole seems to be the most likely drug causative for this severe immune reaction ... With an adequate therapy of the psoriasis and avoidance of the suspected analgetics metamizole and diclofenac, the clinical condition of the skin is stable, as is the dispersal and quantity of the lipomatous metaplasia.'

Sender's comment:

Regional Centre Drug Safety Boehringer Ingelheim

Comment on case 2008-DE-04813GD / METAMIZOLE

Except suberythrodermia and fever the events in this case report are not listed adverse drug reactions for the product in question. Based on the details currently available concerning this case there is no reasonable evidence in favour of a causal association between the reported adverse events and the product in question. Furthermore, the adverse events may be plausibly explained by other characteristics of the case (comed. with diclofenac).

As with all unlisted events, the company will continue to monitor all respective reports received and, based on cumulative experience, will re-evaluate the available evidence on an ongoing basis. No change in the company core safety information or other measures are recommended at this time.

GEIERH

Case assessment created by GE on 18 Aug 2008

Reporter's comment:

PLIVA Research and Development Ltd., 03.11.2008 DE-PLIVA Research and Development Ltd.-107#08#2008-05259

Sender diagnosis: not reported

Narrative

This case was found in scientific literature.

A 69-year-old woman developed acute generalised exanthematic pustulosis (AGEP) leading to lipomatous metaplasia, after receiving dipyrone [therapeutic indication not stated].

The woman had a history of relapsing psoriasis. She was hospitalised twice in 2 months with suberythrodermia, and a generalised pustular exanthema on her head, trunk and in the intertriginous areas. On both occasions, the rash had started 1 day after receiving dipyrone [dosage not stated]. The first occasion was also preceded by administration of diclofenac (brand/trade name unknown) 2 days prior. She also had a fever ($> 39^{\circ}\text{C}$), leucocytosis ($30 \times 10^3/\mu\text{L}$) and increased erythrocyte sedimentation rate (70 mm/h). Histology of an abdominal skin biopsy demonstrated subcorneal and spongiform pustules, consistent with AGEP.

The woman was treated with prednisolone for 20 days, and her skin symptoms subsided. She subsequently developed red to yellow-brownish lucent nodules on her thighs and upper arms. Histology of skin biopsies from those areas showed a band of fatty tissue beneath the papillary dermis, comprising mature adipocytes. She was diagnosed with dermal lipomatous metaplasia, possibly as a result of severe and chronic skin inflammation due to her psoriasis and AGEP.

Four weeks after the last dose of prednisolone, patch testing returned positive reactions to dipyrone and indometacin, at 48 and 72 hours.

Author Comment:

[Dipyrone] seems to be the most likely drug causative for this severe immune reaction ... the chronic and severe inflammation of the skin may have provoked this lipomatous metaplasia of the dermis.

Sender's comment:

Expedited reporting for diclofenac containing drugs of AWD.pharma:

According to the current SmPC of diclofenac containing drugs of AWD.pharma acute generalized exanthematic pustulosis (AGEP) and dermal lipomatous metaplasia are not listed.

The ADRs are not included in master text of BfArM and eMC either.

In the SmPC of the drugs mentioned above exanthema, erythema, purpura, bullous skin reactions such as Stevens-Johnson syndrome and toxic epidermal necrolysis are included.

The author stated that dipyrone seems to be the most likely drug responsible for development of the ADRs. Patch test revealed positive results for indometacin and dipyrone. Therefore it cannot be explicitly excluded that diclofenac contributed to the development of the ADRs. However drug causality cannot be verified due to the lack of information concerning treatment duration and dosage of diclofenac.

Literature search in PubMed revealed no cases associating diclofenac with lipomatous metaplasia. One case without verification was found on diclofenac in association with AGEP.

No case reports were found concerning the latter issue while searching drug safety database.

Risk minimizing measures are not necessary.

Reporter's comment:

PLIVA Research and Development Ltd., 11.11.2008 DE-PLIVA Research and Development Ltd.-107#08#2008-05259

Sender diagnosis: not reported

Narrative

This case was found in scientific literature.

A 69-year-old woman developed acute generalised exanthematic pustulosis (AGEP) leading to lipomatous metaplasia, after receiving dipyrone [therapeutic indication not stated].

The woman had a history of relapsing psoriasis. She was hospitalised twice in 2 months with suberythrodermia, and a generalised pustular exanthema on her head, trunk and in the intertriginous areas. On both occasions, the rash had started 1 day after receiving dipyrone [dosage not stated]. The first occasion was also preceded by administration of diclofenac (brand/trade name unknown) 2 days prior. She also had a fever ($> 39^{\circ}\text{C}$), leucocytosis ($30 \times 10^3/\mu\text{L}$) and increased erythrocyte sedimentation rate (70 mm/h). Histology of an abdominal skin biopsy demonstrated subcorneal and spongiform pustules, consistent with AGEP.

The woman was treated with prednisolone for 20 days, and her skin symptoms subsided. She subsequently developed red to yellow-brownish lucent nodules on her thighs and upper arms. Histology of skin biopsies from those areas showed a band of fatty tissue beneath the papillary dermis, comprising mature adipocytes. She was diagnosed with dermal lipomatous metaplasia, possibly as a result of severe and chronic skin inflammation due to her psoriasis and AGEP.

Four weeks after the last dose of prednisolone, patch testing returned positive reactions to dipyrone and indometacin, at 48 and 72 hours.

Author Comment:

[Dipyrone] seems to be the most likely drug causative for this severe immune reaction ... the chronic and severe inflammation of the skin may have provoked this lipomatous metaplasia of the dermis.

Full text article was received on 31-OCT-2008:

- intramuscular was added as route of administration for diclofenac
- intravenous was added as route of administration and daily dose of 250 mg for prednisolone (drug used to treat ADR)
- no further relevant information was included in the full text article

Sender's comment:

Expedited reporting for diclofenac containing drugs of AWD.pharma:

Follow-up information does not require any further comment. There is no change of the causality assessment.

According to the current SmPC of diclofenac containing drugs of AWD.pharma acute generalized exanthematic pustulosis (AGEP) and dermal lipomatous metaplasia are not listed.

The ADRs are not included in master text of BfArM and eMC either.

In the SmPC of the drugs mentioned above exanthema, erythema, purpura, bullous skin reactions such as Stevens-Johnson syndrome and toxic epidermal necrolysis are included.

The author stated that dipyrone seems to be the most likely drug responsible for development of the ADRs. Patch test revealed positive results for indometacin and dipyrone. Therefore it cannot be explicitly excluded that diclofenac contributed to the development of the ADRs. However drug causality cannot be verified due to the lack of information concerning treatment duration and dosage of diclofenac.

Literature search in PubMed revealed no cases associating diclofenac with lipomatous metaplasia. One case without verification was found on diclofenac in association with AGEP.

No case reports were found concerning the latter issue while searching drug safety database.

Risk minimizing measures are not necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08056499

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin exfoliation					Unknown
2 Skin red					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN EMULGEL	Topical	/	-	Breast inflammation
diclofenac diethylamine			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Breast operation NOS			Unknown	Status after breast surgery (possibly breast cancer)

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 14.05.2008 DE-002147023-CHNY2008DE01857

Sender diagnosis: not reported

Narrative

Initial pharmacist report received on 05 May 2008. This patient with a history of status after breast surgery (possible breast cancer) started Voltaren Emulgel (diclofenac diethylammonium salt) for a non-bacterial breast inflammation in May 2008. She experienced severe local skin redness and massive local skin exfoliation in May 2008. The patient was hospitalised. Voltaren was discontinued in May 2008. Final outcome was unknown.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Hospitalisation). The event Skin exfoliation was assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The event Erythema was already labelled in the product information.

Reporter's comment:

NOVARTIS, 22.05.2008 DE-NOVNCHHQ-CHNY2008DE01857

Sender diagnosis: not reported

Narrative

Initial pharmacist report received on 05 May 2008. This patient with a history of status after breast surgery (possible breast cancer) started Voltaren Emulgel (diclofenac diethylammonium salt) for a non-bacterial breast inflammation in May 2008. She experienced severe local skin redness and massive local skin exfoliation in May 2008. The patient was hospitalised. Voltaren was discontinued in May 2008. Final outcome was unknown.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Hospitalisation). The event Skin exfoliation was assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The event Erythema was already labelled in the product information.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-08079051

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No	No	No	No	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 72 Year	weight (kg): 68	BMI_int: 27.94	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthematic drug eruption	20080428			1 Day	Unknown
2 Urticarial rash	20080428			1 Day	Unknown
3 Urticaria drug-induced	20080428			1 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclofenac AL diclofenac sodium		25 Mg milligram(s)/Day	20080428- -	Spinal disorder

Primary source

Qualification: Physician observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)ALIUD PHARMA GmbH & Co. KG, 24.07.2008 DE-STADA-011615

Sender diagnosis: not reported

Narrative

This report was received from a physician via German Authority (case no. DE-BFARM-08079051). A 72-year-old female patient received diclofenac (25 mg once daily, on 2008-Apr-28, indication lumbar spine syndrome). On 2008-Apr-28, the patient developed disseminated urticarial drug rash - without circulatory problems - according to the reporter. The outcome of the event was unknown. No actions were taken regarding the event were reported. The patient had received diclofenac before without side effects. The patient's medical history included no allergies. Co-medication was not reported.

Sender's comment:

The MAH assessed the causal relationship between diclofenac and the suspected adverse reaction as possibly related based on the temporal association and the known safety profile of diclofenac which includes skin reactions. Urticaria and drug rash are listed in the current reference SPC. This case does not change the overall benefit-risk balance of the medicinal product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08148233

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 79 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythroderma	20060106			35 Day	fatal
2 Pruritus	20060105	20060209	36 Day	34 Day	fatal
3 Toxic epidermal necrolysis	20060104			33 Day	fatal
4 Genital erosion	20060107			36 Day	fatal
5 Pustule	20060106			35 Day	fatal
6 Redness	20060104			33 Day	fatal
7 Lumbar pain	20060103			32 Day	Unknown
8 Thoracic spine degeneration	20060103			32 Day	Unknown
9 Gastrointestinal erosion	20060107			36 Day	Unknown
10 Lip erosion	20060107			766 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMOXICILLIN amoxicillin	Oral	/	20051203- -	Urinary tract infection
2 ciprobay ciprofloxacin hydrochloride	Oral	/	20051203- -	Urinary tract infection
3 novalgin metamizole	Intravenous (not otherwis /		- -	Pain
4 novalgin metamizole		/	20030204- -	Pain
5 sigaprim forte sulfamethoxazole; trimethoprim	Oral	/	20051203- -	Urinary tract infection
6 TRAMAL tramadol hydrochloride	Intravenous (not otherwis / Injection		- -	Pain
7 TRAMAL tramadol hydrochloride	Oral	/	20030204- -	Pain
8 voltaren diclofenac sodium	Oral	/	20051203- -	Pain

Drugs (concomitant)					
9	ass acetylsalicylic acid	Oral	/	20030204-	Thrombosis
				-	
10	BALDRIAN valeriana officinalis root	Oral	/	-	Sedation
				-	
11	betaisodona povidone-iodine	Topical	/	20051129-	Skin disorder
			Ointment	-	
12	Betavert betahistine mesilate	Oral	/	-	Dizziness
				-	
13	clexane enoxaparin	Subcutaneous	/	-	Thrombosis
				-	
14	CORVATON molsidomine	Oral	/	20030204-	Coronary artery disease
				-	
15	decortin prednisone	Oral	/	20030204-	Arthrosis
				-	
16	dulcolax bisacodyl	Oral	/	-	Constipation
				-	
17	ibuprofen ibuprofen	Oral	/	20051203-	Pain
				-	
18	IMUREK azathioprine	Oral	/	20030204-	Renal transplant
				-	
19	jonosteril calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		-	Fluid replacement
				-	
20	klosterfrau melissengeist ethanol; herbal extracts nos	Oral	/	20051201-	Circulatory depression
				-	
21	Kontrastmittel	Intravenous (not otherwis /		-	Scintigraphy
				-	
22	korodin camphor; convallaria tincture; crataegus extract; menthol; valerian tincture	Oral	/	20051203-	Circulatory depression
				-	
23	limptar aminophylline; quinine sulfate	Oral	/	20051203-	Cramps calf
				-	
24	MONO-EMBOLEX heparin-fraction, sodium salt	Subcutaneous	/	-	Thrombosis
				-	
25	NEXIUM esomeprazole magnesium	Oral	/	20030204-	
				-	
26	nitro-spray isosorbide dinitrate	Oral	/	20051203-	Coronary artery disease
				-	
27	OXAZEPAM oxazepam	Oral	/	-	Sleep disorder
				-	
28	PASPERTIN metoclopramide hydrochloride	Oral	/	20051203-	Nausea
				-	
29	PASPERTIN metoclopramide hydrochloride	Intravenous (not otherwis /		-	
			Injection	-	
30	sortis atorvastatin calcium	Oral	/	20050920-	Hypercholesterolemia
				-	
31	TAGAMET cimetidine	Oral	/	20051203-	
				-	
32	valoron n naloxone hydrochloride; tilidine hydrochloride	Oral	/	20051203-	Pain
				-	
33	VIGANTOLETTEN colecalfiferol	Oral	/	20030204-	Osteoporosis
				-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Circulatory depression			Unknown	
Nausea			Unknown	
Coronary artery disease			Unknown	
Urinary tract infection	200512		Unknown	
Renal transplant	1986		Unknown	
Skin disorder			Unknown	
Osteoporosis			Unknown	
Hypercholesterolemia			Unknown	
Arthrosis			Unknown	
Dizziness			Unknown	
Cramps calf			Unknown	

Primary source

Qualification: Physician observestudytype: Individual patient use
Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 17.12.2008 DE-002147023-PHHY2008DE31718

Sender diagnosis: not reported

Narrative

Initial report received from a Health care professional on 11 Dec 2008 (case originating from non-Novartis patient registry called "RegiSCAR project Documentation of Severe Skin Reactions", case number 9170370): This multimorbid and polymedicated patient with a history of renal transplantation (1986) showed some unclear macular skin changes and mildly bleeding with onset of 29 Nov 2005. In the first week of Dec 2005 the patient suffered from urinary tract infection and she was treated with antibiotics (amoxicillin, Ciprobay) since 3 Dec 2005. Voltaren (diclofenac) therapy was also started on 3 Dec 2005 as a treatment of pain. Further drugs were used concomitantly for different indications. Patient was hospitalised on 3 Jan 2006 due to suspicion of thoracic spine syndrome or lumbalgia (these events were not reported as adverse drug reactions). One day later local skin redness was noticed and pruritus started on 5 Jan 2006. Suberythroderma, skin bladders, and positive Nikolski were found on 6 Jan 2006. On 7 Jan 2006, patient showed erosions at lips, gastric and genital mucosa. The diagnosis of lifethreatening Toxic Epidermal Necrolysis on large-area erythemas was made on 4 Jan 2006. The patient died on 9 Feb 2006, as a consequence of Toxic Epidermal Necrolysis. The specialist from the skin reaction center assessed the following drugs as causally related: Voltaren, Amoxicillin, Ciprobay (ciprofloxacin), Sigaprim forte (trimethoprim, sulfamethoxazol), Novalgin (metamizol) and Tramal (tramadol), both as oral and i.v. treatments. Also, some "possible" involvement of the previous renal transplantation and of the actual urinary tract infection was addressed by the skin reaction specialist. The event outcome was reported as death.

Sender's comment:

Serious adverse drug reaction report, spinal osteoarthritis (medically significant), assessed as not expected according to the Core Data Sheet.

Serious adverse drug reaction report, toxic epidermal necrolysis (death), assessed as not expected according to the Core Data Sheet.

Serious adverse drug reaction report, gastrointestinal erosion (hospitalization), assessed as not expected according to the Core Data Sheet.

The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. Reporting healthcare professional causality was suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-08153261

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 84 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Sleep disturbance					recovered/resolved
2 Stevens-Johnson syndrome	20051019	20051106	19 Day	22 Day	recovered/resolved
3 Transient ischemic attacks					recovered/resolved
4 Mucosal erosion	20051023	20051106	15 Day	26 Day	recovered/resolved
5 Skin xerosis	20051002	20051106	36 Day	5 Day	recovered/resolved
6 Skin disorder	20051022	20051106	16 Day	25 Day	recovered/resolved
7 Seizure					recovered/resolved
8 Restlessness					recovered/resolved
9 Pruritus	20051002	20051106	36 Day	5 Day	recovered/resolved
10 Oral candidiasis					recovered/resolved
11 Obstipation					recovered/resolved
12 Nausea					recovered/resolved
13 Fever	20051022	20051106	16 Day	25 Day	recovered/resolved
14 Exanthema		20051106			recovered/resolved
15 Blistering	20051025	20051105	12 Day	61 Day	recovered/resolved
16 Macular rash	20051019	20051106	19 Day	22 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 CETIRIZINE cetirizine		/	20051020-	
2 CETIRIZINE cetirizine	Oral	/	20051010-20051010	Pruritus
3 cotrim forte heumann sulfamethoxazole; trimethoprim	Oral	/	20051016-20051019	
4 mono-embolex certoparin sodium	Subcutaneous	/	20050928-20051010	Thrombosis prophylaxis
5 novalgin metamizole	Oral	/	20050826-20050927	Pain
6 PASPERTIN /GFR/ metoclopramide hydrochloride	Oral	/	20051014-	Nausea
7 PHENHYDAN phenytoin	Oral	/	20050918-	Seizures
8 PLANUM temazepam	Oral	/	20050928-	Sleep disturbance
9 voltaren dispers diclofenac sodium	Oral	/	20050826-20050927	Pain

Drugs (concomitant)				
10	Acetylsalizylsaeure acetylsalicylic acid	Oral	/	20050918-20051018 Thrombosis prophylaxis 31 Day
11	BIFITERAL "SOLVAY ARZNEIMITTEL" lactulose	Oral	/ Syrup	20050918-20051006 Obstipation 19 Day
12	CARMEN lercanidipine	Oral	/	20050525- Hypertension arterial -
13	dacortin prednisone	Oral	/	20051021- -
14	delix plus hydrochlorothiazide; ramipril	Oral	/	20050918-20050927 Hypertension arterial 10 Day
15	fenistil dimetindene maleate		/ Gel	20051020- Pruritus -
16	FOSAMAX alendronate sodium	Oral	/	20050826-20051005 Osteoporosis 41 Day
17	HALDOL haloperidol decanoate		/	20051006-20051007 Restlessness 2 Day
18	ideos calcium carbonate; colecalciferol	Oral	/ Dispersible tablet	20050826- Osteoporosis -
19	METFORMIN metformin	Oral	/	20050525-20051016 Diabetes mellitus 4 Month
20	movicol macrogol; potassium chloride; sodium bicarbonate; sodium chloride		/	20051006- Obstipation -
21	NEXIUM esomeprazole magnesium		/	20051018-20051020 3 Day
22	PANTOZOL pantoprazole sodium	Oral	/	20050918-20050927 Prophylaxis 10 Day
23	PANTOZOL pantoprazole sodium		/	20051021- -
24	RIVOTRIL clonazepam	Oral	/	20051004-20051007 Restlessness 4 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes mellitus			Unknown	
Osteoporosis			Unknown	
Hypertension arterial			Unknown	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.12.2008 DE-002147023-GXKR2008DE11780

Sender diagnosis: not reported

Narrative

Report received from an intensive monitoring project (German documentation centre for severe skin diseases dZh, Prof. Schöpf, Freiburg, case no. 9170345) on 11 Dec 2008: A patient initiated Voltaren Dispers (diclofenac, 1-0-1/d) and Novalgin (metamizole, 30drops four times daily) for pain and Fosamax 70mg (alendronate sodium, once weekly) and Ideos chewing tablets (calcium carbonate, colecalciferol, 0-1-0/d or 1-0-1/d) for osteoporosis on 26 Aug 2005. Concomitantly the patient received Carmen 10mg (lercanidipine, 1-0-1/d) and metformin 850mg (1-0-0.5/d) for arterial hypertension and diabetes mellitus. The patient was hospitalized on 18 Sep 2005. On the same day treatment with Delix plus 5mg (ramipril, hydrochlorothiazide; 0.5-0-0 tablets/d), Bifiteral Sirup (lactulose, 10-0-0ml/d), ASS 100mg (acetylsalicylic acid, 1-0-0/d), Phenhydan 100mg (phenytoin, 1-1-1/d) and Pantozol 40mg (pantoprazole, 0-0-1/d or 0-0-0.5/d) were started for arterial hypertension, obstipation, thrombosis prophylaxis, seizure during transient ischemic attack and stomach protection. On 27 Sep 2005, Novalgin, Voltaren Dispers and Pantozol were stopped. Mono-Embolex (certoparin, "0.5mg"-0-0/d) and Planum mite (temazepam, 0-0-01 or 0-0-1/d) were started for thrombosis prophylaxis and sleeping disturbance respectively on 28 Sep 2005. On 02 Oct 2005 the patient developed pruritus and skin xerosis. On 04 Oct 2005 Rivotril 0.5mg (clonazepam, 0-0-1/d, 1-0-1/d or 1-1-1/d) and on 06 Oct 2005 Haldol Tropfen (haloperidol, 3x10 drops/d), Pantozol 40mg and Movicol (polyethylene glycol, 1-0-0/d) were started for restlessness, stomach

protection and obstipation respectively. Haldol and Rivotril were discontinued on 07 Oct 2005. On 08 Oct 2005 prednisolone ointment (1-0-1/d) was initiated for skin symptoms. On 10 Oct 2005 Pantozol 40mg and Mono-Embolex were stopped and cetirizine 10mg (0-0-1/d) was given once for pruritus. On 12 Oct 2005 prednisolone ointment was discontinued. On 14 Oct 2005 Paspertin Tropfen (metoclopramide, 15 drops TID) was started for nausea and on 16 Oct 2005 metformin was stopped and Cotrim forte (sulfamethoxazole, trimethoprim; 0-0-1/d) was started. On 18 Oct 2005 ASS 100mg was stopped and Nexium mups 40mg (esomeprazole, 0-0-1/d) was initiated for stomach protection. On 19 Oct 2005 the patient developed Stevens Johnson syndrome and maculae. On the same day Cotrim forte was stopped. On 20 Oct 2005 Nexium mups 40mg was stopped and cetirizine 10mg and Fenistil gel (dimetindene, 1-0-1/d) were started for pruritus and exanthema. On 21 Oct 2005 Pantozol 40mg was reinitiated and Decortin 5mg (prednisolone, p.o.; 1-0-1/d or 4-0-0/d) was started. On 22 Oct 2005 the patient developed fever and the skin symptoms aggravated and on 23 Oct 2008 oral mucosal erosions occurred. On 23 Oct 2005 Ampho-Moronal (amphotericin B, 1-0-1/d) and prednisolon (p.o.; 32 or 64mg; 0-1-0/d) were given once for suspected oral candidiasis and for the exanthema. On 24 Oct 2005 the patient developed lip and genital mucosal erosions and one day later also blistering and nasal mucosal erosions occurred. An infection was excluded. The symptoms lasted until 06 Nov 2005 and the patient recovered completely. The relationship to Phenhydan 100mg was noted as highly and probably suspected and to Cotrim forte as highly and well possibly suspected. The relationship to Mono-Embolex, Planum mite, cetirizine 10mg and Paspertin Tropfen was assessed as possible and the relationship to Bifiteral Sirup, Fosamax 70mg, metformin 850mg, ASS 100mg, Carmen 10mg, Pantozol 40mg, Ideos chewing tablets, Rivotril 0.5mg, Haldol Tropfen, Movicol, prednisolone ointment and Nexium mups 40mg was assessed as questionable. Novalgin and Voltaren Dispers were noted as suspected and also a relationship of not suspected was noted regarding these drugs. No relationship was suspected to Delix plus 5mg, Fenistil Gel, Decortin 5mg, Ampho-Moronal and prednisolone.

Sender's comment:

Mono-Embolex: Serious adverse drug reaction report, Stevens-Johnson syndrome (hospitalisation), assessed as unexpected according to the Core Data Sheet. However, other alternative causes (co-suspect drug) provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

Convulsion and sleep disorder were assessed as not suspected for Mono-Embolex as there is no timely relationship.

Diclofenac: Serious adverse drug reaction report, Stevens-Johnson syndrome and convulsion (hospitalisation, medically significant), assessed as expected according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

The events: oral candidiasis, nausea, constipation, sleep disorder and restlessness were assessed as non-serious.

All remaining reported leading events and associated symptoms were assessed as non-serious by the Novartis Medical Safety Physician.

Reporter's comment:

NOVARTIS, 22.12.2008 DE-SZ09-GXKR2008DE11780

Sender diagnosis: not reported

Narrative

Report received from an intensive monitoring project (German documentation centre for severe skin diseases dZh, Prof. Schöpf, Freiburg, case no. 9170345) on 11 Dec 2008: A patient initiated Voltaren Dispers (diclofenac, 1-0-1/d) and Novalgin (metamizole, 30drops four times daily) for pain and Fosamax 70mg (alendronate sodium, once weekly) and Ideos chewing tablets (calcium carbonate, colecalciferol, 0-1-0/d or 1-0-1/d) for osteoporosis on 26 Aug 2005. Concomitantly the patient received Carmen 10mg (lercanidipine, 1-0-1/d) and metformin 850mg (1-0-0.5/d) for arterial hypertension and diabetes mellitus. The patient was hospitalized on 18 Sep 2005. On the same day treatment with Delix plus 5mg (ramipril, hydrochlorothiazide; 0.5-0-0 tablets/d), Bifiteral Sirup (lactulose, 10-0-0ml/d), ASS 100mg (acetylsalicylic acid, 1-0-0/d), Phenhydan 100mg (phenytoin, 1-1-1/d) and Pantozol 40mg (pantoprazole, 0-0-1/d or 0-0-0.5/d) were started for arterial hypertension, obstipation, thrombosis prophylaxis, seizure during transient ischemic attack and stomach protection. On 27 Sep 2005, Novalgin, Voltaren Dispers and Pantozol were stopped. Mono-Embolex (certoparin, "0.5mg"-0-0/d) and Planum mite (temazepam, 0-0-01 or 0-0-1/d) were started for thrombosis prophylaxis and sleeping disturbance respectively on 28 Sep 2005. On 02 Oct 2005 the patient developed pruritus and skin xerosis. On 04 Oct 2005 Rivotril 0.5mg (clonazepam, 0-0-1/d, 1-0-1/d or 1-1-1/d) and on 06 Oct 2005 Haldol Tropfen (haloperidol, 3x10 drops/d), Pantozol 40mg and Movicol (polyethylene glycol, 1-0-0/d) were started for restlessness, stomach protection and obstipation respectively. Haldol and Rivotril were discontinued on 07 Oct 2005. On 08 Oct 2005 prednisolone ointment (1-0-1/d) was initiated for skin symptoms. On 10 Oct 2005 Pantozol 40mg and Mono-Embolex were stopped and cetirizine 10mg (0-0-1/d) was given once for pruritus. On 12 Oct 2005 prednisolone ointment was discontinued. On 14 Oct 2005 Paspertin Tropfen (metoclopramide, 15 drops TID) was started for nausea and on 16 Oct 2005 metformin was stopped and Cotrim forte (sulfamethoxazole, trimethoprim; 0-0-1/d) was started. On 18 Oct 2005 ASS 100mg was stopped and Nexium mups 40mg (esomeprazole, 0-0-1/d) was initiated for stomach protection. On 19 Oct 2005 the patient developed Stevens Johnson syndrome and maculae. On the same day Cotrim forte was stopped. On 20 Oct 2005 Nexium mups 40mg was stopped and cetirizine 10mg and Fenistil gel (dimetindene, 1-0-1/d) were started for pruritus and exanthema. On 21 Oct 2005 Pantozol 40mg was reinitiated and

Decortin 5mg (prednisolone, p.o.; 1-0-/d or 4-0-0/d) was started. On 22 Oct 2005 the patient developed fever and the skin symptoms aggravated and on 23 Oct 2005 oral mucosal erosions occurred. On 23 Oct 2005 Ampho-Moronal (amphotericin B, 1-0-1/d) and prednisolone (p.o.; 32 or 64mg; 0-1-0/d) were given once for suspected oral candidiasis and for the exanthema. On 24 Oct 2005 the patient developed lip and genital mucosal erosions and one day later also blistering and nasal mucosal erosions occurred. An infection was excluded. The symptoms lasted until 06 Nov 2005 and the patient recovered completely. The relationship to Phenhydan 100mg was noted as highly and probably suspected and to Cotrim forte as highly and well possibly suspected. The relationship to Mono-Embolex, Planum mite, cetirizine 10mg and Paspertin Tropfen was assessed as possible and the relationship to Bifiteral Sirup, Fosamax 70mg, metformin 850mg, ASS 100mg, Carmen 10mg, Pantozol 40mg, Ideos chewing tablets, Rivotril 0.5mg, Haldol Tropfen, Movicol, prednisolone ointment and Nexium mups 40mg was assessed as questionable. Novalgin and Voltaren Dispers were noted as suspected and also a relationship of not suspected was noted regarding these drugs. No relationship was suspected to Delix plus 5mg, Fenistil Gel, Decortin 5mg, Ampho-Moronal and prednisolone.

Sender's comment:

Mono-Embolex: Serious adverse drug reaction report, Stevens-Johnson syndrome (hospitalisation), assessed as unexpected according to the Core Data Sheet. However, other alternative causes (co-suspect drug) provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

Convulsion and sleep disorder were assessed as not suspected for Mono-Embolex as there is no timely relationship.

Diclofenac: Serious adverse drug reaction report, Stevens-Johnson syndrome and convulsion (hospitalisation, medically significant), assessed as expected according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

The events: oral candidiasis, nausea, constipation, sleep disorder and restlessness were assessed as non-serious.

All remaining reported leading events and associated symptoms were assessed as non-serious by the Novartis Medical Safety Physician.

Reporter's comment:

NOVARTIS, 29.12.2008 DE-002147023-GXKR2008DE11780

Sender diagnosis: not reported

Narrative

Report received from an intensive monitoring project (German documentation centre for severe skin diseases dZh, Prof. Schöpf, Freiburg, case no. 9170345) on 11 Dec 2008: A patient initiated Voltaren Dispers (diclofenac, 1-0-1/d) and Novalgin (metamizole, 30drops four times daily) for pain and Fosamax 70mg (alendronate sodium, once weekly) and Ideos chewing tablets (calcium carbonate, colecalciferol, 0-1-0/d or 1-0-1/d) for osteoporosis on 26 Aug 2005. Concomitantly the patient received Carmen 10mg (lercanidipine, 1-0-1/d) and metformin 850mg (1-0-0.5/d) for arterial hypertension and diabetes mellitus. The patient was hospitalized on 18 Sep 2005. On the same day treatment with Delix plus 5mg (ramipril, hydrochlorothiazide; 0.5-0-0 tablets/d), Bifiteral Sirup (lactulose, 10-0-0ml/d), ASS 100mg (acetylsalicylic acid, 1-0-0/d), Phenhydan 100mg (phenytoin, 1-1-1/d) and Pantozol 40mg (pantoprazole, 0-0-1/d or 0-0-0.5/d) were started for arterial hypertension, obstipation, thrombosis prophylaxis, seizure during transient ischemic attack and stomach protection. On 27 Sep 2005, Novalgin, Voltaren Dispers and Pantozol were stopped. Mono-Embolex (certoparin, "0.5mg"-0-0/d) and Planum mite (temazepam, 0-0-01 or 0-0-1/d) were started for thrombosis prophylaxis and sleeping disturbance respectively on 28 Sep 2005. On 02 Oct 2005 the patient developed pruritus and skin xerosis. On 04 Oct 2005 Rivotril 0.5mg (clonazepam, 0-0-1/d, 1-0-1/d or 1-1-1/d) and on 06 Oct 2005 Haldol Tropfen (haloperidol, 3x10 drops/d), Pantozol 40mg and Movicol (polyethylene glycol, 1-0-0/d) were started for restlessness, stomach protection and obstipation respectively. Haldol and Rivotril were discontinued on 07 Oct 2005. On 08 Oct 2005 prednisolone ointment (1-0-1/d) was initiated for skin symptoms. On 10 Oct 2005 Pantozol 40mg and Mono-Embolex were stopped and cetirizine 10mg (0-0-1/d) was given once for pruritus. On 12 Oct 2005 prednisolone ointment was discontinued. On 14 Oct 2005 Paspertin Tropfen (metoclopramide, 15 drops TID) was started for nausea and on 16 Oct 2005 metformin was stopped and Cotrim forte (sulfamethoxazole, trimethoprim; 0-0-1/d) was started. On 18 Oct 2005 ASS 100mg was stopped and Nexium mups 40mg (esomeprazole, 0-0-1/d) was initiated for stomach protection. On 19 Oct 2005 the patient developed Stevens Johnson syndrome and maculae. On the same day Cotrim forte was stopped. On 20 Oct 2005 Nexium mups 40mg was stopped and cetirizine 10mg and Fenistil gel (dimetindene, 1-0-1/d) were started for pruritus and exanthema. On 21 Oct 2005 Pantozol 40mg was reinitiated and Decortin 5mg (prednisolone, p.o.; 1-0-/d or 4-0-0/d) was started. On 22 Oct 2005 the patient developed fever and the skin symptoms aggravated and on 23 Oct 2008 oral mucosal erosions occurred. On 23 Oct 2005 Ampho-Moronal (amphotericin B, 1-0-1/d) and prednisolone (p.o.; 32 or 64mg; 0-1-0/d) were given once for suspected oral candidiasis and for the exanthema. On 24 Oct 2005 the patient developed lip and genital mucosal erosions and one day later also blistering and nasal mucosal erosions occurred. An infection was excluded. The symptoms lasted until 06 Nov 2005 and the patient recovered completely. The relationship to Phenhydan 100mg was noted as highly and probably suspected and to Cotrim forte as highly and well possibly suspected. The relationship to Mono-Embolex, Planum mite, cetirizine 10mg and Paspertin Tropfen was assessed as possible and the relationship to Bifiteral Sirup, Fosamax 70mg, metformin 850mg, ASS 100mg, Carmen 10mg, Pantozol 40mg, Ideos chewing tablets, Rivotril 0.5mg,

Haldol Tropfen, Movicol, prednisolone ointment and Nexium mups 40mg was assessed as questionable. Novalgin and Voltaren Dispers were noted as suspected and also a relationship of not suspected was noted regarding these drugs. No relationship was suspected to Delix plus 5mg, Fenistil Gel, Decortin 5mg, Ampho-Moronal and prednisolone.

Significant follow-up after internal review on 23-Dec-2008. The suspected drug cetirizine had to be recoded to NGX. The assessment and comment were performed.

Sender's comment:

Mono-Embolex: Serious adverse drug reaction report, Stevens-Johnson syndrome (hospitalisation), assessed as unexpected according to the Core Data Sheet. However, other alternative causes (co-suspect drug) provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

Convulsion and sleep disorder were assessed as not suspected for Mono-Embolex as there is no timely relationship.

Cetirizine: Serious adverse drug reaction report, Stevens-Johnson syndrome (hospitalisation), assessed as unexpected according to the Core Data Sheet due to greater severity/specificity. The following term, erythema multiforme, is already included in the Core Data Sheet. Reporting healthcare professional causality is suspected.

Convulsion and sleep disorder were assessed as not suspected for Mono-Embolex as there is no timely relationship.

Diclofenac: Serious adverse drug reaction report, Stevens-Johnson syndrome and convulsion (hospitalisation, medically significant), assessed as expected according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

The events: oral candidiasis, nausea, constipation, sleep disorder and restlessness were assessed as non-serious.

All remaining reported leading events and associated symptoms were assessed as non-serious by the Novartis Medical Safety Physician.

Reporter's comment:

NOVARTIS, 29.12.2008 DE-SZ09-GXKR2008DE11780

Sender diagnosis: not reported

Narrative

Report received from an intensive monitoring project (German documentation centre for severe skin diseases dZh, Prof. Schöpf, Freiburg, case no. 9170345) on 11 Dec 2008: A patient initiated Voltaren Dispers (diclofenac, 1-0-1/d) and Novalgin (metamizole, 30drops four times daily) for pain and Fosamax 70mg (alendronate sodium, once weekly) and Ideos chewing tablets (calcium carbonate, colecalciferol, 0-1-0/d or 1-0-1/d) for osteoporosis on 26 Aug 2005. Concomitantly the patient received Carmen 10mg (lercanidipine, 1-0-1/d) and metformin 850mg (1-0-0.5/d) for arterial hypertension and diabetes mellitus. The patient was hospitalized on 18 Sep 2005. On the same day treatment with Delix plus 5mg (ramipril, hydrochlorothiazide; 0.5-0-0 tablets/d), Bifiteral Sirup (lactulose, 10-0-0ml/d), ASS 100mg (acetylsalicylic acid, 1-0-0/d), Phenhydan 100mg (phenytoin, 1-1-1/d) and Pantozol 40mg (pantoprazole, 0-0-1/d or 0-0-0.5/d) were started for arterial hypertension, obstipation, thrombosis prophylaxis, seizure during transient ischemic attack and stomach protection. On 27 Sep 2005, Novalgin, Voltaren Dispers and Pantozol were stopped. Mono-Embolex (certoparin, "0.5mg"-0-0/d) and Planum mite (temazepam, 0-0-01 or 0-0-1/d) were started for thrombosis prophylaxis and sleeping disturbance respectively on 28 Sep 2005. On 02 Oct 2005 the patient developed pruritus and skin xerosis. On 04 Oct 2005 Rivotril 0.5mg (clonazepam, 0-0-1/d, 1-0-1/d or 1-1-1/d) and on 06 Oct 2005 Haldol Tropfen (haloperidol, 3x10 drops/d), Pantozol 40mg and Movicol (polyethylene glycol, 1-0-0/d) were started for restlessness, stomach protection and obstipation respectively. Haldol and Rivotril were discontinued on 07 Oct 2005. On 08 Oct 2005 prednisolone ointment (1-0-1/d) was initiated for skin symptoms. On 10 Oct 2005 Pantozol 40mg and Mono-Embolex were stopped and cetirizine 10mg (0-0-1/d) was given once for pruritus. On 12 Oct 2005 prednisolone ointment was discontinued. On 14 Oct 2005 Paspertin Tropfen (metoclopramide, 15 drops TID) was started for nausea and on 16 Oct 2005 metformin was stopped and Cotrim forte (sulfamethoxazole, trimethoprim; 0-0-1/d) was started. On 18 Oct 2005 ASS 100mg was stopped and Nexium mups 40mg (esomeprazole, 0-0-1/d) was initiated for stomach protection. On 19 Oct 2005 the patient developed Stevens Johnson syndrome and maculae. On the same day Cotrim forte was stopped. On 20 Oct 2005 Nexium mups 40mg was stopped and cetirizine 10mg and Fenistil gel (dimetindene, 1-0-1/d) were started for pruritus and exanthema. On 21 Oct 2005 Pantozol 40mg was reinitiated and Decortin 5mg (prednisolone, p.o.; 1-0-/d or 4-0-0/d) was started. On 22 Oct 2005 the patient developed fever and the skin symptoms aggravated and on 23 Oct 2008 oral mucosal erosions occurred. On 23 Oct 2005 Ampho-Moronal (amphotericin B, 1-0-1/d) and prednisolon (p.o.; 32 or 64mg; 0-1-0/d) were given once for suspected oral candidiasis and for the exanthema. On 24 Oct 2005 the patient developed lip and genital mucosal erosions and one day later also blistering and nasal mucosal erosions occurred. An infection was excluded. The symptoms lasted until 06 Nov 2005 and the patient recovered completely. The relationship to Phenhydan 100mg was noted as highly and probably suspected and to Cotrim forte as highly and

well possibly suspected. The relationship to Mono-Embolex, Planum mite, cetirizine 10mg and Paspertin Tropfen was assessed as possible and the relationship to Bifiteral Sirup, Fosamax 70mg, metformin 850mg, ASS 100mg, Carmen 10mg, Pantozol 40mg, Ideos chewing tablets, Rivotril 0.5mg, Haldol Tropfen, Movicol, prednisolone ointment and Nexium mups 40mg was assessed as questionable. Novalgin and Voltaren Dispers were noted as suspected and also a relationship of not suspected was noted regarding these drugs. No relationship was suspected to Delix plus 5mg, Fenistil Gel, Decortin 5mg, Ampho-Moronal and prednisolone.

Significant follow-up after internal review on 23-Dec-2008. The suspected drug cetirizine had to be recoded to NGX. The assessment and comment were performed.

Sender's comment:

Mono-Embolex: Serious adverse drug reaction report, Stevens-Johnson syndrome (hospitalisation), assessed as unexpected according to the Core Data Sheet. However, other alternative causes (co-suspect drug) provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

Convulsion and sleep disorder were assessed as not suspected for Mono-Embolex as there is no timely relationship.

Cetirizine: Serious adverse drug reaction report, Stevens-Johnson syndrome (hospitalisation), assessed as unexpected according to the Core Data Sheet due to greater severity/specificity. The following term, erythema multiforme, is already included in the Core Data Sheet. Reporting healthcare professional causality is suspected.

Convulsion and sleep disorder were assessed as not suspected for Mono-Embolex as there is no timely relationship.

Diclofenac: Serious adverse drug reaction report, Stevens-Johnson syndrome and convulsion (hospitalisation, medically significant), assessed as expected according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

The events: oral candidiasis, nausea , constipation, sleep disorder and restlessness were assessed as non-serious.

All remaining reported leading events and associated symptoms were assessed as non-serious by the Novartis Medical Safety Physician.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09002013

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes	Yes					

Patient

Initials:	Sex: Male	Birth: privacy	Age: 44 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20071028	20071117	21 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 allopurinol allopurinol	Oral	300 Mg milligram(s)/Day	20071007-20071029	Pain
2 diclofenac diclofenac	Oral	50 Mg milligram(s)/	20071007-20071029	Gout
			23 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pain			Unknown	
Gout			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09003345

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes					

Patient

Initials:	Sex: Female	Birth: privacy	Age: 78 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20060615	deathcause: TEN
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens Johnson syndrome	20060603	20060615	13 Day		fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 aponal 50 doxepin hydrochloride	Oral	/Day	20060426-	Depression
2 Ass 100 mg acetylsalicylic acid	Oral	/	20060426-	Thrombosis prophylaxis
3 diclofenac diclofenac	Oral	/	20060601-20060602 2 Day	Shoulder pain
4 Glimepirid 3 mg glimepiride	Oral	/Day	20060426-	Diabetes mellitus

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Urinary tract infection			Yes	chronische HWI
Osteoporosis			Unknown	
Hypertension arterial			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09028257

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No		No						

Patient

Initials:	Sex: Female	Birth: privacy	Age: 42 Year	weight (kg): 59	BMI_int: 20.9	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Rash bullous	20090127				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Deflamat diclofenac sodium	Oral	150 Mg milligram(s)/Day	20090121-20090128 8 Day	Lumboischialgia
2 novalgin metamizole	Oral	60 Gtt drop(s)/Day	20090121-20090127 7 Day	Lumboischialgia
3 voltaren diclofenac sodium	Oral	50 Mg milligram(s)/Day	20090121-20090124 4 Day	Lumboischialgia

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Nickel sensitivity			Unknown	
Allergy			Unknown	odorous substance mix
Allergy			Unknown	Perubalsam
Trauma			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09042783

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 26 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Buccal mucosal roughening					not recovered/not resolved
2 Pain oral					not recovered/not resolved
3 Gut pain					not recovered/not resolved
4 Blistering					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo kd 75 diclofenac sodium	Oral	/	- 1 Day	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09043574

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 52 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Swelling					recovered/resolved
2 Redness					recovered/resolved
3 General malaise					recovered/resolved
4 Acute generalised exanthematous pustulosis					recovered/resolved
5 Eyelid oedema					recovered/resolved
6 GPT increased					Unknown
7 Oedema					recovering/resolving
8 Fever					recovering/resolving
9 CRP increased					recovered/resolved
10 Leucocytosis NOS					recovering/resolving
11 Pustule					recovering/resolving
12 Scales					Unknown
13 Common cold					recovered/resolved
14 Sore throat					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Oral	/	-	Joint disorder
diclofenac			-	
2 penicillin		/	-	
benzylpenicillin			-	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Prange B., Marini A., Kalke A., Hodzic-Avdagic N., Ruzicka T., Hengge U.R. Acute localized exanthematous pustulosis. Journal der Deutschen Dermatologischen Gesellschaft. 2005;3(3):210-212	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 06.04.2009 DE-002147023-PHHY2009DE12265

Sender diagnosis: not reported

Narrative

Initial literature report received on 26 Mar 2009: This patient developed acute localized exanthematous pustulosis after intake of penicillin (manufacturer unknown) and diclofenac (manufacturer unknown) for joints complaints on unknown dates. She presented with a common cold with sore throat, and one day after penicillin therapy, she complained of generalized malaise, redness and swelling in front of the right ear and on the right cheeks as well as in the area of the right lower eyelid, and fever. At the time of the presentation, she a large painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localized exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with penicillin and diclofenac was negative. The reporter stated that the event was probably caused by penicillin or diclofenac.

Sender's comment:

Serious literature report, acute generalised exanthematous pustulosis, transaminase increased, [medically significant], already listed in the Core Data Sheet. All literature reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

The remaining reported leading event was assessed as non-serious by the Novartis Medical Safety Physician.

Reporter's comment:NOVARTIS, 14.04.2009 DE-002147023-PHHY2009DE12265

Sender diagnosis: not reported

Narrative

Initial literature report received on 26 Mar 2009: This patient developed acute localized exanthematous pustulosis after intake of penicillin (manufacturer unknown) and diclofenac (manufacturer unknown) for joints complaints on unknown dates. She presented with a common cold with sore throat, and one day after penicillin therapy, she complained of generalized malaise, redness and swelling in front of the right ear and on the right cheeks as well as in the area of the right lower eyelid, and fever. At the time of the presentation, she a large painful swollen erythema with multiple small confluent pustules and oedema on the right cheek. Leukocytosis, elevation of transaminases (GOT, GPT, and GGT), a marked elevation of CRP and the ASL titre were present. Laboratory examinations and dermatohistopathologic findings were suggestive of acute localized exanthematous pustulosis. Following cessation of drugs and corrective treatment, the disease resolved within one week with scaling. Interval epicutaneous testing with penicillin and diclofenac was negative. The reporter stated that the event was probably caused by penicillin or diclofenac.

Follow-up information received on 01 Apr 2009: Full English translation received. Bacterial culture of the affected area showed propionibacteria. The treatment of the patient consisted on discontinuation of the suspect drug and saline-moistened compress.

Sender's comment:

Serious literature report, acute generalised exanthematous pustulosis, transaminase increased, [medically significant], already listed in the Core Data Sheet. All literature reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

The remaining reported leading event was assessed as non-serious by the Novartis Medical Safety Physician.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09080153

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 50 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthematic drug eruption					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	-	
diclofenac			-	

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09075383

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth: privacy	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Desquamation	200905				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dispers diclofenac sodium	Oral	/	200904-20090602	Herpes zoster
			-	

Drugs (concomitant)

2 ass acetylsalicylic acid		/	-	
3 bepanten dexpanthenol	Topical	/	-	Herpes zoster
4 GLIMEPIRIDE glimepiride		/	-	
5 GLUCOPHAGE metformin		/	-	Diabetes mellitus
6 KARVEA irbesartan		/	-	
7 METFORMIN metformin		/	-	Diabetes mellitus

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Vesicles			Unknown	
Pruritus			Unknown	pruritus at the herpes lesion sites
Diabetes mellitus			Unknown	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 16.06.2009 DE-002147023-PHHY2009DE22344

Sender diagnosis: not reported

Narrative

Initial consumer report received on 05 Jun 2009. The patient with diabetes mellitus was on treatment with Voltaren Dispers (diclofenac) b.i.d. for 45 days due to herpes zoster involving vesicles and pruritus at penis, testes, inner parts of upper thigh and after up to the tailbone. A topical treatment with Bepanthen (panthenol) was also used. The patient was under multiple concomitant medications. According to patient, the vesicles had vanished but he was still complaining about pruritus at the herpes lesion sites. Furthermore, the patient noticed desquamating skin at both hands, about 1 to 2 weeks ago, and therefore, he voluntarily stopped Voltaren Dispers therapy on 02 Jun 2009. NAC (acetylcysteine) effervescent tablets were taken as treatment for the event. The outcome was reported as condition unchanged.

Sender's comment:

Serious spontaneous report, skin exfoliation [medically significant], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09082268

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 72 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Eczema weeping					recovered/resolved
2 Skin ulceration	20090523			4 Day	recovered/resolved
3 Skin desquamation					recovered/resolved
4 Redness					recovered/resolved
5 Pain	20090523			4 Day	recovered/resolved
6 Infection localised					recovered/resolved
7 Dry skin					recovered/resolved
8 Blisters	20090523			4 Day	recovered/resolved
9 Toxic epitheliolysis				4 Day	recovered/resolved
10 Itching					recovered/resolved
11 Dry eczema					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN EMULGEL diclofenac diethylamine	Topical	/ 95	20090520-20090523 4 Day	Joint disorder

Drugs (concomitant)

2 ACC acetylcysteine	1 Df dosage form/Day	-	
3 co-diovan hydrochlorothiazide; valsartan	1 Df dosage form/Day	-	
4 CORANGIN "CIBA-GEIGY" isosorbide mononitrate	1 Df dosage form/Day	-	
5 CORTISONE cortisone	4 Mg milligram(s)/Day	-	Nasal disorder
6 CORVATON "CASSELLA-RIEDEL" molsidomine	1 Df dosage form/Day	-	
7 MARCUMAR phenprocoumon	1 Df dosage form/Day	-	
8 NEUROBION FORTE cyanocobalamin; pyridoxine hydrochloride; thiamine disulfide	2 Df dosage form/Day	-	
9 PANTOZOL pantoprazole sodium	2 Df dosage form/Day	-	
10 TEBONIN ginkgo tree leaves extract	1 Df dosage form/Day	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pulmonary embolism			Unknown	Mar 2009.
Nasal disorder			Unknown	
Psoriasis			Unknown	
COPD			Unknown	

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 26.06.2009 DE-NOVNCHHQ-CHNY2009DE02369

Sender diagnosis: not reported

Narrative

This physician report combines initial information received on 08 Jun 2009 and follow-ups received on 09 Jun 2009 and 10 Jun 2009. This 72-year-old patient who is a physician, used Voltaren Schmerzgel (diclofenac diethylammonium) beginning of Jun 2009 on fossa poplitea for knee joint distortion and developed blister on right hand which was used for application and in fossa poplitea (application site) on the third day of treatment. The patient also developed dry eczema on right hand and to a lower intensity also on left hand. He experienced also weeping eczema on left leg with skin desquamation and severe itching on third day of topical application of Voltaren Schmerzgel. Voltaren Schmerzgel was stopped. Following drug treatment with Soventol (bamipine hydrochloride) on the dry eczema of hands and antiseptic Bepanthen on weeping eczema on knee, the reactions improved only slowly. No itching any more on 10 Jun 2009. Outcome: improvement.

Follow-up information received on 22 Jun 2009 from physician (consumer) which upgraded to case to medically significant: the reporting physician, who suffered from psoriasis, specified that he developed blister on right hand and ulceration on the left knee in fossa poplitea of left knee after topical use of Voltaren on 23 May 2009. The following days, he developed epitheliolysis on right hand and in fossa poplitea of left knee. Due to sensitive skin in fossa poplitea of left knee, he developed painful ulceration on skin in fossa poplitea. Following drug treatment with Sofratull and Bepanthen ointment, the reactions were reported as improving. At the time of the report, residual skin areas with epitheliolysis, extremely dry skin on right hand as well as large area phlegmonous skin redness of fossa poplitea of left knee with severe pain were still present. Final outcome was condition improving.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard). The events Stevens-Johnson syndrome and Skin ulcer with related symptoms were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The rest of the event was assessed as non-serious.

Reporter's comment:NOVARTIS, 29.06.2009 DE-002147023-CHNY2009DE02369

Sender diagnosis: not reported

Narrative

This physician report combines initial information received on 08 Jun 2009 and follow-ups received on 09 Jun 2009 and 10 Jun 2009. This 72-year-old patient who is a physician, used Voltaren Schmerzgel (diclofenac diethylammonium) beginning of Jun 2009 on fossa poplitea for knee joint distortion and developed blister on right hand which was used for application and in fossa poplitea (application site) on the third day of treatment. The patient also developed dry eczema on right hand and to a lower intensity also on left hand. He experienced also weeping eczema on left leg with skin desquamation and severe itching on third day of topical application of Voltaren Schmerzgel. Voltaren Schmerzgel was stopped. Following drug treatment with Soventol (bamipine hydrochloride) on the dry eczema of hands and antiseptic Bepanthen on weeping eczema on knee, the reactions improved only slowly. No itching any more on 10 Jun 2009. Outcome: improvement.

Follow-up information received on 22 Jun 2009 from physician (consumer) which upgraded to case to medically significant: the reporting physician, who suffered from psoriasis, specified that he developed blister on right hand and ulceration on the left knee in fossa poplitea of left knee after topical use of Voltaren on 23 May 2009. The following days, he developed epitheliolysis on right hand and in fossa poplitea of left knee. Due to sensitive skin in fossa poplitea of left knee, he developed painful ulceration on skin in fossa poplitea. Following drug treatment with Sofratull and Bepanthen ointment, the reactions were reported as improving. At the time of the report, residual skin areas with epitheliolysis, extremely dry skin on right hand as well as large area phlegmonous skin redness of fossa poplitea of left knee with severe pain were still present. Final outcome was condition improving.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard). The events Stevens-Johnson syndrome and Skin ulcer with related symptoms were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The rest of the event was assessed as non-serious.

Reporter's comment:

NOVARTIS, 29.07.2009 DE-002147023-CHNY2009DE02369

Sender diagnosis: not reported

Narrative

This physician report combines initial information received on 08 Jun 2009 and follow-ups received on 09 Jun 2009 and 10 Jun 2009. This 72-year-old patient who is a physician, used Voltaren Schmerzgel (diclofenac diethylammonium) beginning of Jun 2009 on fossa poplitea for knee joint distortion and developed blister on right hand which was used for application and in fossa poplitea (application site) on the third day of treatment. The patient also developed dry eczema on right hand and to a lower intensity also on left hand. He experienced also weeping eczema on left leg with skin desquamation and severe itching on third day of topical application of Voltaren Schmerzgel. Voltaren Schmerzgel was stopped. Following drug treatment with Soventol (bamipine hydrochloride) on the dry eczema of hands and antiseptic Bepanthen on weeping eczema on knee, the reactions improved only slowly. No itching any more on 10 Jun 2009. Outcome: improvement.

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Upon internal review done on 22 Jul 2009, it was noticed that the correct initial receipt date of the initial report was reported as 05 Jun 2009 instead of 08 Jun 2009. Initial receipt date was corrected.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard). The events Stevens-Johnson syndrome and Skin ulcer with related symptoms were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The rest of the events was assessed as non-serious.

Reporter's comment:

NOVARTIS, 29.07.2009 DE-NOVNCHHQ-CHNY2009DE02369

Sender diagnosis: not reported

Narrative

This physician report combines initial information received on 08 Jun 2009 and follow-ups received on 09 Jun 2009 and 10 Jun 2009. This 72-year-old patient who is a physician, used Voltaren Schmerzgel (diclofenac diethylammonium) beginning of Jun 2009 on fossa poplitea for knee joint distortion and developed blister on right hand which was used for application and in fossa poplitea (application site) on the third day of treatment. The patient also developed dry eczema on right hand and to a lower intensity also on left hand. He experienced also weeping eczema on left leg with skin desquamation and severe itching on third day of topical application of Voltaren Schmerzgel. Voltaren Schmerzgel was stopped. Following drug treatment with Soventol (bamipine hydrochloride) on the dry eczema of hands and antiseptic Bepanthen on weeping eczema on knee, the reactions improved only slowly. No itching any more on 10 Jun 2009. Outcome: improvement.

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Upon internal review done on 22 Jul 2009, it was noticed that the correct initial receipt date of the initial report was reported as 05 Jun 2009 instead of 08 Jun 2009. Initial receipt date was corrected.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard). The events Stevens-Johnson syndrome and Skin ulcer with related symptoms were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The rest of the events was assessed as non-serious.

Reporter's comment:

NOVARTIS, 29.09.2009 DE-002147023-CHNY2009DE02369

Sender diagnosis: not reported

Narrative

This physician report combines initial information received on 08 Jun 2009 and follow-ups received on 09 Jun 2009 and 10 Jun 2009. This 72-year-old patient who is a physician, used Voltaren Schmerzgel (diclofenac diethylammonium) beginning of Jun 2009 on fossa poplitea for knee joint distortion and developed blister on right hand which was used for application and in fossa poplitea (application site) on the third day of treatment. The patient also developed dry eczema on right hand and to a lower intensity also on left hand. He experienced also weeping eczema on left leg with skin desquamation and severe itching on third day of topical application of Voltaren Schmerzgel. Voltaren Schmerzgel was stopped. Following drug treatment with Soventol (bamipine hydrochloride) on the dry eczema of hands and antiseptic Bepanthen on weeping eczema on knee, the reactions improved only slowly. No itching any more on 10 Jun 2009. Outcome: improvement.

Follow-up information received on 22 Jun 2009 from physician (consumer) which upgraded to case to medically significant: the reporting physician, who suffered from psoriasis, specified that he developed blister on right hand and ulceration on the left knee in fossa poplitea of left knee after topical use of Voltaren on 23 May 2009. The following days, he developed epitheliolysis on right hand and in fossa poplitea of left knee. Due to sensitive skin in fossa poplitea of left knee, he developed painful ulceration on skin in fossa poplitea. Following drug treatment with Sofratull and Bepanthen ointment, the reactions were reported as improving. At the time of the report, residual skin areas with epitheliolysis, extremely dry skin on right hand as well as large area phlegmonous skin redness of fossa poplitea of left knee with severe pain were still present. Final outcome was condition improving.

Upon internal review done on 22 Jul 2009, it was noticed that the correct initial receipt date of the initial report was reported as 05 Jun 2009 instead of 08 Jun 2009. Initial receipt date was corrected.

Follow-up received on 23 Sep 2009 from physician (consumer). The patient stated that "the ulcerations on skin of fossa poplitea were closed since approximately 1 month". According to the reporting physician (consumer), there was no sufficient data to provide a causality assessment. Outcome: complete recovery.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard). The events Stevens-Johnson syndrome and Skin ulcer with related symptoms were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The rest of the events was assessed as non-serious.

Reporter's comment:

NOVARTIS, 30.09.2009 DE-NOVNCHHQ-CHNY2009DE02369

Sender diagnosis: not reported

Narrative

This physician report combines initial information received on 08 Jun 2009 and follow-ups received on 09 Jun 2009 and 10 Jun 2009. This 72-year-old

patient who is a physician, used Voltaren Schmerzgel (diclofenac diethylammonium) beginning of Jun 2009 on fossa poplitea for knee joint distortion and developed blister on right hand which was used for application and in fossa poplitea (application site) on the third day of treatment. The patient also developed dry eczema on right hand and to a lower intensity also on left hand. He experienced also weeping eczema on left leg with skin desquamation and severe itching on third day of topical application of Voltaren Schmerzgel. Voltaren Schmerzgel was stopped. Following drug treatment with Soventol (bambipine hydrochloride) on the dry eczema of hands and antiseptic Bepanthen on weeping eczema on knee, the reactions improved only slowly. No itching any more on 10 Jun 2009. Outcome: improvement.

Follow-up information received on 22 Jun 2009 from physician (consumer) which upgraded to case to medically significant: the reporting physician, who suffered from psoriasis, specified that he developed blister on right hand and ulceration on the left knee in fossa poplitea of left knee after topical use of Voltaren on 23 May 2009. The following days, he developed epitheliolysis on right hand and in fossa poplitea of left knee. Due to sensitive skin in fossa poplitea of left knee, he developed painful ulceration on skin in fossa poplitea. Following drug treatment with Sofratull and Bepanthen ointment, the reactions were reported as improving. At the time of the report, residual skin areas with epitheliolysis, extremely dry skin on right hand as well as large area phlegmonous skin redness of fossa poplitea of left knee with severe pain were still present. Final outcome was condition improving.

Upon internal review done on 22 Jul 2009, it was noticed that the correct initial receipt date of the initial report was reported as 05 Jun 2009 instead of 08 Jun 2009. Initial receipt date was corrected.

Follow-up received on 23 Sep 2009 from physician (consumer). The patient stated that "the ulcerations on skin of fossa poplitea were closed since approximately 1 month". According to the reporting physician (consumer), there was no sufficient data to provide a causality assessment. Outcome: complete recovery.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard). The events Stevens-Johnson syndrome and Skin ulcer with related symptoms were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The rest of the events was assessed as non-serious.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-09108231

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 24 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 C-reactive protein increased	200909				Unknown
2 Fuchs' syndrome	200909				Unknown
3 Stevens Johnson syndrome					Unknown
4 Leukocytosis	200909				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 acetylsalicylic acid	Oral	/	-	Common cold
acetylsalicylic acid			-	
2 Amoxicillin	Oral	3000 Mg milligram(s)/Day	-	Pharyngitis
amoxicillin			-	
3 diclofenac	Oral	150 Mg milligram(s)/Day	-	Pharyngitis
diclofenac			-	
4 ibuprofen	Oral	/	-	Common cold
ibuprofen			-	
5 ibuprofen	Unknown	/	-	Fever
ibuprofen			-	
6 Paracetamol	Oral	/	-	Common cold
paracetamol			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Apathy			Unknown	
Fever sore			Unknown	
Common cold			Yes	With odynophagy, rhinitis, general apathy and fever
Odynophagia			Unknown	
Rhinitis			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Jakob M, Stuhmann N, Jordan J, Bootz F, Schröck A. [Stomatitis, cheilitis and conjunctivitis after a common cold.] HNO 2009; doi 10.1007/s00106-009-1931-2.	
Qualification:	Other Health Professional	observestudytype:
Literature:	Jakob M., Stuhmann N., Jordan J., Bootz F., Schröck A. Stomatitis, cheilitis and conjunctivitis after a common cold. HNO 2009 , 57:914-917	

Narratives (nur bei elektronischem Eingang)

JUTA, 18.08.2009 DE-JUTA-1515-09

Sender diagnosis: not reported

Narrative

A 24-year-old female patient developed Stevens Johnson syndrome (Fuchs' syndrome) during treatment with amoxicillin and diclofenac and after treatment with paracetamol, ibuprofen and acetylsalicylic acid. The patient complained of odynophagy, rhinitis, general apathy and fever which had persisted for 5 days. After initial treatment with paracetamol, ibuprofen and acetylsalicylic acid without improvement amoxicillin and diclofenac were prescribed. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered

alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs.

Sender's comment:

Allergic skin reactions are expected adverse drug reactions for amoxicillin. The comedication and the basic disease are alternative explanations. No further measures are considered to be necessary.

Reporter's comment:

Riemser Arzneimittel AG, 24.11.2009 DE-RIEMSER-20090338

Sender diagnosis: not reported

Narrative

This spontaneous literature case concerned a previous healthy female who experienced Fuchs syndrome while receiving paracetamol, ibuprofen and acetylsalicylic acid for a common cold and amoxicillin and diclofenac for acute pharyngitis. The medical history of the patient was unremarkable especially for allergic skin reactions.

On an unspecified date, the patient suffered from a common cold with odynophagia, rhinitis, fatigue and pyrexia and started treatment with paracetamol, ibuprofen and acetylsalicylic acid without medical advice. Three days later, there was no improvement of her symptoms and she consulted an otorhinolaryngologist. The diagnosis of an acute pharyngitis was made and treatment with amoxicillin (1000 mg tid) and diclofenac (50 mg tid) was initiated. During the following two days, the patient suffered from a painless redness of her eyes and from a progression of odynophagia. Thus, she was referred to the Department of Otorhinolaryngology of the University Hospital Bonn, Germany. On admission, the patient presented in a severely reduced general condition with pyrexia. Nutrition and fluid intake was not possible due to excessive odynophagia. There were pronounced erosive, reddened, edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. The lips were red with hemorrhagic scabs. In addition, a conjunctivitis was noticed. Laboratory parameters were remarkable for leukocytosis and elevated CRP. The diagnosis of Fuchs syndrome, a special form of Stevens-Johnson syndrome, was made and the patient was treated with systemic corticosteroids (100 mg daily). Furthermore, antiseptic and anaesthetic mouth rinsings as well as dexpantenol-containing ointments were prescribed. It is obvious, that all drugs were discontinued. During her hospital stay, the patient recovered completely and was discharged three weeks after admission.

Follow up information was received and the following data were added to the case: clinical course and outcome of the patient; causality assessment between the reported event and the used drugs; medical history of the patient.

Sender's comment:

The RIEMSER Arzneimittel AG holds the marketing authorization for paracetamol-containing preparations (e.g. Neopyrin forte) and for amoxicillin-containing preparations (e.g. AmoxiHefa 1000 mg). Because there was no information about the tradenames of the used paracetamol and amoxicillin in this case, it cannot be excluded, that RIEMSER products were administered. Fuchs syndrome is not listed for Neopyrin forte and AmoxiHefa. However, hypersensitivity reactions are listed for Neopyrin forte and for AmoxiHefa 1000 mg, severe allergic skin reactions are listed. The causal relationship between the reported event and paracetamol and amoxicillin was assessed as possible by the company. Alternative explanation is the concomitant medication of ibuprofen, acetylsalicylic acid and diclofenac. Furthermore, the underlying common cold might be a contributing factor for the Fuchs syndrome. The seriousness of this case is assessed as serious because a hospitalization was required. Additionally, the case was classified as medically significant by the company. Based upon this single case, the overall safety profile of the RIEMSER paracetamol- and amoxicillin-containing preparations has not been changed.

Reporter's comment:

The reporting physician assessed, that each of the used drugs might be the causative agent for the occurred Fuchs syndrome in this case.

AXCOUNT, 22.12.2009 DE-AXCOUNT-00019

Sender diagnosis: not reported

Narrative

A 24-year-old patient complained of odynophagy, rhinitis, general apathy and fever (38,5 Â°C) which had persisted for 5 days. Treatment had initially

been a combination of paracetamol, ibuprofen and acetylsalicylic acid and when no improvement occurred, this medication was stopped after 3 days and amoxicillin and diclofenac were prescribed for sore throat. Following, odynophagy was exacerbated and nutrition and fluid intake was not possible. In addition to leukocytosis and increased C-reactive protein, there were oedematous alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The diagnosis was Fuchs' syndrome. The reaction outcome was not mentioned.

Sender's comment:

This case was classified serious because to its medical importance (since the nutrition and fluid intake were not possible due to excessive odynophagy in this case). Regarding the information on this case, it can not be excluded that Amoxicillin account 1000 mg, Tabletten was involved in the adverse reaction because this case occurred in Germany. Fuchs' syndrome is not covered by the current SPC and will be closely monitored by the MAH in the future.

Reporter's comment:

The reporters stated that Fuchs' syndrome was medication induced. The exact role of amoxicillin in the development of Fuchs' syndrome is not nearly posed.

Bausch & Lomb Incorporated, 05.01.2010 DE-BAUSCH-2009BL006723

Sender diagnosis: not reported

Narrative

A report was identified in a literature search indicating that a 24-year-old female patient complained of odynophagy, rhinitis, general apathy and fever (38.5 degrees C) which had persisted for 5 days. Treatment had initially been a combination of PARACETAMOL, IBUPROFEN and ACETYLSALICYLIC ACID (self-medication, manufacturers and doses unknown) and when no improvement occurred on the third day the medication was changed to 1000 mg AMOXICILLIN three times per day and 50 mg DICLOFENAC three times per day by an ENT doctor who made the diagnosis of angina of the salpingopalatine fold. Nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis (LEUCOCYTOSIS) (13.12 g/l) and increased C-reactive protein (C-REACTIVE PROTEIN INCREASED) (193 mg/l), there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs' syndrome (FUCHS' SYNDROME) which is a severe medication-induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy are decisive. The patient received cortisone therapy of 100 mg per day. The erosive alterations of oral mucosa were treated with antiseptic, anaesthetizing and caring mouth irrigations, the lip alterations with dexpanthenol containing ointments. The outcome was not reported. No further information is available.

Source:

JAKOB M, STUHRMANN N, JORDAN J, BOOTZ F, SCHROECK A. STOMATITIS, CHEILITIS AND CONJUNCTIVITIS AFTER A COMMON COLD. HNO 2009 SEP; 57(9): 914-7.

Sender's comment:

A literature report was received indicating a serious unexpected case of FUCHS' SYNDROME in connection with the use of several drugs including paracetamol (manufacturer unknown). As Bausch and Lomb also have a marketing authorisation for paracetamol (Vivimed N gegen Fieber und Kopfschmerzen) this case is reported.

This case was assessed as serious (medical significant), unexpected and possibly related as a causal relationship between the application of paracetamol and the occurrence of this adverse drug reaction cannot be excluded. FUCHS' SYNDROME is a sub-type of the Stevens Johnsons Syndrome with a very rare frequency of about 1 out of one million patients. The Fuchs syndrome like Stevens Johnsons Syndrome may be caused by different drugs including antibiotics and NSAIDs. In this case report the patient had used several drugs that are suspected to cause this type of adverse reaction like penicilline, and several NSAIDs (ibuprofen, ASS, diclofenac, paracetamol). Therefore the contribution of paracetamol to this adverse reaction remains unclear but cannot be ruled out. In the SmPC of our paracetamol-containing product a potential of generalised hypersensitivity reactions after the use of paracetamol is already described. However this special drug reaction is not mentioned, therefore this case is assessed unexpected.

As this report is an isolated case no further measures for risk mitigation are considered necessary.

Reporter's comment:

Dr. August Wolff Arzneimittel, 20.01.2010 DE-DRWOLFF-2010-050

Sender diagnosis: not reported

Narrative

A case of Fuchs syndrome with stomatitis, cheilitis and conjunctivitis was reported in a 24-year-old female patient. The patient had a past medical history of common cold with odynophagy, rhinitis, general apathy and fever. Five days before presentation, she had taken paracetamol, ibuprofen and acetylsalicylic acid for treatment. As her symptoms did not improve, amoxicillin (3x1000mg/d) and diclofenac (3x50 mg/d) were prescribed by her treating otorhinolaryngologist based on a diagnosis of acute pharyngitis two days before presentation. The patient presented to the hospital with stomatitis, cheilitis and conjunctivitis. Physical examination revealed pronounced erosive, reddened oedematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. Relevant laboratory test results included leucocytes 13.12 G/L and CRP 193 mg/L. The patient was diagnosed with Fuchs syndrome. No further case details were reported.

Sender's comment:

This adverse reaction is unexpected for Amoxi-Wolff products. Fuchs syndrome is a subtype of Stevens-Johnson-Syndrome. However, as this term (SJS) is not sufficiently reflected in the current SPCs of Amoxi-Wolff products, the company will revise the wording.

Reporter's comment:

Dr. August Wolff Arzneimittel, 20.01.2010 DE-DRWOLFF-2010-050

Sender diagnosis: not reported

Narrative

A case of Fuchs syndrome with stomatitis, cheilitis and conjunctivitis was reported in a 24-year-old female patient. The patient had a past medical history of common cold with odynophagy, rhinitis, general apathy and fever. Five days before presentation, she had taken paracetamol, ibuprofen and acetylsalicylic acid for treatment. As her symptoms did not improve, amoxicillin (3x1000mg/d) and diclofenac (3x50 mg/d) were prescribed by her treating otorhinolaryngologist based on a diagnosis of acute pharyngitis two days before presentation. The patient presented to the hospital with stomatitis, cheilitis and conjunctivitis. Physical examination revealed pronounced erosive, reddened oedematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. Relevant laboratory test results included leucocytes 13.12 G/L and CRP 193 mg/L. The patient was diagnosed with Fuchs syndrome. No further case details were reported.

Sender's comment:

This adverse reaction is unexpected for Amoxi-Wolff products. Fuchs syndrome is a subtype of Stevens-Johnson-Syndrome. However, as this term (SJS) is not sufficiently reflected in the current SPCs of Amoxi-Wolff products, the company will revise the wording.

Reporter's comment:

Dr. August Wolff Arzneimittel, 21.01.2010 DE-DRWOLFF-2010-050

Sender diagnosis: not reported

Narrative

A case of Fuchs syndrome with stomatitis, cheilitis and conjunctivitis was reported in a 24-year-old female patient. The patient had a past medical history of common cold with odynophagy, rhinitis, general apathy and fever. Five days before presentation, she had taken paracetamol, ibuprofen and acetylsalicylic acid for treatment. As her symptoms did not improve, amoxicillin (3x1000mg/d) and diclofenac (3x50 mg/d) were prescribed by her treating otorhinolaryngologist based on a diagnosis of acute pharyngitis two days before presentation. The patient presented to the hospital with stomatitis, cheilitis and conjunctivitis. Physical examination revealed pronounced erosive, reddened oedematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes. Relevant laboratory test results included leucocytes 13.12 G/L and CRP 193 mg/L. The

patient was diagnosed with Fuchs syndrome. No further case details were reported.

Sender's comment:

This adverse reaction is unexpected for Amoxi-Wolff products. Fuchs syndrome is a subtype of Stevens-Johnson-Syndrome. However, as this term (SJS) is not sufficiently reflected in the current SPCs of Amoxi-Wolff products, the company will revise the wording.

Reporter's comment:

mibe GmbH Arzneimittel, 08.03.2010 DE-MIBE-N02BA01-01032010-A

Sender diagnosis: not reported

Narrative

A 24-year-old female patient complained of odynophagy, rhinitis, general apathy and fever (38.5 degrees C) which had persisted for 5 days. Treatment had initially been a combination of paracetamol, ibuprofen and acetylsalicylic acid and when no improvement occurred amoxicillin was prescribed. Nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs' syndrome which is a severe medication-induced skin

Sender's comment:

mibe comment:

Fuchs syndrome describes a variant of erythema multiforme majus which mainly involves the mucosal surfaces. Severe skin reactions such as Stevens-Johnson syndrome are known side effects on acetylsalicylic acid and ibuprofen and listed in the current SPC of Temagin ASS, Jenaprofen, Ibu-MTT and Fibralflex. Only allergic skin reactions with rash are listed for paracetamol. This case is assessed as serious and the causal relationship as possible for several suspected drugs. No safety concern is elicited from this case.

Reporter's comment:

MIP International Pharma Research GmbH, 31.05.2011 DE-MIP-hamxpo090111-hdcopo090111

Sender diagnosis: not reported

Narrative

A 24-year-old female patient complained of odynophagy, rhinitis, general apathy and fever (38.5 °C) which had persisted for 5 days. Treatment had initially been a combination of paracetamol, ibuprofen and acetylsalicylic acid and when no improvement occurred amoxicillin and diclofenac were prescribed. Nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs' syndrome which is a severe medication-induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy are decisive.

Sender's comment:

The case was detected in the MEDLINE database on 18th May 2011. In the case, no product name or MAH could be identified. The case could theoretically be in connection with amoxicillin- or diclofenac-containing products of the MAH. Therefore, the case was recorded for both active substances in the database of the MAH as case hamxpo090111 and case hdcopo090111. The diagnosed Fuchs' syndrome is a special form of

Stevens-Johnson-Syndrom concerning the eye and oral mucosa. According to the treating physician, the Fuchs' Syndrom belongs to serious drug-induced skin reactions. Skin and subcutaneous disorders like exanthema, rash and allergic reactions are known ADRs of both drugs. The cases were classified as labelled for diclofenac and as unlabelled for amoxicillin. The case was classified as serious due to the medically important condition. Based on the available data, the MAH evaluated the causality as possible for both drugs. The reported case does not change the risk/benefit assessment for Amoxi-saar 500/750/1000 mg Tabletten as well as Diclo-saar 50/100 mg Tabletten. Stevens-Johnson Syndrome is also labelled for the co-medication ASS and Ibuprofen.

Reporter's comment:

McNeil Germany, Local Safety Officers, 17.04.2013 DE-JNJFOC-20130407230

Sender diagnosis: not reported

Narrative

Report was received from literature.

Jakob M, Stuhmann N, Jordan J, Bootz F, Schrock A. Stomatitis, cheilitis and conjunctivitis after a common cold. Hno 2009; 57(9):914-917

This report concerns a 24-year-old female patient. The patient's height, weight and medical history were not reported. The patient had complained of odynophagy, rhinitis, general apathy and fever (38.5degreeC) which had persisted for 5 days. The patient was treated with the combination of Ibuprofen (unspecified formulation and route, batch unspecified) dose and frequency unspecified and non-company suspect drug(s) paracetamol (unspecified formulation and route, batch unspecified) dose and frequency unspecified and acetylsalicylic acid (unspecified formulation and route, batch unspecified) dose and frequency unspecified and when no improvement occurred amoxicillin (unspecified formulation and route, batch unspecified) dose and frequency unspecified was prescribed for odynophagy, rhinitis, general apathy and fever. The patient's nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis. The lips were red with hemorrhagic scabs. The diagnosis was Fuchs syndrome which was a severe medication induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy were decisive. Action taken with ibuprofen, paracetamol, acetylsalicylic acid and amoxicillin were not reported. The patient's outcome was not reported for Fuchs syndrome at the time of this report.

This report is serious (medically significant).<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Reporter's comment:

PFIZERINC, 23.04.2013 DE-PFIZER INC-2013120405

Sender diagnosis: not reported

Narrative

This is a literature report from the Hno 2009; 57(9):914-917, entitled "Stomatitis, cheilitis and conjunctivitis after a common cold", based on information received by Pfizer from Johnson and Johnson (manufacturer control number DE-JNJFOC-20130407230), license party for ibuprofen.

Report was received from literature. Jakob M, Stuhmann N, Jordan J, Bootz F, Schrock A. Stomatitis, cheilitis and conjunctivitis after a common cold. Hno 2009; 57(9):914-917. This report concerns a 24-year-old female patient. The patient's height, weight and medical history were not reported. The patient had complained of odynophagy, rhinitis, general apathy and fever (38.5degreeC) which had persisted for 5 days. The patient was treated with the combination of Ibuprofen (unspecified formulation and route, batch unspecified) dose and frequency unspecified and non-company suspect drug (s) paracetamol (unspecified formulation and route, batch unspecified) dose and frequency unspecified and acetylsalicylic acid (unspecified formulation and route, batch unspecified) dose and frequency unspecified and when no improvement occurred amoxicillin (unspecified formulation and route, batch unspecified) dose and frequency unspecified was prescribed for odynophagy, rhinitis, general apathy and fever. The patient's nutrition and fluid intake was not possible due to excessive odynophagy. In addition to leukocytosis and increased C-reactive protein, there were pronounced erosive, reddened edematous and fibrin-covered alterations of the whole oral, pharyngeal and laryngeal mucous membranes as well as conjunctivitis.

The lips were red with hemorrhagic scabs. The diagnosis was Fuchs syndrome which was a severe medication induced skin reaction. Abstinence from the agent (medication) responsible and early cortisone therapy were decisive. Action taken with ibuprofen, paracetamol, acetylsalicylic acid and amoxicillin were not reported. The patient's outcome was not reported for Fuchs syndrome at the time of this report. This report is serious (medically significant).

Causality Assessment per reporter for the event is "Possible".

Johnson and Johnson Causality assessment: the event of Fuchs syndrome was considered possibly related to ibuprofen.

Pfizer is Marketing Authorization Holder of ibuprofen in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of ibuprofen has submitted the same report to the regulatory authorities.

Sender's comment:

Based on the information provided, a possible contributory role of the suspect drug to the reported event Fuch's syndrome cannot be excluded. The impact of this report on the benefit/risk profile of the Pfizer product is evaluated as part of Pfizer procedures for safety evaluation, including the review and analysis of aggregate data for adverse events. Any safety concern identified as part of this review, as well as any appropriate action in response, will be promptly notified to Regulatory Authorities, Ethics Committees and Investigators, as appropriate.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09122238

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Contact eczema	200908				recovering/resolving
2 Bullous eruption	200908				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN EMULGEL diclofenac diethylamine	Topical	/	200908-200908	Sunburn
			-	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 16.09.2009 DE-002147023-CHNY2009DE03456

Sender diagnosis: not reported

Narrative

This physician (dermatologist) report combines initial information and follow-up report both received on 07 Sep 2009. This female patient born in 1973 used Voltaren Emulgel (diclofenac diethylammonium salt) in Aug 2009 to treat sunburn on ankle (off label use). After several days, she experienced severe bullous contact eczema with generalization to the complete integument. The patient was hospitalized on 15 Aug 2009. She received topical treatment with antiseptics and steroids, and systemic treatment with antihistaminics and antibiotics. On 22 Aug 2009, she was discharged from hospital in a significantly improved condition. Voltaren Emulgel treatment was discontinued in Aug 2009. Final outcome was condition improving.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Hospitalization). The event Dermatitis contact and related symptom were already labelled in the product information (any type of dermatitis and dermatitis bullous).

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09122742

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Contact eczema	200908				recovering/resolving
2 Bullous eruption	200908				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/	200908-200908 -	Sunburn

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 16.09.2009 DE-NOVNCHHQ-CHNY2009DE03456

Sender diagnosis: not reported

Narrative

This physician (dermatologist) report combines initial information and follow-up report both received on 07 Sep 2009. This female patient born in 1973 used Voltaren Emulgel (diclofenac diethylammonium salt) in Aug 2009 to treat sunburn on ankle (off label use). After several days, she experienced severe bullous contact eczema with generalization to the complete integument. The patient was hospitalized on 15 Aug 2009. She received topical treatment with antiseptics and steroids, and systemic treatment with antihistaminics and antibiotics. On 22 Aug 2009, she was discharged from hospital in a significantly improved condition. Voltaren Emulgel treatment was discontinued in Aug 2009. Final outcome was condition improving.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Hospitalization). The event Dermatitis contact and related symptom were already labelled in the product information (any type of dermatitis and dermatitis bullous).

Reporter's comment:

case: DE-BFARM-09125670

Patient

Reactions

Drugs (suspected or interacting)

Primary source

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09132458

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Itching	20090905			1 Day	recovered/resolved
2 Chemical burn					recovered/resolved
3 Blisters	200909				recovered/resolved
4 Skin red					recovered/resolved
5 Skin inflammation					recovered/resolved
6 Wound oozing	200909				recovered/resolved
7 Open wound	200909				recovered/resolved
8 Skin swelling	200909				recovered/resolved
9 Generalized rash					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN SCHMERZGEL diclofenac diethylamine	Topical	1 Df dosage form/Day 95	20090905-20090908 4 Day	Ankle swelling

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 02.10.2009 DE-NOVNCHHQ-CHNY2009DE03501

Sender diagnosis: not reported

Narrative

This consumer report combines initial information and follow-up, both received on 25 Sep 2009. This male consumer with no known allergies, used Voltaren Schmerzgel (diclofenac diethylammonium salt) for the very first time to treat swelling of the lower ankle (off label use) on 2 consecutive days in Sep 2009 during sun exposure (at the beach) without experiencing any adverse event. The third application of Voltaren Schmerzgel was again in Sep 2009 overnight. The application site was capped by a semi-occlusive wound dressing (not specified, maybe Iroxol) and additionally covered by a dressing. The patient developed severe swelling of the skin (looked like severe chemical burn) with severe itching. The fourth application was performed in the same way as the third one. The morning after the 4th application, the patient developed blisters (5-10 cm in length) which were hard, like fins, which bursted open and released liquid. Voltaren Schmerzgel treatment was discontinued in Sep 2009. The patient washed the application site with warm water and treated the reactions with Kamillosan and by rubbing alcohol. Final outcome was condition improving. Follow-up was ongoing.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant) not confirmed by a health professional. The events Skin swelling, Pruritus, and Blisters and related symptoms were assessed as unlabelled according to the product information. The information provided in this individual case does not warrant a change to the product information text.

Reporter's comment:

NOVARTIS, 05.10.2009 DE-002147023-CHNY2009DE03501

Sender diagnosis: not reported

Narrative

This consumer report combines initial information and follow-up, both received on 25 Sep 2009. This male consumer with no known allergies, used Voltaren Schmerzgel (diclofenac diethylammonium salt) for the very first time to treat swelling of the lower ankle (off label use) on 2 consecutive days in Sep 2009 during sun exposure (at the beach) without experiencing any adverse event. The third application of Voltaren Schmerzgel was again in Sep 2009 overnight. The application site was capped by a semi-occlusive wound dressing (not specified, maybe Iroxol) and additionally covered by a dressing. The patient developed severe swelling of the skin (looked like severe chemical burn) with severe itching. The fourth application was performed in the same way as the third one. The morning after the 4th application, the patient developed blisters (5-10 cm in length) which were hard, like fins, which bursted open and released liquid. Voltaren Schmerzgel treatment was discontinued in Sep 2009. The patient washed the application site with warm water and treated the reactions with Kamillosan and by rubbing alcohol. Final outcome was condition improving. Follow-up was ongoing.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant) not confirmed by a health professional. The events Skin swelling, Pruritus, and Blisters and related symptoms were assessed as unlabelled according to the product information. The information provided in this individual case does not warrant a change to the product information text.

Reporter's comment:

NOVARTIS, 22.10.2009 DE-002147023-CHNY2009DE03501

Sender diagnosis: not reported

Narrative

This consumer report combines initial information and follow-up, both received on 25 Sep 2009. This male consumer with no known allergies, used Voltaren Schmerzgel (diclofenac diethylammonium salt) for the very first time to treat swelling of the lower ankle (off label use) on 2 consecutive days in Sep 2009 during sun exposure (at the beach) without experiencing any adverse event. The third application of Voltaren Schmerzgel was again in Sep 2009 overnight. The application site was capped by a semi-occlusive wound dressing (not specified, maybe Iroxol) and additionally covered by a dressing. The patient developed severe swelling of the skin (looked like severe chemical burn) with severe itching. The fourth application was performed in the same way as the third one. The morning after the 4th application, the patient developed blisters (5-10 cm in length) which were hard, like fins, which bursted open and released liquid. Voltaren Schmerzgel treatment was discontinued in Sep 2009. The patient washed the application site with warm water and treated the reactions with Kamillosan and by rubbing alcohol. Final outcome was condition improving. Follow-up was ongoing.

Follow-up report received from consumer on 13 Oct 2009. Voltaren Schmerzgel was started to be applied on 05 Sep 2009 and covered with a sterile compress in the night. On the same day, he experienced severe itching. After an unspecified period of time, he noticed acid burn with skin inflammation and blisters nearer described as acid burn on approximately 5 cm x 10 cm skin area with severe blisters, severe weeping, severe itching, severe skin redness and severely inflamed tissue. Voltaren Schmerzgel was discontinued on 08 Sep 2009. Following drug treatment with Betagalen cream / Galen (betamethasone valerate), the reactions aggravated dramatically and he experienced skin reaction with blisters and skin rash on whole body. The feet were washed with washing soap (Kernseife) and the adverse symptoms lasted all together for approximately one month. Final outcome was complete recovery. The reporting consumer assessed the causality as not probable.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard) not confirmed by a health professional. The event Generalized rash was assessed as unlabelled according to the product information due to greater specificity/severity. The term Rash is already included in the product information. The events Caustic injury with related symptoms and Skin swelling were assessed as unlabelled according to the product information. The information provided in this individual case does not warrant a change to the product information text. The events Erythema and Dermatitis were already labelled in the product information.

Reporter's comment:

NOVARTIS, 22.10.2009 DE-NOVNCHHQ-CHNY2009DE03501

Sender diagnosis: not reported

Narrative

This consumer report combines initial information and follow-up, both received on 25 Sep 2009. This male consumer with no known allergies, used Voltaren Schmerzgel (diclofenac diethylammonium salt) for the very first time to treat swelling of the lower ankle (off label use) on 2 consecutive days in Sep 2009 during sun exposure (at the beach) without experiencing any adverse event. The third application of Voltaren Schmerzgel was again in Sep 2009 overnight. The application site was capped by a semi-occlusive wound dressing (not specified, maybe Iroxol) and additionally covered by a dressing. The patient developed severe swelling of the skin (looked like severe chemical burn) with severe itching. The fourth application was performed in the same way as the third one. The morning after the 4th application, the patient developed blisters (5-10 cm in length) which were hard, like fins, which bursted open and released liquid. Voltaren Schmerzgel treatment was discontinued in Sep 2009. The patient washed the application site with warm water and treated the reactions with Kamillosan and by rubbing alcohol. Final outcome was condition improving. Follow-up was ongoing.

Follow-up report received from consumer on 13 Oct 2009. Voltaren Schmerzgel was started to be applied on 05 Sep 2009 and covered with a sterile compress in the night. On the same day, he experienced severe itching. After an unspecified period of time, he noticed acid burn with skin inflammation and blisters nearer described as acid burn on approximately 5 cm x 10 cm skin area with severe blisters, severe weeping, severe itching, severe skin redness and severely inflamed tissue. Voltaren Schmerzgel was discontinued on 08 Sep 2009. Following drug treatment with Betagalen cream / Galen (betamethasone valerate), the reactions aggravated dramatically and he experienced skin reaction with blisters and skin rash on whole body. The feet were washed with washing soap (Kernseife) and the adverse symptoms lasted all together for approximately one month. Final outcome was complete recovery. The reporting consumer assessed the causality as not probable.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazard) not confirmed by a health professional. The event Generalized rash was assessed as unlabelled according to the product information due to greater specificity/severity. The term Rash is already included in the product information. The events Caustic injury with related symptoms and Skin swelling were assessed as unlabelled according to the product information. The information provided in this individual case does not warrant a change to the product information text. The events Erythema and Dermatitis were already labelled in the product information.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09158678

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 55 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin ulcer	200911				recovered/resolved
2 Open wound	20091111				recovered/resolved
3 Blister	20091111				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN SCHMERZGEL diclofenac diethylamine	Topical	/ 95	200910-20091111 -	Foot injury
2 VOLTAREN SCHMERZGEL diclofenac diethylamine		/ 95	- -	Pain in ankle

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 25.11.2009 DE-002147023-CHNY2009DE03597

Sender diagnosis: not reported

Narrative

This pharmacist report combines initial and follow-up information both received on 16 Nov 2009. A 55-year-old female patient started Voltaren Schmerzgel (diclofenac diethylammonium) in Oct 2009 to treat foot distortion with pain in ankles on right ankle for 2 weeks and subsequently on left ankle for 2 weeks. On 11 Nov 2009, she developed closed blisters which subsequently broken open and which formed large open blister/ulcera on 3 fingers on her left hand. On her right hand, which also came in contact with Voltaren Schmerzgel, the patient developed pinpoint large small vesicles which were closed at the date of the report. Voltaren was discontinued on 11 Nov 2009. Following drug treatment with unspecified wound covering plus wound treatment, adverse symptoms lasted all together for 5 days. The reporting pharmacist assessed the causality as possible. Final outcome was reported as complete recovery.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazards). The event Skin ulcer was assessed as unlabelled according to the product information. The information provided in this individual case does not warrant a change to the product information text. The event Blister with related symptoms was assessed as unlabelled according to the product information due to greater specificity. The term Dermatitis bullous is already included in the product information.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09159159

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 55 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blister	20091111				recovered/resolved
2 Skin ulcer	200911				recovered/resolved
3 Open wound	20091111				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/ 95	200910-20091111 -	Foot injury
2 voltaren schmerzgel diclofenac diethylamine		/ 95	- -	Pain in ankle

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

[NOVARTIS, 25.11.2009 DE-NOVNCHHQ-CHNY2009DE03597](#)

Sender diagnosis: not reported

Narrative

This pharmacist report combines initial and follow-up information both received on 16 Nov 2009. A 55-year-old female patient started Voltaren Schmerzgel (diclofenac diethylammonium) in Oct 2009 to treat foot distortion with pain in ankles on right ankle for 2 weeks and subsequently on left ankle for 2 weeks. On 11 Nov 2009, she developed closed blisters which subsequently broken open and which formed large open blister/ulcera on 3 fingers on her left hand. On her right hand, which also came in contact with Voltaren Schmerzgel, the patient developed pinpoint large small vesicles which were closed at the date of the report. Voltaren was discontinued on 11 Nov 2009. Following drug treatment with unspecified wound covering plus wound treatment, adverse symptoms lasted all together for 5 days. The reporting pharmacist assessed the causality as possible. Final outcome was reported as complete recovery.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant hazards). The event Skin ulcer was assessed as unlabelled according to the product information. The information provided in this individual case does not warrant a change to the product information text. The event Blister with related symptoms was assessed as unlabelled according to the product information due to greater specificity. The term Dermatitis bullous is already included in the product information.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-09167412

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 43 Year	weight (kg): 70	BMI_int: 24.8	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering	20091021			1 Day	Unknown
2 Enanthema	20091021			10 Day	Unknown
3 Erythema exudativum multiforme					Unknown
4 Eczema	20091012			1 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ALLOPURINOL		/	20091012-	
allopurinol			-	
2 DICLOFENAC		/	20091012-	
diclofenac			-	
3 NOVALGIN		/	20091012-	Ache
metamizole			-	
4 QUENSYL		500 Mg milligram(s)/Day	200908-	Lupus erythematosus
hydroxychloroquine sulfate			-	
5 TRAMAL		/	20091012-	Ache
tramadol hydrochloride			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Ache			Unknown	
Lupus erythematosus			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 21.12.2009 DE-002147023-PHHY2009DE55851

Sender diagnosis: not reported

Narrative

Initial physician report received from the Health Authority (DE-BFARM-09167412) on 14 Dec 2009: This patient was on treatment with diclofenac (manufacturer unknown) since 12 Oct 2009 for an unspecified indication. The patient was also on treatment with Allopurinol since 12 Oct 2009, Novalgin since 12 Oct 2009 for ache, Quensyl since Aug 2009 for lupus erythematosus and Tramal since 12 Oct 2009 for ache. These drugs were considered as co-suspect drugs. He experienced eczema on 12 Oct 2009 and enanthema on 21 Oct 2009. He also experienced blistering on 21 Oct 2009 and the blisters used to appear even when a gloved hand pressed down lightly (on the patient's skin). Skin alterations in agreement with erythema exudativum multiforme was observed. The events met the seriousness criteria of life threatening and the outcome of the events were not reported. The causality was reported as possible. No further information was provided.

Sender's comment:

Serious spontaneous report, erythema multiformae [life-threatening], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

sanofi-aventis, 29.12.2009 DE-SANOFI-AVENTIS-2009SA008680

Sender diagnosis: not reported

Narrative**Initial Report:**

This serious case was extracted from the website of the German health authority (authority number DE-BFARM-09167412), initially reported by an anonymized physician via the Drug Commission of the German Medical Association (case no.: DE-DCGMA-09151255) to the BfArM. The case involves a 43-year-old male patient. He was treated with hydroxychloroquine sulfate (Quensyl since Aug-2009) for lupus erythematosus, metamizole sodium (Novalgin since 12-Oct-2009) for ache, diclofenac sodium (Diclofenac since 12-Oct-2009), tramadol hydrochloride (since 12-Oct-2009) for ache and allopurinol (since 12-Oct-2009). Concomitant medications were not provided. On 12-Oct-2009 the patient developed rosette-shaped skin lesions (eczema). Nine days after this event he experienced blistering and enanthema. The action taken concerning the therapy and the outcome of the events were not reported. The Drug Commission assessed the causal relationship between the events and all suspected drugs as possible.

Sender's comment:

Sanofi-Aventis Company Comment dated 16-DEC-2009:

Based on the available information and temporal chronology a causal relationship between the suspected drugs and the reactions cannot be completely excluded. However, this case lacks further information such as detailed patient relevant medical history, relevant diagnosis, outcome, and action taken for all medications to make a proper medical assessment.

29-DEC-2009 Sanofi-Aventis Germany

All reported clinical diagnoses are considered locally labelled for all suspected products and thus consistent with the known benefit risk ratio of the product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10005201

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg): 67	BMI_int: 23.18	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pain					recovered/resolved
2 Therapeutic response decreased					Unknown
3 Blisters					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/ 95	20100104-20100107 4 Day	Bruise
2 voltaren schmerzgel diclofenac diethylamine		/ 95	-	Back pain

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 18.01.2010 DE-002147023-CHPA2010DE00833

Sender diagnosis: not reported

Narrative

This report combines initial information received on 07 Jan 2010, from a consumer via Novartis Pharma, and follow-up information received on 11 Jan 2010, from a pharmacist. A consumer reported, her father, used VOLTAREN SCHMERZGEL (diclofenac diethylammonium salt) to treat bruises, site unspecified, and complained that "the effect was very poor". Additionally, he developed "blistering at his fingers, with subsequent burst open of the skin and severe pain". The pharmacist added the patient used the product probably to treat back pain and developed the initially reported adverse events. It was added that the fingers were cleaned after each application and that the skin at the application site was not affected. Voltaren Schmerzgel was probably discontinued. The outcome of the events was not reported.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically Significant). The events Blister and Pain were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:NOVARTIS, 26.01.2010 DE-002147023-CHPA2010DE00833

Sender diagnosis: not reported

Narrative

This report combines initial information received on 07 Jan 2010, from a consumer via Novartis Pharma, and follow-up information received on 11 Jan

2010, from a pharmacist. A consumer reported, her father, used VOLTAREN SCHMERZGEL (diclofenac diethylammonium salt) to treat bruises, site unspecified, and complained that "the effect was very poor". Additionally, he developed "blistering at his fingers, with subsequent burst open of the skin and severe pain". The pharmacist added the patient used the product probably to treat back pain and developed the initially reported adverse events. It was added that the fingers were cleaned after each application and that the skin at the application site was not affected. Voltaren Schmerzgel was probably discontinued. The outcome of the events was not reported.

Additional information received on 18 Jan 2010. Updated patient data, reaction onset, details regarding the event, suspect drug information, measures taken, information related to suspected drug and adverse reaction and outcome. The patient's daughter reported this male patient used Voltaren Schmerzgel once daily, to treat bruises, starting treatment on 04 Jan 2010 and developed "blistering at both hands (area of the fingers) with burst open of the skin and severe pain, despite prompt cleaning of his hands after application". She stated that "the patient was badly impaired" and that "the skin at the application site NOS was not affected". Voltaren Schmerzgel was discontinued after 3 days, on 07 Jan 2010. The hands were treated with cortisone ointment NOS. It was stated that the adverse events persisted for several days. Causality assessment by reporting daughter of patient was "probable".

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically Significant). The events Blister and Pain were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10004730

Serious Yes	Death	Lifethr. No	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 60 Year	weight (kg): 125	BMI_int: 43.25	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Purpura	20091226		7 Day	2 Day	recovering/resolving
2 Generalised oedema	20091226		7 Day	2 Day	recovering/resolving
3 Blood creatinine increased	20091226		7 Day	2 Day	not recovered/not resolved
4 Stevens-Johnson syndrome	20091226		7 Day	2 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC diclofenac	Intramuscular	75 Mg milligram(s)/Day	20091225-20091226 2 Day	Back pain

Drugs (concomitant)

2 dexamethason dexamethasone	Intramuscular	/	20091225-20091226 2 Day	Back pain
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Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 27.01.2010 DE-002147023-PHHY2010DE02889

Sender diagnosis: not reported

Narrative

Initial report received from physician via Health Authority (reference number: DE-BFARM-10004730) on 18 Jan 2010: The patient was on treatment with diclofenac (manufacturer unknown) 75 mg per day intramuscular route for back pain from 25 Dec 2009. On 26 Dec 2009 the patient developed Stevens-Johnson Syndrome with purpura and generalized oedema, the patient also had increased blood creatinine. The therapy with diclofenac was discontinued on 26 Dec 2009. The outcome for purpura and generalized oedema was reported to be condition improving whereas, outcome for Stevens-Johnson Syndrome and blood creatinine increased was reported to be condition unchanged.

Sender's comment:

Serious spontaneous report, Stevens-Johnson syndrome and blood creatinine increased [medically significant], already listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10005572

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg): 67	BMI_int: 23.18	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blisters					recovered/resolved
2 Pain					recovered/resolved
3 Therapeutic response decreased					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine		/	-	Back pain
		95	-	
2 voltaren schmerzgel diclofenac diethylamine	Topical	/	20100104-20100107	Bruise
		95	4 Day	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

[NOVARTIS, 18.01.2010 DE-NOVNCHHQ-CHPA2010DE00833](#)

Sender diagnosis: not reported

Narrative

This report combines initial information received on 07 Jan 2010, from a consumer via Novartis Pharma, and follow-up information received on 11 Jan 2010, from a pharmacist. A consumer reported, her father, used VOLTAREN SCHMERZGEL (diclofenac diethylammonium salt) to treat bruises, site unspecified, and complained that "the effect was very poor". Additionally, he developed "blistering at his fingers, with subsequent burst open of the skin and severe pain". The pharmacist added the patient used the product probably to treat back pain and developed the initially reported adverse events. It was added that the fingers were cleaned after each application and that the skin at the application site was not affected. Voltaren Schmerzgel was probably discontinued. The outcome of the events was not reported.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically Significant). The events Blister and Pain were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

[NOVARTIS, 26.01.2010 DE-NOVNCHHQ-CHPA2010DE00833](#)

Sender diagnosis: not reported

Narrative

This report combines initial information received on 07 Jan 2010, from a consumer via Novartis Pharma, and follow-up information received on 11 Jan

2010, from a pharmacist. A consumer reported, her father, used VOLTAREN SCHMERZGEL (diclofenac diethylammonium salt) to treat bruises, site unspecified, and complained that "the effect was very poor". Additionally, he developed "blistering at his fingers, with subsequent burst open of the skin and severe pain". The pharmacist added the patient used the product probably to treat back pain and developed the initially reported adverse events. It was added that the fingers were cleaned after each application and that the skin at the application site was not affected. Voltaren Schmerzgel was probably discontinued. The outcome of the events was not reported.

Additional information received on 18 Jan 2010. Updated patient data, reaction onset, details regarding the event, suspect drug information, measures taken, information related to suspected drug and adverse reaction and outcome. The patient's daughter reported this male patient used Voltaren Schmerzgel once daily, to treat bruises, starting treatment on 04 Jan 2010 and developed "blistering at both hands (area of the fingers) with burst open of the skin and severe pain, despite prompt cleaning of his hands after application". She stated that "the patient was badly impaired" and that "the skin at the application site NOS was not affected". Voltaren Schmerzgel was discontinued after 3 days, on 07 Jan 2010. The hands were treated with cortisone ointment NOS. It was stated that the adverse events persisted for several days. Causality assessment by reporting daughter of patient was "probable".

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically Significant). The events Blister and Pain were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10019034

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No		No						

Patient

Initials:	Sex: Male	Birth: privacy	Age: 81 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Desquamation	200909				not recovered/not resolved
2 Burning skin	200909				not recovered/not resolved
3 Erythema exudativum	200909				not recovered/not resolved
4 Allergic eczema	20091125		4 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Solaraze 3% Gel		/	200909-200911	Actinic keratosis
diclofenac sodium		95	62 Day	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Basal cell carcinoma			Unknown	Verdacht auf Basaliom
Senile keratosis			Unknown	
Hypertension			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10022672

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 58 Year	weight (kg): 69	BMI_int: 24.45	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Itching	200807				recovered/resolved
2 Open wound	200807				recovered/resolved
3 Blisters	200807				recovered/resolved
4 Allergic reaction	200807				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN EMULGEL diclofenac sodium	Topical	/	200807-200807	
		95	-	

Drugs (concomitant)

2 CARVEDILOL carvedilol	/	-	-	
3 gingium ginkgo tree leaves extract	/	-	-	
4 HYDROCHLOROTHIAZIDE hydrochlorothiazide	/	-	-	
5 SILICUR silybum marianum	/	-	-	
6 votum olmesartan medoxomil	/	-	-	

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 10.02.2010 DE-NOVNCHHQ-CHPA2010DE00835

Sender diagnosis: not reported

Narrative

Initial consumer report received on 07 Jan 2010. A consumer reported other individuals used VOLTAREN EMULGEL (diclofenac diethylammonium salt) and developed "blistering at the fingers, with subsequent burst open and severe pain". She knew from a good source NOS.

Follow up information received and the case was upgraded to Serious on 03 Feb 2010: According to follow-up information, the "good source NOS" of the initial reporter was a pharmacist, who was also the respective patient. She applied VOLTAREN EMULGEL (diclofenac diethylammonium salt) on her feet of an acquaintance in Jul 2008, forgot once to thoroughly wash her hands and developed "allergic reaction between her fingers, (itching, blistering and open wounds)". Voltaren emulgel was stopped and the adverse reactions were treated with healing ointment and cortisone ointment. She added that "the adverse reaction were very unpleasant, because the healing process lasted for about 6 weeks". Former treatment with Voltaren Emulgel to treat knee disorders NOS also led to allergic reaction NOS (no further information regarding this former adverse reaction reported). Outcome: Recovery.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically Significant). The events Hypersensitivity, Open Wound, Blister, Pruritus were already labelled in the product information.

Reporter's comment:

NOVARTIS, 11.02.2010 DE-002147023-CHPA2010DE00835

Sender diagnosis: not reported

Narrative

Initial consumer report received on 07 Jan 2010. A consumer reported other individuals used VOLTAREN EMULGEL (diclofenac diethylammonium salt) and developed "blistering at the fingers, with subsequent burst open and severe pain". She knew from a good source NOS.

Follow up information received and the case was upgraded to Serious on 03 Feb 2010: According to follow-up information, the "good source NOS" of the initial reporter was a pharmacist, who was also the respective patient. She applied VOLTAREN EMULGEL (diclofenac diethylammonium salt) on her feet of an acquaintance in Jul 2008, forgot once to thoroughly wash her hands and developed "allergic reaction between her fingers, (itching, blistering and open wounds)". Voltaren emulgel was stopped and the adverse reactions were treated with healing ointment and cortisone ointment. She added that "the adverse reaction were very unpleasant, because the healing process lasted for about 6 weeks". Former treatment with Voltaren Emulgel to treat knee disorders NOS also led to allergic reaction NOS (no further information regarding this former adverse reaction reported). Outcome: Recovery.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically Significant). The events Hypersensitivity, Open Wound, Blister, Pruritus were already labelled in the product information.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10025938

Serious Yes	Death Yes	Lifethr. No	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 59 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blisters	20070307			29 Day	fatal
2 Chapped lips	20070307			29 Day	fatal
3 Oral mucosa erosion	20070307			29 Day	fatal
4 Nikolsky's sign	20070307			29 Day	fatal
5 Pruritus	20070304			26 Day	fatal
6 Pyrexia	20070304			26 Day	fatal
7 Freckles	20070304			26 Day	fatal
8 Stevens-Johnson syndrome	20070304			26 Day	fatal
9 Redness of eyes	20070304			26 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ALLOPURINOL allopurinol	Oral	300 Mg milligram(s)/Day	20070209-20070224 16 Day	Hyperuricaemia
2 ALLOPURINOL allopurinol	Oral	/	20070224- -	
3 DICLOFENAC diclofenac	Oral	/	20070224- -	Pain
4 DULCOLAX bisacodyl	Oral	/	20070209-20070223 15 Day	Constipation
5 ENAHEXAL enalapril maleate	Oral	/	20070214- -	Cardiac insufficiency
6 HYDROCHLOROTHIAZIDE hydrochlorothiazide	Oral	12.5 Mg milligram(s)/Day	20070207-20070223 17 Day	Edema
7 madopar benserazide hydrochloride; levodopa	Oral	1 Df dosage form/Day	20070221-20070223 3 Day	
8 madopar benserazide hydrochloride; levodopa	Oral	/	20070224- -	
9 mono-embolex certoparin sodium	Subcutaneous	1 Df dosage form/Day	20070207-20070223 17 Day	Thrombosis prophylaxis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetic nephropathy			k.A.	

Primary source

Qualification:	Physician	observestudytype:	Individual patient use
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.02.2010 DE-002147023-PHHY2010DE09218

Sender diagnosis: not reported

Narrative

Information regarding this patient's case, which was evaluated in the course of the non Novartis multinational patient Registry "RegiSCAR" collecting data about Severe Cutaneous Adverse Reactions (SCAR), received on 15 Feb 2010: This poly medicated patient with a history of diabetic

nephropathy was treated with Mono-Embolex (certoparin) for thrombosis prophylaxis and hydrochlorothiazide (manufacture unknown) for edema from 07 Feb 2007 to 23 Feb 2007 and diclofenac (manufacture unknown) for pain on 24 Feb 2007. Other co-suspect medications included Dulcolax (bisacodyl) for constipation on 09 Feb 2007, allopurinol 300 mg for hyperuricaemia on 24 Feb 2007, Enahexal (enalapril) 10 mg for cardiac insufficiency on 14 Feb 2007 and Madopar (levodopa + benserazide) 62.5 mg for an unspecified indication. Therapy with Mono-Embolex, hydrochlorothiazide, Dulcolax was discontinued on 23 Feb 2007 and diclofenac discontinued on 24 Feb 2007. On 04 Mar 2007, she developed a Stevens-Johnson-Syndrome starting with pruritus, pyrexia, spots and redness of the eyes and was hospitalized on 07 Mar 2007 with blisters, Nikolski's sign, MSH erosions and erosions of the lips. The patient died on 25 Mar 2007. A dermatology expert committee assessed the causal relationship to Mono-Embolex and hydrochlorothiazide as possible.

Sender's comment:

For Mono-embolex & hydrochlorothiazide

Serious adverse drug reaction report, Stevens-Johnson-Syndrome [death], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. Reporting healthcare professional causality is suspected.

For diclofenac

Serious adverse drug reaction report, Stevens-Johnson-Syndrome [death], assessed as listed according to the Core Data Sheet.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10023001

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg): 80	BMI_int: 28.01	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema					recovered/resolved
2 Allergic reaction					recovered/resolved
3 Vesicles					recovered/resolved
4 Swelling of eyelid					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 solaraze diclofenac sodium	Topical	2* /Day	-	Actinic keratosis
			-	

Drugs (concomitant)

2 MARCUMAR phenprocoumon	/	-	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Trigeminal neuralgia			Unknown	
Hypertonus			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

ALM-HQ, 22.02.2010 DE-ALM-HQ-DE-2010-0060

Sender diagnosis: not reported

Narrative

Initial information received on 25 Jan 2010.

ICSR of a non-serious adverse drug reaction received from a Health Professional in Germany.

A 80-year-old male patient with relevant medical history of hypertonus and trigeminal neuralgia, applied SOLARAZE (diclofenac sodium) topically, dose and frequency unknown for senile keratosis on unknown dates.

On an unspecified date the patient experienced an allergic reaction with blistering, erythema and eyelid swelling. He was taken by ambulance to the hospital. The patient used SOLARAZE extensively against advice. Treatment with SOLARAZE was stopped. After an unknown time the patient recovered.

The physician has assessed these reactions as non-serious and possibly related to SOLARAZE.

Follow-up information received on 08 Feb 2010.

ICSR initially assessed as non-serious and upgraded to serious due to hospitalisation.

According to follow-up information, the patient was 82-year-old (height 169 cm and weight 80 kg). The frequency of SOLARAZE (diclofenac sodium) was stated as two times a day for senile keratosis on his forehead. The patient was also taking MARCUMAR (phenprocoumon) dose, via of administration and frequency unknown for an unknown indication on unknown dates.

The patient was treated for this event with SDH (Solu-Decortin H), 250 mg intravenous and treatment with SOLARAZE was stopped. After an

undisclosed period of time the patient recovered.

A laboratory test was performed on 01 Feb 2010 showing the following relevant results: Quick's test 24 % (70 - 100%), Triglycerides 337 mg/dL (until 175), Cholesterol 303 mg/dL, Cholesterol LDL 206 mg/dL, Cholesterol LDL/HDL ratio 5.0 (< 3.0), Cholesterol/HDL ratio 7.4 (until 5). Other results were within the normal ranges.

Sender's comment:

Corporate Drug Safety comment:

Hypersensitivity is listed in the CCSI of Solaraze. Blister is regarded listed since application site blistering is listed in the CCSI of Solaraze. Erythema is listed in the CCSI of Solaraze. Eyelid oedema is not listed in the CCSI of Solaraze, but may be a symptom of the hypersensitivity. As treatment was stopped and the patient recovered, dechallenge is positive. Causality is possible.

Corporate Drug Safety comment on follow-up 1 (08 Feb 2010):

It was stated that the patient was hospitalized, therefore the case is considered serious. As Solaraze was withdrawn and the patient recovered, dechallenge is considered positive for all events. Although it was stated that the patient used Solaraze extensively against advice, it is not known what amount was exactly applied. Causality remains possible for all events.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10026789

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 58 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate: 20080124	deathcause: Unknown cause of death
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blister	20080107			4 Day	Unknown
2 Epidermolysis	20080108			19 Day	fatal
3 Erythema diffuse	20080107			4 Day	Unknown
4 Renal insufficiency				4 Day	Unknown
5 Nikolsky's sign	20080109			4 Day	fatal
6 Epidermolysis				4 Day	not recovered/not resolved
7 Toxic epidermal necrolysis	20080107	20080124	18 Day	4 Day	fatal
8 Unknown cause of death				4 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	acc acetylcysteine	Intravenous (not otherwise / 120	20071221-20071231	Chronic obstructive pulmonary disease
2	Amaryl glimepiride	Oral 2 Mg milligram(s)/Day	20041213-20080103	Diabetes mellitus
3	Amiodaron amiodarone	Intravenous (not otherwise / 120	20080104- -	Cardiomyopathy
4	Arterenol norepinephrine hydrochloride	Intravenous (not otherwise / 120	20080104- -	
5	avalox moxifloxacin hydrochloride	Oral /	- -	
6	Bisomerck bisoprolol	Oral 17.5 Mg milligram(s)/Day	20051201-20080103	Cardiomyopathy
7	budecort budesonide	Respiratory (inhalation) 2* /Day	20041213-20080103	Chronic obstructive pulmonary disease
8	CEFUROXIME cefuroxime	Oral 1000 Mg milligram(s)/Day	20071222-20071231	Chronic obstructive airways disease exacerbated
9	CEFUROXIME cefuroxime	Intravenous (not otherwise / 120	20080105-20080110	Surgery
10	Clexane enoxaparin	Subcutaneous 0.8 ml millilitre(s)/Day 120	20071221-20080103	Thrombosis prophylaxis
11	dexpanthenol dexpanthenol	Other 5* /Day 75	20071221-20080103	Eye dryness
12	DICLOFENAC diclofenac	Oral 70 Mg milligram(s)/Day	20041213-20080103	Pain
13	ERYTHROMYCIN erythromycin	Oral 1500 Mg milligram(s)/Day	20071223-20071231	Chronic obstructive airways disease exacerbated
14	Formoterol fumarate formoterol	Respiratory (inhalation) 2* /Day 110	20041213-20080103	Chronic obstructive pulmonary disease
15	fraxiparin nadroparin calcium	Subcutaneous 0.6 ml millilitre(s)/Day 120	20080103-20080103	Thrombosis prophylaxis
16	furosemid furosemide	Intravenous (not otherwise / 120	20080106- -	Diuresis
17	glucose 5% glucose monohydrate	Intravenous (not otherwise /	20080104-20080107	Electrolyte substitution therapy
18	HEPARIN heparin	Intravenous (not otherwise / 120	20080104-20080106	Thrombosis prophylaxis
19	Insulin insulin	Intravenous (not otherwise / 120	20080104- -	Diabetes mellitus
20	Jonosteril calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwise / 124	20080104- -	Electrolyte substitution therapy
21	kcl potassium chloride	Intravenous (not otherwise /	20080104-20080106	Electrolyte abnormality
22	lasix furosemide	Intravenous (not otherwise 80 Mg milligram(s)/Day 120	20071221-20071231	Edema
23	Milrinone milrinone	Intravenous (not otherwise / 120	20080106- -	Cardiomyopathy
24	Natriumchlorid sodium chloride	Intravenous (not otherwise /	20071221-20080103	
25	Propofol propofol	Intravenous (not otherwise / 120	20080104- -	Narcosis
26	Ramipril ramipril	Oral 2.5 Mg milligram(s)/Day	20050101-20080103	Cardiomyopathy
27	salbutamol salbutamol	Respiratory (inhalation) / 110	20071221-20080103	Chronic obstructive pulmonary disease
28	suprarenin epinephrine	Intravenous (not otherwise /	20080104- -	

			120	-
29	Torem torasemide	Oral	/	20071221-20080103 Cardiomyopathy 14 Day
30	Tranexamsaeure tranexamic acid	Intravenous (not otherwis	/	20080104- Cardiomyopathy -
31	Viani forte fluticasone propionate; salmeterol xinafoate	Respiratory (inhalation)	/Day 110	20080103-20080103 Chronic obstructive pulmonary 1 Day disease
32	voltaren ophtha diclofenac sodium	Other	4* /Day	20071221-20080103 Pain 14 Day

Drugs (concomitant)

33	bisomerck plus bisoprolol hemifumarate; hydrochlorothiazide	Oral	1 Df dosage form/Day	20050101-20080103 Cardiomyopathy 3 Year
34	corneregel dexpanthenol		/	20071221-20080103 Dry eye 14 Day
35	decortin prednisone	Oral	/	20041213-20071220 COPD 3 Year
36	Kaliumchlorid potassium chloride	Intravenous (not otherwis	/	20080104-20080105 Potassium replacement 2 Day
37	multiple medication homeopatics nos	Unknown	/	- -
38	Remifentanil remifentanil hydrochloride	Intravenous (not otherwis	5 Mg milligram(s)/Day 120	20080107-20080107 Pain 1 Day
39	theophyllin theophylline	Oral	/	20071214-20071219 6 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Thrombosis prophylaxis			k.A.	
Surgery			k.A.	
Narcosis			Yes	
Renal insufficiency			k.A.	
Necrosis			k.A.	
Pain			Yes	
Hypertension arterial			Yes	
Eye dryness			Yes	
Electrolyte abnormality			Yes	
Edema			Yes	
Diuresis			Yes	
Hospitalisation	20071221		Yes	
Cardiomyopathy			Yes	
Chronic obstructive pulmonary disease			Yes	
Diabetes mellitus			Yes	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			
Qualification:	Other Health Professional	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 22.02.2010 DE-GlaxoSmithKline-D0066482A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of death NOS in a 58-year-old male patient who had discontinued Nadroparine calcium (Fraxiparin) injection for thrombosis prophylaxis, who had discontinued Viani forte for chronic obstructive pulmonary disease, who received Cefuroxim for exacerbation of chronic obstructive pulmonary disease and post surgery and who had discontinued Salbutamol for chronic obstructive pulmonary disease.

This report was received from the Intensified Monitoring from the Documentation Centre for Severe Skin Reactions in Freiburg.

Concurrent medical conditions included cardiomyopathy, chronic obstructive pulmonary disease, diabetes mellitus, diuresis, edema, eye dryness, hypertension arterial, narcosis, pain and substitution of KCl. The patient was hospitalised from 21 December 2007 to 3 January 2008 at cardiological department, from 3 to 4 January 2008 at surgical department and since 4 January 2008 at the intensive care unit.

Co-suspect medication included Erythromycin, Diclofenac, Voltaren eye drops, ACC, Lasix, Clexane, Torem, Amaryl, Formoterol fumarate (Formotop), Budecort, Ramipril plus, Bisomerck plus, NaCl as a carrier solution, Dexpanthenol (Corneregel), KCl, Amiodaron, Insulin, Tranexamic acid, Arterenol and Suprarenin for stabilization, Heparin, Glucose 5%, Propofol, Jonosteril, Milrinone and Furosemid.

Concurrent medications included remifentanil hydrochloride (Remifentanil) and 8 other non-GSK concurrent drugs.

On 3 January 2008 the patient received Nadroparine calcium (subcutaneous) at 0.6 ml (single dose).

On 3 January 2008 the patient received Viani forte (inhaled) (single dose).

On 21 December 2007 the patient started Salbutamol (inhaled) with unknown total daily dose.

On 22 December 2007 the patient started Cefuroxim (unknown) at 1000 mg (orally).

On 04 January 2008 the patient started Cefuroxim (intravenously) (single dose and unknown total daily dose).

Approximately 4 days after discontinuing Nadroparine calcium and Viani forte, on 7 January 2008, the patient experienced toxic epidermal necrolysis, diffuse erythema and skin blisters. On 8 January 2008, the patient developed

progression of epidermolysis. On 9 January 2008, nikolsky's sign was positive.

The physician considered the events to be life threatening.

At the time of reporting, the events were fatal.

The reporting physician considered the events were possibly related to treatment with Nadroparine calcium, Viani forte, Cefuroxim and Salbutamol to all co-suspect medication. Renal failure was also considered a possible cause of the events.

The patient died from death NOS. It is unknown whether an autopsy was performed.

Sender's comment:

Die UAW ist unerwartet fuer Viani, Salbutamol, Fraxiparin

Die UAW ist erwartet fuer Cefuroxim

Wir sehen hier einen kausalen Zusammenhang mit Viani, Salbutamol und Fraxiparin als unwahrscheinlich. Am ehesten kommt Cefuroxim als moeglich ursaechlich in Frage.

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

NOVARTIS, 22.06.2010 DE-002147023-PHHY2010DE39149

Sender diagnosis: not reported

Narrative

Initial PMS (post marketing surveillance) report received from the documentation center for severe skin reaction in Germany (dZh) in writing (dZh case no: 9170639) on 15 Jun 2010: This polymedicated patient with medical condition of chronic obstructive pulmonary disease (COPD), diabetes mellitus, cardiomyopathy, renal insufficiency, diuresis, necrosis, dry eye, oedema and surgery (unspecified) was on Voltaren optha (diclofenac), unspecified dose for pain since 21 Dec 2007 to 03 Jan 2008, diclofenac (manufacturer unknown), 70mg daily, for pain since 13 Dec 2004 to 03 Jan 2008, Avalox (moxifloxacin), unspecified dose, unknown duration and indication since 14 Dec 2007 to 31 Dec 2007, cefuroxime (manufacturer unknown) oral 500mg twice daily for exacerbated COPD since 22 Dec 2007 to 31 Dec 2007, cefuroxime (manufacturer unknown) solution for injection, intravenous, unspecified dose, unknown duration and for post surgery since 05 Jan 2008 to 10 Jan 2008 and erythromycin (manufacturer unknown), 500 mg TID, for exacerbated COPD since 23 Dec 2007 to 31 Dec 2007. Patient was hospitalized for an unspecified reason in the cardiology unit of a hospital from 21 Dec 2007 to 03 Jan 2008. On 03 Jan 2008, patient was referred to the surgery department and then to the intensive care unit on 04 Jan 2008. On 07 Jan 2008, a diffuse erythema and blister appeared. On 08 Jan 2008, a progression of epidermolysis was observed. On 09 Jan 2008, nikolski's sign was positive. On 24 Jan 2008, patient died from toxic epidermal necrolysis (TEN). The dZh expert team considered diclofenac, cefuroxime (tablets as well as solution for injection) and erythromycin as suspected drugs. The causal relationship between the occurrence of the TEN and the mentioned suspected drugs was assessed as possible.

Sender's comment:

For Voltaren Ophtha,
Serious adverse drug reaction report,
Toxic epidermal necrolysis [death], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. Reporting healthcare professional causality is suspected.

For Diclofenac oral
Serious adverse drug reaction report,
Toxic epidermal necrolysis [death], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10023742

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 47 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pain of skin	20070220			2 Day	recovered/resolved
2 Stevens-Johnson syndrome	20070219			1 Day	recovered/resolved
3 Allergic reaction	20070221			3 Day	recovered/resolved
4 Blisters	20070219			1 Day	recovered/resolved
5 Oral mucosa erosion	20070221			3 Day	recovered/resolved
6 Fever	20070220			2 Day	recovered/resolved
7 Genital erosion	20070222			4 Day	recovered/resolved
8 Lip erosion	20070222			4 Day	recovered/resolved
9 Nasal disorder	20070222			4 Day	recovered/resolved
10 Drug use for unapproved indication					Unknown
11 Skin disorder NOS	20070220			2 Day	recovered/resolved
12 Pain	20070223			5 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ALLO allopurinol		/	20070219- -	Hyperuricaemia
2 FENISTIL dimetindene maleate	Oral	/	20070221- -	Allergic reaction
3 METAMIZOL "NORMON" metamizole magnesium		/	20070219- -	Podagra
4 VOLTAREN EMULGEL diclofenac diethylamine	Topical	/ 95	20070219-20070219 1 Day	Podagra

Drugs (concomitant)

5 berodual fenoterol hydrobromide; ipratropium bromide	/	19870101- -
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Foot pain	20070218		Unknown	unspecified
Lip swelling	20070219		Unknown	Acute lip swelling
Gout acute	20070219		Unknown	Acute gout attack
Common cold	20070202		Yes	Acute common cold
Herpes NOS	20070201	20070205	No	Acute Herpes eruption
Asthma bronchial			k.A.	
Hyperuricaemia	20070217		Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 23.02.2010 DE-002147023-CHPA2010DE03232

Sender diagnosis: not reported

Narrative

Initial healthcare professional report received from systematically screening of the German documentation centre for severe skin reactions (Case No. 9170513, Mfr. Control No. 133/10) on 15 Feb 2010. A 47 year-old male patient, with a history of pre-existing asthma bronchiale under daily treatment with Berodual Inhalat (ipratropium bromide, fenoterol) since 01 Jan 1987, herpes eruption on 01 Feb 2007, and common cold on 02 Feb 2007, was diagnosed with Stevens-Johnson syndrome on 19 Feb 2007. On 18 Feb 2007, he developed unspecified foot pain. On 19 Feb 2007, he developed an acute gout attack and lip swelling, and was treated on the same day with Voltaren Emulgel (diclofenac diethylammonium salt), single topical treatment, to treat 'podagra' (= gout) and with Allo 300mg (allopurinol) 0-1-1, to treat hyperuricaemia. A treatment with Metamizol (metamizol) 1-0-1 to treat 'podagra' was also initiated from 19 Feb 2007 on. On 20 Feb 2007, the patient developed fever, skin pain, and unspecified skin alterations. On 21 Feb 2007, the patient developed erosions of oral mucosa. The patient was treated with an unspecified oral Fenistil compound (dimethindene maleate) 1-0-1, due to a tentative diagnosis of allergic reaction, on 21 Feb 2007. On 22 Feb 2007, the patient developed lip erosions, genital erosions, and nasal erosions. The patient was hospitalised on an unknown date, and was orally treated with paracetamol (paracetamol) 0-1-1, to treat unspecified pain, on 23 Feb 2007. The index day for first occurrence of blisters and erosions was reported as 19 Feb 2007. The patient was diagnosed with 'Stevens-Johnson syndrome (probable case)' on 19 Feb 2007. For the 'duration of the reaction' the date 03 Mar 2007 was reported. The reaction was rated as non life-threatening. The outcome of the events was recovery.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Hospitalization). The events Stevens-Johnson Syndrome, Pyrexia, Pain of Skin, Skin disorder, Oral mucosa erosion, Hypersensitivity, lip erosion, genital erosion, nasal disorder, pain and blister were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

NOVARTIS, 23.02.2010 DE-NOVNCHHQ-CHPA2010DE03232

Sender diagnosis: not reported

Narrative

Initial healthcare professional report received from systematically screening of the German documentation centre for severe skin reactions (Case No. 9170513, Mfr. Control No. 133/10) on 15 Feb 2010. A 47 year-old male patient, with a history of pre-existing asthma bronchiale under daily treatment with Berodual Inhalat (ipratropium bromide, fenoterol) since 01 Jan 1987, herpes eruption on 01 Feb 2007, and common cold on 02 Feb 2007, was diagnosed with Stevens-Johnson syndrome on 19 Feb 2007. On 18 Feb 2007, he developed unspecified foot pain. On 19 Feb 2007, he developed an acute gout attack and lip swelling, and was treated on the same day with Voltaren Emulgel (diclofenac diethylammonium salt), single topical treatment, to treat 'podagra' (= gout) and with Allo 300mg (allopurinol) 0-1-1, to treat hyperuricaemia. A treatment with Metamizol (metamizol) 1-0-1 to treat 'podagra' was also initiated from 19 Feb 2007 on. On 20 Feb 2007, the patient developed fever, skin pain, and unspecified skin alterations. On 21 Feb 2007, the patient developed erosions of oral mucosa. The patient was treated with an unspecified oral Fenistil compound (dimethindene maleate) 1-0-1, due to a tentative diagnosis of allergic reaction, on 21 Feb 2007. On 22 Feb 2007, the patient developed lip erosions, genital erosions, and nasal erosions. The patient was hospitalised on an unknown date, and was orally treated with paracetamol (paracetamol) 0-1-1, to treat unspecified pain, on 23 Feb 2007. The index day for first occurrence of blisters and erosions was reported as 19 Feb 2007. The patient was diagnosed with 'Stevens-Johnson syndrome (probable case)' on 19 Feb 2007. For the 'duration of the reaction' the date 03 Mar 2007 was reported. The reaction was rated as non life-threatening. The outcome of the events was recovery.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Hospitalization). The events Stevens-Johnson Syndrome, Pyrexia, Pain of Skin, Skin disorder, Oral mucosa erosion, Hypersensitivity, lip erosion, genital erosion, nasal disorder, pain and blister were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

NOVARTIS, 25.02.2010 DE-NOVNCHHQ-CHPA2010DE03232

Sender diagnosis: not reported

Narrative

Initial healthcare professional report received from systematically screening of the German documentation centre for severe skin reactions (Case No. 9170513, Mfr. Control No. 133/10) on 15 Feb 2010. A 47 year-old male patient, with a history of pre-existing asthma bronchiale under daily treatment with Berodual Inhalat (ipratropium bromide, fenoterol) since 01 Jan 1987, herpes eruption on 01 Feb 2007, and common cold on 02 Feb 2007, was diagnosed with Stevens-Johnson syndrome on 19 Feb 2007. On 18 Feb 2007, he developed unspecified foot pain. On 19 Feb 2007, he developed an acute gout attack and lip swelling, and was treated on the same day with Voltaren Emulgel (diclofenac diethylammonium salt), single topical treatment, to treat 'podagra' (= gout) and with Allo 300mg (allopurinol) 0-1-1, to treat hyperuricaemia. A treatment with Metamizol (metamizol) 1-0-1 to treat 'podagra' was also initiated from 19 Feb 2007 on. On 20 Feb 2007, the patient developed fever, skin pain, and unspecified skin alterations. On 21 Feb 2007, the patient developed erosions of oral mucosa. The patient was treated with an unspecified oral Fenistil compound (dimethindene maleate) 1-0-1, due to a tentative diagnosis of allergic reaction, on 21 Feb 2007. On 22 Feb 2007, the patient developed lip erosions, genital erosions, and nasal erosions. The patient was hospitalised on an unknown date, and was orally treated with paracetamol (paracetamol) 0-1-1, to treat unspecified pain, on 23 Feb 2007. The index day for first occurrence of blisters and erosions was reported as 19 Feb 2007. The patient was diagnosed with 'Stevens-Johnson syndrome (probable case)' on 19 Feb 2007. For the 'duration of the reaction' the date 03 Mar 2007 was reported. The reaction was rated as non life-threatening. The outcome of the events was recovery.

Following internal review on 22 Feb 2010 of data received on 15 Feb 2010, Fenistil has been corrected from a treatment drug to a suspect drug.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous (Hospitalization)

Voltaren: The events Stevens-Johnson Syndrome, Pyrexia, Pain of Skin, Skin disorder, Oral mucosa erosion, Hypersensitivity, lip erosion, genital erosion, nasal disorder, pain and blister were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case

Fenistil: The events Stevens-Johnson Syndrome, Pyrexia, Pain of Skin, Skin disorder, Oral mucosa erosion, Hypersensitivity, lip erosion, genital erosion, nasal disorder, pain and blister were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case

Reporter's comment:

NOVARTIS, 26.02.2010 DE-002147023-CHPA2010DE03232

Sender diagnosis: not reported

Narrative

Initial healthcare professional report received from systematically screening of the German documentation centre for severe skin reactions (Case No. 9170513, Mfr. Control No. 133/10) on 15 Feb 2010. A 47 year-old male patient, with a history of pre-existing asthma bronchiale under daily treatment with Berodual Inhalat (ipratropium bromide, fenoterol) since 01 Jan 1987, herpes eruption on 01 Feb 2007, and common cold on 02 Feb 2007, was diagnosed with Stevens-Johnson syndrome on 19 Feb 2007. On 18 Feb 2007, he developed unspecified foot pain. On 19 Feb 2007, he developed an acute gout attack and lip swelling, and was treated on the same day with Voltaren Emulgel (diclofenac diethylammonium salt), single topical treatment, to treat 'podagra' (= gout) and with Allo 300mg (allopurinol) 0-1-1, to treat hyperuricaemia. A treatment with Metamizol (metamizol) 1-0-1 to treat 'podagra' was also initiated from 19 Feb 2007 on. On 20 Feb 2007, the patient developed fever, skin pain, and unspecified skin alterations. On 21 Feb 2007, the patient developed erosions of oral mucosa. The patient was treated with an unspecified oral Fenistil compound (dimethindene maleate) 1-0-1, due to a tentative diagnosis of allergic reaction, on 21 Feb 2007. On 22 Feb 2007, the patient developed lip erosions, genital erosions, and nasal erosions. The patient was hospitalised on an unknown date, and was orally treated with paracetamol (paracetamol) 0-1-1, to treat unspecified pain, on 23 Feb 2007. The index day for first occurrence of blisters and erosions was reported as 19 Feb 2007. The patient was diagnosed with 'Stevens-Johnson syndrome (probable case)' on 19 Feb 2007. For the 'duration of the reaction' the date 03 Mar 2007 was reported. The reaction was rated as non life-threatening. The outcome of the events was recovery.

Following internal review on 22 Feb 2010 of data received on 15 Feb 2010, Fenistil has been corrected from a treatment drug to a suspect drug.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous (Hospitalization)

Voltaren: The events Stevens-Johnson Syndrome, Pyrexia, Pain of Skin, Skin disorder, Oral mucosa erosion, Hypersensitivity, lip erosion, genital erosion, nasal disorder, pain and blister were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case

Fenistil: The events Stevens-Johnson Syndrome, Pyrexia, Pain of Skin, Skin disorder, Oral mucosa erosion, Hypersensitivity, lip erosion, genital erosion, nasal disorder, pain and blister were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case

Reporter's comment:

NOVARTIS, 24.06.2010 DE-NOVNCHHQ-CHPA2010DE03232

Sender diagnosis: not reported

Narrative

Initial healthcare professional report received from systematically screening of the German documentation centre for severe skin reactions (Case No. 9170513, Mfr. Control No. 133/10) on 15 Feb 2010. A 47 year-old male patient, with a history of pre-existing asthma bronchiale under daily treatment with Berodual Inhalat (ipratropium bromide, fenoterol) since 01 Jan 1987, herpes eruption on 01 Feb 2007, and common cold on 02 Feb 2007, was diagnosed with Stevens-Johnson syndrome on 19 Feb 2007. On 18 Feb 2007, he developed unspecified foot pain. On 19 Feb 2007, he developed an acute gout attack and lip swelling, and was treated on the same day with Voltaren Emulgel (diclofenac diethylammonium salt), single topical treatment, to treat 'podagra' (= gout) and with Allo 300mg (allopurinol) 0-1-1, to treat hyperuricaemia. A treatment with Metamizol (metamizol) 1-0-1 to treat 'podagra' was also initiated from 19 Feb 2007 on. On 20 Feb 2007, the patient developed fever, skin pain, and unspecified skin alterations. On 21 Feb 2007, the patient developed erosions of oral mucosa. The patient was treated with an unspecified oral Fenistil compound (dimethindene maleate) 1-0-1, due to a tentative diagnosis of allergic reaction, on 21 Feb 2007. On 22 Feb 2007, the patient developed lip erosions, genital erosions, and nasal erosions. The patient was hospitalised on an unknown date, and was orally treated with paracetamol (paracetamol) 0-1-1, to treat unspecified pain, on 23 Feb 2007. The index day for first occurrence of blisters and erosions was reported as 19 Feb 2007. The patient was diagnosed with 'Stevens-Johnson syndrome (probable case)' on 19 Feb 2007. For the 'duration of the reaction' the date 03 Mar 2007 was reported. The reaction was rated as non life-threatening. The outcome of the events was recovery.

Following internal review on 22 Feb 2010 of data received on 15 Feb 2010, Fenistil has been corrected from a treatment drug to a suspect drug.

Following internal review on 21 Jun 2010 of information received on 15 Feb 2010, the Report Type has been corrected from a Spontaneous Report to a Clinical Trial. The following information was also received in the initial report, but had been incorrectly omitted: The intensive monitoring project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from definite to improbable) as follows: Allo 300mg was rated as '4= well possible'. Voltaren Emulgel and Metamizol were rated as '5= possible'. Berodual Inhalat was rated as '6 =questionable'. Fensitil oral and Paracetamol oral were rated as '7=no causal relationship'. Fensitil oral and Berodual were classified as 'not been used before'. Allo 300mg was classified as 'used before' with 'unknown tolerability result'. Voltaren Emulgel was classified as 'used before' and 'not tolerated well' (no reaction specified). It was added that the patient was suffering from infection (herpes eruption and common cold) as other possible risk factors and that there were no further known pre-existing conditions. Outcome: recovery.

Sender's comment:

This is a serious case (Hospitalization) for two products:

Voltaren: The events Stevens-Johnson Syndrome, Pyrexia, Pain of Skin, Skin disorder, Oral mucosa erosion, Hypersensitivity, lip erosion, genital erosion, nasal disorder, pain and blister were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. Investigator causality was suspected.

Fenistil: The events Stevens-Johnson Syndrome, Pyrexia, Pain of Skin, Skin disorder, Oral mucosa erosion, Hypersensitivity, lip erosion, genital erosion, nasal disorder, pain and blister were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. Investigator causality was not suspected.

Reporter's comment:

NOVARTIS, 24.06.2010 DE-002147023-CHPA2010DE03232

Sender diagnosis: not reported

Narrative

Initial healthcare professional report received from systematically screening of the German documentation centre for severe skin reactions (Case No. 9170513, Mfr. Control No. 133/10) on 15 Feb 2010. A 47 year-old male patient, with a history of pre-existing asthma bronchiale under daily treatment with Berodual Inhalat (ipratropium bromide, fenoterol) since 01 Jan 1987, herpes eruption on 01 Feb 2007, and common cold on 02 Feb 2007, was diagnosed with Stevens-Johnson syndrome on 19 Feb 2007. On 18 Feb 2007, he developed unspecified foot pain. On 19 Feb 2007, he developed an acute gout attack and lip swelling, and was treated on the same day with Voltaren Emulgel (diclofenac diethylammonium salt), single topical treatment, to treat 'podagra' (= gout) and with Allo 300mg (allopurinol) 0-1-1, to treat hyperuricaemia. A treatment with Metamizol (metamizol) 1-0-1 to treat 'podagra' was also initiated from 19 Feb 2007 on. On 20 Feb 2007, the patient developed fever, skin pain, and unspecified skin alterations. On 21 Feb 2007, the patient developed erosions of oral mucosa. The patient was treated with an unspecified oral Fenistil compound (dimethindene maleate) 1-0-1, due to a tentative diagnosis of allergic reaction, on 21 Feb 2007. On 22 Feb 2007, the patient developed lip erosions, genital erosions, and nasal erosions. The patient was hospitalised on an unknown date, and was orally treated with paracetamol (paracetamol) 0-1-1, to treat unspecified pain, on 23 Feb 2007. The index day for first occurrence of blisters and erosions was reported as 19 Feb 2007. The patient was diagnosed with 'Stevens-Johnson syndrome (probable case)' on 19 Feb 2007. For the 'duration of the reaction' the date 03 Mar 2007 was reported. The reaction was rated as non life-threatening. The outcome of the events was recovery.

Following internal review on 22 Feb 2010 of data received on 15 Feb 2010, Fenistil has been corrected from a treatment drug to a suspect drug.

Following internal review on 21 Jun 2010 of information received on 15 Feb 2010, the Report Type has been corrected from a Spontaneous Report to a Clinical Trial. The following information was also received in the initial report, but had been incorrectly omitted: The intensive monitoring project assessed the causality with regard to the patient's multiple medication (using an algorithm with 8 different levels, ranging from definite to improbable) as follows: Allo 300mg was rated as '4= well possible'. Voltaren Emulgel and Metamizol were rated as '5= possible'. Berodual Inhalat was rated as '6 =questionable'. Fensitil oral and Paracetamol oral were rated as '7=no causal relationship'. Fensitil oral and Berodual were classified as 'not been used before'. Allo 300mg was classified as 'used before' with 'unknown tolerability result'. Voltaren Emulgel was classified as 'used before' and 'not tolerated well' (no reaction specified). It was added that the patient was suffering from infection (herpes eruption and common cold) as other possible risk factors and that there were no further known pre-existing conditions. Outcome: recovery.

Sender's comment:

This is a serious case (Hospitalization) for two products:

Voltaren: The events Stevens-Johnson Syndrome, Pyrexia, Pain of Skin, Skin disorder, Oral mucosa erosion, Hypersensitivity, lip erosion, genital erosion, nasal disorder, pain and blister were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. Investigator causality was suspected.

Fenistil: The events Stevens-Johnson Syndrome, Pyrexia, Pain of Skin, Skin disorder, Oral mucosa erosion, Hypersensitivity, lip erosion, genital erosion, nasal disorder, pain and blister were assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. Investigator causality was not suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10029052

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	Yes	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 63 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 TEN	20050811			32 Day	fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	AMIODARONE amiodarone	Oral	50 Mg milligram(s)/Day	20050801-20050805	Heart insufficiency 5 Day
2	APROVEL irbesartan	Oral	300 Mg milligram(s)/Day	20050711- -	Hypertension arterial
3	ASS acetylsalicylic acid	Oral	100 Mg milligram(s)/Day	20040601- -	Thrombosis prophylaxis
4	ATMADISC fluticasone propionate; salmeterol xinafoate	Respiratory (inhalation)	/	20050805- -	COPD
5	DICLOFENAC diclofenac	Intramuscular	/	20050810-20050810	Frozen shoulder 1 Day
6	EUTHYROX levothyroxine sodium		100 µg microgram(s)/Day	20050711- -	Hypothyroidism
7	FORMOTEROL formoterol	Respiratory (inhalation)	/	20050805- -	COPD
8	HOT THERMO benzyl nicotinate; glycol salicylate	Topical	50 Mg milligram(s)/	20050805- -	Pain
9	HYDROCHLOROTHIAZIDE hydrochlorothiazide	Oral	25 Mg milligram(s)/Day	20050711- -	Hypertension arterial
10	LASIX furosemide		/	20050805- -	Hypertension arterial
11	MOVICOL macrogol; potassium chloride; sodium bicarbonate; sodium chloride	Oral	/	20050725- -	Constipation
12	NEXIUM MUPS esomeprazole	Oral	40 Mg milligram(s)/Day	20050711- -	Prophylaxis
13	PALLADON hydromorphone hydrochloride	Oral	24 Mg milligram(s)/Day	20050805- -	Pain
14	PASPERTIN metoclopramide hydrochloride	Oral	/	20050725- -	Nausea
15	PREDNISOLONE prednisolone	Intramuscular	/	20050810-20050810	Frozen shoulder 1 Day
16	SUPERTENDIN cyanocobalamin; dexamethasone; diphenhydramine hydrochloride; lidocaine hydrochloride; pyridoxine hydrochloride	Subcutaneous	20 ml millilitre(s)/Day	20050722-20050722	Shoulder hand syndrome 1 Day
17	SUPERTENDIN cyanocobalamin; dexamethasone; diphenhydramine hydrochloride; lidocaine hydrochloride; pyridoxine hydrochloride	Subcutaneous	40 ml millilitre(s)/Day	20050729-20050729	1 Day
18	TOREM torasemide	Oral	/	20050808- -	
19	TOREM torasemide	Oral	/	20031001-20050805	Hypertension arterial 22 Month
20	TRAMADOL "RATIOPHARM" tramadol hydrochloride	Oral	100 Mg milligram(s)/Day	20050711- -	Pain
21	Zoloft sertraline hydrochloride		/	-	Depression
22	Zoloft sertraline hydrochloride	Oral	50 Mg milligram(s)/Day 82	20050711-20050805 26 Day	Pain

Drugs (concomitant)

23	Morphin morphine		/	20050725-20050725	Pain 1 Day
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Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 25.02.2010 DE-PFIZER INC-2010019307

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170447. This 63-year-old female patient was treated with sertraline (ZOLOFT) 50mg daily from 11Jul2005 to 05Aug2005 for pain/depression, tramadol (TRAMADOL RATIOPHARM) 100mg since 11Jul2005 for pain, irbesartan (APROVEL) 300mg daily since 11Jul2005 for arterial hypertension, torasemide (TOREM) 10mg 2-1-0 since 10Jan2003 (paused from 06Aug2005 to 07Aug2005) for arterial hypertension, hydrochlorothiazide (manufactured by 1A PHARMA) 25mg once daily since 11Jul2005 for arterial hypertension, esomeprazole (NEXIUM MUPS) 40mg once daily since 11Jul2005 for stomach protection, levothyroxine (EUTHYROX) 100ug daily since 11Jul2005 for hypothyreosis, acetylsalicylic acid (ASS) 100mg once daily since 01Jun2004 for thrombosis prophylaxis, dexamethasone/lidocaine/cetylpyridinium chloride (SUPERTENDIN) 10ml twice daily on 22Jul2005 and 20ml once on 29Jul2005 for pain, metoclopramide (PASPERTIN) since 25Jul2005 for nausea, macrogol/sodium chloride/sodium bicarbonate/potassium chloride (MOVICOL BTL) since 25Jul2005 for constipation, amiodarone (AMIODARON RATIOPHARM) 50mg daily from 01Aug2005 to 05Aug2005 for heart insufficiency, hydromorphone (PALLADON RETARD) 8mg three times daily since 05Aug2005 for pain, formoterol (FORMOTOP) once daily since 05Aug2005 for COPD, unspecified contrast media for MRI since 05Aug2005, furosemide (LASIX) since 05Aug2005 for arterial hypertension, hydroxymethylsalicylate/benzyl nicotinate (HOT THERMO SALBE) 50 mg since 05Aug2005 for pain, salmeterol/fluticasone (ATMADISC INHALAT) since 05Aug2005 for COPD, diclofenac once on 10Aug2005 for frozen shoulder and prednisolone once on 10Aug2005 for frozen shoulder. Concomitant medication included morphine on 25Jul2005 for pain and unknown injection for pain on 25Jul2005. On 11Aug2005 the patient was hospitalized due to possible toxic epidermal necrolysis with diffuse erythema, skin blisters, Nikolski and erosion of oral, genital and anal mucosa. The adverse event was documented as life-threatening. Per documentation, the event persisted until 12Aug2005, however, the final outcome of the event was reported as death. Date of death not documented. The category infection was documented as no by the project experts. A possible relationship was suspected by the project experts with renal insufficiency and suspected dermatomyositis. In Aug2006 the project experts assessed the causal relationship as very possible with tramadol, as possible with sertraline, irbesartan, hydrochlorothiazide, esomeprazole, levothyroxine, metoclopramide, amiodarone, hydromorphone, formoterol, contrast media, furosemide, salmeterol/fluticasone and diclofenac and as questionable for torasemide, acetylsalicylic acid, dexamethasone/lidocaine/cetylpyridinium chloride, macrogol/sodium chloride/sodium bicarbonate/potassium chloride, hydroxymethylsalicylate/benzyl nicotinate and prednisolone. Information on medication was available from 14Jul2005 to 11Aug2005.

Pfizer is not the Marketing Authorization Holder for diclofenac, acetylsalicylic acid, morphine sulfate and prednisolone in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of diclofenac, acetylsalicylic acid, morphine sulfate and prednisolone submitted the same report to the regulatory authorities.

Follow-up status: Case closed (15Feb2010)

Sender's comment:

Beim Stevens-Johnson-Syndrom handelt es sich um eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

PFIZERINC, 02.03.2010 DE-PFIZER INC-2010019307

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170447. This 63-year-old female patient was treated with sertraline (ZOLOFT) 50mg daily from 11Jul2005 to 05Aug2005 for pain/depression, tramadol (TRAMADOL RATIOPHARM) 100mg since 11Jul2005 for pain, irbesartan (APROVEL) 300mg daily since 11Jul2005 for arterial hypertension, torasemide (TOREM) 10mg 2-1-0 since 10Jan2003 (paused from 06Aug2005 to 07Aug2005) for arterial hypertension, hydrochlorothiazide (manufactured by 1A PHARMA) 25mg once daily since 11Jul2005 for arterial hypertension, esomeprazole (NEXIUM MUPS) 40mg once daily since 11Jul2005 for stomach protection, levothyroxine (EUTHYROX) 100ug daily since 11Jul2005 for hypothyreosis, acetylsalicylic acid (ASS) 100mg once daily since 01Jun2004 for thrombosis prophylaxis, dexamethasone/lidocaine/cetylpyridinium chloride (SUPERTENDIN) 10ml twice daily on 22Jul2005 and 20ml once on 29Jul2005 for pain, metoclopramide (PASPERTIN) since 25Jul2005 for nausea, macrogol/sodium chloride/sodium bicarbonate/potassium chloride (MOVICOL BTL) since 25Jul2005 for constipation, amiodarone (AMIODARON RATIOPHARM) 50mg daily from 01Aug2005 to 05Aug2005 for heart insufficiency, hydromorphone (PALLADON RETARD) 8mg three times daily since 05Aug2005 for pain, formoterol (FORMOTOP) once daily since

05Aug2005 for COPD, unspecified contrast media for MRI since 05Aug2005, furosemide (LASIX) since 05Aug2005 for arterial hypertension, hydroxymethylsalicylate/benzyl nicotinate (HOT THERMO SALBE) 50 mg since 05Aug2005 for pain, salmeterol/fluticasone (ATMADISC INHALAT) since 05Aug2005 for COPD, diclofenac once on 10Aug2005 for frozen shoulder and prednisolone once on 10Aug2005 for frozen shoulder. Concomitant medication included morphine on 25Jul2005 for pain and unknown injection for pain on 25Jul2005. On 11Aug2005 the patient was hospitalized due to possible toxic epidermal necrolysis with diffuse erythema, skin blisters, Nikolski and erosion of oral, genital and anal mucosa. The adverse event was documented as life-threatening. Per documentation, the event persisted until 12Aug2005, however, the final outcome of the event was reported as death. Date of death not documented. The category infection was documented as no by the project experts. A possible relationship was suspected by the project experts with renal insufficiency and suspected dermatomyositis. In Aug2006 the project experts assessed the causal relationship as very possible with tramadol, as possible with sertraline, irbesartan, hydrochlorothiazide, esomeprazole, levothyroxine, metoclopramide, amiodarone, hydromorphone, formoterol, contrast media, furosemide, salmeterol/fluticasone and diclofenac and as questionable for torasemide, acetylsalicylic acid, dexamethasone/lidocaine/cetylpyridinium chloride, macrogol/sodium chloride/sodium bicarbonate/potassium chloride, hydroxymethylsalicylate/benzyl nicotinate and prednisolone. Information on medication was available from 14Jul2005 to 11Aug2005.

Pfizer is not the Marketing Authorization Holder for diclofenac, acetylic salicylic acid, morphine sulfate and prednisolone in the reporter's country. This may be a duplicate report in situations where another Marketing Authorization Holder of diclofenac, acetylic salicylic acid, morphine sulfate and prednisolone submitted the same report to the regulatory authorities.

Follow-up status: Case closed (15Feb2010)

Follow-up (25Feb2010): This case is not from a Regulatory authority.

Sender's comment:

Beim Stevens-Johnson-Syndrom handelt es sich um eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10030565

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 28 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oral mucosa erosion	20070210			27 Day	recovered/resolved
2 Mucosal erosion	20070213	20070306	22 Day	30 Day	recovered/resolved
3 Nikolsky's sign	20070213	20070306	22 Day	30 Day	recovered/resolved
4 Swelling of lips	20070210	20070306	25 Day	27 Day	recovered/resolved
5 Stevens-Johnson syndrome	20070210	20070306	25 Day	27 Day	recovered/resolved
6 Blisters	20070211	20070306	24 Day	28 Day	recovered/resolved
7 Skin blotches	20070211	20070306	24 Day	28 Day	recovered/resolved
8 Lip erosion	20070211	20070306	24 Day	28 Day	recovered/resolved
9 Pruritis	20070213	20070306	22 Day	30 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMBROXOL ambroxol	Oral	3 Df dosage form/Day	20070203-	
2 diclofenac diclofenac	Oral	2 Df dosage form/Day	20070203-	Pain
3 EUTHYROX levothyroxine sodium	Oral	75 µg microgram(s)/Day	20070122-20070202 12 Day	Hypothyroidism
4 Lyrica pregabalin	Oral	300 Mg milligram(s)/Day 6	-	
5 Lyrica pregabalin	Oral	150 Mg milligram(s)/Day 6	20070131-	Pain
6 LYRICA pregabalin	Oral	/	20070131-	Pain
7 mono-embolex certoparin sodium	Subcutaneous	1 Df dosage form/Day	20070115-20070202 19 Day	Thrombosis prophylaxis
8 MUCOSOLVAN ambroxol hydrochloride	Oral	6 ml millilitre(s)/Day	20070118-20070202 16 Day	
9 NEXIUM esomeprazole magnesium	Oral	/	20070121-	
10 novalgine metamizole	Oral	150 Gtt drop(s)/Day	20070121-20070203 14 Day	Pain
11 THYBON liothyronine hydrochloride	Oral	/	20070201-20070203 3 Day	Hypothyroidism
12 voltaren diclofenac sodium	Oral	100 Mg milligram(s)/Day	20070122-	Pain

Drugs (concomitant)

13	aminoven		/	-	
	amino acids nos			-	
14	CALCIUM	Oral	/	-	
	calcium			-	
15	calcium gluconate		/	-	
	calcium gluconate			-	
16	DIPIDOLOR		/	-	
	piritramide			-	
17	DISOPRIVAN		/	-	
	propofol			-	
18	dulcolax		/	-	Constipation
	bisacodyl			-	
19	FRESH FROZEN PLASMA		/	-	
	plasma protein fraction (human)			-	
20	fresubin	Oral	/	-	
	carbohydrates nos; fatty acids nos; minerals nos; proteins nos; vitamins nos			-	
21	insuman rapid		/	-	
	insulin human; isophane insulin			-	
22	jonosteril		/	-	
	calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride			-	
23	kaliumchlorid		/	-	
	potassium chloride			-	
24	KONAKION		/	-	
	phytomenadione			-	
25	LASIX		/	-	
	furosemide			-	
26	LAXOBERAL	Oral	/	-	
	sodium picosulfate			-	
27	lipofundin		/	-	
	glycerol; lecithin; soya oil			-	
28	MACROGOL	Oral	/	-	
	macrogol			-	
29	multibionta	Oral	/	-	
	ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate			-	
30	MUSARIL	Oral	/	-	
	tetrazepam			-	
31	nacl		/	-	
	sodium chloride			-	
32	PROPOFOL		/	-	20070114-20070117 4 Day
	propofol			-	
33	RAPIFEN		/	-	
	alfentanil hydrochloride			-	
34	RED BLOOD CELLS, CONCENTRATED		/	-	
	red blood cells, concentrated			-	
35	solu-decortin-h		/	-	
	prednisolone sodium succinate			-	
36	TRANXILIUM	Oral	/	-	
	clorazepate dipotassium			-	
37	VOLUVEN		/	-	20090115-20090118 4 Day
	hetastarch			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hospitalization NOS	20070109	20070203	No	surgery department
Thyroid operation	20070115	20070115	No	Underwent surgery.
Hypothyroidism			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Other Health Professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 01.03.2010 DE-002147023-PHHY2010DE09441

Sender diagnosis: not reported

Narrative

Initial report received from a non Novartis multinational patient Registry [RegiSCAR] collecting data about Severe Cutaneous Adverse Reactions (SCAR), was reported on 15 Feb 2010: This poly medicated patient with a history of hypothyroidism and papillary carcinoma of the thyroid gland under went surgery on 09 Jan 2007. One week later, the patient was initiated Mono-Embolex (certoparin sodium) 1 dosage form QD subcutaneously for thrombosis prophylaxis on 15 Jan 2007, Voltaren (diclofenac sodium) 50 mg BID on 22 Jan 2010 and Diclofenac Dura one DF on 03 Feb 2010 for pain. Other co-suspect medications included Mucosolvan (ambroxol hydrochloride) 2 ml TID, orally for secretolysis from 18 Jan 2007, Ambrol AL (ambroxol) 300 mg TID , orally for secretolysis from 03 Feb 2007, Novalgin (metimazole) 30 drops, 5QD, orally for pain from 21 Jan 2007, Nexium (esomeprazole magnesium) Lyrica 75 mg (pregabalin) 150-300 mg daily, orally for pain from 31 Jan 2007, Euthyrox (levothyroxine sodium), 75 ug QD and Thybon 20 mcg (liothyronine hydrochloride) 20-60 ug daily, orally for hypothyroidism since from 01 Feb 2007. Therapy with Mono-Embolex, Mucosolvan, Euthyrox, Novalgin and Thybon was stopped on 03 Feb 2007. The patient was discharged on 03 Feb 2007. One week after discontinuation of Mono-Embolex, Mucosolvan, Euthyrox, Novalgin and Thybon, she developed Stevens-Johnson- Syndrome with a swelling of the lips and MSH (erosion of oral mucosa) erosions and was hospitalized on 11 Feb 2007 with cockades and spots, blisters and erosions of the lips. On 13 Feb 2007 she developed also pruritus, Nikolski's sign and NSH (erosions of nasal mucosa) erosions. The outcome of the events was reported to be complete recovery. A dermatology expert committee assessed the causal relationship to Mono-Embolex and Voltaren as possible.

Sender's comment:

For Mono-Embolex and Voltaren,

Serious adverse drug reaction report, Stevens-Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. Reporting healthcare professional causality is suspected.

Serious adverse drug reaction report, Stevens-Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

All remaining reported leading events and associated symptoms were assessed as non-serious by Novartis Medical Safety Physician.

Reporter's comment:

PFIZERINC, 02.03.2010 DE-PFIZER INC-2010019337

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170505. A contactable physician reported that this 28-year old female patient was hospitalized in the surgery department from 09Jan2007 to 03Feb2007. She was treated due to pain with pregabalin (LYRICA) 2x75 mg/d and 2x150 mg/d since 31Jan2007, metamizole sodium (NOVALGIN) 5x30 DF/d from 21Jan2007 to 03Feb2007, diclofenac sodium (VOLTAREN) 2x50 mg/d since 22Jan2007 and diclofenac dura 2x50 mg/d since 03Feb2007; for thrombosis

prophylaxis with heparin fraction/sodium salt (MONO-EMBOLEX) once per day from 15Jan2007 to 02Feb2007; for secretolysis with ambroxol hydrochloride (MUCOSOLVAN) 3x2 ml/d from 18Jan2007 to 02Feb2007 and ambroxol al 3x300 mg/d since 03Feb2007; for stomach protection with esomeprazole (NEXIUM) 1x 20 mg/d and 2x20 mg/d since 21Jan2007; due to hypothyroidism with levothyroxine sodium (EUTHYROX) 1x75 ug/d since 22Jan2007 and liothyronine hydrochloride (THYBON) 20 ug/40 ug/60 ug from 01Feb2007 to 03Feb2007; for substitution with calcium sherbet 3xd from 23Jan2007 to 02Feb2007 and calcium carbonate (CALCIUM HEXAL) 3x500 mg/d since 03Feb2007 and due to constipation with macrogol since 03Feb2007. Medical history included thyroid gland surgery on 15Jan2007. She was concomitantly treated with propofol for narcosis and hetastarch (VOLUVEN) for substitution from 15Jan2007 to 18Jan2007, esomeprazole (NEXIUM) 1x40 mg/d from 15Jan2007 to 20Jan2007 for stomach protection, metamizole sodium (NOVALGIN) 5x1 g/d from 15Jan2007 to 20Jan2007 due to pain, potassium chloride from 15Jan2007 to 22Jan2007 for substitution, calcium acetate/magnesium acetate/potassium acetate/sodium acetate/sodium chloride (JONOSTERIL) from 15Jan2007 to 17Jan2007 and 19Jan2007 to 24Jan2007, erythrocyte concentration and fresh frozen plasma on 16Jan2007 for substitution, prednisolone sodium succinate (SOLU-DECORTIN-H) 1x100/250 mg/d post-surgery, disoprivan from 16Jan2007 to 17Jan2007 for narcosis, phytomenadione (KONAKION) for substitution from 16Jan2007 to 17Jan2007, carbohydrates/fatty acid/minerals/proteins/vitamins (FRESUBIN) for nutrition from 16Jan2007 to 18Jan2007, furosemide (LASIX) for diuresis from 16Jan2007 to 21Jan2007, piritramide (DIPIDILOR) and diclofenac sodium (VOLTAREN) 3x50 mg/d due to pain from 16Jan2007 to 22Jan2007, calciumgluconat for substitution from 16Jan2007 to 22Jan2007, amino acids (AMINOVEN) for substitution from 16Jan2007 to 18Jan2007 and 21Jan2007 to 22Jan2007, insulin human (INSUMAN RAPID) and sodium chloride 0.9 % on 17Jan2007 for substitution, alfentanil hydrochloride (RAPIFEN) for narcosis on 17Jan2007, bisacodyl (DULCOLAX) on 19Jan2007 due to constipation, sodium picosulfate (LAXOBERAL) 1x10DF/d from 18Jan2007 to 20Jan2007 due to constipation, clorazepate dipotassium (TRANXILIUM) 1x20 mg/d from 20Jan2007 to 21Jan2007 and tetrazepam (MUSARIL) on 21Jan2007 for sedation, MULTIBIONTA on 21Jan2007 and soya oil/xylitol (LIPOFUNDIN) from 21Jan2007 to 22Jan2007 for substitution. On 10Feb2007 she developed Stevens-Johnson syndrome with swelling of the lips and erosion of oral mucosa. She was hospitalized the following day with cockades, spots, blisters on the skin and lip erosion. On 13Feb2007 the patient developed the additional symptoms pruritus, Nikolsky's sign and erosion of nasal mucosa. The patient had recovered from the adverse event on 06Mar2007. In Feb2007 the project experts assessed the causal relationship as quite possible for diclofenac sodium, metamizole sodium and diclofenac dura, as possible for heparin fraction/sodium salt, ambroxol hydrochloride, esomeprazole, levothyroxine sodium, pregabalin, liothyronine hydrochloride and ambroxol and as questionable for calcium sherbet, calcium carbonate and macrogol.

Pfizer is not the Marketing Authorization Holder for heparin fraction/sodium salt, ambroxol hydrochloride, metamizole sodium, esomeprazole, levothyroxine sodium, calcium sherbet, liothyronine hydrochloride, diclofenac, calcium carbonate and macrogol in the reporters country. This may be a duplicate report in situations where another Marketing Authorization Holder of heparin fraction/sodium salt, ambroxol hydrochloride, metamizole sodium, esomeprazole, levothyroxine sodium, calcium sherbet, liothyronine hydrochloride, diclofenac, calcium carbonate and macrogol submitted the same report to the regulatory authorities.

Follow-up status: case closed (15Feb2010).

Sender's comment:

Based on available information, a possible contributory role of the subject drug pregabalin can not be excluded for the reported event Stevens-Johnson syndrome due to temporal relationship. Patient was on multiple medications that may also have contributed to the event. This individual report would not seem to modify the risk-benefit profile of the subject drug. Bei Stevens-Johnson-Syndrom handelt es sich um eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10027526

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 60 Year	weight (kg): 55	BMI_int: 18.59	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Swelling of face	20100104	20100106	3 Day	5 Day	recovered/resolved
2 Vomiting	20100104	20100106	3 Day	5 Day	recovered/resolved
3 Seborrhea					recovered/resolved
4 Facial swelling					recovered/resolved
5 Nausea					recovered/resolved
6 Redness					recovered/resolved
7 Vomiting					recovered/resolved
8 Allergic reaction	20100104	20100106	3 Day	5 Day	recovered/resolved
9 C-reactive protein increased	20100104			5 Day	Unknown
10 Congestion pulmonary					recovered/resolved
11 Ear swelling	20100104	20100106	3 Day	5 Day	recovered/resolved
12 Face red	20100104	20100106	3 Day	5 Day	recovered/resolved
13 Nausea	20100104			6 Day	recovered/resolved
14 Skin peeling					recovered/resolved
15 Sweating					recovered/resolved
16 Swelling of eyelid	20100104	20100106	3 Day	5 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Rectal	/	20091231-20091231	Limb discomfort
diclofenac		242	1 Day	
2 paracetamol 500	Oral	500 Mg milligram(s)/	20091230-20100102	Viral upper respiratory tract
paracetamol		245	4 Day	infection

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Headache	200912		Unknown	
Viral upper respiratory tract infection	200912		Unknown	viraler Infekt mit Fieber und Gliederschmerzen
Limb discomfort	200912		Unknown	Gliederschmerzen bei viralem Infekt mit Fieber
Fever	200912		Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

HELIOS Klinikum Wuppertal, 03.03.2010 DE-PVZIBE-02-10251

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:
allergische Reaktion

KURZZUSAMMENFASSUNG:
allergischen Reaktion auf Paracetamol und Diclofenac
BESCHREIBUNG:

seit dem 30.12.2009 Kopfschmerzen und Fieber bis 38,5°C (im Sinne eines viralen Infektes), daraufhin Einnahme von Paracetamol (vom 30.12.2009 bis 02.01.2010) und Einnahme von Diclofenac einmalig am 31.12.2009;

bei Einweisung Schwellung des Gesichtes, der Ohren, der Augenlider und der Wangen, starke Rötung des Gesichtsbereiches, leichte Übelkeit, einmaliges Erbrechen;

bei Aufnahme kein weiteres Exanthem oder Erythem am Körper, keine bronchiale Spastik, fieberfrei, kein Meningismus, Blutdruck 110/60mmHg; vor dem 30.12.2009 kein Paracetamol eingenommen, so dass dies die erstmalige Anwendung war

THERAPIE:

beim Hausarzt: 100mg Prednisolut i.v. und eine Ampulle Fenistil i.v.;

stationär: parenteraler Volumenersatz, Kreislaufkontrollen

VERLAUF:

laut Beschreibung des Hausarztes Patient bisher völlig gesund;

während des stationären Aufenthaltes starkes Schwitzen, das langsam im Verlauf abklingt;

04.01.2010 Röntgen-Thorax: Zwerchfell glatt abzugrenzen, beginnende peribronchitische Anschoppung nicht ausgeschlossen, sonst kein Nachweis pulmonaler Infiltrationen, kein Anhalt für Pneumonie;

06.01.2010 rückläufiges Erythem der Gesichtshaut, teilweise Ablösen der Gesichtshaut bei insgesamt Neigung des Patienten zur Seborrhoe;

bei symptomatologischer Behandlung allmähliche Befund- und Beschwerderegress

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 04.-06.01.2010;

Krankensein aufgrund einer allergischen Reaktion auf Paracetamol und Diclofenac;

Besserung nach Absetzen der Medikamente und antiallergisch wirksamer Therapie

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: paracetamol 500

BETREFFENDE UAW: allergische Reaktion

ERLÄUTERUNG: Überempfindlichkeitsreaktionen auf Paracetamol mit der Ausprägung der beschriebenen UAW sind in der Fachinfo beschrieben.

MEDIKAMENT: diclofenac

BETREFFENDE UAW: allergische Reaktion

ERLÄUTERUNG: Überempfindlichkeitsreaktionen auf Diclofenac sind bekannt und in der Fachinfo als häufig auftretend beschrieben.<dbtext>

To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Initialmeldung - Zusatzinformationen werden erwartet.

Reporter's comment:

sanofi-aventis, 12.03.2010 DE-SANOFI-AVENTIS-2010SA012485

Sender diagnosis: not reported

Narrative

This ICSR had been created by a regional pharmacovigilance centre and was extracted from the website of the German health authority (authority number DE-BFARM-10027526) on 04-Mar-2010. The pharmacovigilance (PV) centre is conducting a hospitalisation registry.

Within this registry, the patient's medical records are screened for adverse drug reactions that could have led to hospital admission; this screening takes place after the patient's discharge from hospital.

The case involves a 60-year-old male patient. Since 30-Dec-2009 the patient suffered from headache and fever 38.5 °C (in the sense of a viral infection). Therefore, he took paracetamol (no tradename provided) at a dose of 500 mg for viral upper respiratory tract infection from 30-Dec-2009 to 02-Jan-2010 and diclofenac (no tradename provided) once for limb discomfort on 31-Dec-2009. On 04-Jan-2010 the patient experienced a facial swelling (ear, eye lid, cheeks), severe redness of face, mild nausea and unique vomiting. The family doctor administered prednisolone sodium succinate at a dose of 100 mg i.v. and one ampoule of dimetindene maleate i.v. to treat the events. The patient was hospitalised due to an allergic reaction to paracetamol and diclofenac on the same day. On admission no further exanthema on body, no bronchial spasm, no fever and no meningism was observed. The blood pressure was 110/60 mmHg. The patient received volume replacement. During the hospitalisation the patient experienced severe sweating that slowly decays over time. On 04-Jan-2010 an x-ray thorax was performed with the following findings: Delineate

diaphragm smooth, incipient peri bronchial engorgement not excluded, no evidence of pulmonary infiltration or pneumonia. On 06-Jan-2010 a declining erythema of face and partial detachment of the facial skin in tendency to seborrhea were observed. The condition of the patient improved after effective anti-allergic therapy. The patient recovered and was discharged from hospital on 06-Jan-2010.

The reporter assessed the causal relationship between the events and the suspected drugs according to the WHO score as possible.

Explanatory notes to causality:

Paracetamol: The adverse drug reactions are well known and described in the physician's information.

Diclofenac: The adverse drug reactions are well known and described in the physician's information as frequently occurring.

Sender's comment:

Sanofi-aventis Company Comment dated 05-MAR-2010:

Due to temporary relationship the causal role of paracetamol and diclofenac cannot be excluded.

12-MAR-2010 Sanofi-Aventis Germany

All reported clinical diagnoses are considered locally labelled for all suspected products and thus consistent with the known benefit risk ratio of the product(s). Measures according to Paragraph 29 German Drug Law (AMG), section (1) concerning the approval status are not deemed necessary.

Reporter's comment:

NOVARTIS, 17.03.2010 DE-002147023-PHHY2010DE13353

Sender diagnosis: not reported

Narrative

Initial report received from a regional Pharmacovigilance center (intensive monitoring project) via Health Authority (DE-BFARM-10027526) on 03 Mar 2010: This patient with medical history of viral upper respiratory infection and Limb discomfort received diclofenac (manufacturer unknown) suppository and paracetamol (manufacturer unknown) and was hospitalized for allergic reaction. He had headache and fever up to 38.5 degree Celsius (consistent with a viral infection) since 30 Dec 2009, followed by intake of paracetamol, 500 mg tablet daily, orally from 30 Dec 2009 to 02 Jan 2010 and single intake of diclofenac on 31 Dec 2009. On admission to hospital on 04 Jan 2010, he had swelling of the face, ears, eyelids and cheeks, severe reddening of the facial area, slight nausea, single episode of vomiting. After admission to hospital, he had no further exanthema or erythema on the body, no bronchial spasticity, afebrile, no meningism, blood pressure was 110/80 mmHg. He had not taken paracetamol before 30 Dec 2009 and this was therefore the first use. According to the primary health care provider's description, the patient had previously been completely healthy. During hospitalization, severe sweating gradually subsiding during the course. On 04 Jan 2010, Chest radiography showed diaphragm clearly distinguishable, incipient peribronchitic congestion not ruled out, otherwise no evidence of pulmonary infiltrations, no sign of pneumonia. C-reactive protein was 158.3 mg/dL (Reference range: 0 to 5 mg/L), absolute neutrophil count was $9.11 \times 10^9/L$ (Reference range: 1.5 to 7.7) and WBC was $15.32 \times 10^9/L$ (reference range: 4 to 10). He received Prednisolut (prednisolone-21-hydrogen succinate, sodium salt) 100 mg, intravenously and one ampoule of Fenistil (dimetinden) intravenously. He also received parenteral volume replacement and had a circulatory monitoring. On 06 Jan 2010, he had regressive erythema of the facial skin, partial detachment of the facial skin with overall tendency to seborrhea. He had general regression of the findings and complaints with symptomatic treatment. The hospitalization duration was from 04 Jan 2010 to 06 Jan 2010. The event outcome was reported as completely recovered. The regional Pharmacovigilance center assessed as possibly related to diclofenac and paracetamol.

Sender's comment:

Serious adverse drug reaction report, hypersensitivity [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

HELIOS Klinikum Wuppertal, 07.04.2010 DE-PVZIBE-02-10251

Sender diagnosis: not reported

Narrative

ANMERKUNGEN ZU FOLGENDER UAW:

07.03.2016

BfArM - Pharmacovigilance

422 / 652

allergische Reaktion

KURZZUSAMMENFASSUNG:

allergischen Reaktion auf Paracetamol und Diclofenac

BESCHREIBUNG:

seit dem 30.12.2009 Kopfschmerzen und Fieber bis 38,5°C (im Sinne eines viralen Infektes), daraufhin Einnahme von Paracetamol (vom 30.12.2009 bis 02.01.2010) und Einnahme von Diclofenac einmalig am 31.12.2009; seit 2.01. zunächst Rötung des Gesichts, dann zunehmende Schwellung bis 04.01.. daraufhin Arzt aufgesucht

bei Einweisung Schwellung des Gesichtes, der Ohren, der Augenlider und der Wangen, starke Rötung des Gesichtsbereiches, leichte Übelkeit, einmaliges Erbrechen;

bei Aufnahme kein weiteres Exanthem oder Erythem am Körper, keine bronchiale Spastik, fieberfrei, kein Meningismus, Blutdruck 110/60mmHg; vor dem 30.12.2009 kein Paracetamol eingenommen, so dass dies die erstmalige Anwendung war

THERAPIE:

beim Hausarzt: 100mg Prednisolol i.v. und eine Ampulle Fenistil i.v.;

stationär: parenteraler Volumenersatz, Kreislaufkontrollen

VERLAUF:

laut Beschreibung des Hausarztes Patient bisher völlig gesund;

während des stationären Aufenthaltes starkes Schwitzen, das langsam im Verlauf abklingt;

04.01.2010 Röntgen-Thorax: Zwerchfell glatt abzugrenzen, beginnende peribronchitische Anschoppung nicht ausgeschlossen, sonst kein Nachweis pulmonaler Infiltrationen, kein Anhalt für Pneumonie;

06.01.2010 rückläufiges Erythem der Gesichtshaut, teilweise Ablösen der Gesichtshaut bei insgesamt Neigung des Patienten zur Seborrhoe;

bei symptomatologischer Behandlung allmähliche Befund- und Beschwerderegress

ERLÄUTERUNGEN ZUR UAW:

Krankenhausaufenthalt: 04.-06.01.2010;

Krankenhauseinweisung aufgrund einer allergischen Reaktion auf Paracetamol und Diclofenac;

Besserung nach Absetzen der Medikamente und antiallergisch wirksamer Therapie

ERLÄUTERUNGEN ZU DEN KAUSALITÄTEN:

MEDIKAMENT: paracetamol 500

BETREFFENDE UAW: allergische Reaktion

ERLÄUTERUNG: Überempfindlichkeitsreaktionen auf Paracetamol mit der Ausprägung der beschriebenen UAW sind in der Fachinfo beschrieben.

MEDIKAMENT: diclofenac

BETREFFENDE UAW: allergische Reaktion

ERLÄUTERUNG: Überempfindlichkeitsreaktionen auf Diclofenac sind bekannt und in der Fachinfo als häufig auftretend beschrieben.<dbtext>

To_Char_NARRATIVE_2</dbtext>

Sender's comment:

case closed; es werden keine weiteren Zusatzinformationen mehr erwartet.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10041281

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	Yes	No	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 42 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	20090702	20090814	44 Day	1 Day	not recovered/not resolved
2 Restlessness					Unknown
3 Nausea					Unknown
4 Infection	20090702			1 Day	Unknown
5 Conjunctivitis	20090711			10 Day	Unknown
6 Lip erosion	20090707			6 Day	Unknown
7 Blister	20090704			3 Day	Unknown
8 Skin red	20090703			2 Day	Unknown
9 Pruritus	20090703			2 Day	Unknown
10 Edema uvula	20090703			2 Day	Unknown
11 Fever	20090702			1 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Cefuroxim cefuroxime sodium	Oral	2*500 Mg milligram(s)/ Da	20090702-20090703	Infection 2 Day
2 Isocillin phenoxymethylpenicillin potassium	Intravenous (not otherwis	2* / Day 231	20090702-20090702	Infection 1 Day
3 multiple medication homeopatics nos	Unknown	/	- -	
4 novalgine metamizole	Oral	30 Gtt drop(s)/ Day	20090701-20090701	Pain 1 Day
5 Penicillin phenoxymethylpenicillin potassium	Intravenous (not otherwis	/ Day 231	20090701-20090701	Infection prophylaxis 1 Day
6 tracrrium atracurium besilate	Intravenous (not otherwis	/	20090701-20090701	Muscle relaxation 1 Day
7 Tramal tramadol hydrochloride	Oral	/	20090702-20090703	Pain 2 Day
8 Ultiva remifentanil hydrochloride	Intravenous (not otherwis	/	20090701-20090701	Narcosis 1 Day
9 voltaren diclofenac sodium	Oral	/	20090613-20090701	Sore throat 19 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Sore throat			Yes	
Pain			Yes	
Tonsillitis	20090609	20090616	No	
Tonsillectomy	20090701	20090701	Yes	

Primary source

Qualification:	Physician	observestudytype:	Other studies
Literature:			
Qualification:	Other Health Professional	observestudytype:	Other studies
Literature:			

Narratives (nur bei elektronischem Eingang)

GlaxoSmithKline, 18.03.2010 DE-GlaxoSmithKline-D0066739A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of toxic epidermal necrolysis in a 42-year-old female patient who had received Atracurium besylate (Tracrium) and Remifentanyl hydrochloride (Ultiva) injection for muscle relaxation and narcosis. This report was received from the Intensified Monitoring from the Documentation Centre for Severe Skin Reactions in Freiburg.

The patient's past medical history included tonsillitis until 16 June 2009, which had been treated with penicillin from 09 to 13 June 2009.

Concurrent medical conditions included pain and sore throat. On 01 July 2009 the patient underwent tonsillectomy.

Co-suspect medication included Diclofenac (Voltaren), Dipyrrone (Novalgin), Phenoxymethylpenicillin potassium (Penicillin), Phenoxymethylpenicillin potassium (Isocillin), Tramadol hydrochloride (Tramal), Cefuroxime sodium (Cefuroxim) and 10 other drugs for peri-surgical prophylaxis, narcosis and substitution.

On 1 July 2009 the patient underwent tonsillectomy and received Atracurium besylate (intravenous) and Remifentanyl hydrochloride (intravenous), together with several other drugs.

Approximately 1 days after surgery, on 2 July 2009, the patient experienced fever, followed by edema of uvula, itching and skin red on 03 July 2009. On 04 July 2009 the patient developed skin blister, on 07 July 2009 lip erosion and on 11 July 2009 severe conjunctivitis.

The patient was first treated for the suspect of infection, but finally diagnosed with toxic epidermal necrolysis.

The physician considered the events to be life threatening.

Additionally the patient was treated for nausea and restlessness, but no start date of the events was provided.

At the time of reporting, the events were unresolved, but there was also the end date of 14 August 2009 reported for toxic epidermal necrolysis.

The patient was treated with 14 drugs.

The reporting physician considered the events were possibly related to treatment with Diclofenac, Dipyron, Phenoxymethylpenicillin potassium, Tramadol hydrochloride and Cefuroxime sodium and questionably related to Atracurium besilate, Remifentanyl hydrochloride and the other suspect drugs.

Tonsillitis was also considered a possible cause of the events.

No further information will be available.

Sender's comment:

Die UAW ist nicht erwartet.

Wir sehen einen kausalen Zusammenhang mit Ultiva und Tracrium als unwahrscheinlich. Es kommen weitere Arzneimittel in Betracht

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10039872

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Oral mucosa redness	20070925	20071026	32 Day	19 Day	recovered/resolved
2 Blisters	20070925	20071026	32 Day	19 Day	recovered/resolved
3 Eye redness	20070925	20071026	32 Day	19 Day	recovered/resolved
4 Skin lesion	20070919	20071026	38 Day	13 Day	recovered/resolved
5 Pruritus	20070921	20071026	36 Day	15 Day	recovered/resolved
6 Stevens Johnson syndrome	20070919	20071026	38 Day	13 Day	recovered/resolved
7 Nikolsky's sign	20070925	20071026	32 Day	19 Day	recovered/resolved
8 Fever	20070917	20071026	40 Day	11 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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DE-BFARM-10039872

1	acetylcysteine acetylcysteine	Oral	600 Mg milligram(s)/Day	20070905-20070918 14 Day	
2	acetylcysteine acetylcysteine	Oral	900 Mg milligram(s)/Day	20070828-20070828 1 Day	
3	acetylcysteine acetylcysteine	Intravenous (not otherwis	1200 Mg milligram(s)/Day	20070829-20070829 1 Day	
4	ADALAT nifedipine	Oral	/	20070911-20070914 4 Day	Hypertension
5	ALDACTONE potassium canrenoate	Oral	100 Mg milligram(s)/Day 120	20070903-20070903 1 Day	Hypertension
6	AMBROXOL ambroxol	Oral	45 Mg milligram(s)/Day	20070913-20070918 6 Day	Bronchospasm
7	avalox moxifloxacin hydrochloride	Oral	400 Mg milligram(s)/Day	20070916-20070919 4 Day	Psoas abscess
8	BELOC ZOK metoprolol succinate	Oral	2 Df dosage form/Day	20070902- -	Hypertension
9	BERODUAL fenoterol hydrobromide; ipratropium bromide	Respiratory (inhalation)	4 Df dosage form/Day	20070906- -	Respiratory obstruction unspecified
10	catapresan clonidine	Intravenous (not otherwis	/	20070830-20070831 2 Day	Hypertension
11	catapresan clonidine	Oral	450 Mg milligram(s)/Day	20070912- -	
12	CLEXANE heparin-fraction, sodium salt	Subcutaneous	/	20070908- -	Thrombosis prophylaxis
13	COMBACTAM sulbactam sodium	Intravenous (not otherwis	2 G gram(s)/Day	20070828-20070828 1 Day	Psoas abscess
14	diclac diclofenac sodium	Oral	150 Mg milligram(s)/Day	20070908-20070912 5 Day	Pain
15	DIOVAN valsartan	Oral	80 Mg milligram(s)/Day	20070907-20070912 6 Day	Hypertension
16	DIPIDOLOR piritramide	Intravenous (not otherwis	6 Mg milligram(s)/Day	20070828-20070828 1 Day	Pain
17	DIPIDOLOR piritramide	Intravenous (not otherwis	8 Mg milligram(s)/Day	20070908-20070909 2 Day	
18	DIPIDOLOR piritramide	Intravenous (not otherwis	9 Mg milligram(s)/Day	20070830-20070905 7 Day	
19	HYDROCHLOROTHIAZIDE hydrochlorothiazide	Oral	25 Mg milligram(s)/Day	20070907-20070911 5 Day	Diuresis
20	hydrocortisone hydrocortisone	Intravenous (not otherwis	/	20070829-20070901 4 Day	Sepsis
21	hydrocortisone hydrocortisone	Intravenous (not otherwis	/	20070907-20070914 8 Day	
22	INSULIN insulin	Subcutaneous	/	20070905-20070914 10 Day	Hyperglycaemia
23	L-THYROXIN levothyroxine sodium	Oral	75 µg microgram(s)/Day	20070910- -	Hypothyroidism
24	MUCOSOLVAN ambroxol hydrochloride	Oral	45 Mg milligram(s)/Day	20070913- -	
25	MUCOSOLVAN ambroxol hydrochloride	Intravenous (not otherwis	/	20070907-20070912 6 Day	
26	NORVASC amlodipine besilate	Oral	10 Mg milligram(s)/Day	20070902- -	Hypertension
27	NOVALGIN metamizole	Oral	30 Gtt drop(s)/Day	20070923-20070925 3 Day	
28	NOVALGIN metamizole		3 G gram(s)/Day	20070829-20070914 17 Day	Pain

DE-BFARM-10039872

29	PANTOZOL pantoprazole	Intravenous (not otherwis 40 Mg milligram(s)/Day	20070828-20070901	Gastric ulcer prophylaxis
30	PANTOZOL pantoprazole	Oral 40 Mg milligram(s)/Day	5 Day 20070905-	
31	PARACETAMOL paracetamol	Oral /	20070912-20070916	Fever
32	PIPERACILLIN piperacillin	Intravenous (not otherwis 8 G gram(s)/Day	5 Day 20070828-20070828	Psoas abscess
33	RIFAMPICIN rifampicin	Intravenous (not otherwis 600 Mg milligram(s)/Day	1 Day 20070901-20070916	Psoas abscess
34	ROCEPHIN ceftriaxone sodium	Intravenous (not otherwis 4 G gram(s)/Day	16 Day 20070828-20070831	Psoas abscess
35	SAROTEN amitriptyline hydrochloride	Oral 75 Mg milligram(s)/Day	4 Day 20070907-20070914	Pain
36	staphylex flucloxacillin sodium	Intravenous (not otherwis 6 G gram(s)/Day	8 Day 20070831-20070916	Psoas abscess
37	tavor lorazepam	Oral 1 Mg milligram(s)/Day	17 Day 20070920-20070920	
38	tavor lorazepam	Oral 1 Mg milligram(s)/Day	1 Day 20070922-20070922	
39	tavor lorazepam	Oral 1 Mg milligram(s)/Day	1 Day 20070907-20070914	Restlessness
40	tavor lorazepam	Oral 1 Mg milligram(s)/Day	8 Day 20070918-20070918	
41	TRAMAL tramadol hydrochloride	Oral 40 Gtt drop(s)/Day	1 Day 20070908-20070909	Pain
42	TRAMAL tramadol hydrochloride	Oral 60 Gtt drop(s)/Day	2 Day 20070912-20070913	
			2 Day	

Drugs (concomitant)			
43	actrapid mc insulin porcine	Intravenous (not otherwis /	20070829-20070905 Hyperglycaemia 8 Day
44	AMINO ACIDS NOS	Intravenous (not otherwis /	20070828-20070828 1 Day
45	AMINO ACIDS NOS/ELECTROLYTES NOS/S amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis /	20070903-20070905 3 Day
46	AMINO ACIDS NOS/ELECTROLYTES NOS/S amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis /	20070829-20070831 Volume depletion 3 Day
47	aminoven amino acids nos	Intravenous (not otherwis /	20070828-20070828 Parenteral fluid replacement 1 Day
48	ARTERENOL norepinephrine hydrochloride	Intravenous (not otherwis /	20070828-20070831 Instability cardiovascular 4 Day
49	ASCORBIC ACID/CALCIUM PANTOTHENATE ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Intravenous (not otherwis 1 Df dosage form/Day	20070902-20070902 Parenteral nutrition 1 Day
50	BELOC metoprolol tartrate	Intravenous (not otherwis 25 Mg milligram(s)/Day	20070830-20070901 3 Day
51	BELOC metoprolol tartrate	Intravenous (not otherwis 5 Mg milligram(s)/Day	20070828-20070828 Hypertension 1 Day
52	clindamycin clindamycin	Oral 2 Df dosage form/Day	20070920- -
53	clindamycin clindamycin	Intravenous (not otherwis 1800 Mg milligram(s)/Day	20070828-20070831 Psoas abscess 4 Day
54	CYANOCOBALAMIN cyanocobalamin	Intravenous (not otherwis 1 Df dosage form/Day	20090902-20090902 1 Day
55	DIAZEPAM diazepam	Intravenous (not otherwis 12.5 Mg milligram(s)/Day	20070828-20070828 Unrest 1 Day
56	DOLANTIN pethidine hydrochloride	Intravenous (not otherwis 25 Mg milligram(s)/Day	20070902-20070902 Pain 1 Day
57	EBRANTIL urapidil	Intravenous (not otherwis 250 Mg milligram(s)/Day	20070831-20070901 Hypertension 2 Day
58	EUGALAC lactulose	Oral /	20070904-20070905 Constipation 2 Day
59	fenistil dimetindene maleate	Intravenous (not otherwis 1 Df dosage form/Day	20070908-20070908 Respiratory obstruction 1 Day unspecified
60	fenistil dimetindene maleate	Oral 1 Df dosage form/Day	20070919- -
61	Folsaeure folic acid	Intravenous (not otherwis 1 Df dosage form/Day	20070902-20070902 1 Day
62	Glyceroltrinitrat glyceryl trinitrate	Respiratory (inhalation) 2 Df dosage form/Day	20070917-20070917 Hypertension 1 Day
63	HALDOL haloperidol	Intravenous (not otherwis 5 Mg milligram(s)/Day	20070828-20070828 Unrest 1 Day
64	HEPARIN heparin	Intravenous (not otherwis 15000 Iu international unit	20070828-20070828 Thrombosis prophylaxis 1 Day
65	HETASTARCH hetastarch	Intravenous (not otherwis /	20070907-20070913 7 Day
66	HETASTARCH hetastarch	Intravenous (not otherwis /	20070828-20070829 Volume depletion 2 Day
67	INSULIN GLARGINE insulin glargine	Subcutaneous /	20070902-20070902 Hyperglycaemia 1 Day
68	INSUMAN insulin human injection, isophane; insulin zinc protamine injection	Intravenous (not otherwis 8 Iu international unit(s)/D;	20070828-20070828 Hyperglycaemia 1 Day

Drugs (concomitant)

69	INZOLEN aspartate zinc; cobalt hydrogenaspartate; copper hydrogenaspartate; magnesium aspartate; manganese hydrogenaspartate; potassium hydrogen aspartate	Intravenous (not otherwis /		20070912-20070914 3 Day
70	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070920-20070921 2 Day
71	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070828-20070903 Volume depletion 7 Day
72	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070914-20070916 3 Day
73	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070906-20070911 6 Day
74	kalinor potassium chloride	Oral	300 Mg milligram(s)/Day	20070912-20070920 9 Day
75	Kaliumchlorid potassium chloride	Intravenous (not otherwis /		20070828-20070913 Electrolyte replacement 17 Day
76	Kaliumzitrat potassium citrate	Intravenous (not otherwis 2 Df dosage form/Day		20070902-20070903 2 Day
77	LASIX furosemide	/		20070828-20070905 Diuresis 9 Day
78	magnetrans magnesium aspartate hydrochloride	Oral	900 Mg milligram(s)/Day	20070911-20070911 1 Day
79	METOCLOPRAMIDE metoclopramide	Intravenous (not otherwis 30 Mg milligram(s)/Day		20070828-20070901 Nausea 5 Day
80	MONO-EMBOLEX heparin-fraction, sodium salt	Subcutaneous	3000 Iu international unit(s)	20070830-20070907 Thrombosis prophylaxis 9 Day
81	moronal nystatin	/		20070914-20070918 Prophylaxis 5 Day
82	Natriumchlorid sodium chloride	/		- -
83	neostigmine neostigmine	Intravenous (not otherwis /		20070829-20070901 4 Day
84	PERENTEROL yeast dried	Oral	6 Df dosage form/Day	20070916-20070919 Diarrhea 4 Day
85	PERFALGAN paracetamol	Intravenous (not otherwis /		20070828-20070828 Fever 1 Day
86	PERFALGAN paracetamol	Intravenous (not otherwis /		20070924-20070924 1 Day
87	PERFALGAN paracetamol	Intravenous (not otherwis /		20070922-20070922 1 Day
88	PERFALGAN paracetamol	Intravenous (not otherwis /		20070902-20070903 2 Day
89	PHYTOMENADIONE phytomenadione	Intravenous (not otherwis 1 Df dosage form/Day		20070902-20070902 1 Day
90	prednisolone prednisolone	Oral	/	20070921-20070924 Exanthema 4 Day
91	PROPOFOL propofol	Intravenous (not otherwis /		20070828-20070831 Narcosis 4 Day
92	sobelin solubile clindamycin phosphate	Intravenous (not otherwis 1800 Mg milligram(s)/Day		20070828-20070828 Psoas abscess 1 Day
93	solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis 250 Mg milligram(s)/Day		20070920-20070920

Drugs (concomitant)

94	solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwis 250 Mg milligram(s)/Day	1 Day 20070906-20070906 Exanthema
95	SUFENTA sufentanil citrate	Intravenous (not otherwis /	1 Day 20070829-20070829 Narcosis
96	Vitamin B calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	Intravenous (not otherwis 1 Df dosage form/Day	1 Day 20070911-20070911 Oral fluid replacement

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Spondylodiscitis			Yes	
Psoas abscess			Yes	
Diuresis			k.A.	
Fever			k.A.	
Hyperglycaemia			k.A.	
Hypertension			k.A.	
Spondylodiscitis			k.A.	
Pain			k.A.	
Respiratory obstruction unspecified			k.A.	
Restlessness			k.A.	
Sepsis			k.A.	
Psoas abscess			k.A.	
Intervertebral discitis			k.A.	
Hypothyroidism			k.A.	

Primary source

Qualification: Physician observestudytype: Other studies
Literature:

Narratives (nur bei elektronischem Eingang)

Nycomed, 19.03.2010 DE-Nycomed-0210604

Sender diagnosis: not reported

Narrative

The case was received by Pfizer (ref. no. 2010031955) /Wyeth (ref. no. DE-WYE-G05756510) Germany on 12-MAR-2010 from the German Documentation centre for severe skin reactions (RegiSCAR) and was forwarded to Nycomed on 17-MAR-2010.

It concerned a 70-year-old female patient started therapy with Pantozol (pantoprazole), 40 mg daily, intravenous, on 28-AUG-2007, for gastric ulcer prophylaxis. On 05-SEP-2007, intravenous pantoprazole was switched to oral pantoprazole, 20 mg twice daily. Additional suspect medications were piperacillin, sulbactam, ceftriaxone, piritramide, acetylcysteine, metamizole, hydrocortisone, clonidine, flucloxacillin, rifampicin, metoprolol, amlodipine besilate), potassium canrenoate, insulin, fenoterol /ipratropium, hydrochlorothiazide, valsartan, ambroxol, amitriptyline, diclofenac, tramadol heparin-fraction, levothyroxine sodium, nifedipine, paracetamol and moxifloxacin. The patient had a medical history of restlessness, intervertebral discitis, psoas abscess, hypertension, pain, hypothyroidism, unspecified respiratory obstruction, hyperglycaemia, diuresis and fever. She was concomitantly treated with clindamycin, human insulin, haloperidol, diazepam, furosemide, neostigmine, urapidil and prednisolone, amongst other numerous medications. From 19-SEP-2007 (22 days after the first dose of pantoprazole), the patient suffered from exanthema. On 21-SEP-2007, she experienced pruritus and on 22-SEP-2007, she presented with fever. On 25-SEP-2007, she was diagnosed with Stevens-Johnson syndrome. At that time she was additionally suffered from vesicles on the skin, redness of eyes and as well as redness oral mucosa and Nikolsky's signs. The patient was hospitalised. No details regarding clinical course or treatment were provided.

Action taken concerning therapy with pantoprazole was unknown.
The patient recovered on 26-OCT-2007.

The investigator considered Stevens-Johnson syndrome possibly related to pantoprazole and all other suspect medications.

Sender's comment:

Stevens-Johnson syndrome is listed in the current reference safety information on pantoprazole.

Stevens-Johnson Syndrome (SJS) is a rare but severe dermatological condition, considered as a severe type of erythema exsudativum multiforme. It is characterized by erythema with bullous and eroded lesions of skin and mucous membranes. Additional symptoms include fever, general ill feeling and joint aches. SJS is an immune-complex-mediated hypersensitivity reaction that occurs in response to medications (e.g., sulfonamides, penicillins, barbiturates, and phenytoin), infections (e.g., herpes simplex, Mycoplasma and malaria infections), or malignancies (various carcinomas and lymphomas). Stevens-Johnson syndrome (SJS) is idiopathic in 25-50% of cases.

This patient had other risk factors that could have caused or contributed to the development of the reaction e.g. co-suspect medications and several concomitant medications.

This single case report is not considered to change the current knowledge of the safety profile of pantoprazole.

Reporter's comment:

Wyeth-Pharma GmbH, 23.03.2010 DE-WYE-G05756510

Sender diagnosis: not reported

Narrative

This case has been received from Pfizer (Pfizer Case ID #2010031955) and contains a Pfizer co-suspect product. The Pfizer case may qualify for regulatory reporting for the Pfizer co-suspect product. This case may therefore be a duplicate report. Information was received from a healthcare professional via a registry entitled "European Registry of Severe Cutaneous Adverse Reactions to Drugs (RegiSCAR)" regarding a 70-year-old female patient who received Tavor (lorazepam unspc) therapy and experienced Stevens Johnson syndrome.

MEDICAL HISTORY:

The patient's concurrent illnesses include intervertebral discitis and psoas abscess.

PRODUCT DETAILS:

Indication for Tavor was restlessness. The dose regimen was 1 mg 1 time per day (oral) from 07-Sep-2007 to 14-Sep-2007, then was 1 mg (oral) on 18-Sep-2007, then was 1 mg (oral) on 20-Sep-2007 and then was 1 mg (oral) on 22-Sep-2007.

Additional suspect medication included Piperacillin, Combactam (sulbactam sodium), Rocephin (ceftriaxone sodium), Pantozol (pantoprazole unspc), Dipidolor (piritramide), Acetylcysteine, Novalgin (metamizole sodium), Hydrocortisone, Catapresan (clonidine), Staphylex (flucloxacillin sodium), Rifampicin, Beloc Zok (metoprolol succinate), Norvasc (amlodipine besilate), Aldactone (potassium canrenoate injection), Insulin (unspc), Berodual (fenoterol hydrobromide/ipratropium bromide), Hydrochlorothiazide, Diovan (valsartan), Mucosolvan (ambroxol hydrochloride), Saroten (amitriptyline hydrochloride), Diclac (diclofenac sodium), Tramal (tramadol hydrochloride), Clexane (heparin-fraction, sodium salt), L-Thyroxin (levothyroxine sodium), Adalat (nifedipine), Paracetamol and Avalox (moxifloxacin).

CONCOMITANT THERAPY:

Concomitant therapy included Sobelin Solubile (clindamycin phosphate unspc), Insuman (insulin human injection, isophane/insulin zinc protamine injection), Heparin, Haldol (haloperidol), Diazepam, Hetastarch, Amino Acids Nos (unspc), Clindamycin, Arterenol (norepinephrine hydrochloride), Disoprivan (propofol), Beloc (metoprolol tartrate), Metoclopramide, Lasix (furosemide), Potassium Chloride, Jonosteril (calcium acetate/magnesium acetate/potassium acetate/sodium chloride), Perfalgan (paracetamol), Neostigmine, Sufenta (sufentanil citrate), Multibionta (ascorbic acid/calcium pantothenate/cyanocobalamin/nicotinamide/pyridoxine hydrochloride/retinol palmitate/riboflavin/rutoside/thiamine hydrochloride/tocopheryl acetate), Actrapid Mc (insulin), Periplasol (amino acids nos/electrolytes nos/sorbitol/xylitol), Mono-Embolex (heparin-fraction, sodium salt), Ebrantil (urapidil), Lantus (insulin glargine), Dolantin (pethidine hydrochloride), Folic Acid, Vitamin B12 (cyanocobalamin), Vitamin K (phytomenadione), Potassium Citrate, Eugalac (lactulose), Solu-Decortin-H (prednisolone sodium succinate), Fenistil (dimetindene maleate unspc), Magnetrans (magnesium aspartate), Vitamin B, Sodium Chloride (unspc), Inzolen (aspartate zinc/cobalt hydrogenaspartate/copper hydrogenaspartate/magnesium aspartate/manganese hydrogenaspartate/potassium hydrogen aspartate), Kalinor (potassium chloride), Moronal (nystatin), Perenterol (yeast dried), Nitro (glyceryl trinitrate unspc) and Prednisolon (prednisolone).

EVENT DETAILS:

Starting from 19-Sep-2007 the patient suffered from exanthema. On 21-Sep-2007 she experienced pruritus and on 22-Sep-2007 additionally fever. Then on 25-Sep-2007 she was diagnosed with Stevens-Johnson syndrome (Stevens-Johnson syndrome). At this time she was additionally suffering from vesicles of skin, redness of eyes and as well as redness oral mucosa and Nikolsky's signs. No details regarding clinical course or treatment were provided. However she has recovered by 26-Oct-2007.

RELATEDNESS ASSESSMENT:

The investigator considered Stevens-Johnson syndrome possibly related to Tavor as well as to Piperacillin, Combactam, Rocephin, Pantozol, Dipidolor, Acetylcysteine, Novalgin, Hydrocortisone, Catapresan, Staphylex, Rifampicin, Beloc Zok, Norvasc, Aldactone, Insulin (unspc), Berodual, Hydrochlorothiazide, Diovan, Mucosolvan, Saroten, Diclac, Tramal, Clexane, L-Thyroxin, Adalat, Paracetamol and Avalox.

Sender's comment:

Wyeth comment:

Initial (v1) 16-Mar-2010

STEVENS-JOHNSON-SYNDROM nach Tavor

Dieser Fall wurde von Pfizer erhalten (Fallnummer 2010031955) und enthält ein verdächtiges Pfizer-Präparat, weshalb es zu einer Doppelmeldung kommen kann.

Fall aus dem RegiSCAR-Register der Uni Freiburg:

Es wurde über eine 70-jährige Patientin berichtet, die während eines stationären Aufenthaltes bei Spondylodisitis mit Psoasabszessen aufgrund von Unruhe vom 07.09.07 bis zum 15.09.07 sowie am 18., 20. und 22.09.07 Tavor erhielt. Sie bekam eine umfangreiche weitere medikamentöse Behandlung. Ab dem 19.09.07 entwickelte sie dermale Symptome, die sich bis zum 25.09.07 zu einem Steven-Johnson-Syndrom verschlimmerten. Ausser zu Tavor wurde ein möglicher Zusammenhang zur Behandlung mit Piperacillin, Combactam (Sulbactam), Rocephin (Ceftriaxon), Pantozol, Dipidolor (Piriramid), Acetylcystein, Novalgin (Metamizole), Hydrocortison, Catapresan (Clonidin), Staphylex (Flucloxacillin), Rifampicin, Beloc Zok (Metoprolol), Norvasc (Amlodipin), Aldactone, Insulin (unspez.), Berodual (Fenoterol/Ipratropiumbromid), Hydrochlorothiazid, Diovan (Valsartan), Mucosolvan (Ambroxol), Saroten (Amitriptyline), Diclac (Diclofenac), Tramal (Tramadol), Clexane (Heparin-Fraktion), L-Thyroxin (Levothyroxin), Adalat (Nifedipin), Paracetamol und Avalox (Moxifloxacin) gesehen. Weitere Informationen zum klinischen Verlauf und zur Behandlung lagen nicht vor. Bis zum 26.10.07 erholte sie sich jedoch von den Ereignissen.

Vom RegiSCAR-Register wird ein möglicher Zusammenhang für die o.g. Arzneimittel angenommen.

Die berichteten Ereignisse sind in der FI zu Tavor nicht aufgeführt und somit unbekannt.

Basierend auf der aktuellen Information führt dieser Bericht nicht zu einer Änderung des Nutzen-Risiko-Verhältnisses. Zu diesem Zeitpunkt sind keine weiteren Maßnahmen vorgesehen. Sobald Änderungen hierzu bekannt werden, werden wir darüber unverzüglich und unaufgefordert informieren.

Dr. D. Kracht

Reporter's comment:

NOVARTIS, 30.06.2010 DE-002147023-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodisitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess, Sobelin iv (clindamycine) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain, potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (propacetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, Periplusa iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. On 31 Aug 2007 Catapresan was stopped, Ebrantil iv (urapidil) for hypertension, Staphylex iv (flucloxacillin) for psoasabscess and rifampicin iv for psoasabscess were started. On 01 Sep Ebrantil was stopped, a single dose of Lantus sc (insulin glargine) for hyperglycaemia, beloc zok oral (metoprolol) for hypertension, Norvasc oral (amlodipine) for hypertension and Aldactone oral (spironolactone) for hypertension were started. On 02 Sep the patient received a single dose of Dolantin iv (pethidine) for pain, of folic acid iv, of vitamin B 12 iv and of vitamin K iv for substitution. Furthermore potassium oral and Lasix oral (furosemide) for diuresis were started. Potassium oral was stopped on 03 Sep. On 04 Sep 2007 the patient received single doses of Novalgin oral (metamizole), of Kitiva iv (unknown ingredients) for pain and of Eugalac oral (lactulose) for constipation. On 05 Sep Novalgin oral for pain, insulin sc for hyperglycaemia and Pantozol oral for stomach protection were started. On 06 Sep 2007 a single dose of Solu-Decortin H iv (prednisolone) for stridor and an exanthema was given. Berodual inhalation (ipratropium bromide/fenoterol) for pulmonary obstruction was started. On 07 Sep 2007 Mono-Embolex was stopped and HCT oral (hydrochlorothiazide) for diuresis, Diovan oral (valsartan) for hypertension, Mucosolvan iv (ambroxol) for secretolysis, Tavor oral (lorazepam) for unrest and Saroten oral (amitriptyline) for pain were started. On 08 Sep single doses of Fenistil iv (dimetidine) and erythrocyte concentrate iv for anaemia were given. Also Diclac oral (diclofenac) for pain, Tramal oral (tramadol) for pain and Clexane sc (enoxaparin) for thrombosis prophylaxis were started. On 10 Sep 2007 levothyroxine oral for hypothyroidism was started. On 11 Sep 2007 the patient was given a single dose of Magnetran forte oral (magnesium oxide) and of vitamin B iv for substitution. Furthermore Adalat oral (nifedipine) for

hypertension and sodium chloride iv to keep the injection entry open were started. HCT was stopped on the same day. On 12 Sep 2007 Diovan, Mucosolvan and Diclac were stopped. Inzolen iv (potassium aspartate, magnesium aspartate, copper hydrogenaspartate, zinc aspartate, manganese hydrogenaspartate) for substitution, paracetamol oral for fever, Kalinor oral (potassium citrate, potassium hydrogencarbonate, citric acid) for substitution and Catapresan oral (clonidine) for hypertension were started. On 13 Sep 2007 Tramal was stopped, ambroxol oral for secretolysis was started. On 14 Sep 2007 Novalgin oral, insulin sc, Saroten oral, Adalat oral and Inzolen iv were stopped, Moronal top (nystatin) for prophylaxis of soor and Xaramitis oral (unknown ingredients) were started. The patient was transferred on this day to a surgical unit. On 15 Sep 2007 Perenterol oral (saccharomyces boulardii) for diarrhoea was started. On 16 Sep 2007 Staphylex iv, rifampicin iv and paracetamol oral were stopped, Avalox oral (moxifloxacin) for psoasabscess was started. On 17 Sep 2007 the patient developed fever which lasted until 20 Sep 2007. She was given a single dose of Nitro oral (glyceryl trinitrate) for hypertension. On 18 Sep 2007 ambroxol oral and Moronal oral were stopped. On 19 Sep the patient developed skin lesions. A Stevens-Johnson syndrome was diagnosed. Perenterol and Avalox were stopped. Fenistil oral (dimetidine) for rash was started. On 20 Sep 2007 the patient was given a single dose of Solu- Decortin H iv (prednisolone) for pulmonary obstruction. Kalinor oral and Xaramitis were stopped, Clinda saar oral (clindamycin) for psoasabscess was started. On 21 Sep 2007 the patient developed pruritus, prednisolone oral for exanthema was started. On 22 Sep 2007 the patient developed again fever. On 23 Sep 2007 Novaminsulfon drops (metamizole) for fever was started and was given for two days. On 24 Sep 2007 prednisolone oral was stopped. On 25 Sep 2007 the patient developed also blisters on the skin, Nikolskys sign and redness of the eyes and oral mucosa. The Stevens-Johnson syndrome lasted until 26 Oct 2007. The patient had fully recovered. The causal relationship between the Stevens-Johnson syndrome and the medications was assessed by a dermatology expert panel as follows.: To Pipril iv, Sobelin iv, Combactam iv, Insuman Rapid iv, heparin iv, Haldol iv, diazepam iv, Haes iv, Aminoven iv, Rocephin iv, clindamycin iv, Arterenol iv, Disoprivan iv, Beloc iv, MCP iv, Pantozol iv, Lasix iv, Perfalgan iv, ACC iv, neostigmine iv, Sufenta iv, Novalgin iv, Actrapid iv, PeriPlasmasol iv, Ebrantil iv, Lantus sc, Dolantin iv, folic acid iv, vitamin B12 iv, vitamin K iv, potassium oral, Novalgin oral, Kitiva iv, Eugalac oral, Fenistil oral, Clinda Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnetrans forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

NOVARTIS, 01.07.2010 DE-NOVNCHHQ-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodiscitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess,

Sobelin iv (clindamycine) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain, potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (propacetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, Periplasma iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. On 31 Aug 2007 Catapresan was stopped, Ebrantil iv (urapidil) for hypertension, Staphylex iv (flucloxacillin) for psoasabscess and rifampicin iv for psoasabscess were started. On 01 Sep Ebrantil was stopped, a single dose of Lantus sc (insulin glargine) for hyperglycaemia, belok zok oral (metoprolol) for hypertension, Norvasc oral (amlodipine) for hypertension and Aldactone oral (spironolactone) for hypertension were started. On 02 Sep the patient received a single dose of Dolantin iv (pethidine) for pain, of folic acid iv, of vitamin B 12 iv and of vitamin K iv for substitution. Furthermore potassium oral and Lasix oral (furosemide) for diuresis were started. Potassium oral was stopped on 03 Sep. On 04 Sep 2007 the patient received single doses of Novalgin oral (metamizole), of Kitiva iv (unknown ingredients) for pain and of Eugalac oral (lactulose) for constipation. On 05 Sep Novalgin oral for pain, insulin sc for hyperglycaemia and Pantozol oral for stomach protection were started. On 06 Sep 2007 a single dose of Solu-Decortin H iv (prednisolone) for stridor and an exanthema was given. Berodual inhalation (ipratropium bromide/fenoterol) for pulmonary obstruction was started. On 07 Sep 2007 Mono-Embolex was stopped and HCT oral (hydrochlorothiazide) for diuresis, Diovan oral (valsartan) for hypertension, Mucosolvan iv (ambroxol) for secretolysis, Tavor oral (lorazepam) for unrest and Saroten oral (amitriptyline) for pain were started. On 08 Sep single doses of Fenistil iv (dimetidine) and erythrocyte concentrate iv for anaemia were given. Also Diclac oral (diclofenac) for pain, Tramal oral (tramadol) for pain and Clexane sc (enoxaparin) for thrombosis prophylaxis were started. On 10 Sep 2007 levothyroxine oral for hypothyroidism was started. On 11 Sep 2007 the patient was given a single dose of Magnetans forte oral (magnesium oxide) and of vitamin B iv for substitution. Furthermore Adalat oral (nifedipine) for hypertension and sodium chloride iv to keep the injection entry open were started. HCT was stopped on the same day. On 12 Sep 2007 Diovan, Mucosolvan and Diclac were stopped. Inzolen iv (potassium aspartate, magnesium aspartate, copper hydrogenaspartate, zinc aspartate, manganese hydrogenaspartate) for substitution, paracetamol oral for fever, Kalinor oral (potassium citrate, potassium hydrogencarbonate, citric acid) for substitution and Catapresan oral (clonidine) for hypertension were started. On 13 Sep 2007 Tramal was stopped, ambroxol oral for secretolysis was started. On 14 Sep 2007 Novalgin oral, insulin sc, Saroten oral, Adalat oral and Inzolen iv were stopped, Moronal top (nystatin) for prophylaxis of soor and Xaramitis oral (unknown ingredients) were started. The patient was transferred on this day to a surgical unit. On 15 Sep 2007 Perenterol oral (saccharomyces boulardii) for diarrhoea was started. On 16 Sep 2007 Staphylex iv, rifampicin iv and paracetamol oral were stopped, Avalox oral (moxifloxacin) for psoasabscess was started. On 17 Sep 2007 the patient developed fever which lasted until 20 Sep 2007. She was given a single dose of Nitro oral (glyceryl trinitrate) for hypertension. On 18 Sep 2007 ambroxol oral and Moronal oral were stopped. On 19 Sep the patient developed skin lesions. A Stevens-Johnson syndrome was diagnosed. Perenterol and Avalox were stopped. Fenistil oral (dimetidine) for rash was started. On 20 Sep 2007 the patient was given a single dose of Solu-Decortin H iv (prednisolone) for pulmonary obstruction. Kalinor oral and Xaramitis were stopped, Clinda saar oral (clindamycin) for psoasabscess was started. On 21 Sep 2007 the patient developed pruritus, prednisolone oral for exanthema was started. On 22 Sep 2007 the patient developed again fever. On 23 Sep 2007 Novaminsulfon drops (metamizole) for fever was started and was given for two days. On 24 Sep 2007 prednisolone oral was stopped. On 25 Sep 2007 the patient developed also blisters on the skin, Nikolskys sign and redness of the eyes and oral mucosa. The Stevens-Johnson syndrome lasted until 26 Oct 2007. The patient had fully recovered. The causal relationship between the Stevens-Johnson syndrome and the medications was assessed by a dermatology expert panel as follows.: To Pipril iv, Sobelin iv, Combactam iv, Insuman Rapid iv, heparin iv, Haldol iv, diazepam iv, Haes iv, Aminoven iv, Rocephin iv, clindamycin iv, Arterenol iv, Disoprivan iv, Beloc iv, MCP iv, Pantozol iv, Lasix iv, Perfalgan iv, ACC iv, neostigmine iv, Sufenta iv, Novalgin iv, Actrapid iv, Periplasma iv, Ebrantil iv, Lantus sc, Dolantin iv, folic acid iv, vitamin B12 iv, vitamin K iv, potassium oral, Novalgin oral, Kitiva iv, Eugalac oral, Fenistil oral, Clinda Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnetans forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event.

In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

NOVARTIS, 01.07.2010 DE-002147023-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodiscitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess, Sobelin iv (clindamycine) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain, potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (propacetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, Periplasma iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. On 31 Aug 2007 Catapresan was stopped, Ebrantil iv (urapidil) for hypertension, Staphylex iv (flucloxacillin) for psoasabscess and rifampicin iv for psoasabscess were started. On 01 Sep Ebrantil was stopped, a single dose of Lantus sc (insulin glargine) for hyperglycaemia, beloc zok oral (metoprolol) for hypertension, Norvasc oral (amlodipine) for hypertension and Aldactone oral (spironolactone) for hypertension were started. On 02 Sep the patient received a single dose of Dolantin iv (pethidine) for pain, of folic acid iv, of vitamin B 12 iv and of vitamin K iv for substitution. Furthermore potassium oral and Lasix oral (furosemide) for diuresis were started. Potassium oral was stopped on 03 Sep. On 04 Sep 2007 the patient received single doses of Novalgin oral (metamizole), of Kitiva iv (unknown ingredients) for pain and of Eugalac oral (lactulose) for constipation. On 05 Sep Novalgin oral for pain, insulin sc for hyperglycaemia and Pantozol oral for stomach protection were started. On 06 Sep 2007 a single dose of Solu-Decortin H iv (prednisolone) for stridor and an exanthema was given. Berodual inhalation (ipratropium bromide/fenoterol) for pulmonary obstruction was started. On 07 Sep 2007 Mono-Embolex was stopped and HCT oral (hydrochlorothiazide) for diuresis, Diovan oral (valsartan) for hypertension, Mucosolvan iv (ambroxol) for secretolysis, Tavor oral (lorazepam) for unrest and Saroten oral (amitriptyline) for pain were started. On 08 Sep single doses of Fenistil iv (dimetidine) and erythrocyte concentrate iv for anaemia were given. Also Diclac oral (diclofenac) for pain, Tramal oral (tramadol) for pain and Clexane sc (enoxaparin) for thrombosis prophylaxis were started. On 10 Sep 2007 levothyroxine oral for hypothyroidism was started. On 11 Sep 2007 the patient was given a single dose of Magnetans forte oral (magnesium oxide) and of vitamin B iv for substitution. Furthermore Adalat oral (nifedipine) for hypertension and sodium chloride iv to keep the injection entry open were started. HCT was stopped on the same day. On 12 Sep 2007 Diovan, Mucosolvan and Diclac were stopped. Inzolen iv (potassium aspartate, magnesium aspartate, copper hydrogenaspartate, zinc aspartate, manganese hydrogenaspartate) for substitution, paracetamol oral for fever, Kalinor oral (potassium citrate, potassium hydrogencarbonate, citric acid) for substitution and Catapresan oral (clonidine) for hypertension were started. On 13 Sep 2007 Tramal was stopped, ambroxol oral for secretolysis was started. On 14 Sep 2007 Novalgin oral, insulin sc, Saroten oral, Adalat oral and Inzolen iv were stopped, Moronal top (nystatin) for prophylaxis of soor and Xaramitis oral (unknown ingredients) were started. The patient was transferred on this day to a surgical unit. On 15 Sep 2007 Perenterol oral (saccharomyces boulardii) for diarrhoea was started. On 16 Sep 2007 Staphylex iv, rifampicin iv and paracetamol oral were stopped, Avalox oral (moxifloxacin) for psoasabscess was started. On 17 Sep 2007 the patient developed fever which lasted until 20 Sep 2007. She was given a single dose of Nitro oral (glyceryl trinitrate) for hypertension. On 18 Sep 2007 ambroxol oral and Moronal oral were stopped. On 19 Sep the patient developed skin lesions. A Stevens-Johnson syndrome was diagnosed. Perenterol and Avalox were stopped. Fenistil oral (dimetidine) for rash was started. On 20 Sep 2007 the patient was given a single dose of Solu- Decortin H iv (prednisolone) for pulmonary obstruction. Kalinor oral and Xaramitis were stopped, Clinda saar oral (clindamycin) for psoasabscess was started. On 21 Sep 2007 the patient developed pruritus, prednisolone oral for exanthema was started. On 22 Sep 2007 the patient developed again fever. On 23 Sep 2007 Novaminsulfon drops (metamizole) for fever was started and was given for two days. On 24 Sep 2007 prednisolone oral was stopped. On 25 Sep 2007 the patient developed also blisters on the skin, Nikolskys sign and redness of the eyes and oral mucosa. The Stevens-Johnson syndrome lasted until 26 Oct 2007. The patient had fully recovered. The causal relationship between the Stevens-Johnson syndrome and the medications was assessed by a dermatology expert panel as follows.: To

Pipril iv, Sobelin iv, Combactam iv, Insuman Rapid iv, heparin iv, Haldol iv, diazepam iv, Haes iv, Aminoven iv, Rocephin iv, clindamycin iv, Arterenol iv, Disoprivan iv, Beloc iv, MCP iv, Pantozol iv, Lasix iv, Perfalgan iv, ACC iv, neostigmine iv, Sufenta iv, Novalgin iv, Actrapid iv, PeriPlasma iv, Ebrantil iv, Lantus sc, Dolantin iv, folic acid iv, vitamin B12 iv, vitamin K iv, potassium oral, Novalgin oral, Kitiva iv, Eugalac oral, Fenistil oral, Clinda Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnetran forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

Reporter's comment:

NOVARTIS, 22.08.2011 DE-002147023-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodiscitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess, Sobelin iv (clindamycin) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain, potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (paracetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, PeriPlasma iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. On 31 Aug 2007 Catapresan was stopped, Ebrantil iv (urapidil) for hypertension, Staphylex iv (flucloxacillin) for psoasabscess and rifampicin iv for psoasabscess were started. On 01 Sep Ebrantil was stopped, a single dose of Lantus sc (insulin glargine) for hyperglycaemia, beloc zok oral (metoprolol) for hypertension, Norvasc oral (amlodipine) for hypertension and Aldactone oral (spironolactone) for hypertension were started. On 02 Sep the patient received a single dose of Dolantin iv (pethidine) for pain, of folic acid iv, of vitamin B 12 iv and of vitamin K iv for substitution. Furthermore potassium oral and Lasix oral

(furosemide) for diuresis were started. Potassium oral was stopped on 03 Sep. On 04 Sep 2007 the patient received single doses of Novalgin oral (metamizole), of Kitiva iv (unknown ingredients) for pain and of Eugalac oral (lactulose) for constipation. On 05 Sep Novalgin oral for pain, insulin sc for hyperglycaemia and Pantozol oral for stomach protection were started. On 06 Sep 2007 a single dose of Solu-Decortin H iv (prednisolone) for stridor and an exanthema was given. Berodual inhalation (ipratropium bromide/fenoterol) for pulmonary obstruction was started. On 07 Sep 2007 Mono-Embolex was stopped and HCT oral (hydrochlorothiazide) for diuresis, Diovan oral (valsartan) for hypertension, Mucosolvan iv (ambroxol) for secretolysis, Tavor oral (lorazepam) for unrest and Saroten oral (amitriptyline) for pain were started. On 08 Sep single doses of Fenistil iv (dimetidine) and erythrocyte concentrate iv for anaemia were given. Also Diclac oral (diclofenac) for pain, Tramal oral (tramadol) for pain and Clexane sc (enoxaparin) for thrombosis prophylaxis were started. On 10 Sep 2007 levothyroxine oral for hypothyroidism was started. On 11 Sep 2007 the patient was given a single dose of Magnettrans forte oral (magnesium oxide) and of vitamin B iv for substitution. Furthermore Adalat oral (nifedipine) for hypertension and sodium chloride iv to keep the injection entry open were started. HCT was stopped on the same day. On 12 Sep 2007 Diovan, Mucosolvan and Diclac were stopped. Inzolen iv (potassium aspartate, magnesium aspartate, copper hydrogenaspartate, zinc aspartate, manganese hydrogenaspartate) for substitution, paracetamol oral for fever, Kalinor oral (potassium citrate, potassium hydrogencarbonate, citric acid) for substitution and Catapresan oral (clonidine) for hypertension were started. On 13 Sep 2007 Tramal was stopped, ambroxol oral for secretolysis was started. On 14 Sep 2007 Novalgin oral, insulin sc, Saroten oral, Adalat oral and Inzolen iv were stopped, Moronal top (nystatin) for prophylaxis of soor and Xaramitis oral (unknown ingredients) were started. The patient was transferred on this day to a surgical unit. On 15 Sep 2007 Perenterol oral (saccharomyces boulardii) for diarrhoea was started. On 16 Sep 2007 Staphylex iv, rifampicin iv and paracetamol oral were stopped, Avalox oral (moxifloxacin) for psoasabscess was started. On 17 Sep 2007 the patient developed fever which lasted until 20 Sep 2007. She was given a single dose of Nitro oral (glyceryl trinitrate) for hypertension. On 18 Sep 2007 ambroxol oral and Moronal oral were stopped. On 19 Sep the patient developed skin lesions. A Stevens-Johnson syndrome was diagnosed. Perenterol and Avalox were stopped. Fenistil oral (dimetidine) for rash was started. On 20 Sep 2007 the patient was given a single dose of Solu- Decortin H iv (prednisolone) for pulmonary obstruction. Kalinor oral and Xaramitis were stopped, Clinda saar oral (clindamycin) for psoasabscess was started. On 21 Sep 2007 the patient developed pruritus, prednisolone oral for exanthema was started. On 22 Sep 2007 the patient developed again fever. On 23 Sep 2007 Novaminsulfon drops (metamizole) for fever was started and was given for two days. On 24 Sep 2007 prednisolone oral was stopped. On 25 Sep 2007 the patient developed also blisters on the skin, Nikolskys sign and redness of the eyes and oral mucosa. The Stevens-Johnson syndrome lasted until 26 Oct 2007. The patient had fully recovered. The causal relationship between the Stevens-Johnson syndrome and the medications was assessed by a dermatology expert panel as follows.: To Pipril iv, Sobelin iv, Combactam iv, Insuman Rapid iv, heparin iv, Haldol iv, diazepam iv, Haes iv, Aminoven iv, Rocephin iv, clindamycin iv, Arterenol iv, Disoprivan iv, Beloc iv, MCP iv, Pantozol iv, Lasix iv, Perfalgan iv, ACC iv, neostigmine iv, Sufenta iv, Novalgin iv, Actrapid iv, PeriPlasmal iv, Ebrantil iv, Lantus sc, Dolantin iv, folic acid iv, vitamin B12 iv, vitamin K iv, potassium oral, Novalgin oral, Kitiva iv, Eugalac oral, Fenistil oral, Clinda Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnettrans forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

Following an internal review on 10 Aug 2011, of the information received on 15 Jun 2010: Following significant correction was done. Mono-embolex was recoded with Novartis dictionary.

Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

For Mono-Embolex,

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event.

In the absence of the reporting healthcare professional causality, Novartis has processed the case as suspected for reporting purposes. The case will be reassessed on receipt of follow-up information.

MR: The event could have been confounded by pantozol.

Reporter's comment:

NOVARTIS, 22.08.2011 DE-NOVNCHHQ-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodiscitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess, Sobelin iv (clindamycine) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain, potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (propacetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, Peri plasma iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. On 31 Aug 2007 Catapresan was stopped, Ebrantil iv (urapidil) for hypertension, Staphylex iv (flucloxacillin) for psoasabscess and rifampicin iv for psoasabscess were started. On 01 Sep Ebrantil was stopped, a single dose of Lantus sc (insulin glargine) for hyperglycaemia, beloc zok oral (metoprolol) for hypertension, Norvasc oral (amlodipine) for hypertension and Aldactone oral (spironolactone) for hypertension were started. On 02 Sep the patient received a single dose of Dolantin iv (pethidine) for pain, of folic acid iv, of vitamin B 12 iv and of vitamin K iv for substitution. Furthermore potassium oral and Lasix oral (furosemide) for diuresis were started. Potassium oral was stopped on 03 Sep. On 04 Sep 2007 the patient received single doses of Novalgin oral (metamizole), of Kitiva iv (unknown ingredients) for pain and of Eugalac oral (lactulose) for constipation. On 05 Sep Novalgin oral for pain, insulin sc for hyperglycaemia and Pantozol oral for stomach protection were started. On 06 Sep 2007 a single dose of Solu-Decortin H iv (prednisolone) for stridor and an exanthema was given. Berodual inhalation (ipratropium bromide/fenoterol) for pulmonary obstruction was started. On 07 Sep 2007 Mono-Embolex was stopped and HCT oral (hydrochlorothiazide) for diuresis, Diovan oral (valsartan) for hypertension, Mucosolvan iv (ambroxol) for secretolysis, Tavor oral (lorazepam) for unrest and Saroten oral (amitriptyline) for pain were started. On 08 Sep single doses of Fenistil iv (dimetidine) and erythrocyte concentrate iv for anaemia were given. Also Diclac oral (diclofenac) for pain, Tramal oral (tramadol) for pain and Clexane sc (enoxaparin) for thrombosis prophylaxis were started. On 10 Sep 2007 levothyroxine oral for hypothyroidism was started. On 11 Sep 2007 the patient was given a single dose of Magnettrans forte oral (magnesium oxide) and of vitamin B iv for substitution. Furthermore Adalat oral (nifedipine) for hypertension and sodium chloride iv to keep the injection entry open were started. HCT was stopped on the same day. On 12 Sep 2007 Diovan, Mucosolvan and Diclac were stopped. Inzolen iv (potassium aspartate, magnesium aspartate, copper hydrogenaspartate, zinc aspartate, manganese hydrogenaspartate) for substitution, paracetamol oral for fever, Kalinor oral (potassium citrate, potassium hydrogencarbonate, citric acid) for substitution and Catapresan oral (clonidine) for hypertension were started. On 13 Sep 2007 Tramal was stopped, ambroxol oral for secretolysis was started. On 14 Sep 2007 Novalgin oral, insulin sc, Saroten oral, Adalat oral and Inzolen iv were stopped, Moronal top (nystatin) for prophylaxis of soor and Xaramitis oral (unknown ingredients) were started. The patient was transferred on this day to a surgical unit. On 15 Sep 2007 Perenterol oral (saccharomyces boulardii) for diarrhoea was started. On 16 Sep 2007 Staphylex iv, rifampicin iv and paracetamol oral were stopped, Avalox oral (moxifloxacin) for psoasabscess was started. On 17 Sep 2007 the patient developed fever which lasted until 20 Sep 2007. She was given a single dose of Nitro oral (glyceryl trinitrate) for hypertension. On 18 Sep 2007 ambroxol oral and Moronal oral were stopped. On 19 Sep the patient developed skin lesions. A Stevens-Johnson syndrome was diagnosed. Perenterol and Avalox were stopped. Fenistil oral (dimetidine) for rash was started. On 20 Sep 2007 the patient was given a single dose of Solu-Decortin H iv (prednisolone) for pulmonary obstruction. Kalinor oral and Xaramitis were stopped, Clinda saar oral (clindamycin) for psoasabscess was started. On 21 Sep 2007 the patient developed pruritus, prednisolone oral for exanthema was started. On 22 Sep 2007 the patient developed again fever. On 23 Sep 2007 Novaminsulfon drops (metamizole) for fever was started and was given for two days. On 24 Sep 2007 prednisolone oral was stopped. On 25 Sep 2007 the patient developed also blisters on the skin, Nikolskys sign and redness of the eyes and oral mucosa. The Stevens-Johnson syndrome lasted until 26 Oct 2007. The patient had fully recovered. The causal relationship between the Stevens-Johnson syndrome and the medications was assessed by a dermatology expert panel as follows.: To Pipril iv, Sobelin iv, Combactam iv, Insuman Rapid iv, heparin iv, Haldol iv, diazepam iv, Haes iv, Aminoven iv, Rocephin iv, clindamycin iv, Arterenol iv, Disoprivan iv, Beloc iv, MCP iv, Pantozol iv, Lasix iv, Perfalgan iv, ACC iv, neostigmine iv, Sufenta iv, Novalgin iv, Actrapid iv, Peri plasma iv, Ebrantil iv, Lantus sc, Dolantin iv, folic acid iv, vitamin B12 iv, vitamin K iv, potassium oral, Novalgin oral, Kitiva iv, Eugalac oral, Fenistil oral, Clinda

Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnetrans forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

Following an internal review on 10 Aug 2011, of the information received on 15 Jun 2010: Following significant correction was done. Mono-emblex was recoded with Novartis dictionary.

Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

For Mono-Embolex,

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event.

In the absence of the reporting healthcare professional causality, Novartis has processed the case as suspected for reporting purposes. The case will be reassessed on receipt of follow-up information.

MR: The event could have been confounded by pantozol.

Reporter's comment:

NOVARTIS, 22.08.2011 DE-SZ09-PHHY2010DE40691

Sender diagnosis: not reported

Narrative

Initial report received from a physician on 15 Jun 2010: This patient was enrolled in the non-Novartis multinational patient Registry RegiSCAR collecting data about Severe Cutaneous Adverse Reactions (SCAR). This patient with a history of a spondylodiscitis with an psoas abscess was admitted to an intensive care unit on 24 Aug 2008. She was treated with a multiple drug regimen including Pipril iv (piperacillin) for psoasabscess, Sobelin iv (clindamycine) for psoasabscess, Combactam iv (sulbactam) for psoasabscess, Insuman Rapid iv (insulin human) for hyperglycaemia, heparin sodium iv for thrombosis prophylaxis, Haldol iv (haloperidol) for unrest, diazepam iv for unrest, Haes iv (polyhydroxyethyl starch, sodium chloride) for volume substitution, Aminoven iv (isoleucine, leucine, lysine, methionine, phenylalanine) for substitution, Rocephin iv (ceftriaxone) for psoasabscess, Arterenol iv (nor epinephrine) for stabilization of the circulatory system, Disoprivan iv (propofol) for narcosis, Beloc iv (metoprolol) for hypertension, MCP iv (metoclopramide) for nausea, Pantozol iv (pantoprazole) for stomach protection and Lasix iv (furosemide) for diuresis. From 28 Aug 2007 until 29 Aug 2007 the patient had to be transferred to an area for shock treatment. She was also given Dipidolor iv (piritramide) for pain,

potassium chloride iv for substitution, Voluven iv (polyhydroxyethyl starch, sodium chloride) for substitution, NAC oral (acetylcysteine) for secretolysis, Jonosteril iv (sodium chloride, sodium acetate, potassium acetate, calcium acetate, magnesium acetate) for volume substitution, Perfalgan iv (propacetamol) for fever, ACC iv (acetylcysteine) for secretolysis, neostigmine iv for stimulation of the gut, Sufenta iv (sufentanil) for narcosis, Novalgin iv (metamizole) for pain, Actrapid iv (insulin) for hyperglycaemia, PeriPlasma iv (amino acids, glucose, sodium chloride, potassium chloride, magnesium chloride) for volume substitution, hydrocortisone iv for sepsis/stridor and a contrast media for a CT and angiography. On 30 Aug 2007 Catapresan iv (clonidine) for hypertension and Mono-Embolex 3000 IU sc (certoparin) for thrombosis prophylaxis were started. On 31 Aug 2007 Catapresan was stopped, Ebrantil iv (urapidil) for hypertension, Staphylex iv (flucloxacillin) for psoasabscess and rifampicin iv for psoasabscess were started. On 01 Sep Ebrantil was stopped, a single dose of Lantus sc (insulin glargine) for hyperglycaemia, belok zok oral (metoprolol) for hypertension, Norvasc oral (amlodipine) for hypertension and Aldactone oral (spironolactone) for hypertension were started. On 02 Sep the patient received a single dose of Dolantin iv (pethidine) for pain, of folic acid iv, of vitamin B 12 iv and of vitamin K iv for substitution. Furthermore potassium oral and Lasix oral (furosemide) for diuresis were started. Potassium oral was stopped on 03 Sep. On 04 Sep 2007 the patient received single doses of Novalgin oral (metamizole), of Kitiva iv (unknown ingredients) for pain and of Eugalac oral (lactulose) for constipation. On 05 Sep Novalgin oral for pain, insulin sc for hyperglycaemia and Pantozol oral for stomach protection were started. On 06 Sep 2007 a single dose of Solu-Decortin H iv (prednisolone) for stridor and an exanthema was given. Berodual inhalation (ipratropium bromide/fenoterol) for pulmonary obstruction was started. On 07 Sep 2007 Mono-Embolex was stopped and HCT oral (hydrochlorothiazide) for diuresis, Diovan oral (valsartan) for hypertension, Mucosolvan iv (ambroxol) for secretolysis, Tavor oral (lorazepam) for unrest and Saroten oral (amitriptyline) for pain were started. On 08 Sep single doses of Fenistil iv (dimetidine) and erythrocyte concentrate iv for anaemia were given. Also Diclac oral (diclofenac) for pain, Tramal oral (tramadol) for pain and Clexane sc (enoxaparin) for thrombosis prophylaxis were started. On 10 Sep 2007 levothyroxine oral for hypothyroidism was started. On 11 Sep 2007 the patient was given a single dose of Magnettrans forte oral (magnesium oxide) and of vitamin B iv for substitution. Furthermore Adalat oral (nifedipine) for hypertension and sodium chloride iv to keep the injection entry open were started. HCT was stopped on the same day. On 12 Sep 2007 Diovan, Mucosolvan and Diclac were stopped. Inzolen iv (potassium aspartate, magnesium aspartate, copper hydrogenaspartate, zinc aspartate, manganese hydrogenaspartate) for substitution, paracetamol oral for fever, Kalinor oral (potassium citrate, potassium hydrogencarbonate, citric acid) for substitution and Catapresan oral (clonidine) for hypertension were started. On 13 Sep 2007 Tramal was stopped, ambroxol oral for secretolysis was started. On 14 Sep 2007 Novalgin oral, insulin sc, Saroten oral, Adalat oral and Inzolen iv were stopped, Moronal top (nystatin) for prophylaxis of soor and Xaramitis oral (unknown ingredients) were started. The patient was transferred on this day to a surgical unit. On 15 Sep 2007 Perenterol oral (saccharomyces boulardii) for diarrhoea was started. On 16 Sep 2007 Staphylex iv, rifampicin iv and paracetamol oral were stopped, Avalox oral (moxifloxacin) for psoasabscess was started. On 17 Sep 2007 the patient developed fever which lasted until 20 Sep 2007. She was given a single dose of Nitro oral (glyceryl trinitrate) for hypertension. On 18 Sep 2007 ambroxol oral and Moronal oral were stopped. On 19 Sep the patient developed skin lesions. A Stevens-Johnson syndrome was diagnosed. Perenterol and Avalox were stopped. Fenistil oral (dimentidine) for rash was started. On 20 Sep 2007 the patient was given a single dose of Solu- Decortin H iv (prednisolone) for pulmonary obstruction. Kalinor oral and Xaramitis were stopped, Clinda saar oral (clindamycin) for psoasabscess was started. On 21 Sep 2007 the patient developed pruritus, prednisolone oral for exanthema was started. On 22 Sep 2007 the patient developed again fever. On 23 Sep 2007 Novaminsulfon drops (metamizole) for fever was started and was given for two days. On 24 Sep 2007 prednisolone oral was stopped. On 25 Sep 2007 the patient developed also blisters on the skin, Nikolskys sign and redness of the eyes and oral mucosa. The Stevens-Johnson syndrome lasted until 26 Oct 2007. The patient had fully recovered. The causal relationship between the Stevens-Johnson syndrome and the medications was assessed by a dermatology expert panel as follows.: To Pipril iv, Sobelin iv, Combactam iv, Insuman Rapid iv, heparin iv, Haldol iv, diazepam iv, Haes iv, Aminoven iv, Rocephin iv, clindamycin iv, Arterenol iv, Disoprivan iv, Beloc iv, MCP iv, Pantozol iv, Lasix iv, Perfalgan iv, ACC iv, neostigmine iv, Sufenta iv, Novalgin iv, Actrapid iv, PeriPlasma iv, Ebrantil iv, Lantus sc, Dolantin iv, folic acid iv, vitamin B12 iv, vitamin K iv, potassium oral, Novalgin oral, Kitiva iv, Eugalac oral, Fenistil oral, Clinda Saar oral, prednisolone oral and Novaminsulfon oral not related. To potassium chloride iv, Voluven iv, Jonosteril iv, Catapresan iv, Mono-Embolex sc, Solu-Decortin H iv, Fenistil iv, erythrocyte concentrate iv, Magnettrans forte oral, vitamin B iv, sodium chloride iv, Inzolen iv, Kalinor oral, Moronal top, Perenterol oral and Nitro oral as doubtful. To Dipidolor iv, NAC oral, Tavor oral, hydrocortisone iv, contrast media iv, Beloc zok oral, Norvasc oral, Aldactone oral, Lasix oral, insulin sc, Pantozol oral, Berodual inh, HCT oral, Diovan oral, Mucosolvan oral, Saroten oral, Clexane sc, levothyroxine oral, Adalat oral, paracetamol oral, Catapresan oral, ambroxol oral, Xaramitis oral and Avalox oral as possible. To Staphylex iv, rifampicin iv, Novalgin drops oral, Diclac oral and Tramal drops oral as quite possible. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

Following an internal review on 10 Aug 2011, of the information received on 15 Jun 2010: Following significant correction was done. Mono-embolex was recorded with Novartis dictionary.

Sender's comment:

For Diovan and hydrochlorothiazide

Serious adverse drug reaction report: Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet. However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. Reporting healthcare professional causality is suspected.

For potassium chloride

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet . However other alternative causes [other drugs] provide a possible explanation for the reported adverse event. In the absence of a health care professional's causality, the Novartis Medical Safety Physician has provisionally assessed a relationship between the event Stevens Johnson syndrome " and " potassium chloride " as suspected based on the current available information.

For Pantozol

Stevens Johnson syndrome [hospitalization], assessed as listed according to the Core Data Sheet. Reporting healthcare professional causality is suspected.

For Mono-Embolex,

Stevens Johnson syndrome [hospitalization], assessed as unlisted according to the Core Data Sheet .
However other alternative causes [other drugs] provide a possible explanation for the reported adverse event.

In the absence of the reporting healthcare professional causality, Novartis has processed the case as suspected for reporting purposes. The case will be reassessed on receipt of follow-up information.

MR: The event could have been confounded by pantozol.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10039985

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20070919	20071026	38 Day	19 Day	recovered/resolved
2 Stevens-Johnson syndrome	20070919	20071026	38 Day	19 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
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1	ADALAT nifedipine	Oral	10 Mg milligram(s)/Day	20070911- -	Hypertension arterial
2	ADALAT nifedipine	Oral	5 Mg milligram(s)/Day	-20070914 -	
3	ALDACTONE spironolactone	Oral	100 Mg milligram(s)/Day	20070901- -	Hypertension arterial
4	avalox moxifloxacin hydrochloride	Oral	400 Mg milligram(s)/Day	20070916-20070919 4 Day	Psoas abscess
5	BELOC ZOK metoprolol succinate	Oral	/	20070901- -	Hypertension arterial
6	BERODUAL fenoterol hydrobromide; ipratropium bromide	Respiratory (inhalation)	/	20070906- -	Pulmonary disorder
7	catapresan clonidine	Intravenous (not otherwis	150 Mg milligram(s)/	20070830-20070831 2 Day	Hypertension arterial
8	catapresan clonidine	Intravenous (not otherwis	150 Mg milligram(s)/	20070907-20070907 1 Day	
9	catapressan clonidine	Oral	450 Mg milligram(s)/Day	20070912- -	Hypertension arterial
10	CLEXANE heparin-fraction, sodium salt	Subcutaneous	/	20070908- -	Thrombosis prophylaxis
11	diclac diclofenac sodium	Oral	150 Mg milligram(s)/Day	20070908-20070912 5 Day	Pain
12	DIOVAN "NOVARTIS" valsartan	Oral	80 Mg milligram(s)/Day	20070907-20070912 6 Day	Hypertension arterial
13	DIPIDOLOR piritramide	Intravenous (not otherwis	8 Mg milligram(s)/Day	20070908-20070909 2 Day	
14	DIPIDOLOR piritramide	Intravenous (not otherwis	6 Mg milligram(s)/Day	20070827-20070828 2 Day	Pain
15	DIPIDOLOR piritramide	Intravenous (not otherwis	9 Mg milligram(s)/Day	20070830-20070905 7 Day	
16	INZOLEN aspartate zinc; cobalt hydrogenaspartate; copper hydrogenaspartate; magnesium aspartate; manganese hydrogenaspartate; potassium hydrogen aspartate	Intravenous (not otherwis /		20070912-20070914 3 Day	Supplementation therapy
17	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070923-20070923 1 Day	
18	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070920-20070921 2 Day	
19	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070824-20070903 11 Day	Supplementation therapy
20	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070905-20070911 7 Day	
21	JONOSTERIL calcium acetate; magnesium acetate; potassium acetate; sodium acetate; sodium chloride	Intravenous (not otherwis /		20070914-20070916 3 Day	
22	kalinor potassium chloride	Oral	/	20070912-20070920 9 Day	Supplementation therapy
23	LASIX furosemide	Oral	40 Mg milligram(s)/Day	- -	
24	LASIX furosemide	Oral	160 Mg milligram(s)/Day	20070902- -	Diuresis
25	L-THYROXIN levothyroxine sodium	Oral	75 Mg milligram(s)/Day	20070910-	

				-	
26	magnetrans forte magnesium oxide	Oral	600 Mg milligram(s)/Day	20070911-20070911	Supplementation therapy 1 Day
27	MONO-EMBOLEX heparin-fraction, sodium salt	Subcutaneous	3000 Iu international unit(s)	20070830-20070907	Thrombosis prophylaxis 9 Day
28	moronal nystatin	Topical	/	20070914-20070918	Prophylaxis 5 Day
29	MUCOSOLVAN ambroxol hydrochloride	Intravenous (not otherwise)	/	20070907-20070912	Secretion discharge 6 Day
30	nitro glyceryl trinitrate	Sublingual	/	20070917-20070917	Hypertension arterial 1 Day
31	Norvasc amlodipine besilate	Oral	10 Mg milligram(s)/Day 245	20070901-	Hypertension arterial -
32	NOVALGIN metamizole	Oral	160 Gtt drop(s)/Day	20070905-20070914	Pain 10 Day
33	PANTOZOL pantoprazole sodium	Oral	20 Mg milligram(s)/	20070905-	Gastric ulcer prophylaxis -
34	PERENTEROL yeast dried	Oral	/	20070915-20070919	Diarrhea 5 Day
35	SAROTEN amitriptyline hydrochloride	Oral	75 Mg milligram(s)/Day	20070907-20070914	Pain 8 Day
36	solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwise)	250 Mg milligram(s)/	20070906-20070906	Stridor 1 Day
37	solu-decortin-h prednisolone sodium succinate	Intravenous (not otherwise)	250 Mg milligram(s)/	20070920-20070920	Exanthema 1 Day
38	staphylex flucloxacillin sodium	Intravenous (not otherwise)	6 G gram(s)/Day	20070831-20070916	Psoas abscess 17 Day
39	tavor lorazepam	Oral	1 Mg milligram(s)/Day	20070922-20070922	 1 Day
40	tavor lorazepam	Oral	1 Mg milligram(s)/Day	20070918-20070918	 1 Day
41	tavor lorazepam	Oral	1 Mg milligram(s)/Day	20070907-20070915	 9 Day
42	tavor lorazepam	Oral	1 Mg milligram(s)/Day	20070827-	Unrest -
43	tavor lorazepam	Oral	1 Mg milligram(s)/Day	20070920-20070920	 1 Day
44	TRAMAL tramadol hydrochloride	Oral	40 Gtt drop(s)/Day	20070908-20070909	Pain 2 Day
45	TRAMAL tramadol hydrochloride	Oral	60 Gtt drop(s)/Day	20070912-20070913	 2 Day
46	VITAMIN B COMPLEX calcium pantothenate; nicotinamide; pyridoxine hydrochloride; riboflavin; thiamine hydrochloride	Intravenous (not otherwise)	/	20070911-20070911	Supplementation therapy 1 Day
47	VOLUVEN hetastarch	Intravenous (not otherwise)	500 ml millilitre(s)/	20070827-20070829	Supplementation therapy 3 Day
48	VOLUVEN hetastarch	Intravenous (not otherwise)	500 ml millilitre(s)/	20070907-20070914	 8 Day

Drugs (concomitant)

49	acc acetylcysteine	Intravenous (not otherwis 1200 Mg milligram(s)/Day	20070829-20070829	Secretion discharge 1 Day
50	ACTRAPID insulin	Intravenous (not otherwis /	20070829-20070905	Hyperglycemia 8 Day
51	aminoven amino acids nos	Intravenous (not otherwis /	20070828-20070828	Supplementation therapy 1 Day
52	ARTERENOL norepinephrine hydrochloride	Intravenous (not otherwis /	20070828-20070831	Circulatory insufficiency 4 Day
53	BELOC metoprolol tartrate	Intravenous (not otherwis 5 Mg milligram(s)/Day	20070828-20070828	Hypertension arterial 1 Day
54	BELOC metoprolol tartrate	Intravenous (not otherwis 5 Mg milligram(s)/	20070830-20070901	 3 Day
55	COMBACTAM sulbactam sodium	Intravenous (not otherwis 2 G gram(s)/Day	20070827-20070828	Psoas abscess 2 Day
56	DISOPRIVAN "ASTRA ZENECA" propofol	Intravenous (not otherwis /	20070828-20070831	Narcosis 4 Day
57	DOLANTIN pethidine hydrochloride	Intravenous (not otherwis /	20070902-20070902	Pain 1 Day
58	EBRANTIL FOR INJECTION urapidil hydrochloride	Intravenous (not otherwis 250 Mg milligram(s)/	20070831-20070901	Hypertension arterial 2 Day
59	EUGALAC lactulose	Oral 20 ml millilitre(s)/Day	20070904-20070905	Constipation 2 Day
60	haes-steril hetastarch; sodium chloride	Intravenous (not otherwis 6 % percent/	20070828-20070828	Supplementation therapy 1 Day
61	HALDOL haloperidol	Intravenous (not otherwis /	20070827-20070827	Unrest 1 Day
62	HALDOL haloperidol	Intravenous (not otherwis /	20070828-20070828	 1 Day
63	insuman rapid insulin human; isophane insulin	Intravenous (not otherwis 8 lu international unit(s)/	20070827-20070828	Hyperglycemia 2 Day
64	KALIUM "VERLA" potassium citrate	Oral /	20070902-20070903	Supplementation therapy 2 Day
65	LANTUS insulin glargine	Subcutaneous /	20070901-20070901	Hyperglycemia 1 Day
66	LASIX furosemide	Intravenous (not otherwis /	20070828-20070905	Diuresis 9 Day
67	MULTIBIONTA ascorbic acid; calcium pantothenate; cyanocobalamin; nicotinamide; pyridoxine hydrochloride; retinol palmitate; riboflavin; rutoside; thiamine hydrochloride; tocopheryl acetate	Intravenous (not otherwis /	20070902-20070902	Parenteral nutrition 1 Day
68	NOVALGIN metamizole	Oral 3 G gram(s)/Day	20070904-20070904	Pain 1 Day
69	NOVALGIN metamizole	Intravenous (not otherwis 3 G gram(s)/Day	20070829-20070904	 7 Day
70	NOVALGIN metamizole	Intravenous (not otherwis 3 G gram(s)/Day	20070824-	Pain -
71	PANTOZOL pantoprazole sodium	Intravenous (not otherwis 40 Mg milligram(s)/Day	20070827-20070901	Gastric ulcer prophylaxis 6 Day
72	PERIPLASMAL amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis /	20070903-20070905	 3 Day
73	PERIPLASMAL amino acids nos; electrolytes nos; sorbitol; xylitol	Intravenous (not otherwis /	20070829-20070831	Supplementation therapy 3 Day
74	PIPRIL piperacillin sodium	Intravenous (not otherwis 8 G gram(s)/Day	20070827-20070828	Psoas abscess 2 Day

Drugs (concomitant)

75	ROCEPHIN ceftriaxone sodium	Intravenous (not otherwis 4 G gram(s)/Day	20070828-20070831	Psoas abscess 4 Day
76	sobelin clindamycin hydrochloride	Intravenous (not otherwis 1800 Mg milligram(s)/Day	20090828-20090828	Psoas abscess 1 Day
77	SUFENTA sufentanil citrate	Intravenous (not otherwis /	20070829-20070831	Narcosis 3 Day
78	VITAMIN B12 cyanocobalamin	Intravenous (not otherwis /	20070902-20070902	Supplementation therapy 1 Day
79	VITAMIN K phytomenadione	Intravenous (not otherwis /	20070902-20070902	Supplementation therapy 1 Day

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Restlessness			Unknown	
Sepsis			Yes	
Intervertebral discitis			Yes	
Spondylodiscitis			Unknown	
Psoas abscess			Unknown	
Hyperglycemia			Unknown	
Unrest			Unknown	
Narcosis			Unknown	
Hypertension arterial			Unknown	
Nausea			Unknown	
Diuresis			Unknown	
Pain			Unknown	
Bronchial secretion retention			Unknown	
Fever			Unknown	
Parenteral nutrition			Unknown	
Constipation			Unknown	
Hypothyroidism			Yes	
Respiratory obstruction unspecified			Yes	

Primary source

Qualification: Other Health Professional observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 23.03.2010 DE-PFIZER INC-2010031955

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the Documentation Center for Severe Skin Reactions (dZh):9170608.

A contactable physician reported that this 70-year old female patient was hospitalized from 24Aug2007 to 28Aug2007 in the intensive care unit (ICU). From 28Aug2007 to 29Aug2007 she was in the resuscitation room, from 29Aug2007 to 14Sep2007 she was again in the ICU and from 14Sep2007 to 25Sep2007 she was in the trauma surgery unit. Reason for hospitalisation was not provided. The patient was treated with the following suspect drugs: oral amlodipine besilate (NORVASC) 10 mg once daily due to arterial hypertension from 01Sep2007 onwards, intravenous hydrocortisone sodium succinate (HYDROCORTISONE) 140 mg, 200 mg, 50 mg, 25 mg (as reported) due to sepsis and stridor from 29Aug2007 to 01Sep2007 and from 07Sep2007 to 14Sep2007, oral acetylcysteine (ACETYLCYSTEINE) 300 mg 3x/day from 27Aug2007 to 28Aug2007 and 2x300 mg daily from 05Sep2007 to 18Sep2007 for secretolysis, oral lorazepam (TAVOR) 1 mg/day due to unrest from 27Aug2007 onwards (no stop date provided), from 07Sep2007 to 15Sep2007, on 18Sep2007, on 20Sep2007 and on 22Sep2007, intravenous flucloxacillin sodium (STAPHYLEX) 3 x 2 g daily due to psoas abscess from 31Aug2007 to 16Sep2007, intravenous rifampicin (RIFAMPICIN) 600 mg/day due of psoas abscess from 31Aug2007 to 16Sep2007, intravenous potassium chloride (POTASSIUM CHLORIDE) for substitution from 27Aug2007 to 13Sep2007, subcutaneous heparin-fraction, sodium salt (MONO-EMBOLEX) for thrombosis prophylaxis from 30Aug2007 to 07Sep2007, oral metoprolol succinate (BELOC-ZOK) twice daily due to arterial hypertension from 01Sep2007 onwards, oral spironolactone (ALDACTONE) 100 mg once daily due to arterial hypertension from 01Sep2007 onwards, intravenous prednisolone sodium succinate (SOLU-DECORTIN-H) 250 mg due to stridor and exanthema on 6Sep2007 and on 20Sep2007, subcutaneous insulin (INSULIN) due to hyperglycemia from 05Sep2007 to 14Sep2007, oral furosemide (LASIX) starting with 4 x 40

mg/day (reduced on an unknown date to 40 mg/day) for diuresis from 02Sep2007 onwards, respiratory fenoterol hydrobromide, ipratropium bromide (BERODUAL) 2 puffs in the morning, 2 puffs in the evening due to pulmonary obstruction from 06Sep2007 onwards, oral hydrochlorothiazide (HYDROCHLOROTHIAZIDE) 25 mg/day from 07Sep2007 to 11Sep2007, intravenous ambroxol hydrochloride (MUCOSOLVAN) for secretolysis from 07Sep2007 to 12Sep2007, oral diclofenac sodium (DICLAC) 75 mg 2x/day due to pain from 08Sep2007 to 12Sep2007, intravenous piritramide (DIPIDOLOR) due to pain 2 mg 3x/day on 27Aug2007, 3 mg 3x/day from 30Aug2007 to 05Sep2007 and 2 mg 4x/day from 08Sep2007 to 09Sep2007, intravenous hetastarch (VOLUVEN) 500 ml for substitution from 27Aug2007 to 29Aug2007 and from 07Sep2007 to 14Sep2007, oral levothyroxine sodium (L-THYROXIN) 75 mg/day due to hypothyroidism from 10Sep2007 onwards, oral magnesium oxide (MAGNETRANS FORTE) 2x 300 mg in the morning, 300 mg in the evening for substitution on 11Sep2007, oral nifedipine (ADALAT) 5 mg twice (reduced to 5 mg/day on an unknown date in Sep2007) due to arterial hypertension from 11Sep2007 to 14Sep2007, intravenous sodium chloride (SODIUM CHLORIDE) to keep the injection entry open from 11Sep2007 onwards, oral paracetamol (PARACETAMOL) 1 g 2x/day (increased on an unknown date in Sep2007 to 1 g 3x/day) due to fever from 12Sep2007 to 16Sep2007, oral ambroxol (AMBROXOL) 15 mg three times daily for secretolysis from 13Sep2007 to 18Sep2007, oral pantoprazole sodium (PANTOZOL) 2x20 mg in the morning for stomach protection from 05Sep2007 onwards, oral tramadol hydrochloride (TRAMAL) due to pain 20 Gtt, 2x/day from 08Sep2007 to 09Sep2007 and 20 Gtt, 3x/day from 12Sep2007 to 13Sep2007, intravenous calcium acetate, magnesium acetate, potassium acetate, sodium acetate, sodium chloride (JONOSTERIL) for substitution of volume from 24Aug2007 to 03Sep2007, from 05Sep2007 to 11Sep2007, from 14Sep2007 to 16Sep2007, from 20Sep2007 to 21Sep2007 and on 23Sep2007, intravenous CONTRAST MEDIA for computerized tomography/angiography on 29Aug2007, 04Sep2007, 11Sep2007, 21Sep2007 and on 23Sep2007, intravenous clonidine (CATAPRESAN) 150 mg due to arterial hypertension from 30Aug2007 to 31Aug2007 and on 07Sep2007, oral metamizole sodium (NOVALGIN) drops due to pain 40 Gtt 4x/day from 05Sep2007 to 14Sep2007, oral valsartan (DIOVAN) due to arterial hypertension 80 mg daily from 07Sep2007 to 12Sep2007, oral amitriptyline hydrochloride (SAROTEN) due to pain 75 mg daily from 07Sep2007 to 14Sep2007, subcutaneous heparin-fraction, sodium salt (CLEXANE) for thrombosis prophylaxis from 08Sep2007 onwards, intravenous calcium pantothenate, nicotinamide, pyridoxine hydrochloride, riboflavin, thiamine hydrochloride (VITAMIN B COMPLEX) for substitution on 11Sep2007, intravenous aspartate zinc, cobalt hydrogenaspartate, copper hydrogenaspartate, magnesium aspartate, manganese hydrogenaspartate, potassium hydrogen aspartate (INZOLEN) for substitution from 12Sep2007 to 14Sep2007, oral potassium chloride (KALINOR) twice daily for substitution from 12Sep2007 to 20Sep2007, oral clonidine (CATAPRESAN) due to arterial hypertension 150 mg, 3x/day from 12Sep2007 onwards, topical nystatin (MORONAL) for prophylaxis of thrush from 14Sep2007 to 18Sep2007, oral (unknown ingredients) (XARAMITIS) (unknown indication) 40 (unk) 4x/day from 14Sep2007 to 20Sep2007, oral yeast dried (PERENTEROL) due to diarrhea (2 in the morning, 2 at noon, 2 in the evening) from 15Sep2007 to 19Sep2007, oral moxifloxacin (AVALOX) 400 mg once daily due to psoas abscess from 16Sep2007 to 19Sep2007, oral (per report, but probably sublingual) trinitrate (NITRO) 2 puffs due to arterial hypertension on 17Sep2007 and intravenous red blood cell concentrate due to anemia on 08Sep2007. Concomitant medication included: piperacillin sodium (PIPRIL), clindamycin hydrochloride (SOBELIN), sulbactam sodium (COMBACTAM), insulin human (INSUMAN RAPID), heparin (HEPARIN), haloperidol (HALDOL), diazepam (DIAZEPAM), hyetellose, sodium chloride (HAES-STERIL), amino acids nos (AMINOVEN), ceftriaxone sodium (ROCEPHIN), clindamycin (CLINDAMYCIN), norepinephrine hydrochloride (ARTERENOL), propofol (DISOPRIVAN), metoprolol tartrate (BELOC), metoclopramide (METOCLOPRAMIDE), pantoprazole sodium (PANTOZOL), furosemide (LASIX), propacetamol (PERFALGAN), acetylcysteine (ACC), neostigmine (NEOSTIGMINE), sufentanil citrate (SUFENTA), ascorbic acid, calcium pantothenate, cyanocobalamin, nicotinamide, pyridoxine hydrochloride, retinol palmitate, riboflavin, rutoside, thiamine hydrochloride, tocopheryl acetate (MULTIBIONTA), metamizole sodium (NOVALGIN), insulin (ACTRAPID), amino acids nos, electrolytes nos, sorbitol, xylitol (PERIPLASMAL), urapidil hydrochloride (EBRANTIL FOR INJECTION), insulin glargine (LANTUS), pethidine hydrochloride (DOLANTIN), folic acid (FOLIC ACID), cyanocobalamin (VITAMIN B12), phytomenadione (VITAMIN K), potassium citrate (KALIUM VERLA), metamizole sodium (NOVALGIN), unknown ingredient (KITIVA), lactulose (EUGALAC) and an unknown drug NOS. The patient had taken the following drugs in the past (therapy dates and indications unknown): piritramide (DIPIDOLOR), which was well tolerated without adverse event, lorazepam (TAVOR), hydrocortisone sodium succinate (HYDROCORTISONE), heparin-fraction, sodium salt (MONO-EMBOLEX), metamizole sodium (NOVALGIN), pantoprazole (PANTOZOL), tramadol hydrochloride (TRAMAL), levothyroxine sodium (L-Thyroxin). The occurrence of any adverse reactions related to these medicines is unknown. The patient had a medical history of spondylodiscitis, psoas abscess, hyperglycemia, unrest, narcosis, arterial hypertension, nausea, diuresis, pain, secretolysis, parenteral nutrition, and constipation. On 17Sep2007 the patient experienced fever lasting until 20Sep2007. On 19Sep2007 spots occurred and Stevens-Johnson-Syndrome was diagnosed on the same day. The event was not considered life-threatening by the reporting experts. On the next days the following symptoms occurred: pruritus on 21Sep2007, fever on 22Sep2007 and blisters of skin, Nikolsky's sign and redness of eyes and of oral mucosa occurred on 25Sep2007. The following suspect drugs were continued unchanged: amlodipine besilate, metoprolol succinate, spironolactone, furosemide, fenoterol hydrobromide, ipratropium bromide, levothyroxine sodium, sodium chloride, pantoprazole sodium, heparin-fraction, potassium chloride, clonidine and XARAMITIS. Yeast dried and moxifloxacin were discontinued on the day Stevens-Johnson-Syndrome was diagnosed. After 19Sep2007 the patient was treated additionally with the following oral drugs: dimetindene (FENISTIL), clindamycin (CLINDAMYCIN), prednisolone (PREDNISOLON) and metamizole sodium (NOVALGIN). On 26Oct2007 the patient completely recovered from Stevens-Johnson-Syndrome. Observation period including information on medication was available from 28Aug2007 until 25Sep2007. In Sep2008 the experts assessed the causal relationship as very possible with flucloxacillin sodium, rifampicin, metamizole sodium (drops), diclofenac sodium and tramadol hydrochloride, as possible with amlodipine besilate, hydrocortisone sodium succinate, piritramide, acetylcysteine, lorazepam, CONTRAST MEDIA, metoprolol succinate, spironolactone, furosemide, insulin, pantoprazole sodium, fenoterol hydrobromide, ipratropium bromide, hydrochlorothiazide, valsartan, ambroxol hydrochloride, amitriptyline hydrochloride, heparin-fraction, sodium salt, levothyroxine sodium, nifedipine, paracetamol, clonidine, XARAMITIS and moxiflocacin, as dubious with potassium chloride, hetastarch, calcium acetate/magnesium acetate/potassium acetate/sodium acetate/sodium chloride, dimetindene, clonidine, heparin-fraction, sodium salt, prednisolone, red blood cell concentrate, magnesium oxide, calcium pantothenate, nicotinamide, pyridoxine hydrochloride, riboflavin, thiamine hydrochloride, sodium chloride, aspartate zinc, cobalt hydrogenaspartate, copper hydrogenaspartate, magnesium aspartate, manganese hydrogenaspartate, potassium hydrogen aspartate, potassium chloride, nystatin, saccharomyces boulardii and glyceryl trinitrate. Per project experts a relationship spondylodiscitis with psoas abscess was also possible.

Follow-up status: case closed (12Mar2010).

Sender's comment:

Due to the several medications the patient was taking, a causal relationship for each of them in inducing the reported events can not be established. Bei Stevens-Johnson-Syndrom handelt es sich um eine fuer Amlodipin bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung. Stevens-Johnson-Syndrom ist in der Produktinformation von Hydrocortison als moegliche Nebenwirkung nicht beschrieben. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

PFIZERINC, 11.01.2011 DE-PFIZER INC-2010031955

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the Documentation Center for Severe Skin Reactions (dZh):9170608.

A contactable physician reported that this 70-year old female patient was hospitalized from 24Aug2007 to 28Aug2007 in the intensive care unit (ICU). From 28Aug2007 to 29Aug2007 she was in the resuscitation room, from 29Aug2007 to 14Sep2007 she was again in the ICU and from 14Sep2007 to 25Sep2007 she was in the trauma surgery unit. Reason for hospitalisation was not provided. The patient was treated with the following suspect drugs: oral amlodipine besilate (NORVASC) 10 mg once daily due to arterial hypertension from 01Sep2007 onwards, intravenous hydrocortisone sodium succinate (HYDROCORTISONE) 140 mg, 200 mg, 50 mg, 25 mg (as reported) due to sepsis and stridor from 29Aug2007 to 01Sep2007 and from 07Sep2007 to 14Sep2007, oral acetylcysteine (ACETYLCYSTEINE) 300 mg 3x/day from 27Aug2007 to 28Aug2007 and 2x300 mg daily from 05Sep2007 to 18Sep2007 for secretolysis, oral lorazepam (TAVOR) 1 mg/day due to unrest from 27Aug2007 onwards (no stop date provided), from 07Sep2007 to 15Sep2007, on 18Sep2007, on 20Sep2007 and on 22Sep2007, intravenous flucloxacillin sodium (STAPHYLEX) 3 x 2 g daily due to psoas abscess from 31Aug2007 to 16Sep2007, intravenous rifampicin (RIFAMPICIN) 600 mg/day due of psoas abscess from 31Aug2007 to 16Sep2007, intravenous potassium chloride (POTASSIUM CHLORIDE) for substitution from 27Aug2007 to 13Sep2007, subcutaneous heparin-fraction, sodium salt (MONO-EMBOLEX) for thrombosis prophylaxis from 30Aug2007 to 07Sep2007, oral metoprolol succinate (BELOC-ZOK) twice daily due to arterial hypertension from 01Sep2007 onwards, oral spironolactone (ALDACTONE) 100 mg once daily due to arterial hypertension from 01Sep2007 onwards, intravenous prednisolone sodium succinate (SOLU-DECORTIN-H) 250 mg due to stridor and exanthema on 6Sep2007 and on 20Sep2007, subcutaneous insulin (INSULIN) due to hyperglycemia from 05Sep2007 to 14Sep2007, oral furosemide (LASIX) starting with 4 x 40 mg/day (reduced on an unknown date to 40 mg/day) for diuresis from 02Sep2007 onwards, respiratory fenoterol hydrobromide, ipratropium bromide (BERODUAL) 2 puffs in the morning, 2 puffs in the evening due to pulmonary obstruction from 06Sep2007 onwards, oral hydrochlorothiazide (HYDROCHLOROTHIAZIDE) 25 mg/day from 07Sep2007 to 11Sep2007, intravenous ambroxol hydrochloride (MUCOSOLVAN) for secretolysis from 07Sep2007 to 12Sep2007, oral 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times daily for secretolysis from 13Sep2007 to 18Sep2007, oral pantoprazole sodium (PANTOZOL) 2x20 mg in the morning for stomach protection from 05Sep2007 onwards, oral tramadol hydrochloride (TRAMAL) due to pain 20 Gtt, 2x/day from 08Sep2007 to 09Sep2007 and 20 Gtt, 3x/day from 12Sep2007 to 13Sep2007, intravenous calcium acetate, magnesium acetate, potassium acetate, sodium acetate, sodium chloride (JONOSTERIL) for substitution of volume from 24Aug2007 to 03Sep2007, from 05Sep2007 to 11Sep2007, from 14Sep2007 to 16Sep2007, from 20Sep2007 to 21Sep2007 and on 23Sep2007, intravenous CONTRAST MEDIA for computerized tomography/angiography on 29Aug2007, 04Sep2007, 11Sep2007, 21Sep2007 and on 23Sep2007, intravenous clonidine (CATAPRESAN) 150 mg due to arterial hypertension from 30Aug2007 to 31Aug2007 and on 07Sep2007, oral metamizole sodium (NOVALGIN) drops due to pain 40 Gtt 4x/day from 05Sep2007 to 14Sep2007, oral valsartan (DIOVAN) due to arterial hypertension 80 mg daily from 07Sep2007 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diarrhea (2 in the morning, 2 at noon, 2 in the evening) from 15Sep2007 to 19Sep2007, oral moxifloxacin (AVALOX) 400 mg once daily due to psoas abscess from 16Sep2007 to 19Sep2007, oral (per report, but probably sublingual) trinitrate (NITRO) 2 puffs due to arterial hypertension on 17Sep2007 and intravenous red blood cell concentrate due to anemia on 08Sep2007. Concomitant medication included: piperacillin sodium (PIPRIL), clindamycin hydrochloride (SOBELIN), sulbactam sodium (COMBACTAM), insulin human (INSUMAN RAPID), heparin (HEPARIN), haloperidol (HALDOL), diazepam (DIAZEPAM), hyetellose, sodium chloride (HAES-STERIL), amino acids

nos (AMINOVEN), ceftriaxone sodium (ROCEPHIN), clindamycin (CLINDAMYCIN), norepinephrine hydrochloride (ARTERENOL), propofol (DISOPRIVAN), metoprolol tartrate (BELOC), metoclopramide (METOCLOPRAMIDE), pantoprazole sodium (PANTOZOL), furosemide (LASIX), propacetamol (PERFALGAN), acetylcysteine (ACC), neostigmine (NEOSTIGMINE), sufentanil citrate (SUFENTA), ascorbic acid, calcium pantothenate, cyanocobalamin, nicotinamide, pyridoxine hydrochloride, retinol palmitate, riboflavin, rutoside, thiamine hydrochloride, tocopheryl acetate (MULTIBIONTA), metamizole sodium (NOVALGIN), insulin (ACTRAPID), amino acids nos, electrolytes nos, sorbitol, xylitol (PERIPLASMAL), urapidil hydrochloride (EBRANTIL FOR INJECTION), insulin glargine (LANTUS), pethidine hydrochloride (DOLANTIN), folic acid (FOLIC ACID), cyanocobalamin (VITAMIN B12), phytomenadione (VITAMIN K), potassium citrate (KALIUM VERLA), metamizole sodium (NOVALGIN), unknown ingredient (KITIVA), lactulose (EUGALAC) and an unknown drug NOS. The patient had taken the following drugs in the past (therapy dates and indications unknown): piritramide (DIPIDOLOR), which was well tolerated without adverse event, lorazepam (TAVOR), hydrocortisone sodium succinate (HYDROCORTISONE), heparin-fraction, sodium salt (MONO-EMBOLEX), metamizole sodium (NOVALGIN), pantoprazole (PANTOZOL), tramadol hydrochloride (TRAMAL), levothyroxine sodium (L-Thyroxin). The occurrence of any adverse reactions related to these medicines is unknown. The patient had a medical history of spondylodiscitis, psoas abscess, hyperglycemia, unrest, narcosis, arterial hypertension, nausea, diuresis, pain, secretolysis, parenteral nutrition, and constipation. On 17Sep2007 the patient experienced fever lasting until 20Sep2007. On 19Sep2007 spots occurred and Stevens-Johnson-Syndrome was diagnosed on the same day. The event was not considered life-threatening by the reporting experts. On the next days the following symptoms occurred: pruritus on 21Sep2007, fever on 22Sep2007 and blisters of skin, Nikolsky's sign and redness of eyes and of oral mucosa occurred on 25Sep2007. The following suspect drugs were continued unchanged: amlodipine besilate, metoprolol succinate, spironolactone, furosemide, fenoterol hydrobromide, ipratropium bromide, levothyroxine sodium, sodium chloride, pantoprazole sodium, heparin-fraction, potassium chloride, clonidine and XARAMITIS. Yiest dried and moxifloxacin were discontinued on the day Stevens-Johnson-Syndrome was diagnosed. After 19Sep2007 the patient was treated additionally with the following oral drugs: dimetindene (FENISTIL), clindamycin (CLINDAMYCIN), prednisolone (PREDNISOLON) and metamizole sodium (NOVALGIN). On 26Oct2007 the patient completely recovered from Stevens-Johnson-Syndrome. Observation period including information on medication was available from 28Aug2007 until 25Sep2007. In Sep2008 the experts assessed the causal relationship as very possible with flucloxacillin sodium, rifampicin, metamizole sodium (drops), diclofenac sodium and tramadol hydrochloride, as possible with amlodipine besilate, hydrocortisone sodium succinate, piritramide, acetylcysteine, lorazepam, CONTRAST MEDIA, metoprolol succinate, spironolactone, furosemide, insulin, pantoprazole sodium, fenoterol hydrobromide, ipratropium bromide, hydrochlorothiazide, valsartan, ambroxol hydrochloride, amitriptyline hydrochloride, heparin-fraction, sodium salt, levothyroxine sodium, nifedipine, paracetamol, clonidine, XARAMITIS and moxiflocacin, as dubious with potassium chloride, hetastarch, calcium acetate/magnesium acetate/potassium acetate/sodium acetate/sodium chloride, dimetindene, clonidine, heparin-fraction, sodium salt, prednisolone, red blood cell concentrate, magnesium oxide, calcium pantothenate, nicotinamide, pyridoxine hydrochloride, riboflavin, thiamine hydrochloride, sodium chloride, aspartate zinc, cobalt hydrogenaspartate, copper hydrogenaspartate, magnesium aspartate, manganese hydrogenaspartate, potassium hydrogen aspartate, potassium chloride, nystatin, saccharomyces boulardii and glyceryl trinitrate. Per project experts a relationship spondylodiscitis with psoas abscess was also possible.

Follow-up status: case closed (12Mar2010).

Follow up (17Dec2010): Case 2010031955 and case DE-WYE-G05756510 are found to be duplicates resulting from case sharing between Pfizer and Wyeth after the merger in Oct2009. All subsequent follow-up information will be reported under 2010031955 which will be the retained case. Case DE-WYE-G05756510 is invalid and will be deleted from the database.

Sender's comment:

Due to the several medications the patient was taking, a causal relationship for each of them in inducing the reported events can not be established. The follow-up information received does not alter the previous company clinical evaluation. Stevens-Johnson-Syndrom ist fuer Amlodipin (NORVASC) eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung, fuer TAVOR und Hydrocortison nicht. Die Anzeige des Follow-up Reports erfolgt nur wegen Duplikat-Zusammenfuehrung (Zusammenfuehrung der Wyeth und Pfizer AMS-Datenbanken); keine Zusatzinformationen. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

RANBAXY, 21.09.2011 DE-RANBAXY-2010US-45778

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2010US-45778 from GERMANY refers to a 70 Years-old Female, who experience Stevens-Johnson syndrome on 19-SEP-2007 after consuming Paracetamol UNK, UNK on 12-SEP-2007 for fever. The event required hospitalization.

The case was received by Pfizer (ref. no. 2010031955) / Wyeth (ref. no.

DE-WYE-005756510) Germany on 12-MAR-2010 from the German Documentation centre for severe skin reactions (RegiSCAR) and was forwarded to Nycomed on 17-MAR-2010.

It concerned a 70-year-old female patient started therapy with Pantozol (pantoprazole), 40 mg daily, intravenous, on 28-AUG-2007, for gastric ulcer prophylaxis. On 05-SEP-2007, intravenous pantoprazole was switched to oral pantoprazole, 20 mg twice daily. Additional suspect medications were piperacillin, sulbactam, ceftriaxone, piritramide, acetylcysteine, metamizole, hydrocortisone, clonidine, flucloxacillin, rifampicin, metoprolol, amlodipine besilate), potassium canrenoate, insulin, fenoterol /ipratropium, hydrochlorothiazide, valsartan, ambroxol, amitriptyline, diclofenac, tramadol heparin-fraction, levothyroxine sodium, Nifedipine, paracetamol and moxifloxacin.

The patient had a medical history of restlessness, intervertebral discitis, psoas abscess, hypertension, pain, hypothyroidism, unspecified respiratory obstruction, hyperglycaemia, diuresis and fever. She was concomitantly treated with clindamycin, human insulin, haloperidol, diazepam, furosemide, neostigmine, urapidil and prednisolone, amongst other numerous medications. From 19-SEP-2007 (22 days after the first dose of pantoprazole), the patient suffered from exanthema. On 21-SEP-2007, she experienced pruritus and on 22-SEP-2007, she presented with fever. On 25-SEP-2007, she was diagnosed with Stevens-Johnson syndrome. At that time she was additionally suffered from vesicles on the skin, redness of eyes and as well as redness of oral mucosa and Nikolskys signs. The patient was hospitalized. No details regarding clinical course or treatment were provided.

Action taken concerning therapy with pantoprazole was unknown.

The patient recovered on 26-OCT-2007.

The investigator considered Stevens-Johnson syndrome possibly related to pantoprazole and all other suspect medications.

The outcome of the event Stevens-Johnson syndrome was reported as Resolved on 26-OCT-2007 at the time of this report.

Case outcome: Resolved.

The case was deemed serious. The event Stevens-Johnson syndrome was found to be unexpected as per PI. Medical Reviewer considered the case to be possibly related to suspect drug due to its temporal association as per WHO UMC system for standardized causality assessment.

Sender's comment:

The case was deemed serious. The event Stevens-Johnson syndrome was found to be unexpected as per PI. Medical Reviewer considered the case to be possibly related to suspect drug due to its temporal association as per WHO UMC system for standardized causality assessment.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-10041735

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 29 Year	weight (kg): 72	BMI_int: 24.91	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Lyell's syndrome	19921229				fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren diclofenac sodium	Topical	/ 95	19921228-19921228	Arthralgia
2 voltaren diclofenac sodium	Oral	/ 94	19921224-19921224	Pain in joint
			1 Day	

Drugs (concomitant)

3 BUPIVACAIN bupivacaine	Intra-articular	/ 231	19921227-19921227	Pain in joint
4 TRIAMCINOLONE triamcinolone	Intra-articular	/ 231	19921227-19921227	Pain in joint
			1 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:	ARZNEIMITTELINDUZIERTES LYELL-SYNDROM. ARZTRECHTLICHE ASPEKTE. (DRUG-INDUCED LYELL-SYNDROME. MEDICAL LEGAL ASPECTS) . MED. WELT. 1995;46:280-284	
Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 26.03.2010 DE-002147023-PHBS1993DE03836

Sender diagnosis: not reported

Narrative

This case was previously recorded as S9334001 ;

CASE DESCRIPTION:A 29-YEAR-OLD FEMALE PATIENT RECEIVED VOLTAROL 6 DAYS BEFORE SHE DEVELOPED A LYELL SYNDROME. ONE DAY LATER SHE DIED BECAUSE OF CIRCULATORY INSUFFICIENCY. 06. June 95: Report was published. This 29-year-old female patient took 2 tablets of diclofenac for pain in her right shoulder. Three days later she received an injection with bupivacain and triamcinolon in the right shoulder. One day after the injection she rubbed with diclofenac gel. One day later (5 days after the first diclofenac- intake) she was hospitalized because of an exanthema at the trunk with haemorrhagic vesicles. During the following day she developed circulatory insufficiency and a pleural effusion. Despite intensive medical care she died 24 hours after admission secondary to a protracted shock. Autopsy results: Epidermal scaling and intradermal bleeding in 60 % of the body surface. shock lungs, shock kidneys, brain edema, gastric mucosal erosions, pleural effusions bilateral. Right shoulder joint with no morphological changes. ;

CIOMS Remarks:The initial report was a spontaneous report from Prof. B. Madea. ;

Article Title:ARZNEIMITTELINDUZIERTES LYELL-SYNDROM. ARZTRECHTLICHE ASPEKTE. : (DRUG-INDUCED LYELL-SYNDROME. MEDICAL LEGAL ASPECTS) : ;

Publication Name:MED. WELT

Publication Volume:46

Publication Issue:5

Article Start Page:280

Article End Page:284

Publication Year:1995

After an internal review on 23 Mar 2010 of the information received on 30 Jun 1993, the following significant correction was made: The suspect drug Voltaren (diclofenac sodium) emulgel was recoded to Voltaren (diclofenac diethylammonium salt) emulgel.

Sender's comment:

Reporter's comment:

NOVARTIS, 26.03.2010 DE-NOVNCHHQ-PHBS1993DE03836

Sender diagnosis: not reported

Narrative

This case was previously recorded as S9334001 ;

CASE DESCRIPTION: A 29-YEAR-OLD FEMALE PATIENT RECEIVED VOLTAROL 6 DAYS BEFORE SHE DEVELOPED A LYELL SYNDROME. ONE DAY LATER SHE DIED BECAUSE OF CIRCULATORY INSUFFICIENCY. 06. June 95: Report was published. This 29-year-old female patient took 2 tablets of diclofenac for pain in her right shoulder. Three days later she received an injection with bupivacain and triamcinolon in the right shoulder. One day after the injection she rubbed with diclofenac gel. One day later (5 days after the first diclofenac- intake) she was hospitalized because of an exanthema at the trunk with haemorrhagic vesicles. During the following day she developed circulatory insufficiency and a pleural effusion. Despite intensive medical care she died 24 hours after admission secondary to a protracted shock. Autopsy results: Epidermal scaling and intradermal bleeding in 60 % of the body surface. shock lungs, shock kidneys, brain edema, gastric mucosal erosions, pleural effusions bilateral. Right shoulder joint with no morphological changes. ;

CIOMS Remarks: The initial report was a spontaneous report from Prof. B. Madea. ;

Article Title: ARZNEIMITTELINDUZIERTES LYELL-SYNDROM. ARZTRECHTLICHE ASPEKTE. : (DRUG-INDUCED LYELL-SYNDROME. MEDICAL LEGAL ASPECTS) : ;

Publication Name: MED. WELT

Publication Volume: 46

Publication Issue: 5

Article Start Page: 280

Article End Page: 284

Publication Year: 1995

After an internal review on 23 Mar 2010 of the information received on 30 Jun 1993, the following significant correction was made: The suspect drug Voltaren (diclofenac sodium) emulgel was recoded to Voltaren (diclofenac diethylammonium salt) emulgel.

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10055156

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		No

Patient

Initials:	Sex: Male	Birth: privacy	Age: 73 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vesicles	2007				recovered/resolved
2 Hematoma NOS	2007				recovered/resolved
3 Poor peripheral perfusion	2007				recovered/resolved
4 Skin blotches	2007				recovered/resolved
5 Leg ulcer					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac		/	2007-2007	Arthritic pains
diclofenac			-	

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 21.04.2010 DE-002147023-PHHY2010DE22225

Sender diagnosis: not reported

Narrative

Initial report received from consumer via Novartis Consumer Health (local number 268/10) on 09 Apr 2010: This patient received two injections of a diclofenac (manufacturer unknown) into the big toe joint in 2007 (3 years ago) for arthrotic pain. One to 2 weeks later, the skin turned blotchy blue-red down to the lower leg, followed by development of bursting vesicles at three toes, ulcer at the tibia, and hematoma below the toe sole. Reportedly, the patient was admitted to hospital, several times, and an amputation had threatened, at the time. The treating physicians assessed the event was as a perfusion disorder. After a long period of time, the skin healed.

Sender's comment:

Serious spontaneous report,

- poor peripheral circulation, skin ulcer and hematoma [hospitalization], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

- rash macular and blister [hospitalization], already listed in the core data sheet

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10061500

Serious Yes	Death No	Lifethr. Yes	Hospital. Yes	Disabling No	Congenital No	Other No	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 58 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20090215			260 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ascorvit ascorbic acid	Oral	/	20090212-	Vitamin C deficiency
2 BELOC-ZOK MITE metoprolol succinate	Oral	47.5 Mg milligram(s)/Day	20080101-	Hypertension arterial
3 ciprobay ciprofloxacin hydrochloride	Oral	2* /Day	20090203-20090210 8 Day	Cystitis
4 CLEXANE heparin-fraction, sodium salt	Subcutaneous	0.4 ml millilitre(s)/Day	20090212-	Thrombosis prophylaxis
5 fortecortin dexamethasone	Oral	8 Mg milligram(s)/	20090108-	Prophylaxis
6 fortecortin dexamethasone	Intravenous (not otherwis	32 Mg milligram(s)/Day	20090213-20090216 4 Day	Seizure
7 LEFAX simeticone	Oral	2* /Day	20080101-	Meteorism
8 Lyrica pregabalin	Oral	150 Mg milligram(s)/ 6	20080601-	Polyneuropathy
9 MOVICOL macrogol; potassium chloride; sodium bicarbonate; sodium chloride	Oral	/Day	20090204-20090217 14 Day	Constipation
10 Natriumchlorid sodium chloride	Oral	/	20090203-	Hyponatremia
11 NOVORAPID insulin aspart	Subcutaneous	3* /Day	20080101-	Diabetes mellitus
12 OLMESARTAN MEDOXOMIL olmesartan medoxomil	Oral	20 Mg milligram(s)/	20080101-	Hypertension arterial
13 ORFIRIL "DESITIN" valproate sodium	Oral	600 Mg milligram(s)/	20090212-	Seizure
14 PROTAPHAN insulin isophane human biosynthetic	Subcutaneous	/Day	20080101-	Diabetes mellitus
15 SIMVASTATIN simvastatin	Oral	20 Mg milligram(s)/Day	20080101-	Hypercholesterolemia
16 Tegretal retard carbamazepine	Oral	400 Mg milligram(s)/Day	20090204-20090212 9 Day	Seizure
17 TIMONIL - SLOW RELEASE carbamazepine	Oral	200 Mg milligram(s)/	20090105-	Seizure
18 voltaren diclofenac sodium	Topical	/ 156	20090206-20090206 1 Day	Pain
19 ZOLPIDEM zolpidem	Oral	10 Mg milligram(s)/Day	20090123-	Sleep disorder

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Renal cell carcinoma			Unknown	
Metastases to brain			Unknown	
Radiotherapy		20090128	No	

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 30.04.2010 DE-PFIZER INC-2010050211

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170803.

A contactable physician reported that this 58-year old female patient who was in hospital for unknown reason, from 04Jan2009 until 04Feb2009 in neurology and from 20Feb2009 in dermatology. The patient was treated with the following suspect drugs: Pregabalin (LYRICA) from 01Jun2008 at dose of 150 mg (1-0-1, 1-0-3/2, 1-0-2) per oral route due to polyneuropathy, simeticone (LEFAX) from 01Jan2008 until 04Feb2009 and, possibly again from 05Jan2009 until 12Jan2009, twice daily (dose unknown) per oral route due to meteorism, insulin aspart (NOVORAPID) from 01Jan2008 three times daily (dose unknown), subcutaneously, due to diabetes mellitus, insulin isophane/human biosynthetic (PROTAPHANE) from 01Jan2008 once daily (dose unknown), subcutaneously, due to diabetes mellitus, carbamazepine (TIMONIL retard) from 05Jan2009 until 03Feb2009 and again from 13Feb2009 200 mg (1-0-2) per oral route due to seizure, carbamazepine was taken as generic from 05 until 07Jan2009, olmesartan medoxomil (VOTUM) from 01Jan2008 at dose of 20 mg (1-0-0, 1/2-0-0) per oral route due to arterial hypertension, metoprolol succinate (BELOC-ZOK mite) from 01Jan2008 at dose of 47.5 mg once daily per oral route due to arterial hypertension, dexamethasone (FORTECORTIN) from 08Jan2009 at dose of 8mg (1-1-1, 3x1/2 per oral route due to prophylaxis of cerebral edema, simvastatin (SIMVASTATIN) from 01Jan2008 at dose of 20 mg once daily due to hypercholesterolemia, zolpidem (ZOLDEM) from 23Jan2009 until 06Feb2009 and again from 18Feb2009 at dose of 10 mg at night per oral route due to sleep disorder, ciprofloxacin hydrochloride (CIPROBAY) from 03Feb2009 until 10Feb2009 twice daily (dose unknown) per oral route due to cystitis, sodium chloride (SODIUM CHLORIDE) from 03Feb2009 at unknown dose (1-1-0, 2-2-2) per oral route due to hyponatremia, carbamazepine (TEGRETAL - SLOW RELEASE) from 04Feb2009 until 12Feb2009 at dose of 200 mg twice daily per oral route due to seizure, macrogol/potassium chloride/sodium bicarbonate/sodium chloride (MOVICOL) from 04Feb2009 until 17Feb2009 once daily per oral route at unknown dose due to constipation, diclofenac sodium (VOLTAREN) ointment on 06Feb2009 topically due to pain, (MALUOKLIST, uncodeable) suppositories on 09Feb2009 at unknown dose due to constipation, valproate sodium (ORFIRIL liquid) from 12Feb2009 at dose of 600 mg (1-0-0, 1-0-1/2) per oral route due to seizure, heparin-fraction/sodium salt (CLEXANE) from 12Feb2009 at dose of 0.4 ml once daily per subcutaneous route for prophylaxis of thrombosis, ascorbic acid (ASCORVIT) from 12Feb2009 until 15Feb2009 and from 18Feb2009 three times daily (dose unknown) per oral route due for substitution and dexamethasone (FORTECORTIN) from 13Feb2009 until 16Feb2009 at dose of 8 mg four times daily per intravenous route due to seizure. Concomitant medication was not reported. It was reported that the patient received the "wrong tablets" (NOS) in hospital for two days in the period between 04Feb2009 and 12Feb2009. Medical history included cystitis and renal cell carcinoma with cerebral metastases as well as radiotherapy which had lasted until 28Jan2009. On 12Feb2009 the patient developed a reddening of her head and cockades and spots on 15Feb2009. Stevens-Johnson-Syndrome was diagnosed on 15Feb2009. Ascorbic acid was interrupted on 15Feb2009 and intravenous dexamethasone was discontinued on 16Feb2009, macrogol/potassium chloride/sodium bicarbonate/sodium chloride was discontinued on 17Feb2009, all of them unknown whether due to event. On 18Feb2009 the patient developed blisters on her skin and anal erosions. Ascorbic acid reintroduced on 18Feb2009. On 19Feb2009 the patient developed also erosions on lips and genital mucous membrane were stated (however, documentation was only until 18Feb2009). The event was considered life-threatening and caused prolonged inpatient hospitalization. Therapy with pregabalin, insulin aspart, insulin isophane/human biosynthetic, carbamazepine, olmesartanmedoxomil, metoprolol succinate, oral dexamethasone and simvastatin, sodium chloride, valproate sodium and heparin-fraction/sodium salt was ongoing by 18Feb2009, thereafter no further documentation of these drugs. ZOLDEM was restarted on 18Feb2009 and the patient also put on NOVALGIN drops (4 x 30 drops) for pain and SAB SIMPLEX 3 x 25 (units not provided) per oral route due to constipation. Outcome was reported as not yet recovered. It was unknown whether the patient was still in hospital. In Sep2009 the project experts assessed the causal relationship as dubious with pregabalin, as likely with carbamazepine (TIMONIL retard) and carbamazepine (TEGRETAL - SLOW RELEASE), as very possible with ciprofloxacin hydrochloride, as possible with oral dexamethasone and macrogol/potassium chloride/sodium bicarbonate/sodium chloride, as dubious with simeticone, insulin aspart, insulin isophane/human biosynthetic, olmesartanmedoxomil, metoprolol succinate, simvastatin, zolpidem (ZOLDEM), sodium chloride, diclofenac sodium, MALUOKLIST (ingredients not specified), valproate sodium, heparin-fraction/sodium salt, ascorbic acid and intravenous dexamethasone. Infection (cystitis) as alternative cause was documented as "possible". Renal cell carcinoma with cerebral metastasation as alternative cause was documented as "possible" by the project experts.

Sender's comment:

Bei Stevens-Johnson-Syndrome handelt es sich um eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10065128

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 51 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20090331	20090416	17 Day	29 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 LAMOTRIGINE lamotrigine	Oral	25 Mg milligram(s)/	20090306-20090331	Epilepsy
2 Planum temazepam	Oral	/Day 7	20090303- -	Sleep disorder
3 VOLTAREN RESINAT "NOVARTIS" diclofenac resinate	Oral	/	20090303- -	Pain

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

[PFIZERINC, 06.05.2010 DE-PFIZER INC-2010051371](#)

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, sponsored by the German Regulatory Authority, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the documentation center for severe skin reactions (dZh): 9170822.

A contactable physician reported that a 51-year-old male patient started to receive oral temazepam (PLANUM) once at night for sleep disorder on 03Mar2009 and unspecified dose of oral diclofenac resinate (VOLTAREN RESINAT) for pain. He also started oral lamotrigine (LAMOTRIGINE) 25 mg 1 or 2 dose form once daily on 06Mar2009 for epilepsy.

On 31Mar2009 the patient developed spots and cockades as well as lip erosions. Lamotrigine was discontinued the same day. It's uncertain if temazepam and diclofenac resinate were continued but, if so, at least until 07Apr2009.

On 31Mar2009, lamotrigine was substituted with diazepam 5 mg once in the morning for epilepsy, continued until 07Apr2009. Acetylcysteine (ACC) 500 mg once daily was probably introduced on 01Apr2009 due to fever and discontinued on 06Apr2009. For the same reason, also valproic acid 150 mg once daily was introduced from 04Apr2009 and continued until 07Apr2009 at least. Temazepam (PLANUM) and acetylcysteine (ACC) had been taken before and tolerated well by the patient. On 05Apr2009 the patient presented with blisters on his skin and erosions on his oral mucosa. On 06Apr2009 there were also erosions on his nasal mucosa. On 07Apr2009 the patient presented with reddening in the region of the genital mucosa and he was hospitalized the same day. Stevens-Johnson-syndrome was diagnosed with start date of 31Mar2009. On 16Apr2009 the Stevens-Johnson-Syndrome resolved.

In Sep2009 the project experts assessed the causal relationship as possible with temazepam, as likely with lamotrigine and as very possible with diclofenac resinate. Infection or other alternative causes was documented as not seen.

No follow-up attempts needed, follow-up automatically provided by dZh.

Sender's comment:

Causality for temazepam is difficult to assess due to intake of additional cosuspect drugs as a confounding factor. Stevens-Johnson-Syndrome ist in der Produktinformation als moegliche Nebenwirkung nicht beschrieben. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10069559

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 54 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20090503	20090518	16 Day	123 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ACERBON lisinopril	Oral	20 Mg milligram(s)/	20090101- -	Hypertension arterial
2 ACTOS pioglitazone	Oral	30 Mg milligram(s)/	20090101- -	Diabetes mellitus
3 BISOPROLOL bisoprolol	Oral	5 Mg milligram(s)/	20090101- -	Hypertension arterial
4 CARBAMAZEPINE carbamazepine	Oral	400 Mg milligram(s)/Day	20090101- -	Diabetic polyneuropathy
5 CLOMIPRAMINE clomipramine	Oral	10 Mg milligram(s)/	20090403- -	Diabetic polyneuropathy
6 DICLOFENAC diclofenac	Oral	50 Mg milligram(s)/	20090101- -	Pain
7 DOXYCYCLINE HYCLATE doxycycline hyclate	Oral	400 Mg milligram(s)/Day	20090428-20090503 6 Day	Prostatitis
8 FAKTU AKUT bismuth subgallate; bufexamac; lidocaine hydrochloride; titanium dioxide	Topical	/	20090401- -	Hemorrhoids
9 GABAPENTIN gabapentin	Oral	/	20090403- -	Diabetic polyneuropathy
10 METFOGAMMA metformin hydrochloride	Oral	850 Mg milligram(s)/	20090101- -	Diabetes mellitus
11 Norvasc amlodipine besilate	Oral	5 Mg milligram(s)/ 245	20090101- -	Hypertension arterial
12 OMEPR omeprazole	Oral	/	20090101- -	Gastritis prophylaxis
13 trimipramin trimipramine	Oral	/	20090403-20090428 26 Day	Diabetic polyneuropathy

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

[PFIZERINC, 11.05.2010 DE-PFIZER INC-2010053375](#)

Sender diagnosis: not reported

Narrative

This is a spontaneous report from a project, set up to collect suspected drug induced severe skin reactions leading to hospital admission. Reference number of the Documentation Center for Severe Skin Reactions (dZh): 9170830.

A contactable physician reported that a 54-year-old male patient started to receive, on 01Jan2009, oral administration of amlodipine besilate (NORVASC) 5mg, lisinopril (ACERBON) 20mg and bisoprolol 5mg for arterial hypertension, diclofenac 50mg, intermittently for pain, pioglitazone (ACTOS) 30mg and metformin hydrochloride (METFOGAMMA) 850mg for diabetes mellitus, carbamazepine 400mg once every evening for diabetic

polyneuropathy, an unspecified dose of omeprazole (OMEP) for stomach protection and topical bismuth subgallate, buprenorphine, lidocaine hydrochloride, titanium dioxide (FAKTU AKUT) intermittently for hemorrhoids. On 03Apr2009 he also started oral administration of unspecified dose of gabapentin (manufacturer not specified), trimipramine intermittently (until 28Apr2009) and clomipramine 10mg for diabetic polyneuropathy. On 28Apr2009, an unspecified dose of oral doxycycline (manufacturer not specified) was started for prostatitis.

In the past the patient had already been treated with diclofenac for unspecified indication which he tolerated well.

There were no concomitant medications.

On 03May2009 the patient developed pain of oral mucosa, cockades, spots, and reddening of the eyes and Steven-Johnson-Syndrome was diagnosed. The treatment of oral doxycycline was discontinued on 03May2009 and substituted, on 04May2009, with an unspecified dose of oral levofloxacin (TAVANIC). On 04May2009 patient was observed to have progression of Steven-Johnson-Syndrome; an unspecified dose of oral dimetindene maleate (FENISTIL) and calcium was started for the suspected allergic reaction.

On 05May2009 the patient developed erosions of lips, oral and nasal mucosa so he was admitted to hospital and intravenously treated with prednisolone (PREDNISOLUT) 125mg due to allergic exanthema.

On 09May2009 the patient developed conjunctivitis. At the time of the report, all suspected medications, except of doxycycline which was discontinued on 03May2009, were continued. On 18May2009 the patient recovered completely. Hospital discharge date was not provided.

The project experts assessed the causal relationship as likely with carbamazepine, as very possible with doxycycline, possible with gabapentin, diclofenac, metformin hydrochloride, trimipramine, clomipramine and as dubious with amlodipine besilate (NORVASC), omeprazole, bisoprolol, lisinopril (ACERBON), pioglitazone (ACTOS) and bismuth subgallate, buprenorphine, lidocaine hydrochloride, titanium dioxide (FAKTU AKUT).

A possible relationship was suspected by the project experts with the infection prostatitis.

No other causes were considered.

Pfizer is the Marketing Authorization Holder of gabapentin in Germany. This may be a duplicate report in situations where another Marketing Authorization Holder gabapentin has submitted the same report to the regulatory authorities.

This case has been received from Pfizer (9170830) and contains a Wyeth co-suspect product. The Pfizer case may qualify for regulatory reporting for the Wyeth co-suspect product. This case may therefore be a duplicate report.

No follow-up attempts needed/possible. No further information expected.

Sender's comment:

Causality of the company products gabapentin and amlodipine is not assessable due to multiple cosuspects as a confounding factor. Bei Steven-Johnson-Syndrom handelt es sich um eine bekannte, in der Produktinformation ausreichend praezise deklarierte Nebenwirkung von Norvasc. Allerdings kamen sehr viele Medikamente zum Einsatz, die das Event ver- bzw. mitverursacht haben koennen. Wir sehen zum gegenwaertigen Zeitpunkt keinen Anlass fuer eine geaenderte Bewertung des Nutzen-/Risikoverhaeltnisses.

Reporter's comment:

FIRST LOOK

Country: Deutschland Type: Spontaneous report case: DE-BFARM-10073371

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes Yes

Patient

Initials: Sex: Female Birth: privacy Age: weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin peeling					Unknown
2 Rash					Unknown
3 Application site erythema					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm gel	Topical	100 Mg milligram(s)/Day	-	
diclofenac sodium		95	-	

Primary source

Qualification: Pharmacist observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

ratiopharm GmbH, 20.05.2010 DE-RATIOPH-2010R0503596

Sender diagnosis: not reported

Narrative

Reported terms: Skin peeling(Skin exfoliation) Rash(Rash)

Case reference number 2010R0503596 is a spontaneous case report sent by a pharmacist which refers to a female of unknown age. It was in connection with a complaint and return of drug.

The patient's past medical history included no medical records. There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started taking Diclofenac-ratiopharm Gel (DICLOFENAC), indication unknown on an unknown date. The patient developed Skin peeling (Skin exfoliation)(start date ?) and Rash (Rash)(start date ?).

The following action was taken: for Diclofenac-ratiopharm Gel (DICLOFENAC) - Unknown

The outcome for Rash (Rash) is Unknown. The outcome for Skin peeling (Skin exfoliation) is Unknown.

Rechallenge information for Diclofenac-ratiopharm Gel is not applicable. Dechallenge information for Diclofenac-ratiopharm Gel is not applicable.

The reporter's causality for the case is Unknown. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

Skin peeling is not exactly worded in our RSI, however it is covered by the chapter side effects: "local skin reactions, such as skin redness, pruritus, a burning sensation, skin rash, sometimes with pustules or wheals." Causal relationship between suspected drug and events is rated as "possible".

Reporter's comment:

ratiopharm GmbH, 16.06.2010 DE-RATIOPH-2010R0503596

Sender diagnosis: not reported

Narrative

Reported terms: Skin peeling(Skin exfoliation) Rash(Rash)

Case reference number 2010R0503596 is a spontaneous case report sent by a pharmacist which refers to a female of unknown age. It was in connection with a complaint and return of drug.

The patient's past medical history included no medical records. There is no information about risk factors. There is no information about concomitant diseases. No prior drugs are known to have been taken by the patient.

The patient started taking Diclofenac-ratiopharm Gel (DICLOFENAC), indication unknown on an unknown date. The patient developed Skin peeling (Skin exfoliation)(start date ?) and Rash (Rash)(start date ?).

The following action was taken: for Diclofenac-ratiopharm Gel (DICLOFENAC) - Unknown

The outcome for Rash (Rash) is Unknown. The outcome for Skin peeling (Skin exfoliation) is Unknown.

Rechallenge information for Diclofenac-ratiopharm Gel is not applicable. Dechallenge information for Diclofenac-ratiopharm Gel is not applicable.

The reporter's causality for the case is Unknown.

Follow up 07-JUN-2010:

The case concerns to a female patient born on 28.03.1966. The outcome for Rash (Rash) and for Skin peeling (Skin exfoliation) is recovered / resolved. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

Skin peeling is not exactly worded in our RSI, however it is covered by the chapter side effects: "local skin reactions, such as skin redness, pruritus, a burning sensation, skin rash, sometimes with pustules or wheals." Causal relationship between suspected drug and events is rated as "possible".

Follow up 07-JUN-2010:

Overall causality has not changed.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10073372

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes		Yes						

Patient

Initials:	Sex: Female	Birth: privacy	Age: 41 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Quincke's edema	200903		1 Week		recovered/resolved
2 Rhinoconjunctivitis	200903		1 Week		recovered/resolved
3 Tachycardia	200903		1 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclofenac-ratiopharm 75 mg SL Retardkapseln diclofenac sodium	Oral	75 Mg milligram(s)/Day 208	200903- -	Headache

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Drug allergy			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

[ratiopharm GmbH, 20.05.2010 DE-RATIOPH-2010R0403196](#)

Sender diagnosis: not reported

Narrative

Reported terms: Quincke's edema(Angioedema) Tachycardia(Tachycardia) Rhinoconjunctivitis(Conjunctivitis)

Case reference number 2010R0403196 is a spontaneous case report sent by a physician which refers to a female aged 41 years.

The patient's past medical history included no medical records. There is no information about risk factors. Concomitant diseases include Ranitidine allergy (Drug hypersensitivity). The following drugs are known to have been taken by the patient prior to the event: Ranitidine (indication unknown, reaction: Tachycardia, started on an unknown date) and Ranitidine (indication unknown, reaction: Erythema, started on an unknown date).

The patient started taking Diclofenac-ratiopharm 75 mg SL Retardkapseln (DICLOFENAC), indication Headache on ??-Mar-2009. The patient developed Quincke's edema (Angioedema)(start date ??-Mar-2009), Tachycardia (Tachycardia)(start date ??-Mar-2009) and Rhinoconjunctivitis (Conjunctivitis)(start date ??-Mar-2009). The event was life threatening.

The following action was taken: for Diclofenac-ratiopharm 75 mg SL Retardkapseln (DICLOFENAC) - Drug withdrawn

The outcome for Tachycardia (Tachycardia), Quincke's edema (Angioedema) and Rhinoconjunctivitis (Conjunctivitis) is Recovered / Resolved.

Rechallenge information for Diclofenac-ratiopharm 75 mg SL Retardkapseln is not applicable. Dechallenge information for Diclofenac-ratiopharm 75 mg SL Retardkapseln is not applicable.

The reporter's assessment for drug Diclofenac-ratiopharm 75 mg SL Retardkapseln related with Tachycardia, Rhinoconjunctivitis and Quincke's edema is as follows: Probable. CLASSIFICATION: Adverse Event: No, Adverse Reaction: Yes, Drug / Drug Interaction: No, Abuse / Misuse: No, Pregnancy / Lactation tracking: No

Sender's comment:

Tachycardia is not exactly worded in our RSI however it may be covered by listed palpitations. Rhinoconjunctivitis is unlisted. This is the first case in our databank concerning this event after the intake of diclofenac.

Overall causality assessment was rated as 'possible', because there is no information on possible alternative explanations. Although, because of the negative allergy test, a causal relationship seems questionable.

No action is necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10082686

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hepatotoxicity	2010				Unknown
2 Skin disorder	2010				Unknown
3 Fever	2010				Unknown
4 Erythroderma	2010				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclophenac diclofenac	Unknown	/	-	-
2 Pandemrix H1N1 a/h1n1 influenza pandemic vaccine	Unknown	/ Day 120	201002-201002 -	Prophylaxis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Back pain			Yes	
Skin disorder			Yes	
Diarrhea			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)GlaxoSmithKline, 27.05.2010 DE-GlaxoSmithKline-D0067560A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of hepatotoxic reaction in a subject of unspecified age and gender who was vaccinated with swine flu split dresden (Pandemrix H1N1, GlaxoSmithKline) for prophylaxis.

In February 2010 the subject received unspecified dose of Pandemrix H1N1 (unknown route and injection site).

In 2010, an unspecified time after vaccination with Pandemrix H1N1, the subject experienced hepatotoxic reaction with fever and skin change.

The subject was hospitalised.

At the time of reporting the outcome of the events was unspecified.

Follow up information was received on 19 May 2010.

The subject experienced erythrodermia and hepatotoxic reaction (liver values 20-fold increased).

As additional possible triggering substance the subject received Diclophenac for back pain. Apart from that the subject received medications only rarely.

At the time of reporting the outcome of the events was unspecified.

Further information has been requested.

Sender's comment:

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

GlaxoSmithKline, 22.06.2010 DE-GlaxoSmithKline-D0067560A

Sender diagnosis: not reported

Narrative

This case was reported by a physician and described the occurrence of hepatotoxic reaction in a subject of unspecified age and gender who was vaccinated with swine flu split dresden (Pandemrix H1N1, GlaxoSmithKline) for prophylaxis.

In February 2010 the subject received unspecified dose of Pandemrix H1N1 (unknown route and injection site).

In 2010, an unspecified time after vaccination with Pandemrix H1N1, the subject experienced hepatotoxic reaction with fever and skin change.

The subject was hospitalised.

At the time of reporting the outcome of the events was unspecified.

Follow up information was received on 19 May 2010.

The subject experienced erythrodermia and hepatotoxic reaction (liver values 20-fold increased).

As additional possible triggering substance the subject received Diclophenac for back pain. Apart from that the subject received medications only rarely.

At the time of reporting the outcome of the events was unspecified.

Further information has been requested.

Written Follow up information was received on 21 June 2010 from the reporting physician: "Following allergy anamnesis and diagnostics, the subject's complaints are no longer suspected to be related to the vaccination with Pandemrix. Skin changes and diarrhea had occurred prior to the vaccination, possibly as a sign of intolerance to diclophenac. Allergy testing with thiomersal and diclophanc were negative. The adverse event report should be revoked."

Sender's comment:

Auf der Basis dieses Einzelfalles ergibt sich keine Aenderung des wissenschaftlichen Kenntnisstandes und der Nutzen/Risiko-Relation. Weitergehende eigenverantwortliche Massnahmen halten wir derzeit nicht fuer erforderlich.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10086782

Serious Yes	Death	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 67 Year	weight (kg): 55	BMI_int: 20.45	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Sepsis	200912			1 Month	recovered/resolved
2 Stomatitis	20100404			4 Day	recovered/resolved
3 Infected finger	200912			1 Month	recovered/resolved
4 Oral lesion	20100404			4 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 solaraze diclofenac sodium	Topical	2* /Day	20100401-20100409 9 Day	
2 solaraze diclofenac sodium	Topical	2* /Day	20091113- -	Actinic keratosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension			Unknown	
Hypothyroidism			Unknown	

Primary source

Qualification: Physician

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

ALM-HQ, 15.06.2010 DE-ALM-HQ-DE-2010-0426

Sender diagnosis: not reported

Narrative

Initial information received on 09 Apr 2010

ICSR of an initially reported non-serious ADR which was upgraded to serious by the company (hospitalisation), received from a consumer (patient herself) in Germany.

A female patient, of unknown age and unknown medical history, applied SOLARAZE (diclofenac sodium) to the back of the hand and the right temple, dosage, frequency and indication for use not provided, beginning on 13 Nov 2009. On an undisclosed date in the middle of December, the patient experienced a purulent thumb on the same hand where SOLARAZE was applied. The purulent thumb developed to sepsis for which she was hospitalised for four days (admission and discharge dates unknown). The patient recovered from the sepsis on an unknown date. Solaraze was withdrawn four weeks after its start date. On 01 Apr 2010, the patient re-applied SOLARAZE for actinic keratosis on the right temple. On 04 Apr 2010, four days after reintroducing SOLARAZE, the patient experienced a 1 cm big red spot with white focus at the roof of the mouth. The spot burns during drinking and eating. The patient consulted a dentist on 09 Apr 2010. On 13 Apr 2010 the patient consulted the dentist again and received local treatment (unspecified). It was unknown whether treatment with SOLARAZE was stopped. At the time of this report, the patient still had not recovered from the red spot. It was unknown if SOLARAZE was still ongoing.

As this case was provided by a non-healthcare professional, no causality assessment was available.

Closure information received on 20 Apr 2010.

Solaraze was withdrawn immediately after sepsis occurred. In Dec 2009 the treating physician did not see causal relationship between Solaraze and sepsis as stated. The patient does not want to contact her dermatologist again. The case is considered closed.

Follow-up information received on 07 Jun 2010.

This case was re-opened as medically confirmed information was received from a Health Professional in Germany.

According to follow-up information this 67-year-old female patient (weight: 55 kg, height: 1,64 m) with concomitant diseases of hypertension and hypothyroidism, applied SOLARAZE (diclofenac sodium) twice per day for actinic keratosis first from 13 Nov 2009 during 4 weeks and from 01 Apr 2010 to 09 Apr 2010. In mid Dec 2009 the patient experienced an infection in the finger and one week after stopping the treatment with SOLARAZE the patient experienced stomatitis. Treatment with SOLARAZE was stopped on 09 Apr 2010. The patient was treated with a dental mouth ointment. After an unknown period of time the patient recovered.

The physician has assessed these reactions as serious and probably related to SOLARAZE.

Corporate Drug Safety comment:

The patient used Solaraze on the back of her hand. Approximately one month after the start of therapy, she experienced a purulent thumb on the same hand where Solaraze was applied to. It was reported that the purulent thumb developed to sepsis for which she was hospitalized for four days. Sepsis is unlisted in the CCSI of Solaraze. It was not reported if there were any other factors than Solaraze (e.g. injury, insect bite etc.) that may have caused the purulent thumb. It was also unknown if the patient was suffering from any concomitant diseases that may have made her more prone to develop the purulent thumb and the sepsis (e.g. diabetes). It is important that the application site of Solaraze (back of the hand) was distinct to the thumb. As the purulent thumb developed on the same hand where Solaraze was applied to, it is also unlikely that the thumb regularly came into contact with Solaraze due to application (Solaraze was obviously applied with the other hand or by somebody else). For this reason, it is unlikely that the application of Solaraze to the back of the hand triggered the purulent thumb and in consequence the sepsis. It is also unlikely that systemic absorption of the topically applied diclofenac significantly contributed to this event, as systemic availability of Solaraze is limited (mean absorption through the skin varies between < 1-12% with large inter individual variability). Also, the area of the back of the hand is limited. For the mentioned reasons, causality is unlikely for Sepsis.

Approximately 3 1/2 months after experiencing the purulent thumb, Solaraze was reintroduced. Four days after reintroducing Solaraze, the patient experienced a 1 cm big red spot with white focus at the roof of the mouth, which burned during drinking and eating. This event is unlisted in the CCSI of Solaraze. Although the site of application of Solaraze was not clearly stated, it can be assumed that the patient used the product at the same site like previously or at least on skin like recommended and not inside her mouth. Although non-steroidal anti-inflammatory drugs (NSAIDs) are known to promote ulcers in the gastrointestinal tract, such a lesion like described may also have been caused by viral, bacterial or fungal infections or be a symptom of other diseases. Given the limited systemic availability of Solaraze, it seems more likely that alternative causes like explained were responsible for the lesion. Therefore, causality for this event is also unlikely.

Follow-up Corporate Drug Safety comments (20 Apr 2010): Confirmation that no further information will be available has been received and the case is considered closed. No further information relevant for case assessment was provided. Company causality remains as unlikely.

Corporate Drug Safety comment on Follow-up 1 (07 Jun 2010):

The event of infection at the finger is described (event not listed) to have happened in middle of December 2009, this matches the purulent thumb like described in the initial report. Like outlined in the initial report comment, it is unlikely that this event was triggered by Solaraze use. It was also stated that the patient experienced stomatitis one week after cessation of Solaraze therapy. It was not clearly stated if this event happened after the first or the second period of Solaraze application. This event is unlisted in the CCSI of Solaraze. As this event appeared one week after cessation of therapy, there is no plausible temporal relationship between Solaraze use and the event. Other causes of stomatitis (e.g. viral or fungal infections) are more likely. Therefore, causality is unlikely for this event. The reporter stated that the reaction reappeared after reintroduction of the drug, but no further details (e.g. which reaction reappeared) were provided. Therefore, rechallenge is deemed positive for both reactions (infection at the finger and stomatitis) at the time of this report.

Sender's comment:

Reporter's comment:

ALM-HQ, 16.07.2010 DE-ALM-HQ-DE-2010-0426

Sender diagnosis: not reported

Narrative

Initial information received on 09 Apr 2010

ICSR of an initially reported non-serious ADR which was upgraded to serious by the company (hospitalisation), received from a consumer (patient

herself) in Germany.

A female patient, of unknown age and unknown medical history, applied SOLARAZE (diclofenac sodium) to the back of the hand and the right temple, dosage, frequency and indication for use not provided, beginning on 13 Nov 2009. On an undisclosed date in the middle of December, the patient experienced a purulent thumb on the same hand where SOLARAZE was applied. The purulent thumb developed to sepsis for which she was hospitalised for four days (admission and discharge dates unknown). The patient recovered from the sepsis on an unknown date. Solaraze was withdrawn four weeks after its start date. On 01 Apr 2010, the patient re-applied SOLARAZE for actinic keratosis on the right temple. On 04 Apr 2010, four days after reintroducing SOLARAZE, the patient experienced a 1 cm big red spot with white focus at the roof of the mouth. The spot burns during drinking and eating. The patient consulted a dentist on 09 Apr 2010. On 13 Apr 2010 the patient consulted the dentist again and received local treatment (unspecified). It was unknown whether treatment with SOLARAZE was stopped. At the time of this report, the patient still had not recovered from the red spot. It was unknown if SOLARAZE was still ongoing.

As this case was provided by a non-healthcare professional, no causality assessment was available.

Closure information received on 20 Apr 2010.

Solaraze was withdrawn immediately after sepsis occurred. In Dec 2009 the treating physician did not see causal relationship between Solaraze and sepsis as stated. The patient does not want to contact her dermatologist again. The case is considered closed.

Follow-up information received on 07 Jun 2010.

This case was re-opened as medically confirmed information was received from a Health Professional in Germany.

According to follow-up information this 67-year-old female patient (weight: 55 kg, height: 1,64 m) with concomitant diseases of hypertension and hypothyroidism. applied SOLARAZE (diclofenac sodium) twice per day for actinic keratosis first from 13 Nov 2009 during 4 weeks and from 01 Apr 2010 to 09 Apr 2010. In mid Dec 2009 the patient experienced an infection in the finger and one week after stopping the treatment with SOLARAZE the patient experienced stomatitis. Treatment with SOLARAZE was stopped on 09 Apr 2010. The patient was treated with a dental mouth ointment. After an unknown period of time the patient recovered.

The physician has assessed these reactions as serious and probably related to SOLARAZE.

Follow-up information received on 05 July 2010.

According to follow-up information received from the dentist, stomatitis occurred during the second period of treatment. The reported term "red spot with white focus in the middle" from the consumer corresponded to the reported term "stomatitis".

The infection at the finger appeared after the first use of SOLARAZE. SOLARAZE was stopped. This reaction can not be confirmed by the reporting physician, because she had not seen it. After reintroduction of SOLARAZE the patient experienced stomatitis.

Corporate Drug Safety comment:

The patient used Solaraze on the back of her hand. Approximately one month after the start of therapy, she experienced a purulent thumb on the same hand where Solaraze was applied to. It was reported that the purulent thumb developed to sepsis for which she was hospitalized for four days. Sepsis is unlisted in the CCSI of Solaraze. It was not reported if there were any other factors than Solaraze (e.g. injury, insect bite etc.) that may have caused the purulent thumb. It was also unknown if the patient was suffering from any concomitant diseases that may have made her more prone to develop the purulent thumb and the sepsis (e.g. diabetes). It is important that the application site of Solaraze (back of the hand) was distinct to the thumb. As the purulent thumb developed on the same hand where Solaraze was applied to, it is also unlikely that the thumb regularly came into contact with Solaraze due to application (Solaraze was obviously applied with the other hand or by somebody else). For this reason, it is unlikely that the application of Solaraze to the back of the hand triggered the purulent thumb and in consequence the sepsis. It is also unlikely that systemic absorption of the topically applied diclofenac significantly contributed to this event, as systemic availability of Solaraze is limited (mean absorption through the skin varies between < 1-12% with large inter individual variability). Also, the area of the back of the hand is limited. For the mentioned reasons, causality is unlikely for Sepsis.

Approximately 3 1/2 months after experiencing the purulent thumb, Solaraze was reintroduced. Four days after reintroducing Solaraze, the patient experienced a 1 cm big red spot with white focus at the roof of the mouth, which burned during drinking and eating. This event is unlisted in the CCSI of Solaraze. Although the site of application of Solaraze was not clearly stated, it can be assumed that the patient used the product at the same site like previously or at least on skin like recommended and not inside her mouth. Although non-steroidal anti-inflammatory drugs (NSAIDs) are known to promote ulcers in the gastrointestinal tract, such a lesion like described may also have been caused by viral, bacterial or fungal infections or be a symptom of other diseases. Given the limited systemic availability of Solaraze, it seems more likely that alternative causes like explained were responsible for the lesion. Therefore, causality for this event is also unlikely.

Follow-up Corporate Drug Safety comments (20 Apr 2010): Confirmation that no further information will be available has been received and the case is considered closed. No further information relevant for case assessment was provided. Company causality remains as unlikely.

Corporate Drug Safety comment on Follow-up 1 (07 Jun 2010):

The event of infection at the finger is described (event not listed) to have happened in middle of December 2009, this matches the purulent thumb like described in the initial report. Like outlined in the initial report comment, it is unlikely that this event was triggered by Solaraze use. It was also stated

that the patient experienced stomatitis one week after cessation of Solaraze therapy. It was not clearly stated if this event happened after the first of the second period of Solaraze application. This event is unlisted in the CCSI of Solarze. As this event appeared one week after cessation of therapy, there is no plausible temporal relationship between Solaraze use and the event. Other causes of stomatitis (e.g. viral or fungal infections) are more likely. Therefore, causality is unlikely for this event. The reporter stated that the reaction reappeared after reintroduction of the drug, but no further details (e.g. which reaction reappeared) were provided. Therefore, rechallenge is deemed positive for both reactions (infection at the finger and stomatitis) at the time of this report.

Corporate Drug Safety comment on Follow-Up 2 (05 July 2010):

It was confirmed that stomatitis occurred after the second therapy cycle. It was also confirmed that the inflammation at the finger occurred during the first therapy cycle. Then, during the second therapy cycle, the patient experienced stomatitis. The events of sepsis and localised infection (finger) are different to stomatitis. Therefore, the rechallenge for the localised infection (finger) is negative. Also, it was not reported that the patient experienced sepsis after reintroduction, but stomatitis. Therefore, rechallenge is also deemed negative for sepsis. Rechallenge is unknown for stomatitis. For the reasons described in the previous assessments, causality remains unlikely for all three events.

Sender's comment:

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Report from study

case: DE-BFARM-10098330

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 79 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blisters	20080714	20080805	23 Day	196 Day	recovered/resolved
2 Pruritis	20080711	20080805	26 Day	193 Day	recovered/resolved
3 Erythema facial	20080710	20080805	27 Day	192 Day	recovered/resolved
4 Oral mucosa erosion	20080714	20080805	23 Day	196 Day	recovered/resolved
5 Stevens Johnson syndrome	20080710	20080805	27 Day	192 Day	recovered/resolved
6 Mucosal erosion	20080718	20080805	19 Day	200 Day	recovered/resolved
7 Macule	20080711	20080805	26 Day	193 Day	recovered/resolved
8 Lip erosion	20080716	20080805	21 Day	198 Day	recovered/resolved
9 Anal erosion	20080718	20080805	19 Day	200 Day	recovered/resolved
10 Eye redness	20080715	20080805	22 Day	197 Day	recovered/resolved
11 Tiredness	20080707	20080805	30 Day	189 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ALLOPURINOL allopurinol	Oral	/	20080623- -	Hyperuricaemia
2 ENAHXAL enalapril maleate	Oral	1 Df dosage form/Day	20080101- -	Renal insufficiency
3 MARCUMAR phenprocoumon	Oral	/	20080101- -	Atrial fibrillation
4 METOHEXAL metoprolol tartrate	Oral	1 Df dosage form/Day	20080101- -	Hypertension
5 OXYBUTYNIN oxybutynin	Oral	10 Mg milligram(s)/Day	20080101- -	Pollakiuria
6 RASILEZ aliskiren	Oral	/	20080101- -	Hypertension
7 TORASEMIDE torasemide	Oral	1 Df dosage form/Day	20080101- -	Renal insufficiency
8 VOLTAREN RESINAT diclofenac resinate	Oral	/	20080101- -	Pain

Primary source

Qualification:	Physician	observestudytype:	Individual patient use
Literature:			

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 01.07.2010 DE-002147023-PHHY2010DE39382

Sender diagnosis: not reported

Narrative

This case was received from a regional pharmacovigilance center via health authority on 15 June 2010: This poly medicated patient was on Rasilez 300 mg (aliskiren) for arterial hypertension and Voltaren Resinat (diclofenac) started on 01 Jan 2008 (indication not specified). On 23 Jun 2008 the patient additionally started Allopurinol. On 7 Jul 2008 the patient developed a feeling of tiredness, on 10 Jul 2008 facial erythema, on 11 Jul 2008 pruritus and maculae, on 14 Jul 2008 skin blisters and erosions of oral mucosa, on 15 Jul 2008 eye redness. On 12 Jul 2008 the patient was treated with Tavegil i.v. (clemastine), cetirizin (per os), Solu-Decortin H (i.v., prednisolone), ranitidine (i.e.) and Linolacort (topically, betamethasone). On 16 Jul 2008 the patient develop lip erosions and on 18 Jul 2008 anal and nasal erosions. A diagnosis of Stevens-Johnson-syndrome was made

(assessed as definite case) with 10 Jul 2008 as onset date. The event lasted until 5 Aug 2008. The patient made a complete recovery. The causal relationship was assessed as very probable for Allopurinol and as dubious for Rasilez, Voltaren Resinat, Ferro sanol duodenal (ferrous glycine sulfate), Marcumar acc. Quick (phenprocoumon), Enahexal (enalapril), Metohexal Succ 95 mg (metoprolol), Torasemid Hexal (torasemide) and Oxybutynin STADA (oxybutynin). Alternative explanations in the form of infections or "other" (not specified) were ruled out.

Sender's comment:

Serious spontaneous report:

For Rasilez and Metohexal Succ

Stevens-Johnson Syndrome [Hospitalization] assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

For Voltaren

Stevens-Johnson Syndrome [Hospitalization] already listed in core data sheet.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10099355

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 74 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin erosion	20100421			3 Year	not recovered/not resolved
2 Ear disorder NOS	20070507	20070707	62 Day	11 Day	recovered/resolved
3 Chondritis	20100421			3 Year	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 solaraze diclofenac sodium	Topical	2* /Day	20070426-20070507 12 Day	Actinic keratosis
2 solaraze diclofenac sodium		/	20100304- -	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Basal cell carcinoma			No	
Paronychia			No	
Onychomycosis			Unknown	
Lyme borreliosis			No	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

ALM-HQ, 05.07.2010 DE-ALM-HQ-DE-2010-1189

Sender diagnosis: not reported

Narrative

Initial information received on 31 May 2010:

ICSR of a non-serious adverse drug reaction received from a Health Professional in Germany.

A 77-year-old male patient (further details unknown) with unknown medical history applied SOLARAZE (diclofenac sodium) to ear helix (frequency unknown) for a long period of time for actinic keratosis. No concomitant medication was reported. On 21 Apr 2010 the patient experienced chondritis. Treatment with SOLARAZE was stopped on an undisclosed date. The patient was not recovered.

Follow-up information received on 21 Jun 2010.

ICSR upgraded to serious (other medically important condition) by the Health Professional.

ICSR ref. number DE-2010-1189 was linked in the safety database with ICSR ref. number DE-2010-1185 as it was reported by the same Healthcare professional.

A 77-year-old male patient with a medical history of basal cell carcinoma, borreliosis, onychomycosis and paronychia, applied SOLARAZE (Diclofenac Sodium) twice daily for actinic keratosis, first period from 26 April 2007 to 07 May 2007 and second period from 04 Mar 2010 up to an unreported date. From 07 May 2007 to 07 July 2007 the patient experienced ear injury. Solaraze was stopped. The patient was treated for this reaction with BEPANTHEN (Panthenol, Benzalkonium Chloride) cream.

On 21 Apr 2010, the patient experienced erosion. The patient was treated with FUCICORT (Fusidic Acid, Betamethasone) Ointment (frequency unknown), AUREOMYCIN (chlortetracycline) Ointment (frequency unknown) and CLINDAMYCIN (clindamycin) 600 tablets (three per day). Treatment with SOLARAZE was stopped on an unreported date. The patient has not recovered yet.

The physician has assessed these reactions as serious and probably related to SOLARAZE.

Sender's comment:

Corporate Drug Safety comment:

A 77 y/o male applied diclofenac sodium to ear helix for actinic keratosis and experienced chondritis. Chondritis can be listed under application site inflammation (in this case, cartilage inflammation). Company causality is possible.

Corporate Drug Safety comment on follow-up 1 (21 Jun 2010):

The case was upgraded to serious by reporter (medically important condition). Two additional events were reported: ear wound (application period in 2007) and skin erosion (application period in 2010). As all three events describe a similar application site reaction likely at the same site, dechallenge and rechallenge can be regarded positive. It is unknown if there were any other risk factors that may have promoted the events. Ear wound (ear disorder) can be regarded listed as an application site reaction. Skin erosion can be regarded listed since the more severe condition skin ulcer is listed. Causality is possible for all events.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10109463

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Male Birth: privacy Age: 61 Year weight (kg): 72 BMI_int: 26.45 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Acute generalised exanthematous pustulosis	20100712		5 Day		recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 novalgin metamizole	Oral	60 Gtt drop(s)/Day	-	-
2 voltaren resinat diclofenac resinate	Oral	2 Df dosage form/Day	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergy			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10127214

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 64 Year	weight (kg): 57	BMI_int: 21.19	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Allergic reaction	20100722			2 Day	recovered/resolved
2 Skin reaction localised					not recovered/not resolved
3 Pruritus aggravated					not recovered/not resolved
4 Swelling of hands	20100722			2 Day	recovered/resolved
5 Itching	20100722			2 Day	recovered/resolved
6 Redness	20100722			2 Day	recovered/resolved
7 Skin peeling	20100722			2 Day	recovered/resolved
8 Skin discomfort					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Voltaren Spray 40 mg/g Spray zur Anwendung	Topical	3 Df dosage form/Day	20100721-20100724	Pain
diclofenac sodium		26	4 Day	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 24.08.2010 DE-002147023-CHPA2010DE13476

Sender diagnosis: not reported

Narrative

Initial report received on 20 Aug 2010. A pharmacist reports, via the German Pharmacist's Drug Commission (number 154816), a female patient used VOLTAREN (diclofenac sodium) Spray, three times daily, for pain. She used the product from 22 Jul 2010 to 24 Jul 2010. In Jul 2010, she developed "a severe allergic skin reaction at her hands with redness and severe itching. The skin at her hands peeled off completely although the patient had washed her hands after application". She consulted a physician and cortisone NOS (internal and external) was used for a short time to treat the adverse events. At the time of the report, no further information was available. A quality assurance investigation was requested. According to the report, the product's batch number was 908630-21. The product returned was batch 909496-21. Final outcome of the events was reported as unchanged. Follow-up information was requested in written form in week 34, 2010.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event, Hypersensitivity and all associated events, was already labelled in the product information.

Reporter's comment:

NOVARTIS, 24.08.2010 DE-NOVNCHHQ-CHPA2010DE13476

Sender diagnosis: not reported

Narrative

VOLTAREN (diclofenac sodium) Spray, three times daily, for pain. She used the product from 22 Jul 2010 to 24 Jul 2010. In Jul 2010, she developed "a severe allergic skin reaction at her hands with redness and severe itching. The skin at her hands peeled off completely although the patient had washed her hands after application". She consulted a physician and cortisone NOS (internal and external) was used for a short time to treat the adverse events. At the time of the report, no further information was available. A quality assurance investigation was requested. According to the report, the product's batch number was 908630-21. The product returned was batch 909496-21. Final outcome of the events was reported as unchanged. Follow-up information was requested in written form in week 34, 2010.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event, Hypersensitivity and all associated events, was already labelled in the product information.

Reporter's comment:

NOVARTIS, 02.09.2010 DE-002147023-CHPA2010DE13476

Sender diagnosis: not reported

Narrative

Initial report received on 20 Aug 2010. A pharmacist reports, via the German Pharmacist's Drug Commission (number 154816), a female patient used VOLTAREN (diclofenac sodium) Spray, three times daily, for pain. She used the product from 22 Jul 2010 to 24 Jul 2010. In Jul 2010, she developed "a severe allergic skin reaction at her hands with redness and severe itching. The skin at her hands peeled off completely although the patient had washed her hands after application". She consulted a physician and cortisone NOS (internal and external) was used for a short time to treat the adverse events. At the time of the report, no further information was available. A quality assurance investigation was requested. According to the report, the product's batch number was 908630-21. The product returned was batch 909496-21. Final outcome of the events was reported as unchanged. Follow-up information was requested in written form in week 34, 2010.

Additional information received on 26 Aug 2010. The pharmacist specified that the patient developed 'massive allergic skin reaction with skin redness, severe itching, skin desquamation and swelling of hand inspite washing-off the solution after topical application' of VOLTAREN Spray. The reporter specified the onset date as '22 Jul 2010'. VOLTAREN Spray was stopped. Following drug treatment with cortisone (internal and external) treated by physician the adverse symptoms lasted all together for one week and four days. Causality rating of reporter was probable. Final outcome was reported as recovered.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event, Hypersensitivity and all associated events, was already labelled in the product information.

Reporter's comment:

NOVARTIS, 02.09.2010 DE-NOVNCHHQ-CHPA2010DE13476

Sender diagnosis: not reported

Narrative

Initial report received on 20 Aug 2010. A pharmacist reports, via the German Pharmacist's Drug Commission (number 154816), a female patient used VOLTAREN (diclofenac sodium) Spray, three times daily, for pain. She used the product from 22 Jul 2010 to 24 Jul 2010. In Jul 2010, she developed "a severe allergic skin reaction at her hands with redness and severe itching. The skin at her hands peeled off completely although the patient had washed her hands after application". She consulted a physician and cortisone NOS (internal and external) was used for a short time to treat the adverse events. At the time of the report, no further information was available. A quality assurance investigation was requested. According to the report, the product's batch number was 908630-21. The product returned was batch 909496-21. Final outcome of the events was reported as unchanged. Follow-up information was requested in written form in week 34, 2010.

Additional information received on 26 Aug 2010. The pharmacist specified that the patient developed 'massive allergic skin reaction with skin redness, severe itching, skin desquamation and swelling of hand inspite washing-off the solution after topical application' of VOLTAREN Spray. The reporter specified the onset date as '22 Jul 2010'. VOLTAREN Spray was stopped. Following drug treatment with cortisone (internal and external) treated by physician the adverse symptoms lasted all together for one week and four days. Causality rating of reporter was probable. Final outcome was reported as recovered.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event, Hypersensitivity and all associated events, was already labelled in the product information.

Reporter's comment:

NOVARTIS, 16.12.2010 DE-002147023-CHPA2010DE13476

Sender diagnosis: not reported

Narrative

Initial report received on 20 Aug 2010. A pharmacist reports, via the German Pharmacist's Drug Commission (number 154816), a female patient used VOLTAREN (diclofenac sodium) Spray, three times daily, for pain. She used the product from 22 Jul 2010 to 24 Jul 2010. In Jul 2010, she developed "a severe allergic skin reaction at her hands with redness and severe itching. The skin at her hands peeled off completely although the patient had washed her hands after application". She consulted a physician and cortisone NOS (internal and external) was used for a short time to treat the adverse events. At the time of the report, no further information was available. A quality assurance investigation was requested. According to the report, the product's batch number was 908630-21. The product returned was batch 909496-21. Final outcome of the events was reported as unchanged. Follow-up information was requested in written form in week 34, 2010.

Additional information received on 26 Aug 2010. The pharmacist specified that the patient developed 'massive allergic skin reaction with skin redness, severe itching, skin desquamation and swelling of hand inspite washing-off the solution after topical application' of VOLTAREN Spray. The reporter specified the onset date as '22 Jul 2010'. VOLTAREN Spray was stopped. Following drug treatment with cortisone (internal and external) treated by physician the adverse symptoms lasted all together for one week and four days. Causality rating of reporter was probable. Final outcome was reported as recovered.

Additional information received on 01 Dec 2010 consisted of technical complaint investigation results. All values are in full compliance with the specification. Batch documentation without deviation. (Quality complaint number: 10602146)

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event, Hypersensitivity and all associated events, was already labelled in the product information.

Reporter's comment:

NOVARTIS, 16.12.2010 DE-NOVNCHHQ-CHPA2010DE13476

Sender diagnosis: not reported

Narrative

Initial report received on 20 Aug 2010. A pharmacist reports, via the German Pharmacist's Drug Commission (number 154816), a female patient used VOLTAREN (diclofenac sodium) Spray, three times daily, for pain. She used the product from 22 Jul 2010 to 24 Jul 2010. In Jul 2010, she developed "a severe allergic skin reaction at her hands with redness and severe itching. The skin at her hands peeled off completely although the patient had washed her hands after application". She consulted a physician and cortisone NOS (internal and external) was used for a short time to treat the

adverse events. At the time of the report, no further information was available. A quality assurance investigation was requested. According to the report, the product's batch number was 908630-21. The product returned was batch 909496-21. Final outcome of the events was reported as unchanged. Follow-up information was requested in written form in week 34, 2010.

Additional information received on 26 Aug 2010. The pharmacist specified that the patient developed 'massive allergic skin reaction with skin redness, severe itching, skin desquamation and swelling of hand inspite washing-off the solution after topical application' of VOLTAREN Spray. The reporter specified the onset date as '22 Jul 2010'. VOLTAREN Spray was stopped. Following drug treatment with cortisone (internal and external) treated by physician the adverse symptoms lasted all together for one week and four days. Causality rating of reporter was probable. Final outcome was reported as recovered.

Additional information received on 01 Dec 2010 consisted of technical complaint investigation results. All values are in full compliance with the specification. Batch documentation without deviation. (Quality complaint number: 10602146)

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event, Hypersensitivity and all associated events, was already labelled in the product information.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10134839

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blisters	201008				not recovered/not resolved
2 Pustule	201008				not recovered/not resolved
3 Itching	201008				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN SCHMERZGEL diclofenac diethylamine	Topical	/ 95	201008-201008 1 Day	Shoulder pain

Drugs (concomitant)

2 CARDIAC THERAPY	/	-
3 cortisone cortisone	/	-
4 Folsaeure folic acid	/	-
5 METHOTREXATE methotrexate	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Rheumatism			Unknown	

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 03.09.2010 DE-002147023-CHPA2010DE13999

Sender diagnosis: not reported

Narrative

Initial report received on 30 Aug 2010. A male consumer reported he used VOLTAREN SCHMERZGEL (diclofenac diethylammonium salt), twice, in Aug 2010 for shoulder pain. One day after the application, he developed "pea-sized, itching and soaking blisters, which changed to hard and in part bloody pustules in the meantime". The VOLTAREN SCHMERZGEL was discontinued after two applications. The consumer planned to see a physician. At the time of the report, additional information was not available. Final outcome reported as unchanged. Follow-up information was requested in written form, including consent, on 31 Aug 2010.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant) not confirmed by a health professional. The event, Blister, was assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The events, Rash pustular and Pruritus, were already labelled in the product information.

Reporter's comment:

NOVARTIS, 03.09.2010 DE-NOVNCHHQ-CHPA2010DE13999

Sender diagnosis: not reported

Narrative

Initial report received on 30 Aug 2010. A male consumer reported he used VOLTAREN SCHMERZGEL (diclofenac diethylammonium salt), twice, in Aug 2010 for shoulder pain. One day after the application, he developed "pea-sized, itching and soaking blisters, which changed to hard and in part bloody pustules in the meantime". The VOLTAREN SCHMERZGEL was discontinued after two applications. The consumer planned to see a physician. At the time of the report, additional information was not available. Final outcome reported as unchanged. Follow-up information was requested in written form, including consent, on 31 Aug 2010.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant) not confirmed by a health professional. The event, Blister, was assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case. The events, Rash pustular and Pruritus, were already labelled in the product information.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10169591

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 83 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Application site eczema	201010				recovering/resolving
2 Eczema	201010				recovering/resolving
3 Skin red	201010				recovering/resolving
4 Desquamation	201010				recovering/resolving
5 Pruritus aggravated	201010				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclo schmerzgel diclofenac sodium	Topical	2 Df dosage form/Day 95	20100915-201010 -	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergy multiple			Yes	
Drug intolerance			Yes	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

case: DE-BFARM-10169778

Patient

Reactions

Drugs (suspected or interacting)

Primary source

487 / 652

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10170722

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Male Birth: privacy Age: 71 Year weight (kg): 108 BMI_int: 38.72 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Redness	20100919	20100928	10 Day		recovered/resolved
2 Itch	20100919	20100928	10 Day		recovered/resolved
3 Blistering	20100919	20100928	10 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Solaraze 3 % Gel diclofenac sodium	Topical	2 Df dosage form/Day	20100915-20100928 14 Day	Actinic keratosis
2 Solaraze 3 % Gel diclofenac sodium	Topical	2 Df dosage form/Day	20101014-20101018 5 Day	Actinic keratosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes			Unknown	
Hypertension			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10185892

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex:	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pain burning	20101002			2 Day	recovered/resolved with sequel
2 Skin inflammation	2010100212			2 Day	recovered/resolved with sequel
3 Blisters	20101002			2 Day	recovered/resolved with sequel

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN SCHMERZGEL	Topical	/	20101001-20101001	Back pain
diclofenac diethylamine		95	1 Day	

Drugs (concomitant)

2 BELOC ZOK	Oral	190 Mg milligram(s)/Day	-
metoprolol succinate		245	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Blood pressure increased			Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 30.11.2010 DE-002147023-CHPA2010DE16594

Sender diagnosis: not reported

Narrative

Initial report received on 15 Oct 2010. A pharmacist reports a female patient, more than 70 years old, used VOLTAREN SCHMERZGEL (diclofenac diethylammonium salt) topically on a large scale skin area, on her back, to treat back pain and developed "burn blisters and had the feeling as if the skin was suffering from a burn". The pharmacist specified that the VOLTAREN SCHMERZGEL was used several times before without any problems. It was unknown if product use continued. Final outcome of the events was unknown. Follow-up information was requested in written form in week 42, 2010.

Additional information received on 25 Oct 2010. A pharmacist reports the patient returned the VOLTAREN SCHMERZGEL to the pharmacy and the product was sent in at German affiliate QA. The corrected batch number was W8564. A QA investigation was requested. Product use was discontinued. Final outcome was unknown.

Follow up information received from a pharmacist on 22 Nov 2010, which upgrades this case to serious (medically significant). The reporting pharmacist specified that the patient, suffering from elevated blood pressure, developed skin inflammation with weeping blisters on her back, on a skin area of approximately 30 cm x 30 cm, nearer described as burning pain, after a single topical application of Voltaren Schmerzgel. The product was applied once as a thin layer on 01 Oct 2010, around 08.00 in the morning. The reactions onset date was 02 Oct 2010, approximately around 12:00. Voltaren was discontinued after the reactions occurrence. The patient received unspecified treatment prescribed by physician. The symptoms lasted for about 6 weeks. The reported outcome was: recovered with sequelae (persisting skin scars). No other information was available.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The terms dermatitis and related event, and blisters are already included in the product information.

Reporter's comment:NOVARTIS, 30.11.2010 DE-NOVNCHHQ-CHPA2010DE16594

Sender diagnosis: not reported

Narrative

Initial report received on 15 Oct 2010. A pharmacist reports a female patient, more than 70 years old, used VOLTAREN SCHMERZGEL (diclofenac diethylammonium salt) topically on a large scale skin area, on her back, to treat back pain and developed "burn blisters and had the feeling as if the skin was suffering from a burn". The pharmacist specified that the VOLTAREN SCHMERZGEL was used several times before without any problems. It was unknown if product use continued. Final outcome of the events was unknown. Follow-up information was requested in written form in week 42, 2010.

Additional information received on 25 Oct 2010. A pharmacist reports the patient returned the VOLTAREN SCHMERZGEL to the pharmacy and the product was sent in at German affiliate QA. The corrected batch number was W8564. A QA investigation was requested. Product use was discontinued. Final outcome was unknown.

Follow up information received from a pharmacist on 22 Nov 2010, which upgrades this case to serious (medically significant). The reporting pharmacist specified that the patient, suffering from elevated blood pressure, developed skin inflammation with weeping blisters on her back, on a skin area of approximately 30 cm x 30 cm, nearer described as burning pain, after a single topical application of Voltaren Schmerzgel. The product was applied once as a thin layer on 01 Oct 2010, around 08.00 in the morning. The reactions onset date was 02 Oct 2010, approximately around 12:00. Voltaren was discontinued after the reactions occurrence. The patient received unspecified treatment prescribed by physician. The symptoms lasted for about 6 weeks. The reported outcome was: recovered with sequelae (persisting skin scars). No other information was available.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The terms dermatitis and related event, and blisters are already included in the product information.

Reporter's comment:NOVARTIS, 10.12.2010 DE-002147023-CHPA2010DE16594

Sender diagnosis: not reported

Narrative

Initial report received on 15 Oct 2010. A pharmacist reports a female patient, more than 70 years old, used VOLTAREN SCHMERZGEL (diclofenac diethylammonium salt) topically on a large scale skin area, on her back, to treat back pain and developed "burn blisters and had the feeling as if the skin was suffering from a burn". The pharmacist specified that the VOLTAREN SCHMERZGEL was used several times before without any problems. It was unknown if product use continued. Final outcome of the events was unknown. Follow-up information was requested in written form in week 42, 2010.

Additional information received on 25 Oct 2010. A pharmacist reports the patient returned the VOLTAREN SCHMERZGEL to the pharmacy and the product was sent in at German affiliate QA. The corrected batch number was W8564. A QA investigation was requested. Product use was discontinued. Final outcome was unknown.

Follow up information received from a pharmacist on 22 Nov 2010, which upgrades this case to serious (medically significant). The reporting pharmacist specified that the patient, suffering from elevated blood pressure, developed skin inflammation with weeping blisters on her back, on a skin area of approximately 30 cm x 30 cm, nearer described as burning pain, after a single topical application of Voltaren Schmerzgel. The product was applied once as a thin layer on 01 Oct 2010, around 08.00 in the morning. The reactions onset date was 02 Oct 2010, approximately around 12:00. Voltaren was discontinued after the reactions occurrence. The patient received unspecified treatment prescribed by physician. The symptoms lasted for about 6 weeks. The reported outcome was: recovered with sequelae (persisting skin scars). No other information was available.

Follow up received on 02 Dec 2010 consisted of technical complaint investigation results. We do not analyze the returned sample or our file sample for lack of efficacy and/or side effect complaints in principle. We decide whether some analytical tests should be performed on a case-by-case basis, as this is our usual practice. In this instance, we have decided not to carry out an analysis. This decision rests on the following facts: This lot met the release requirements and the present case is the first of its kind for this lot. So, it is our expert opinion that there is no need to analyze any sample at this time. (Quality complaint number: 10628674)

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The terms dermatitis and related event, and blisters are already included in the product information.

Reporter's comment:

NOVARTIS, 10.12.2010 DE-NOVNCHHQ-CHPA2010DE16594

Sender diagnosis: not reported

Narrative

Initial report received on 15 Oct 2010. A pharmacist reports a female patient, more than 70 years old, used VOLTAREN SCHMERZGEL (diclofenac diethylammonium salt) topically on a large scale skin area, on her back, to treat back pain and developed "burn blisters and had the feeling as if the skin was suffering from a burn". The pharmacist specified that the VOLTAREN SCHMERZGEL was used several times before without any problems. It was unknown if product use continued. Final outcome of the events was unknown. Follow-up information was requested in written form in week 42, 2010.

Additional information received on 25 Oct 2010. A pharmacist reports the patient returned the VOLTAREN SCHMERZGEL to the pharmacy and the product was sent in at German affiliate QA. The corrected batch number was W8564. A QA investigation was requested. Product use was discontinued. Final outcome was unknown.

Follow up information received from a pharmacist on 22 Nov 2010, which upgrades this case to serious (medically significant). The reporting pharmacist specified that the patient, suffering from elevated blood pressure, developed skin inflammation with weeping blisters on her back, on a skin area of approximately 30 cm x 30 cm, nearer described as burning pain, after a single topical application of Voltaren Schmerzgel. The product was applied once as a thin layer on 01 Oct 2010, around 08.00 in the morning. The reactions onset date was 02 Oct 2010, approximately around 12:00. Voltaren was discontinued after the reactions occurrence. The patient received unspecified treatment prescribed by physician. The symptoms lasted for about 6 weeks. The reported outcome was: recovered with sequelae (persisting skin scars). No other information was available.

Follow up received on 02 Dec 2010 consisted of technical complaint investigation results. We do not analyze the returned sample or our file sample for lack of efficacy and/or side effect complaints in principle. We decide whether some analytical tests should be performed on a case-by-case basis, as this is our usual practice. In this instance, we have decided not to carry out an analysis. This decision rests on the following facts: This lot met the release requirements and the present case is the first of its kind for this lot. So, it is our expert opinion that there is no need to analyze any sample at this time. (Quality complaint number: 10628674)

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The terms dermatitis and related event, and blisters are already included in the product information.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-10189622

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 68 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin necrosis	20101106			4 Day	Unknown
2 Wound	20101106			4 Day	Unknown
3 Wrong technique in drug usage process					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/	20101103-20101103	
		95	1 Day	
2 voltaren emulgel diclofenac diethylamine	Topical	/	20101106-20101106	
		95	1 Day	

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 03.12.2010 DE-NOVNCHHQ-CHNY2010DE00533

Sender diagnosis: not reported

Narrative

Initial spontaneous report received from a physician on 23 Nov 2010. A 68-year old female patient developed a local skin reaction with skin necrosis on her left leg after the second application of Voltaren Emulgel using iontophoresis. The product was applied twice, on 03 Nov 2010 and on 06 Nov 2010. The reaction occurred after the second application and the time elapsed between the application and the event onset was not specified. The outcome was not reported. No other information was provided.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event skin Necrosis was assessed as unlisted to the product information. The available information was considered inadequate to fully assess the case. The other event was assessed as non-serious.

Reporter's comment:

NOVARTIS, 06.12.2010 DE-002147023-CHNY2010DE00533

Sender diagnosis: not reported

Narrative

Initial spontaneous report received from a physician on 23 Nov 2010. A 68-year old female patient developed a local skin reaction with skin necrosis on her left leg after the second application of Voltaren Emulgel using iontophoresis. The product was applied twice, on 03 Nov 2010 and on 06 Nov

2010. The reaction occurred after the second application and the time elapsed between the application and the event onset was not specified. The outcome was not reported. No other information was provided.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event skin Necrosis was assessed as unlisted to the product information. The available information was considered inadequate to fully assess the case. The other event was assessed as non-serious.

Reporter's comment:

NOVARTIS, 13.12.2010 DE-002147023-CHNY2010DE00533

Sender diagnosis: not reported

Narrative

Initial spontaneous report received from a physician on 23 Nov 2010. A 68-year old female patient developed a local skin reaction with skin necrosis on her left leg after the second application of Voltaren Emulgel using iontophoresis. The product was applied twice, on 03 Nov 2010 and on 06 Nov 2010. The reaction occurred after the second application and the time elapsed between the application and the event onset was not specified. The outcome was not reported. No other information was provided.

Additional information received on 07 Dec 2010. A physician reports this female patient developed "a secondary, curative wound with subsequent skin necrosis" after a second iontophoresis treatment. The reporter did not fill in the follow-up questionnaire and did not provide further information. Causality assessment by reporting physician: not related. The outcome was not reported.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event skin Necrosis was assessed as unlisted to the product information. The available information was considered inadequate to fully assess the case. The other event was assessed as non-serious.

Reporter's comment:

NOVARTIS, 13.12.2010 DE-NOVNCHHQ-CHNY2010DE00533

Sender diagnosis: not reported

Narrative

Initial spontaneous report received from a physician on 23 Nov 2010. A 68-year old female patient developed a local skin reaction with skin necrosis on her left leg after the second application of Voltaren Emulgel using iontophoresis. The product was applied twice, on 03 Nov 2010 and on 06 Nov 2010. The reaction occurred after the second application and the time elapsed between the application and the event onset was not specified. The outcome was not reported. No other information was provided.

Additional information received on 07 Dec 2010. A physician reports this female patient developed "a secondary, curative wound with subsequent skin necrosis" after a second iontophoresis treatment. The reporter did not fill in the follow-up questionnaire and did not provide further information. Causality assessment by reporting physician: not related. The outcome was not reported.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event skin Necrosis was assessed as unlisted to the product information. The available information was considered inadequate to fully assess the case. The other event was assessed as non-serious.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11046534

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth: privacy	Age: 52 Year	weight (kg): 85	BMI_int: 28.73	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blisters	20110228			3 Day	not recovered/not resolved
2 Drug hypersensitivity	20110227			2 Day	not recovered/not resolved
3 Swelling of feet	20110228			3 Day	not recovered/not resolved
4 Hives	20110227			2 Day	not recovered/not resolved
5 Redness	20110227			2 Day	not recovered/not resolved
6 Itching	20110227			2 Day	not recovered/not resolved
7 Contact dermatitis	20110224	20110324	29 Day	3 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm	Topical	/	20110226-20110227	Sprained ankle
diclofenac sodium		95	2 Day	

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)TEVA, 15.03.2011 DE-TEVA-271053GER

Sender diagnosis: not reported

Narrative

09-Mar-2011, Spontaneous Serious Report.

A patient reported the case about himself of unknown age who experienced ALLERGIC REACTION, BLISTERS ON FOOT, SWELLING OF FOOT, HIVES, REDNESS OF SKIN, ITCHING while on DICLOFENAC-ratiopharm (Teva's drug).

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events)).

Patient experienced sprained foot ankle on 25-Feb-2011. Therefore, patient used Diclofenac-ratiopharm Gel on 26-Feb-and 27-Feb-2011. In the evening of 27-Feb-2011 patient's skin was red, with hives and itching. Patient foot became more swollen on the next day and itching increased. As patient tried to dress his shoes the foot was swollen, extremely red and showed blisters. Patient consulted emergency physician. Therapy: Administration of cortisone injection, following high-dosage cortisone therapy for 4 days orally. Patient stated, since allergic reaction does not improve soon, he will go to the hospital.

Teva Comment: due to striking temporal relationship symptoms reported are possibly related to Diclofenac-ratiopharm Gel. As there is only few information available contribution of other reasons and circumstances cannot be excluded.

This is a spontaneous case and therefore considered and handled as a related case.

Sender's comment:

Serious, spontaneous (TEVA product could not be excluded) case report from Germany. Duty for notification according to German Drug Law (§ 63b). Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: expected. The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

TEVA, 04.04.2011 DE-TEVA-271053GER

Sender diagnosis: not reported

Narrative

09-Mar-2011, Spontaneous Serious Report.

A patient reported the case about himself of unknown age who experienced ALLERGIC REACTION, BLISTERS ON FOOT, SWELLING OF FOOT, HIVES, REDNESS OF SKIN, ITCHING while on DICLOFENAC-ratiopharm (Teva's drug).

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events)).

Patient experienced sprained foot ankle on 25-Feb-2011. Therefore, patient used Diclofenac-ratiopharm Gel on 26-Feb-and27-Feb-2011. In the evening of 27-Feb-2011 patient's skin was red, with hives and itching. Patient foot became more swollen on the next day and itching increased. As patient tried to dress his shoes the foot was swollen, extremely red and showed blisters. Patient consulted emergency physician. Therapy: Administration of cortisone injection, following high-dosage cortisone therapy for 4 days orally. Patient stated, since allergic reaction does not improve soon, he will go to the hospital.

Teva Comment: due to striking temporal relationship symptoms reported are possibly related to Diclofenac-ratiopharm Gel. As there is only few information available contribution of other reasons and circumstances cannot be excluded.

This is a spontaneous case and therefore considered and handled as a related case.

28-Mar-2011:

Patient weight, height, birth date and age of 52 years available.

No new medical assessment necessary.

Sender's comment:

Serious, spontaneous TEVA product could not be excluded) (follow-up) case report from Germany. Duty for notification according to German Drug Law (§ 63b). Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: expected. The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

TEVA, 18.04.2011 DE-TEVA-271053GER

Sender diagnosis: not reported

Narrative

09-Mar-2011, Spontaneous Serious Report.

A patient reported the case about himself of unknown age who experienced ALLERGIC REACTION, BLISTERS ON FOOT, SWELLING OF FOOT, HIVES, REDNESS OF SKIN, ITCHING while on DICLOFENAC-ratiopharm (Teva's drug).

This case was considered serious based on the following criteria: (Other Serious (Important Medical Events)).

Patient experienced sprained foot ankle on 25-Feb-2011. Therefore, patient used Diclofenac-ratiopharm Gel on 26-Feb-and27-Feb-2011. In the evening of 27-Feb-2011 patient's skin was red, with hives and itching. Patient foot became more swollen on the next day and itching increased. As patient tried to dress his shoes the foot was swollen, extremely red and showed blisters. Patient consulted emergency physician. Therapy: Administration of cortisone injection, following high-dosage cortisone therapy for 4 days orally. Patient stated, since allergic reaction does not improve soon, he will go to the hospital.

Teva Comment: due to striking temporal relationship symptoms reported are possibly related to Diclofenac-ratiopharm Gel. As there is only few

information available contribution of other reasons and circumstances cannot be excluded.

This is a spontaneous case and therefore considered and handled as a related case.

28-Mar-2011:

Patient weight, height, birth date and age of 52 years available.

No new medical assessment necessary.

11-Apr-2011:

Furthermore, patient reported contact dermatitis with papulos exanthema related to Diclofenac-ratiopharm gel. Contact dermatitis occurred one day prior Diclofenac-ratiopharm was used.

Teva Comment: As contact dermatitis occurred 1 day prior to Diclofenac use, the relationship is rated as "not related" due to implausible temporal relationship.

Sender's comment:

Serious, spontaneous TEVA product could not be excluded) (follow-up2) case report from Germany. Duty for notification according to German Drug Law (§ 63b). Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: expected. The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11084620

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 54 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	20101031			3 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 DICLOFENAC diclofenac	Intramuscular	/	20101029-	Back pain
			-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)NOVARTIS, 20.05.2011 DE-002147023-PHHY2011DE40006

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE40006, is an initial spontaneous report from a physician via Drug Commission of the German Medical Association received on 11 May 2011. This report refers to a female patient. The patient's medical history and concomitant medications were not reported. The patient received diclofenac (manufacturer unknown) for back pain intramuscularly 29 Oct 2010. On 31 Oct 2010, after receiving the drug she experienced Stevens-Johnson syndrome. The patient was hospitalized on unspecified date. The outcome of the event was not reported. The causality of the event was reported as possible.

Sender's comment:

Serious spontaneous report- Stevens-Johnson syndrome [hospitalization], assessed as listed in the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11096429

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 33 Year	weight (kg): 63	BMI_int: 21.8	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Hyperpigmentation skin					not recovered/not resolved
2 Scar					not recovered/not resolved
3 Cutaneous vasculitis	2004				not recovered/not resolved
4 Ankle ulcer	2004				not recovered/not resolved
5 Thrombosis	2007				not recovered/not resolved
6 Capillaritis	2007				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Acetylsalizylsaeure acetylsalicylic acid	Oral	500 Mg milligram(s)/	-	
2 ibuprofen ibuprofen	Oral	400 Mg milligram(s)/	-	
3 voltaren dispers diclofenac sodium	Oral	/ 245	2001-201105	Headache
4 voltaren resinat diclofenac resinate	Oral	/	2007-	Muscle pain

Drugs (concomitant)

5 folio folic acid	Oral	150 µg microgram(s)/	-	
6 FOLIO FORTE	Oral	10 µg microgram(s)/	-	
7 FOLIO FORTE	Oral	800 µg microgram(s)/	201104-	
8 vitamin c ascorbic acid		500 Mg milligram(s)/	201103-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Joint pain			Unknown	
Drug allergy			Unknown	
Pain			Unknown	
Ulcer			Unknown	
Pain in spine			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

[NOVARTIS, 13.05.2011 DE-002147023-PHHY2011DE37257](#)

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE37257, is an initial spontaneous report received from physician via phone call on 28 Apr 2011. This report refers to a 33-year-old female patient. The patient received Voltaren resinat (diclofenac resinat) orally as needed for muscle and joint pain since 2007 (for about 5

years). About half a year after the start of treatment with Voltaren resinat, the patient developed ulceration on ankles. Reportedly the ulceration had worsened after intake of Voltaren resinat. A specimen of ulcerated skin tissue showed capillary inflammation and thrombosis of capillaries. Peripheral arterial disease and venous disorders had been excluded. Dechallenge of Voltaren Resinat had been planned. The event outcome was reported as ongoing and causality was not explicitly mentioned.

Sender's comment:

Serious spontaneous report- Skin ulcer, [medically significant] assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Based on Review of available reported data does not allow for a proper causality assessment. Reported event(s) is (are) unassessable

Reporter's comment:

NOVARTIS, 11.07.2011 DE-002147023-PHHY2011DE37257

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE37257, is an initial spontaneous report received from physician via phone call on 28 Apr 2011 with follow up received on 27 Jun 2011: This report refers to a 33-year-old female patient. This polymedicated patient had medical history of amoxicillin allergy. Concomitant medication included Vitamin C and Folio forte tablets The patient received Voltaren dispers (diclofenac free acid) tablets orally for headache, vertebral pain and pain from ulcers since 2001 approximately one to two tablets daily. The patient also received ibuprofen (manufacturer unknown) 600 mg and acetylsalicylic acid (manufacturer unknown) 500 mg orally. In end of 2004, the patient developed cutaneous vasculitis at both ankles. The patient developed ulceration on ankles. A specimen of ulcerated skin tissue showed capillary inflammation and thrombosis of capillaries. Peripheral arterial disease and venous disorders had been excluded. The diameter of ulcerations was approximately 5 mm and was relapsing every 3 to 10 months until recently. Ulcerations have merged with a diameter of a single lesion of up to 10 mm. Vasculitis was treated with prednisolone 60 mg and pain with Valoron drops orally. No new lesion had occurred since cessation of voltaren dispers on May 2011. The patient had not yet recovered and sequelae of scar and hyperpigmentation have remained. The causality was assessed by reporting physician as likely to be related to Voltaren dispers, however ibuprofen 600 mg and acetylsalicylic acid 500 mg were mentioned as possible alternative explanations.

Follow-up information received from physician on 27 Jun 2011: Suspect drug changed from Voltaren resinat to Voltaren dispers, updated start date of suspect drug, indication, concomitant medication, medical history of allergy, events, treatment medication, co suspect medication, action taken with suspect drug and causality.

Sender's comment:

Serious spontaneous report,

For Voltaren dispers,

Cutaneous vasculitis, [medically significant] assessed as listed according to the Core Data Sheet.

For acetyl salicylic acid and ibuprofen,

Cutaneous vasculitis [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

For acetyl salicylic acid and ibuprofen, the event is confounded by Voltaren dispers.

Reporter's comment:

NOVARTIS, 11.07.2011 DE-SZ09-PHHY2011DE37257

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE37257, is an initial spontaneous report received from physician via phone call on 28 Apr 2011 with follow up received on 27 Jun 2011: This report refers to a 33-year-old female patient. This polymedicated patient had medical history of amoxicillin allergy. Concomitant medication included Vitamin C and Folio forte tablets The patient received Voltaren dispers (diclofenac free acid) tablets orally for headache, vertebral pain and pain from ulcers since 2001 approximately one to two tablets daily. The patient also received ibuprofen (manufacturer unknown) 600 mg and acetylsalicylic acid (manufacturer unknown) 500 mg orally. In end of 2004, the patient developed cutaneous vasculitis at both ankles. The patient developed ulceration on ankles. A specimen of ulcerated skin tissue showed capillary inflammation and thrombosis of capillaries. Peripheral arterial disease and venous disorders had been excluded. The diameter of ulcerations was approximately 5 mm and was relapsing every 3 to 10 months until recently. Ulcerations have merged with a diameter of a single lesion of up to 10 mm. Vasculitis was treated with prednisolone 60 mg and pain with Valoron drops orally. No new lesion had occurred since cessation of voltaren dispers on May 2011. The patient had not yet recovered and sequelae of scar and hyperpigmentation have remained. The causality was assessed by reporting physician as likely to be related to Voltaren dispers, however ibuprofen 600 mg and acetylsalicylic acid 500 mg were mentioned as possible alternative explanations.

Follow-up information received from physician on 27 Jun 2011: Suspect drug changed from Voltaren resinat to Voltaren dispers, updated start date of suspect drug, indication, concomitant medication, medical history of allergy, events, treatment medication, co suspect medication, action taken with suspect drug and causality.

Sender's comment:

Serious spontaneous report,

For Voltaren dispers,

Cutaneous vasculitis, [medically significant] assessed as listed according to the Core Data Sheet.

For acetyl salicylic acid and ibuprofen,

Cutaneous vasculitis [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

For acetyl salicylic acid and ibuprofen, the event is confounded by Voltaren dispers.

Reporter's comment:

NOVARTIS, 11.07.2011 DE-NOVNCHHQ-PHHY2011DE37257

Sender diagnosis: not reported

Narrative

Case number PHHY2011DE37257, is an initial spontaneous report received from physician via phone call on 28 Apr 2011 with follow up received on 27 Jun 2011: This report refers to a 33-year-old female patient. This polymedicated patient had medical history of amoxicillin allergy. Concomitant medication included Vitamin C and Folio forte tablets The patient received Voltaren dispers (diclofenac free acid) tablets orally for headache, vertebral pain and pain from ulcers since 2001 approximately one to two tablets daily. The patient also received ibuprofen (manufacturer unknown) 600 mg and acetylsalicylic acid (manufacturer unknown) 500 mg orally. In end of 2004, the patient developed cutaneous vasculitis at both ankles. The patient developed ulceration on ankles. A specimen of ulcerated skin tissue showed capillary inflammation and thrombosis of capillaries. Peripheral arterial disease and venous disorders had been excluded. The diameter of ulcerations was approximately 5 mm and was relapsing every 3 to 10 months until recently. Ulcerations have merged with a diameter of a single lesion of up to 10 mm. Vasculitis was treated with prednisolone 60 mg and pain with Valoron drops orally. No new lesion had occurred since cessation of voltaren dispers on May 2011. The patient had not yet recovered and sequelae of scar and hyperpigmentation have remained. The causality was assessed by reporting physician as likely to be related to Voltaren dispers, however ibuprofen 600 mg and acetylsalicylic acid 500 mg were mentioned as possible alternative explanations.

Follow-up information received from physician on 27 Jun 2011: Suspect drug changed from Voltaren resinat to Voltaren dispers, updated start date of suspect drug, indication, concomitant medication, medical history of allergy, events, treatment medication, co suspect medication, action taken with suspect drug and causality.

Sender's comment:

Serious spontaneous report,

For Voltaren dispers,

Cutaneous vasculitis, [medically significant] assessed as listed according to the Core Data Sheet.

For acetyl salicylic acid and ibuprofen,

Cutaneous vasculitis [medically significant], assessed as unlisted according to the Core Data Sheet. The information provided in this individual case does not warrant a change to the Core Data Sheet text. The topic will be monitored closely.

All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

For acetyl salicylic acid and ibuprofen, the event is confounded by Voltaren dispers.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11091661

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 41 Year	weight (kg): 56	BMI_int: 21.87	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Desquamation	20110430			1 Day	Unknown
2 Skin lesion	20110430			1 Day	Unknown
3 Pruritus	20110430			1 Day	Unknown
4 Skin red	20110430			1 Day	Unknown
5 Skin rash	20110430			1 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 arthrex Schmerzgel	Cutaneous	1 Df dosage form/Day	20110430-20110430	
diclofenac sodium		95	1 Day	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11128636

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 55 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering	20110609			5 Day	recovered/resolved
2 Allergy	20110609			5 Day	recovered/resolved
3 Wrong technique in drug usage process					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel	Topical	/	20110605-20110609	
diclofenac diethylamine		95	5 Day	

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)NOVARTIS, 22.06.2011 DE-002147023-CHPA2011DE08145

Sender diagnosis: not reported

Narrative

Initial healthcare professional report received on 16 Jun 2011. This physician reports that a female consumer, born in 1956, developed "severe contact allergy with blistering at her wrist", after the first application (dose unknown) of Voltaren Emulgel (diclofenac diethylammonium salt). It was unknown if the product was taken under medical advice, and was used for an unknown indication. After treatment with steroids NOS, the disorders abated. The product was discontinued. The outcome was reported as reaction resolving. No further information was received. Follow-up information requested in written form in week 24, 2011.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically Significant). The event, blister, was assessed as unlabelled according to the product information. The event, Hypersensitivity, was already labelled in the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:NOVARTIS, 20.09.2011 DE-002147023-CHPA2011DE08145

Sender diagnosis: not reported

Narrative

Initial healthcare professional report received on 16 Jun 2011. This physician reports that a female consumer, born in 1956, developed "severe contact allergy with blistering at her wrist", after the first application (dose unknown) of Voltaren Emulgel (diclofenac diethylammonium salt). It was unknown if the product was taken under medical advice, and was used for an unknown indication. After treatment with steroids NOS, the disorders abated. The product was discontinued. The outcome was reported as reaction resolving. No further information was received. Follow-up information

requested in written form in week 24, 2011.

Multiple attempts to obtain additional information have been unsuccessful. Case was closed with lost to follow-up.

Follow-up healthcare professional report received on 15 Sep 2011. A dermatologist reported that a 55 year-old female patient developed severe contact allergy with a subsequent positive epicutan test result for diclofenac allergy (date not reported) after topical application of Voltaren Emulgel (indication unknown). The onset date was reported as 09 Jun 2011. The stated daily dose was "locally rubbed in, unspecified bandage". Voltaren Emulgel was discontinued. Following treatment with "Prednicarbat ointment" and Steroids NOS, the symptoms lasted altogether for several days. Causality rating of reporter was probable. Outcome was reported as recovered. No additional information was provided.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically Significant). The event, blister, was assessed as unlabelled according to the product information. The event, Hypersensitivity, was already labelled in the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11129476

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials: Sex: Female Birth: privacy Age: 55 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Wrong technique in drug usage process					Unknown
2 Blistering	20110609			5 Day	recovered/resolved
3 Allergy	20110609			5 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/ 95	20110605-20110609 5 Day	

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 22.06.2011 DE-NOVNCHHQ-CHPA2011DE08145

Sender diagnosis: not reported

Narrative

Initial healthcare professional report received on 16 Jun 2011. This physician reports that a female consumer, born in 1956, developed "severe contact allergy with blistering at her wrist", after the first application (dose unknown) of Voltaren Emulgel (diclofenac diethylammonium salt). It was unknown if the product was taken under medical advice, and was used for an unknown indication. After treatment with steroids NOS, the disorders abated. The product was discontinued. The outcome was reported as reaction resolving. No further information was received. Follow-up information requested in written form in week 24, 2011.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically Significant). The event, blister, was assessed as unlabelled according to the product information. The event, Hypersensitivity, was already labelled in the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

NOVARTIS, 20.09.2011 DE-NOVNCHHQ-CHPA2011DE08145

Sender diagnosis: not reported

Narrative

Initial healthcare professional report received on 16 Jun 2011. This physician reports that a female consumer, born in 1956, developed "severe contact allergy with blistering at her wrist", after the first application (dose unknown) of Voltaren Emulgel (diclofenac diethylammonium salt). It was unknown if the product was taken under medical advice, and was used for an unknown indication. After treatment with steroids NOS, the disorders abated. The product was discontinued. The outcome was reported as reaction resolving. No further information was received. Follow-up information

requested in written form in week 24, 2011.

Multiple attempts to obtain additional information have been unsuccessful. Case was closed with lost to follow-up.

Follow-up healthcare professional report received on 15 Sep 2011. A dermatologist reported that a 55 year-old female patient developed severe contact allergy with a subsequent positive epicutan test result for diclofenac allergy (date not reported) after topical application of Voltaren Emulgel (indication unknown). The onset date was reported as 09 Jun 2011. The stated daily dose was "locally rubbed in, unspecified bandage". Voltaren Emulgel was discontinued. Following treatment with "Prednicarbat ointment" and Steroids NOS, the symptoms lasted altogether for several days. Causality rating of reporter was probable. Outcome was reported as recovered. No additional information was provided.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically Significant). The event, blister, was assessed as unlabelled according to the product information. The event, Hypersensitivity, was already labelled in the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11163931

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 71 Year	weight (kg): 79	BMI_int: 24.38	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Drug ineffective for unapproved indication	20110725	20110726	2 Day	1 Day	Unknown
2 Bullous dermatitis	20110727	20110805	10 Day	3 Day	recovered/resolved
3 Application site discharge	20110727	20110805	10 Day	3 Day	recovered/resolved
4 Application site redness	20110726	20110805	11 Day	2 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN WIRKSTOFF-PFLASTER diclofenac epolaminum	Transdermal	/ 250	20110725-20110726 2 Day	Sciatica

Primary source

Qualification: Physician observestudytype:

Literature:

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 17.08.2011 DE-NOVNCHHQ-CHPA2011DE10433

Sender diagnosis: not reported

Narrative

Initial physician report received on 02 Aug 2011. This physician reported that a male patient who was born in 1939 and had a history of allergic reactions to Voltaren Emulgel (diclofenac diethylammonium salt), which were not confirmed in allergy tests, used Voltaren Wirkstoff-Pflaster (diclofenac epolamine) around end of Jul 2011 and developed "pronounced dermatitis at the application site (below the glued surface)". The product was discontinued in end of Jul 2011. Consumer had not yet recovered at the time of this report. No further information was available at date of this report. Follow-up information was requested in written form in week on 04 Aug 2011. (Related case: CHPA2011DE10437)

Follow up information received from a physician (follow-up report form) and consumer (letter) on 10 Aug 2011 upgraded the case to meet the serious criterion of medically significant. This 71 year old male patient had a history of an "allergic local skin reaction" after use of Voltaren Emulgel (diclofenac diethylamine) a few years ago and specified that the allergy testing for diclofenac was negative. Per physician, the patient started to use Voltaren Wirkstoff-Pflaster (diclofenac epolamine) (batch no 1012012) 1 patch daily on 27 Jul 2011 for "left-sided lumbar sciatic pain". However, the patient stated that he noticed pain in left leg especially during night time on 23 and 24 Jul 2011 so that he presumed muscle tenseness in gluteal muscle and that he applied Voltaren Wirkstoff-Pflaster at the central area of pre-existing pain in the evening on 25 Jul 2011. Patient reported that he removed the patch and noticed "skin redness" in the morning of 26 Jul 2011. He visited the local thermal spa and sauna on 26 Jul 2011. Wirkstoff-Pflaster was stopped on 26 Jul 2011. On 27 Jul 2011, he noticed "blisters at the application side" which had partly opened spontaneously during the night and added that the skin area at the application side of the patch was severely weeping. As per physician, he developed "dermatitis", described as "bullous dermatitis" after patch application of Voltaren Wirkstoff-Pflaster. The patient consulted his general practitioner on 28 Jul 2011. The physician treated the bullous dermatitis by opening/lancing the closed blisters (internal hint: rated as significant) and subsequently covering skin area with "Urgotl Silver" and absorbant dressings. On 02 Aug 2011, the wound was dry and had almost healed up completely. On 03 Aug 2011, skin rags dropped from healed-up skin. On 05 Aug 2011, the skin was rated as healed-up by the treating physician. The patient added that he "did not notice a pain relieving effect" after application of Voltaren Wirkstoff-Pflaster. Per physician, the adverse symptoms lasted all together for 8 days. Patient had recovered from skin redness, blister, and weeping. Outcome of "no pain relieving effect" was not reported. Causality rating of reporting physician was probable. (Related case: CHPA2011DE10437)

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant). The event, Dermatitis bullous and all associated symptoms, was assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:NOVARTIS, 17.10.2011 DE-NOVNCHHQ-CHPA2011DE10433

Sender diagnosis: not reported

Narrative

Initial physician report received on 02 Aug 2011. This physician reported that a male patient who was born in 1939 and had a history of allergic reactions to Voltaren Emulgel (diclofenac diethylammonium salt), which were not confirmed in allergy tests, used Voltaren Wirkstoff-Pflaster (diclofenac epolamine) around end of Jul 2011 and developed "pronounced dermatitis at the application site (below the glued surface)". The product was discontinued in end of Jul 2011. Consumer had not yet recovered at the time of this report. No further information was available at date of this report. Follow-up information was requested in written form in week on 04 Aug 2011. (Related case: CHPA2011DE10437)

Follow up information received from a physician (follow-up report form) and consumer (letter) on 10 Aug 2011 upgraded the case to meet the serious criterion of medically significant. This 71 year old male patient had a history of an "allergic local skin reaction" after use of Voltaren Emulgel (diclofenac diethylamine) a few years ago and specified that the allergy testing for diclofenac was negative. Per physician, the patient started to use Voltaren Wirkstoff-Pflaster (diclofenac epolamine) (batch no 1012012) 1 patch daily on 27 Jul 2011 for "left-sided lumbar sciatic pain". However, the patient stated that he noticed pain in left leg especially during night time on 23 and 24 Jul 2011 so that he presumed muscle tenseness in gluteal muscle and that he applied Voltaren Wirkstoff-Pflaster at the central area of pre-existing pain in the evening on 25 Jul 2011. Patient reported that he removed the patch and noticed "skin redness" in the morning of 26 Jul 2011. He visited the local thermal spa and sauna on 26 Jul 2011. Wirkstoff-Pflaster was stopped on 26 Jul 2011. On 27 Jul 2011, he noticed "blisters at the application side" which had partly opened spontaneously during the night and added that the skin area at the application side of the patch was severely weeping. As per physician, he developed "dermatitis", described as "bullous dermatitis" after patch application of Voltaren Wirkstoff-Pflaster. The patient consulted his general practitioner on 28 Jul 2011. The physician treated the bullous dermatitis by opening/lancing the closed blisters (internal hint: rated as significant) and subsequently covering skin area with "Urgotll Silver" and absorbant dressings. On 02 Aug 2011, the wound was dry and had almost healed up completely. On 03 Aug 2011, skin rags dropped from healed-up skin. On 05 Aug 2011, the skin was rated as healed-up by the treating physician. The patient added that he "did not notice a pain relieving effect" after application of Voltaren Wirkstoff-Pflaster. Per physician, the adverse symptoms lasted all together for 8 days. Patient had recovered from skin redness, blister, and weeping. Outcome of "no pain relieving effect" was not reported. Causality rating of reporting physician was probable. (Related case: CHPA2011DE10437)

Follow up received on 13 Oct 2011 consisted of technical complaint investigation results (10725974). All values were within specification at release.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant) confirmed by a health care professional. The event, Dermatitis bullous and all associated symptoms, was assessed as unlabelled according to the product information. The available information was considered inadequate to fully assess the case.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11225221

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 52 Year	weight (kg): 98	BMI_int: 32	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Bullous dermatitis	20120109			73 Day	recovered/resolved
2 Blistering	20111031	20120118	80 Day	3 Day	recovered/resolved
3 Allergic reaction	20111031	20120118	80 Day	3 Day	recovered/resolved
4 Itching	20111031	20120118	80 Day	3 Day	recovered/resolved
5 Skin rash	201110	20120118			recovered/resolved
6 Pain of skin	201110				Unknown
7 Muscle pain	201110				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cetebe		4 Df dosage form/Day	-	
ascorbic acid		5	-	
2 voltaren schmerzgel	Topical	/	20111029-20111030	Muscle tension
diclofenac diethylamine		95	2 Day	

Drugs (concomitant)

3 thyronajod	/	-
levothyroxine sodium; potassium iodide		-
4 VOLTAREN EMULGEL "NOVARTIS"	/	-
diclofenac diethylamine		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Dermatitis			Unknown	Reported as "actinic dermatitis".
Sun sensitivity			Unknown	Reported as "sun allergy".

Primary source

Qualification:	Physician	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 10.11.2011 DE-002147023-CHPA2011DE015619

Sender diagnosis: not reported

Narrative

Initial consumer report received on 07 Nov 2011. This female consumer used Voltaren Schmerzgel (diclofenac diethylammonium salt) due to a "muscle tension" in her right arm. It was unknown if the product was taken under medical advice. After the third application within 3 days, she developed a "severe rash NOS persisting tenaciously since one and a half weeks" (Oct 2011). Consumer treated her rash with Anaesthesulf lotio (lauromacrogol 400), Soventol Hydrocort (hydrocortisone) and Fenistil Tropfen (dimetindene maleate). She received "cortisone injections NOS" by her physician for her rash, however, "only with short term improvement without healing". Consumer added that she "has suffered not only from muscle pain but also from skin pain since 3 weeks now." Action taken with product was unknown. Reaction of rash continued. No further information was available at date of report. Follow-up information requested in written form (including consent) on 08 Nov 2011.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) not confirmed by

a health care professional. The adverse event, Rash was assessed as listed according to the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit-risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 04.01.2012 DE-002147023-CHPA2011DE015619

Sender diagnosis: not reported

Narrative

Initial consumer report received on 07 Nov 2011. This female consumer used Voltaren Schmerzgel (diclofenac diethylammonium salt) due to a "muscle tension" in her right arm. It was unknown if the product was taken under medical advice. After the third application within 3 days, she developed a "severe rash NOS persisting tenaciously since one and a half weeks" (Oct 2011). Consumer treated her rash with Anaesthesulf lotio (lauromacrogol 400), Soventol Hydrocort (hydrocortisone) and Fenistil Tropfen (dimetindene maleate). She received "cortisone injections NOS" by her physician for her rash, however, "only with short term improvement without healing". Consumer added that she "has suffered not only from muscle pain but also from skin pain since 3 weeks now." Action taken with product was unknown. Reaction of rash continued. No further information was available at date of report. Follow-up information requested in written form (including consent) on 08 Nov 2011.

Additional information was received from the consumer on 02 Jan 2012. This 52-year-old female patient used Voltaren Schmerzgel due to a "muscle tension/motion pain" in her forearm and applied the drug finely on 29 and 30 Oct 2011 twice daily. On 31 Oct 2011 she developed "an allergic reaction with blistering and severe itching, like varicella or "sun allergy" respectively", which persisted for 3 weeks. The patient discontinued the drug. Upon the recommendation of her family doctor the patient used over one and a half week in each case Anaesthesulf lotio (lauromacrogol 400), 3-times daily, and Fenistil Tropfen (dimetindene maleate), 30 drops twice daily, to treat the adverse events. Her family doctor also administered two "cortisone injections NOS". He also advised her to use Ingelan Puder (isoprenalinsulfate, salicylic acid), highly dosed magnesium and Fucidine® H 20 mg/g + 10 mg/g (fusidic acid / hydrocortisone acetate), but it was not evident from the reported data whether the patient actually used these drugs. Patient added that she did not suffer from such a disease previously, apart from "sun allergy". The patient assessed the causal relationship as highly probably. Patient provided a photo documentation of her adverse skin reaction. No further information available at date of report. Outcome: recovery.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) not confirmed by a health care professional. The adverse events, Hypersensitivity and all associated symptoms and Rash were assessed as listed according to the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 30.01.2012 DE-002147023-CHPA2011DE015619

Sender diagnosis: not reported

Narrative

Initial consumer report received on 07 Nov 2011. This female consumer used Voltaren Schmerzgel (diclofenac diethylammonium salt) due to a "muscle tension" in her right arm. It was unknown if the product was taken under medical advice. After the third application within 3 days, she developed a "severe rash NOS persisting tenaciously since one and a half weeks" (Oct 2011). Consumer treated her rash with Anaesthesulf lotio (lauromacrogol 400), Soventol Hydrocort (hydrocortisone) and Fenistil Tropfen (dimetindene maleate). She received "cortisone injections NOS" by her physician for her rash, however, "only with short term improvement without healing". Consumer added that she "has suffered not only from muscle pain but also from skin pain since 3 weeks now." Action taken with product was unknown. Reaction of rash continued. No further information was available at date of report. Follow-up information requested in written form (including consent) on 08 Nov 2011.

Additional information was received from the consumer on 02 Jan 2012. This 52-year-old female patient used Voltaren Schmerzgel due to a "muscle tension/motion pain" in her forearm and applied the drug finely on 29 and 30 Oct 2011 twice daily. On 31 Oct 2011 she developed "an allergic reaction with blistering and severe itching, like varicella or "sun allergy" respectively", which persisted for 3 weeks. The patient discontinued the drug. Upon the recommendation of her family doctor the patient used over one and a half week in each case Anaesthesulf lotio (lauromacrogol 400), 3-times daily, and Fenistil Tropfen (dimetindene maleate), 30 drops twice daily, to treat the adverse events. Her family doctor also administered two "cortisone injections NOS". He also advised her to use Ingelan Puder (isoprenalinsulfate, salicylic acid), highly dosed magnesium and Fucidine® H 20 mg/g + 10 mg/g (fusidic acid / hydrocortisone acetate), but it was not evident from the reported data whether the patient actually used these drugs. Patient added that she did not suffer from such a disease previously, apart from "sun allergy". The patient assessed the causal relationship as highly probably. Patient provided a photo documentation of her adverse skin reaction. No further information available at date of report. Outcome: recovery.

Additional information received on 23 Jan 2012. This 52 year old female patient reported that she developed "consecutive symptoms after use of Voltaren Schmerzgel" nearer described as "the same symptoms like after use of Voltaren Schmerzgel (rubbed in) after intake of the first capsule of Cetebe Vitamin C Retard 500 (batch number 900423)". The consecutive symptoms was described as "again skin rash on whole lower arm and on left hand" and rated as "bullous dermatitis". The onset date of these consecutive symptoms was specified as "09 Jan 2012". The daily dose for Cetebe Vitamin C Retard 500 was specified as "1 capsule in the morning and 1 capsule in the evening". The reporter specified that the reaction was treated like the reaction before with 'the rest of the tube of Fucidine H 20/+10 (fusidic acid / hydrocortisone acetate)' and that the reactions had not disappeared completely until 18 Jan 2012. Outcome for consecutive symptoms were "not reported". However, it was internally rated as outcome for initial reactions as recovery and outcome for consecutive symptoms as improvement.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) not confirmed by a health care professional. The adverse events, Hypersensitivity and all associated symptoms and Rash were assessed as listed according to the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 27.02.2012 DE-002147023-CHPA2011DE015619

Sender diagnosis: not reported

Narrative

Initial consumer report received on 07 Nov 2011. This female consumer used Voltaren Schmerzgel (diclofenac diethylammonium salt) due to a "muscle tension" in her right arm. It was unknown if the product was taken under medical advice. After the third application within 3 days, she developed a "severe rash NOS persisting tenaciously since one and a half weeks" (Oct 2011). Consumer treated her rash with Anaesthesulf lotio (lauromacrogol 400), Soventol Hydrocort (hydrocortisone) and Fenistil Tropfen (dimetindene maleate). She received "cortisone injections NOS" by her physician for her rash, however, "only with short term improvement without healing". Consumer added that she "has suffered not only from muscle pain but also from skin pain since 3 weeks now." Action taken with product was unknown. Reaction of rash continued. No further information was available at date of report. Follow-up information requested in written form (including consent) on 08 Nov 2011.

Additional information was received from the consumer on 02 Jan 2012. This 52-year-old female patient used Voltaren Schmerzgel due to a "muscle tension/motion pain" in her forearm and applied the drug finely on 29 and 30 Oct 2011 twice daily. On 31 Oct 2011 she developed "an allergic reaction with blistering and severe itching, like varicella or "sun allergy" respectively", which persisted for 3 weeks. The patient discontinued the drug. Upon the recommendation of her family doctor the patient used over one and a half week in each case Anaesthesulf lotio (lauromacrogol 400), 3-times daily, and Fenistil Tropfen (dimetindene maleate), 30 drops twice daily, to treat the adverse events. Her family doctor also administered two "cortisone injections NOS". He also advised her to use Ingelan Puder (isoprenalinsulfate, salicylic acid), highly dosed magnesium and Fucidine® H 20 mg/g + 10 mg/g (fusidic acid / hydrocortisone acetate), but it was not evident from the reported data whether the patient actually used these drugs. Patient added that she did not suffer from such a disease previously, apart from "sun allergy". The patient assessed the causal relationship as highly probably. Patient provided a photo documentation of her adverse skin reaction. No further information available at date of report. Outcome: recovery.

Additional information received on 23 Jan 2012. This 52 year old female patient reported that she developed "consecutive symptoms after use of Voltaren Schmerzgel" nearer described as "the same symptoms like after use of Voltaren Schmerzgel (rubbed in) after intake of the first capsule of Cetebe Vitamin C Retard 500 (batch number 900423)". The consecutive symptoms was described as "again skin rash on whole lower arm and on left hand" and rated as "bullous dermatitis". The onset date of these consecutive symptoms was specified as "09 Jan 2012". The daily dose for Cetebe Vitamin C Retard 500 was specified as "1 capsule in the morning and 1 capsule in the evening". The reporter specified that the reaction was treated

like the reaction before with 'the rest of the tube of Fucidine H 20/+10 (fusidic acid / hydrocortisone acetate)' and that the reactions had not disappeared completely until 18 Jan 2012. Outcome for consecutive symptoms were "not reported". However, it was internally rated as outcome for initial reactions as recovery and outcome for consecutive symptoms as improvement.

Additional information received on 20 Feb 2012. This dermatologist reported that the 52 year-old female patient developed "skin rash" after topical application of Voltaren Schmerzgel (diclofenac diethylammonium salt). Voltaren Schmerzgel was stopped by the patient. The dermatologist added that the patient visited the dermatologist only once on 04 Nov 2011 and that the following information was unknown to the dermatologist: batch number, onset date of reaction, duration of reaction, exact indication for use (it was only known that Voltaren Schmerzgel was prescribed by a different physician), therapy dates, other relevant history, concomitant drugs and date of administration. The reporting dermatologist assessed the causal relationship as probable. The dermatologist prescribed Fucidine H Salbe to treat the skin rash. Final outcome was not reported with handwritten note: "The patient did not consult the dermatologist again".

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) not confirmed by a health care professional. The adverse events, Hypersensitivity and all associated symptoms and Rash were assessed as listed according to the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-11225341

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 52 Year	weight (kg): 98	BMI_int: 32	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Muscle pain	201110				Unknown
2 Pain of skin	201110				Unknown
3 Allergic reaction	20111031	20120118	80 Day	3 Day	recovered/resolved
4 Bullous dermatitis	20120109			73 Day	recovered/resolved
5 Itching	20111031	20120118	80 Day	3 Day	recovered/resolved
6 Blistering	20111031	20120118	80 Day	3 Day	recovered/resolved
7 Skin rash	201110	20120118			recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 cetebe		4 Df dosage form/Day	-	
ascorbic acid		5	-	
2 voltaren schmerzgel	Topical	/	20111029-20111030	Muscle tension
diclofenac diethylamine		95	2 Day	

Drugs (concomitant)

3 thyronajod	/	-
levothyroxine sodium; potassium iodide		-
4 VOLTAREN EMULGEL "NOVARTIS"	/	-
diclofenac diethylamine		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Dermatitis			Unknown	Reported as "actinic dermatitis".
Sun sensitivity			Unknown	Reported as "sun allergy".
Pain			k.A.	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 11.11.2011 DE-NOVNCHHQ-CHPA2011DE015619

Sender diagnosis: not reported

Narrative

Initial consumer report received on 07 Nov 2011. This female consumer used Voltaren Schmerzgel (diclofenac diethylammonium salt) due to a "muscle tension" in her right arm. It was unknown if the product was taken under medical advice. After the third application within 3 days, she developed a "severe rash NOS persisting tenaciously since one and a half weeks" (Oct 2011). Consumer treated her rash with Anaesthesulf lotio (lauromacrogol 400), Soventol Hydrocort (hydrocortisone) and Fenistil Tropfen (dimetindene maleate). She received "cortisone injections NOS" by her physician for her rash, however, "only with short term improvement without healing". Consumer added that she "has suffered not only from muscle pain but also from skin pain since 3 weeks now." Action taken with product was unknown. Reaction of rash continued. No further information was available at date of report. Follow-up information requested in written form (including consent) on 08 Nov 2011.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) not confirmed by a health care professional. The adverse event, Rash was assessed as listed according to the Reference Safety Information. While a single

spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit-risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 04.01.2012 DE-NOVNCHHQ-CHPA2011DE015619

Sender diagnosis: not reported

Narrative

Initial consumer report received on 07 Nov 2011. This female consumer used Voltaren Schmerzgel (diclofenac diethylammonium salt) due to a "muscle tension" in her right arm. It was unknown if the product was taken under medical advice. After the third application within 3 days, she developed a "severe rash NOS persisting tenaciously since one and a half weeks" (Oct 2011). Consumer treated her rash with Anaesthesulf lotio (lauromacrogol 400), Soventol Hydrocort (hydrocortisone) and Fenistil Tropfen (dimetindene maleate). She received "cortisone injections NOS" by her physician for her rash, however, "only with short term improvement without healing". Consumer added that she "has suffered not only from muscle pain but also from skin pain since 3 weeks now." Action taken with product was unknown. Reaction of rash continued. No further information was available at date of report. Follow-up information requested in written form (including consent) on 08 Nov 2011.

Additional information was received from the consumer on 02 Jan 2012. This 52-year-old female patient used Voltaren Schmerzgel due to a "muscle tension/motion pain" in her forearm and applied the drug finely on 29 and 30 Oct 2011 twice daily. On 31 Oct 2011 she developed "an allergic reaction with blistering and severe itching, like varicella or "sun allergy" respectively", which persisted for 3 weeks. The patient discontinued the drug. Upon the recommendation of her family doctor the patient used over one and a half week in each case Anaesthesulf lotio (lauromacrogol 400), 3-times daily, and Fenistil Tropfen (dimetindene maleate), 30 drops twice daily, to treat the adverse events. Her family doctor also administered two "cortisone injections NOS". He also advised her to use Ingelan Puder (isoprenalinsulfate, salicylic acid), highly dosed magnesium and Fucidine® H 20 mg/g + 10 mg/g (fusidic acid / hydrocortisone acetate), but it was not evident from the reported data whether the patient actually used these drugs. Patient added that she did not suffer from such a disease previously, apart from "sun allergy". The patient assessed the causal relationship as highly probably. Patient provided a photo documentation of her adverse skin reaction. No further information available at date of report. Outcome: recovery.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) not confirmed by a health care professional. The adverse events, Hypersensitivity and all associated symptoms and Rash were assessed as listed according to the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 30.01.2012 DE-NOVNCHHQ-CHPA2011DE015619

Sender diagnosis: not reported

Narrative

Initial consumer report received on 07 Nov 2011. This female consumer used Voltaren Schmerzgel (diclofenac diethylammonium salt) due to a "muscle tension" in her right arm. It was unknown if the product was taken under medical advice. After the third application within 3 days, she developed a "severe rash NOS persisting tenaciously since one and a half weeks" (Oct 2011). Consumer treated her rash with Anaesthesulf lotio (lauromacrogol 400), Soventol Hydrocort (hydrocortisone) and Fenistil Tropfen (dimetindene maleate). She received "cortisone injections NOS" by her physician for her rash, however, "only with short term improvement without healing". Consumer added that she "has suffered not only from muscle pain but also from skin pain since 3 weeks now." Action taken with product was unknown. Reaction of rash continued. No further information was available at date of report. Follow-up information requested in written form (including consent) on 08 Nov 2011.

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Additional information received on 23 Jan 2012. This 52 year old female patient reported that she developed "consecutive symptoms after use of Voltaren Schmerzgel" nearer described as "the same symptoms like after use of Voltaren Schmerzgel (rubbed in) after intake of the first capsule of Cetebe Vitamin C Retard 500 (batch number 900423)". The consecutive symptoms was described as "again skin rash on whole lower arm and on left hand" and rated as "bullous dermatitis". The onset date of these consecutive symptoms was specified as "09 Jan 2012". The daily dose for Cetebe Vitamin C Retard 500 was specified as "1 capsule in the morning and 1 capsule in the evening". The reporter specified that the reaction was treated like the reaction before with 'the rest of the tube of Fucidine H 20/+10 (fusidic acid / hydrocortisone acetate)' and that the reactions had not disappeared completely until 18 Jan 2012. Outcome for consecutive symptoms were "not reported". However, it was internally rated as outcome for initial reactions as recovery and outcome for consecutive symptoms as improvement.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) not confirmed by a health care professional. The adverse events, Hypersensitivity and all associated symptoms and Rash were assessed as listed according to the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 27.02.2012 DE-NOVNCHHQ-CHPA2011DE015619

Sender diagnosis: not reported

Narrative

Initial consumer report received on 07 Nov 2011. This female consumer used Voltaren Schmerzgel (diclofenac diethylammonium salt) due to a "muscle tension" in her right arm. It was unknown if the product was taken under medical advice. After the third application within 3 days, she developed a "severe rash NOS persisting tenaciously since one and a half weeks" (Oct 2011). Consumer treated her rash with Anaesthesulf lotio (lauromacrogol 400), Soventol Hydrocort (hydrocortisone) and Fenistil Tropfen (dimetindene maleate). She received "cortisone injections NOS" by her physician for her rash, however, "only with short term improvement without healing". Consumer added that she "has suffered not only from muscle pain but also from skin pain since 3 weeks now." Action taken with product was unknown. Reaction of rash continued. No further information was available at date of report. Follow-up information requested in written form (including consent) on 08 Nov 2011.

Additional information was received from the consumer on 02 Jan 2012. This 52-year-old female patient used Voltaren Schmerzgel due to a "muscle tension/motion pain" in her forearm and applied the drug finely on 29 and 30 Oct 2011 twice daily. On 31 Oct 2011 she developed "an allergic reaction with blistering and severe itching, like varicella or "sun allergy" respectively", which persisted for 3 weeks. The patient discontinued the drug. Upon the recommendation of her family doctor the patient used over one and a half week in each case Anaesthesulf lotio (lauromacrogol 400), 3-times daily, and Fenistil Tropfen (dimetindene maleate), 30 drops twice daily, to treat the adverse events. Her family doctor also administered two "cortisone injections NOS". He also advised her to use Ingelan Puder (isoprenalinsulfate, salicylic acid), highly dosed magnesium and Fucidine® H 20 mg/g + 10 mg/g (fusidic acid / hydrocortisone acetate), but it was not evident from the reported data whether the patient actually used these drugs. Patient added that she did not suffer from such a disease previously, apart from "sun allergy". The patient assessed the causal relationship as highly probably. Patient provided a photo documentation of her adverse skin reaction. No further information available at date of report. Outcome: recovery.

Additional information received on 23 Jan 2012. This 52 year old female patient reported that she developed "consecutive symptoms after use of Voltaren Schmerzgel" nearer described as "the same symptoms like after use of Voltaren Schmerzgel (rubbed in) after intake of the first capsule of Cetebe Vitamin C Retard 500 (batch number 900423)". The consecutive symptoms was described as "again skin rash on whole lower arm and on left hand" and rated as "bullous dermatitis". The onset date of these consecutive symptoms was specified as "09 Jan 2012". The daily dose for Cetebe Vitamin C Retard 500 was specified as "1 capsule in the morning and 1 capsule in the evening". The reporter specified that the reaction was treated like the reaction before with 'the rest of the tube of Fucidine H 20/+10 (fusidic acid / hydrocortisone acetate)' and that the reactions had not

disappeared completely until 18 Jan 2012. Outcome for consecutive symptoms were "not reported". However, it was internally rated as outcome for initial reactions as recovery and outcome for consecutive symptoms as improvement.

Additional information received on 20 Feb 2012. This dermatologist reported that the 52 year-old female patient developed "skin rash" after topical application of Voltaren Schmerzgel (diclofenac diethylammonium salt). Voltaren Schmerzgel was stopped by the patient. The dermatologist added that the patient visited the dermatologist only once on 04 Nov 2011 and that the following information was unknown to the dermatologist: batch number, onset date of reaction, duration of reaction, exact indication for use (it was only known that Voltaren Schmerzgel was prescribed by a different physician), therapy dates, other relevant history, concomitant drugs and date of administration. The reporting dermatologist assessed the causal relationship as probable. The dermatologist prescribed Fucidine H Salbe to treat the skin rash. Final outcome was not reported with handwritten note: "The patient did not consult the dermatologist again".

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) not confirmed by a health care professional. The adverse events, Hypersensitivity and all associated symptoms and Rash were assessed as listed according to the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12006430

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No	No	No	No	No	No	No		

Patient

Initials: Sex: Male Birth: privacy Age: 76 Year weight (kg): 78 BMI_int: 24.07 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Swelling	20111230		7 Day	14 Day	not recovered/not resolved
2 Skin erosion	20111230		7 Day	14 Day	not recovered/not resolved
3 Burning sensation	20111230		7 Day	14 Day	not recovered/not resolved
4 Itching	20111230		7 Day	14 Day	not recovered/not resolved
5 Skin tightness	20111230		7 Day	14 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Solaraze 3% Gel diclofenac sodium	Topical	/	20111217-20111230 14 Day	Actinic keratosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Senile keratosis			Unknown	
Actinic keratosis			Unknown	
Hypertension			Unknown	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12051132

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 61 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Itch burning	20120229				not recovered/not resolved
2 Skin peeling	20120229				not recovered/not resolved
3 Skin red	20120229				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
unbekannt			k.A.	Entzündung des Handgelenks

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12055703

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 45 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Unknown cause of death
deathdate:	deathcause: Toxic epidermal necrolysis

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	199901				fatal
2 TEN	199901				fatal
3 Stevens-Johnson syndrome					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 amoxicillin	Unknown	/	-	Drug use for unknown indication
amoxicillin			-	indication
2 AZATHIOPRINE	Unknown	/	-	Drug use for unknown indication
azathioprine			-	indication
3 diclofenac	Unknown	/	-	Drug use for unknown indication
diclofenac			-	indication
4 prednisolone	Unknown	/	-	Drug use for unknown indication
prednisolone			-	indication
5 prednisone	Unknown	/	-	Drug use for unknown indication
prednisone			-	indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Systemic lupus erythematosus	1980		Yes	in 1980

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Ziemer, Mirjana. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology. 2012;166(3):575-600	

Narratives (nur bei elektronischem Eingang)

PFIZERINC, 16.03.2012 DE-PFIZER INC-2012064981

Sender diagnosis: not reported

Narrative

This is a literature report from the British Journal of Dermatology, 2012, vol. 166 (3), pgs. 575-600, entitled, Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature.

For this retrospective analysis, cases of Stevens-Johnson syndrome (SJS)/ toxic epidermal necrolysis (TEN), ascertained and validated by the German registry, based on a large population-based national registry, were used to evaluate clinically and histopathologically probable or definite cases of SJS/TEN with a history of systemic or other lupus erythematosus [(S)LE]. All cases are represented by patients of European descent. Among 1366 patients with SJS/TEN, 17 with a sufficiently documented history of (S)LE and representative histological material could be identified, suggesting a considerable over-representation of LE in patients with SJS/TEN. Eight of these showed clinically and/or histopathologically some LE-characteristic features interfering with the diagnosis of SJS/TEN. Differentiation could be elaborated on clinical and histopathological grounds: four patients were classified as SJS/TEN with a preceding (S)LE exacerbation and/or LE-typical histopathological features, and four as TEN-like (S)LE. Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

This case refers to patient # 10 in table 2.

A 45-year-old-female patient was diagnosed with definite TEN in Jan1999 after use of amoxicillin and diclofenac. The medical history was relevant for LE, which duration time was: in 1980 SLE.

LE criteria clinically and previous changes: Antiphospholipid syndrome, involvement of skin, heart, lung, kidney; last exacerbation 1995 with extensive skin and liver involvement.

LE characteristic laboratory values: Antiphospholipid, elevated transaminases.

She received immunosuppressive therapy before event: prednisolone/prednisone, azathioprine.

She had no skin lesions before event.

The time duration index-day to day of the maximum blisters/erosions period was 7.

The skin involvement was relevant for: Atypical targets flat, spots widespread.

There was no LE typical photodistribution.

There was involvement of palms, soles.

There was involvement of mucous membranes: lips, oral.

Histopathological characteristics (biopsy) presented multiple necrotic keratinocytes within entire epidermis and complete epidermal necrosis; superficial lymphocytic infiltrate.

The outcome was death.

Potential cause of SJS/TEN: Possible amoxicillin, diclofenac.

The authors stated that the potential cause of SJS/TEN was possible amoxicillin and diclofenac.

The authors concluded that most patients with SJS/TEN and a history of (S)LE demonstrate clinical and histopathological properties allowing clear differentiation. However, occasionally acute cutaneous manifestations of (S)LE and SJS/TEN can be phenotypically similar, caused by extensive epidermal necrosis. Although no feature by itself is conclusive, a combination of recent (S)LE exacerbation, evident photodistribution, annular lesions and absent or only mild focal erosive mucosal involvement may favour LE over SJS/TEN clinically. Histopathologically, in particular, junctional vacuolar alteration, and the presence of solitary necrotic keratinocytes at lower epidermal levels, combined with moderate to dense peridnexal and perivascular lymphocytic infiltrates with a variable presence of melanophages, and mucin point to a LE-related origin.

Sender's comment:

Based on the information available in the case, the Company considers this individual report would not modify the benefit/risk profile of the suspect drug.

Reporter's comment:

sanofi-aventis, 23.03.2012 DE-SANOFI-AVENTIS-2012SA019613

Sender diagnosis: not reported

Narrative

This case is issued from a publication in which sixteen other cases were reported: 2012SA017469, 2012SA019605, 2012SA019606, 2012SA019607, 2012SA019608, 2012SA019609, 2012SA019610, 2012SA019611, 2012SA019612, 2012SA019614, 2012SA019615, 2012SA019616, 2012SA019617, 2012SA019618, 2012SA019619 and 2012SA019620 (cluster).

This unsolicited literature case was identified on 13-March-2012 via an article, Ziemer M, Kardaun S H, Liss Y, Mockenhaupt M, Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology 2012; 166(3):575-600.

This case concerns a 45 year old female patient (patient 10) who died due to toxic epidermal necrolysis whilst receiving prednisolone, prednisone, amoxicillin, diclofenac sodium (diclofenac) and azathioprine all taken for unknown indications.

On unknown dates, the patient commenced treatment with prednisolone, prednisone, amoxicillin, diclofenac sodium and azathioprine (route, dose, form, frequency, expiry date and lot no. unknown for all). In January 1999, she experienced definite toxic epidermal necrolysis and unspecified time later she was hospitalized. She had flat targets, widespread spots on the palms, soles involving lips and oral mucous membrane. Seven days after the index day the patient developed maximum blisters and erosions. In the histology sample, the patient did not have hyperorthokeratosis, intrafollicular hyperkeratosis, atrophic epidermis, solitary necrotic keratinocytes within lower epidermis, vacuolar degeneration, thickened basement membrane, plasma cells, peri-adnexal involvement of sweat glands, necrotic keratinocytes and lymphocytic infiltrate, melanophages, and mucin. She had multiple necrotic keratinocytes within entire epidermis and complete epidermal necrosis. She had sparse and superficial lymphocytic infiltrate. Peri-adnexal involvement included infundibular necrotic keratinocytes and there was no considerable inflammatory infiltrate. On an unknown date, she died due to toxic epidermal necrolysis. No autopsy information was provided.

The therapy status of prednisolone, prednisone, amoxicillin, diclofenac sodium and azathioprine at the time of patient's death was not reported.

The patient's medical history included systemic lupus erythematosus in 1980, exacerbation of lupus erythematosus in 1995 with extensive skin and liver involvement and antiphospholipid syndrome with the involvement of skin, heart, lung and kidney. She did not have skin lesions before the event. No past drugs, concomitant drugs, or concurrent conditions were reported.

The author stated that the potential cause of toxic epidermal necrolysis was possible use of amoxicillin and diclofenac sodium. The author further stated that other co-dependent risk factors included immunosuppressive drugs.

Sender's comment:

Sanofi-Aventis company comment dated 23rd Mar 2012: This case concerns a 45 year old female patient who died due to toxic epidermal necrolysis whilst receiving prednisolone, prednisone, amoxicillin, diclofenac sodium and azathioprine. In this case based on the temporal association although the role of drugs cannot be denied for the event of toxic epidermal necrolysis leading to fatal outcome, however their individual role cannot be assessed in isolation. Contributory role of co-suspect drug azathioprine cannot be underestimated either. Autopsy report will aid in comprehensive assessment of this case.

Reporter's comment:

STADA Arzneimittel AG, 09.05.2012 DE-STADA-046837

Sender diagnosis: not reported

Narrative

The following information was published in the scientific literature.

Ziener M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. Br-J-Dermatol 2012; 166(3) 575-600 -

Summary:

A 45-year-old woman with systemic lupus erythematosus was receiving amoxicillin, diclofenac, azathioprine and either prednisolone or prednisone. She developed widespread atypical skin target lesions, with involvement of the lip and oral mucous membranes. Skin biopsy findings included multiple necrotic keratinocytes throughout the epidermis. She was diagnosed with definite toxic epidermal necrolysis, with possible causes being amoxicillin and diclofenac. She later died. <dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

This case was classified as serious.

AMOXICILLIN, DICLOFENAC

The causal relationship between the suspected drug and the suspected adverse reaction was assessed as possibly related based on the temporal association and the known safety profile.

The suspected adverse reaction is listed according to the current Reference Safety Informations (RSIs).

This case does not change the overall benefit-risk balance of the medicinal products.

Reporter's comment:

'Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.'

DEXPHARM, 15.05.2012 DE-DEXPHARM-20120439

Sender diagnosis: not reported

Narrative

Report received from literature.

Reference: Ziemer, M.; Kardaun, S. H.; Liss, Y.; Mockenhaupt, M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology 2012, V166, P575-600

A retrospective study identified 15 patients [see table] with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died.

Editorial Comment: The authors identified a further two patients with either TEN or SJS/TEN overlap; however, drug causality could not be assigned in these cases.

Patient 9 from the table:

A 45 year-old female patient with lupus erythematosus developed toxic epidermal necrolysis (TEN) after receiving diclofenac and amoxicillin [details of drug administrations not stated]. After the onset of the skin reaction, biopsy findings included necrotic keratinocytes throughout the epidermis. She subsequently died. Causality was determined as possible for diclofenac and amoxicillin by the author.

Author Comment: "Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

Company assessment: This 45 years old female with lupus erythematosus was taking amoxicillin and diclofenac (durations unspecified) and developed definite TEN. There is little information but it is possible that diclofenac was causally related; it is not possible to distinguish the relative risks associated with the two products.

Sender's comment:Reporter's comment:

RANBAXY, 16.05.2012 DE-RANBAXY-2012RR-56249

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2012RR-56249 "Patient 9", from GERMANY refers to a 45 year-old female who experienced toxic epidermal necrolysis which was fatal, following treatment with amoxicillin and diclofenac.

This case was reported in the literature.

Patient's medical history:

Lupus erythematosus.

The patient received the following suspect product:

Amoxicillin: dosage, route and indication not stated.

Diclofenac: dosage, route and indication not stated.

The patient developed toxic epidermal necrolysis, which was fatal, while receiving amoxicillin and diclofenac.

Verbatim text: "A retrospective study identified 15 patients with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic

epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died.

The authors identified a further two patients with either TEN or SJS/TEN overlap; however, drug causality could not be assigned in these cases."

The outcome of the event was reported as fatal at the time of this report.

The case was considered serious because the patient died.

Author Comment "Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

Sender's comment:

Temporal association suggests a possible causality with each drug use in this case, but for an accurate assessment of the role on each suspect drug more information is necessary .

Toxic epidermal necrolysis is stated in Diclofenac and also in Amoxicillin SmPC.

Stevens-Johnson syndrome (SJS) and toxic epidermal necrolysis (TEN) are severe cutaneous hypersensitivity reactions. Drugs, especially sulfa drugs, antiepileptics, and antibiotics, are the most common causes. The exact mechanism is unknown; however, one theory holds that altered drug metabolism in some patients cause formation of reactive metabolites that bind to and alter cell proteins, triggering a T-cell-mediated cytotoxic reaction to drug antigens in keratinocytes.

Another possible mechanism involves interactions between Fas (a cell-surface receptor that induces apoptosis) and its ligand, particularly a soluble form of Fas ligand released from mononuclear cells. Recent findings suggest that granulysin released from cytotoxic T cells and natural killer cells might play a role in keratinocyte death.

References:

SmPC of Diclofenac and Amoxicillin.

http://www.merckmanuals.com/professional/dermatologic_disorders/hypersensitivity_and_inflammatory_disorders/stevens-johnson_syndrome_sjs_and_toxic_epidermal_necrolysis_ten.html

Reporter's comment:

The case is rated as serious, possible and expected with both suspect drugs

Temmler Pharma GmbH & Co. KG, 21.05.2012 DE-TEMMLER06-005082

Sender diagnosis: not reported

Narrative

This literature case was received by Temmler Pharma on 07 May 2012 from Adis Reactions via literature research:

A 45-year-old woman with systemic lupus erythematosus was receiving amoxicillin, diclofenac, azathioprine and either prednisolone or prednisone. She developed widespread atypical skin target lesions, with involvement of the lip and oral mucous membranes. Skin biopsy findings included multiple necrotic keratinocytes throughout the epidermis. She was diagnosed with definite toxic epidermal necrolysis, with possible causes being amoxicillin and diclofenac. She later died.

Temmler ID: DIC-2012-043<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Company comment Temmler Pharma:

TEN is listed, but not fatal outcome.

This case is medically confirmed and the overall benefit-risk-ratio is not changed.

Reporter's comment:

Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

drreddys, 22.05.2012 DE-drreddys-GER/GER/12/0024373

Sender diagnosis: not reported

Narrative

This is a literature case report which described Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus. This retrospective study identified 17 patients with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs. Five patients later died.

This case pertains to a 45-year-old female patient who developed toxic epidermal necrolysis and subsequently died while on therapy with amoxicillin and diclofenac for an unknown indication.

The patient's medical history included systemic lupus erythematosus in 1980 and anti-phospholipid syndrome (involvement of skin, heart, lung and kidney). Last exacerbation was in 1995 with extensive skin and liver involvement. Concomitant medications included immunosuppressive therapy with prednisolone/prednisone and azathioprine.

On an unspecified date, the patient started amoxicillin and diclofenac at an unspecified route, dose and frequency. In January 1999, the patient developed toxic epidermal necrolysis (Skin involvement: 'Atypical targets flat spots widespread; involvement of palms, soles, lips and oral'). Laboratory results showed anti-phospholipids and elevated transaminases. The patient subsequently died on an unspecified date.

The reporter assessed the events as possibly related to amoxicillin and diclofenac therapy.

The reporter assessed the case as serious (Fatal outcome, Hospitalization).

The reporter commented that 'Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety'.

Literature reference:

Zimer M., Kardaun S.H., Liss Y., Mocjenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology. 2012. 166; 575-600.

Also, please refer GER/GER/12/0024372, GER/GER/12/0024374, GER/GER/12/0024375, GER/GER/12/0024376, GER/GER/12/00024377, GER/GER/12/00024378, GER/GER/12/00024379, GER/GER/12/0024380 and GER/GER/12/0024381 for other patients presented in this article.

Sender's comment:

This patient with systemic lupus erythematosus developed toxic epidermal necrolysis (TEN) and subsequently died while on amoxicillin and diclofenac therapy. Although the concomitant medications could have contributed to the reported event, a role of amoxicillin and diclofenac could not be excluded in the reported event.

This case was considered serious (fatal).

Fatal TEN was unexpected per Amoxicillin and diclofenac reference labels.

Reporter's comment:

KRKAPR, 23.05.2012 DE-KRKA-DE2012K1305LIT

Sender diagnosis: not reported

Narrative

Various drugs

Stevens-Johnson syndrome and toxic epidermal necrolysis: 15 case reports - serious

A retrospective study identified 15 patients [see table] with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died.

Author Comment "Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

Patient: 9

sex: F

age (y): 45

Suspect drugs (causality): amoxicillin, diclofenac (possible)

Diagnosis: definite TEN

EDITORIAL COMMENT

The authors identified a further two patients with either TEN or SJS/TEN overlap; however, drug causality could not be assigned in these cases.

CITATION DETAILS

Ziener M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology 166: 575-600, No. 3, Mar 2012.

Available from: URL: <http://dx.doi.org/10.1111/j.1365-2133.2011.10705.x> (English)

Krka's comment: S, UL, possibly related

- amoxicillin: S, UL, possibly related. Could also be explained by other drug (diclofenac).

- diclofenac: S, UL possibly related

Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

Sender's comment:

Reporter's comment:

MIP International Pharma Research GmbH, 23.05.2012 DE-MIP-hdcopo160412

Sender diagnosis: not reported

Narrative

A 45-year-old female patient with systemic lupus erythematosus (SLE) suffered from toxic epidermal necrolysis with fatal outcome after administration of products containing Diclofenac and Amoxicillin. Flat atypical targets and widespread spots were observed and palms and soles were involved as well as mucous membranes (lips, oral). Furthermore, the patient was treated with immunosuppressive therapy.

Sender's comment:

On 09-MAY-2012, the MAH received a literature report concerning an adverse drug reaction occurred in Germany. The reporting obligation results from a co-marketing authorization concerning Diclo-saar 75 SF mg / Diclo-saar 50 mg. Skin and subcutaneous tissue disorders like exanthema, erythema, purpura, as well as bullous skin reactions like Stevens-Johnson syndrome and toxic epidermal necrolysis are known for Diclofenac therapy and are labelled for Diclo-saar 75 SF mg / Diclo-saar 50 mg. The case was classified as serious since the patient had to be treated inpatient and the outcome was fatal. The causality was judged as possible for Diclofenac and the co-medication Amoxicillin. The reported case does not change the risk-benefit assessment for Diclo-saar 75 SF mg / Diclo-saar 50 mg.

Reporter's comment:

Dr. August Wolff GmbH & Co. KG Arzneimittel, 24.05.2012 DE-DRWOLFF-2012-238

Sender diagnosis: not reported

Narrative

This case was reported within the context of a retrospective study on validated cases of Stevens-Johnson syndrome/Toxic epidermal necrolysis (SJS /TEN) with a history of systemic lupus erythematosus ((S)LE), based on a large population-based national registry. A 45-year-old female patient with a history of systemic lupus erythematosus developed TEN in January 1999. She had been treated with amoxicillin and diclofenac one week before the onset of TEN. Causality was assessed as possible for both drugs. Skin examination revealed atypical targets, widespread flat spots with involvement of palms and soles as well as oral mucosa. The patient died.

Sender's comment:

Serious, expected adverse reaction. Causality assessment per company: possible. This individual case identified from literature does not indicate a change in the assessment of the safety and benefit-risk-ratio of the amoxicillin containing products Amoxi-Wolff. No further safety-relevant measure is currently deemed necessary.

Reporter's comment:

Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

MEDA Pharma GmbH & Co. KG, 01.06.2012 DE-MEDA-M-EU-2012050276

Sender diagnosis: not reported

Narrative

A literature report was received by YES Services (DE-VIGILIT-700155) - literature search project VIGILIT - on 22-May-2012. The original article was received on 25-May-2012.

Three female patients developed definite/probable SJS after use of amoxicillin, co-trimoxazole (only pat. 5), diclofenac (only pat. 10) and multiple medication use including cefazolin, metamizole, and clavulanic acid (only pat.13). The patients were part of a retrospective study in which 17 patients with lupus erythematosus (LE) developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs. The analysis was based on cases which had been ascertained and validated by a German registry.

Patient No. 10 in table 2 of the article: a 45-year-old female patient was diagnosed with SLE (systemic LE) in 1980. In January 1999 she was diagnosed with definite TEN. Signs of LE included anti-phospholipid syndrome, involvement of skin, heart, lung, and kidney. Last exacerbation was in 1995 with extensive skin and liver involvement. LE characteristic laboratory values showed: anti-phospholipid antibodies and elevated transaminases. The patient received immunosuppressive therapy prior to the event (prednisolone/prednisone, azathioprine). Time period index-day to day of the maximum blisters/erosions was 7. Skin involvement included atypical flat targets and widespread spots. Soles, palms as well as mucous membranes of lips and mouth were also involved. The patient died. The possible causative agents for TEN were amoxicillin and diclofenac.

The authors state that drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety. The authors further mention that the prevalence of LE among patients with SJS/TEN is considerably higher than in the general population. Whether or not SLE is an independent risk factor for SJS in 1994 (pat.5). Questionable SLE in 2003, and proven SLE in 2005 (pat.13). In 1980 (pat.10)/TEN would be a matter of debate. The proportional over-representation in this study might also be due to co-dependent risk factors such as use of immunosuppressive drugs, or other shared risk factors such as genetic make-up resulting in a possible shared mechanism of acute diffuse epidermal apoptosis with associated production of inflammatory cytokines.

Literature source:

Zierner M., Kardaun S.H., Liss Y., Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology. 2012; 166:575-600.

Case is linked to the first of a series of cases created by MEDA on the basis of the article, M-EU-2012050250. Cases were classified as solicited reports.

Sender's comment:

MEDA comment

Serious literature report (fatal outcome)

Toxic epidermal necrolysis (TEN) is considered listed for amoxicillin according to MEDA RSI. Patient had also been treated with diclofenac that is also known to cause SJS. There is no further information on the therapy dates regarding both drugs. Patient also had severe underlying lupus erythematosus (LE) with different organ manifestations and was thus probably pre-disposed for the development of SJS and a more severe outcome. She had been on immunosuppressive treatment. Histopathological LE features were present. With the available information it cannot be clarified whether the event was caused by amoxicillin. The causality to amoxicillin cannot be assessed.

Reporter's comment:

The authors state that drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety. The authors further mention that the prevalence of LE among patients with SJS/TEN is considerably higher than in the general population. Whether or not SLE is an independent risk factor for SJS/TEN would be a matter of debate.

The proportional over-representation in this study mi

RANBAXY, 04.06.2012 DE-RANBAXY-2012RR-56249

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2012RR-56249 "Patient 9", from GERMANY refers to a 45 year-old female who experienced toxic epidermal necrolysis which was fatal, following treatment with amoxicillin and diclofenac.

This case was reported in the literature.

Patient's medical history:

Lupus erythematosus.

The patient received the following suspect product:

Amoxicillin: dosage, route and indication not stated.

Diclofenac: dosage, route and indication not stated.

The patient developed toxic epidermal necrolysis, which was fatal, while receiving amoxicillin and diclofenac.

Verbatim text: "A retrospective study identified 15 patients with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died.

The authors identified a further two patients with either TEN or SJS/TEN overlap; however, drug causality could not be assigned in these cases."

The outcome of the event was reported as fatal at the time of this report.

The case was considered serious because the patient died.

Author Comment "Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

Follow up report (full text article) received on 21-MAY-2012 containing further information:

Reporter details.

Laboratory tests.

Current condition, systemic lupus erythematosus was started in 1980, with the previous changes: antiphospholipid syndrome, involvement of skin, heart, lung, kidney; last exacerbation 1995 with extensive skin and liver involvements.

Concomitant medication:

Prednisolone: for lupus erythematosus.

Azathioprine: for lupus erythematosus.

Patient link: 803070132_10-002 (patient 9 from summary is patient 10 from full text article).

Sender's comment:

Temporal association suggests a possible causality with each drug use in this case, but for an accurate assessment of the role on each suspect drug more information is necessary .

Toxic epidermal necrolysis is stated in Diclofenac and also in Amoxicillin SmPC.

Stevens-Johnson syndrome (SJS) and toxic epidermal necrolysis (TEN) are severe cutaneous hypersensitivity reactions. Drugs, especially sulfa drugs, antiepileptics, and antibiotics, are the most common causes. The exact mechanism is unknown; however, one theory holds that altered drug metabolism in some patients cause formation of reactive metabolites that bind to and alter cell proteins, triggering a T-cell-mediated cytotoxic reaction to drug antigens in keratinocytes.

Another possible mechanism involves interactions between Fas (a cell-surface receptor that induces apoptosis) and its ligand, particularly a soluble form of Fas ligand released from mononuclear cells. Recent findings suggest that granulysin released from cytotoxic T cells and natural killer cells might play a role in keratinocyte death.

References:

SmPC of Diclofenac and Amoxicillin.

http://www.merckmanuals.com/professional/dermatologic_disorders/hypersensitivity_and_inflammatory_disorders/stevens-johnson_syndrome_sjs_and_toxic_epidermal_necrolysis_ten.html

Reporter's comment:

The case is rated as serious, possible and expected with both suspect drugs.

Follow up 21.05.2012: Based on the new available information the assessment of the case remains unchanged.

Dolorgiet GmbH, 08.06.2012 DE-DOLORGIET-2012diclo03759

Sender diagnosis: not reported

Narrative

A retrospective study identified 15 patients [see Table 1] with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died.

Sender's comment:

The authors identified a further two patients with either TEN or SJS/TEN overlap; however, drug causality could not be assigned in these cases.

Reporter's comment:

Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

MENARINIP, 10.08.2012 DE-MEN-27449

Sender diagnosis: not reported

Narrative

This case, identified within the international literature, was presented within a publication concerning a descriptive study of 17 validated cases of SJS/TEN in patients with a history of systemic or other lupus erythematosus, from a large population-based national registry.

For this retrospective analysis, probable or definite cases of SJS/TEN (evaluation criteria of Bastuji-Garin), ascertained and validated by the German registry from 1990 to 2006 were used.

All consecutive cases validated as probable or definite SJS/TEN with an affirmative answer to SLE in their history were selected. Among a total of

2590 cases, 1366 had been validated as definite or probable SJS/TEN, 27 of which had a history of (S)LE. Because of insufficiently documented LE diagnosis, six cases were excluded. Out of the remaining 21 cases, representative histopathological slides or sufficient paraffin material were provided for 17 patients: one EMM/SJS, seven SJS, six SJS/TEN-overlap and three TEN. Clinical characteristics of these 17 cases were summarized within a table. In three of these patients metamizol treatment was identified as potential cause of SJS/TEN, and in each two cases sulfamethoxazole/trimethoprim or diclofenac was identified as culprit drug.

Patient 10 involved a 45-year-old female patient, who developed Toxic epidermal necrolysis in Jan-1999 and was hospitalised.

Diclofenac (possible) and amoxicillin (possible) were identified as causative drugs; information regarding therapy duration and daily dosages was not available.

The patient had a medical history of systemic lupus erythematosus (SLE) since 1980 with involvement of skin, heart, lung and kidney, antiphospholipid syndrome; last exacerbation of SLE was 1995 with extensive skin and liver involvement. Before development of Toxic epidermal necrolysis she had immunosuppressive therapies with prednisolone/prednisone and azathioprine. Laboratory analysis had revealed underlying elevated transaminases. The woman had no skin lesions before event.

She developed widespread Toxic epidermal necrolysis (skin involvement: atypical targets flat, spots widespread) with involvement of palms and soles, and oral mucous membranes involvement and lips. Time from index-day to the day of maximum blisters and erosions was 7 days. The patient died.

Author comment: "Stevens-Johnson syndrome (SJS)/toxic epidermal necrolysis (TEN) may show resemblance with lupus erythematosus (LE), clinically and histopathologically. The prevalence of LE among patients with SJS/TEN is considerably higher than in the general population."

This case was derived from the same publication as DE-MEN-27446 and DE-MEN-27447 (sulfamethoxazole/trimethoprim), DE-MEN-27448 (diclofenac), DE-MEN-27450, DE-MEN-27451 and DE-MEN-27452 (metamizol).

Sender's comment:

Although this case lacks crucial information regarding therapy dates, daily doses, further concomitant medication and action taken with the suspected drugs, as all cases in the study were validated and causality assessment was performed using the definition proposed by Bastuji-Garin, we consider the event as possibly related to diclofenac as well as amoxicillin. However, an assignment to a single drug is not possible. Underlying SLE could have been an influence factor. Toxic epidermal necrolysis is a known side effect of diclofenac and amoxicillin. Presently no measures are deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12061933

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 31 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermal necrosis					Unknown
2 Oral lesion					Unknown
3 Anal lesion					Unknown
4 Toxic epidermal necrolysis	200006				recovered/resolved
5 Stevens-Johnson syndrome	200006				recovered/resolved
6 Overlap syndrome					Unknown
7 Lip disorder					Unknown
8 Bullae					Unknown
9 Skin erosion	200006				Unknown
10 Spot pigmented	200006				Unknown
11 Toxic epidermal necrolysis					Unknown
12 Erythema					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AZATHIOPRINE azathioprine	Unknown	/	-	Immunosuppressant drug therapy
2 diclofenac sodium diclofenac sodium	Unknown	/	-	Drug use for unknown indication
3 prednisolone prednisolone	Unknown	/	-	Drug use for unknown indication
4 prednisone prednisone	Unknown	/	-	Drug use for unknown indication
5 ROFECOXIB rofecoxib	Unknown	/	-	Drug use for unknown indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Anaemia			Unknown	
Skin lesion NOS			Yes	over approximately 6w prior to ADR following sun exposure
Leucopenia			Unknown	
Lupus erythematosus			Yes	
Systemic lupus erythematosus	1996		Yes	in 1996
Polyserositis			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Ziemer M, Kardaun S H, Liss Y, Mockenhaupt M, Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology 2012; 166(3):575-600.	

Narratives (nur bei elektronischem Eingang)

[sanofi-aventis, 23.03.2012 DE-SANOFI-AVENTIS-2012SA019609](#)

Sender diagnosis: not reported

Narrative

This case is issued from a publication in which sixteen other cases were reported: 2012SA017469, 2012SA019605, 2012SA019606, 2012SA019607, 2012SA019608, 2012SA019610, 2012SA019611, 2012SA019612, 2012SA019613, 2012SA019614, 2012SA019615, 2012SA019616, 2012SA019617, 2012SA019618, 2012SA019619 and 2012SA019620 (cluster).

This unsolicited literature case was identified on 13-March-2012 via an article

Zierner M, Kardaun S H, Liss Y, Mockenhaupt M, Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology 2012; 166(3):575-600.

This case concerns a 31 year old female patient (patient 6) who experienced Stevens-Johnson syndrome and toxic epidermal necrosis whilst receiving prednisone, prednisolone, diclofenac sodium (diclofenac), rofecoxib and azathioprine all for an unknown indication.

On an unknown date, the patient commenced treatment with prednisone, prednisolone, diclofenac sodium, rofecoxib and azathioprine. In June 2000 (few weeks after the therapy), she experienced definite Stevens-Johnson syndrome and toxic epidermal necrolysis and was hospitalised. She had widespread spots, erosions in photodistribution and had localized bulla sole involving oral, lips and anal mucous membranes. 11 days after the index day, the patient developed maximum blisters and erosions. In the histology sample, the patient did not have hyperorthokeratosis, intrafollicular hyperkeratosis, solitary necrotic keratinocytes within lower epidermis, atrophic epidermis and melanophages. Plasma cells were not involved. She had multiple necrotic keratinocytes within entire epidermis and complete epidermal necrosis, peri-adnexal involvement of sweat glands and hair follicles, necrotic keratinocytes and lymphocytic infiltrate. Mucin was also involved. She had moderate and superficial and deep lymphocytic infiltrate. Direct immunofluorescence showed linear fibrin, granular C3 blister roof. It was reported that the patient was discharged. Outcome of the events of Stevens-Johnson syndrome and toxic epidermal necrolysis was unknown. Therapy with prednisone, prednisolone, diclofenac sodium, rofecoxib and azathioprine was completed.

The patient's medical history included systemic lupus erythematosus in 1996, with hematological and cardiac involvement. She had development of erythema face, decollete and upper arms after sun exposure, 6 weeks prior to event. Her past drugs included 12 courses of cyclophosphamide in 1997 to 1998. No concomitant drugs and concurrent conditions were reported.

The author stated that the potential cause of Stevens-Johnson syndrome and toxic epidermal necrolysis was possibly rofecoxib and diclofenac. The author further stated that other co-dependent risk factors included immunosuppressive drugs.

Sender's comment:

Sanofi Aventis company comment: This case concerns a 31 year old female patient who experienced Stevens-Johnson syndrome and toxic epidermal necrosis whilst receiving prednisone, prednisolone, diclofenac sodium (diclofenac), rofecoxib and azathioprine all for an unknown indication. Based on the drug-event temporal relationship the role of drugs cannot be denied; however, their individual role cannot be assessed in isolation for the events.

Reporter's comment:

STADA Arzneimittel AG, 09.05.2012 DE-STADA-046832

Sender diagnosis: not reported

Narrative

The following information was published in the scientific literature.

Zierner M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. Br-J-Dermatol 2012; 166(3) 575-600 -

Summary:

A 31-year-old woman with systemic lupus erythematosus was receiving rofecoxib, diclofenac, azathioprine and either prednisolone or prednisone. She developed widespread spots and erosions with photodistribution and mucous membrane involvement. Skin biopsy findings included multiple necrotic keratinocytes throughout the epidermis. She was diagnosed with definite Stevens-Johnson syndrome/toxic epidermal necrolysis overlap, with possible causes being rofecoxib and diclofenac. She was later discharged.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

This case was classified as serious.

DICLOFENAC

The causal relationship between the suspected drug and the suspected adverse reactions was assessed as possibly related based on the temporal association and the known safety profile.

The suspected adverse reactions are listed according to the current Reference Safety Information (RSI).

This case does not change the overall benefit-risk balance of the medicinal product.

Reporter's comment:

'Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.'

NOVARTIS, 09.05.2012 DE-SZ09-PHHY2012DE038749

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE038749 is an initial literature report received on 04 May 2012. The author discussed about Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus. This case referred to a 31-year-old female patient (Tab 2, No 6). This patient's medical history included systemic lupus erythematosus (1996), polyserositis, anaemia and leucopenia. She had drug history of cyclophosphamide (1997/98), prednisolone/prednisone and azathioprine as immunosuppressants. The patient was on treatment with diclofenac (manufacturer, dose route, formulation and indication were unknown) and rofecoxib from an unreported date. On an unreported date in Jun 2000, she was diagnosed with definite Stevens-Johnson syndrome (SJS)/toxic epidermal necrolysis (TEN). She was hospitalized on an unreported date. Skin involvement included widespread spots and erosions in photodistribution. Localized bulla sole, and lips, oral, anal mucous membranes were also involved. She had extensive erythema with large areas of erosion, erosive cheilitis and accentuation on sun-exposed skin. Treatment received in response to the events was unknown. On an unreported date the patient was discharged. The author stated that potential cause of SJS/TEN was possible diclofenac and rofecoxib.

Sender's comment:

Serious literature report, Toxic epidermal necrolysis, Stevens-Johnson syndrome [hospitalization], already listed in the Core Data Sheet. All literature reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

DEXPHARM, 15.05.2012 DE-DEXPHARM-20120438

Sender diagnosis: not reported

Narrative

Report received from literature.

Reference: Ziemer, M.; Kardaun, S. H.; Liss, Y.; Mockenhaupt, M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology 2012, V166, P575-600

A retrospective study identified 15 patients [see table] with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died.

Editorial Comment: The authors identified a further two patients with either TEN or SJS/TEN overlap; however, drug causality could not be assigned in these cases.

Patient 5 from the table:

A 31 year old female patient with lupus erythematosus developed Stevens-Johnson syndrome (SJS)/ toxic epidermal necrolysis (TEN) overlap after receiving diclofenac and rofecoxib [details of drug administrations not stated]. After the onset of the skin reaction, biopsy findings included necrotic keratinocytes throughout the epidermis. She was subsequently discharged. Causality was determined as possible for diclofenac and rofecoxib by the author.

Author Comment: "Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

Company assessment: This 31 years old female with lupus erythematosus was taking rofecoxib and diclofenac (durations unspecified) and developed definite SJS/TEN. There is little information but it is possible that diclofenac was causally related; it is not possible to distinguish the relative risks associated with the two products.

Sender's comment:

Reporter's comment:

RANBAXY, 16.05.2012 DE-RANBAXY-2012RR-56245

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2012RR-56245 "Patient 5", from GERMANY refers to a 31 year-old female who experienced Stevens-Johnson syndrome/ toxic epidermal necrolysis overlap which was medically significant and required patient's hospitalization, following treatment with rofecoxib and diclofenac.

This case was reported in the literature.

Patient's medical history
Lupus erythematosus.

The patient received the following suspect product:
Rofecoxib: dosage, route and indication not stated.
Diclofenac: dosage, route and indication not stated.

The patient developed overlap between Stevens-Johnson syndrome and toxic epidermal necrolysis, while receiving treatment with rofecoxib and diclofenac.

Verbatim text: "A retrospective study identified 15 patients with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died. The authors identified a further two patients with either TEN or SJS/TEN overlap; however, drug causality could not be assigned in these cases."

The outcome of the events was reported as the patient was "subsequently discharged" at the time of this report.

The case was considered serious because the ADRs were medically significant and the patient was hospitalised

Author Comment "Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

Sender's comment:

Temporal association suggests a possible causality with each drug use in this case, but for an accurate assessment of the role of each suspect drug more information is necessary.

Both Stevens-Johnson syndrome and TEN are stated in Diclofenac's SmPC.

Stevens-Johnson syndrome (SJS) and toxic epidermal necrolysis (TEN) are severe cutaneous hypersensitivity reactions. Drugs, especially sulfa drugs, antiepileptics, and antibiotics, are the most common causes. The exact mechanism is unknown; however, one theory holds that altered drug metabolism in some patients cause formation of reactive metabolites that bind to and alter cell proteins, triggering a T-cell-mediated cytotoxic reaction to drug antigens in keratinocytes.

Another possible mechanism involves interactions between Fas (a cell-surface receptor that induces apoptosis) and its ligand, particularly a soluble form of Fas ligand released from mononuclear cells. Recent findings suggest that granulysin released from cytotoxic T cells and natural killer cells might play a role in keratinocyte death.

References:

SmPC of Diclofenac

http://www.merckmanuals.com/professional/dermatologic_disorders/hypersensitivity_and_inflammatory_disorders/stevens-johnson_syndrome_sjs_and_toxic_epidermal_necrolysis_ten.html

Reporter's comment:

The case is rated as serious and possible with both suspect drugs and is expected with diclofenac use.

Kappler Pharma Consult GmbH, 16.05.2012 DE-KAPPLER-000160

Sender diagnosis: not reported

Narrative

Literature case:

A retrospective study identified 15 patients with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs.

Patient 06, a 31-year old female patient developed Stevens-Johnson syndrome (SJS) and toxic epidermal necrolysis (TEN) during treatment with diclofenac and rofecoxib in June 2000. The time from index-day to the day of maximum blisters and erosions was 11 days. She experienced an exacerbation of systemic lupus erythematosus prior to the event. Over 6 weeks prior to event development of erythema face, decollete and upper arms after sun exposure. She was also receiving immunosuppressive therapy.

The causality was assessed as possible related. The characteristics laboratory were: ANA 1:1280, anti-Ro 1412- positive, anti-La 154:6 borderline; antihistone 42:5 positive; anti-dsDNA positive; anaemia, leucopenia, C3 decreased; DIF: fibrin band-like; C3 granular at blister floor. She was discharged.

Sender's comment:

The case is assessed as serious due to the hospitalisation. Steven-Johnson syndrome and toxic epidermal necrolysis are listed in current CCSI of diclofenac. The ADRs are possible related to diclofenac medication.

Reporter's comment:

Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety

Temmler Pharma GmbH & Co. KG, 21.05.2012 DE-TEMMLER06-005080

Sender diagnosis: not reported

Narrative

This literature case was received by Temmler Pharma on 07 May 2012 from Adis Reactions via literature research:

A 31-year-old woman with systemic lupus erythematosus was receiving rofecoxib, diclofenac, azathioprine and either prednisolone or prednisone. She developed widespread spots and erosions with photodistribution and mucous membrane involvement. Skin biopsy findings included multiple necrotic

keratinocytes throughout the epidermis. She was diagnosed with definite Stevens-Johnson syndrome/toxic epidermal necrolysis overlap, with possible causes being rofecoxib and diclofenac. She was later discharged.

Temmler ID: DIC-2012-042<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Company comment Temmler Pharma:

This case is medically confirmed and the overall benefit-risk-ratio is not changed.

Reporter's comment:

Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

drreddys, 22.05.2012 DE-drreddys-GER/GER/12/0024375

Sender diagnosis: not reported

Narrative

This is a literature case report which described Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus. This retrospective study identified 17 patients with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs. Five patients later died.

This case pertains to a 31-year-old female patient who developed Stevens-Johnson syndrome/ toxic epidermal necrolysis while on therapy with diclofenac for an unknown indication.

The patient's medical history included systemic lupus erythematosus in 1996, polyserositis (hematological, cardiac involvement) and 12 cyclophosphamide courses in 1997/98. Co-suspect medication included rofecoxib. Concomitant medications included immunosuppressive therapy with azathioprine and prednisolone/prednisone.

On an unspecified date, the patient started diclofenac at an unspecified route, dose and frequency. In June 2000, the patient developed Stevens-Johnson syndrome/ toxic epidermal necrolysis (Skin involvement: 'spots widespread, erosions in photodistribution, localized bulla sole, involvement of mucus membranes of lips, oral and anal'). It was reported that, six weeks before event occurrence, development of erythema in face and upper arms after sun exposure was noted. Laboratory test revealed ANA 1: 1280, anti-Ro 1412.7 (positive), anti-La 154.6 (borderline), antihistone 43.5 (positive), anti ds-DNA (positive), anaemia, leucopenia, C3 decreased, DIF:fibrin band-like and C3 granular at blister floor. At the time of reporting, the patient was discharged and the outcome of the event was unknown.

The reporter assessed events as possibly related to diclofenac and rofecoxib therapy.

The reporter assessed the case as serious (Hospitalization).

The reporter commented that 'Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety'.

Literature reference:

Zimer M., Kardaun S.H., Liss Y., Mocjenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology. 2012. 166; 575-600.

Also, please refer GER/GER/12/0024372, GER/GER/12/0024373, GER/GER/12/0024374, GER/GER/12/0024376, GER/GER/12/00024377, GER/GER/12/00024378, GER/GER/12/00024379, GER/GER/12/0024380 and GER/GER/12/0024381 for other patients presented in this article.

Sender's comment:

This patient with systemic lupus erythematosus experienced Stevens-Johnson syndrome (SJS) and toxic epidermal necrolysis (TEN) while on diclofenac and rofecoxib therapy. Although the co-suspect medication rofecoxib and the concomitant medications could have contributed to the reported events, a role of diclofenac could not be excluded.

This case was considered serious (Hospitalization).

SJS and TEN were expected per diclofenac reference label for Germany.

Reporter's comment:

Dr. R. Pflieger Chemische Fabrik GmbH, 23.05.2012 DE-PFLEGER-001783

Sender diagnosis: not reported

Narrative

Literature report:

A retrospective study identified 15 patients [see Table 1] with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died.

Table 1:

Pat. 5, Sex: Female, Age: 31 Years, Suspect drugs: Rofecoxib, Diclofenac, causality: possible, Diagnosis: definite SJS, TEN<dbtext>

To_Char_NARRATIVE_2</dbtext>

Sender's comment:

Changes of the Reference Safety Information are not indicated.

Reporter's comment:

Drug causality was evaluated based on an algorithm including relevant exposure time before the Index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

MIP International Pharma Research GmbH, 23.05.2012 DE-MIP-hdcopo160312

Sender diagnosis: not reported

Narrative

A 31-year-old female patient with systemic lupus erythematosus (SLE) suffered from Stevens-Johnson syndrome and TEN after administration of a product containing Diclofenac and Rofecoxib. Widespread spots and erosions in photodistribution were observed as well as localized bulla sole and an involvement of mucous membranes (lips, oral, anal). Furthermore, the patient was treated with immunosuppressive therapy.

Sender's comment:

On 09-MAY-2012, the MAH received a literature report concerning an adverse drug reaction occurred in Germany. The reporting obligation results from a co-marketing authorization concerning Diclo-saar 75 SF mg / Diclo-saar 50 mg. Skin and subcutaneous tissue disorders like exanthema, erythema, purpura, as well as bullous skin reactions like Stevens-Johnson syndrome and toxic epidermal necrolysis are known for Diclofenac therapy and are labelled for Diclo-saar 75 SF mg / Diclo-saar 50 mg. The case was classified as serious since the patient had to be treated inpatient. The causality was judged as possible for Diclofenac and Rofecoxib. The reported case does not change the risk-benefit assessment for Diclo-saar 75 SF mg / Diclo-saar 50 mg.

Reporter's comment:

KRKAPR, 23.05.2012 DE-KRKA-DE2012K1304LIT

Sender diagnosis: not reported

Narrative

Various drugs

Stevens-Johnson syndrome and toxic epidermal necrolysis: 15 case reports

A retrospective study identified 15 patients [see table] with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died.

Author Comment "Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

Patient: 5

sex: F

age (y): 31

Suspect drugs (causality): rofecoxib, diclofenac (possible)

Diagnosis: definite SJS/TEN

EDITORIAL COMMENT

The authors identified a further two patients with either TEN or SJS/TEN overlap; however, drug causality could not be assigned in these cases.

CITATION DETAILS

Ziener M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology 166: 575-600, No. 3, Mar 2012.

Available from: URL: <http://dx.doi.org/10.1111/j.1365-2133.2011.10705.x> (English)

Krka's comment: S, UL, possibly related

Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

Sender's comment:

Reporter's comment:

MEDA Pharma GmbH & Co. KG, 01.06.2012 DE-MEDA-M-EU-2012050255

Sender diagnosis: not reported

Narrative

A literature report was received by YES Services (DE-VIGILIT-700160) - literature search project VIGILIT - on 22-May-2012. The original article was received on 25-May-2012.

Two female patients developed definite TEN and definite SJS/TEN after use of diclofenac, rofecoxib (only pat. 6), and amoxicillin (only pat. 10). The patients were part of a retrospective study in which 17 patients with lupus erythematosus (LE) developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs. The analysis was based on cases which had been ascertained and validated by a German registry.

Patient No. 6 in table 2 of the article: a 31-year-old female patient was diagnosed with SLE (systemic LE) in 1996. In June 2000, she was diagnosed with definite SJS/TEN. Clinical signs of LE included polyserositis, haematological and cardiac involvement. She received twelve cyclophosphamide

courses 1997/98. LE characteristic laboratory values: in the year 2000 ANA (antinuclear antibodies) 1: 1280, anti-Ro 1412.7 positive, anti-La 154.6 borderline, antihistone 42.5 positive. Anti-dsDNA positive. Anaemia, leucopenia, C3 decreased. DIF (direct immunofluorescence): fibrin band-like, C3 granular at blister floor. The patient received immunosuppressive therapy prior to the event (prednisolone/prednisone, azathioprine). Skin lesions before SJS/TEN included development of erythema in the face, décolleté and upper arms after sun exposure over approx. six weeks prior to the event. Time period index-day to day of the maximum blisters/erosions was 11. Skin involvement included widespread spots and erosions in photodistribution. LE typical photodistribution was located in face, décolleté, upper back, and extensor site of arms. Localised bullae at the sole were noted. as Additionally, oral and anal mucous membranes as well as the lips were involved. The patient was subsequently discharged. Possible causes of SJS/TEN were diclofenac and rofecoxib.

The authors state that drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety. The authors further mention that the prevalence of LE among patients with SJS/TEN is considerably higher than in the general population. Whether or not SLE is an independent risk factor for SJS/TEN would be a matter of debate.

The proportional over-representation in this study might also be due to co-dependent risk factors such as use of immunosuppressive drugs, or other shared risk factors such as genetic make-up resulting in a possible shared mechanism of acute diffuse epidermal apoptosis with associated production of inflammatory cytokines.

Literature source:

Ziener M., Kardaun S.H., Liss Y., Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology. 2012; 166:575-600.

Case is linked to the first of a series of cases created by MEDA on the basis of the article, M-EU-2012050250. Cases were classified as solicited reports.

Sender's comment:

MEDA comment

Serious literature report (hospitalisation)

Toxic epidermal necrolysis (TEN)/ Stevens-Johnson syndrome (SJS) is considered listed for diclofenac according to MEDA RSI. Patient had also been treated with rofecoxib that is also known to cause SJS. There is no further information on the therapy dates regarding both drugs. Patient also had underlying lupus erythematosus (LE) and was on immunosuppressive treatment, so that she was probably pre-disposed for the development of SJS. The patient had both clinical and histopathological LE features. She had recently experienced an acute LE exacerbation and was finally diagnosed with SJS/TEN like SLE. It is therefore questionable whether the event was drug induced at all. The causality to diclofenac cannot be assessed.

Reporter's comment:

The authors state that drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety. The authors further mention that the prevalence of LE among patients with SJS/TEN is considerably higher than in the general population. Whether or not SLE is an independent risk factor for SJS/TEN would be a matter of debate.

The proportional over-representation in this study mi

RANBAXY, 04.06.2012 DE-RANBAXY-2012RR-56245

Sender diagnosis: not reported

Narrative

This case, manufacturer control number 2012RR-56245 "Patient 5", from GERMANY refers to a 31 year-old female who experienced Stevens-Johnson syndrome/ toxic epidermal necrolysis overlap which was medically significant and required patient's hospitalization, following treatment with rofecoxib and diclofenac.

This case was reported in the literature.

Patient's medical history

Lupus erythematosus.

The patient received the following suspect product:

Rofecoxib: dosage, route and indication not stated.

Diclofenac: dosage, route and indication not stated.

The patient developed overlap between Stevens-Johnson syndrome and toxic epidermal necrolysis, while receiving treatment with rofecoxib and

diclofenac.

Verbatim text: "A retrospective study identified 15 patients with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died.

The authors identified a further two patients with either TEN or SJS/TEN overlap; however, drug causality could not be assigned in these cases."

The outcome of the events was reported as the patient was "subsequently discharged" at the time of this report.

The case was considered serious because the ADRs were medically significant and the patient was hospitalised

Author Comment "Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

Follow up report (full text article) received on 21-MAY-2012 containing further information:

Reporter details.

Lab tests.

Lupus erythematosus previous changes: polyserositis, haematological, cardiac involvement; 12 cyclophosphamide courses 1997/1998.

Concomitant medication:

Prednisolone: for lupus erythematosus.

Azathioprine: for lupus erythematosus.

The outcome of the events was updated to improved.

Patient link: 803070132_06-002 (patient 5 from summary is patient 6 in full text).

Sender's comment:

Temporal association suggests a possible causality with each drug use in this case, but for an accurate assessment of the role of each suspect drug more information is necessary.

Both Stevens-Johnson syndrome and TEN are stated in Diclofenac's SmPC.

Stevens-Johnson syndrome (SJS) and toxic epidermal necrolysis (TEN) are severe cutaneous hypersensitivity reactions. Drugs, especially sulfa drugs, antiepileptics, and antibiotics, are the most common causes. The exact mechanism is unknown; however, one theory holds that altered drug metabolism in some patients cause formation of reactive metabolites that bind to and alter cell proteins, triggering a T-cell-mediated cytotoxic reaction to drug antigens in keratinocytes.

Another possible mechanism involves interactions between Fas (a cell-surface receptor that induces apoptosis) and its ligand, particularly a soluble form of Fas ligand released from mononuclear cells. Recent findings suggest that granulysin released from cytotoxic T cells and natural killer cells might play a role in keratinocyte death.

References:

SmPC of Diclofenac

http://www.merckmanuals.com/professional/dermatologic_disorders/hypersensitivity_and_inflammatory_disorders/stevens-johnson_syndrome_sjs_and_toxic_epidermal_necrolysis_ten.html

Reporter's comment:

The case is rated as serious and possible with both suspect drugs and is expected with diclofenac use.

Follow up 21.05.2012: Based on the new available information the assessment of the case remains unchanged.

Dolorgiet GmbH, 08.06.2012 DE-DOLORGIET-2012diclo03758

Sender diagnosis: not reported

Narrative

A retrospective study identified 15 patients [see Table 1] with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were

subsequently discharged, while the remaining five patients died.

Sender's comment:

The authors identified a further two patients with either TEN or SJS/TEN overlap; however, drug causality could not be assigned in these cases.

Reporter's comment:

Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

MENARINIP, 10.08.2012 DE-MEN-27448

Sender diagnosis: not reported

Narrative

This case, identified within the international literature, was presented within a publication concerning a descriptive study of 17 validated cases of SJS/TEN in patients with a history of systemic or other lupus erythematosus, from a large population-based national registry.

For this retrospective analysis, probable or definite cases of SJS/TEN (evaluation criteria of Bastuji-Garin), ascertained and validated by the German registry from 1990 to 2006 were used.

All consecutive cases validated as probable or definite SJS/TEN with an affirmative answer to SLE in their history were selected. Among a total of 2590 cases, 1366 had been validated as definite or probable SJS/TEN, 27 of which had a history of (S)LE. Because of insufficiently documented LE diagnosis, six cases were excluded. Out of the remaining 21 cases, representative histopathological slides or sufficient paraffin material were provided for 17 patients: one EMM/SJS, seven SJS, six SJS/TEN-overlap and three TEN. Clinical characteristics of these 17 cases were summarized within a table. In three of these patients metamizol treatment was identified as potential cause of SJS/TEN, and in each two cases sulfamethoxazole/trimethoprim or diclofenac was identified as culprit drug.

Patient 6 involved a 31-year-old female patient, who developed Toxic epidermal necrolysis in Jun-2000 and was hospitalised. Diclofenac (possible) and rofecoxib (possible) were identified as causative drugs; information regarding therapy duration and daily dosages was not available.

The patient had a medical history of systemic lupus erythematosus (SLE) since 1996 with polyserositis and haematological and cardiac involvement; she had 12 cyclophosphamide courses between 1997 and 1998 and immunosuppressive therapies with prednisolone/prednisone and azathioprine. laboratory analysis had revealed underlying anaemia and leucopenia.

Over six weeks before event she had developed erythema on face, décolleté and upper arms before sun exposure.

The woman developed widespread Toxic epidermal necrolysis (skin involvement: spots widespread, erosions in photodistribution) with localized bulla sole, involvement of oral and anal mucous membranes, and lips. Time from index-day to the day of maximum blisters and erosions was 11 days. The patient was discharged.

Author comment: "Stevens-Johnson syndrome (SJS)/toxic epidermal necrolysis (TEN) may show resemblance with lupus erythematosus (LE), clinically and histopathologically. The prevalence of LE among patients with SJS/TEN is considerably higher than in the general population."

This case was derived from the same publication as DE-MEN-27446 and DE-MEN-27447 (sulfamethoxazole/trimethoprim), DE-MEN-27449 (diclofenac), DE-MEN-27450, DE-MEN-27451 and DE-MEN-27452 (metamizol).

Sender's comment:

Although this case lacks crucial information regarding therapy dates, daily doses, further concomitant medication and action taken with the suspected drugs, as all cases in the study were validated and causality assessment was performed using the definition proposed by Bastuji-Garin, we consider the event as possibly related to diclofenac as well as rofecoxib. However, an assignment to a single drug is not possible. Underlying SLE could have been an influence factor. Toxic epidermal necrolysis is a known side effect of diclofenac but not of rofecoxib. Presently no measures are deemed necessary.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12081926

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Male Birth: privacy Age: 72 Year weight (kg): 96 BMI_int: 30.99 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pruritus	201108		3 Week		recovered/resolved
2 Dermatitis exfoliative	201108		3 Week		recovered/resolved
3 Dandruff	201108		3 Week		recovered/resolved
4 Application site discomfort	201108		3 Week		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 solaraze 3% Gel diclofenac sodium	Topical	2* /Day	201107-201108 32 Day	Solar keratosis

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension			Unknown	
Type 2 diabetes mellitus			Unknown	

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12085737

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 78 Year	weight (kg): 90	BMI_int: 26.58	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Open wound	201202				recovered/resolved
2 Incorrect drug administration duration	2008				Unknown
3 Crust	201202				recovered/resolved
4 Blister	201202				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/ 95	2008-20120416	Back pain

Drugs (concomitant)

2 DRUG THERAPY NOS	/	-	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bedridden			Unknown	
Osteoporosis			Unknown	Osteoporosis with status after rip fractures causing pain.

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		
Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 26.04.2012 DE-002147023-CHPA2012DE008760

Sender diagnosis: not reported

Narrative

Initial pharmacist report received on 16 Apr 2012. An over 60 year old, bedridden male patient was treated with Voltaren Schmerzgel (diclofenac diethylammonium salt) at his back due to pain for at least several months, from the end of 2011 until 16 Apr 2012. According to his wife, the patient thereby developed an open wound at his back (exact date of reaction onset unknown, presumably several weeks ago). Voltaren Schmerzgel was stopped on 16 Apr 2012. No further information was available at date of this report. Outcome: reaction continues. Follow-up information was requested in written form (including consent) in week 16, 2012.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant) confirmed by a health care professional. The adverse event(s), Open wound, was assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 08.05.2012 DE-002147023-CHPA2012DE008760

Sender diagnosis: not reported

Narrative

Initial pharmacist report received on 16 Apr 2012. An over 60 year old, bedridden male patient was treated with Voltaren Schmerzgel (diclofenac diethylammonium salt) at his back due to pain for at least several months, from the end of 2011 until 16 Apr 2012. According to his wife, the patient thereby developed an open wound at his back (exact date of reaction onset unknown, presumably several weeks ago). Voltaren Schmerzgel was stopped on 16 Apr 2012. No further information was available at date of this report. Outcome: reaction continues. Follow-up information was requested in written form (including consent) in week 16, 2012.

Additional information received on 30 Apr 2012. A pharmacist reported this 82 year old bed ridden male patient developed 'approx 2-Euro coin large skin spots, which were previously blister-like skin areas and which were currently in process of forming crusts below costal arch at the back' after topical application of Voltaren Schmerzgel (diclofenac diethylammonium) used to treat 'pain at back'. Therapy dates were specified as start date of therapy 2008. The reporter specified the onset date as 'Feb 2012'. The stated daily dose was '2-3x topically applied per day'. Voltaren Schmerzgel was stopped. The reporter ticked the box for 'dose reduction' of Voltaren Schmerzgel (no more information provided). Following drug treatment with 'aloe vera spray' adverse symptoms lasted all together for 8-10 weeks. The reporter ticked the box that the reaction was not treated by a physician. Causality rating of reporter was possible. The consent form was returned without signing the permission to contact the treating physicians and with handwritten note 'no medical treatment of the adverse events conducted'. The reporter ticked the box 'Outcome: improvement'. Due to the stated limited duration of the reactions the outcome was internally rated as 'recovery'.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant) confirmed by a health care professional. The adverse event(s), Open wound, Blister, and Scab were assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12087304

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 78 Year	weight (kg): 90	BMI_int: 26.58	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Open wound					Unknown
2 Blister	201202				recovered/resolved
3 Crust	201202				recovered/resolved
4 Open wound	201202				recovered/resolved
5 Incorrect drug administration duration	2008				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/ 95	2011-20120416 -	Back pain

Drugs (concomitant)

2 DRUG THERAPY NOS	/	-	-
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Bedridden			Unknown	
Osteoporosis			k.A.	Osteoporosis with status after rip fractures causing pain.

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 26.04.2012 DE-NOVNCHHQ-CHPA2012DE008760

Sender diagnosis: not reported

Narrative

Initial pharmacist report received on 16 Apr 2012. An over 60 year old, bedridden male patient was treated with Voltaren Schmerzgel (diclofenac diethylammonium salt) at his back due to pain for at least several months, from the end of 2011 until 16 Apr 2012. According to his wife, the patient thereby developed an open wound at his back (exact date of reaction onset unknown, presumably several weeks ago). Voltaren Schmerzgel was stopped on 16 Apr 2012. No further information was available at date of this report. Outcome: reaction continues. Follow-up information was requested in written form (including consent) in week 16, 2012.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant) confirmed by a health care professional. The adverse event(s), Open wound, was assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 07.05.2012 DE-NOVNCHHQ-CHPA2012DE008760

Sender diagnosis: not reported

Narrative

Initial pharmacist report received on 16 Apr 2012. An over 60 year old, bedridden male patient was treated with Voltaren Schmerzgel (diclofenac diethylammonium salt) at his back due to pain for at least several months, from the end of 2011 until 16 Apr 2012. According to his wife, the patient thereby developed an open wound at his back (exact date of reaction onset unknown, presumably several weeks ago). Voltaren Schmerzgel was stopped on 16 Apr 2012. No further information was available at date of this report. Outcome: reaction continues. Follow-up information was requested in written form (including consent) in week 16, 2012.

Additional information received on 30 Apr 2012. A pharmacist reported this 82 year old bed ridden male patient developed 'approx 2-Euro coin large skin spots, which were previously blister-like skin areas and which were currently in process of forming crusts below costal arch at the back' after topical application of Voltaren Schmerzgel (diclofenac diethylammonium) used to treat 'pain at back'. Therapy dates were specified as start date of therapy 2008. The reporter specified the onset date as 'Feb 2012'. The stated daily dose was '2-3x topically applied per day'. Voltaren Schmerzgel was stopped. The reporter ticked the box for 'dose reduction' of Voltaren Schmerzgel (no more information provided). Following drug treatment with 'aloe vera spray' adverse symptoms lasted all together for 8-10 weeks. The reporter ticked the box that the reaction was not treated by a physician. Causality rating of reporter was possible. The consent form was returned without signing the permission to contact the treating physicians and with handwritten note 'no medical treatment of the adverse events conducted'. The reporter ticked the box 'Outcome: improvement'. Due to the stated limited duration of the reactions the outcome was internally rated as 'recovery'.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (medically significant) confirmed by a health care professional. The adverse event(s), Open wound, Blister, and Scab were assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12097414

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 31 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stevens-Johnson syndrome	200006				recovering/resolving
2 Toxic epidermal necrolysis	200006				recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac	Unknown	/	-	
diclofenac			-	
2 ROFECOXIB	Unknown	/	-	
rofecoxib			-	

Drugs (concomitant)

3 AZATHIOPRINE	Unknown	/	-	Immunosuppressant drug
azathioprine			-	therapy

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Systemic lupus erythematosus	1996		Yes	
Skin lesion NOS			Yes	over approximately 6w prior to ADR following sun exposure
Immunosuppressant drug therapy			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Ziemer M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. Br-J-Dermatol 2012; 166(3) 575-600	

Narratives (nur bei elektronischem Eingang)

Generics [UK] Ltd, 10.05.2012 DE-MYLANLABS-2012S1009200

Sender diagnosis: not reported

Narrative

Ziemer M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. Br-J-Dermatol 2012; 166(3) 575-600

A 31-year-old woman with systemic lupus erythematosus was receiving rofecoxib, diclofenac, azathioprine and either prednisolone or prednisone. She developed widespread spots and erosions with photodistribution and mucous membrane involvement. Skin biopsy findings included multiple necrotic keratinocytes throughout the epidermis. She was diagnosed with definite Stevens-Johnson syndrome/toxic epidermal necrolysis overlap, with possible causes being rofecoxib and diclofenac. She was later discharged.

Sender's comment:

Stevens-Johnson-syndrome and toxic epidermal necrolysis are listed undesirable effects of diclofenac. Causality is assessed as possible. Co-suspect medication included rofecoxib.

Reporter's comment:

"Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

TEVA, 18.05.2012 DE-TEVA-337298GER

Sender diagnosis: not reported

Narrative

5 of 15

07-May-2012, Serious Literature Report received from Reaction (803070132). (WOLTERS KLUWER HEALTH; NZ-ADIS-3070132-25371)

Case of a Female patient 31 Years of age who experienced STEVENS-JOHNSON SYNDROME, TOXIC EPIDERMAL NECROLYSIS OVERLAP while on Diclofenac(DICLOFENAC) (Teva's product cannot be excluded); Rofecoxib(ROFECOXIB) (Non-Teva product).

A 31-year-old woman with systemic lupus erythematosus was receiving rofecoxib, diclofenac, azathioprine and either prednisolone or prednisone. She developed widespread spots and erosions with photodistribution and mucous membrane involvement. Skin biopsy findings included multiple necrotic keratinocytes throughout the epidermis. She was diagnosed with definite Stevens-Johnson syndrome/toxic epidermal necrolysis overlap, with possible causes being rofecoxib and diclofenac. She was later discharged.

Author comment: Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

At the time of the report the outcome of the AEs were: STEVENS-JOHNSON SYNDROME : recovering/resolving, TOXIC EPIDERMAL NECROLYSIS OVERLAP : recovering/resolving.

Concomitant Medication:

Azathioprine(AZATHIOPRINE)

Medical History:

SYSTEMIC LUPUS ERYTHEMATOSUS(Concomitant) ,SKIN LESION NOS(Concomitant) ,IMMUNOSUPPRESSANT DRUG THERAPY(Concomitant)

Action Taken with suspect Drugs: Rofecoxib - Unknown ,Diclofenac - Unknown

Literature Reference:

Ziener M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. Br-J-Dermatol 2012; 166(3) 575-600

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: Due to data available and mentioned results from causality evaluation contribution of Diclofenac cannot be excluded: possibly related.

Sender's comment:

Serious, spontaneous / literature (TEVA product could not be excluded) case report from Germany. Duty for notification according to German Drug Law. Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: Diclofenac: expected. The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

"Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12098703

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 45 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	199901				not recovered/not resolved
2 Unknown cause of death					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMOXICILLIN	Unknown	/	-	
amoxicillin			-	
2 diclofenac	Unknown	/	-	
diclofenac			-	

Drugs (concomitant)

3 AZATHIOPRINE	Unknown	/	-	Immunosuppressant drug
azathioprine			-	therapy

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Systemic lupus erythematosus	1980		Yes	exacerbation in 1995, skin/liver involvement
Immunosuppressant drug therapy			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Ziemer M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. Br-J-Dermatol 2012; 166(3) 575-600	

Narratives (nur bei elektronischem Eingang)

Generics [UK] Ltd, 11.05.2012 DE-MYLANLABS-2012S1009206

Sender diagnosis: not reported

Narrative

Ziemer M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. Br-J-Dermatol 2012; 166(3) 575-600

A 45-year-old woman with systemic lupus erythematosus was receiving amoxicillin, diclofenac, azathioprine and either prednisolone or prednisone. She developed widespread atypical skin target lesions, with involvement of the lip and oral mucous membranes. Skin biopsy findings included multiple necrotic keratinocytes throughout the epidermis. She was diagnosed with definite toxic epidermal necrolysis, with possible causes being amoxicillin and diclofenac. She later died.

Sender's comment:

Toxic epidermal necrolysis is a listed undesirable effect of amoxicillin and diclofenac, fatal outcomes are not described. Causality is assessed as possible.

Reporter's comment:

"Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

TEVA, 18.05.2012 DE-TEVA-337308GER

Sender diagnosis: not reported

Narrative

9 of 15

07-May-2012, Serious Literature Report received from Reaction (803070132). (WOLTERS KLUWER HEALTH; NZ-ADIS-3070132-25375)

Case of a Female patient 45 Years of age who experienced UNKNOWN CAUSE OF DEATH, TOXIC EPIDERMAL NECROLYSIS while on Amoxicillin (AMOXICILLIN) (Teva's products cannot be excluded); DICLOFENAC (Teva's products cannot be excluded).

A 45-year-old woman with systemic lupus erythematosus was receiving amoxicillin, diclofenac, azathioprine and either prednisolone or prednisone. She developed widespread atypical skin target lesions, with involvement of the lip and oral mucous membranes. Skin biopsy findings included multiple necrotic keratinocytes throughout the epidermis. She was diagnosed with definite toxic epidermal necrolysis, with possible causes being amoxicillin and diclofenac. She later died.

Author comment: Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

At the time of the report the outcome of the AEs were: Toxic epidermal necrolysis : not recovered/not resolved.

Concomitant Medication:

Azathioprine(AZATHIOPRINE)

Medical History:

SYSTEMIC LUPUS ERYTHEMATOSUS(Concomitant) ,IMMUNOSUPPRESSANT DRUG THERAPY(Concomitant)

Action Taken with suspect Drugs: Amoxicillin - Unknown ,DICLOFENAC - Unknown

Literature Reference:

Ziener M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. Br-J-Dermatol 2012; 166(3) 575-600

This case was considered serious based on the following criteria: (Death, Other Serious (Important Medical Events))

Teva Comment: Due to data available and results from data evaluation mentioned above contribution of Amoxicillin and Diclofenac cannot be excluded: possibly related.

Unknown Cause of Death is not assessable in relation to Amoxicillin and diclofenac: no clinical and temporal data are available, no information whether there was any association suspected with suspected drugs or ADR.

Sender's comment:

Serious, spontaneous / literature (TEVA product could not be excluded) case report from Germany. Duty for notification according to German Drug Law. Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: amoxicillin: unexpected; Diclofenac: partially expected. The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

"Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety."

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12111658

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	Yes	No	No	No		

Patient

Initials:	Sex:	Birth: privacy	Age: 49 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis					fatal
2 Stevens-Johnson syndrome					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Allopurinol		/	-	Drug use for unknown indication
allopurinol			-	indication
2 Amitriptyline		/	-	Drug use for unknown indication
amitriptyline			-	indication
3 Amoxicillin		/	-	Drug use for unknown indication
amoxicillin			-	indication
4 Amoxicillin/Clavulanic acid		/	-	Drug use for unknown indication
amoxicillin; clavulanic acid			-	indication
5 Cefazolin		/	-	Drug use for unknown indication
cefazolin			-	indication
6 ciprofloxacin		/	-	Drug use for unknown indication
ciprofloxacin			-	indication
7 Cotrimoxazole		/	-	Drug use for unknown indication
sulfamethoxazole; trimethoprim			-	indication
8 diclofenac		/	-	Drug use for unknown indication
diclofenac			-	indication
9 Doxycycline		/	-	Drug use for unknown indication
doxycycline			-	indication
10 Etoricoxib		/	-	Drug use for unknown indication
etoricoxib			-	indication
11 hydroxychloroquine		/	-	Drug use for unknown indication
hydroxychloroquine			-	indication
12 Imipenem		/	-	Drug use for unknown indication
imipenem			-	indication
13 Metamizole		/	-	Drug use for unknown indication
metamizole			-	indication
14 Phenoxymethylpenicillin		/	-	Drug use for unknown indication
phenoxymethylpenicillin			-	indication
15 Piperacillin/Tazobactam		/	-	Drug use for unknown indication
piperacillin; tazobactam			-	indication
16 Rofecoxib		/	-	Drug use for unknown indication
rofecoxib			-	indication
17 Sertraline		/	-	Drug use for unknown indication
sertraline			-	indication
18 Tramadol		/	-	Drug use for unknown indication
tramadol			-	indication
19 Valaciclovir		/	-	Drug use for unknown indication
valaciclovir			-	indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Lupus erythematosus			Unknown	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Ziemer M, Kardaun SH, Liss Y, Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: a descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology 2012;166:575-600.	

Narratives (nur bei elektronischem Eingang)

Heumann Pharma GmbH & Co. Generika KG, 24.05.2012 DE-HEUMANN1-001980

Sender diagnosis: not reported

Narrative

This case has been identified in the literature.

A retrospective study identified 15 patients with lupus erythematosus who developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs [details of drug administrations not stated]. Five patients later died.

The average age was 49.2 years (range 22-80 years); two patients were male (12%) and 15 female (88%). Mortality rate was 35% (six of 17). Drugs potentially causal for SJS/TEN could be identified in 15 of 17 patients (88%), one drug in five cases, two in eight and three in two cases. Eleven patients were also receiving immunosuppressive therapy that included prednisolone or prednisone, azathioprine, cyclophosphamide, leflunomide and chloroquine. After the onset of the skin reaction, biopsy findings in all patients included necrotic keratinocytes throughout the epidermis. Seven patients also had histological characteristic features of concurrent lupus erythematosus. Patients 1, 2, 4-8 and 12-14 were subsequently discharged, while the remaining five patients died.<dbtext>To_Char_NARRATIVE_2</dbtext>

Sender's comment:

These cases were classified serious (hospitalisation/death) and were identified in the literature as part of a prospective study. Regarding the information given on the suspected drugs and since the cases described in this study occurred in Germany, it cannot be excluded that the respective products of Heumann Pharma GmbH & Co. Generika KG were involved. TEN is listed for all suspected products of Heumann Pharma GmbH & Co. Generika KG. SJS is listed for Allopurinol Heumann, Sertralin Heumann, Diclofenac Heumann and Ciprofloxacin Heumann, whereas it is unlisted for Amoxicillin Heumann, Doxycyclin Heumann (although severe and life-threatening skin reactions are mentioned) and Tramadol Heumann. However, in most of the cases more than one potential cause for SJS or TEN, respectively, was identified. Only in two cases allopurinol was the only potential cause. All patients had a history of systemic or other lupus erythematosus. Regarding amoxicillin, no further case report of SJS were identified in the company's database. In the scientific literature, several publications regarding amoxicillin were identified, however, mainly dealing with a combination of amoxicillin with clavulanic acid for which SJS is a known side effect. Regarding tramadol, one further case report of SJS was identified in the company's database and one further study was identified in the scientific literature. However, regarding the identified case report, the patient was concomitantly treated with 19 further suspect drugs and in the identified study the authors mentioned that although Tramadol was associated with cases of SJS, a high percentage of co-medication with highly suspected drugs was identified. Therefore, the causality is assessed as possible for all suspected products of Heumann Pharma GmbH & Co. Generika KG. These cases do not affect the positive benefit/risk ratios of Allopurinol Heumann, Sertralin Heumann, Amoxicillin Heumann, Doxycyclin Heumann, Diclofenac Heumann, Tramadol

Reporter's comment:

Drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety.

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12116455

Serious Yes	Death Yes	Lifethr.	Hospital. Yes	Disabling	Congenital	Other	Pregnancy	Med. conf. Yes
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Patient

Initials:	Sex: Female	Birth: privacy	Age: 45 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Toxic epidermal necrolysis	199901				fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 AMOXICILLIN		/	-	
amoxicillin			-	
2 diclofenac		/	-	
diclofenac			-	

Drugs (concomitant)

3 AZATHIOPRINE	/	-	Immunosuppression
azathioprine		-	
4 prednisolone	/	-	Immunosuppression
prednisolone		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Systemic lupus erythematosus	1980		Yes	in 1980

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:	Ziemer M,et al. . Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature.. British Journal of Dermatology 2012; 166: 575-600	

Narratives (nur bei elektronischem Eingang)

MEDA Pharma GmbH & Co. KG, 01.06.2012 DE-MEDA-M-EU-2012050272

Sender diagnosis: not reported

Narrative

A literature report was received by YES Services (DE-VIGILIT-700160) - literature search project VIGILIT - 22-May-2012. The original article was received on 25-May-2012.

Two female patients developed definite TEN and definite SJS/TEN after use of diclofenac, rofecoxib (only pat. 6), and amoxicillin (only pat. 10). The patients were part of a retrospective study in which 17 patients with lupus erythematosus (LE) developed either Stevens-Johnson syndrome (SJS), toxic epidermal necrolysis (TEN) or SJS/TEN overlap after receiving various drugs. The analysis was based on cases which had been ascertained and validated by a German registry.

Patient No. 10 in table 2 of the article: a 45-year-old female patient was diagnosed with SLE (systemic LE) in 1980. In January 1999 she was diagnosed with definite TEN. Clinical signs of LE included anti-phospholipid syndrome, involvement of skin, heart, lung, and kidney. Last exacerbation was in 1995 with extensive skin and liver involvement. LE characteristic laboratory values: anti-phospholipid antibodies and elevated transaminases. The patient received immunosuppressive therapy prior to the event (prednisolone/prednisone, azathioprine). Time period index-day to day of the maximum blisters/erosions was 7. Skin involvement showed atypical flat targets and widespread spots. Soles, palms, lips and oral mucous membranes were involved. The patient died. Possible causes of TEN were amoxicillin and diclofenac. The authors state that drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety. The authors further mention that whether or not SLE is an independent risk factor for SJS/TEN is a matter of debate. The proportional over-representation in this study might also be due to co-dependent risk factors such as use of immunosuppressive drugs, or other shared risk factors such as genetic make-up resulting in a possible shared mechanism of acute diffuse epidermal apoptosis with associated production of inflammatory cytokines.

Literature source:

Ziemer M., Kardaun S.H., Liss Y., Mockenhaupt M. Stevens-Johnson syndrome and toxic epidermal necrolysis in patients with lupus erythematosus: A descriptive study of 17 cases from a national registry and review of the literature. British Journal of Dermatology. 2012; 166:575-600.

Case is linked to the first of a series of cases created by MEDA on the basis of the article, M-EU-2012050250. Cases were classified as solicited reports.

Sender's comment:

MEDA comment

Serious literature report (fatal outcome)

Toxic epidermal necrolysis (TEN) is considered listed for diclofenac according to MEDA RSI. Patient had also been treated with amoxicillin that is also known to cause SJS. There is no further information on the therapy dates regarding both drugs. Patient also had severe underlying lupus erythematosus (LE) with different organ manifestations and was thus probably pre-disposed for the development of SJS and a more severe outcome. She had been on immunosuppressive treatment. Histopathological LE features were present. With the available information it cannot be clarified whether the event was caused by diclofenac. The causality to diclofenac cannot be assessed.

Reporter's comment:

The authors state that drug causality was evaluated based on an algorithm including relevant exposure time before the index-day, potential earlier reactions to the drug, potential alternative causes and drug notoriety. The authors further mention that the prevalence of LE among patients with SJS/TEN is considerably higher than in the general population. Whether or not SLE is an independent risk factor for SJS/TEN would be a matter of debate.

The proportional over-representation in this study mi

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12123320

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 11 Year	weight (kg): 30	BMI_int: 15.31	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Allergic skin reaction					not recovered/not resolved
2 Urticaria generalised					not recovered/not resolved
3 Swelling					not recovered/not resolved
4 Rash					not recovered/not resolved
5 Pustule					not recovered/not resolved
6 Blistering					not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ARTHREX SCHMERZGEL		/	20120515-	Tendonitis
diclofenac sodium		95	-	

Primary source

Qualification:	Other Health Professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 13.06.2012 DE-SZ09-GXKR2012DE001933

Sender diagnosis: not reported

Narrative

Case number GXKR2012DE001933 is an initial spontaneous report from a physician assistant received on 06 Jun 2012. This report refers to an 11-years-old male patient. The patient's medical history and concomitant medication was not reported. The patient received Arthrex (diclofenac) for tendonitis from unreported date. On an unreported date, the patient experienced Allergic reaction with blister and urticaria spread over the whole body. The patient needed a hospitalization. The events were considered as serious by MAH. No further details or causality assessment were provided by the reporter.

Sender's comment:

Serious spontaneous report, Hypersensitivity (hospitalisation), assessed as listed according to the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements

Reporter's comment:NOVARTIS, 18.06.2012 DE-SZ09-GXKR2012DE001933

Sender diagnosis: not reported

Narrative

Case number GXKR2012DE001933 is a spontaneous report initially received from a physician assistant on 06Jun2012, with follow-up from a physician received on 12Jun2012. This report refers to an 11year-old male patient. Medical history and concomitant medications were not reported. The patient received arthrex Schmerzgel (diclofenac) for the treatment of tendonitis once on 15May2012 (dose not reported), and, on the same day (15May2012) experienced allergic skin reaction with blistering, urticaria on whole body and face, swelling, rash and pustules on both feet. The event

caused hospitalisation for 4 days. Treatment with arthrex Schmerzgel was stopped (date not reported). Outcome of the event was reported as not recovered. The physician reported the events were suspected to be certainly related to treatment with arthrex Schmerzgel.

Follow-up received on 12Jun2012: updated reporter, patient details, adverse events, therapy date and dose regimen, onset date, reporters causality assessment, Novartis comment and outcome.

Sender's comment:

Serious spontaneous report, dermatitis allergic (hospitalisation), assessed as listed according to the Core Data Sheet. All spontaneous reports are considered suspected for reporting purposes or in accordance with national regulatory requirements.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12145923

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 49 Year	weight (kg): 79.9	BMI_int: 28.31	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Erythema exudativum multiforme	20120630			1 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac 1 A Pharma diclofenac	Unknown	75 Mg milligram(s)/Day	20120630-20120630	Back pain
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension arterial			Yes	
Anxiety disorder			Yes	
Hyperlipidaemia			Yes	
Diabetes mellitus			Yes	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12168491

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Female Birth: privacy Age: 44 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pain NOS	201206		1 Week		Unknown
2 Vesicles	201206		1 Week	2 Day	Unknown
3 Itch	201206		1 Week	2 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 diclofenac-ratiopharm gel diclofenac sodium	Topical	/	20120618-20120618	-

Primary source

Qualification: Physician observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-12188431

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 57 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Exanthema	20120812			1 Day	recovered/resolved
2 Vesicular rash	20120812			1 Day	not recovered/not resolved
3 Blistering	20120812			1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN EMULGEL diclofenac diethylamine	Topical	/ 95	20120812-20120812 1 Day	Inflammation

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 27.08.2012 DE-002147023-CHPA2012DE018337

Sender diagnosis: not reported

Narrative

Initial report was received from pharmacist on 17 Aug 2012 via German Pharmacist's Drug Commission (number [171722]). This 57-year-old male patient used Voltaren Emulgel (diclofenac diethylamine salt) as "topical inflammatory treatment on arms and legs" once on 12 Aug 2012. On the same day, the patient developed "exanthema and blistering NOS on his extremities".

The patient was treated with Telfast (fexofenadine hydrochloride , Rx-medicinal-drug) and Fenistil Wundheilgel (acid colloidal hydrocolloid, medical device). Around date of this report, patient's adverse symptoms had improved but were still persisting. This case was assessed as medically significant because it involved treatment with an Rx-medicinal- drug. The causal relationship was judged as being related by the reporting pharmacist. No further information available at date of this report. Outcome: reaction improving. Product use was discontinued. Further follow-up information requested in written in week 34, 2012.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) confirmed by a health care professional. The adverse event(s), Blister, was assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. The adverse event, Rash, is listed in the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit-risk balance of the medicinal product.

Reporter's comment:NOVARTIS, 27.08.2012 DE-NOVNCHHQ-CHPA2012DE018337

Sender diagnosis: not reported

Narrative

Initial report was received from pharmacist on 17 Aug 2012 via German Pharmacist's Drug Commission (number [171722]). This 57-year-old male

patient used Voltaren Emulgel (diclofenac diethylamine salt) as "topical inflammatory treatment on arms and legs" once on 12 Aug 2012. On the same day, the patient developed "exanthema and blistering NOS on his extremities".

The patient was treated with Telfast (fexofenadine hydrochloride , Rx-medicinal-drug) and Fenistil Wundheilgel (acid colloidal hydrocolloid, medical device). Around date of this report, patient's adverse symptoms had improved but were still persisting. This case was assessed as medically significant because it involved treatment with an Rx-medicinal- drug. The causal relationship was judged as being related by the reporting pharmacist. No further information available at date of this report. Outcome: reaction improving. Product use was discontinued. Further follow-up information requested in written in week 34, 2012.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) confirmed by a health care professional. The adverse event(s), Blister, was assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. The adverse event, Rash, is listed in the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit-risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 31.08.2012 DE-002147023-CHPA2012DE018337

Sender diagnosis: not reported

Narrative

Initial report was received from pharmacist on 17 Aug 2012 via German Pharmacist's Drug Commission (number [171722]). This 57-year-old male patient used Voltaren Emulgel (diclofenac diethylamine salt) as "topical inflammatory treatment on arms and legs" once on 12 Aug 2012. On the same day, the patient developed "exanthema and blistering NOS on his extremities". The patient was treated with Telfast (fexofenadine hydrochloride , Rx-medicinal-drug) and Fenistil Wundheilgel (acid colloidal hydrocolloid, medical device). Around date of this report, patient's adverse symptoms had improved but were still persisting. This case was assessed as medically significant because it involved treatment with an Rx-medicinal- drug. The causal relationship was judged as being related by the reporting pharmacist. No further information available at date of this report. Outcome: reaction improving. Product use was discontinued. Further follow-up information requested in written in week 34, 2012.

Additional information received from a pharmacist on 27 Aug 2012. This 57 year old male patient developed "exanthema NOS" after topical application of Voltaren Emulgel (diclofenac diethylammonium salt) used as "topical anti-inflammatory treatment on arms and legs". Therapy dates were specified as 12 Aug 2012 to 12 Aug 2012 (used once). The stated daily dose was "topically applied once". Voltaren Emulgel was stopped. Following drug treatment with Telfast (fexofenadine hydrochloride , Rx-medicinal-drug) and Fenistil Wundheilgel (acid colloidal hydrocolloid, medical device) prescribed by treating physician, the reaction was reported as improving. The reporter rated the reaction as "medically significant" due to the treatment with an Rx-medical-drug. Causality rating of reporter was probable. Outcome was reported as improvement.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) confirmed by a health care professional. The adverse event(s), Blister, was assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. The adverse event, Rash, is listed in the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit-risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 31.08.2012 DE-NOVNCHHQ-CHPA2012DE018337

Sender diagnosis: not reported

Narrative

Initial report was received from pharmacist on 17 Aug 2012 via German Pharmacist's Drug Commission (number [171722]). This 57-year-old male patient used Voltaren Emulgel (diclofenac diethylamine salt) as "topical inflammatory treatment on arms and legs" once on 12 Aug 2012. On the same day, the patient developed "exanthema and blistering NOS on his extremities". The patient was treated with Telfast (fexofenadine hydrochloride , Rx-medicinal-drug) and Fenistil Wundheilgel (acid colloidal hydrocolloid, medical device). Around date of this report, patient's adverse symptoms had improved but were still persisting. This case was assessed as medically significant because it involved treatment with an Rx-medicinal- drug. The causal relationship was judged as being related by the reporting pharmacist. No further information available at date of this report. Outcome: reaction improving. Product use was discontinued. Further follow-up information requested in written in week 34, 2012.

Additional information received from a pharmacist on 27 Aug 2012. This 57 year old male patient developed "exanthema NOS" after topical application of Voltaren Emulgel (diclofenac diethylammonium salt) used as "topical anti-inflammatory treatment on arms and legs". Therapy dates were specified as 12 Aug 2012 to 12 Aug 2012 (used once). The stated daily dose was "topically applied once". Voltaren Emulgel was stopped. Following drug treatment with Telfast (fexofenadine hydrochloride , Rx-medicinal-drug) and Fenistil Wundheilgel (acid colloidal hydrocolloid, medical device) prescribed by treating physician, the reaction was reported as improving. The reporter rated the reaction as "medically significant" due to the treatment with an Rx-medical-drug. Causality rating of reporter was probable. Outcome was reported as improvement.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (Medically significant) confirmed by a health care professional. The adverse event(s), Blister, was assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. The adverse event, Rash, is listed in the Reference Safety Information. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit-risk balance of the medicinal product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-12227019

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 49 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin necrosis					Unknown
2 Postoperative wound infection					Unknown
3 Wrong injection technique					recovering/resolving
4 Necrosis skin					recovering/resolving
5 Drug administered via inappropriate route					recovering/resolving
6 Fat necrosis					recovering/resolving
7 Necrosis					Unknown
8 Inappropriate dose of drug administered					Unknown
9 Inappropriate schedule of drug administration					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Dexamethason 4 mg	Intramuscular	4 Mg milligram(s)/	-	Back pain
dexamethasone		120	-	
2 diclofenac	Intramuscular	/	-	Back pain
diclofenac			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Surgery			Unknown	
Spinal disorder			Yes	Chronic recurrent.
Obesity			Yes	
Back pain			Yes	due to "chronic recurrent spinal syndrome"
Liposuction			k.A.	for removal of subcutaneous fat from the buttock.

Primary source

Qualification:	Physician	observestudytype:
Literature:	Holland C, Jaeger L, Smentkowski U, Weber B, Otto C. Septic and aseptic complications of corticosteroid injections: An assessment of 278 cases reviewed by expert commissions and mediation boards from 2005 to 2009. Deuts-Arztebl 2012; 109(24) 425-430	
Qualification:	Other Health Professional	observestudytype:
Literature:	Holland C, Jaeger L, Smentkowski U, Weber B, Otto C.. Septic and Aseptic Complications of Corticosteroid Injections. Dtsch Arztebl Int. 2012;109(24):425-30	

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 13.09.2012 DE-SZ09-PHHY2012DE078319

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE078319, is an initial literature report received on 07 Sep 2012. The author discussed about septic and aseptic complications of corticosteroid injections. This report refers about 49-years-old female patient. The patient received dexamethasone phosphate 4mg (unknown manufacturer), dexamethasone (unknown manufacturer) 4 mg and diclofenac injections for chronic recurrent spinal syndrome on unspecified date and dose. Approximately 6 weeks after her last injection, she required surgery for the excision of necrotic skin and subcutaneous fat from her buttock. Following resection, she developed an infection that arose at the resection site that required further hospitalizations. An expert panel determined that two treatment errors had occurred that injection was administered into adipose tissue, rather than the underlying musculature and repeated parenteral administration of the drugs, rather than switching to the oral route. Seriousness reported as serious (medically significant) by the

author. The outcome of the event was not reported. The author concluded that It is known that corticosteroid injections in too rapid succession can promote the development of infection and/or frequently lead to local aseptic tissue damage. This conclusion was even more certain with respect to gluteal injections at an inadequate depth which often cause a typical pattern of necrosis in the skin and subcutaneous fat of the buttock.

Sender's comment:

MAC: Based on review of available reported data there is a reasonable possibility of causal relationship for the event skin necrosis and infection.

Reporter's comment:

NOVARTIS, 14.09.2012 DE-002147023-PHHY2012DE078319

Sender diagnosis: not reported

Narrative

Case number PHHY2012DE078319, is an initial literature report received on 07 Sep 2012. The author discussed about septic and aseptic complications of corticosteroid injections. This report refers about 49-years-old female patient. The patient received dexamethasone phosphate 4mg (unknown manufacturer), dexamethasone (unknown manufacturer) 4 mg and diclofenac injections for chronic recurrent spinal syndrome on unspecified date and dose. Approximately 6 weeks after her last injection, she required surgery for the excision of necrotic skin and subcutaneous fat from her buttock. Following resection, she developed an infection that arose at the resection site that required further hospitalizations. An expert panel determined that two treatment errors had occurred that injection was administered into adipose tissue, rather than the underlying musculature and repeated parenteral administration of the drugs, rather than switching to the oral route. Seriousness reported as serious (medically significant) by the author. The outcome of the event was not reported. The author concluded that It is known that corticosteroid injections in too rapid succession can promote the development of infection and/or frequently lead to local aseptic tissue damage. This conclusion was even more certain with respect to gluteal injections at an inadequate depth which often cause a typical pattern of necrosis in the skin and subcutaneous fat of the buttock.

Sender's comment:

MAC: Based on review of available reported data there is a reasonable possibility of causal relationship for the event skin necrosis and infection.

Reporter's comment:

TEVA, 20.09.2012 DE-TEVA-358673GER

Sender diagnosis: not reported

Narrative

10-Sep-2012, Serious Literature Report received from Reaction (#803077013). (SPRINGER HEALTHCARE; NZ-ADIS-3077013-28624)
Case of a Female patient 49 Years of age who experienced NECROTIC SKIN, SUBCUTANEOUS FAT ON THE BUTTOCK (NECROSIS), AN INFECTION THAT AROSE AT THE RESECTION SITE, INJECTION INTO ADIPOSE TISSUE, RATHER THAN INTO DEEPER-LYING MUSCULATURE, REPEATED PARENTERAL APPLICATION OF THE SUBSTANCES, INSTEAD OF A SWITCH TO ORAL ADMINISTRATION while on Dexamethasone 4 mg(DEXAMETHASONE) (Teva's product cannot be excluded).

In a retrospective review of 278 patients, a 49-year-old obese woman was identified after she developed extensive necrosis following administration of dexamethasone. An expert panel later determined that treatment errors had occurred. The woman, who had back pain due to "chronic recurrent spinal syndrome", received multiple intragluteal injections of dexamethasone phosphate 4mg, dexamethasone 4mg (frequency and duration of treatment not stated) and diclofenac. Approximately 6 weeks after her last injection, she required surgery for the excision of necrotic skin and subcutaneous fat from her buttock. Following resection, she developed an infection that arose at the resection site that required further hospitalisations. An expert panel determined that two treatment errors had occurred: injection into adipose tissue, rather than the underlying musculature, and repeated parenteral administration of the drugs, rather than switching to the oral route.

Author comment: With respect to gluteal injections at an inadequate depth (as in Case 2), (these) often cause a typical pattern of necrosis in the skin and subcutaneous fat of the buttock.

At the time of the report the outcome of the AEs were: NECROTIC SKIN : recovering/resolving, SUBCUTANEOUS FAT ON THE BUTTOCK (NECROSIS) : recovering/resolving, AN INFECTION THAT AROSE AT THE RESECTION SITE : Unknown, INJECTION INTO ADIPOSE TISSUE, RATHER THAN INTO DEEPER-LYING MUSCULATURE : recovering/resolving, REPEATED PARENTERAL APPLICATION OF THE SUBSTANCES, INSTEAD OF A SWITCH TO ORAL ADMINISTRATION : recovering/resolving.

Concomitant Medication:

Diclofenac(diclofenac)

Medical History:

BACK PAIN(Concomitant) ,OBESITY(Concomitant)

Action Taken with suspect Drugs: Dexamethasone 4 mg - Not applicable

Literature Reference:

Holland C, Jaeger L, Smentkowski U, Weber B, Otto C. Septic and aseptic complications of corticosteroid injections: An assessment of 278 cases reviewed by expert commissions and mediation boards from 2005 to 2009. Deuts-Arztebl 2012; 109(24) 425-430

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: Reported events "necrotic skin", "subcutaneous fat on the buttock (necrosis)" as well as "an infection that arose at the resection site" are possibly related to treatment with Dexamethasone erroneously injected into adipose tissue rather than intramuscular. "Injection into adipose tissue, rather than into deeper-lying musculature" as well as "repeated parenteral application of the substances, instead of switch to oral administration" are not related to Dexamethasone itself but provided the basis for the development of previously reported adverse events.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case.

Sender's comment:

Serious literature report (TEVA product could not be excluded) case report from Germany. Duty for notification according to German Drug Law. Causality assessment: see case narrative TEVA comment. Assessment of expectedness according to current German SmPC and PIL: unexpected (except for postoperative wound infection). The risk-benefit ratio of the treatment with the drug in general can be regarded as unchanged. Changes in the statements of the German SmPC and PIL regarding special warnings, special precautions for use and undesirable effects due to this case seem not to be justified at present.

Reporter's comment:

KRKAPR, 26.09.2012 DE-KRKA-DE2012K3237LIT

Sender diagnosis: not reported

Narrative

Dexamethasone/triamcinolone

Abscesses and extensive necrosis: 2 case reports - SERIOUS

In a retrospective review of 278 patients, two women were identified; one woman developed multiple abscesses after receiving triamcinolone, while the other developed extensive necrosis after receiving dexamethasone. An expert panel later determined that treatment errors had occurred.

A 49-year-old obese woman, with back pain due to "chronic recurrent spinal syndrome", received multiple intragluteal injections of dexamethasone phosphate 4mg, dexamethasone 4mg [frequency and duration of treatment not stated] and diclofenac. Approximately 6 weeks after her last injection, she required surgery for the excision of necrotic skin and subcutaneous fat from her buttock. Following resection, she developed an infection that arose at the resection site that required further hospitalisations. An expert panel determined that two treatment errors had occurred: injection into adipose tissue, rather than the underlying musculature, and repeated parenteral administration of the drugs, rather than switching to the oral route. Author Comment "[I]t is known that corticosteroid injections in too rapid succession can promote the development of infection and/or frequently lead to local aseptic tissue damage. . . This conclusion is even more certain with respect to gluteal injections at an inadequate depth (as in Case 2), which often cause a typical pattern of necrosis in the skin and subcutaneous fat of the buttock".

CITATION DETAILS Holland C, Jaeger L, Smentkowski U, Weber B, Otto C. Septic and aseptic complications of corticosteroid injections: An

assessment of 278 cases reviewed by expert commissions and mediation boards from 2005 to 2009. Deutsches Arzteblatt 109: 425-430, No. 24, 15 Jun 2012. Available from: URL: <http://dx.doi.org/10.3238/arztebl.2012.0425> (English)

Krka's comment:

- diclofenac: S, UL, unlikely related

Other drugs (dexamethasone, dexamethasone phosphate) provide plausible explanation.

- for dexamethasone: case is S, UL, possibly related. The patient was concomitantly receiving diclofenac. Injections were probably given into adipose tissue, rather than the underlying musculature.

Sender's comment:

Reporter's comment:

PANPHARMAP, 02.10.2012 DE-PANPHARMAP-201200359

Sender diagnosis: not reported

Narrative

The initial information was published in the scientific literature:

Holland C, Jaeger L, Smentkowski U, Weber B, Otto C.. Septic and aseptic complications of corticosteroid injections: an assessment of 278 cases reviewed by expert commissions and mediation boards from 2005 to 2009.. Dtsch Arztebl Int.. 2012 Jun;109(24):425-30.

BACKGROUND:

Local corticosteroid injections can have serious septic and aseptic complications.

METHODS: From 2005 to 2009, medical expert committees and mediation boards reviewed 1528 cases of alleged treatment errors relating to injections.

RESULTS: 278 cases were identified in which complications arose after local glucocorticosteroid injections. The injections were intra-articular, paravertebral, intramuscular, and at other sites. In 39.6% of cases, treatment errors or patient information errors of the following types were found: aseptic technique was not maintained, injections were performed in the absence of an indication, time intervals between injections were too short, excessive doses were administered, infections were not diagnosed, erroneous injections were performed, patients were not informed of the risks, and there were errors of organization and documentation.

CASE:

Extensive necrosis after intragluteal injection for back pain.

An obese 49-year-old woman who was said to be suffering from a "chronic, recurrent spinal syndrome" received multiple intragluteal injections of 4 mg of dexamethasone dihydrogen phosphate, 4 mg of dexamethasone, and diclofenac. About six weeks after the last injection of one of these substances, she required surgery for the removal of 500 g of necrotic skin and subcutaneous fat from the buttock. An infection that arose at the resection site necessitated multiple further hospitalizations. The expert panel determined that there were two treatment errors: injection into adipose tissue, rather than into the deeper-lying musculature, and the repeated parenteral application of the substances, instead of a switch to oral administration.

CONCLUSIONS: Injections of glucocorticosteroids must be performed in strict adherence to the manufacturer's instructions with respect to the composition of the solution to be injected, the quantity per injection, and the intervals between injections. Repeated injections with too little time between them raise the risk of infection. Physicians should pay more attention to this fact, particularly when deciding on the indication for paravertebral injections. Aseptic technique should be strictly maintained. The indication for the injection should be clearly documented. When glucocorticosteroids are injected into small joints and tendon spaces, the introduction of crystals into the subcutaneous tissue and adipose tissue should be avoided. The intramuscular administration of depot glucocorticosteroids should be avoided. Patients should be informed of the risk of infection and/or tissue atrophy, as well as of alternative forms of treatment.

Sender's comment:

According to the French SmPC used as Reference Safety Information, skin necrosis and injection site necrosis are unlisted.

In the present misuse case and according to the Company, dexamethasone dihydrogen phosphate, diclofenac and dexamethasone were considered as suspect drug and the causal relationship was assessed as dubious (C1 S2 I1) according to the French methodology of causality assessment and probable according to the adapted WHO-UMC causality assessment (chronology compatible (6 weeks after the last injection of one of these

substances), events occurred at the injection site and two treatment errors have been performed).

Reporter's comment:

NOVARTIS, 13.04.2015 DE-002147023-PHHY2015DE041440

Sender diagnosis: not reported

Narrative

Case number PHHY2015DE041440, is an initial literature report received on 01 Apr 2015. The authors discussed about the septic and aseptic complications of corticosteroid injections. This report refers to a 49-year-old female patient. The patient's medical history and concomitant medications were not reported. On an unknown date the patient received multiple intragluteal injections of 4 mg of dexamethasone dihydrogen phosphate, 4 mg dexamethasone and diclofenac (manufacturer and frequency unknown) for the treatment of chronic, recurrent spinal syndrome. Six weeks after the last injection of one of the three substances the patient presented with necrotic skin (500 g removed during surgery) and subcutaneous fat accumulation in buttock. As a treatment patient underwent a surgery to remove the fat. Following which the patient was hospitalised for an infection at the resection site. Action taken with dexamethasone and diclofenac were not reported. Outcome of the event was not reported. The experts determined it to be medication errors of injection into adipose tissue, rather than into deeper-lying musculature and repeated parenteral application of substance, instead of a switch to oral administration. Author assessed event as suspected.

Sender's comment:

The events are considered suspected considering the temporal association and the lack of alternative explanations and the event being listed for the drug

Reporter's comment:

SANDOZ, 13.04.2015 DE-SZ09-PHHY2015DE041440

Sender diagnosis: not reported

Narrative

Case number PHHY2015DE041440, is an initial literature report received on 01 Apr 2015. The authors discussed about the septic and aseptic complications of corticosteroid injections. This report refers to a 49-year-old female patient. The patient's medical history and concomitant medications were not reported. On an unknown date the patient received multiple intragluteal injections of 4 mg of dexamethasone dihydrogen phosphate, 4 mg dexamethasone and diclofenac (manufacturer and frequency unknown) for the treatment of chronic, recurrent spinal syndrome. Six weeks after the last injection of one of the three substances the patient presented with necrotic skin (500 g removed during surgery) and subcutaneous fat accumulation in buttock. As a treatment patient underwent a surgery to remove the fat. Following which the patient was hospitalised for an infection at the resection site. Action taken with dexamethasone and diclofenac were not reported. Outcome of the event was not reported. The experts determined it to be medication errors of injection into adipose tissue, rather than into deeper-lying musculature and repeated parenteral application of substance, instead of a switch to oral administration. Author assessed event as suspected.

Sender's comment:

The events are considered suspected considering the temporal association and the lack of alternative explanations and the event being listed for the drug

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13028833

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		No

Patient

Initials:	Sex: Male	Birth: privacy	Age: 73 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Allergic reaction	201302				Unknown
2 Herpes zoster	20130222			8 Day	Unknown
3 Skin discoloration	201302				Unknown
4 Pustule	201302				Unknown
5 Redness	201302				Unknown
6 Pain	201302				Unknown
7 Skin red	201302				Unknown
8 Vesicles	201302				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN EMULGEL	Topical	/	20130215-20130219	Shoulder pain
diclofenac diethylamine		95	5 Day	

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 08.03.2013 DE-NOVNCHHQ-CHPA2013DE004060

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from daughter of patient on 21 Feb 2013. This 73-year-old male patient applied Voltaren Schmerzgel (diclofenac diethylamine salt) (date of expiry 12/2014) on his neck a few days ago due to severe neck pain. The patient had used the drug very often before, but this time he developed "a severe allergic reaction with pustules, redness, severe pain and black spots...". The patient was appalled and not able to understand the reaction. No further information available at date of this report. A QA investigation was requested. Outcome: not reported. Follow-up information requested in written form in week 9, 2013.

Follow-up information was received on 04 Mar 2013 upgraded the case to meet the serious criteria of hospitalization. The consumer's daughter report that the consumer received in-patient treatment in hospital with unspecified infusions. Updated: patient data, kind of reaction, CIOMS criteria, indication for use, therapy dates and outcome. This 73 year old male patient developed 'tentative allergic skin reaction, skin redness' after topical application of Voltaren Schmerzgel (diclofenac diethylamine salt) used to treat pre-existing 'severe neck pain' on 18 Feb 2013. On 20 Feb 2013 he consulted his family doctor who 'diagnosed an allergic skin reaction' and treated the reactions with unspecified 'cortisone ointment' and unspecified 'analgesics'. On 22 Feb 2013 he could not bear the pain and consulted a dermatologist who 'diagnosed instead acute herpes zoster'. The patient was hospitalized and was treated as in-patient with unspecified infusions since 22 Feb 2013 (upgrading the case to serious). Therapy dates were specified as 18 Feb 2013 to not reported. The reactions were still present at date of follow-up report. Outcome was not reported.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (hospitalization) not confirmed by a health care professional. The adverse event(s) Herpes zoster was/were assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 21.03.2013 DE-NOVNCHHQ-CHPA2013DE004060

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from daughter of patient on 21 Feb 2013. This 73-year-old male patient applied Voltaren Schmerzgel (diclofenac diethylamine salt) (date of expiry 12/2014) on his neck a few days ago due to severe neck pain. The patient had used the drug very often before, but this time he developed "a severe allergic reaction with pustules, redness, severe pain and black spots...". The patient was appalled and not able to understand the reaction. No further information available at date of this report. A QA investigation was requested. Outcome: not reported. Follow-up information requested in written form in week 9, 2013.

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Follow up received on 18 Mar 2013 is a result of a technical complaint investigation: Technical complaint# 10991972. Due to the given information and without sample a deeper investigation is not possible. There are different possible root causes and none of them can be finally excluded. Therefore it is urgently necessary to get the complaint sample. At least without sample we cannot confirm the quality defect of the product. The API manufacturing site is not informed by us as important information for investigation is missing. The HA is not involved thus this is a packaging issue. Risk assessment cannot be performed with the given information. The medical risk assessment has to be done by the pharmacovigilance/DS&E. We classify this case as unclear and critical.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (hospitalization) not confirmed by a health care professional. The adverse event(s) Herpes zoster was/were assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 26.03.2013 DE-NOVNCHHQ-CHPA2013DE004060

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from daughter of patient on 21 Feb 2013. This 73-year-old male patient applied Voltaren Schmerzgel (diclofenac diethylamine salt) (date of expiry 12/2014) on his neck a few days ago due to severe neck pain. The patient had used the drug very often before, but this time he developed "a severe allergic reaction with pustules, redness, severe pain and black spots...". The patient was appalled and not able to understand the reaction. No further information available at date of this report. A QA investigation was requested. Outcome: not reported. Follow-up information requested in written form in week 9, 2013.

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'analgesics'. On 22 Feb 2013 he could not bear the pain and consulted a dermatologist who 'diagnosed instead acute herpes zoster'. The patient was hospitalized and was treated as in-patient with unspecified infusions since 22 Feb 2013 (upgrading the case to serious). Therapy dates were specified as 18 Feb 2013 to not reported. The reactions were still present at date of follow-up report. Outcome was not reported.

Follow up received on 18 Mar 2013 is a result of a technical complaint investigation: Technical complaint# 10991972. Due to the given information and without sample a deeper investigation is not possible. There are different possible root causes and none of them can be finally excluded. Therefore it is urgently necessary to get the complaint sample. At least without sample we cannot confirm the quality defect of the product. The API manufacturing site is not informed by us as important information for investigation is missing. The HA is not involved thus this is a packaging issue. Risk assessment cannot be performed with the given information. The medical risk assessment has to be done by the pharmacovigilance/DS&E. We classify this case as unclear and critical.

Follow-up information was received on 21 Mar 2013. Updated: patient data, kind of reaction, suspected drug, indication for use, daily dose, indication for use, therapy dates, information related to suspected drug and adverse reaction, concomitant treatment, outcome and causality rating. This 73 year old male patient used Voltaren Emulgel (diclofenac diethylamine salt) to treat 'shoulder pain in self-medication' and had rubbed in the "Emulgel" topically at skin of shoulder several times per day" until he developed 'vesicles and skin redness which led to wrong diagnose of contact allergy by family doctor but were in fact the skin symptoms and signs of herpes zoster'. Therapy dates were specified as 15 Feb 2013 to 19 Feb 2013 (continuous treatment). The stated daily dose was "Emulgel" topically rubbed in at skin of shoulder several times per day'. Voltaren Emulgel was stopped on 19 Feb 2013. The reporting physician ticked the box that the reaction was treated by physician. The reporter added that there was no causal relationship between the zoster and the use of Voltaren Emulgel. The reporting physician ticked the box for 'involved or prolonged inpatient hospitalisation' underlined the sentence part 'in timely relationship' with the use of Voltaren Emulgel and added handwritten hint: 'but not in causal relationship with the use of Voltaren Emulgel. The 'reactivation of herpeszoster virus infection' was reported as alternative cause for the reaction. Causality rating of reporter was independent. Outcome: not reported.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (hospitalization) not confirmed by a health care professional. The adverse event(s) Herpes zoster was/were assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13031328

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		No

Patient

Initials:	Sex: Male	Birth: privacy	Age: 73 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Allergic reaction	201302				Unknown
2 Herpes zoster	20130222			8 Day	Unknown
3 Skin discoloration	201302				Unknown
4 Vesicles	201302				Unknown
5 Skin red	201302				Unknown
6 Pain	201302				Unknown
7 Redness	201302				Unknown
8 Pustule	201302				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN EMULGEL	Topical	/	20130215-20130219	Shoulder pain
diclofenac diethylamine		95	5 Day	

Primary source

Qualification: Consumer or other non health professional observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 08.03.2013 DE-002147023-CHPA2013DE004060

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from daughter of patient on 21 Feb 2013. This 73-year-old male patient applied Voltaren Schmerzgel (diclofenac diethylamine salt) (date of expiry 12/2014) on his neck a few days ago due to severe neck pain. The patient had used the drug very often before, but this time he developed "a severe allergic reaction with pustules, redness, severe pain and black spots...". The patient was appalled and not able to understand the reaction. No further information available at date of this report. A QA investigation was requested. Outcome: not reported. Follow-up information requested in written form in week 9, 2013.

Follow-up information was received on 04 Mar 2013 upgraded the case to meet the serious criteria of hospitalization. The consumer's daughter report that the consumer received in-patient treatment in hospital with unspecified infusions. Updated: patient data, kind of reaction, CIOMS criteria, indication for use, therapy dates and outcome. This 73 year old male patient developed 'tentative allergic skin reaction, skin redness' after topical application of Voltaren Schmerzgel (diclofenac diethylamine salt) used to treat pre-existing 'severe neck pain' on 18 Feb 2013. On 20 Feb 2013 he consulted his family doctor who 'diagnosed an allergic skin reaction' and treated the reactions with unspecified 'cortisone ointment' and unspecified 'analgesics'. On 22 Feb 2013 he could not bear the pain and consulted a dermatologist who 'diagnosed instead acute herpes zoster'. The patient was hospitalized and was treated as in-patient with unspecified infusions since 22 Feb 2013 (upgrading the case to serious). Therapy dates were specified as 18 Feb 2013 to not reported. The reactions were still present at date of follow-up report. Outcome was not reported.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (hospitalization) not confirmed by a health care professional. The adverse event(s) Herpes zoster was/were assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 21.03.2013 DE-002147023-CHPA2013DE004060

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from daughter of patient on 21 Feb 2013. This 73-year-old male patient applied Voltaren Schmerzgel (diclofenac diethylamine salt) (date of expiry 12/2014) on his neck a few days ago due to severe neck pain. The patient had used the drug very often before, but this time he developed "a severe allergic reaction with pustules, redness, severe pain and black spots...". The patient was appalled and not able to understand the reaction. No further information available at date of this report. A QA investigation was requested. Outcome: not reported. Follow-up information requested in written form in week 9, 2013.

Follow-up information was received on 04 Mar 2013 upgraded the case to meet the serious criteria of hospitalization. The consumer's daughter report that the consumer received in-patient treatment in hospital with unspecified infusions. Updated: patient data, kind of reaction, CIOMS criteria, indication for use, therapy dates and outcome. This 73 year old male patient developed 'tentative allergic skin reaction, skin redness' after topical application of Voltaren Schmerzgel (diclofenac diethylamine salt) used to treat pre-existing 'severe neck pain' on 18 Feb 2013. On 20 Feb 2013 he consulted his family doctor who 'diagnosed an allergic skin reaction' and treated the reactions with unspecified 'cortisone ointment' and unspecified 'analgesics'. On 22 Feb 2013 he could not bear the pain and consulted a dermatologist who 'diagnosed instead acute herpes zoster'. The patient was hospitalized and was treated as in-patient with unspecified infusions since 22 Feb 2013 (upgrading the case to serious). Therapy dates were specified as 18 Feb 2013 to not reported. The reactions were still present at date of follow-up report. Outcome was not reported.

Follow up received on 18 Mar 2013 is a result of a technical complaint investigation: Technical complaint# 10991972. Due to the given information and without sample a deeper investigation is not possible. There are different possible root causes and none of them can be finally excluded. Therefore it is urgently necessary to get the complaint sample. At least without sample we cannot confirm the quality defect of the product. The API manufacturing site is not informed by us as important information for investigation is missing. The HA is not involved thus this is a packaging issue. Risk assessment cannot be performed with the given information. The medical risk assessment has to be done by the pharmacovigilance/DS&E. We classify this case as unclear and critical.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (hospitalization) not confirmed by a health care professional. The adverse event(s) Herpes zoster was/were assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

NOVARTIS, 26.03.2013 DE-002147023-CHPA2013DE004060

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from daughter of patient on 21 Feb 2013. This 73-year-old male patient applied Voltaren Schmerzgel (diclofenac diethylamine salt) (date of expiry 12/2014) on his neck a few days ago due to severe neck pain. The patient had used the drug very often before, but this time he developed "a severe allergic reaction with pustules, redness, severe pain and black spots...". The patient was appalled and not able to understand the reaction. No further information available at date of this report. A QA investigation was requested. Outcome: not reported. Follow-up information requested in written form in week 9, 2013.

Follow-up information was received on 04 Mar 2013 upgraded the case to meet the serious criteria of hospitalization. The consumer's daughter report that the consumer received in-patient treatment in hospital with unspecified infusions. Updated: patient data, kind of reaction, CIOMS criteria, indication for use, therapy dates and outcome. This 73 year old male patient developed 'tentative allergic skin reaction, skin redness' after topical application of Voltaren Schmerzgel (diclofenac diethylamine salt) used to treat pre-existing 'severe neck pain' on 18 Feb 2013. On 20 Feb 2013 he consulted his family doctor who 'diagnosed an allergic skin reaction' and treated the reactions with unspecified 'cortisone ointment' and unspecified

'analgesics'. On 22 Feb 2013 he could not bear the pain and consulted a dermatologist who 'diagnosed instead acute herpes zoster'. The patient was hospitalized and was treated as in-patient with unspecified infusions since 22 Feb 2013 (upgrading the case to serious). Therapy dates were specified as 18 Feb 2013 to not reported. The reactions were still present at date of follow-up report. Outcome was not reported.

Follow up received on 18 Mar 2013 is a result of a technical complaint investigation: Technical complaint# 10991972. Due to the given information and without sample a deeper investigation is not possible. There are different possible root causes and none of them can be finally excluded. Therefore it is urgently necessary to get the complaint sample. At least without sample we cannot confirm the quality defect of the product. The API manufacturing site is not informed by us as important information for investigation is missing. The HA is not involved thus this is a packaging issue. Risk assessment cannot be performed with the given information. The medical risk assessment has to be done by the pharmacovigilance/DS&E. We classify this case as unclear and critical.

Follow-up information was received on 21 Mar 2013. Updated: patient data, kind of reaction, suspected drug, indication for use, daily dose, indication for use, therapy dates, information related to suspected drug and adverse reaction, concomitant treatment, outcome and causality rating. This 73 year old male patient used Voltaren Emulgel (diclofenac diethylamine salt) to treat 'shoulder pain in self-medication' and had rubbed in the "Emulgel" topically at skin of shoulder several times per day" until he developed 'vesicles and skin redness which led to wrong diagnose of contact allergy by family doctor but were in fact the skin symptoms and signs of herpes zoster'. Therapy dates were specified as 15 Feb 2013 to 19 Feb 2013 (continuous treatment). The stated daily dose was "Emulgel" topically rubbed in at skin of shoulder several times per day'. Voltaren Emulgel was stopped on 19 Feb 2013. The reporting physician ticked the box that the reaction was treated by physician. The reporter added that there was no causal relationship between the zoster and the use of Voltaren Emulgel. The reporting physician ticked the box for 'involved or prolonged inpatient hospitalisation' underlined the sentence part 'in timely relationship' with the use of Voltaren Emulgel and added handwritten hint: 'but not in causal relationship with the use of Voltaren Emulgel. The 'reactivation of herpeszoster virus infection' was reported as alternative cause for the reaction. Causality rating of reporter was independent. Outcome: not reported.

Sender's comment:

All spontaneous reports are considered suspected for reporting purposes. This is a serious spontaneous report (hospitalization) not confirmed by a health care professional. The adverse event(s) Herpes zoster was/were assessed as unlisted according to the Reference Safety Information. The available information was considered inadequate to fully assess the case. While a single spontaneous report of an adverse reaction may contain sufficient information to suggest a causal association with the suspected medicinal product, NCH also examines the safety profile of the product with signal detection activities to fully assess the impact of the safety information on the benefit- risk balance of the medicinal product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13131230

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
Yes

Patient

Initials: Sex: Female Birth: privacy Age: 23 Year weight (kg): 65 BMI_int: 25.71 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering	20130405	20130412	8 Day		not recovered/not resolved
2 Skin injury	20130405	20130412	8 Day		not recovered/not resolved
3 Burning skin	20130405	20130412	8 Day		not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclofenac-ratiopharm	Topical	/	-	
diclofenac sodium			-	

Drugs (concomitant)

2 atmadisc	/	-	
fluticasone propionate; salmeterol xinafoate		-	
3 ibuprofen 600	/	-	
ibuprofen		-	
4 salbutamol	/	-	
salbutamol		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Tenosynovitis			k.A.	

Primary source

Qualification: Pharmacist observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13336305

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Male Birth: privacy Age: 79 Year weight (kg): 76 BMI_int: 22.94 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pruritus	20131015			2 Day	recovering/resolving
2 Papuloerythematous drug eruption	20131015			2 Day	recovering/resolving
3 Reduced general condition	20131015			2 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 solaraze diclofenac sodium	Topical	/	20131014- -	Actinic keratosis

Drugs (concomitant)

2 azathioprin azathioprine	Oral	50 Mg milligram(s)/Day	- -	Crohn's
3 kreon 40.000 pancreatin	Oral	/	- -	
4 pantoprazol pantoprazole	Oral	20 Mg milligram(s)/Day	- -	Intestinal resection

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Crohn's disease			Unknown	schwerer M. Crohn mit mass. Gangrän und großräumiger OP 2001; wohl auch Pankreasteilresektion;
Primary sclerosing cholangitis	201309		Unknown	Vd. primär sklerosierende Cholangitis gestellt (DHC-Stenose, evtl. Stenose re. Gallengang)
Anaemia macrocytic	201309		Unknown	makrozytäre Anämie mit Ausschluß Systemerkrankung durch Knochenmarksbiopsie

Primary source

Qualification: Physician observestudytype:
Literature:

Narratives (nur bei elektronischem Eingang)

Drug Commission of the German Medical Association, 23.10.2013 DE-DCGMA-13161763

Sender diagnosis: not reported

Narrative

Sender's comment:

Reporter's comment:

12.09.-20.09.2013 Stationär wegen GI-Blutung; dabei umfangreiche Untersuchungen/Vorbereitungen mit nicht ru rekapitulierender Begleitmedikation. Dabei KEINE Beschwerden.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13362156

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 62 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin disorder					not recovered/not resolved
2 Wound weeping	20131002				recovering/resolving
3 Itch	20131002				recovering/resolving
4 Redness	20131002				recovering/resolving
5 Pustule	20131002				recovering/resolving
6 Skin necrosis					not recovered/not resolved
7 Skin inflammation					not recovered/not resolved
8 Incorrect dose administered					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel	Topical	/Month	2006-	Pain
diclofenac diethylamine		95	-	
2 voltaren emulgel		/	-	Off label use
diclofenac diethylamine		95	-	

Drugs (concomitant)

3 chamomile	Unknown	/	-
chamomile			-
4 diclofenac	Unknown	/	-
diclofenac			-
5 METFORMIN ATID	Unknown	1000 Mg milligram(s)/	-
metformin hydrochloride			-
6 ONBREZ BREEZHALER	Respiratory (inhalation)	150 µg microgram(s)/	-
indacaterol maleate			-
7 pantoprazole	Unknown	40 Mg milligram(s)/	-
pantoprazole			-
8 SIMVASTATIN 1A PHARMA	Unknown	/	-
simvastatin			-
9 TORASEMID HEXAL	Unknown	20 Mg milligram(s)/	-
torasemide			-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Environmental allergy			Unknown	Allergic reaction to "perfume composition".
Diabetes mellitus			Unknown	
Overweight			Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 15.11.2013 DE-002147023-CHNY2013DE002460

Sender diagnosis: not reported

Narrative

Initial report from a pharmacist received on 04 Nov 2013. This male patient (born 1951) used Voltaren Emulgel (diclofenac diethylamine salt) to treat 'severe pain' and developed 'pitting at skin of lower leg, necrotic and inflamed skin alterations (at the application site, respectively) like in case of open leg or not healing lower leg ulcer' after topical application. Therapy dates were specified as 'since approximately 2006 to not reported.' Daily dose was specified as 'mean dose of 2 to 6x 100g per month'. The patient was going to consult the treating physician. The reporter added that a recently conducted allergy test resulted in allergy to perfume composition and that Voltaren Emulgel was also applied on the open skin areas. Outcome: reaction continued. Follow up information requested in written form in week 46, 2013.

Sender's comment:

Dermatitis has been reported following use of Voltaren Emulgel (CDS-April 2012) and is considered suspect. Due to missing biological plausibility (events are unlisted) and lack of information regarding underlying medical condition(s) (e.g. diabetes), the events Skin disorder and Skin necrosis are considered as not assessable.

Reporter's comment:

NOVARTIS, 21.01.2014 DE-002147023-CHNY2013DE002460

Sender diagnosis: not reported

Narrative

Initial report from a pharmacist received on 04 Nov 2013. This male patient (born 1951) used Voltaren Emulgel (diclofenac diethylamine salt) to treat 'severe pain' and developed 'pitting at skin of lower leg, necrotic and inflamed skin alterations (at the application site, respectively) like in case of open leg or not healing lower leg ulcer' after topical application. Therapy dates were specified as 'since approximately 2006 to not reported.' Daily dose was specified as 'mean dose of 2 to 6x 100g per month'. The patient was going to consult the treating physician. The reporter added that a recently conducted allergy test resulted in allergy to perfume composition and that Voltaren Emulgel was also applied on the open skin areas. Outcome: reaction continued. Follow up information requested in written form in week 46, 2013.

Follow up information from reporting pharmacist received on 07 Jan 2014. Updated: patient data, reaction onset, suspect drug information, concomitant drugs and history, measures taken, information related to suspected drug and adverse reaction, causality assessment by reporting pharmacist and outcome. This 62 year old male patient with a history of pain in limbs, artificial hip joint, diabetes mellitus type II, hypertension, overweight and several allergies used Voltaren Emulgel (diclofenac diethylamine salt) to treat joint pain/pain in the limbs starting therapy before 2006. The daily dose was described as: "application as needed, in winter 2 times daily, in summer with breaks or 1 daily application only". On about 02 Oct 2013, the patient developed "redness and pustules on the hip; 3 different, palm sized spots on the leg, 1 below the knee and 2 in the area of the ankles. The skin reddened, than itched, than weeping spots". Concomitantly, the patient took multiple medication of metformin atid 100mg (indication unknoww), kamillin extern rogugen soludtion (chamomile), diclofenac retard heumann tablets (100), onbrez breezhaler 150ug capsusles for inhalation (indacaterol), torasemid hexal 20mg, simvastatin 1A pharma, and pantropazol actavis 40mg (all medications indication unknown. Use of Voltaren Emulgel (diclofenac diethylamine salt) was stopped end of Oct 2013. The patient received "cortisone ointment" (betamethasone 0.1 %, gentamycin 1.0 %, basic creme to 30.0) to treat the adverse reactions. It was confirmed that the patient was allergic to "perfume composition" in addition to several other allergies (propolis cera, methylchloroisothiazolinone and methylisothiazolinone, sorbitan sesquioleate). Causality assessment of reporting pharmacist was probable. Outcome: reaction resolving.

Sender's comment:

Dermatitis has been reported following use of Voltaren Emulgel (CDS-April 2012) and is considered suspect. Patient medical history of diabetes mellitus could provide the alternative explanation for events Skin disorder and Skin necrosis. However, overall insufficient information is provided for proper causality assessment and the events Skin disorder and Skin necrosis are considered as not assessable.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13362107

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Papule					recovered/resolved
2 Pustule	201310				recovered/resolved
3 Redness	201310				recovered/resolved
4 Skin rash					recovered/resolved
5 Vesicles					recovered/resolved
6 Itchy	201310				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN SCHMERZGEL FORTE diclofenac diethylamine		/ 95	-	Pain in arm
2 VOLTAREN SCHMERZGEL FORTE diclofenac diethylamine	Topical	/ 95	20131029-20131030 2 Day	Contusion

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fall			Unknown	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)NOVARTIS, 15.11.2013 DE-002147023-CHNY2013DE002446

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from pharmacist on 04 Nov 2013. This male patient, born 1936, developed "red, itchy pustules on his arm" after singular application of Voltaren Schmerzgel forte (diclofenac diethylamine salt) in Oct 2013 for treatment of bruises on his upper arm and lower arm after fall. No further information available at date of this report. Outcome was unknown. Followup information requested in written form in week 45, 2013.

Follow up information received from pharmacist on 06 Nov 2013 which reassessed the case to medically significant. Updated: measures taken (treatment of adverse event by a physician). According to the reporting pharmacist the patient received prescription of a cortisone ointment NOS from a physician to treat the skin reactions caused by the use of Voltaren Schmerzgel forte (diclofenac diethylamine salt). Outcome: unknown. Follow up information requested in written form in week 45, 2013.

Sender's comment:

Erythema, Pruritus and Rash pustular have been reported following use of Voltaren. Due to the timely relationship between product application and the report of Erythema, Pruritus and Rash pustular, the events are considered as suspected; the causal role of the medication could not be ruled out.

Reporter's comment:

NOVARTIS, 28.11.2013 DE-002147023-CHNY2013DE002446

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from pharmacist on 04 Nov 2013. This male patient, born 1936, developed "red, itchy pustules on his arm" after singular application of Voltaren Schmerzgel forte (diclofenac diethylamine salt) in Oct 2013 for treatment of bruises on his upper arm and lower arm after fall. No further information available at date of this report. Outcome was unknown. Followup information requested in written form in week 45, 2013.

Follow up information received from pharmacist on 06 Nov 2013 which reassessed the case to medically significant. Updated: measures taken (treatment of adverse event by a physician). According to the reporting pharmacist the patient received prescription of a cortisone ointment NOS from a physician to treat the skin reactions caused by the use of Voltaren Schmerzgel forte (diclofenac diethylamine salt). Outcome: unknown. Follow up information requested in written form in week 45, 2013.

Follow up information was received from a pharmacist on 18 Nov 2013: Updated: kind of reaction, duration of reaction, reaction onset, indication for use, therapy dates, therapy duration until onset, batch number, information related to suspected drug and adverse reaction, causality rating and readministration of drug. This male patient (born 1936) developed 'severely itchy skin rash, skin redness, papules, and open bursted vesicles' a few hours after topical application of Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch no. N03041A) used after 'fall with contusion and pain in arm'. Therapy dates were specified as '29 Oct 2013 to 30 Oct 2013'. The reporter specified the onset date as 'approx. 30 or 31 Oct 2013'. Voltaren Schmerzgel forte was stopped. Following drug treatment with 'Betagalen Creme' (betamethasone) adverse symptoms lasted all together for several days. Causality rating of reporter was probable. The reporter ticked the box 'Outcome: unknown'. Due to the stated limited duration of the reactions the outcome was internally rated as 'recovery'.

Sender's comment:

Erythema, Pruritus and Rash pustular have been reported following use of Voltaren. Due to the timely relationship between product application and the report of Erythema, Pruritus and Rash pustular, the events are considered as suspected; the causal role of the medication could not be ruled out. 26 Nov 2013: Assessment remains unchanged from the follow-up information received.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13363802

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 62 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Itch	20131002				recovering/resolving
2 Wound weeping	20131002				recovering/resolving
3 Redness	20131002				recovering/resolving
4 Pustule	20131002				recovering/resolving
5 Skin disorder					recovering/resolving
6 Skin necrosis					recovering/resolving
7 Skin inflammation					recovering/resolving
8 Incorrect dose administered					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/ 95	2006-201310 -	Off label use
2 voltaren emulgel diclofenac diethylamine	Topical	/Month 95	2006-201310 -	Pain

Drugs (concomitant)

3 CHAMOMILE chamomile	Unknown	/	-	-
4 DICLOFENAC - SLOW RELEASE "HEUMANN" diclofenac	Unknown	/	-	-
5 METFORMIN ATID metformin hydrochloride	Unknown	1000 Mg milligram(s)/	-	-
6 ONBREZ BREEZHALER indacaterol maleate	Respiratory (inhalation)	150 µg microgram(s)/	-	-
7 PANTOPRAZOLE ACTAVIS pantoprazole	Unknown	40 Mg milligram(s)/	-	-
8 SIMVASTATIN 1A PHARMA simvastatin	Unknown	/	-	-
9 TORASEMID HEXAL torasemide	Unknown	20 Mg milligram(s)/	-	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Environmental allergy			Yes	Allergic reaction to "perfume composition", PROPOLIS CERA, METHYLCHLOROISOTHIAZOLINONE AND METHYLISO
Hypertension			Yes	
Hip replacement			Yes	
Allergy			Yes	Severe allergies
Overweight			Yes	
Type 2 diabetes mellitus			Yes	

Primary source

Qualification:	Pharmacist	observestudytype:
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Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 15.11.2013 DE-NOVNCHHQ-CHNY2013DE002460

Sender diagnosis: not reported

Narrative

Initial report from a pharmacist received on 04 Nov 2013. This male patient (born 1951) used Voltaren Emulgel (diclofenac diethylamine salt) to treat 'severe pain' and developed 'pitting at skin of lower leg, necrotic and inflamed skin alterations (at the application site, respectively) like in case of open leg or not healing lower leg ulcer' after topical application. Therapy dates were specified as 'since approximately 2006 to not reported.' Daily dose was specified as 'mean dose of 2 to 6x 100g per month'. The patient was going to consult the treating physician. The reporter added that a recently conducted allergy test resulted in allergy to perfume composition and that Voltaren Emulgel was also applied on the open skin areas. Outcome: reaction continued. Follow up information requested in written form in week 46, 2013.

Sender's comment:

Dermatitis has been reported following use of Voltaren Emulgel (CDS-April 2012) and is considered suspect. Due to missing biological plausibility (events are unlisted) and lack of information regarding underlying medical condition(s) (e.g. diabetes), the events Skin disorder and Skin necrosis are considered as not assessable.

Reporter's comment:NOVARTIS, 20.01.2014 DE-NOVNCHHQ-CHNY2013DE002460

Sender diagnosis: not reported

Narrative

Initial report from a pharmacist received on 04 Nov 2013. This male patient (born 1951) used Voltaren Emulgel (diclofenac diethylamine salt) to treat 'severe pain' and developed 'pitting at skin of lower leg, necrotic and inflamed skin alterations (at the application site, respectively) like in case of open leg or not healing lower leg ulcer' after topical application. Therapy dates were specified as 'since approximately 2006 to not reported.' Daily dose was specified as 'mean dose of 2 to 6x 100g per month'. The patient was going to consult the treating physician. The reporter added that a recently conducted allergy test resulted in allergy to perfume composition and that Voltaren Emulgel was also applied on the open skin areas. Outcome: reaction continued. Follow up information requested in written form in week 46, 2013.

Follow up information from reporting pharmacist received on 07 Jan 2014. Updated: patient data, reaction onset, suspect drug information, concomitant drugs and history, measures taken, information related to suspected drug and adverse reaction, causality assessment by reporting pharmacist and outcome. This 62 year old male patient with a history of pain in limbs, artificial hip joint, diabetes mellitus type II, hypertension, overweight and several allergies used Voltaren Emulgel (diclofenac diethylamine salt) to treat joint pain/pain in the limbs starting therapy before 2006. The daily dose was described as: "application as needed, in winter 2 times daily, in summer with breaks or 1 daily application only". On about 02 Oct 2013, the patient developed "redness and pustules on the hip; 3 different, palm sized spots on the leg, 1 below the knee and 2 in the area of the ankles. The skin reddened, than itched, than weeping spots". Concomitantly, the patient took multiple medication of metformin atid 100mg (indication unknoww), kamillin extern rogugen soludtion (chamomile), diclofenac retard heumann tablets (100), onbrez breezhaler 150ug capsusles for inhalation (indacaterol), torasemid hexal 20mg, simvastatin 1A pharma, and pantropazol actavis 40mg (all medications indication unknown. Use of Voltaren Emulgel (diclofenac diethylamine salt) was stopped end of Oct 2013. The patient received "cortisone ointment" (betamethasone 0.1 %, gentamycin 1.0 %, basic creme to 30.0) to treat the adverse reactions. It was confirmed that the patient was allergic to "perfume composition" in addition to several other allergies (propolis cera, methylchloroisothiazolinone and methylisothiazolinone, sorbitan sesquioleate). Causality assessment of reporting pharmacist was probable. Outcome: reaction resolving.

Sender's comment:

Dermatitis has been reported following use of Voltaren Emulgel (CDS-April 2012) and is considered suspect. Patient medical history of diabetes mellitus could provide the alternative explanation for events Skin disorder and Skin necrosis. However, overall insufficient information is provided for proper causality assessment and the events Skin disorder and Skin necrosis are considered as not assessable.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13363801

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Redness	201310				recovered/resolved
2 Pustule	201310				recovered/resolved
3 Papule					recovered/resolved
4 Skin rash					recovered/resolved
5 Vesicles					recovered/resolved
6 Itchy	201310				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN SCHMERZGEL FORTE diclofenac diethylamine		/ 95	-	Pain in arm
2 VOLTAREN SCHMERZGEL FORTE diclofenac diethylamine	Topical	/ 95	20131029-20131030 2 Day	Contusion

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Fall			Unknown	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 15.11.2013 DE-NOVNCHHQ-CHNY2013DE002446

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from pharmacist on 04 Nov 2013. This male patient, born 1936, developed "red, itchy pustules on his arm" after singular application of Voltaren Schmerzgel forte (diclofenac diethylamine salt) in Oct 2013 for treatment of bruises on his upper arm and lower arm after fall. No further information available at date of this report. Outcome was unknown. Followup information requested in written form in week 45, 2013.

Follow up information received from pharmacist on 06 Nov 2013 which reassessed the case to medically significant. Updated: measures taken (treatment of adverse event by a physician). According to the reporting pharmacist the patient received prescription of a cortisone ointment NOS from a physician to treat the skin reactions caused by the use of Voltaren Schmerzgel forte (diclofenac diethylamine salt). Outcome: unknown. Follow up information requested in written form in week 45, 2013.

Sender's comment:

Erythema, Pruritus and Rash pustular have been reported following use of Voltaren. Due to the timely relationship between product application and the report of Erythema, Pruritus and Rash pustular, the events are considered as suspected; the causal role of the medication could not be ruled out.

Reporter's comment:

NOVARTIS, 28.11.2013 DE-NOVNCHHQ-CHNY2013DE002446

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from pharmacist on 04 Nov 2013. This male patient, born 1936, developed "red, itchy pustules on his arm" after singular application of Voltaren Schmerzgel forte (diclofenac diethylamine salt) in Oct 2013 for treatment of bruises on his upper arm and lower arm after fall. No further information available at date of this report. Outcome was unknown. Followup information requested in written form in week 45, 2013.

Follow up information received from pharmacist on 06 Nov 2013 which reassessed the case to medically significant. Updated: measures taken (treatment of adverse event by a physician). According to the reporting pharmacist the patient received prescription of a cortisone ointment NOS from a physician to treat the skin reactions caused by the use of Voltaren Schmerzgel forte (diclofenac diethylamine salt). Outcome: unknown. Follow up information requested in written form in week 45, 2013.

Follow up information was received from a pharmacist on 18 Nov 2013: Updated: kind of reaction, duration of reaction, reaction onset, indication for use, therapy dates, therapy duration until onset, batch number, information related to suspected drug and adverse reaction, causality rating and readministration of drug. This male patient (born 1936) developed 'severely itchy skin rash, skin redness, papules, and open bursted vesicles' a few hours after topical application of Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch no. N03041A) used after 'fall with contusion and pain in arm'. Therapy dates were specified as '29 Oct 2013 to 30 Oct 2013'. The reporter specified the onset date as 'approx. 30 or 31 Oct 2013'. Voltaren Schmerzgel forte was stopped. Following drug treatment with 'Betagalen Creme' (betamethasone) adverse symptoms lasted all together for several days. Causality rating of reporter was probable. The reporter ticked the box 'Outcome: unknown'. Due to the stated limited duration of the reactions the outcome was internally rated as 'recovery'.

Sender's comment:

Erythema, Pruritus and Rash pustular have been reported following use of Voltaren. Due to the timely relationship between product application and the report of Erythema, Pruritus and Rash pustular, the events are considered as suspected; the causal role of the medication could not be ruled out. 26 Nov 2013: Assessment remains unchanged from the follow-up information received.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13382181

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Redness	20131124			1 Day	not recovered/not resolved
2 Rash	20131124			1 Day	not recovered/not resolved
3 Pustule	20131124			1 Day	not recovered/not resolved
4 Condition aggravated	20131125			2 Day	not recovered/not resolved
5 Blisters	20131124			1 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dolo	Oral	25 Mg milligram(s)/Day	20131124-20131126	Pain
diclofenac potassium		82	3 Day	

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 05.12.2013 DE-002147023-CHNY2013DE002744

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from patient on 26 Nov 2013. This female patient took one Voltaren Dolo Extra 25 mg (diclofenac potassium) on 24 Nov 2013 for treatment of pain and developed "rash with redness and pustules" on 24 Nov 2013. On 25 Nov 2013 the female patient took another Voltaren Dolo Extra 25 mg (diclofenac potassium) and "the symptoms worsened". The patient reported on "rash with water blisters at her back and chest, especially at the cardiac region". The drug was discontinued on 26 Nov 2013. No further information available at date of this report. Outcome: reaction continues. No contact details were provided and therefore follow up not feasible. Case was closed with lost to follow up on 29 Nov 2013.

Sender's comment:

The events Rash pustular, Blister, Rash, Erythema and Condition aggravated are considered suspect due to biological plausibility and timely relationship to Voltaren Dolo; the causal role of the medication could not be ruled out.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13383461

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Pustule	20131124			1 Day	not recovered/not resolved
2 Rash	20131124			1 Day	not recovered/not resolved
3 Redness	20131124			1 Day	not recovered/not resolved
4 Blisters	20131124			1 Day	not recovered/not resolved
5 Condition aggravated	20131125			2 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dolo	Oral	25 Mg milligram(s)/Day	20131124-20131126	Pain
diclofenac potassium		82	3 Day	

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 05.12.2013 DE-NOVNCHHQ-CHNY2013DE002744

Sender diagnosis: not reported

Narrative

Initial report with insufficient information received from patient on 26 Nov 2013. This female patient took one Voltaren Dolo Extra 25 mg (diclofenac potassium) on 24 Nov 2013 for treatment of pain and developed "rash with redness and pustules" on 24 Nov 2013. On 25 Nov 2013 the female patient took another Voltaren Dolo Extra 25 mg (diclofenac potassium) and "the symptoms worsened". The patient reported on "rash with water blisters at her back and chest, especially at the cardiac region". The drug was discontinued on 26 Nov 2013. No further information available at date of this report. Outcome: reaction continues. No contact details were provided and therefore follow up not feasible. Case was closed with lost to follow up on 29 Nov 2013.

Sender's comment:

The events Rash pustular, Blister, Rash, Erythema and Condition aggravated are considered suspect due to biological plausibility and timely relationship to Voltaren Dolo; the causal role of the medication could not be ruled out.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-13399278

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 70 Year	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Multiple organ failure
deathdate:	deathcause: General physical health deterioration

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin red					Unknown
2 Blister					Unknown
3 Burn					Unknown
4 Death sudden					fatal

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN SCHMERZGEL FORTE	Topical	/	20131120-20131120	Back pain
diclofenac diethylamine		95	1 Day	

Drugs (concomitant)

2 acyclovir	/	20131125-	Herpes infection
aciclovir	245	-	
3 allopurinol	/	-	
allopurinol		-	
4 bisoprolol	/	-	
bisoprolol		-	
5 levetiracetam	/	-	
levetiracetam		-	
6 metoclopramid	/	-	
metoclopramide hydrochloride		-	
7 novaminsulfon	/	-	
metamizole sodium	158	-	
8 oxycodon	/	-	
oxycodone	245	-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Sickness			Unknown	
Multiple organ failure			Unknown	
Herpes infection			Unknown	Possibly initial phase of herpes treated with Aciclovir tablets
General physical health deterioration			Unknown	
Exposure to radiation			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 23.12.2013 DE-002147023-CHNY2013DE002882

Sender diagnosis: not reported

Narrative

Case number CHNY2013DE002882, is an initial spontaneous report from a pharmacist via German Pharmacist's Drug Commission (number [182301]) received on 13 Dec 2013. This report refers to a female patient, born in 1943. Historical drug included Voltaren Schmerzgel (diclofenac diethylamine salt) that was well tolerated. Concurrent conditions included "possibly initial phase of herpes" and "no uv-irradiation at sunbed". Concomitant medication included Acyclovir (aciclovir). The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N04183A for the treatment of back pain from 12 Nov 2013 at an unknown dose. The patient reported on the second day of therapy, she "developed severe skin redness/ after start of topical application" (application site erythema), "developed signs of skin burn/ after start of topical application" (application site burn), "developed blisters/after start of topical application" (application site vesicles). Treatment with Voltaren Schmerzgel Forte was stopped on an unknown date. The reaction recurred after reintroduction and the previous reactions were described as "aggravated after reintroduction". At the time of this report, outcome of the events was reaction continues. The pharmacist assessed the severe skin redness, signs of skin burn, and blisters as suspected to be related to treatment with Voltaren Schmerzgel Forte. The causality was not provided. Follow up information requested in written form in week 51, 2013.

Sender's comment:

Various dermatological reactions have been reported with diclofenac. Due to the timely relationship between application of the product and the events of Application site burn, erythema, and vesicles, the events are considered as suspected; however, it should be also considered that the consumer's concurrent conditions, including "possibly initial phase of herpes" and "no uv-irradiation at sunbed", may provide alternative explanations. A positive rechallenge was reported; the causal role of the product could not be ruled out.

Reporter's comment:

NOVARTIS, 23.12.2013 DE-NOVNCHHQ-CHNY2013DE002882

Sender diagnosis: not reported

Narrative

Case number CHNY2013DE002882, is an initial spontaneous report from a pharmacist via German Pharmacist's Drug Commission (number [182301]) received on 13 Dec 2013. This report refers to a female patient, born in 1943. Historical drug included Voltaren Schmerzgel (diclofenac diethylamine salt) that was well tolerated. Concurrent conditions included "possibly initial phase of herpes" and "no uv-irradiation at sunbed". Concomitant medication included Acyclovir (aciclovir). The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N04183A for the treatment of back pain from 12 Nov 2013 at an unknown dose. The patient reported on the second day of therapy, she "developed severe skin redness/ after start of topical application" (application site erythema), "developed signs of skin burn/ after start of topical application" (application site burn), "developed blisters/after start of topical application" (application site vesicles). Treatment with Voltaren Schmerzgel Forte was stopped on an unknown date. The reaction recurred after reintroduction and the previous reactions were described as "aggravated after reintroduction". At the time of this report, outcome of the events was reaction continues. The pharmacist assessed the severe skin redness, signs of skin burn, and blisters as suspected to be related to treatment with Voltaren Schmerzgel Forte. The causality was not provided. Follow up information requested in written form in week 51, 2013.

Sender's comment:

Various dermatological reactions have been reported with diclofenac. Due to the timely relationship between application of the product and the events of Application site burn, erythema, and vesicles, the events are considered as suspected; however, it should be also considered that the consumer's concurrent conditions, including "possibly initial phase of herpes" and "no uv-irradiation at sunbed", may provide alternative explanations. A positive rechallenge was reported; the causal role of the product could not be ruled out.

Reporter's comment:

NOVARTIS, 03.01.2014 DE-NOVNCHHQ-CHNY2013DE002882

Sender diagnosis: not reported

Narrative

Case number CHNY2013DE002882, is an initial spontaneous report from a pharmacist via German Pharmacist's Drug Commission (number [182301]) received on 13 Dec 2013. This report refers to a female patient, born in 1943. Historical drug included Voltaren Schmerzgel (diclofenac diethylamine salt) that was well tolerated. Concurrent conditions included "possibly initial phase of herpes" and "no uv-irradiation at sunbed". Concomitant medication included Acyclovir (aciclovir). The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N04183A for the treatment of back pain from 12 Nov 2013 at an unknown dose. The patient reported on the second day of therapy, she "developed severe skin redness/ after start of topical application" (application site erythema), "developed signs of skin burn/ after start of topical application" (application site burn), "developed blisters/after start of topical application" (application site vesicles). Treatment with Voltaren Schmerzgel Forte was stopped on an unknown date. The reaction recurred after reintroduction and the previous reactions were described as "aggravated after reintroduction". At the time of this report, outcome of the events was reaction continues. The pharmacist assessed the severe skin redness, signs of skin burn, and blisters as suspected to be related to treatment with Voltaren Schmerzgel Forte. The causality was not provided. Follow up information requested in written form in week 51, 2013.

Follow-up information was received from a pharmacist via German Pharmacist's Drug Commission (number [182301]) on 20 Dec 2013. Updated: kind of reaction, therapy dates, therapy duration until onset, concomitant drugs and outcome. This female patient developed 'extreme skin erythema, signs of skin burn, and blisters at a skin area of 50x50cm up to 60x60cm at her hip and back region down to her upper leg' after first and single use of Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch-no. N04183A) used to treat back pain. Therapy dates were specified as '20 Nov 2013 to 20 Nov 2013'. Voltaren Schmerzgel forte was stopped immediately due to the reactions. Other relevant medical history included no uv-irradiation at sunbed; possibly initial phase of herpes treated with Aciclovir tablets starting from 25 Nov 2013. The pharmacist provided 3 pictures showing the skin reaction. Outcome: not reported.

Sender's comment:

Various dermatological reactions have been reported with diclofenac. Due to the timely relationship between application of the product and the events of Application site burn, erythema, and vesicles, the events are considered as suspected; however, it should be also considered that the consumer's concurrent conditions, including "possibly initial phase of herpes" and "no uv-irradiation at sunbed", may provide alternative explanations. A positive rechallenge was reported; the causal role of the product could not be ruled out.

Reporter's comment:

NOVARTIS, 03.01.2014 DE-002147023-CHNY2013DE002882

Sender diagnosis: not reported

Narrative

Case number CHNY2013DE002882, is an initial spontaneous report from a pharmacist via German Pharmacist's Drug Commission (number [182301]) received on 13 Dec 2013. This report refers to a female patient, born in 1943. Historical drug included Voltaren Schmerzgel (diclofenac diethylamine salt) that was well tolerated. Concurrent conditions included "possibly initial phase of herpes" and "no uv-irradiation at sunbed". Concomitant medication included Acyclovir (aciclovir). The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N04183A for the treatment of back pain from 12 Nov 2013 at an unknown dose. The patient reported on the second day of therapy, she "developed severe skin redness/ after start of topical application" (application site erythema), "developed signs of skin burn/ after start of topical application" (application site burn), "developed blisters/after start of topical application" (application site vesicles). Treatment with Voltaren Schmerzgel Forte was stopped on an unknown date. The reaction recurred after reintroduction and the previous reactions were described as "aggravated after reintroduction". At the time of this report, outcome of the events was reaction continues. The pharmacist assessed the severe skin redness, signs of skin burn, and blisters as suspected to be related to treatment with Voltaren Schmerzgel Forte. The causality was not provided. Follow up information requested in written form in week 51, 2013.

Follow-up information was received from a pharmacist via German Pharmacist's Drug Commission (number [182301]) on 20 Dec 2013. Updated: kind of reaction, therapy dates, therapy duration until onset, concomitant drugs and outcome. This female patient developed 'extreme skin erythema, signs of skin burn, and blisters at a skin area of 50x50cm up to 60x60cm at her hip and back region down to her upper leg' after first and single use of Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch-no. N04183A) used to treat back pain. Therapy dates were specified as '20 Nov 2013 to 20 Nov 2013'. Voltaren Schmerzgel forte was stopped immediately due to the reactions. Other relevant medical history included no uv-irradiation at sunbed; possibly initial phase of herpes treated with Aciclovir tablets starting from 25 Nov 2013. The pharmacist provided 3 pictures showing the skin reaction. Outcome: not reported.

Sender's comment:

Various dermatological reactions have been reported with diclofenac. Due to the timely relationship between application of the product and the events

of Application site burn, erythema, and vesicles, the events are considered as suspected; however, it should be also considered that the consumer's concurrent conditions, including "possibly initial phase of herpes" and "no uv-irradiation at sunbed", may provide alternative explanations. A positive rechallenge was reported; the causal role of the product could not be ruled out.

Reporter's comment:

NOVARTIS, 13.02.2014 DE-002147023-CHNY2013DE002882

Sender diagnosis: not reported

Narrative

Case number CHNY2013DE002882, is an initial spontaneous report from a pharmacist via German Pharmacist's Drug Commission (number [182301]) received on 13 Dec 2013. This report refers to a female patient, born in 1943. Historical drug included Voltaren Schmerzgel (diclofenac diethylamine salt) that was well tolerated. Concurrent conditions included "possibly initial phase of herpes" and "no uv-irradiation at sunbed". Concomitant medication included Acyclovir (aciclovir). The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N04183A for the treatment of back pain from 12 Nov 2013 at an unknown dose. The patient reported on the second day of therapy, she "developed severe skin redness/ after start of topical application" (application site erythema), "developed signs of skin burn/ after start of topical application" (application site burn), "developed blisters/after start of topical application" (application site vesicles). Treatment with Voltaren Schmerzgel Forte was stopped on an unknown date. The reaction recurred after reintroduction and the previous reactions were described as "aggravated after reintroduction". At the time of this report, outcome of the events was reaction continues. The pharmacist assessed the severe skin redness, signs of skin burn, and blisters as suspected to be related to treatment with Voltaren Schmerzgel Forte. The causality was not provided. Follow up information requested in written form in week 51, 2013.

Follow-up information was received from a pharmacist via German Pharmacist's Drug Commission (number [182301]) on 20 Dec 2013. Updated: kind of reaction, therapy dates, therapy duration until onset, concomitant drugs and outcome. This female patient developed 'extreme skin erythema, signs of skin burn, and blisters at a skin area of 50x50cm up to 60x60cm at her hip and back region down to her upper leg' after first and single use of Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch-no. N04183A) used to treat back pain. Therapy dates were specified as '20 Nov 2013 to 20 Nov 2013'. Voltaren Schmerzgel forte was stopped immediately due to the reactions. Other relevant medical history included no uv-irradiation at sunbed; possibly initial phase of herpes treated with Aciclovir tablets starting from 25 Nov 2013. The pharmacist provided 3 pictures showing the skin reaction. Outcome: not reported. No additional information is expected.

Follow-up information was received from pharmacist on 10 Feb 2014 via follow-up letter and follow-up call. Updated: CIOMS criteria, other relevant history, outcome. According to follow-up information the reporting pharmacist returned an empty questionnaire and informed us that this patient had died on not reported date. The pharmacist reported via phone call that this patient "had been sick for longer". Reportedly, this patient died of "a worsening of the general state of health, probably multiple organ failure". The pharmacist did not see any causal relationship between Voltaren Schmerzgel forte (diclofenac diethylamine salt) and the patient's death. According to the pharmacist the patient had "discontinued the drug quite a while ago". Outcome: death - autopsy performed unknown.

Sender's comment:

Various dermatological reactions have been reported with diclofenac. Due to the timely relationship between application of the product and the events of Application site burn, erythema, and vesicles, the events are considered as suspected; however, it should be also considered that the consumer's concurrent conditions, including "possibly initial phase of herpes" and "no uv-irradiation at sunbed", may provide alternative explanations. A positive rechallenge was reported; the causal role of the product could not be ruled out.

Additional event reported: Death is not assessable. The lack of information did not allow the meaningful causality assessment for this event.

Reporter's comment:

NOVARTIS, 13.02.2014 DE-NOVNCHHQ-CHNY2013DE002882

Sender diagnosis: not reported

Narrative

Case number CHNY2013DE002882, is an initial spontaneous report from a pharmacist via German Pharmacist's Drug Commission (number [182301]) received on 13 Dec 2013. This report refers to a female patient, born in 1943. Historical drug included Voltaren Schmerzgel (diclofenac diethylamine salt) that was well tolerated. Concurrent conditions included "possibly initial phase of herpes" and "no uv-irradiation at sunbed". Concomitant medication included Acyclovir (aciclovir). The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N04183A for the treatment of back pain from 12 Nov 2013 at an unknown dose. The patient reported on the second day of therapy, she "developed severe skin redness/ after start of topical application" (application site erythema), "developed signs of skin burn/ after start of topical application" (application site burn), "developed blisters/after start of topical application" (application site vesicles). Treatment with Voltaren Schmerzgel Forte was stopped on an unknown date. The reaction recurred after reintroduction and the previous reactions were described as "aggravated after reintroduction". At the time of this report, outcome of the events was reaction continues. The pharmacist assessed the severe skin redness, signs of skin burn, and blisters as suspected to be related to treatment with Voltaren Schmerzgel Forte. The causality was not provided. Follow up information requested in written form in week 51, 2013.

Follow-up information was received from a pharmacist via German Pharmacist's Drug Commission (number [182301]) on 20 Dec 2013. Updated: kind of reaction, therapy dates, therapy duration until onset, concomitant drugs and outcome. This female patient developed 'extreme skin erythema, signs of skin burn, and blisters at a skin area of 50x50cm up to 60x60cm at her hip and back region down to her upper leg' after first and single use of Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch-no. N04183A) used to treat back pain. Therapy dates were specified as '20 Nov 2013 to 20 Nov 2013'. Voltaren Schmerzgel forte was stopped immediately due to the reactions. Other relevant medical history included no uv-irradiation at sunbed; possibly initial phase of herpes treated with Aciclovir tablets starting from 25 Nov 2013. The pharmacist provided 3 pictures showing the skin reaction. Outcome: not reported. No additional information is expected.

Follow-up information was received from pharmacist on 10 Feb 2014 via follow-up letter and follow-up call. Updated: CIOMS criteria, other relevant history, outcome. According to follow-up information the reporting pharmacist returned an empty questionnaire and informed us that this patient had died on not reported date. The pharmacist reported via phone call that this patient "had been sick for longer". Reportedly, this patient died of "a worsening of the general state of health, probably multiple organ failure". The pharmacist did not see any causal relationship between Voltaren Schmerzgel forte (diclofenac diethylamine salt) and the patient's death. According to the pharmacist the patient had "discontinued the drug quite a while ago". Outcome: death - autopsy performed unknown.

Sender's comment:

Various dermatological reactions have been reported with diclofenac. Due to the timely relationship between application of the product and the events of Application site burn, erythema, and vesicles, the events are considered as suspected; however, it should be also considered that the consumer's concurrent conditions, including "possibly initial phase of herpes" and "no uv-irradiation at sunbed", may provide alternative explanations. A positive rechallenge was reported; the causal role of the product could not be ruled out.

Additional event reported: Death is not assessable. The lack of information did not allow the meaningful causality assessment for this event.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14036341

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Elevated liver enzyme levels					recovering/resolving
2 Skin peeling					recovering/resolving
3 Blood magnesium abnormal					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren dolo diclofenac potassium	Oral	50 Mg milligram(s)/ 82	20140109-20140123 15 Day	Back pain

Primary source

Qualification: Consumer or other non health professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 07.02.2014 DE-NOVNCHHQ-CHNY2014DE000644

Sender diagnosis: not reported

Narrative

Case number CHNY2014DE000644, is an initial spontaneous report from a consumer received on 28 Jan 2014. This report refers to a female patient, born on an unknown date in 1967. Historical conditions were not reported. No concomitant medication was reported. The patient received Voltaren Dolo (diclofenac potassium) film-coated tablet at 50 mg (2 tablets at lunch time, 50mg) for the treatment of back pain from 09 Jan 2014 at 50 mg for the duration of 15 days. On an unknown onset date after the first dose of the suspect drug the patient reported "elevated liver parameters" (hepatic enzyme increased), "skin peeling palm of hands, blister like in case of burn blister" (which opened and the skin could be peeled off like old rags from the palm of her hands with new skin below the peeled off skin (no raw flesh below the blisters), (skin exfoliation) and "blood magnesium <0.1" (blood magnesium abnormal). The following significant lab tests were recorded: hepatic enzyme increased on an unknown date was unknown result (elevated). The following significant lab tests were recorded: blood magnesium on an unknown date was <0.1. At the time of this report, outcome of the event skin peeling palm of hands, blister like in case of burn blister (skin peeling), blood magnesium < 0.1 (blood magnesium abnormal) and elevated liver parameters (elevated liver enzyme levels) was reported as condition improved. Follow up information requested in written form in week 6, 2014.

Sender's comment:

Medical assessment comment: Based on the available information, the events of skin peeling, low magnesium levels and hepatic enzyme levels increased are currently not assessable. There is no information on the patient's age, any relevant medical history or concomitant medication.

Reporter's comment:NOVARTIS, 10.02.2014 DE-002147023-CHNY2014DE000644

Sender diagnosis: not reported

Narrative

Case number CHNY2014DE000644, is an initial spontaneous report from a consumer received on 28 Jan 2014. This report refers to a female patient,

born on an unknown date in 1967. Historical conditions were not reported. No concomitant medication was reported. The patient received Voltaren Dolo (diclofenac potassium) film-coated tablet at 50 mg (2 tablets at lunch time, 50mg) for the treatment of back pain from 09 Jan 2014 at 50 mg for the duration of 15 days. On an unknown onset date after the first dose of the suspect drug the patient reported "elevated liver parameters" (hepatic enzyme increased), "skin peeling palm of hands, blister like in case of burn blister" (which opened and the skin could be peeled off like old rags from the palm of her hands with new skin below the peeled off skin (no raw flesh below the blisters), (skin exfoliation) and "blood magnesium <0.1" (blood magnesium abnormal). The following significant lab tests were recorded: hepatic enzyme increased on an unknown date was unknown result (elevated). The following significant lab tests were recorded: blood magnesium on an unknown date was <0.1. At the time of this report, outcome of the event skin peeling palm of hands, blister like in case of burn blister (skin peeling), blood magnesium < 0.1 (blood magnesium abnormal) and elevated liver parameters (elevated liver enzyme levels) was reported as condition improved. Follow up information requested in written form in week 6, 2014.

Sender's comment:

Medical assessment comment: Based on the available information, the events of skin peeling, low magnesium levels and hepatic enzyme levels increased are currently not assessable. There is no information on the patient's age, any relevant medical history or concomitant medication.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14068788

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No		No	No	No				

Patient

Initials:	Sex: Male	Birth: privacy	Age: 82 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin inflammation	20140207			12 Day	not recovered/not resolved
2 Skin scaly	20140207			12 Day	not recovered/not resolved
3 Rash both legs	20140207			12 Day	not recovered/not resolved
4 Dry skin	20140207			12 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Voltaren Emulgel diclofenac diethylamine	Cutaneous	/	20140127-20140209	Knee pain
			14 Day	

Drugs (concomitant)

2 enalapril enalapril	Oral	/	-	Hypertension
			-	

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14057452

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin peeling	201401				recovered/resolved
2 Skin rash	201401				recovered/resolved
3 Wound weeping	201401				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/ 95	20140124-20140202 10 Day	Shoulder pain

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 25.02.2014 DE-NOVNCHHQ-CHPA2014DE002865

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE002865, is an initial spontaneous report from a pharmacist received on 17 Feb 2014 via Drug commission of German Pharmacists (number [183930]). This report refers to a female patient, either 80 or 81 year old. Historical conditions were not reported. No concomitant medication was reported. The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N05074A) for the treatment of musculoskeletal pain (pain at shoulder) from 24 Jan 2014 at an unknown dose, topically applied 1 to 2 times daily, for the duration of 10 days. On an approximately 29 Jan 2014, the patient reported "weeping of wound" (wound secretion), "skin rash" (rash) and "partly skin peeling of first skin layer" (skin exfoliation). Treatment with Voltaren Schmerzgel Forte was stopped on 02 Feb 2014. Following wound dressing and treatment with Bepanthen ointment the events were reported as improving. The reactions were rated by reporter as transiently significantly impairing. At the time of this report, outcome of the event skin rash (rash), partly skin peeling of first skin layer (skin peeling) and weeping of wound (wound weeping) was reported as condition improved. Seriousness and causality were not provided. No additional information was provided. Follow up information was requested in written form in week 9, 2014.

Sender's comment:

The event Rash is listed in current RSI. The remaining two events are not listed in current RSI. The causal role of the medication in events Wound secretion, Skin exfoliation and Rash could not be ruled out due to temporal plausibility and pharmacological properties of the Voltaren Gel.

Reporter's comment:NOVARTIS, 04.03.2014 DE-NOVNCHHQ-CHPA2014DE002865

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE002865, is an initial spontaneous report from a pharmacist received on 17 Feb 2014 via Drug commission of German Pharmacists (number [183930]). This report refers to a female patient, either 80 or 81 year old. Historical conditions were not reported. No

concomitant medication was reported. The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N05074A) for the treatment of musculoskeletal pain (pain at shoulder) from 24 Jan 2014 at an unknown dose, topically applied 1 to 2 times daily, for the duration of 10 days. On an approximately 29 Jan 2014, the patient reported "weeping of wound" (wound secretion), "skin rash" (rash) and "partly skin peeling of first skin layer" (skin exfoliation). Treatment with Voltaren Schmerzgel Forte was stopped on 02 Feb 2014. Following wound dressing and treatment with Bepanthen ointment the events were reported as improving. The reactions were rated by reporter as transiently significantly impairing. At the time of this report, outcome of the event skin rash (rash), partly skin peeling of first skin layer (skin peeling) and weeping of wound (wound weeping) was reported as condition improved. Seriousness and causality were not provided. No additional information was provided. Follow up information was requested in written form in week 9, 2014.

Follow up information was received from a pharmacist on 25 FEB 2014. This female patient developed 'skin rash with partly skin peeling of first skin layer and weeping of wound' after topical application of Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch no. N05074A) used to treat 'pain at shoulder'. Therapy dates were specified as 'approx 24 Jan 2014 to approx 02 Feb 2014' (continuous treatment). Voltaren Schmerzgel forte was stopped. Following drug treatment with wound dressing for weeping wound with subsequent treatment with Bepanthen ointment adverse symptoms lasted all together for 2 to 3 weeks. The reporter ticked the box that the reaction was not treated by a physician. Causality rating of reporter was probable. The reporter ticked the box Outcome as recovery.

Sender's comment:

The event Rash is listed in current RSI. The remaining two events are not listed in current RSI. The causal role of the medication in events Wound secretion, Skin exfoliation and Rash could not be ruled out due to temporal plausibility and pharmacological properties of the Voltaren Gel.

03 Mar 2014: Assessment remains unchanged from the follow-up information received.

Reporter's comment:

NOVARTIS, 11.04.2014 DE-NOVNCHHQ-CHPA2014DE002865

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE002865, is an initial spontaneous report from a pharmacist received on 17 Feb 2014 via Drug commission of German Pharmacists (number [183930]). This report refers to a female patient, either 80 or 81 year old. Historical conditions were not reported. No concomitant medication was reported. The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N05074A) for the treatment of musculoskeletal pain (pain at shoulder) from 24 Jan 2014 at an unknown dose, topically applied 1 to 2 times daily, for the duration of 10 days. On an approximately 29 Jan 2014, the patient reported "weeping of wound" (wound secretion), "skin rash" (rash) and "partly skin peeling of first skin layer" (skin exfoliation). Treatment with Voltaren Schmerzgel Forte was stopped on 02 Feb 2014. Following wound dressing and treatment with Bepanthen ointment the events were reported as improving. The reactions were rated by reporter as transiently significantly impairing. At the time of this report, outcome of the event skin rash (rash), partly skin peeling of first skin layer (skin peeling) and weeping of wound (wound weeping) was reported as condition improved. Seriousness and causality were not provided. No additional information was provided. Follow up information was requested in written form in week 9, 2014.

Follow up information was received from a pharmacist on 25 FEB 2014. This female patient developed 'skin rash with partly skin peeling of first skin layer and weeping of wound' after topical application of Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch no. N05074A) used to treat 'pain at shoulder'. Therapy dates were specified as 'approx 24 Jan 2014 to approx 02 Feb 2014' (continuous treatment). Voltaren Schmerzgel forte was stopped. Following drug treatment with wound dressing for weeping wound with subsequent treatment with Bepanthen ointment adverse symptoms lasted all together for 2 to 3 weeks. The reporter ticked the box that the reaction was not treated by a physician. Causality rating of reporter was probable. The reporter ticked the box Outcome as recovery.

Significant follow up report received on 09 Apr 2014 which consisted of technical complaint investigation results (11159999): No defect observed during bulk batch manufacturing or packaging process. At release, analysis were conform and retest done on retention samples are also compliant. The case is not confirmable from a technical point of view. However, a trend is detected for this issue on bulk 145275 and currently under investigation: PR 130392: COMP Trend analysis on ADR Voltaren 2 percent. Case will be investigated by DSP. No additional information was available.

Sender's comment:

The event Rash is listed in current RSI. The remaining two events are not listed in current RSI. The causal role of the medication in events Wound secretion, Skin exfoliation and Rash could not be ruled out due to temporal plausibility and pharmacological properties of the Voltaren Gel.

03 Mar 2014: Assessment remains unchanged from the follow-up information received.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14059004

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin peeling	201401				recovered/resolved
2 Skin rash	201401				recovered/resolved
3 Wound weeping	201401				recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/ 95	20140124-20140202	Shoulder pain
			10 Day	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)NOVARTIS, 27.02.2014 DE-002147023-CHPA2014DE002865

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE002865, is an initial spontaneous report from a pharmacist received on 17 Feb 2014 via Drug commission of German Pharmacists (number [183930]). This report refers to a female patient, either 80 or 81 year old. Historical conditions were not reported. No concomitant medication was reported. The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N05074A) for the treatment of musculoskeletal pain (pain at shoulder) from 24 Jan 2014 at an unknown dose, topically applied 1 to 2 times daily, for the duration of 10 days. On an approximately 29 Jan 2014, the patient reported "weeping of wound" (wound secretion), "skin rash" (rash) and "partly skin peeling of first skin layer" (skin exfoliation). Treatment with Voltaren Schmerzgel Forte was stopped on 02 Feb 2014. Following wound dressing and treatment with Bepanthen ointment the events were reported as improving. The reactions were rated by reporter as transiently significantly impairing. At the time of this report, outcome of the event skin rash (rash), partly skin peeling of first skin layer (skin peeling) and weeping of wound (wound weeping) was reported as condition improved. Seriousness and causality were not provided. No additional information was provided. Follow up information was requested in written form in week 9, 2014.

Sender's comment:

The event Rash is listed in current RSI. The remaining two events are not listed in current RSI. The causal role of the medication in events Wound secretion, Skin exfoliation and Rash could not be ruled out due to temporal plausibility and pharmacological properties of the Voltaren Gel.

Reporter's comment:NOVARTIS, 04.03.2014 DE-002147023-CHPA2014DE002865

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE002865, is an initial spontaneous report from a pharmacist received on 17 Feb 2014 via Drug commission of German Pharmacists (number [183930]). This report refers to a female patient, either 80 or 81 year old. Historical conditions were not reported. No

concomitant medication was reported. The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N05074A) for the treatment of musculoskeletal pain (pain at shoulder) from 24 Jan 2014 at an unknown dose, topically applied 1 to 2 times daily, for the duration of 10 days. On an approximately 29 Jan 2014, the patient reported "weeping of wound" (wound secretion), "skin rash" (rash) and "partly skin peeling of first skin layer" (skin exfoliation). Treatment with Voltaren Schmerzgel Forte was stopped on 02 Feb 2014. Following wound dressing and treatment with Bepanthen ointment the events were reported as improving. The reactions were rated by reporter as transiently significantly impairing. At the time of this report, outcome of the event skin rash (rash), partly skin peeling of first skin layer (skin peeling) and weeping of wound (wound weeping) was reported as condition improved. Seriousness and causality were not provided. No additional information was provided. Follow up information was requested in written form in week 9, 2014.

Follow up information was received from a pharmacist on 25 FEB 2014. This female patient developed 'skin rash with partly skin peeling of first skin layer and weeping of wound' after topical application of Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch no. N05074A) used to treat 'pain at shoulder'. Therapy dates were specified as 'approx 24 Jan 2014 to approx 02 Feb 2014' (continuous treatment). Voltaren Schmerzgel forte was stopped. Following drug treatment with wound dressing for weeping wound with subsequent treatment with Bepanthen ointment adverse symptoms lasted all together for 2 to 3 weeks. The reporter ticked the box that the reaction was not treated by a physician. Causality rating of reporter was probable. The reporter ticked the box Outcome as recovery.

Sender's comment:

The event Rash is listed in current RSI. The remaining two events are not listed in current RSI. The causal role of the medication in events Wound secretion, Skin exfoliation and Rash could not be ruled out due to temporal plausibility and pharmacological properties of the Voltaren Gel.

03 Mar 2014: Assessment remains unchanged from the follow-up information received.

Reporter's comment:

NOVARTIS, 11.04.2014 DE-002147023-CHPA2014DE002865

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE002865, is an initial spontaneous report from a pharmacist received on 17 Feb 2014 via Drug commission of German Pharmacists (number [183930]). This report refers to a female patient, either 80 or 81 year old. Historical conditions were not reported. No concomitant medication was reported. The patient received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel (batch/lot number: N05074A) for the treatment of musculoskeletal pain (pain at shoulder) from 24 Jan 2014 at an unknown dose, topically applied 1 to 2 times daily, for the duration of 10 days. On an approximately 29 Jan 2014, the patient reported "weeping of wound" (wound secretion), "skin rash" (rash) and "partly skin peeling of first skin layer" (skin exfoliation). Treatment with Voltaren Schmerzgel Forte was stopped on 02 Feb 2014. Following wound dressing and treatment with Bepanthen ointment the events were reported as improving. The reactions were rated by reporter as transiently significantly impairing. At the time of this report, outcome of the event skin rash (rash), partly skin peeling of first skin layer (skin peeling) and weeping of wound (wound weeping) was reported as condition improved. Seriousness and causality were not provided. No additional information was provided. Follow up information was requested in written form in week 9, 2014.

Follow up information was received from a pharmacist on 25 FEB 2014. This female patient developed 'skin rash with partly skin peeling of first skin layer and weeping of wound' after topical application of Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch no. N05074A) used to treat 'pain at shoulder'. Therapy dates were specified as 'approx 24 Jan 2014 to approx 02 Feb 2014' (continuous treatment). Voltaren Schmerzgel forte was stopped. Following drug treatment with wound dressing for weeping wound with subsequent treatment with Bepanthen ointment adverse symptoms lasted all together for 2 to 3 weeks. The reporter ticked the box that the reaction was not treated by a physician. Causality rating of reporter was probable. The reporter ticked the box Outcome as recovery.

Significant follow up report received on 09 Apr 2014 which consisted of technical complaint investigation results (11159999): No defect observed during bulk batch manufacturing or packaging process. At release, analysis were conform and retest done on retention samples are also compliant. The case is not confirmable from a technical point of view. However, a trend is detected for this issue on bulk 145275 and currently under investigation: PR 130392: COMP Trend analysis on ADR Voltaren 2 percent. Case will be investigated by DSP. No additional information was available.

Sender's comment:

The event Rash is listed in current RSI. The remaining two events are not listed in current RSI. The causal role of the medication in events Wound secretion, Skin exfoliation and Rash could not be ruled out due to temporal plausibility and pharmacological properties of the Voltaren Gel.

03 Mar 2014: Assessment remains unchanged from the follow-up information received.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14063910

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Dry skin	20140207			22 Day	recovered/resolved
2 Skin scaly	20140207			22 Day	recovered/resolved
3 Skin inflammation	20140207			22 Day	recovered/resolved
4 Skin rash	20140207			22 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN EMULGEL diclofenac diethylamine	Topical	/ 95	20140117-20140217	Knee pain

Drugs (concomitant)

2 ENALAPRIL enalapril	Unknown	/	-	Hypertension
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Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypertension			Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 04.03.2014 DE-002147023-CHPA2014DE003284

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE003284, is an initial spontaneous report from a pharmacist received on 25 Feb 2014. This report refers to a male patient of unknown age. Concurrent conditions included hypertension. Concomitant medications included enalapril. The patient received Voltaren Emulgel (diclofenac diethylammonium salt) for the treatment of knee pain (arthralgia) from 27 Jan 2014 at an unknown dose for the duration of 14 days. It was unknown if the product was taken under medical advice. On 07 Feb 2014, 12 days. after the first dose of the suspect drug the patient reported "skin rash and red skin spots at both shinbones" (rash), "dry skin", "skin scaly and peeling" (skin exfoliation), "inflamed skin areas (approx 5 cent coin large)" (dermatitis). Treatment with Voltaren Emulgel was stopped on 09 Feb 2014. The patient's treatment included Cetirizin and Linola Fett. The events were ongoing at the time of report. The seriousness and causality was not reported. Follow up information requested in written form in week 10, 2014.

Sender's comment:

The causal role of the medication in the events Rash, Dermatitis could not be ruled out due to temporal plausibility and pharmacological properties of the drug

Reporter's comment:

NOVARTIS, 04.03.2014 DE-NOVNCHHQ-CHPA2014DE003284

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE003284, is an initial spontaneous report from a pharmacist received on 25 Feb 2014. This report refers to a male patient of unknown age. Concurrent conditions included hypertension. Concomitant medications included enalapril. The patient received Voltaren Emulgel (diclofenac diethylammonium salt) for the treatment of knee pain (arthralgia) from 27 Jan 2014 at an unknown dose for the duration of 14 days. It was unknown if the product was taken under medical advice. On 07 Feb 2014, 12 days. after the first dose of the suspect drug the patient reported "skin rash and red skin spots at both shinbones" (rash), "dry skin", "skin scaly and peeling" (skin exfoliation), "inflamed skin areas (approx 5 cent coin large)" (dermatitis). Treatment with Voltaren Emulgel was stopped on 09 Feb 2014. The patient's treatment included Cetirizin and Linola Fett. The events were ongoing at the time of report. The seriousness and causality was not reported. Follow up information requested in written form in week 10, 2014.

Sender's comment:

The causal role of the medication in the events Rash, Dermatitis could not be ruled out due to temporal plausibility and pharmacological properties of the drug

Reporter's comment:NOVARTIS, 10.03.2014 DE-002147023-CHPA2014DE003284

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE003284, is an initial spontaneous report from a pharmacist received on 25 Feb 2014. This report refers to a male patient of unknown age. Concurrent conditions included hypertension. Concomitant medications included enalapril. The patient received Voltaren Emulgel (diclofenac diethylammonium salt) for the treatment of knee pain (arthralgia) from 27 Jan 2014 at an unknown dose for the duration of 14 days. It was unknown if the product was taken under medical advice. On 07 Feb 2014, 12 days. after the first dose of the suspect drug the patient reported "skin rash and red skin spots at both shinbones" (rash), "dry skin", "skin scaly and peeling" (skin exfoliation), "inflamed skin areas (approx 5 cent coin large)" (dermatitis). Treatment with Voltaren Emulgel was stopped on 09 Feb 2014. The patient's treatment included Cetirizin and Linola Fett. The events were ongoing at the time of report. The seriousness and causality was not reported. Follow up information requested in written form in week 10, 2014.

Follow up information was received from a pharmacist on 06 MAR 2014. Updated: duration of reaction, therapy dates and concomitant drugs. This male patient (age 82-83 years old) developed the initially reported reactions after topical application of Voltaren Emulgel (diclofenac diethylamine salt) used to treat pre-existing 'knee pain'. Therapy dates were specified as '17 Jan 2014 to 17 Feb 2014' (continuous treatment). Voltaren Emulgel was stopped. Following drug treatment with Cetirizin and Linola Fett by treating physician adverse symptoms lasted all together for 10 days. The reporter ticked the box 'Outcome: unknown'. Due to the stated limited duration of the reactions the outcome was internally rated as 'recovery'. No additional information was provided.

Sender's comment:

The causal role of the medication in the events Rash, Dermatitis could not be ruled out due to temporal plausibility and pharmacological properties of the drug

Reporter's comment:NOVARTIS, 11.03.2014 DE-NOVNCHHQ-CHPA2014DE003284

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE003284, is an initial spontaneous report from a pharmacist received on 25 Feb 2014. This report refers to a male patient of unknown age. Concurrent conditions included hypertension. Concomitant medications included enalapril. The patient received Voltaren Emulgel (diclofenac diethylammonium salt) for the treatment of knee pain (arthralgia) from 27 Jan 2014 at an unknown dose for the duration of 14 days. It was unknown if the product was taken under medical advice. On 07 Feb 2014, 12 days after the first dose of the suspect drug the patient reported "skin rash and red skin spots at both shinbones" (rash), "dry skin", "skin scaly and peeling" (skin exfoliation), "inflamed skin areas (approx 5 cent coin large)" (dermatitis). Treatment with Voltaren Emulgel was stopped on 09 Feb 2014. The patient's treatment included Cetirizin and Linola Fett. The events were ongoing at the time of report. The seriousness and causality was not reported. Follow up information requested in written form in week 10, 2014.

Follow up information was received from a pharmacist on 06 MAR 2014. Updated: duration of reaction, therapy dates and concomitant drugs. This male patient (age 82-83 years old) developed the initially reported reactions after topical application of Voltaren Emulgel (diclofenac diethylamine salt) used to treat pre-existing 'knee pain'. Therapy dates were specified as '17 Jan 2014 to 17 Feb 2014' (continuous treatment). Voltaren Emulgel was stopped. Following drug treatment with Cetirizin and Linola Fett by treating physician adverse symptoms lasted all together for 10 days. The reporter ticked the box 'Outcome: unknown'. Due to the stated limited duration of the reactions the outcome was internally rated as 'recovery'. No additional information was provided.

Sender's comment:

The causal role of the medication in the events Rash, Dermatitis could not be ruled out due to temporal plausibility and pharmacological properties of the drug

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14075647

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	Yes	No	No	No	No	No		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Patientdeath

deathdate:	deathcause: Death unexplained
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin erythema					recovered/resolved
2 Skin inflammation					recovered/resolved
3 Burning skin					recovered/resolved
4 Death unexplained					fatal
5 Skin peeling					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN SCHMERZGEL FORTE diclofenac diethylamine	Topical	1 Df dosage form/ 95	- -	Back pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Pustule			Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 13.03.2014 DE-002147023-CHNY2014DE000857

Sender diagnosis: not reported

Narrative

Case number CHNY2014DE000857, is a spontaneous report from a pharmacist received on 10 Mar 2014 with subsequent follow up received on 11 Mar 2014. This report refers to a female consumer of unknown age. Historical conditions included rash pustular. No concomitant medication was reported. The consumer received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel for the treatment of back pain from an unknown date at a dose of 1 DF QHS. The pharmacist reported that the consumer reported "death unexplained" (death), "skin inflammation of enlarged skin area after topical application" (dermatitis), "skin peeling of of enlarged skin area after topical application" (skin exfoliation), "skin redness of enlarged skin area after topical application" (erythema), "skin burn of enlarged skin area after topical application" (skin burning sensation). At the time of this report, outcome of the event skin redness of enlarged skin area after topical application (skin erythema), skin burn of enlarged skin area after topical application (burning skin) and skin inflammation of enlarged skin area after topical application (skin inflammation) was reported as recovered due to the stated limited duration of the reactions. Upon follow up the patient died on an unknown date. It was unknown if an autopsy was performed. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed the these events death (death) were serious. The pharmacist assessed death unexplained as suspected to be related to treatment with Voltaren Schmerzgel Forte. The AQWA number was reported as Quality complaint No. 11159968.

Sender's comment:

Review of the available data does not allow for a proper causality assessment. Reported event of Death is not assessable.

Reporter's comment:

NOVARTIS, 14.03.2014 DE-NOVNCHHQ-CHNY2014DE000857

Sender diagnosis: not reported

Narrative

Case number CHNY2014DE000857, is a spontaneous report from a pharmacist received on 10 Mar 2014 with subsequent follow up received on 11 Mar 2014. This report refers to a female consumer of unknown age. Historical conditions included rash pustular. No concomitant medication was reported. The consumer received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel for the treatment of back pain from an unknown date at a dose of 1 DF QHS. The pharmacist reported that the consumer reported "death unexplained" (death), "skin inflammation of enlarged skin area after topical application" (dermatitis), "skin peeling of of enlarged skin area after topical application" (skin exfoliation), "skin redness of enlarged skin area after topical application" (erythema), "skin burn of enlarged skin area after topical application" (skin burning sensation). At the time of this report, outcome of the event skin redness of enlarged skin area after topical application (skin erythema), skin burn of enlarged skin area after topical application (burning skin) and skin inflammation of enlarged skin area after topical application (skin inflammation) was reported as recovered due to the stated limited duration of the reactions. Upon follow up the patient died on an unknown date. It was unknown if an autopsy was performed. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed the these events death (death) were serious. The pharmacist assessed death unexplained as suspected to be related to treatment with Voltaren Schmerzgel Forte. The AQWA number was reported as Quality complaint No. 11159968.

Following internal review on 13 Mar 2014 of information received on 10 Mar 2014 and 11 Mar 2014. The female consumers birth year, 1951 was added. The consumer was on a concomitant medication: unspecified neuroleptic drug. The causality and causality statement were ammended. The initially reporting pharmacist rated the death of the patient as not related to the use of Voltaren Schmerzgel forte (diclofenac diethylamine salt). Causality rating of reporter was independent with regard to causal relationship between death of patient and use of Voltaren Schmerzgel forte.

Sender's comment:

Review of the available data does not allow for a proper causality assessment. Reported event of Death is not assessable.

13 Mar 2014: Patients advanced age and the concomitant medication, unspecified neuroleptic could be an alternative explanation for the SAE Death.

Reporter's comment:

NOVARTIS, 11.04.2014 DE-NOVNCHHQ-CHNY2014DE000857

Sender diagnosis: not reported

Narrative

Case number CHNY2014DE000857, is a spontaneous report from a pharmacist received on 10 Mar 2014 with subsequent follow up received on 11 Mar 2014. This report refers to a female consumer of unknown age. Historical conditions included rash pustular. No concomitant medication was reported. The consumer received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel for the treatment of back pain from an unknown date at a dose of 1 DF QHS. The pharmacist reported that the consumer reported "death unexplained" (death), "skin inflammation of enlarged skin area after topical application" (dermatitis), "skin peeling of of enlarged skin area after topical application" (skin exfoliation), "skin redness of enlarged skin area after topical application" (erythema), "skin burn of enlarged skin area after topical application" (skin burning sensation). At the time of this report, outcome of the event skin redness of enlarged skin area after topical application (skin erythema), skin burn of enlarged skin area after topical application (burning skin) and skin inflammation of enlarged skin area after topical application (skin inflammation) was reported as recovered due to the stated limited duration of the reactions. Upon follow up the patient died on an unknown date. It was unknown if an autopsy was performed. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed the these events death (death) were serious. The pharmacist assessed death unexplained as suspected to be related to treatment with Voltaren Schmerzgel Forte. The AQWA number was reported as Quality complaint No. 11159968.

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Additional information received on 09 Apr 2014 in the form of a technical complaint investigation results (11159968): No defect observed during bulk

batch manufacturing or packaging process. At release, analysis were conform and retest done on retention samples are also compliant. The case is not confirmable from a technical point of view. However, a trend is detected for this issue on bulk 145275 and currently under investigation: PR 130392: COMP - Trend analysis on ADR Voltaren 2% Case will be investigated by DSP.

Sender's comment:

Review of the available data does not allow for a proper causality assessment. Reported event of Death is not assessable.

13 Mar 2014: Patients advanced age and the concomitant medication, unspecified neuroleptic could be an alternative explanation for the SAE Death.

Reporter's comment:

NOVARTIS, 11.04.2014 DE-002147023-CHNY2014DE000857

Sender diagnosis: not reported

Narrative

Case number CHNY2014DE000857, is a spontaneous report from a pharmacist received on 10 Mar 2014 with subsequent follow up received on 11 Mar 2014. This report refers to a female consumer of unknown age. Historical conditions included rash pustular. No concomitant medication was reported. The consumer received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel for the treatment of back pain from an unknown date at a dose of 1 DF QHS. The pharmacist reported that the consumer reported "death unexplained" (death), "skin inflammation of enlarged skin area after topical application" (dermatitis), "skin peeling of of enlarged skin area after topical application" (skin exfoliation), "skin redness of enlarged skin area after topical application" (erythema), "skin burn of enlarged skin area after topical application" (skin burning sensation). At the time of this report, outcome of the event skin redness of enlarged skin area after topical application (skin erythema), skin burn of enlarged skin area after topical application (burning skin) and skin inflammation of enlarged skin area after topical application (skin inflammation) was reported as recovered due to the stated limited duration of the reactions. Upon follow up the patient died on an unknown date. It was unknown if an autopsy was performed. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed the these events death (death) were serious. The pharmacist assessed death unexplained as suspected to be related to treatment with Voltaren Schmerzgel Forte. The AQWA number was reported as Quality complaint No. 11159968.

Following internal review on 13 Mar 2014 of information received on 10 Mar 2014 and 11 Mar 2014. The female consumers birth year, 1951 was added. The consumer was on a concomitant medication: unspecified neuroleptic drug. The causality and causality statement were ammended. The initially reporting pharmacist rated the death of the patient as not related to the use of Voltaren Schmerzgel forte (diclofenac diethylamine salt). Causality rating of reporter was independent with regard to causal relationship between death of patient and use of Voltaren Schmerzgel forte.

Additional information received on 09 Apr 2014 in the form of a technical complaint investigation results (11159968): No defect observed during bulk batch manufacturing or packaging process. At release, analysis were conform and retest done on retention samples are also compliant. The case is not confirmable from a technical point of view. However, a trend is detected for this issue on bulk 145275 and currently under investigation: PR 130392: COMP - Trend analysis on ADR Voltaren 2% Case will be investigated by DSP.

Sender's comment:

Review of the available data does not allow for a proper causality assessment. Reported event of Death is not assessable.

13 Mar 2014: Patients advanced age and the concomitant medication, unspecified neuroleptic could be an alternative explanation for the SAE Death.

Reporter's comment:

NOVARTIS, 16.04.2014 DE-NOVNCHHQ-CHNY2014DE000857

Sender diagnosis: not reported

Narrative

Case number CHNY2014DE000857, is a spontaneous report from a pharmacist received on 10 Mar 2014 with subsequent follow up received on 11 Mar 2014. This report refers to a female consumer of unknown age. Historical conditions included rash pustular. No concomitant medication was reported. The consumer received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel for the treatment of back pain from an unknown date at a dose of 1 DF QHS. The pharmacist reported that the consumer reported "death unexplained" (death), "skin inflammation of enlarged skin area after topical application" (dermatitis), "skin peeling of of enlarged skin area after topical application" (skin exfoliation), "skin redness of enlarged skin area after topical application" (erythema), "skin burn of enlarged skin area after topical application" (skin burning sensation). At the time of this report, outcome of the event skin redness of enlarged skin area after topical application (skin erythema), skin burn of enlarged skin area after topical application (burning skin) and skin inflammation of enlarged skin area after topical application (skin inflammation) was reported as recovered due to the stated limited duration of the reactions. Upon follow up the patient died on an unknown date. It was unknown if an autopsy was performed. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed the these events death (death) were serious. The pharmacist assessed death unexplained as suspected to be related to treatment with Voltaren Schmerzgel Forte. The AQWA number was reported as Quality complaint No. 11159968.

Following internal review on 13 Mar 2014 of information received on 10 Mar 2014 and 11 Mar 2014. The female consumers birth year, 1951 was added. The consumer was on a concomitant medication: unspecified neuroleptic drug. The causality and causality statement were amended. The initially reporting pharmacist rated the death of the patient as not related to the use of Voltaren Schmerzgel forte (diclofenac diethylamine salt). Causality rating of reporter was independent with regard to causal relationship between death of patient and use of Voltaren Schmerzgel forte.

Additional information received on 09 Apr 2014 in the form of a technical complaint investigation results (11159968): No defect observed during bulk batch manufacturing or packaging process. At release, analysis were conform and retest done on retention samples are also compliant. The case is not confirmable from a technical point of view. However, a trend is detected for this issue on bulk 145275 and currently under investigation: PR 130392: COMP - Trend analysis on ADR Voltaren 2% Case will be investigated by DSP.

Following internal review on 15 Apr 2014 of information received on 10 Mar 2014, the female's birth year of 1915 was added.

Sender's comment:

Review of the available data does not allow for a proper causality assessment. Reported event of Death is not assessable.

13 Mar 2014: Patients advanced age and the concomitant medication, unspecified neuroleptic could be an alternative explanation for the SAE Death.

Reporter's comment:

NOVARTIS, 16.04.2014 DE-002147023-CHNY2014DE000857

Sender diagnosis: not reported

Narrative

Case number CHNY2014DE000857, is a spontaneous report from a pharmacist received on 10 Mar 2014 with subsequent follow up received on 11 Mar 2014. This report refers to a female consumer of unknown age. Historical conditions included rash pustular. No concomitant medication was reported. The consumer received Voltaren Schmerzgel Forte (diclofenac diethylammonium salt) gel for the treatment of back pain from an unknown date at a dose of 1 DF QHS. The pharmacist reported that the consumer reported "death unexplained" (death), "skin inflammation of enlarged skin area after topical application" (dermatitis), "skin peeling of of enlarged skin area after topical application" (skin exfoliation), "skin redness of enlarged skin area after topical application" (erythema), "skin burn of enlarged skin area after topical application" (skin burning sensation). At the time of this report, outcome of the event skin redness of enlarged skin area after topical application (skin erythema), skin burn of enlarged skin area after topical application (burning skin) and skin inflammation of enlarged skin area after topical application (skin inflammation) was reported as recovered due to the stated limited duration of the reactions. Upon follow up the patient died on an unknown date. It was unknown if an autopsy was performed. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed the these events death (death) were serious. The pharmacist assessed death unexplained as suspected to be related to treatment with Voltaren Schmerzgel Forte. The AQWA number was reported as Quality complaint No. 11159968.

Following internal review on 13 Mar 2014 of information received on 10 Mar 2014 and 11 Mar 2014. The female consumers birth year, 1951 was added. The consumer was on a concomitant medication: unspecified neuroleptic drug. The causality and causality statement were amended. The initially reporting pharmacist rated the death of the patient as not related to the use of Voltaren Schmerzgel forte (diclofenac diethylamine salt). Causality rating of reporter was independent with regard to causal relationship between death of patient and use of Voltaren Schmerzgel forte.

Additional information received on 09 Apr 2014 in the form of a technical complaint investigation results (11159968): No defect observed during bulk

batch manufacturing or packaging process. At release, analysis were conform and retest done on retention samples are also compliant. The case is not confirmable from a technical point of view. However, a trend is detected for this issue on bulk 145275 and currently under investigation: PR 130392: COMP - Trend analysis on ADR Voltaren 2% Case will be investigated by DSP.

Following internal review on 15 Apr 2014 of information received on 10 Mar 2014, the female's birth year of 1915 was added.

Sender's comment:

Review of the available data does not allow for a proper causality assessment. Reported event of Death is not assessable.

13 Mar 2014: Patients advanced age and the concomitant medication, unspecified neuroleptic could be an alternative explanation for the SAE Death.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14110745

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth: privacy	Age: 66 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Crust					recovering/resolving
2 Pustule					recovering/resolving
3 Scales					recovering/resolving
4 Skin wound					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/ 95	201403- -	Shoulder pain

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

[NOVARTIS, 15.04.2014 DE-NOVNCHHQ-CHPA2014DE005213](#)

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE005213, is an initial spontaneous report from a consumer received on 10 Apr 2014. This report refers to a 66 year old male patient. Historical conditions were not reported. No concomitant medication was reported. The patient received Voltaren Schmerzgel (diclofenac diethylammonium salt) emulgel to treat shoulder pain at an unknown dose and developed "pustules, crust forming, and scaling (at arms, legs, feet, back, repectively)" after topical application (rash pustular) (scab) (skin exfoliation). Therapy dates were specified as "Mar 2014 to Apr 2014". Voltaren Schmerzgel was stopped. Following drug treatment with Bepanthen Salbe in self medication, cortisone containing ointment prescribed by treating physician (leading only to slight relief), and Kaliumpermanganat solution to treat the wound skin areas (skin wound) which were still present at date of report the reactions were reported as resolving. The patient was going to consult the treating physician. The patient refused consent that Novartis contacts the treating physician. Outcome: reaction resolving. Follow up request not feasible since no contact data were provided. Case closed with lost to follow up on 10 APR 2014.

Sender's comment:

The events skin Wound, skin exfoliation, rash and scab are reported with a plausible temporal relationship and have appeared at the application site. So, a possible causal role of the medication could not be ruled out for product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14111063

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Male	Birth: privacy	Age: 66 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Crust					recovering/resolving
2 Pustule					recovering/resolving
3 Scales					recovering/resolving
4 Skin wound					recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	/ 95	201403- -	Shoulder pain

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

[NOVARTIS, 15.04.2014 DE-002147023-CHPA2014DE005213](#)

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE005213, is an initial spontaneous report from a consumer received on 10 Apr 2014. This report refers to a 66 year old male patient. Historical conditions were not reported. No concomitant medication was reported. The patient received Voltaren Schmerzgel (diclofenac diethylammonium salt) emulgel to treat shoulder pain at an unknown dose and developed "pustules, crust forming, and scaling (at arms, legs, feet, back, repectively)" after topical application (rash pustular) (scab) (skin exfoliation). Therapy dates were specified as "Mar 2014 to Apr 2014". Voltaren Schmerzgel was stopped. Following drug treatment with Bepanthen Salbe in self medication, cortisone containing ointment prescribed by treating physician (leading only to slight relief), and Kaliumpermanganat solution to treat the wound skin areas (skin wound) which were still present at date of report the reactions were reported as resolving. The patient was going to consult the treating physician. The patient refused consent that Novartis contacts the treating physician. Outcome: reaction resolving. Follow up request not feasible since no contact data were provided. Case closed with lost to follow up on 10 APR 2014.

Sender's comment:

The events skin Wound, skin exfoliation, rash and scab are reported with a plausible temporal relationship and have appeared at the application site. So, a possible causal role of the medication could not be ruled out for product.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14225887

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Female Birth: privacy Age: 74 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Stomatitis	20140524			2 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclo Dispers diclofenac sodium	Oral	3 Df dosage form/Day	20140523-	-

Drugs (concomitant)

2 Actonel plus Calcium calcium carbonate; risedronate sodium	Oral	/	-	Osteoporosis
3 Bisoprolol AbZ 5 mg bisoprolol	Oral	1 Df dosage form/	-	
4 Xalatan ATR latanoprost	Intraocular	/	-	Glaucoma

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14222171

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	Yes		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 72 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin ulceration					recovered/resolved
2 Application site redness	20140611			1 Day	recovered/resolved
3 Pain	20140611			1 Day	recovered/resolved
4 Blisters	20140611			1 Day	recovered/resolved
5 Edema					recovered/resolved
6 Foot fracture					recovered/resolved
7 Open wound	20140611			1 Day	recovered/resolved
8 Burning sensation	20140611			1 Day	recovered/resolved
9 Skin discoloration	20140611			1 Day	recovered/resolved
10 Allergic reaction	20140611			1 Day	recovered/resolved
11 Condition aggravated	20140711			31 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel forte diclofenac diethylamine	Topical	3* /Day 95	20140611-20140612	Pain in heel
			2 Day	

Drugs (concomitant)

2 ALNA OCAS tamsulosin hydrochloride	.4 Mg milligram(s)/	-
3 AMARYL glimepiride	3 Mg milligram(s)/	-
4 ENALAPRIL enalapril	5 Mg milligram(s)/	-
5 GLUCOPHAGE metformin hydrochloride	1000 Mg milligram(s)/	-
6 lovastatin lovastatin	20 Mg milligram(s)/	-
7 LOVASTATIN lovastatin	20 Mg milligram(s)/	-
8 voltaren diclofenac sodium	/	-
9 voltaren dispers diclofenac sodium	/	-
10 voltaren resinat diclofenac resinate	/	-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypercholesterolemia			Yes	
Hepatitis C			Yes	
Diabetes			Yes	Diabetes Mellitus type 2
Allergy to chemicals			Yes	Allergy to preservative substances
Fall			Unknown	Slipped on boat, got caught in engine

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)NOVARTIS, 25.07.2014 DE-NOVNCHHQ-CHPA2014DE009096

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE009096, is a spontaneous report from a pharmacist received on 04 Jul 2014 with subsequent follow up received on 21 Jul 2014. This 72 year old male patient with a history of diabetes mellitus type 2; hepatitis C; hypertension; hypercholesterolemia and allergy to preservative substances had slipped in engine room of his boat, got caught in engine with metal, oil/grease and had suffered from pre existing foot injury with multiple scraps on foot. He applied Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch no. illegible) on 11 Jun 2014 and developed a 'skin reaction at the application site, respectively, with: burning sensation, skin redness, burst open skin, and black discoloration of the open skin areas after 4 days'. It was unknown if the product was taken under medical advice. The reaction was nearer described as: 'approx 5 hours after topical application of Voltaren Schmerzgel forte a moderate burning sensation at the application site occurred; approx 24 hours after topical application blister forming, aggravation of skin redness, hourly aggravation of reactions; skin burst open; moderate pain.' The reactions improved after 5 to 6 days. The indication for use was specified as 'pain after fall at left-sided heel, swelling but no open skin wounds'. Therapy dates were specified as '11 Jun 2014 to 12 Jun 2014' (continuous treatment). The reporter specified the onset date as '11 Jun 2014'. The daily dose was not reported but nearer specified as 'hazelnut large quantity topically applied and rubbed-in three times'; the reporter also stated that Voltaren Schmerzgel forte was topically applied two times. Voltaren Schmerzgel forte was stopped. Concomitantly, the patient received multiple drug therapy: long term medication, respectively: Glucophage (metformin hydrochloride) 1000mg; Amaryl (glimepiride) 3mg; Lovastatin (lovastatin) 20mg; Voltaren Dispers / Resinat (diclofenac); Alna Ocas (tamsulosin hydrochloride) 0.4mg; Enalapril (enalapril maleate) 5mg. The patient consulted the local hospital and was diagnosed and treated as outpatient. On admission the hospital diagnosed an 'extensive oedema and skin ulceration at the foot' (internal hint: partly illegible hand-written note); the radiologist diagnosed in plane xray at 15:13 on 12 Jun 2014 a 'comminuted fracture involving calcaneus with extension into the posterior facet of the subtalar joint': presumed 'probably extension of the fracture into the calcaneal cuboid joint surface'. laboratory test results showed abnormal values for RBC (red blood cell count) 3.86 [4.20 to 5.80] x10¹²/L, hemoglobin 130 [135 to 170] g/L, hematocrit 0.36 [0.40 to 0.50], MPV 8.6 [9.5 to 12.5] x10⁹/L, lymphocytes 0.8 [1.2 to 3-5] x10⁹/L, c reactive protein 20.6 [less than 3.1] mg/L, AST 50 [10 to 38] U/L, creatine kinase 421 [40 to 275] U/L, and for urine analysis: sodium 129 [135 to 145] mmol/L and chloride 94 [95 to 107] mmol/L. Following drug treatment with unspecified 'topically applied medicinal products', cleansing of wound, covering of wound with wound gaze, and bandage by treating surgeon adverse symptoms lasted all together for 5 days. The patient fully recovered from blister on an unknown date, application site erythema on an unknown date, hypersensitivity on an unknown date, burning sensation on an unknown date, wound on an unknown date, skin discolouration on an unknown date, condition aggravated on an unknown date, oedema on an unknown date, skin ulcer on an unknown date, foot fracture on an unknown date and pain on an unknown date. These events condition aggravated (medically significant), blister (medically significant), skin discolouration (medically significant), wound (medically significant), burning sensation (medically significant), hypersensitivity (medically significant), application site erythema (medically significant), oedema (hospitalization), pain (medically significant), foot fracture (hospitalization), skin ulcer (hospitalization) were considered serious by the pharmacist. The pharmacist assessed hypersensitivity, pain, foot fracture, skin ulcer, oedema, condition aggravated, skin discolouration, wound, burning sensation and application site erythema, blister as suspected to be related to treatment with Voltaren Schmerzgel Forte. Causality rating of reporter was probable. A QA investigation was requested (AQWA: 11199076 and 11199077).

Follow up information was received from a pharmacist on 21 Jul 2014: Updated patient data, kind of reaction, duration of reaction, reaction onset, CIOMS criteria, daily dose, indication for use, therapy dates, therapy duration until onset, other relevant history, batch number, concomitant drugs, measures taken, information related to suspected drug and adverse reaction, outcome, causality rating, readministration of drug, and narrative.

Sender's comment:

The foot injury suffered by the patient confounds the causal assessment of the events oedema and pain even though these occurred at the application site. These events are considered as not assessable.

The event Foot fracture is probably related to the foot injury suffered by the patient and is unlikely to be related to the topical gel.

All other events happened at the application site and are reported with a plausible time to onset. These events are considered as suspected.

Reporter's comment:NOVARTIS, 21.08.2014 DE-NOVNCHHQ-CHPA2014DE009096

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE009096, is a spontaneous report from a pharmacist received on 04 Jul 2014 with subsequent follow up received on 21 Jul 2014 and on 15 Aug 2014. This 72 year old male patient with a history of diabetes mellitus type 2; hepatitis C; hypertension; hypercholesterolemia and allergy to preservative substances had slipped in engine room of his boat, got caught in engine with metal, oil/grease and had suffered from pre existing foot injury with multiple scraps on foot. He applied Voltaren Schmerzgel forte (diclofenac diethylamine salt) (batch no. illegible) on 11 Jun 2014 and developed a 'skin reaction at the application site, respectively, with: burning sensation, skin redness, burst open skin, and black discoloration of the open skin areas after 4 days'. It was unknown if the product was taken under medical advice. The reaction was nearer described as: 'approx 5 hours after topical application of Voltaren Schmerzgel forte a moderate burning sensation at the application site occurred; approx 24 hours after topical application blister forming, aggravation of skin redness, hourly aggravation of reactions; skin burst open; moderate pain.' The reactions improved after 5 to 6 days. The indication for use was specified as 'pain after fall at left-sided heel, swelling but no open skin wounds'. Therapy dates were specified as '11 Jun 2014 to 12 Jun 2014' (continuous treatment). The reporter specified the onset date as '11 Jun 2014'. The daily dose was not reported but nearer specified as 'hazelnut large quantity topically applied and rubbed-in three times'; the reporter also stated that Voltaren Schmerzgel forte was topically applied two times. Voltaren Schmerzgel forte was stopped. Concomitantly, the patient received multiple drug therapy: long term medication, respectively: Glucophage (metformin hydrochloride) 1000mg; Amaryl (glimepiride) 3mg; Lovastatin (lovastatin) 20mg; Voltaren Dispers / Resinat (diclofenac); Alna Ocas (tamsulosin hydrochloride) 0.4mg; Enalapril (enalapril maleate) 5mg. The patient consulted the local hospital and was diagnosed and treated as outpatient. On admission the hospital diagnosed an 'extensive oedema and skin ulceration at the foot' (internal hint: partly illegible hand-written note); the radiologist diagnosed in plane xray at 15:13 on 12 Jun 2014 a 'comminuted fracture involving calcaneus with extension into the posterior facet of the subtalar joint': presumed 'probably extension of the fracture into the calcaneal cuboid joint surface'. Laboratory test results showed abnormal values for RBC (red blood cell count) 3.86 [4.20 to 5.80] x10¹²/L, hemoglobin 130 [135 to 170] g/L, hematocrit 0.36 [0.40 to 0.50], MPV 8.6 [9.5 to 12.5] x10⁹/L, lymphocytes 0.8 [1.2 to 3.5] x10⁹/L, c reactive protein 20.6 [less than 3.1] mg/L, AST 50 [10 to 38] U/L, creatine kinase 421 [40 to 275] U/L, and for urine analysis: sodium 129 [135 to 145] mmol/L and chloride 94 [95 to 107] mmol/L. Following drug treatment with unspecified 'topically applied medicinal products', cleansing of wound, covering of wound with wound gaze, and bandage by treating surgeon adverse symptoms lasted all together for 5 days. The patient fully recovered from blister on an unknown date, application site erythema on an unknown date, hypersensitivity on an unknown date, burning sensation on an unknown date, wound on an unknown date, skin discoloration on an unknown date, condition aggravated on an unknown date, oedema on an unknown date, skin ulcer on an unknown date, foot fracture on an unknown date and pain on an unknown date. These events condition aggravated (medically significant), blister (medically significant), skin discoloration (medically significant), wound (medically significant), burning sensation (medically significant), hypersensitivity (medically significant), application site erythema (medically significant), oedema (hospitalization), pain (medically significant), foot fracture (hospitalization), skin ulcer (hospitalization) were considered serious by the pharmacist. The pharmacist assessed hypersensitivity, pain, foot fracture, skin ulcer, oedema, condition aggravated, skin discoloration, wound, burning sensation and application site erythema, blister as suspected to be related to treatment with Voltaren Schmerzgel Forte. Causality rating of reporter was probable. A QA investigation was requested (AQWA numbers: 11199076 and 11199077). Technical complaint investigation results (11199076): Stability data are conform. Batch record review and Retention sample cannot be tested. Trend is investigated under PR 130392. Case cannot be confirmed from a technical point of view. Case will be investigated by DSP. Technical complaint investigation results (11199077): The defect is confirmed on the complaint sample received: only traces are visible. Investigated reveals that the ink was probably correctly printed on the tube, but was erased during utilization: ink does not last sufficiently. Actions are ongoing: CAPA 112275: Study how to ensure that the ink last longer than currently. CC 120898: Implement a new technology of primary printing for laminated tube. CAPA 111255: Installation of a camera to control the primary printing on the tube on line at 100 percent. The safety risk was assessed by DSP.

Follow up information was received from a pharmacist on 21 Jul 2014: Updated patient data, kind of reaction, duration of reaction, reaction onset, CIOMS criteria, daily dose, indication for use, therapy dates, therapy duration until onset, other relevant history, batch number, concomitant drugs, measures taken, information related to suspected drug and adverse reaction, outcome, causality rating, readministration of drug, and narrative.

Additional information received on 15 Aug 2014 which consisted of technical complaint investigation results (11199076 and 11199077).

Sender's comment:

The foot injury suffered by the patient confounds the causal assessment of the events oedema and pain even though these occurred at the application site. These events are considered as not assessable.

The event Foot fracture is probably related to the foot injury suffered by the patient and is unlikely to be related to the topical gel.

All other events happened at the application site and are reported with a plausible time to onset. These events are considered as suspected.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14240787

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Female Birth: privacy Age: 83 Year weight (kg): BMI_int: Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Blistering	20140523		2 Week	1 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	1 Df dosage form/Day	20140523-20140523	Back pain

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14259471

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Female Birth: privacy Age: 28 Year weight (kg): 80 BMI_int: 26.12 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Scales	20140727			2 Day	not recovered/not resolved
2 Pruritus	20140727			2 Day	not recovered/not resolved
3 Skin flushed	20140727			2 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclofenac-Actavis Gel diclofenac sodium	Cutaneous	/	20140726-20140727 2 Day	Contusion of wrist

Primary source

Qualification: Pharmacist observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14259590

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		No

Patient

Initials:	Sex: Female	Birth: privacy	Age: 31 Year	weight (kg): 55	BMI_int: 21.76	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vesicles	20140819			5 Day	recovered/resolved
2 Redness	20140819			5 Day	recovered/resolved
3 Itching	20140819			5 Day	recovered/resolved
4 Burning sensation	20140820			6 Day	recovered/resolved
5 Application site reaction	20140818			4 Day	recovered/resolved
6 Skin swelling	20140819			5 Day	recovered/resolved
7 Lack of drug effect					Unknown
8 Condition aggravated					Unknown
9 Papulopustular rash	20140819			5 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN SPRAY 4% diclofenac sodium	Topical	/	20140815-20140817 3 Day	Pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hypothyroidism			Yes	
Penicillin allergy			Yes	Allergy to Amoxicillin

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 27.08.2014 DE-NOVNCHHQ-CHPA2014DE011090

Sender diagnosis: not reported

Narrative

Initial report from a consumer received at German affiliate DSP on 21 AUG 2014. This (age not reported) female patient used Voltaren Spray (diclofenac sodium) for not reported indication and complained about 'no real improvement of pre existing pain', therefore, she stopped the treatment with Voltaren Spray. Therapy dates were specified as '15 Aug 2014 to 17 Aug 2014'. Daily dose was not reported but nearer specified as 'total dose: 5 topical applications between 15 Aug 2014 to 17 Aug 2014'. Since 18 Aug 2014 she detected 'impure skin at shoulder and upper back region at the application site. On 19 Aug 2014 she developed 'red skin spots with a lot of small vesicles at her neck'. On 20 Aug 2014 'the reactions aggravated more and more with spreading of reactions, local skin swelling, itching, and burning sensation'. The reporter provided 6 pictures showing the skin reaction at the shoulder and neck region (internal description of skin reaction: papulo pustular skin rash with prominent skin redness especially at rear side and left side of her back of neck). Following drug treatment with cortisone ointment by treating family doctor, the outcome was not reported. The case was forwarded to the QA dept. (AQWA pending). Follow up information requested in written form in week 35, 2014.

Sender's comment:

The event Application site reaction is reported with a plausible temporal relationship and this event is biologically plausible. A possible causal role of the medication is likely.

Reporter's comment:

NOVARTIS, 04.09.2014 DE-NOVNCHHQ-CHPA2014DE011090

Sender diagnosis: not reported

Narrative

Case number# CHPA2014DE011090, is a spontaneous report from a consumer received on 21 Aug 2014 with subsequent follow up received on 29 Aug 2014. This report refers to female patient of unknown age. Historical conditions were not reported. No concomitant medication was reported. The patient received Voltaren Spray 4% (diclofenac sodium) spray (batch/lot number: 149935-21 (expiration date: 30 Sep 2016) for the treatment of pain from 15 Aug 2014 at a dose of unknown dose (topical) for the duration of 3 days. On 18 Aug 2014, 4 days after the first dose of the suspect drug the patient developed "impure skin at application site" (application site reaction). On 19 Aug 2014, 5 days after the first dose of the suspect drug the patient developed "red skin spots with small vesicles at neck" (erythema). On 20 Aug 2014, 6 days after the first dose of the suspect drug the patient developed "burning sensation", itching (pruritus) and "papulo pustular skin rash" (rash), "local skin swelling" (skin swelling). On an unknown date the patient developed "no real improvement of pre existing pain" (drug ineffective), "reactions aggravated" (condition aggravated). The patient treatment included Cortisone. The outcome of the event application site reaction, erythema, rash, skin swelling, pruritus, burning sensation, condition aggravated and drug ineffective is unknown. The event application site reaction was not assessed for seriousness by the consumer. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed application site reaction (medically significant) as serious. The seriousness assessment for erythema, rash, burning sensation, pruritus, skin swelling, condition aggravated, drug ineffective was not reported by the consumer. The consumer assessed application site reaction as suspected to be related to treatment with Voltaren Spray 4%. The AQWA number was reported as Quality complaint No. 11208668.

Follow-up information was received from consumer on 29 Aug 2014. Updated manufacturer information and AQWA number updated.

Sender's comment:

The event Application site reaction is reported with a plausible temporal relationship and this event is biologically plausible. A possible causal role of the medication is likely.

Reporter's comment:

NOVARTIS, 17.09.2014 DE-NOVNCHHQ-CHPA2014DE011090

Sender diagnosis: not reported

Narrative

Case number# CHPA2014DE011090, is a spontaneous report from a consumer received on 21 Aug 2014 with subsequent follow up received on 29 Aug 2014 and on 11 Sep 2014. This 31 year old female adult patient developed 'skin rash with vesicles like in case of acid burn' after topical application of Voltaren Spray (diclofenac sodium) (batch no. 14993521) used to treat pre existing 'pain at shoulder and neck region subsequently after unpleasant massage'. The reaction was nearer described as 'on 18 Aug 2014 she detected 'impure skin at shoulder and upper back region at the application site (application site reaction). On 19 Aug 2014 she developed 'a red skin spot (erythema). at her neck which got larger and larger with subsequent forming of vesicles (blisters) at neck and shoulder blades'; 'the intensity and the affected skin area increased more and more'; 'the reactions improved not until use of the second cortisone ointment and after 7 days of treatment with cortisone ointment'. On 19 Aug 2014, the patient developed "burning sensation", itching (pruritus) and "papulo pustular skin rash" (rash), "local skin swelling" (skin swelling). On an unknown date the patient developed "no real improvement of pre existing pain" (drug ineffective), "reactions aggravated" (condition aggravated). The reporter specified the therapy dates as '21 Aug 2014 to 03 Sep 2014' (unclear data) but also stressed that Voltaren Spray was topically sprayed and rubbed in for 3 consecutive days and that the last topical administration of Voltaren Spray was on 17 Aug 2014 (internally rated as medical treatment of reported side reactions conducted from 21 Aug 2014 to 03 Sep 2014; therapy dates corrected to initially reported dates: 15 Aug 2014 to 17 Aug 2014). The reporter specified the reaction onset date as '18 Aug 2014'. Voltaren Spray was stopped. Following drug treatment with 'cortisone ointments' by treating physician nearer specified as 4 days with Dermatop Creme (prednicarbate) and 10 days with Nerisona Creme 0.1% (diflucortolone valerate) by physician adverse symptoms lasted all together for 2.5 weeks. Causality rating of reporter was probable. The reporter ticked the box 'Outcome: unknown'. Due to the stated limited duration of the reactions the outcome was internally rated as 'recovery'. The case was forwarded to the QA dept. The AQWA number was reported as Quality complaint No. 11208668.

Follow up information was received from consumer on 29 Aug 2014. Updated manufacturer information and AQWA number updated.

Follow up information was received from a consumer on 11 Sep 2014: Updated: patient data, kind of reaction, duration of reaction, reaction onset, indication for use, therapy dates, other relevant history, information related to suspected drug and adverse reaction, causality rating and readministration of drug.

Sender's comment:

The event Application site reaction is reported with a plausible temporal relationship and this event is biologically plausible. A possible causal role of the medication is likely.

Reporter's comment:

NOVARTIS, 07.11.2014 DE-NOVNCHHQ-CHPA2014DE011090

Sender diagnosis: not reported

Narrative

Case number# CHPA2014DE011090, is a spontaneous report from a consumer received on 21 Aug 2014 with subsequent follow up received on 29 Aug 2014 and on 11 Sep 2014 and follow up reports in the form of technical complaint investigation results (AQWA numbers 11208668 and 11208669) received on 06 Nov 2014. This 31 year old female adult patient developed 'skin rash with vesicles like in case of acid burn' after topical application of Voltaren Spray (diclofenac sodium) (batch no. 14993521) used to treat pre existing 'pain at shoulder and neck region subsequently after unpleasant massage'. The reaction was nearer described as 'on 18 Aug 2014 she detected 'impure skin at shoulder and upper back region at the application site (application site reaction). On 19 Aug 2014 she developed 'a red skin spot (erythema). at her neck which got larger and larger with subsequent forming of vesicles (blisters) at neck and shoulder blades'; 'the intensity and the affected skin area increased more and more'; 'the reactions improved not until use of the second cortisone ointment and after 7 days of treatment with cortisone ointment'. On 19 Aug 2014, the patient developed "burning sensation", itching (pruritus) and "papulo pustular skin rash" (rash), "local skin swelling" (skin swelling). On an unknown date the patient developed "no real improvement of pre existing pain" (drug ineffective), "reactions aggravated" (condition aggravated). The reporter specified the therapy dates as '21 Aug 2014 to 03 Sep 2014' (unclear data) but also stressed that Voltaren Spray was topically sprayed and rubbed in for 3 consecutive days and that the last topical administration of Voltaren Spray was on 17 Aug 2014 (internally rated as medical treatment of reported side reactions conducted from 21 Aug 2014 to 03 Sep 2014; therapy dates corrected to initially reported dates: 15 Aug 2014 to 17 Aug 2014). The reporter specified the reaction onset date as '18 Aug 2014'. Voltaren Spray was stopped. Following drug treatment with 'cortisone ointments' by treating physician nearer specified as 4 days with Dermatop Creme (prednicarbate) and 10 days with Nerisona Creme 0.1% (diflucortolone valerate) by physician adverse symptoms lasted all together for 2.5 weeks. Causality rating of reporter was probable. The reporter ticked the box 'Outcome: unknown'. Due to the stated limited duration of the reactions the outcome was internally rated as 'recovery'. The case was forwarded to the QA dept. The technical complaint investigation results stated (AQWA number 11208668): No further actions required. The complaint is not explicable from analytical/technical point of view. Severeness: The risk for the patient have to be evaluated from our DSP. Occurrence: It is an isolated case. Detectability: Not detectable, because no sample available. There is no risk for the business/market and no need for further escalation. The technical complaint investigation results stated (AQWA number 11208669): No further actions required. The complaint is not explicable from analytical/technical point of view. Severeness: The risk for the patient have to be evaluated from our DSP. Occurrence: The occurrence is low. Detectability: Not detectable, because no sample available. There is no risk for the business/market and no need for further escalation.

Follow up information was received from consumer on 29 Aug 2014. Updated manufacturer information and AQWA number updated.

Follow up information was received from a consumer on 11 Sep 2014: Updated: patient data, kind of reaction, duration of reaction, reaction onset, indication for use, therapy dates, other relevant history, information related to suspected drug and adverse reaction, causality rating and readministration of drug.

Follow up information received on 06 Nov 2014 in the form of technical complaint investigation results (AQWA numbers 11208668 and 11208669). QA results updated.

Sender's comment:

The event Application site reaction is reported with a plausible temporal relationship and this event is biologically plausible. A possible causal role of the medication is likely.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14278671

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Female Birth: privacy Age: 44 Year weight (kg): 63 BMI_int: 24.61 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Tongue swelling non-specific	20140728			5 Day	recovering/resolving
2 Numbness localized	20140728			5 Day	not recovered/not resolved
3 Oral mucosal irritation	20140728			5 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren forte schmerzgel diclofenac diethylamine	Topical	3 Df dosage form/Day	20140724-20140731	Fall
			8 Day	

Primary source

Qualification: Pharmacist

observestudytype:

Literature:

case: DE-BFARM-14278749

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Initials: Sex: Male Birth: privacy Age: 83 Year weight (kg): 73 BMI int: 21.1 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Epidermolysis bullosa	2014	0815			not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel		/	-	
diclofenac diethylamine			2 Year	

Drugs (concomitant)

2	digimerck	/	-
	digitoxin		-
3	ferro sanol	/	-
	ferroglycine sulfate complex		-
4	finasteride	/	-
	finasteride		-
5	furosemid	/	-
	furosemide		-
6	marcumar	/	-
	phenprocoumon		-
7	metformin	/	-
	metformin		-
8	omeprazol	/	-
	omeprazole		-
9	ramipril	/	-
	ramipril		-
10	selegilin	/	-
	selegiline		-
11	spironolactone	/	-
	spironolactone		-
12	tamsulosin	/	-
	tamsulosin		-

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Artificial cardiac pacemaker wearer			Unknown	
Parkinson's disease			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14303836

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 79 Year	weight (kg): 64	BMI_int: 23.8	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Open wound	20140504			3 Day	not recovered/not resolved
2 Skin peeling	20140504			3 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel diclofenac diethylamine	Topical	3* /Day 95	20140502-20140504 3 Day	Joint inflammation

Drugs (concomitant)

2 ASS 100 1A Pharma acetylsalicylic acid	Oral	1 Df dosage form/Day	-	Cerebrovascular accident prophylaxis
3 insuman rapid insulin human	Subcutaneous	/Day	-	Diabetes
4 MetoHEXAL Succ 47,5 mg Retardtabletten metoprolol succinate	Oral	47.5 Mg milligram(s)/Day	-	Hypertension
5 RamiLich 2,5 mg Tabletten ramipril	Oral	2.5 Mg milligram(s)/Day	-	Hypertension

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Type 2 diabetes mellitus	2007		Yes	Type 2 treatment with insuline since 2007
Hypertension			Yes	
Foot injury			k.A.	
Pain			k.A.	
Ankle injury			k.A.	status after acute right-sided ankle of foot injury due to twist of her ankle with pre-existing loca

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

[NOVARTIS, 01.10.2014 DE-002147023-CHPA2014DE013099](#)

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE013099 is an initial spontaneous report from a pharmacist via German Pharmacist's Drug Commission (number [188709]) received on 26 Sep 2014. This report refers to an elderly female patient born in 1934. Medical history was not reported. Concurrent conditions included diabetes mellitus and hypertension. Concomitant medications included Ass (acetylsalicylic acid), Insuman Rapid (insulin human), Metohexal (metoprolol tartrate) and RamiLich (ramipril). The patient received Voltaren Schmerzgel (diclofenac diethylammonium salt) for the treatment of arthritis (joint inflammation) unknown if under medical advice from 02 May 2014 at an unknown dose, TID (topical) for the duration of 3 days. On 04 May 2014 the patient developed "deep open wound" (wound), "spot like skin peeling" (skin exfoliation). Reaction was treated with an unspecified wound treatment by treating physician. Treatment with Voltaren Schmerzgel was temporarily stopped on 04 May 2014. Dechallenge was negative and rechallenge was positive. These events wound, skin exfoliation were not assessed for seriousness by the pharmacist. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed wound (medically significant), skin exfoliation (medically significant) as serious. The pharmacist assessed skin exfoliation, wound as suspected to be related to treatment with Voltaren Schmerzgel.

Sender's comment:

The events Skin exfoliation and Wound are reported with a plausible temporal relationship and rechallenge is positive. A possible causal role of the medication is likely.

Reporter's comment:

NOVARTIS, 01.10.2014 DE-NOVNCHHQ-CHPA2014DE013099

Sender diagnosis: not reported

Narrative

Case number CHPA2014DE013099 is an initial spontaneous report from a pharmacist via German Pharmacist's Drug Commission (number [188709]) received on 26 Sep 2014. This report refers to an elderly female patient born in 1934. Medical history was not reported. Concurrent conditions included diabetes mellitus and hypertension. Concomitant medications included Ass (acetylsalicylic acid), Insuman Rapid (insulin human), Metohexal (metoprolol tartrate) and Ramilich (ramipril). The patient received Voltaren Schmerzgel (diclofenac diethylammonium salt) for the treatment of arthritis (joint inflammation) unknown if under medical advice from 02 May 2014 at an unknown dose, TID (topical) for the duration of 3 days. On 04 May 2014 the patient developed "deep open wound" (wound), "spot like skin peeling" (skin exfoliation). Reaction was treated with an unspecified wound treatment by treating physician. Treatment with Voltaren Schmerzgel was temporarily stopped on 04 May 2014. Dechallenge was negative and rechallenge was positive. These events wound, skin exfoliation were not assessed for seriousness by the pharmacist. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed wound (medically significant), skin exfoliation (medically significant) as serious. The pharmacist assessed skin exfoliation, wound as suspected to be related to treatment with Voltaren Schmerzgel.

Sender's comment:

The events Skin exfoliation and Wound are reported with a plausible temporal relationship and rechallenge is positive. A possible causal role of the medication is likely.

Reporter's comment:

NOVARTIS, 20.10.2014 DE-NOVNCHHQ-CHPA2014DE013099

Sender diagnosis: not reported

Narrative

Case number# CHPA2014DE013099, is a spontaneous report from a pharmacist received on 26 Sep 2014 with subsequent follow up received on 13 Oct 2014. This report refers to an elderly female patient. Medical history included limb injury, joint injury (status after acute right sided ankle of foot injury due to twist of her ankle with pre existing local swelling and pain) and pre existing pain. Concurrent conditions included diabetes mellitus (type 2) and hypertension. Concomitant medications included Ass (acetylsalicylic acid), Insuman Rapid (insulin human), Metohexal (metoprolol tartrate) and Ramilich (ramipril). The patient received Voltaren Schmerzgel (diclofenac diethylammonium salt) for the treatment of arthritis joint inflammation from 02 May 2014 at a dose of unknown dose, TID (topical) for the duration of 3 days. On 04 May 2014, the patient developed "deep open wound" (wound), "spot like skin peeling" (skin exfoliation). Treated with unspecified wound treatment was used. Treatment with Voltaren Schmerzgel was stopped on 04 May 2014. Dechallenge was negative and rechallenge was positive. These events wound, skin exfoliation were not assessed for seriousness by the pharmacist. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed wound (medically significant) and skin exfoliation (medically significant) as serious. The pharmacist assessed skin exfoliation, wound as suspected to be related to treatment with Voltaren Schmerzgel.

Follow up information was received from a pharmacist via German Pharmacist's Drug Commission (number [188709]) on 13 Oct 2014: Updated: kind of reaction, concomitant drugs, and other relevant history.

Sender's comment:

The events Skin exfoliation and Wound are reported with a plausible temporal relationship and rechallenge is positive. A possible causal role of the

medication is likely.

Reporter's comment:

NOVARTIS, 20.10.2014 DE-002147023-CHPA2014DE013099

Sender diagnosis: not reported

Narrative

Case number# CHPA2014DE013099, is a spontaneous report from a pharmacist received on 26 Sep 2014 with subsequent follow up received on 13 Oct 2014. This report refers to an elderly female patient. Medical history included limb injury, joint injury (status after acute right sided ankle of foot injury due to twist of her ankle with pre existing local swelling and pain) and pre existing pain. Concurrent conditions included diabetes mellitus (type 2) and hypertension. Concomitant medications included Ass (acetylsalicylic acid), Insuman Rapid (insulin human), Metohexal (metoprolol tartrate) and Ramilich (ramipril). The patient received Voltaren Schmerzgel (diclofenac diethylammonium salt) for the treatment of arthritis joint inflammation from 02 May 2014 at a dose of unknown dose, TID (topical) for the duration of 3 days. On 04 May 2014, the patient developed "deep open wound" (wound), "spot like skin peeling" (skin exfoliation). Treated with unspecified wound treatment was used. Treatment with Voltaren Schmerzgel was stopped on 04 May 2014. Dechallenge was negative and rechallenge was positive. These events wound, skin exfoliation were not assessed for seriousness by the pharmacist. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed wound (medically significant) and skin exfoliation (medically significant) as serious. The pharmacist assessed skin exfoliation, wound as suspected to be related to treatment with Voltaren Schmerzgel.

Follow up information was received from a pharmacist via German Pharmacist's Drug Commission (number [188709]) on 13 Oct 2014: Updated: kind of reaction, concomitant drugs, and other relevant history.

Sender's comment:

The events Skin exfoliation and Wound are reported with a plausible temporal relationship and rechallenge is positive. A possible causal role of the medication is likely.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Other

case: DE-BFARM-14344932

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes			Yes					Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 90 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin erosion					Unknown
2 Erythema					Unknown
3 Purulent discharge					Unknown
4 Scar					Unknown
5 Wound healing disturbance of					Unknown
6 Crust					Unknown
7 Hyperkeratosis					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 ALDARA	Topical	/	-	Actinic keratosis
imiquimod		17	-	
2 DICLOFENAC SANDOZ	Topical	/	-	Actinic keratosis
diclofenac		95	-	
3 solaraze	Topical	/	-	Actinic keratosis
diclofenac sodium		95	-	

Drugs (concomitant)

4 godamed	/Day	-	Coronary heart disease
acetylsalicylic acid		-	
5 SIMVASTATIN	/Day	-	Hyperlipidemia
simvastatin		-	
6 TRIAMTEREN 50/25	/Day	-	Renal insufficiency
triamterene		-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hyperlipidemia			Unknown	
Hypertension arterial			Unknown	
Coronary heart disease			Unknown	
Renal insufficiency			Unknown	
Androgenetic alopecia			Unknown	
Skin graft			k.A.	
Wound dressing			k.A.	fusidic acid gauze and salve dressings
Biopsy			k.A.	

Primary source

Qualification:	Physician	observestudytype:
Literature:	Kowalzik L. Persistent Disturbance of Wound Healing Following Topical Therapy of Field Cancerization of the Scalp with Imiquimod or Diclofenac. Akt Dermatol. 2014 OCT 29;.	

Narratives (nur bei elektronischem Eingang)

ALM-HQ, 11.11.2014 DE-ALM-HQ-DE-2014-2374

Sender diagnosis: not reported

Narrative

ICSR of serious AE report detected in the medical literature in Germany through our affiliate in Germany: Kowalzik et al. Title: Persistent Disturbance of Wound Healing Following Topical Therapy of Field Cancerization of the Scalp with Imiquimod or Diclofenac. Akt Dermatol. DOI: 10.1055/s-0034-1390877.

A male patient of 90 years of age at the time of this report, with a medical history of Hyperlipidemia, Arterial hypertension, Coronary heart disease, Renal insufficiency and Androgenetic development of bald head stadium IV, applied SOLARAZE 3% gel (diclofenac sodium), topical, dosage and frequency were not provided for Actinic keratosis on largely bald scalp from an unknown date.

No concomitant medications were reported.

On an undisclosed date approximately two years before this report, the patient developed an erosive wound healing disturbance following a topical therapy with SOLARAZE. One year and a half prior this report patient was hospitalized also for exclusion of a squamous carcinoma. Massive, partly yellowish crusts on a 11 × 7 cm area in the apex region were found. After removal of the crusts extensive erosion, partly with hypertrophic granulations, was found. The smear showed growth of *Pseudomonas aeruginosa*. Biopsies of erosive area showed no malignancies but actinic damage. Patient was treated systemically with ciprofloxacin (trade name unknown) and locally with Argentum nitricum (trade name unknown). Then patient was re-admitted with a 13 cm diameter area with erythema, erosions, hyperkeratosis, serous and hemorrhagic crusts with subjacent purulence and scarred parts. Four biopsies again showed a moderate chronic and focally florid inflammation with marked actinic elastosis. In a biopsy, an actinic keratosis was without evidence of malignancy. In a swab abundantly *Pseudomonas aeruginosa* could be found. Patient was treated topically with triamcinolone (trade name unknown), Triclosan (trade name unknown) and eosin solution (trade name unknown) 1%.

At the time of this report, action taken with SOLARAZE was unknown and the event outcome was unknown.

The authors believe that the therapy-related destruction of vellus hair follicles in androgenetic alopecia create an imbalance between a too rapid formation of granulation tissue and a too slow epithelialization, because the vellus hair follicles as starting points for a multifocal epithelialization are missing.

The authors believe that the typical secondary colonization of large sores with pathogens could also be a pathogenetic factor, since bacterial toxins and proteases of granulocytes also interfere with or even undo the re-epithelialization.

ICSR ref. number DE-2014-2374 has been linked in the safety database with ICSR ref. number DE-2014-2415 as related to the same literature article.

Sender's comment:

Corporate drug safety comments (31 Oct 2014):

serious report (hospitalization) concerning a 90 year-old male patient, with a medical history of hyperlipidemia, arterial hypertension, coronary heart disease, renal insufficiency and androgenetic development of bald head stadium IV, who received SOLARAZE (Diclofenac sodium), 3% gel, topical route, dosage and frequency were not provided, for actinic keratosis focused on bald scalp, from an unknown date to an unknown date. Concomitant medications unknown.

On an undisclosed date (approximately 2012), the patient developed an impaired healing of one erosive wound in the bald head. Subsequently the patient was hospitalized in two occasions for worsening of the actinic erosive area with secondary colonization for *Pseudomonas aeruginosa*. Patient was treated systemically with ciprofloxacin and locally with Argentum. During a second phase (i.e. after second hospitalization) patient was treated topically with triamcinolone. Triclosan and eosin solution 1%. Lab results unknown.

At the time of this report, action taken with SOLARAZE was unknown and the event outcome was unknown.

Impaired healing is unlisted in the RSI of SOLARAZE. Despite a temporal relationship exists, there are other alternative explanations for the event. According to the authors the androgenetic alopecia predisposes for slow epithelialization, and this fact could correlate with a secondary colonization, which in turn could would prolong the epithelialization time. Therefore, the company causality is not assessable.

Reporter's comment:

ALM-HQ, 12.11.2014 DE-ALM-HQ-DE-2014-2374

Sender diagnosis: not reported

Narrative

Initial information received on 31 Oct 2014.

ICSR of serious AE report detected in the medical literature in Germany through our affiliate in Germany: Kowalzik et al. Title: Persistent Disturbance

of Wound Healing Following Topical Therapy of Field Cancerization of the Scalp with Imiquimod or Diclofenac. Akt Dermatol. DOI: 10.1055/s-0034-1390877.

A male patient of 90 years of age at the time of this report, with a medical history of Hyperlipidemia, Arterial hypertension, Coronary heart disease, Renal insufficiency and Androgenetic development of bald head stadium IV, applied SOLARAZE 3% gel (diclofenac sodium), topical, dosage and frequency were not provided for Actinic keratosis on largely bald scalp from an unknown date.

No concomitant medications were reported.

On an undisclosed date approximately two years before this report, the patient developed an erosive wound healing disturbance following a topical therapy with SOLARAZE. One year and a half prior this report patient was hospitalized also for exclusion of a squamous carcinoma. Massive, partly yellowish crusts on a 11 × 7 cm area in the apex region were found. After removal of the crusts extensive erosion, partly with hypertrophic granulations, was found. The smear showed growth of *Pseudomonas aeruginosa*. Biopsies of erosive area showed no malignancies but actinic damage. Patient was treated systemically with ciprofloxacin (trade name unknown) and locally with Argentum nitricum (trade name unknown). Then patient was re-admitted with a 13 cm diameter area with erythema, erosions, hyperkeratosis, serous and hemorrhagic crusts with subjacent purulence and scarred parts. Four biopsies again showed a moderate chronic and focally florid inflammation with marked actinic elastosis. In a biopsy, an actinic keratosis was without evidence of malignancy. In a swab abundantly *Pseudomonas aeruginosa* could be found. Patient was treated topically with triamcinolone (trade name unknown), Triclosan (trade name unknown) and eosin solution (trade name unknown) 1%.

At the time of this report, action taken with SOLARAZE was unknown and the event outcome was unknown.

The authors believe that the therapy-related destruction of vellus hair follicles in androgenetic alopecia create an imbalance between a too rapid formation of granulation tissue and a too slow epithelialization, because the vellus hair follicles as starting points for a multifocal epithelialization are missing.

The authors believe that the typical secondary colonization of large sores with pathogens could also be a pathogenetic factor, since bacterial toxins and proteases of granulocytes also interfere with or even undo the re-epithelialization.

ICSR ref. number DE-2014-2374 has been linked in the safety database with ICSR ref. number DE-2014-2415 as related to the same literature article.

Sender's comment:

Corporate drug safety comments (31 Oct 2014):

serious report (hospitalization) concerning a 90 year-old male patient, with a medical history of hyperlipidemia, arterial hypertension, coronary heart disease, renal insufficiency and androgenetic development of bald head stadium IV, who received SOLARAZE (Diclofenac sodium), 3% gel, topical route, dosage and frequency were not provided, for actinic keratosis focused on bald scalp, from an unknown date to an unknown date. Concomitant medications unknown.

On an undisclosed date (approximately 2012), the patient developed an impaired healing of one erosive wound in the bald head. Subsequently the patient was hospitalized in two occasions for worsening of the actinic erosive area with secondary colonization for *Pseudomonas aeruginosa*. Patient was treated systemically with ciprofloxacin and locally with Argentum. During a second phase (i.e. after second hospitalization) patient was treated topically with triamcinolone. Triclosan and eosin solution 1%. Lab results unknown.

At the time of this report, action taken with SOLARAZE was unknown and the event outcome was unknown.

Impaired healing is unlisted in the RSI of SOLARAZE. Despite a temporal relationship exists, there are other alternative explanations for the event. According to the authors the androgenetic alopecia predisposes for slow epithelialization, and this fact could correlate with a secondary colonization, which in turn could would prolong the epithelialization time. Therefore, the company causality is not assessable.

Reporter's comment:

ALM-HQ, 10.12.2014 DE-ALM-HQ-DE-2014-2374

Sender diagnosis: not reported

Narrative

Initial information received on 31 Oct 2014.

ICSR of serious AE report detected in the medical literature in Germany through our affiliate in Germany: Kowalzik et al. Title: Persistent Disturbance of Wound Healing Following Topical Therapy of Field Cancerization of the Scalp with Imiquimod or Diclofenac. Akt Dermatol. DOI: 10.1055/s-0034-1390877.

A male patient of 90 years of age at the time of this report, with a medical history of Hyperlipidemia, Arterial hypertension, Coronary heart disease, Renal insufficiency and Androgenetic development of bald head stadium IV, applied SOLARAZE 3% gel (diclofenac sodium), topical, dosage and frequency were not provided for Actinic keratosis on largely bald scalp from an unknown date.

No concomitant medications were reported.

On an undisclosed date approximately two years before this report, the patient developed an erosive wound healing disturbance following a topical therapy with SOLARAZE. One year and a half prior this report patient was hospitalized also for exclusion of a squamous carcinoma. Massive, partly yellowish crusts on a 11 × 7 cm area in the apex region were found. After removal of the crusts extensive erosion, partly with hypertrophic granulations, was found. The smear showed growth of *Pseudomonas aeruginosa*. Biopsies of erosive area showed no malignancies but actinic damage. Patient was treated systemically with ciprofloxacin (trade name unknown) and locally with Argentum nitricum (trade name unknown). Then patient was re-admitted with a 13 cm diameter area with erythema, erosions, hyperkeratosis, serous and hemorrhagic crusts with subjacent purulence and scarred parts. Four biopsies again showed a moderate chronic and focally florid inflammation with marked actinic elastosis. In a biopsy, an actinic keratosis was without evidence of malignancy. In a swab abundantly *Pseudomonas aeruginosa* could be found. Patient was treated topically with triamcinolone (trade name unknown), Triclosan (trade name unknown) and eosin solution (trade name unknown) 1%.

At the time of this report, action taken with SOLARAZE was unknown and the event outcome was unknown.

The authors believe that the therapy-related destruction of vellus hair follicles in androgenetic alopecia create an imbalance between a too rapid formation of granulation tissue and a too slow epithelialization, because the vellus hair follicles as starting points for a multifocal epithelialization are missing.

The authors believe that the typical secondary colonization of large sores with pathogens could also be a pathogenetic factor, since bacterial toxins and proteases of granulocytes also interfere with or even undo the re-epithelialization.

ICSR ref. number DE-2014-2374 has been linked in the safety database with ICSR ref. number DE-2014-2415 as related to the same literature article.

Follow up information received on 01 Dec 2014:

Information received from the physician.

Regarding the patient's medical history, the dates of onset of hyperlipidemia, arterial hypertension, coronary heart disease, renal insufficiency and androgenetic development of bald head stadium IV were not known.

No information concerning topical route, dosage, frequency and exact therapy dates of SOLARAZE treatment could be provided as Solaraze treatment was done by "outpatient clinic colleague".

The patient was taking concomitantly Simvastatin (trade name unknown), one dosage form daily for hyperlipidemia; GODAMED (acetylsalicylic acid), one dosage form daily for coronary heart disease, TRIAMTEREN (triamterene) 50/25: one dosage form daily for renal insufficiency.

Adverse drug reaction (ADR) started anamnestic under therapy. Reason for first hospitalization was suspicion of spinalioma.

The patient was treated with ciprofloxacin (trade name unknown) and Argentum nigricum (silver nitrate) and was reported that they did not lead to healing of ADR.

At the time of this report, actions taken with Solaraze after the two reported hospitalizations were not known; the outcome of the event was still unknown.

The reporting physician assessed the causal relationship between the event and SOLARAZE as probable.

Sender's comment:

Corporate drug safety comments (31 Oct 2014, 01 Dec 2014): serious report (hospitalization) concerning a 90 year-old male patient, with a medical history of hyperlipidemia, arterial hypertension, coronary heart disease, renal insufficiency and androgenetic development of bald head stadium IV, who received SOLARAZE (Diclofenac sodium), 3% gel, topical route, dosage and frequency were not provided, for actinic keratosis focused on bald scalp, from an unknown date to an unknown date. Concomitant medications included: Simvastatin for hyperlipidemia, Acetylsalicylic acid for coronary heart disease, Triamterene for renal insufficiency.

On an undisclosed date (approximately 2012), the patient developed an impaired healing of one erosive wound in the bald head while treatment with Solaraze. Subsequently the patient was hospitalized (exact dates unknown). The reason of the first hospitalization was suspicion of spinalioma, and in the second was worsening of the actinic erosive area with secondary colonization for *P. aeruginosa*. Patient was treated systemically with ciprofloxacin and locally with Argentum which did not lead to healing of the lesions. During a second phase (i.e. after second hospitalization) patient was treated topically with Triamcinolone, Triclosan and Eosin solution 1%. Lab results unknown.

At the time of this report, action taken with SOLARAZE was unknown and the event outcome was unknown.

Impaired healing is unlisted in the RSI of SOLARAZE. Despite a temporal relationship exists, there are other alternative explanations for the event. According to the authors the androgenetic alopecia predisposes for slow epithelisation, and this fact could correlate with a secondary colonization, which in turn could prolong the epithelisation time. Therefore, the company causality remains not assessable.

Reporter's comment:

SANDOZ, 13.04.2015 DE-SZ09-PHHY2015DE042870

Sender diagnosis: not reported

Narrative

Case number PHHY2015DE042870, is an initial literature report received on 30 Mar 2015. The author discussed about persisting disturbance of wound healing following topical therapy of field cancerisation of the scalp with imiquimod or diclofenac. This report refers to a 78-year-old male patient. The patient had developed multiple actinic keratoses of the scalp and showed markedly hyperkeratotic skin lesions on the scalp under continued treatment with Solaraze (diclofenac) 3% gel on follow-up examination performed 1 year. After 8 months of therapy, he was readmitted with scalp erosion with greasy yellowish coating in parts. He started receiving Aldara (imiquimod) 5% cream following topical treatment with diclofenac. He had developed the non-healing erosion three months prior to his admission. His erosion did not heal following use of dressings containing silver. His punch biopsy detected focally ulcerating inflammation with pronounced actinic elastosis. He underwent local treatment with octenidine and eosin solution, fusidic acid gauze and salve dressings, which led to a rapid reduction in the size of the wound. About 2¼ years following the onset of the disturbance to wound healing, he presented again with a residual wound with central erosion, peripheral crusts surrounded by a reepithelialised margin with appearance of scarring in parts. He underwent successful split-skin graft 6 weeks later.

Sender's comment:

MAC: Based on review of available information the causal role of suspect drug in leading upto the events cannot be ruled out.

Reporter's comment:

SANDOZ, 13.04.2015 DE-SZ09-PHHY2015DE042841

Sender diagnosis: not reported

Narrative

Case number PHHY2015DE042841, is an initial literature report received on 30 Mar 2015. The author discussed about persisting disturbance of wound healing following topical therapy of field cancerisation of the scalp with imiquimod or diclofenac. This report refers to a 90-year-old male patient. The patient received topical Solaraze (diclofenac) 3% gel treatment given for extensive actinic keratoses on his largely bald scalp. the patient developed an erosive disturbance to wound healing. The erosion consisted of comparable area, along with hypertrophic granulation. Skin biopsies of the erosive area showed only signs of actinic damage. He underwent treatment with ciprofloxacin and silver nitrate. He was subsequently readmitted and showed erythemas, erosions, hyperkeratoses, serous and haemorrhagic crusts with underlying pus and areas of scarring. The biopsies revealed a moderate, chronic and focally florid inflammation, with pronounced actinic elastosis. He received topical treatment with triamcinolone, triclosan and eosin. The action taken with the suspect drug and final event outcome were not reported. Seriousness assessment of the event was upgraded based on the information available in the source document. The author commented that, therapy-induced destruction of the small vellus hair follicles in the androgenetic alopecia may play a role since these structures play a role as keratinocyte producers for a multifocal reepithelisation.

Sender's comment:

MAC: Based on review of available information the causal role of suspect drug in leading upto the events cannot be ruled out.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14360921

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 20 Year	weight (kg): 40	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Suicide attempt					Unknown
2 Vision yellow					Unknown
3 Abdominal pain					Unknown
4 Conjunctivitis					Unknown
5 Overdose					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclofenac dura 25 mg	Oral	/	20141112-20141112	
diclofenac		134	1 Day	
2 voltaren	Oral	/	20141112-20141112	
diclofenac sodium		245	1 Day	

Primary source

Qualification: Other Health Professional

observestudytype:

Literature:

Narratives (nur bei elektronischem Eingang)

MYLANLABS, 25.11.2014 DE-MYLANLABS-2014M1010774

Sender diagnosis: not reported

Narrative

Spontaneous report received on 12-Nov-2014.

This case, received from a health professional in Germany, involved a 20-year-old female patient who took Diclofenac dura 25 mg (diclofenac sodium) and Voltaren 25 mg (diclofenac) in an act of suicide attempt and experienced yellow vision, abdominal pain and conjunctivitis due to overdose.

The patient's medical history and concomitant medication were not reported.

Non-company suspect medication included Voltaren (diclofenac).

12-Nov-2014: The patient took an overdose of Diclofenac dura 25 mg (diclofenac sodium) (oral, unknown amount) and Voltaren (diclofenac) (oral, unknown amount) 40 tablets in total in an act of suicidal attempt and was hospitalized.

Unknown date: In hospital patient experienced yellow vision, conjunctivitis and abdominal pain.

Poison Information Centre (GIZ) advised drug screening, CK (creatinine kinase) level control, kidney level control, blood gas analysis, and monitoring and inpatient treatment.

At the time of the report, the outcome of the events was unknown.

Sender's comment:

Serious: Abdominal pain and overdose are listed events of diclofenac as per RSI; causality has been assessed as possible based on the known safety profile of the drug. Suicide attempt, xanthopsia and conjunctivitis are unlisted events of diclofenac as per RSI; causality is unassessable due to lack of information. The contribution of overdose to the events cannot be ruled out.

Reporter's comment:

Serious: Abdominal pain and overdose are listed events of diclofenac as per RSI; causality has been assessed as possible based on the known safety profile of the drug. Suicide attempt, xanthopsia and conjunctivitis are unlisted events of diclofenac as per RSI; causality is unassessable due to lack of

information. The contribution of overdose to the events cannot be ruled out.

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14366004

Serious Death Lifethr. Hospital. Disabling Congenital Other Pregnancy Med. conf.
No

Patient

Initials: Sex: Male Birth: privacy Age: weight (kg): 80 BMI_int: 24.69 Age fetus:

Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vesicles					recovered/resolved
2 Redness					recovered/resolved
3 Wheals					recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren resinat diclofenac resinate	Oral	2 Df dosage form/Day	20141030-20141104 6 Day	

Primary source

Qualification: Physician observestudytype:
Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-14373331

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 87 Year	weight (kg): 72	BMI_int: 23.51	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin flushed	20141104			1 Day	not recovered/not resolved
2 Burning sensation	20141104			1 Day	not recovered/not resolved
3 Blistering	20141104			1 Day	not recovered/not resolved
4 Itching	20141104			1 Day	not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Voltaren Schmerzgel forte diclofenac diethylamine	Topical	/	20141104-20141106 3 Day	Leg pain

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Allergy			Yes	Nickelallergie

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-15052153

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		

Patient

Initials:	Sex: Male	Birth: privacy	Age: 54 Year	weight (kg): 121	BMI_int: 39.97	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Wound	20150115			2 Day	recovering/resolving
2 Blisters	20150115			2 Day	recovering/resolving

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 VOLTAREN SCHMERZGEL FORTE diclofenac diethylamine	Topical	/Day 95	20150114-20150114 1 Day	Calf discomfort

Drugs (concomitant)

2 ENALAPRIL enalapril	Unknown	/	-	Product used for unknown indication
3 HYDROCHLOROTHIAZID DEXCEL hydrochlorothiazide	Unknown	/	-	Product used for unknown indication
4 METFORMIN metformin	Unknown	/	-	Product used for unknown indication
5 VICTOZA liraglutide	Unknown	/	-	Product used for unknown indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Diabetes	2013		Yes	seit zwei Jahren Diabetiker

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 23.02.2015 DE-NOVNCHHQ-CHPA2015DE002603

Sender diagnosis: not reported

Narrative

Initial report from a pharmacist via German Pharmacist's Drug Commission (number [DE AMK 192598]) received on 18 Feb 2015. This 54 year old male adult patient used Voltaren Schmerzgel Forte (diclofenac diethylamine salt) for not reported indication and developed 'bursting open calf of the leg' with 'very large blisters' after topical application. Therapy dates and daily dose were not reported. The patient was hospitalized and was treated with the following medicinal drugs or wound care products: 'Prednisolon Acis 5 g, Penhexal 1.5 Mega, Zetovit (suction compress), Pantoprazol activis 40 mg, Mollelast (bandage), Garin (compress), Mollelast (bandage), and Crassolind (ointment compress)'. The reporter added that the skin was normal without any pathological symptom or sign before use of Voltaren Schmerzgel Forte; the reaction was nearer described as: 'water blisters which had developed during the night and which were open at time point of the next morning, no sign for skin ulceration; no previous history of circulatory disorders at the extremities; diabetes present for 2 years (BMI unknown), no known allergy. Outcome: not reported.

Sender's comment:

The lack of information regarding patient medical history and clinical details did not allow a meaningful causality assessment of events Wound and Blister

Reporter's comment:

NOVARTIS, 17.03.2015 DE-NOVNCHHQ-CHPA2015DE002603

Sender diagnosis: not reported

Narrative

Initial report from a pharmacist via German Pharmacist's Drug Commission (number [DE AMK 192598]) received on 18 Feb 2015, with subsequent follow up information received on 11 Mar 2015. This 53 year old male adult patient had a history of diabetes for 2 years (BMI unknown), no previous circulatory disorders at the extremities, and no known allergy. The patient used Voltaren Schmerzgel Forte (diclofenac diethylamine salt) (batch no. P01401A) to treat "hardened calf of leg on recommendation of pharmacist" and developed "bursting open calf of the leg" with "very large blisters" after topical application. He developed "blister filled with water on extended skin area (especially lower leg)" the next morning after topical application of Voltaren Schmerzgel forte. Therapy dates were specified as 14 Jan 2015 to 14 Jan 2015 (noncontinuous treatment). The reporter specified the onset date as 14 Jan 2015 (internally corrected to 15 Jan 2015). Daily dose was not reported but nearer specified as "thin layer rubbed in once in the evening". Voltaren Schmerzgel Forte was stopped. The patient was hospitalized and was treated with the following medicinal drugs or wound care products: "Prednisolon Acis 5 g, Penhexal 1.5 Mega, Zetovit (suction compress), Pantoprazol activis 40 mg, Mollelast (bandage), Garin (compress), Mollelast (bandage), and Crassolind (ointment compress)". The reporter added that the skin was normal without any pathological symptom or sign before use of Voltaren Schmerzgel Forte, the reaction was nearer described as: "water blisters which had developed during the night and which were open at time point of the next morning, no sign for skin ulceration. On follow up received 11 Mar 2015, it was reported that following drug treatment with "antibiotics and bandages by physician" the outcome was reported as reaction resolving. The nonmedical reporter did not mention the hospitalisation again. Causality rating of reporting consumer was probable. Outcome: reaction resolving.

Follow up information was received from a consumer on 11 MAR 2015: Updated: patient data, kind of reaction, reaction onset, daily dose, indication for use, therapy dates, therapy duration until onset, batch number, concomitant drugs, information related to suspected drug and adverse reaction, outcome, causality rating and readministration of drug.

Sender's comment:

The indication used 'hardened calf' and the presence of diabetes are likely confounders. Also, the lack of information regarding patient medical history and clinical details did not allow a meaningful causality assessment of events Wound and Blister

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-15108734

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin reaction				2 Day	not recovered/not resolved
2 Flaking skin				2 Day	not recovered/not resolved
3 Application site discoloration				2 Day	not recovered/not resolved
4 Drug use for unapproved indication	201501				Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren SPRAY 4% diclofenac sodium	Topical	3* /Day	201501-	Arthrosis
2 VOLTAREN SPRAY 4% diclofenac sodium		/	-	Swelling of fingers

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Plasmacytoma			k.A.	with transplantation therapy

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 08.04.2015 DE-NOVNCHHQ-CHPA2015DE004706

Sender diagnosis: not reported

Narrative

Case number# CHPA2015DE004706, is an initial spontaneous report from a pharmacist received on 02 Apr 2015 combined with follow up information received on 02 Apr 2015 from the pharmacist. This report refers to female patient of unknown age. Medical history included plasmacytoma with transplantation therapy. No concomitant medication was reported. The patient received Voltaren Spray 4% (diclofenac sodium) spray for the treatment of pre existing swelling due to arthrosis of thumb saddle joint (osteoarthritis) (off label use) from an unknown date in approximately 3 months ago at an unknown dose TID (topically applied at skin of thumb saddle joint). On an unknown date approximately 2 days after topical application of Voltaren Spray, the patient developed "blue violet skin discolouration like in case of nevus flammeus at the application site" (application site discolouration) with subsequent "spreading of the skin reaction over her wrist" (skin reaction) and "slight flaking of the skin" (skin exfoliation). The reporter specified that there was no local trauma which could have caused the skin reaction (no haematoma) and that there was no itching and no wheal present together with the skin reaction. Following topical treatment of reaction with glucocorticoids, antihistaminics, and dexpathenol the skin reaction was still spreading. The consulted physicians and dermatologists (5 physicians were consulted) rated the causality with regard to the reaction and use of Voltaren Spray as related but also stated that it was known that Voltaren may cause skin reactions but that they had no explanation for the kind and severity of the skin reaction; no diagnosis or tentative diagnosis was reported by the physicians and almost all recommended to conduct a biopsy of the skin reaction which was planned for May 2015. The patient was severely concerned about the planned biopsy of the skin reaction. The patient was going to consult the treating physician again. The outcome of the events was condition unchanged. These events application site discolouration, skin exfoliation, skin reaction were not assessed for seriousness by the pharmacist. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed application site discolouration (medically significant), skin exfoliation (medically significant), skin reaction (medically significant) as serious. Follow up information requested in written form (with consent to contact the treating physician) in week 15, 2015.

Sender's comment:

The events Application site discolouration, Skin reaction and Exfoliation are reported at the application site and with a plausible temporal relationship. A possible causal role of the medication could not be ruled out.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-15152582

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
No								Yes

Patient

Initials:	Sex: Male	Birth: privacy	Age: 79 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Contact dermatitis	20150427		10 Day	15 Day	recovered/resolved
2 Desquamation	20150427		10 Day	15 Day	recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 solaraze diclofenac sodium	Topical	2 Df dosage form/Day	20150413-20150502 20 Day	Keratosi

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Keratosi			Unknown	Aktinische Keratose
Carcinoma in situ			Unknown	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)Drug Commission of the German Medical Association, 21.05.2015 DE-DCGMA-15166330

Sender diagnosis: not reported

Narrative

Maßnahme und Verlauf: Nach Absetzen und topischer Kortisontherapie (Monovo Emulsion) rasche Besserung. Epikutantestung ist vorgesehen für Anfang Juli.

Sender's comment:Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-15162093

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	No	No	Yes		Yes

Patient

Initials:	Sex: Female	Birth: privacy	Age: 49 Year	weight (kg): 75	BMI_int: 31.22	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Broken blood vessel in eye	201504				not recovered/not resolved
2 Earache	201504				not recovered/not resolved
3 Sore throat	201504				not recovered/not resolved
4 Itching	201504				not recovered/not resolved
5 Skin rash	201504				not recovered/not resolved
6 Allergic reaction	201504				not recovered/not resolved
7 Vulvovaginal rash	201504				not recovered/not resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclofenac-ratiopharm 75 mg SL Retardkapseln	Oral	75 Mg milligram(s)/Day	20150413-20150419	
diclofenac		5	7 Day	

Drugs (concomitant)

2 cetirizin	Oral	/	-	Hay fever
cetirizine			-	
3 Momecutan Fettcreme	Topical	/	20150424-20150427	
mometasone furoate			4 Day	
4 prednisolon 20 mg	Oral	/	20150425-20150429	
prednisolone			5 Day	
5 Thomapyrin	Oral	/	-	Headache
acetylsalicylic acid; caffeine; phenacetin			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Hay fever			Yes	HAY FEVER
Headache			Unknown	HEADACHE

Primary source

Qualification: Pharmacist observestudytype:

Literature:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-15212241

Serious Yes	Death	Lifethr.	Hospital.	Disabling	Congenital	Other Yes	Pregnancy	Med. conf.
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Patient

Initials:	Sex: Male	Birth: privacy	Age: 74 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Quincke's oedema	20150625			16 Day	recovered/resolved
2 Generalised urticaria	20150625			16 Day	recovered/resolved
3 Crust	20150625			16 Day	recovered/resolved
4 Desquamation	20150625			16 Day	Unknown
5 Redness	20150625			16 Day	Unknown
6 Swelling	20150625			16 Day	Unknown
7 Localised itching					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 solaraze diclofenac sodium	Topical	/ 95	20150610- -	Actinic keratosis

Drugs (concomitant)

2 ATACAND candesartan cilexetil	/	-	-	Hypertension
3 SIMVASTATIN simvastatin	1 Df dosage form/Day	-	-	

Primary source

Qualification:	Physician	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

ALM-HQ, 08.07.2015 DE-ALM-HQ-DE-2015-1436

Sender diagnosis: not reported

Narrative

Initial information received on 26 Jun 2015.

ICSR of serious AEs (Other serious: important medical event) received from a physician via a company representative in Germany.

A 74-year-old male patient with unknown medical history received SOLARAZE (Diclofenac sodium), dose, frequency, indication for use and period treatment were not provided.

No concomitant medications were provided.

On an unknown date, the patient experienced generalised urticaria, Quincke's oedema on face-eyes and forming of crust in the scalp. It was unknown if the patient received any corrective treatment. No other relevant physical findings or laboratory assessments were provided.

At the time of this report, the action taken with SOLARAZE was withdrawn and the outcome of the events was not recovered.

The reporting physician assessed the seriousness of this report as non-serious and did not provide the causal relationship between SOLARAZE and the events. The company upgraded the case to serious (Other serious: important medical event).

Sender's comment:

Corporate drug safety comments (26 Jun 2015). This report upgrade to serious by the company (important medical event) concerns a 74 year-old male patient with unknown medical history and concomitant medication, who received SOLARAZE (Diclofenac sodium) dose, frequency, indication for

use and period treatment unknown.

On an unknown date (time after start of the treatment with Diclofenac Sodium unknown), the patient experienced generalized urticaria, Quincke's edema on face and eyes and forming of crust in the scalp (lab results, test results and corrective treatment unknown)

Diclofenac sodium was withdrawn on an unknown date. Outcome was not recovered.

Angioedema and urticaria are listed in the RSI of Diclofenac sodium gel. Scab is unlisted. The most causes of angioedema can be: mast cell-mediated etiologies (mast cell-mediated angioedema is associated with urticaria and/or pruritus in most cases), bradykinin-mediated etiologies, and etiologies of unknown mechanism. The scab could be related with urticaria. Relevant information (complete medical history, concomitant medication, dates of the event, corrective treatment and outcome of the events) is not available. Due to lack of data, the company causality for all the events is not assessable.

Reporter's comment:

ALM-HQ, 07.08.2015 DE-ALM-HQ-DE-2015-1436

Sender diagnosis: not reported

Narrative

Initial information received on 26 Jun 2015.

ICSR of serious AEs (Other serious: important medical event) received from a physician via a company representative in Germany.

A 74-year-old male patient with unknown medical history received SOLARAZE (Diclofenac sodium), dose, frequency, indication for use and period treatment were not provided.

No concomitant medications were provided.

On an unknown date, the patient experienced generalised urticaria, Quincke's oedema on face-eyes and forming of crust in the scalp. It was unknown if the patient received any corrective treatment. No other relevant physical findings or laboratory assessments were provided.

At the time of this report, the action taken with SOLARAZE was withdrawn and the outcome of the events was not recovered.

The reporting physician assessed the seriousness of this report as non-serious and did not provide the causal relationship between SOLARAZE and the events. The company upgraded the case to serious (Other serious: important medical event).

Follow information received on 31 Jul 2015.

The patient medical history included a "questionable" medical reaction in 2012 probably due to a statin and/or an unspecified drug for high blood pressure.

The patient was taking concomitantly ATACAND (candesartan) and simvastatin (trade name unknown), one dosage form daily for years.

SOLARAZE 3%, gel, was applied topically on scalp (thinly), in the evenings, for multiple, flat, actinic, partly proliferative keratosis ("distinct light damage of the scalp" (half bald)), from 10 Jun 2015.

The Quincke's edema was developed massively in the eyes and lids, the scab formation was developed in the head and face and additionally reddening, swelling, desquamation and itchiness on arms were reported. The onset date for the itchiness was not reported and for the rest of the events was 25 Jun 2015.

No laboratory tests were performed.

The patient used as a treatment URBASON (methylprednisolone) tablet, TELFART (fexofenadine) and other antihistaminics (trade name unknown) for urticaria; DERMATOP (prednicarbate), locally, for Quincke's oedema and eye ointments with cortisone (trade name unknown) for scab formation.

The outcome of the events was recovered for urticaria, Quincke's oedema and scab formation and unknown for reddening, swelling, desquamation and itchiness on arms.

Sender's comment:

Corporate drug safety comments (26 Jun-31 Jul 2015). This report upgraded to serious by the company (IME) concerns a 74 y-o male patient with medical history of "questionable" medical reaction in 2012 probably due to a statin and/or an unspecified antihypertensive drug, who received SOLARAZE 3%, gel (Diclofenac sodium). It was applied on scalp (thinly), in the evenings, for multiple, flat, actinic, partly proliferative keratosis ("distinct light damage of the scalp" (half bald)), from 10 Jun 2015. Concomitant medication: candesartan and simvastatin (one dosage form daily for years). On 25 Jun 2015, the patient experienced generalized urticaria, Quincke's edema on the eyes and lids, scab formation on the head and face, reddening, swelling and desquamation. On an unknown date, the patient experienced itchiness on arms. No tests were performed. Corrective treatment included: methylprednisolone, fexofenadine and other antihistaminics (details unknown), prednicarbate and cortisone. Diclofenac sodium was withdrawn on an unknown date. The outcome for reddening, swelling, desquamation and itchiness on arms was unknown; and for urticaria, Quincke's oedema and scab formation was recovered.

Angioedema, urticaria, erythema and pruritus are listed in the RSI of company's Diclofenac sodium containing Solaraze gel. Swelling is not listed as such, but is regarded as listed under oedema in the RSI. Scab and skin exfoliation are unlisted. The most causes of angioedema can be: mast cell-mediated etiologies (mast cell-mediated angioedema is associated with urticaria and/or pruritus in most cases), bradykinin-mediated etiologies and etiologies of unknown mechanism. The scab and skin exfoliation could be related with urticaria. Although relevant information (complete medical history, and outcome of some events) is not available, taking into account the follow-up information, the temporal relationship and the biological plausibility, the company causality for all the events is possible.

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-15321283

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	No	Yes	No	Yes		No

Patient

Initials:	Sex: Female	Birth: privacy	Age: 72 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Skin peeling					recovered/resolved
2 General physical condition abnormal					not recovered/not resolved
3 Diabetes mellitus					recovered/resolved
4 Inappropriate schedule of drug administration	20150915			1 Day	Unknown
5 Drug use for unapproved indication	20150915			1 Day	Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren emulgel diclofenac diethylamine	Topical	/Day 95	20150915-20150921 7 Day	Arthrosis

Drugs (concomitant)

2 AMLODIPIN DEXCEL amlodipine maleate	Unknown	5 Mg milligram(s)/Day	2014- -	Product used for unknown indication
3 CALCIUM +VIT D calcium; colecalciferol	Unknown	/	2014- -	Product used for unknown indication
4 L-THYROXIN-HENNING levothyroxine sodium	Unknown	75 µg microgram(s)/Day	- -	Product used for unknown indication
5 M-BETA morphine sulfate	Unknown	/	- -	Product used for unknown indication

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Wheelchair user			Yes	
Asthma			Yes	
Osteoporosis			Yes	
Psoriasis			Yes	
Thyroid disorder			Yes	

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

NOVARTIS, 07.10.2015 DE-NOVNCHHQ-CHPA2015DE011667

Sender diagnosis: not reported

Narrative

Case number# CHPA2015DE011667, is an initial report from a consumer received on 02 Oct 2015. This report refers to female patient of unknown age. Medical history was not reported. No concomitant medication was reported. This female patient of unknown age, resident in a retirement home and previously mobile without need for a wheelchair, was treated with Voltaren Emulgel (diclofenac diethylamine salt) to treat preexisting "pain in right sided foot" prescribed by orthopaedist and developed "severe skin peeling" (skin peeling), with unspecified "worsening of preexisting health status" (general physical condition abnormal) and was subsequently "confined to a wheelchair" starting on 6th day after topical application. Therapy dates and daily dose were not reported. Dosage and mode of application according to recommendations and applied by skilled personnel. The outcome of the event was not reported. These events skin exfoliation and general physical condition abnormal were not assessed for seriousness by the consumer. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed skin exfoliation (medically significant) and general physical condition abnormal (disability) as serious. The causality of the events was not reported by the consumer.

Sender's comment:

Considering that the skin exfoliation seems to have happened at the application site, the event and consequent worsening of physical condition are considered to be suspected to be due to the drug.

Reporter's comment:

NOVARTIS, 08.10.2015 DE-002147023-CHPA2015DE011667

Sender diagnosis: not reported

Narrative

Case number# CHPA2015DE011667, is an initial report from a consumer received on 02 Oct 2015. This report refers to female patient of unknown age. Medical history was not reported. No concomitant medication was reported. This female patient of unknown age, resident in a retirement home and previously mobile without need for a wheelchair, was treated with Voltaren Emulgel (diclofenac diethylamine salt) to treat preexisting "pain in right sided foot" prescribed by orthopaedist and developed "severe skin peeling" (skin peeling), with unspecified "worsening of preexisting health status" (general physical condition abnormal) and was subsequently "confined to a wheelchair" starting on 6th day after topical application. Therapy dates and daily dose were not reported. Dosage and mode of application according to recommendations and applied by skilled personnel. The outcome of the event was not reported. These events skin exfoliation and general physical condition abnormal were not assessed for seriousness by the consumer. In the absence of reported seriousness by the HCP, Novartis safety physician has assessed skin exfoliation (medically significant) and general physical condition abnormal (disability) as serious. The causality of the events was not reported by the consumer.

Sender's comment:

Considering that the skin exfoliation seems to have happened at the application site, the event and consequent worsening of physical condition are considered to be suspected to be due to the drug.

Reporter's comment:

NOVARTIS, 23.10.2015 DE-NOVNCHHQ-CHPA2015DE011667

Sender diagnosis: not reported

Narrative

Case number# CHPA2015DE011667, is an initial report from a consumer received on 02 Oct 2015 with subsequent follow up received on 09 Oct 2015. This 72 year old female elderly patient resident in a retirement home developed 'skin peeling' after topical application of Voltaren Emulgel (diclofenac diethylamine salt) used to treat 'arthrosis at foot prescribed by orthopaedist'; in follow up the patient did not mention again that she was bound to wheelchair after use of Voltaren Emulgel. Therapy dates were specified as '15 Sep 2015 to used for 7 days'. Daily dose was not reported but nearer specified as 'topically applied 1x per day'. Voltaren Emulgel was stopped. Following drug treatment with unspecified healing cream and prednisolon acis (prednisolone) 10mg from 08 Oct 2015 to ongoing (under monitoring in hospital) prescribed by orthopaedist the patient recovered completely. Causality rating of reporting consumer was probable. With regard to the reported elective hospitalisation the patient did not check the box 'involved or prolonged inpatient hospitalisation' in the follow up report form and added that a 'newly diagnosed diabetes mellitus' resulted in the elective hospitalisation and that the treatment with prednisolon acis (prednisolone) 10mg was prescribed by treating orthopaedist and due to the hospitalisation subsequently monitored by the clinicians in hospital (internal hint: due to the reported causality of the hospitalisation the CIOMS criterion 'involved or prolonged inpatient hospitalisation' was not checked by local Affiliate). Outcome: recovery.

Follow up information was received from a consumer on 09 Oct 2015. Updated: patient data, kind of reaction, daily dose, indication for use, therapy dates, other relevant history, concomitant drugs, measures taken, information related to suspected drug and adverse reaction, outcome, causality rating and readministration of drug.

Sender's comment:

The event Skin exfoliation is assessed as suspected based on temporal plausibility. The events Diabetes mellitus and General physical condition abnormal are assessed as unassessable due to the lack of information

Reporter's comment:

NOVARTIS, 23.10.2015 DE-002147023-CHPA2015DE011667

Sender diagnosis: not reported

Narrative

Case number# CHPA2015DE011667, is an initial report from a consumer received on 02 Oct 2015 with subsequent follow up received on 09 Oct 2015. This 72 year old female elderly patient resident in a retirement home developed 'skin peeling' after topical application of Voltaren Emulgel (diclofenac diethylamine salt) used to treat 'arthrosis at foot prescribed by orthopaedist'; in follow up the patient did not mention again that she was bound to wheelchair after use of Voltaren Emulgel. Therapy dates were specified as '15 Sep 2015 to used for 7 days'. Daily dose was not reported but nearer specified as 'topically applied 1x per day'. Voltaren Emulgel was stopped. Following drug treatment with unspecified healing cream and prednisolon acis (prednisolone) 10mg from 08 Oct 2015 to ongoing (under monitoring in hospital) prescribed by orthopaedist the patient recovered completely. Causality rating of reporting consumer was probable. With regard to the reported elective hospitalisation the patient did not check the box 'involved or prolonged inpatient hospitalisation' in the follow up report form and added that a 'newly diagnosed diabetes mellitus' resulted in the elective hospitalisation and that the treatment with prednisolon acis (prednisolone) 10mg was prescribed by treating orthopaedist and due to the hospitalisation subsequently monitored by the clinicians in hospital (internal hint: due to the reported causality of the hospitalisation the CIOMS criterion 'involved or prolonged inpatient hospitalisation' was not checked by local Affiliate). Outcome: recovery.

Follow up information was received from a consumer on 09 Oct 2015. Updated: patient data, kind of reaction, daily dose, indication for use, therapy dates, other relevant history, concomitant drugs, measures taken, information related to suspected drug and adverse reaction, outcome, causality rating and readministration of drug.

Sender's comment:

The event Skin exfoliation is assessed as suspected based on temporal plausibility. The events Diabetes mellitus and General physical condition abnormal are assessed as unassessable due to the lack of information

Reporter's comment:

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-16004713

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes						Yes		

Patient

Initials:	Sex: Female	Birth: privacy	Age: 75 Year	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Vesicles	20151220		3 Day		recovered/resolved
2 Pruritus	20151220		3 Day		recovered/resolved
3 Allergic skin reaction	20151220		3 Day		recovered/resolved

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 voltaren schmerzgel forte 23.2 mg/g diclofenac diethylamine	Topical	2* /Day	-	Joint pain
			-	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Arthrosis			Unknown	
Hypertension			Unknown	

Primary source

Qualification:	Pharmacist	observestudytype:
Literature:		

FIRST LOOK

Country: Deutschland

Type: Spontaneous report

case: DE-BFARM-16046667

Serious	Death	Lifethr.	Hospital.	Disabling	Congenital	Other	Pregnancy	Med. conf.
Yes	No	No	Yes	No	No	No		No

Patient

Initials:	Sex: Female	Birth: privacy	Age:	weight (kg):	BMI_int:	Age fetus:
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Reactions

MedDRA LLT	Begin	End	duration	TtO	outcome
1 Contact eczema					Unknown
2 Conjunctivitis					Unknown
3 Face oedematous					Unknown
4 Fever					Unknown
5 Pain					Unknown
6 Burning sensation					Unknown
7 Nausea					Unknown

Drugs (suspected or interacting)

drug name (substance)	route	dose/ pharmac. form	Begin-End/ Durat.	indication
1 Diclo-ratiopharm Schmerzgel	Topical	/	-	Knee pain
diclofenac		95	3 Week	

Medical History and concurrent conditions

MedDRA-term	Begin	End	Cont.	Comment
Knee pain			Unknown	KNEE PAIN

Primary source

Qualification:	Consumer or other non health professional	observestudytype:
Literature:		

Narratives (nur bei elektronischem Eingang)

TEVA, 10.02.2016 DE-TEVA-632843GER

Sender diagnosis: not reported

Narrative

03-Feb-2016, Spontaneous, Patient/Consumer Serious report.

A female patient patient of unknown age reported about herself. She received Diclo-ratiopharm Schmerzgel (DICLOFENAC, Teva product).

The patient took Diclo-ratiopharm Schmerzgel for KNEE PAIN (DICLOFENAC, Gel, Topical) on a great area for a duration of about 3 weeks.

Since two weeks, the patient experienced a BURNING SENSATION OF KNEES, WHOLE BODY, FOOT SOLES, FACE, UPPER ABDOMEN(Serious); NAUSEA(Serious); BODY TEMPERATURE OF 38.2 DEGREES AT NIGHT(Serious). She additionally reported to have a CONTACT ECZEMA (Serious) and a CONJUNCTIVITIS(Serious). Her FACE OEDEMATOUSLY SWOLLEN (Serious) and she felt PAIN WHILE LYING (Serious) . Due to these symptoms, the patient was admitted to a hospital.

At the time of the report the outcome of the AEs were: Unknown.
 Action taken with suspect drugs: Diclo-ratiopharm Schmerzgel - Drug discontinued.

The patient had medical history of KNEE PAIN(Unknown if Continuing).

The patient's concomitant medication were unspecified.
 The patient's past medication were unspecified.

Lab tests
 body temperature: 38.2 Celsius

This case was considered serious based on the following criteria: (Hospitalization Required)

Teva Comment: Contribution of DICLOFENAC to BURNING SENSATION OF KNEES, WHOLE BODY, FOOT SOLES, FACE, UPPER ABDOMEN and additionally reported NAUSEA, BODY TEMPERATURE OF 38.2 DEGREES AT NIGHT, CONTACT ECZEMA, CONJUNCTIVITIS, FACE OEDEMATOUSLY SWOLLEN and PAIN WHILE LYING cannot be excluded: possibly related. Clinical information including results derived from differential diagnostics are missing and, since no data on underlying diseases or concomitant medication was provided, alternative causative reasons are conceivable.

Because this is a spontaneous case, regulatory distribution will be handled as though it is a related case.

Sender's comment:

Reporter's comment:

number of reports: 322